

THE TECHNIQUE OF PSYCHOTHERAPY FOURTH EDITION LEWIS R.
WOLBERG, M.D.

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Among these are (1) the availability of new psychopharmacological agents that are singularly effective for symptoms of anxiety, panic, obsessive reactions, depression, mania and schizophrenia; (2) a greater use of short-term approaches not only because third party payers have placed limitations on the number of reimbursable sessions, but also because brief therapy has established its validity beyond traditional limited goals; (3) a shift toward the sicker end of the spectrum of patients seeking help in outpatient clinics and private practice due to the reduction of inpatient facilities, and emphasis on short-term hospitalization, deinstitutionalization, and many other factors; (4) the continuing development of creative symptom-oriented and problemsolving, supportive, behavioral, milieu and outreach approaches as well as family, marital, and group treatments; (5) greater efforts toward more scientific treatment planning with emphasis on differential therapeutics; (6) expanding experimentation in reconstructive therapy through methods oriented around object relations theory and self-psychology, which have led to changes in the way the more severe personality problems are

being conceptualized.

While the field of psychotherapy is virtually bursting with luxuriant techniques and expanding dimensions of application, little attempt has been made to integrate the information that we have available today.

This poses a problem for students learning ways of managing emotional illness.

Added to their befuddlement is the persistent fervor of some established groups to maintain ideological purity.

Sugary pieties about the need for unity have not succeeded in halting adversarial verbal dueling.

What we have come to realize more and more is that psychotherapy is not a homogeneous operation, being burdened by many contradictory variables.

It is not only the actual interventions that determine the outcome, but also the degree to which the www.freepsychotherapybooks.org 4 patient accepts and utilizes them, the skill of the therapist, and the climate of the therapeutic alliance.

Countless snags and resistances arise in treatment that can sabotage the efforts of the most dedicated therapists and best motivated patients.

And many nonspecific factors influence therapeutic results for the good and bad.

Yet there is a possibility that all good therapists operate in somewhat similar ways if they are to achieve worthwhile results, and this is irrespective of the theories they espouse and the explanations they offer of what they do.

When the original edition of this book was published in 1954, partial solutions for some of the quandaries in psychotherapy were offered.

I had acquired a bulk of data of how therapists with different orientations conducted psychotherapy that registered a positive effect on their patients.

The locus of this information was a community health center in New York City, the Postgraduate Center for Mental Health, which my wife, a psychiatric social worker and clinical psychologist, and I founded in 1945 under the name The New York Consultation Center.

The Center provided a laboratory for study, and the relatively large numbers of patients being treated served as a rich resource for empirical observations.

Our case load in the early days consisted of a sizeable number of civilian patients as well as veterans of World War II who had come back from the European and Asian theaters and who were sent to us for treatment as a contract clinic of the Veterans Administration.

In a short while we were deluged with veterans being discharged from the service, and to treat this mass of emotionally disturbed patients as well as our habitual caseload of civilians in need of help, we had to enlarge our staff greatly.

World War II having ended, the army medical officers with psychiatric training were going back into private practice.

Drawing on this group, we developed a large staff of psychiatrists and a few clinical psychologists, all of whom had been trained in diverse methods of psychotherapy and belonged to practically every school of psychological thought existing at the time.

Although their theoretical belief systems varied widely, and their focus on pathology differed, we soon discovered that there were those who obtained good results and those who did not.

We tried to substantiate what it was that successful therapists did.

www.freepsychotherapybooks.org 5 Our initial hypothesis was that good therapists, irrespective of discipline, established rapid rapport with their patients, displaying empathy in dealing with expressed emotion.

They developed a theory about the patient's illness and around this formulated a treatment plan involving the patient in at least some of the decision making.

They took the time to resolve unreasonable expectations and to enhance motivation for therapy.

They adapted their techniques to the needs of each patient, e.g. utilization of structured approaches with confused and helpless patients and less structured ones with more highly organized individuals.

They did not hesitate to employ confrontation with patients with whom they had a good relationship in order to create a better learning medium through affect arousal.

They worked within the framework of the patient's belief systems where possible, but when these interfered with treatment they tactfully attempted to alter distorted cognitions.

Analytically oriented therapists did not hesitate to work toward symptom alleviation where symptoms were self-defeating, diverting the patient from selfobservation.

Aware of transferential projections, they controlled their own untoward countertransferential feelings.

The more intuitive therapists employed their personal feelings to divine what was on the patient's mind, conscious and unconscious.

They aggressively dealt with evasions and resistances, recognizing the carry-over of defenses and value distortions from the past.

They provided reinforcements for constructive behavior.

Finally, where the goal was not personality reconstruction, which was most often the case, they terminated therapy at a point where the patient could reasonably manage on his or her own in order to avoid a therapeutic stalemate in dependency.

All of these maneuvers were done irrespective of cherished theories.

As the Center expanded, it developed an organized interdisciplinary training program in psychotherapy and psychoanalysis, obtaining a charter for this from the Board of Regents of the State of New York.

It became one of the largest outpatient continuous treatment centers in the country, servicing a steady caseload of over 1600 patients in its Adult Clinic, Clinic for Children and Adolescents, Social Rehabilitation Clinic, Group Therapy Clinic, Family Therapy Clinic and Clinic for Alcoholism.

Its clinics were licensed by the New York State Department of Mental Hygiene and approved by the Joint Commission on Accreditation of Psychiatric Facilities of the AMA.

Apart from the www.freepsychotherapybooks.org 6 Fellowship program in psychotherapy and psychoanalysis, training programs for qualified personnel were developed within each of the clinics in child psychiatry, analytic group therapy, family therapy, supervision of the therapeutic process, and alcoholism.

The Department of Community Services and Education developed a counseling service and a training program in counseling for social workers, clergymen, teachers, rehabilitation workers, and other personnel dealing with problems encountered in their work.

A twoyear part-time mental health consultation course for certified psychotherapists enabled the servicing of over 600 agencies, institutions, and industrial groups in the community.

As part of public education an ongoing lecture series was developed for the general public.

Continuing education programs for mental health professionals rounded out the activities of the Center.

All of the foregoing activities enabled detailed observation of aspects related to the practice of psychotherapy and, coupled with studies of cases in my own private practice, inspired later editions of this book.

Because of the wide range of its coverage and step-by-step delineation of techniques, the book became known as the "cookbook of psychotherapy," and was also referred to as a treatment manual.

However, it was never intended as either a "cookbook" or treatment manual with standard recipes of operation.

Too many variables exist in the continuum of patient-therapist-environment to allow for universal formulas applicable for all patients and therapists under differing environmental circumstances.

Although the methods described in the book have been tested, they are best utilized as guidelines to be altered to the needs of individual patients and the styles and personalities of practicing therapists.

Throughout the book allusions have been made to a dynamic way of thinking about what goes on in psychotherapy.

The reason for this is that even though the methods employed by the therapist may be non-analytic, one sometimes cannot escape intrapsychic interferences such as those that issue out of the pool of early developmental conditionings.

The form of these interferences are embodied in transferential corruptions and manifold resistances, identification of which would seem to fall within the premises of www.freepsychotherapybooks.org 7 psychoanalysis.

Although in the past few decades we have witnessed the erosion of psychoanalysis as a preferred method of treatment, this has not lessened its value in understanding operative forces in psychotherapy.

“Psychoanalytically oriented psychotherapy” accordingly has become the most common form of psychotherapy in use today.

The psychotherapist who has not been trained as an analyst may still be able to identify transference and resistance, and one may recognize some countertransferential responses within oneself.

In utilizing psychoanalytic concepts in treatment, as in the identification and resolution of resistance, the therapist may still be effective even though not practicing formal psychoanalysis.

Since over 90 percent of patients cannot afford, or tolerate, or constructively utilize intensive psychoanalysis, psychotherapy will constitute the best approach for most patients. Blending dynamics with a skilled use of techniques, the therapist may enable the patient, not only to achieve symptomatic relief, but also to gain sufficient self-understanding to effectuate some important reconstructive personality changes.

The present volumes detail methods through which such changes may be accomplished. It is impossible to include all of the sources I have drawn on for help in the research and writing of this new edition.

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www.freepsychotherapybooks.org 9 Part I.

The Scope, Types, and General Principles of Psychotherapy www.freepsychotherapybooks.org 10 1
What Is Psychotherapy?

Few words in the lexicon of the mental health field are as ambiguous as the term psychotherapy. It is loosely employed to connote, among other meanings, helping, treating, advising, guiding, educating, and even influencing.

Definitions of psychotherapy are often bridled to fields of disciplinary operation, e.g., psychiatry, psychology, casework, etc., sanctuaries for such characterizations being sought in specialized societies.

Diffuseness, in definition, has converted the arena of psychotherapy into a swamp of murky ideas, fostering many divergent theories and techniques.

Yet a brief and precise description of therapy is important if no more than to circumscribe boundaries of operation and for purposes of hypothetical construction and empirical study.

A comprehensive working definition might be the following: Psychotherapy is the treatment, by psychological means, of problems of an emotional nature in which a trained person deliberately establishes a professional relationship with the patient with the object of (1) removing, modifying, or retarding existing symptoms, (2) mediating disturbed patterns of behavior, and (3) promoting positive personality growth and development.

This formulation requires additional elaboration.

Psychotherapy is the treatment.

No matter how much we attempt to dilute what we do in psychotherapy, it constitutes a form of treatment.

Such terms as reeducation, helping process, and guidance are merely descriptive of what happens in the course of treatment and do not really disguise the therapeutic nature of the process.

Forms of intervention other than therapy do exist in the mental health field and will be described later.

Psychological means.

Psychotherapy is a generic term covering the entire spectrum of psychological treatment methods.

These range from designed maneuvers of the therapist-patient relationship to indoctrinations fashioned to change value systems, to www.freepsychotherapybooks.org 11 tactics aimed at intrapsychic processes, and to conditioning techniques that attempt to alter neural mechanisms.

The repertoire of strategies is thus legion, and formats are varied, e.g., individuals, couples, and groups.

They are all, nevertheless, dependent upon the establishment of adequate communication, verbal and nonverbal.

Excluded are such modalities as somatic therapies (drugs, convulsive therapy, surgery, etc.) and "trial action" therapies such as occupational therapy, dance therapy, music therapy, psychodrama, etc.

that, though psychotherapeutic in effect, are not, strictly speaking, forms of psychotherapy.

Problems of an emotional nature.

Emotional problems are diverse, influencing every facet of human functioning.

They are manifest in distortions in the individual's psychic, somatic, interpersonal, and community life.

Manifestations of emotional illness are thus multiple, involving the total human being. In view of this totality of disturbance, it is arbitrary and unsound to dissociate psychic from interpersonal, social, and psychophysiological difficulties, aspects of which are usually concurrent, though not always obvious.

A trained person.

In search of relief, the individual is apt to involve oneself in a relationship with a friend or authority.

The motivations that prompt such a relationship are disabling symptoms or a realization that one's happiness and productivity are being sabotaged by inner forces that one is neither able to understand nor to control.

Sometimes the consequences of this relationship are registered in a restoration of homeostasis, a product of healing forces liberated by the helping process.

At other times, particularly when attempts are made to handle the sufferer's emotional turmoil in depth, the relationship may become disastrous to both participants.

Dealing most adequately with an emotional problem requires a high degree of skill that may best be acquired through extensive postgraduate training and experience.

Deliberately establishing a professional relationship.

The relationship, the core of the therapeutic process, is deliberately planned and nurtured by the therapist.

Unlike nonprofessional relationships, which are part of the social nature of man, the therapeutic relationship is a collaborative undertaking, started and maintained on a professional level toward specific therapeutic objectives.

More than one therapist (co-therapist, multiple therapists) may work together.

The patient.

An individual in psychotherapy receiving treatment is best called a patient rather than some other designation such as a client.

The therapist may relate capably to more than one patient during a session, as in marital or group therapy.

The object is removing existing symptoms.

A prime goal in therapy is to eliminate the patient's suffering as well as to remove the handicaps imposed by symptoms.

Modifying existing symptoms.

Despite our wish for complete relief, certain circumstances may militate against this objective.

Chief deterrents are inadequate motivation, diminutive ego strength,¹ and limitations in the patient's available time or finances.

These will impose restrictions on the extent of help that can be rendered and make for modification rather than cure of the patient's symptoms.

Retarding existing symptoms.

There are some malignant forms of emotional illness, such as certain fulminating schizophrenic and organic brain disorders, in which psychotherapy, no matter how adroitly applied, serves merely to delay an inevitable deteriorative process.

This palliative effect is eminently worthwhile, however, often helping to preserve the patient's contact with reality.

Mediating disturbed patterns of behavior.

The recognition in recent years that many occupational, educational, marital, interpersonal, and social problems are emotionally inspired has extended the use of psychotherapy into fields hitherto considered provinces of the psychologist, teacher, sociologist, religious leader, and lawmaker.

Realization that the character structure is involved in all emotional illness has broadened the objectives of psychotherapy from mere symptom relief or removal to correction of disturbed interpersonal patterns and relationships.

Promoting positive personality growth and development.

The final use of www.freepsychotherapybooks.org 13 psychotherapy is as a vehicle for personality maturation.

This has introduced a new dimension into the field of psychotherapy—a dimension that deals, on the one hand, with problems of immaturity of the so-called normal person and, on the other, with characterologic difficulties associated with inhibited growth previously considered inaccessible to treatment.

Here psychotherapy aims at a resolution of blocks in psychosocial development in order that the individual may aspire to a more complete creative self-fulfillment, more productive attitudes toward life, and more gratifying relationships with people.

The goals of psychotherapy thus extend from the limited objective of helping to control symptoms to the liberation of the rich resources of the human mind from neurotic obstructions that thwart its purpose and stunt its growth.

IS THE MEDICAL MODEL OUTMODED?

Therapists who venture into rendering essential human services in mental health will inevitably find that they must penetrate into zones alien to the medical model.

Of necessity, they will have to adapt to concepts and interventions that are more related to the social than to the biological sciences.

For example, if one seeks to engage in preventive work or in some forms of short-term therapy, it is necessary to become familiar with the dynamics of social systems and the world of learning and rehabilitative procedures.

The information one already possesses of biological and psychological systems will, of course, be fed into these areas in a way that adapts one to the exigencies with which the therapist must deal.

Unless the therapist is willing to restrict operations to the handling of a narrow band of problems, models of helping will have to be employed that go beyond training in traditional psychotherapeutic education.

Whether we need to call such types of helping psychotherapy is a moot question.

Shall we restrict the word to the treatment of outright emotional illness, or shall we extend it to dealing with behavioral and adjustment problems that are not conspicuously pathological?

A good deal of the confusion about psychotherapy stems from the fact that over the years psychological help has been extended to larger and larger groups of people with increasingly diverse complaints.

These range from physiological disruptions of www.freepsychotherapybooks.org 14 anxiety involving almost any organ, to depressive manifestations, to phobias and compulsions, to psychotic dislocations, to habit disorders, to behavioral aberrations, to interpersonal difficulties, to marital and family disturbances, to educational and work blockages, to sexual malfunctions, to addictions, and to a host of other vexations that can plague human beings. The models most appropriate for the understanding of and the dealing with this plethora of troubles must be spread over a number of fields.

Indeed, in the course of helping the same individual we may have to apply information from medical, sociological, educational, rehabilitative, and other sources.

In a way, the term psychotherapy is a limiting one because literally it means treatment of mental or nervous disorders.

This suggests a restrictive medical model.

To encompass all the troubles for which psychological help is sought under the term

psychotherapy will necessitate a borrowing from theories and stratagems more appropriate to structures other than medical ones, such as sociological or rehabilitative and learning models. Functionally, this is what has happened over the years, often with protests from professionals in disciplines of sociology, anthropology, psychology, and education, who resent encroachment of their domains.

Actually, psychotherapy should not be regarded as a cormorant intent on swallowing diverse disciplines.

It is a body of procedures that overlap techniques used in counseling, social casework, education, and rehabilitation, even though its goals may be different.

There are advantages and disadvantages to the medical model per se; but, on the whole, it has a proven utility for a bulk of problems seen by the psychotherapist.

It is possible that Third Force psychology embracing humanistic aims, as well as the human growth potential movement, may some day provide a viable alternative.

As matters stand today, we have not yet achieved this goal.

Substituting for the concept of "mental health/mental illness," "different modes of coping with life" does not necessarily lead to greater clarification of the many problems with which we have to deal in psychotherapy.

The fact that therapists work within the orbit of the medical model does not mean that they must propel themselves into an absurdly authoritative position, thus

www.freepsychotherapybooks.org 15 perpetuating a parent-child relationship.

Authoritativeness is more a product of the personality of the individual therapist than the model that an individual pursues in practice.

Nor does it follow that patients need be depreciated by being labeled "sick" or "mentally ill," thus pinning on them awkward pathological labels.

Nor, if we adopt the medical model, need they be deprived of their freedom or liberty, nor robbed of the option of deciding for themselves their suitability for therapy, nor forced to sacrifice responsibility for their own destiny and the right to their own sense of values, nor prevented from being active participants in their own treatment.

The medical model does not necessarily have to restrict the focus purely on disease.

Expanded psychological growth and development are within its purview, much as physical hygiene is within the scope of good medical practice.

Merely because our present nosological systems are not entirely satisfactory does not sanction the abandonment of diagnosis that is an essential aspect of the medical model.

A proper diagnosis can be helpful to the institution of a rational therapeutic program.

For example, violence may appear as a symptom of a variety of causes.

It may be a simple behavior problem nurtured by situational dislocations.

It may be a habitual egosyntonic display in a psychopathic personality.

It may be a manifestation of failing repressive control in a borderline patient.

It may be the expression of a delusional system in a schizophrenic.

Or it may be a symptom of the manic phase of a bipolar disorder.

Unless a correct diagnosis is established, we may fail miserably in providing effective help.

Thus, elimination of violence due to environmental difficulties can be helpful in simple behavioral problems, but it will usually be ineffective in the other conditions cited.

Anyone who has witnessed the ameliorating effect of neuroleptic medication in schizophrenia and of lithium in manic disorders will attest to the value of a diagnostic survey.

Violence in a borderline case will require special psychotherapeutic management that might not be applicable to other conditions.

Diagnosis can be as important in psychological as in physical problems.

On the other hand, a disadvantage fostered by the medical model is that it concerns itself with

techniques of therapeutic intervention that are sometimes dissociated from the
www.freepsychotherapybooks.org 16 daily life of the individual.

Diagnosis and pathology, legitimate as they may be in disease areas, are sometimes not applicable to certain behavioral zones.

To classify these as normal and abnormal or as symptoms and defenses neglects considerations of background, culture, and life style, which require a different perspective.

Moreover, while the medical model has in the past sponsored a limited training perspective, it has rarely equipped trainees to deal with many behavioral difficulties that are disruptive to the individual and the community.

It would seem appropriate then in a comprehensive training program to expand the education of psychotherapists toward a wider understanding of the behavioral sciences and toward the use of a broad range of techniques additional to the conventional psychotherapeutic procedures.

Under these circumstances, the therapists would be better equipped to move beyond the boundaries of the medical model toward a more pragmatic commitment to the spectrum of problems challenging them in their practices.

PSYCHOTHERAPY VERSUS PSYCHOANALYSIS According to Webster's New Universal Unabridged Dictionary, psychotherapy connotes "the application of the various forms of mental treatment, as hypnosis, suggestion, psychoanalysis, etc., to nervous and mental disorders." This generic definition is, in mental health circles, supplemented by a more specific usage of the term in relation to goals and methods of treatment as contrasted with psychoanalysis.

Psychoanalysis aims at a systematic and total resolution of unconscious conflicts with structural alteration of defenses, and the character organization.

Psychotherapy is less ambitious, reaching for the practical and less arduously achieved goals of resolving some conflicts, modifying others, and even retaining and strengthening certain neurotic defenses that permit individuals to contain their anxiety and to function.

This does not necessarily make psychoanalysis a better kind of treatment than psychotherapy or vice versa.

Some patients fail miserably at one and do quite well with the other.

The key issue is the proper selection of cases for the two different techniques.

Patients exposed to formal analysis, centered around evolvment of a transference neurosis and its resolution www.freepsychotherapybooks.org 17 through interpretation, will require so many qualifications in terms of personality characteristics, motivation, available time, finances, etc.

that they are relatively few in number.

Most patients, on the other hand, will qualify for psychotherapy.

Accepting the fact that psychoanalysis has provided us with concepts and techniques that can lead to the recognition and exposure of conflicts that operate beyond the zone of awareness, what is of concern to clinicians is how useful this information is in treating and resolving emotional problems.

Most students are no longer willing to accept psychological theories on the basis of faith or literary elegance.

Some are relatively unimpressed with both the pronouncements and achievements of psychoanalysis, moving toward psychotherapy with its active approaches directed at symptom relief and problem-solving.

Among the expressed doubts are (1) that psychoanalysis is the best treatment for most problems of an emotional nature; (2) that unconscious conflict is necessarily at the root of all emotional difficulties; (3) that every communication of the patient to the therapist during a session has an unconscious meaning, and that through free association one eventually can reach this repudiated core; (4) that verbal unburdening has a greater impact on the individual than

behavioral solutions to a problem; (5) that supportive and educational interventions are temporary and inevitably lead to greater avoidance and repression; (6) that an adequate cure of a neurosis necessitates its duplication in treatment through the relationship with the therapist (transference); (7) that psychoanalytic theories can be validated through either research or careful clinical inquiry; and (8) that psychoanalysis is the only method through which reconstructive change can be achieved.

The upshot of these questions is that students in progressively larger numbers are doubting the clinical usefulness of psychoanalysis and the need for personal training in psychoanalytic techniques, with the sacrifices of time, energy, and money that disciplined analytic studies would entail.

More insidiously dynamic theory, which is useful in understanding psychopathology, is downgraded and credited with little clinical utility and therefore considered not worthy
www.freepsychotherapybooks.org 18 of study.

This is unfortunate since insight into anachronistic coping patterns, so essential in reconstructive treatment, will draw upon certain psychoanalytic concepts such as unconscious ideation, repression, resistance, and the survival in the present of needs and defenses rooted in the development past.

Among some psychoanalysts, psychotherapy is sometimes employed to distinguish a wide variety of superficial supportive and reeducative procedures aimed at more conscious mental processes, from psychoanalysis that supports techniques of "depth therapy" focused on the unconscious. While utilizing methodologies for expediency's sake, which may be condoned under certain circumstances, they consider it a baser metal than the pure gold of analysis.

The latter alone of all therapies is targeted at the surviving nucleus of emotional illness in the residual "infantile neurosis." Psychoanalysis, they say, offers itself as a technique that may, in the cases where it can be successfully employed, promote maturity by eliminating the infantile neurosis as a source of emotional pollution.

Psychotherapy is more modest in its goals.

It can help to strengthen the individual's defenses so as to prevent the infantile neurosis from interfering too much with a reality adaptation.

It can also provide guidelines for more competent coping with everyday stress.

In this way the individual is better able to live with the infantile neurosis and to make an adjustment that is no better or worse than the normal individual who possesses some neurotic defenses, but never sees the inside of a psychotherapist's office and still gets along satisfactorily with life and people.

Tarachow's (1963) differentiation of psychoanalysis from psychotherapy is still serviceable.

He considers that in the former "transference, repression, other ego defenses, and resistances are all freely subjected to analysis and resolved," while "psychotherapy, on the other hand, is a selective, limited treatment in which a rearrangement rather than a resolution of these elements is aimed at." Furthermore, as more and more defections from Freudian theory have occurred, classical (orthodox) Freudians have tended to claim priority for the term psychoanalysis and to label any derivative neo-Freudian method as psychotherapy.

Factional quarrels have accordingly developed.

www.freepsychotherapybooks.org 19 Since psychoanalytic doctrines have permeated into the fiber of mental health practices and theories, the question of where psychoanalysis belongs is an arbitrary one.

A well-trained psychotherapist is usually schooled in analytic doctrines and methods and is capable of applying these as part of the treatment program.

PSYCHOANALYSIS VERSUS PSYCHOANALYTICALLY ORIENTED PSYCHOTHERAPY A good deal of strife among psychoanalysts who adhere to classical theory and those who have deviated in their

ideas and methods centers around the word psychoanalysis.

Classical analysts claim, with some justification, that the term is being watered down to include stratagems that are not even remotely related to psychoanalysis.

They insist that psychoanalysis is restricted to a specific mode of treatment, focused on the unconscious, in which the uncovering of repressed childhood conflicts is achieved through the gradual resolution of resistance.

This aim is accomplished through an intensive therapist-patient relationship, insured by frequent treatment sessions (preferably five times weekly) and the employment of the techniques of free association, dream analysis, and the evolvment and “working through” of a “transference neurosis.” The latter embraces the projected distortions in the therapeutic relationship of traumatic experiences with early parental figures.

The transferential development is guaranteed best by the employment of passive, neutral, anonymous, and nondirective attitudes on the part of the therapist.

This tends to mobilize the most repressed components of conflict (the infantile neurosis) and to permit the patient to work through with a new parental image (as embodied in the therapist) more perspicacious attitudes toward authority, toward oneself, and toward one’s own impulses. Resultant is a modification of the severity of the superego, a releasing of the strangulations that characterize the archaic defensive maneuvers of the ego, and a freeing of the constructive elements of the id.

Since there are patients whose problems, motivations, and life circumstances are such that they are unable to respond to the passive methods of psychoanalysis, and since there are therapists who are unable to function in an anonymous, nondirective, nonauthoritative manner, dictated by classical technique, a number of modifications have www.freepsychotherapybooks.org 20 been devised.

Because these serve some patients effectively, the revisions have been proposed as “refinements” and “improvements” over the orthodox procedures.

The hard core of psychoanalysts connected with the Freudian school have challenged such modifications, considering them a reversion to preanalytic methods or a blocking of the true aims of psychoanalysis—which deals intensively with the unconscious and the noninterfering resolution of the transference neurosis.

When we examine the therapeutic tactics of the schools that advocate modifications, we do find a diversion from classical aims and techniques of psychoanalysis— such as the institution of activity in the relationship, a reduction in the number of weekly sessions, a substitution of the focused interview for free association, a consideration in the interview of the present rather than the past, the introduction of adjunctive devices, the proffering of suggestions and directives, coordinate interviews when necessary with other family members, and even the restraining of the development of a transference neurosis.

While Freudian analysts generally do not object to these innovations, and even admit that they may serve some patients better than formal psychoanalysis, they do object strenuously to the labeling of these newer techniques as psychoanalysis.

Neo-Freudians, on the other hand, object to the narrow definition of psychoanalysis defined by the Freudians.

After all, they insist Freud himself defined psychoanalysis as any method that dealt with resistance and transference.

Why circumscribe it to a special kind of orientation?

Freud (1952) persistently contended that no person was a psychoanalyst who did not accept the foundations of the theory of psychoanalysis, namely the existence of unconscious mental processes, the recognition of the theory of repression and resistance, and the importance of sexuality and the Oedipus Complex.

Some authorities now employ the term “psychoanalytically oriented psychotherapy” or “dynamic psychotherapy” to those approaches that accept some, but not all of Freud’s premises.

Others consider a psychoanalytically oriented psychotherapy the modulated utilization of the techniques of psychoanalysis that circumvent or dilute a transference

www.freepsychotherapybooks.org 21 neurosis, and that deal with the understanding in dynamic terms of a limited area of the pathology.

However, because the word psychoanalysis carries with it connotations of “depth” and “thoroughness” and because there is still a status and economic advantage in some parts of the country in being known as a psychoanalyst and in doing psychoanalysis, therapists resist debasing their activities with a term that might be interpreted as second best.

Where psychoanalysis ends and psychoanalytically oriented psychotherapy begins has become a matter of opinion.

One may remember the fruitless struggle of the American Psychoanalytic Association to establish, through its Committee on Evaluation of Psychoanalytic Therapy, a baseline from which to approach the contradistinction between psychoanalysis and psychoanalytic psychotherapy.

The effort bogged down, and a report was issued to the effect that even investigation of possible differences mobilized resistance among the members of the Society (Rangell, 1954).

Orthodox Freudians insist that psychoanalysis presupposes an acceptance of the instinct theory (libido theory) and the primacy of early sexual conflicts.

They contend that deviants of cultural or sociological theoretical persuasions are simply not doing psychoanalysis, even though there is an employment of free association and dreams, a delving into childhood conditionings, an uncovering and resolution of resistance, and a setting up and working through by means of interpretation of a transference neurosis.

This position is gradually being softened with the current interest in cognition and “ego psychology,” which considers behavior too complex to be accounted for solely by innate psychological events.

There is then a tendency to embrace as psychoanalysis only those treatment processes that (1) are executed by trained psychoanalysts, (2) have as their goal the overcoming of resistances to unconscious conflicts, whatever their nature may be, (3) deal with a continuity of experience back to early childhood, and (4) encourage the building up of a transference neurosis and its resolution through interpretation.

Techniques that diverge from these methodologic objectives, and yet employ some of the uncovering www.freepsychotherapybooks.org 22 procedures for delving into the unconscious, such as dreams, free association, and analysis of resistance and transference (which is not to be confused with the more intense transference neurosis) are best classified as

“psychoanalytically oriented psychotherapy,” or “dynamic psychotherapy,” or “exploratory psychotherapy.” OTHER DEFINITIONS OF PSYCHOTHERAPY The comprehensive definition of psychotherapy given above has many advantages.

However, other explanations of the meaning of psychotherapy exist that may be interesting to review.

The sundry published definitions of psychotherapy agree on one point— namely, that psychotherapy constitutes a form of approach to many problems of an emotional nature.

They do not agree on other aspects, such as the techniques employed, the processes included, the goals approximated, or the personnel involved.

Typical definitions are these: 1.

“Psychotherapy is the formal treatment of patients using psychological rather than physical or chemical agents, principally verbal communication.” 2.

“Psychotherapy may be defined as the treatment of emotional and personality problems and disorders by psychological means.” 3.

"We shall define psychotherapy as any type of professional interpersonal situation between a therapist and his patient designed to help the patient to resolve emotional problems." 4.

"Psychotherapy is primarily a transaction between the patient and his therapist." 5.

"Psychotherapy is ...

a developing transaction between two people, one suffering from some type of distress or exhibiting disordered behavior, the other offering amelioration as part of his professional activity." 6.

Psychotherapy consists of "techniques derived from established psychological principles, by persons qualified through training and experience to understand these principles and to apply these techniques with the www.freepsychotherapybooks.org 23 intention of assisting individuals to modify such personal characteristics as feelings, values, attitudes, and behaviors judged to be maladaptive or maladjustive." 7.

"For a very simple realistic definition, one could say that psychotherapy is the utilization of psychological measures in the treatment of sick people." 8.

[Psychotherapy endeavors] "to alter the behavior and change the attitudes of a maladjusted person toward a more constructive outcome." 9.

[Psychotherapy alludes] "to the entire collection of approaches attempting to influence or assist a patient toward more desirable ways of thinking, feeling, and behaving." 10.

"By psychotherapy is meant the use of measures which it is believed will act upon the patient's mind and thereby promote his mental health and aid his adjustment to the particular problems which have disturbed his happiness or adaptation." 11.

"Psychotherapy is a form of treatment in psychiatry in which the psychiatrist, by his scientific thinking and understanding, attempts to change the thinking and feeling of people who are suffering from distorted mental or emotional processes." 12.

Psychotherapy "aims to help the impaired individual by influencing his emotional processes, his evaluation of himself and of others, his evaluation of and his manner of coping with the problems of life.

It may also include changing his environment ... and simultaneously increasing his potentialities of mastery and integration." 13.

"[Psychotherapy includes] a multitude of psychological methods, all having one thing in common—the intent to help a suffering individual through psychological means." 14.

"Psychotherapy is a planned and systematic application of psychological facts and theories to the alleviation of a large variety of human ailments and disturbances, particularly those of psychogenic origin." 15.

"In general, psychotherapy can be defined as the provision by the physician of new life-experiences which can influence the patient in the direction of health."

www.freepsychotherapybooks.org 24 16.

"Psychotherapy is the art of combating disease and promoting health by mental influences." 17.

"[Psychotherapy] connotes the use of definitive psychological techniques designed to relieve demonstrable disturbances in psycho-social adjustment." 18.

"Psychotherapy includes all kinds and ways of utilizing psychologic means to achieve beneficial psychobiologic changes." 19.

"Psychotherapy consists of any considered and competent medical endeavor directed toward the improvement of the emotional health of the individual, based upon the understanding of the psychodynamics involved, and of the need of the individual under treatment." 20.

[Psychotherapy is the] "treatment of mental or emotional disorder or of related bodily ills by psychological means." 21.

[Psychotherapy is] "the use of any psychological technique in the treatment of mental disorder or maladjustment... .

The term carries no implication about the seriousness of the disorder ... the duration or intensity of treatment, or the theoretical orientation of the therapist.” 22.

[Psychotherapy is] “an emotional exchange [process] in an interpersonal relationship which accelerates the growth of one or both participants.” 23.

“Psychotherapy is ... a cooperative enterprise for clarifying purposes and modifying attitudes in the direction of greater integrity of personality.” 24.

“Psychotherapy is a certain kind of social relationship between two persons who hold periodic conversations in pursuit of certain goals: namely, the lessening of emotional discomfort and the alteration of various other aspects of client behavior.” 25.

“Psychotherapy may be defined as the treatment of emotional and personality problems and disorders by psychological means... .

Types of psychotherapy fall into two general groups.

One may be described as genetic-dynamic, the other as supportive, suppressive, non-exploratory, or non-specific.” 26.

“Psychotherapy: The generic term for any type of treatment which is based primarily upon verbal or nonverbal communication with the patient, in www.freepsychotherapybooks.org 25 distinction to the use of drugs, surgery, or physical measures, such as electroshock or insulin shock, hydrotherapy, and others.” 27.

“Psychotherapy is a process in which changes in an individual's behavior are achieved as a result of experiences in a relationship with a person trained in understanding behavior.” 28.

“In its classic sense, psychotherapy is defined as the restructuring of the malfunctioning personality.” 29.

“... psychotherapy is a form of help-giving in which a trained, socially sanctioned healer tries to relieve a sufferer's distress by facilitating certain changes in his feelings, attitudes, and behavior, through the performance of certain activities with him, often with the participation of a group.” 30.

“Psychotherapy is a form of treatment of a client by a therapist for disorders of emotional adjustment, with the purpose of bringing about his readjustment so that he will become more comfortable in his emotions, thoughts, and attitudes, and so that his social relationship will be improved.” 31.

Psychotherapy is a “process recurring between two (or more) individuals in which one (the therapist), by virtue of his position and training, seeks systematically to apply psychological knowledge and interventions in an attempt to understand, influence, and ultimately modify the psychic experience, mental function and behavior of the other (the patient).” 32.

[Psychotherapy is] “a procedural arrangement between a therapist and patient in which the therapist, by explicit or implicit contract, attempts to improve, by psychological means, conditions and behavior which interfere with the patient's well-being— and which are ascribed wholly or in part to psychological causes.” 33.

“Psychotherapy: The generic term for any type of treatment that is based primarily upon verbal or nonverbal communication with the patient as distinguished from the use of drugs, surgery, or physical measure such as electroconvulsive treatment.” 34.

“Psychotherapy in its broadest sense is the systematic effort of a person or group to relieve distress or disability by influencing the sufferer's mental state, attitudes and behavior.”

www.freepsychotherapybooks.org 26 35.

[Psychotherapy is the] “use of psychological techniques in the treatment of mental disorders. Psychotherapeutic methods are employed by a trained psychotherapist who helps the patient by means of verbal and emotional communication.” 36.

“The science and art of influencing behavior so as to make it (a) more efficient and satisfactory to the individual and (b) more compatible with social norms.” 37.

Psychotherapy is “a process in which a person who wishes to relieve symptoms or resolve problems in living or seeking personal growth enters into an implicit or explicit contract to interact in a prescribed way with a psychotherapist.” 38.

“Psychotherapy is an interpersonal process designed to bring about modifications of feelings, cognitions, attitudes, and behavior which have proven troublesome to the person seeking help from a trained professional.” 39.

“Psychotherapy is a confiding, emotionally charged relationship between a trained, socially sanctioned healer and a sufferer.

The healer seems to relieve the patient’s suffering and disability ...

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New York, Praeger, 1984 www.freepsychotherapybooks.org 30 2 Varieties of Psychotherapy An
impartial observer of the contemporary psychotherapeutic scene would be forced to admit that
present-day theories about how people become emotionally ill are exceeded in number only by the

available remedies for making them well again.

The individual seeking help for an emotional problem, the student in quest of training in psychotherapy, the investigator searching for answers to the puzzling problems of process and outcome research—all are confronted with an imponderable dilemma of choice.

Indeed, the field of psychotherapy has become a vast supermarket where consumers may shop for stores of interventions that can satisfy the most capricious tastes.

These range from mundane traditional interview methods to more exotic excursions into marathons, mantras, and massage.

There is no need to agonize over reasons for the predicament that psychotherapy finds itself in today.

Empirical studies have shed little light on how treatment processes operate and when they are most effective.

Emotional problems are difficult to treat.

Welltrained therapists are hard to find.

Good therapy is expensive and, even when implemented, encounters great resistance in some patients.

Professionals are constantly devising unique approaches that coordinate with their personalities, philosophies, and styles of relating.

Often they become so convinced of the virtues of their systems that they incorporate in it the entire world of psychopathology.

Vast pockets of need remain unfulfilled and into these areas pour the less trained, less experienced practitioners, as well as a host of charlatans, drum beaters, and mental health evangelists whose enthusiasm for innovation is matched by their ignorance of things psychological.

Lacking the discipline of experience, they promise rapid cures with a murky collage of half-truths, naive opinions, and outright distortions.

The average person is thus in a quandary when exploring the field in the hope of finding an adequate resource for personal emotional difficulties.

Inquiries will generally www.freepsychotherapybooks.org 31 bring as many opinions about preferred courses of action as there are people to consult.

The family physician may admonish to “slow down and relax,” supplementing this advice with such medications as tranquilizers, vitamins, energizers, hypnotics, or placebos.

A minister may enjoin more assiduous devotion to religion and faith that God will show the path to follow.

A lawyer may counsel a long vacation in order to get away from the press of business responsibilities.

Friends may urge one to leave a job, or to divorce a mate, or to find an absorbing hobby, or to take up meditation, or to see a chiropractor, or to read selfhelp books.

If the average person decides to get professional help, there will be no less confounding advice, particularly if a resident of a large city where there are many representative types of therapy.

If such is the case, what should he or she do?

Should he choose a therapist who practices psychoanalysis in spite of the current unfavorable publicity regarding its values?

Should she go to a behavior therapist even though some of her friends have warned her that symptom alleviation will not get down to the bedrock of her troubles?

What about group therapy, or marital therapy, or Scientology, or primal scream therapy, or transcendental meditation, or reality therapy, or any of the scores of other approaches that he or she has heard or read about?

Perhaps his body needs balancing with psychotropic drugs, or maybe she can gain control over her physiological responses with biofeedback, or is hypnosis or strategic therapy the answer? It is confusing, even for the average professional person, to view the multiform methods of treatment that are promulgated for emotional illness and to listen to the exaggerated claims of their devotees and the violent denunciations of their critics.

Equally puzzling is the fact that published statistical data, tabulating percentages of cure and improvement and failure, reveal that results obtained by various methods of treatment are strikingly similar.

Indeed, people seem to be benefited by all kinds of therapy—by those that have a scientific stamp of approval as well as those on the fringe of quackery.

How can we explain such inconsistencies?

How can we, for instance, explain the failure of people to respond to years of intensive and skillful psychoanalysis who, www.freepsychotherapybooks.org 32 succumbing to the blandishments of untutored charlatans, lose their symptoms in a few weeks and take up life with renewed vigor?

Are there differences in the quality of improvement with varied forms of treatment, or do certain kinds of problems respond better to specific types of treatment methods?

Are there differences in the permanency of the results obtained by the respective procedures?

Can we say that there is a “best” treatment for neurosis?

Added to these confusing issues is the fact that in some instances an individual suffering from a severe emotional problem may experience considerable relief, and even a so-called cure, without the formality of having received any kind of treatment whatsoever.

How much of a spontaneous reparative element, therefore, is present during the course of any therapy?

Answers to these questions will be considered in the chapters that follow.

To describe the countless permutations of therapy that exist today would be an almost impossible task since all practitioners evolve a unique format influenced by their background and character structure.

Corsini (1981) lists no less than 250 different kinds of psychotherapy, and Herink (1980) describes as many in his book.

Nevertheless, there are identifiable pools within which practitioners flexibly float.

It may be useful to classify the chief varieties of psychotherapy in relation, first, to the objectives they pursue (Table 2-1) and, second, to the fields from which they derive their substance (Table 2-2).

The varieties of psychotherapy may conveniently be divided into three main groupings: supportive therapy, reeducative therapy, and reconstructive therapy.

The distinctions among these as they relate to objectives and the more common approaches are outlined in Table 2-1.

The many psychotherapies in supportive, reeducative, and reconstructive categories become more diverse from year to year as an increasing number of professionals enter the psychotherapeutic field, introducing into it their unique technical modifications.

Schools of psychotherapy have crystallized around the various approaches, each of which has its zealous disciples as well as its staunch critics.

Each lays claims to multifarious successes and admits to some failures.

Actually, radical divergencies in technique are more apparent than real, many distinctions vanishing as soon as semantic differences are resolved (Rosenzweig, 1936; Watson, 1940).

Some www.freepsychotherapybooks.org 33 differences, however, do exist and will be considered in detail in the next chapters.

Schools of psychotherapy are characterized by the aggregation under one banner of a host of

professionals, trained to respect one or another of the pioneers in the biological, psychological, social, and philosophic fields and espousing, often fanatically, a general point of view and basic postulates, which, upon close examination, reflect wide personal differences in interpretation and application.

Indeed, the variations and shades of divergence between members of the same school are often greater than between members of rival schools.

Professionals develop individual styles of doing psychotherapy, as artists evolve unique ways of painting.

They then become wedded to these styles, perfect them, find them effective to their satisfaction, and then sometimes promote them as the best of all treatments.

A general theory is fabricated sooner or later to provide a rationale which, more or less, draws from their personal lives and intrapsychic experiences.

If they are sufficiently persuasive, they may attract a host of followers who are searching for answers to the dilemmas of treatment.

And if they possess an abundance of charisma, they may even initiate a revolution in the form of a new school, which lasts for a period until sufficient failures in therapy accumulate to convince therapists that the psychiatric messiah has not yet arrived.

When we distill out the important essences of all of the existing therapies, the differences are not as profound as they seem on the surface, although several classes of therapies do exist with distinctive goals and operational modes.

While all schools of psychotherapy explicitly avow an empirical dedication rooted in observation and experiment, some find their pith in ontologic and even theologic-like precepts. Difference is registered in the degree of allegiance to biological as contrasted with psychological and sociological principles.

On the one hand, there are behavioristic schools that orient themselves around a stimulus-response psychology or social learning model, focused on objective observation, accepting psychic phenomena merely as intervening variables.

There are hormic schools, which include the dynamic systems, that www.freepsychotherapybooks.org 34 deny that behavior is explicable in purely mechanistic or physiochemical terms but rather requires the consideration of contents of mind as objects of awareness.

The various approaches to emotional illness involve many affiliated fields (see Table 2-2).

In the main, the following avenues are pursued: 1.

Organic-physiological regimens that, rooted in the soil of biology, are strictly speaking not psychotherapeutic.

2.

Behavioral-conditioning techniques, psychologically oriented, that deal with the effects of conflict and depend on relearning and the retraining of responses without concerning themselves too much with conflictual sources.

3.

Supportive relationship tactics, sociologically based, that draw upon factors that are operative in any "helping process." 4.

Dynamic-reconstructive methodologies, psychologically and sociologically inspired, that focus on insight, delineating the origins of conflict, its effect on intrapsychic functioning, its manifestations in character structure, and its impact on problem solving and other vital behavioral activities.

5.

Philosophic-persuasive ideologies aimed at alteration of values and meaning systems, toward the development of more adaptive ways of approaching life's burdens.

In actual practice there is often a fusion of approaches, although the followers of specific schools may be loathe to admit this.

www.freepsychotherapybooks.org 35 3 Extratherapeutic (non-specific) Healing Aids: I.

The "Spontaneous" Cure Psychotherapy attempts to alleviate emotional suffering and to enhance personality adjustment through planned psychological interventions.

It is by no means the only medium through which such benefits may be achieved.

Indeed, operative constantly on the individual are a variety of forces that serve to ameliorate neurotic symptoms and, even, under fortunate circumstances, to sponsor personality growth.

These forces work so subtly that they are usually overlooked.

They are an inherent and inevitable part of the living experience.

Because they exert their effect during psychotherapy, as well as outside of it, it is difficult to know the proportion of benefit the individual has derived from these congenial non-specific agents and how much that person has gained from the specific psychotherapeutic maneuvers themselves.

More importantly, if we are able to define the forces that nurture emotional health apart from psychotherapy, we may be able to incorporate these into our therapeutic program, or at least prevent ourselves from inhibiting them.

Among the coincidental factors associated with psychological change are (1) "spontaneous" remissions or cures, and (2) influences that automatically issue out of any "helping" situation.

The first of these will be considered in this chapter, the second in Chapter 4.

Spontaneous cures probably occur more frequently than we are wont to admit, for both physical and emotional difficulties are associated with periods of exacerbation and periods of remission and, often, without apparent cause, vanish of their own accord (Stevenson, 1961).

Even the most pernicious forms of psychoses show a tendency toward spontaneous remission, and any institutional psychiatrist, even before the era of psychotropic drugs, is acquainted with the patient who, having spent years in the hinterlands of a chronic mental ward, suddenly returns to the world of reality and expresses eagerness to take up his place as a member of society (Wolberg, 1944).

www.freepsychotherapybooks.org 36 The circumstances surrounding such a "spontaneous cure" are so nebulous that it is customary to look on the phenomenon, when it occurs, as a sort of fortuitous development.

Were we, however, able to peer into the complex mechanisms of the psyche, we might be able to discern definite laws of cause and effect relating to the spontaneous recovery.

It is unfortunate that so much emphasis has been placed on the evil consequences of illness that we are prone to concern ourselves chiefly with destructive pathologic effects and forget that constructive regenerative influences may be coincidentally present.

It is as if in an infection, we were to concentrate on morbid elements of the disease without considering such protective manifestations as the antibody defenses and the activities of the reticuloendothelial system.

Any somatic disease whips into action reparative mechanisms in the direction of health.

Cases of unquestionable cancer that spontaneously resolved themselves without benefit of any treatment have been authenticated (Medical Tribune, 1963).

This indicates that the individual has internal healing forces that are capable of altering an even presumably fatal illness.

A mental or emotional disorder, too, will set into action defensive devices to restore the individual to homeostatic equilibrium; indeed, a study of the symptoms of emotional illness, whether the difficulty be a mild behavior problem or a pernicious form of schizophrenia, will always demonstrate a fusion of dynamisms representative both of the conflict and of the

defenses the organism elaborates in an attempt at cure.

The exact mechanisms involved in spontaneous remissions or cures are not exactly known, but a number of operative factors, singly or in combination, suggest themselves.

First, life circumstances may change and open up opportunities for gratification of important but vitiated needs, normal and neurotic.

Thus a man with a passive personality organization may, in fighting against passive impulses, elevate himself to a high political post that enables him to exercise secret cravings for power.

The expression of these may neutralize inner fears of weakness and lead to feelings of well-being.

A masochistic individual may appease a strong sense of guilt through the incidence of a physical illness, www.freepsychotherapybooks.org 37 an accident, or a catastrophic life happening.

A man with strong dependency strivings may flounder around helplessly until he chances onto a stronger individual, an alliance with whom infuses him with strength and vitality.

A woman with schizoid tendencies may be afforded, through advantageous circumstances, an acceptable opportunity to detach herself in her interpersonal relationships.

A shift in the current situation may, therefore, work in the interest of promoting psychic stability.

Second, provocative stress sources may disappear as a result of the removal of the initiating environmental irritant, or because individuals extricate themselves from it.

A child, selected by a classroom bully as a focus for sadistic attacks will, in all probability, lose tensions and fear with the forceful removal from the class of the disturbed assailant.

A young dependent adult overprotected by a neurotic parent may decide to move away from home and be rewarded with an expanding sense of independence.

A man, promoted into a highly competitive position that sponsors fears of failure, may, by returning to his previous, more mediocre job, overcome his anxiety and depression.

A leave of absence or a vacation may, in a similar manner, remove a person from a disturbing life situation, helping in self stabilization.

Third, crumbling or shattered defenses, whose failure promotes adaptive collapse, may be restored to their original strength, or be reinforced by new, more adequate, and less disabling defenses.

The return of a sense of mastery in the course of buttressing failing defenses, will help to restore the individual to functional equilibrium.

One way this is accomplished is by consolidating the mechanism of repression, sealing off disturbing conflicts that have managed to get out of control.

For instance, a detached individual who has remained celibate all his life due to deep fears of sexuality, falls in love with a young woman.

Her demands for reciprocal affection excite sexual fantasies and impulses that frighten him.

Anxiety and psychosomatic symptoms precipitately develop.

The man then discovers that he is completely impotent and that his interest in the young woman abates and dies.

Restored repression of his sexual desires, with a return to isolation from women, brings about a recovery from symptoms.

Sometimes, however, the defenses of the individual undergo modification in the direction of greater adaptability to inner demands www.freepsychotherapybooks.org 38 and external pressures.

More constructive ways of dealing with current difficulties may be evolved.

The means by which this is accomplished will vary with the circumstances and with the

opportunities at one's disposal.

Not only may the old neurotic balances of power be restored, but surprisingly, without design, enduring personality changes may develop.

No matter how victimized by past unfortunate conditionings, each individual is always capable of some determinate learning under propitious circumstances.

The expectation of failure, the misinterpretations of the reality situation, the transferential projections, the repetitive compulsive reenactments of self-defeating behavior will tend, of course, to sabotage successes.

Inferential discoveries, nevertheless, are going on around the matrix of the neurotic responses, reconditioning at least some neurotically structured reactions.

Instances of reconditioning in even severe neurotic and psychotic disorders have been recorded in persons who have by chance come under the guidance of a well-disposed and intelligent authority or group or who have found a congenial environment that is different from their habitual milieu.

These changes are not fortuitous; they follow certain laws of learning.

Where the individual finds an environment that does not actively repeat the punitive, frustrating conditions of the past; where it provides a modicum of security and some means of gratifying vital psychological needs; where it gives a feeling of knowing what is expected of each person and some ability of living up to these expectations; where it nurtures in the individual a sense of belonging and of being wanted; where it provides opportunities to expand capacities and skills—there will be optimal conditions for learning.

Obviously, those individuals whose emotional problems have undergone minimal structuralization in the form of neurotic and character disorders will have the best chances to learn in a favorable environment.

Where extensive organization has occurred, conflictual strivings will tend to be perpetuated even in a good setting.

There still may be spontaneous periods of improvement, particularly where the environment absorbs and deals in a tolerant and constructive way with the individual's neurotic impulses and behavior without excessive punishment or withdrawal of love.

www.freepsychotherapybooks.org 39 Alleviation of symptoms may follow change in one's philosophies and altered ways of looking at things.

For example, some people adapting to difficult life situations from which there is no escape may insulate themselves by making a virtue out of suffering or by maintaining a stoical attitude accepting their plight as inevitable, and their endurance a test of competence.

Some may even make capital out of personal disasters and secure equanimity through martyrdom. Many of the present-day mystical philosophies stress the unimportance of adversity and external hardship, focusing the individual's attention on establishing contact with the inner self, the essence of being.

In this way the individual detaches from suffering and seeks joy in meditation and self-contemplation.

An individual may resort to other devices in the belief that one has control of one's destiny, for instance, through reliance on astrology.

By classifying oneself in a special category, subject to designated occult influences of the stars, security is gained through regulation of one's life according to a dictated design.

The patient may also evolve the tactic of pushing out of mind sensations of pain, substituting for them thoughts of health and pleasure, hereby eliminating customary preoccupations with suffering.

Through proper thinking habits, like "looking at the cheery side of life," "counting one's blessings," and engaging in thoughts of happiness and health, attempts are made to sidestep

calamities and illness.

Troubles may be rationalized by assuming that one is not alone in strife, that there are others more disabled, more unfortunate.

Striving for success and perfection are false objectives that can bring only exhaustion and disappointment.

It is better to scale down ambitions and content oneself with modest goals that are within the range of immediate fulfillment.

Some people make an inventory of their good qualities and recognize that there is much for them to be thankful.

They may rule themselves to live in the spirit of forbearance, tolerance, sympathy, and altruism, reviving, in the course, religious convictions.

By turning to religion, some are able to help themselves tolerate the destructive effects of their neuroses.

In the hands of a Power stronger than a human being who can lead one to paths of safety and glory, one can be reassured.

In union with God, one does not have to struggle alone; one may be helped to endure travail, to conquer evil thoughts www.freepsychotherapybooks.org 40 and impulses, and to find confidence and strength in living.

In addition to philosophic and religious aids, the individual may indulge other modes of gaining relief.

Remediable, stressful environmental distortions that sustain and exaggerate tension must be rectified.

Moving to a residential area, for instance, that is not so riddled with quarreling and disturbed neighbors may eliminate one source of irritation.

Or, some difficulties may spontaneously cease to exist, as the case where a destructive supervisor or foreman is replaced with a genial one, or a hostile member of one's family may regain control over explosive emotions or takes up residence in another city.

The patient may also find relief from symptomatic preoccupation by externalizing interests in arts, crafts, hobbies, music, games, dancing, physical exercise, sports, and recreation.

Diverting energies into useful channels of activity may thus have a salubrious effect.

Engagement in social activities, joining clubs and other groups, in which there is a sharing of experiences can be most constructive.

The patient may also gain relief from some personal problems by running away from trouble sources.

A woman may rightfully decide that her difficulty is stirred by a destructive home environment, or an impossible work situation, or a relationship problem with which she cannot cope.

She may consequently remove herself bodily from her predicament by going on a long vacation or permanently rupturing her contacts.

Thus, she may seek a divorce, quit her job, or abandon her boyfriend; and these disengagements may, if auxiliary problems do not intervene, temporarily at least, relieve her distress.

She may act more destructively to escape her suffering by indulging in excessive drinking that deadens her sensations, take tranquilizers that ameliorate them, or take sleeping potions that submerge them.

Some people seek peace of mind by providing what they consider rational answers to explain what they are experiencing.

They may do this through self-probing, examining their reactions, questioning their motives, or cogitating on their fantasies.

Under situations of stress individuals are particularly disposed to such self-analysis.

Adolescents particularly attempt to resolve warring elements within themselves through

selfwww.freepsychotherapybooks.org 41 searching.

Such efforts are not always successful since sources of difficulties may be hidden from their rationalizing probes.

The capacity for insight varies.

The desire to reach an understanding of one's turmoil will invoke different degrees of logical compromise.

Education helps especially through proper readings and personal therapy.

Those individuals who have evolved reasonable understanding are most capable of illuminating dark recesses within themselves.

Spontaneously, a recovery of forgotten important memories, a relating of present-day patterns to past experiences, a recognition of inexorable and self-defeating demands they make on themselves may occur.

Ultimately, new solutions for old problems may emerge.

Sometimes such self-understanding is mobilized by single interviews with a psychotherapist with no further professional help.

Malan et al.

(1975) at the Tavistock Clinic made a study of untreated neurotic patients who had exposed themselves to one initial interview and had been seen by a therapist no more than twice in their lives.

On follow-up, 51 percent were found to have improved symptomatically, and 24 percent were judged to have improved dynamically, some even to the point of recovery.

This is a significant number.

Considering the fact that the work was done at the clinic by established psychoanalysts who utilized rigid standards of measurement, we, therefore, have evidence that even single encounters may prove of great benefit to some patients.

Whatever activity was mobilized during the interview, the patients continued spontaneously on their own to work through residues of their problems.

It will be seen from the above that the so-called spontaneous remission or cure does not take place in a vacuum.

It results from the operation of many forces that exert a healing influence.

Even though these processes are not deliberately designed, but obtrude themselves through what appears to be chance, each individual takes advantage of some healthful elements in the environment and within the self to reduce suffering and pain.

As a general rule, spontaneous remissions or cures are of a palliative nature, with no great change coming about in the existing mental mechanisms and intrapsychic apparatus.

In a few instances, however, rather surprising spontaneous structural alterations develop www.freepsychotherapybooks.org 42 in the personality organization itself.

Most recalcitrant to spontaneous resolve are deepseated neuroses linked to repression of infantile anxieties, which are so split off from access to awareness that they do not lend themselves to dissolution.

Nevertheless, even here they may be more readily held at bay by a spontaneous strengthening of repressive forces, or improved by avoiding situations that tend to activate them.

Focal conflicts may in this way be isolated sufficiently to enable the individual to function satisfactorily without being aware of their content or even existence.

It will be recognized that some of the undercurrent processes in spontaneous cure are similar to those in psychotherapy.

Whereas in spontaneous cure the individual inadvertently arrives at conditions in which it is possible to work out problems, in psychotherapy, circumstances conducive to relief or recovery are actively manipulated.

Many opportunities exist in the environment that would lend themselves to a corrective influence on the patient.

Unfortunately, the average person does not often take advantage of them because there is no awareness of their potential or because their meaning is distorted, or because they pose a threat to neurotic defenses.

During psychotherapy, the resistances of the individual are handled so that the most can be made of the opportunities encountered.

In contrast to the fortuitous nature of the spontaneous development, psychotherapy is a planned effort to promote symptom alleviation and constructive learning both within and outside of the psychotherapeutic setting.

www.freepsychotherapybooks.org 43 4 Extratherapeutic (non-specific) Healing Aids: II.

The "Helping" Situation Turning to another human being for help is an inevitable consequence of a shattered sense of mastery.

It represents acknowledgment of the inability to cope with difficulties through one's own resources.

Contact with a trusted individual can inspire rapid relief from tension and even restore adaptive functioning.

Physicians, ministers, lawyers, teachers, psychiatrists, psychologists, social workers, and even considerate friends may serve as elegant resources for a helping relationship.

The effect, while therapeutic, is epiphenomenal.

It reinforces the impact of formal psychotherapy.

It is a vital, indeed inescapable, constituent of the therapeutic gestalt.

Most prominent among the liberated forces are the placebo effect, relationship dimension, emotional catharsis, suggestion, and group dynamics.

THE PLACEBO EFFECT If a patient and therapist are both firmly convinced that eating oatmeal will cure the patient's symptoms, it is likely that relief will be forthcoming as long as the proper rituals have been organized around the taking of the cereal, and the patient maintains faith in its efficacy.

If as a consequence of the conviction of being helped the patient dispels tension and anxiety sufficiently to restore to oneself a sense of control, he or she will possibly return to habitual routines and sooner or later reach an optimal level of functioning.

Moreover, if the problems are not too serious and one is fortunate enough to live in a constructive environment, particularly in a productive relationship with some other human being, he may even undergo deeper personality changes.

The placebo phenomenon responsible for these permutations is usually minimized by therapists who have been seduced by a belief in the scientific validity of their systems.

This is not to minimize the impact of a rational psychotherapy, but it is essential to give credit for part of the benefit to adventitious elements other than technical interventions.

www.freepsychotherapybooks.org 44 The placebo has been called a "nuisance variable," and yet it historically has served as an important, albeit often unacknowledged, aid in medicine (Shapiro, 1964).

Indeed, the prescription of inert substances was a standard part of medical practice for many years, being embodied in the tradition of giving pink sugar pills and injecting sterile water for their suggestive effects.

These effects may be quite powerful, even to the point where direct or suggestive statements regarding a presumed drug action induce a psychological reaction diametrically opposed to the true pharmacological effects of the drug (Wolf S, 1950).

Research explorations on medicaments in the form of double-blind studies pay respect to the placebo factor, recognizing that a mental influence may contaminate the impartial appraisal of

a substance.

There is ample experimental and clinical evidence to substantiate this view.

Laboratory reports indicate that pain thresholds are heightened and pain tolerance increased by placebos.

Beecher (1970) has stated that placebos relieve pain one-half to two-thirds as effectively as optimal doses of morphine.

Apart from the considerable relief of pain, various physical ailments may be helped (Lasagna et al, 1954; Wolf & Pinsky, 1954).

The placebo effect probably accounts for the fact that most patients who visit a general medical physician improve with almost any symptomatic treatment.

Hypodermic injections of distilled water accompanied by statements that a new remedy for bleeding ulcers was being administered helped 70 percent of patients over a long-term period (Volgyesi, 1954).

A study of vaccines for the common cold revealed that while 55 percent of persons injected with the vaccine showed a reduction in the number of yearly colds, fully 61 percent was recorded in a control group who had received nothing more than harmless injections of saline solution (Diehl et al, 1940).

Patients with chronic headaches experienced relief with placebos in 60 percent of cases (Jellinek, 1946), and the same percentage of patients noted a reduction of the frequency of herpes simplex with placebo therapy alone (Conant, 1974).

Beecher (1955), evaluating 15 studies, including 7 of his own, involving a total of 1082 patients, found that placebos had significant effectiveness of 35.2 percent $\pm$ 2.2 percent. He also found that the effect was most powerful when stress, discomfort, and pain were greatest.

His remark that “many a drug has been extolled on the basis of clinical impression when the only power it had was that of a placebo” has been www.freepsychotherapybooks.org 45 repeatedly confirmed.

The impact on the individual of a placebo is all the more astonishing when we consider that through its suggestive influence it can produce objective physiological changes (Abbot et al, 1952).

Thus placebos given to patients with duodenal ulcers produced the healing of ulcers in 65 percent of the cases (Hollander & Harlan, 1973).

A significant number of side effects may even be credited to an inert material (Feather & Rhoads, 1972; Rickels et al, 1965).

Toxic phenomena have been induced by substances to which the individual could not possibly have been sensitive, solely on the basis of the individual's conviction that he could not help responding in an untoward way.

Sugar pills, for example, have produced weakness, palpitation, nausea, maculopapular dermatitis, epigastric pain, watery diarrhea, urticaria, and angioneurotic edema (Wolf & Pinsky, 1954).

According to S.

Wolf (1950), physiological effects are brought about by a placebo through “objective changes at the end organ which may exceed those attributable to potent pharmacologic action.” In patients with anxiety placebos have produced physiological responses similar to those of corticotropin (ACTH) in normal patients.

How placebos work is not known.

Faith in a minister, guru, charismatic leader, shaman, physician, spiritualist, shrine, acupuncture, or scientific method—irrespective of on whom or what the individual pins one's hope—this can promote high levels of wellbeing and healing that on the surface sound

miraculous.

But modern research points out that the miracle is chemical and not spiritual.

Concentrated in the hypothalamic-pituitary region, opiatelike pentapeptides, the enkephalins and endorphins, are apparently liberated through a placebo effect producing analgesia that modulates pain and activates drive and reward systems in the brain.

These endogenous substances also interact with other neurotransmitter systems influencing neuroendocrine functions that sponsor important psychoadaptive effects.

The healing value of faith is very well documented in popular and scientific literature.

Its operation may make the difference between death and survival in serious accidents, illnesses and surgical operations.

When individuals with tension, anxiety, and other neurotic symptoms take recourse

www.freepsychotherapybooks.org 46 to medicaments in order to subdue their suffering, the effectiveness of the drug is greatly enhanced if there is a conviction that it will work.

In double-blind studies with tranquilizers, a sizable number of subjects report marked relief of their symptoms with placebos that are packaged similar to the tranquilizers under test because the deceit excites confidence in the inactive substance.

However, even when the patients are told that the material given them is inert, they will still respond positively (Park & Covi, 1965), probably on the basis of faith, hope, and expectancy.

In many parts of the world charms and amulets are still worn for their placebo effect.

These objects work their wonders among the less educated by dispelling evil spirits, invoking benevolent deities, and generally protecting the individual against misfortune and disease.

Counterparts of the ancient mandrake root and unicorn horn are still sold to quell ague, to staunch blood, to cure headaches, to facilitate childbirth, and generally to promote health and well-being.

In some areas the sale of herbs and curious nostrums flourishes, such as in Chinese herb shops in Hong Kong where dried sea horses are prescribed for chest congestion, ground pearl powder for pimply complexions, rhinoceros horn for heart ailments and impotence, tiger bones for rheumatism, and fungus from a coffin growing near the corpse's mouth for tuberculosis.

Among the better educated, more refined and highly personalized tokens are coveted to bring good luck and to allay misfortune.

Confidence in the charm or token is, as in drug placebos, the essential ingredient.

The conviction that a powerful protective agent is at work to support one's claims on life, and to protect against dangers, is sufficient to calm anxiety, to lessen tension, to restore physiological homeostasis, and to promote self-confidence and assertiveness.

A more accurate perception of possibilities of action leads to mobilization of effective resources and to more constructive problem solving.

Shattered repressions may be restored, and the individual brought back to a functional personality equilibrium.

What holds true for drugs probably holds true for systems of psychotherapy (Rosenthal & Frank, 1956; Frank, 1961; Friedman, 1963; Campbell & Rosenbaum, 1967).

Individuals project into these systems their expectations of cure, and they may react quite remarkably to techniques and agencies that they enshrine with powers that the objects www.freepsychotherapybooks.org 47 may not really possess.

This is suggested by studies of psychiatric outpatients where the sole therapy was oral placebos and in whom a significant (55 percent) improvement rate was actually recorded (Gliedman et al, 1958).

Relief of discomfort and symptomatic amelioration have lasted for years solely on the basis of an expectancy of help (Frank et al, 1959).

There is a definite correlation between conviction and cure.

Thus sitting in the waiting room of an outpatient clinic has resulted in signal improvement in patients (Frank et al, 1963).

Some time ago I was consulted by a man whose sole complaint was that he smoked too much. He wanted me to help him to control his craving so that he could forever eliminate tobacco from his life.

He had, he confessed, at one time been a schizophrenic but was now completely cured, having been treated by a therapist who employed a unique method.

Twice weekly the patient would appear for his sessions.

He would strip to nudity and lie on a couch near an open window.

Verbalizations were forbidden.

Instead he was ordered to bear, Spartan-like, the discomfort of cold and drafts in complete silence.

The therapist, during this ordeal, sat near the patient, huddled in a blanket, recording the patient's muscle movements and general bodily reactions.

Interpretations were not offered.

The object of his tribulation was to raise the patient's frustration tolerance by exposing him to external suffering so severe as to induce a psychological callus formation that would ultimately encapsulate his weak and sensitive ego.

The treatment, while uncomfortable, was, according to the judgment of the patient, eminently successful.

In fact, the therapist was the only one of a series of four psychiatrists who had given him any help whatsoever.

The previous therapists, applying the conventional psychotherapeutic procedures, had failed completely.

While many elements probably entered into the helping process, the fact that a friend of the patient had praised the therapist to the skies had undoubtedly mobilized the patient's expectations of hope and had convinced him that he would be cured.

Therapies that in the mind of the patient possess esoteric or mystical qualities are most likely to induce a placebo effect.

Thus, hypnosis and psychoanalysis, charged as they are with supposed meta-psychological powers, are apt to instill in the patient a feeling of www.freepsychotherapybooks.org 48 magical influence.

This instrumentality, although an artifact, may promote confidence in the treatment method.

By the same token, a negative placebo effect may be obtained, if, by virtue of what is imagined will take place, the patient has lack of faith in, or fears being hurt by, a special kind of therapy.

The chances are then that this attitude will inhibit the effectiveness of treatment.

Medicaments, amulets, and techniques are potent placebo sources, but not nearly so powerful as a human authority with an inspirational flair.

It is here that the quack has an advantage over the ethical practitioner, who, fettered by factual propriety, is unable to flaunt his or her wizardry, and becomes for the patient a vehicle of hope.

In their helpless desperation, many victims of illness reach for a magical helper.

Medicine man, shaman, priest, physician, or psychotherapist, the patient importunes the agency for relief earnestly engaging in avid prayer at the promise of divine reward, and while eagerly incorporating any worthy theological, philosophic, or psychological formulations as a pledge of trust.

The placebo effect of prayer, and of conversion to a system of religion, philosophy, or psychology, are explicable in terms of the bounties the supplicant expects to receive as a

result of his dedication.

In psychotherapy we are mindful of the phenomenon of patients who regurgitate the prophetic revelations of their therapists concerning the cause, meaning, and future of their illness.

How much this “insight” is an invocation for support and how much it constitutes a true discernment of the underlying difficulty must carefully be evaluated.

Unfortunately, the therapist is not the most objective and unbiased referee for this task.

Interestingly, a theoretical system may act as a placebo for the therapist.

If there is faith in its verity, the therapist will approach the patient with dogged confidence, and will undoubtedly unearth in the patient the exact constituents that compose his or her theoretical scheme.

In this way a placebo feedback becomes operative between therapist and patient.

The conviction on the part of the patient that the therapist knows what is wrong with him or her and has ways that will bring health encourages restoration of mastery.

This is irrespective of how valid the therapist's system may be.

Certainly with the abundant theories rampant about health and disease, and the varied psychotherapeutic www.freepsychotherapybooks.org 49 techniques that are being practiced, we may rightfully be persuaded that some systems are at least fanciful if not actually wrong.

But right or wrong, a patient will be benefited by all systems if there is enough confidence in the therapist.

This is probably because the enthusiasm and firm belief of the therapist has aroused the patient's expectations of cure.

On the other hand, if the therapist has doubts about the validity of the therapeutic system, it is possible to communicate pessimism, insecurity, and helplessness, nonverbally if not verbally, and thus the element of faith that can kindle hope will be extinguished.

In this way the therapist will rob the patient of an important element in the healing process.

Many observers have commented on the powerful factor of confidence in the treatment method as an ingredient in cure.

Among native groups who are imbedded in magic and witchcraft, the most adroit application of dynamic psychotherapy produces meager results probably because it does not accord with their belief systems.

Where the victim of an emotional problem is convinced that the suffering is the consequence of a curse or other evil magic from an offended spirit, the adroit exorcism of the force presumably responsible for the bewitchery will, in a surprising number of instances, bring about the improvement or cure of even long-standing neurotic and psychotic illnesses.

Assignment of the illness to its presumable responsible determinants tends to alleviate fear of the unknown.

It matters little whether the identified source is factual or not.

As long as the patient believes it, the catastrophic sense of helplessness is palliated.

The healer may diagnose the condition as due to infestation with evil spirits, or regression to an anachronistic psychosexual level of development, or operations of unconscious conflict, or a disrupted biochemical balance within the body.

If the patient accepts the proffered “insight,” the very focusing on a presumed source of mischief opens up new possibilities of action.

The helping agency, having delivered an explanation, proceeds to deal with the matrix of trouble.

Rituals to exorcise offended spirits or to destroy them, free associations to liberate unconscious foci of conflict, medicaments to reinstate the biochemical balances, conditioning to restore the individual to healthy habit patterns— whatever the theory of etiology—pertinent measures are executed to resolve the problem source.

If the patient has faith in the healer, the virtue of the methods will be endorsed, www.freepsychotherapybooks.org 50 whether these be anomalous or scientific.

Thus an individual, impressed by a practitioner dedicated to meditation, will do better with relaxing exercises than with insight therapy prosecuted by a dynamically schooled therapist in whom there is little confidence.

The feeling that one is being helped, that a curative agency is at work, subdues anxiety and diverts the individual from self-defeating defenses, toward more effective dealings with existing problem situations.

The disruptive physiological effects of anxiety are brought under control, and restoration of self-confidence and assertiveness ensues.

Repressions are restored, and psychological homeostasis is reestablished.

Placebo effects are usually, but not always, temporary.

In many instances the improvement acts, as in the "spontaneous cure," as a vehicle for the reorientation of the individual in the total adjustment.

Generally, the resulting security feeling leads to a heightened capacity to handle challenging relationships with people.

The placebo may thus act as a basis for reorganization of attitudes, which may then become reinforced and entrenched in a favorable milieu.

The operation of the placebo element helps to explain why patients may be helped by psychotherapies that seem scientifically unsound, provided that there is faith and trust in their validity and power.

THE RELATIONSHIP DIMENSION Every helping situation is characterized by a special kind of relationship that develops between the authority and subject.

In this relationship the individual invests the authority with benevolent protective powers and relates to the latter with expectant trust.

Implicit, if not explicit, is the understanding that the authority has the knowledge, the skill, and the desire to help the person overcome the problem for which professional services have been sought.

The more bewildered and helpless the person, the greater the reliance that individual places on experts.

This is certainly the case in the sick patient afflicted with a physical ailment who seeks relief from pain and distress from a physician.

It is a most important factor in the psychotherapeutic situation, particularly at the beginning of treatment.

www.freepsychotherapybooks.org 51 One way that the relationship operates is that while the patient may want to cling to his or her neurotic patterns, the patient may be willing to experiment with different behaviors solely to please the therapist.

Once the patient receives rewards in the form of approving gestures and words for changes in attitudes and behavior, such changes may become solidified.

Of importance here is an alteration of the self-image, which contributes to the permanence of change.

Modeling oneself after an idealized therapist is another change mechanism and may gear the patient toward new constructive experiences.

A relationship with the therapist helps the patient to tolerate, explore, and accept personal aspects that have been repudiated and repressed.

The incorporation of a new image of authority as embodied in the more tolerant, non-judgmental therapist helps alter a punitive, intolerant superego.

As a consequence of a trusting relationship, the patient may be willing to accept a proffered rationale or myth to explain the existing problem, and this may lessen tension if it accords

with the patient's belief system.

Through the instrumentality of the relationship other important processes are liberated that may exert a healing effect on the patient.

First, the patient, terrified by the disruptive emotions that are out of control, has a chance to put his or her destiny in the hands of a helpful, understanding, protective, and non-punitive authority.

There is a tendency to regard the latter as an idealized parental image who is more or less infallible.

The sicker and more anxious the patient, the more godlike the projected image and the less realistic evidence the patient will need to reinforce the illusion.

As a consequence of this union, morale may be restored to the patient, a contingency believed by some authorities to be the primary function of all the rituals lumped under the term of psychotherapy (Frank, 1974).

Second, superimposed on this dependency is a need to obtain from the authority a factual explanation for the emotional turmoil in terms that are understandable and acceptable to the patient.

The validity of the proposed facts, as has been explained previously, is not as important as the patient's willingness to accept them.

Third, the patient demands a formula from the authority that will rectify the actual or imagined sources of the difficulty.

Fourth, the patient utilizes the relationship for encouragement or help in putting into effect the actions that will resolve troubles.

Fifth, the patient finds in the relationship a medium in which to review attitudes and values.

www.freepsychotherapybooks.org 52 Since these are in a direct or indirect way associated with concepts of authority, a constructive relationship serves to alter ideas about hostile or rejecting authority.

The content of the interviews, and the activities inherent in carrying out the treatment plan, may not be as important as the nonverbal emotional crosscurrents that can potentially provide for the patient a truly corrective experience.

Impressive changes under these circumstances can eventuate.

The patient will tend to interpret the past and present in a new and less threatening light.

These processes operate in all helping relationship situations whether or not they are dignified with the name of psychotherapy, conditioning approaches, pharmacotherapy, etc.

The importance of this relationship factor is too often minimized, and complete credit for ensuing benefits is falsely extended to the special techniques and maneuvers executed by the therapist.

It is important to realize that, irrespective of the brand, the depth and the real worth of the healing measures that are being employed, improvement may be sustained in some instances for an indefinite period as a pure product of the relationship.

These "transference cures" are not always temporary as has generally been believed.

Sometimes they have lasted for years, but if stress sources have been corrected, transference distortions worked through with alteration of perceptions of authority and the patient's self, and if personal values and meaning systems have deviated in the direction of a healthy integration, the change may be a permanent one.

On the other hand, where associations with previous authority figures have been disturbingly stormy, the patient from the start may deploy defenses against a trusting involvement.

Or after a brief positive period defiant attitudes may erupt to confound both patient and therapist, the sources of which are rooted, not in reality but in residues of early unresolved childhood ambivalencies toward parental and other important adult figures.

Such “transference” outbreaks will interrupt or explode therapeutic progress. The restoration of basic trust is the foremost task in all forms of psychotherapy (Strupp, 1972a).

This is most effectively accomplished by a therapist who does not have residual interactional difficulties (countertransference) that he or she projects into the therapeutic situation. www.freepsychotherapybooks.org 53 While the relationship dimension is operative in every human encounter, it is most effectively utilized by a trained psychotherapist who knows how to deal with the resistance pitfalls of transference and the obstructive ravages of countertransference.

These are the usual impediments in nonprofessional relationships or in associations between patients and untrained surrogates.

THE FACTOR OF EMOTIONAL CATHARSIS The sheer act of talking can provide an individual with considerable emotional palliation (54).

It furnishes a motor outlet for the release of tension.

It softens inhibitions and liberates conflicts that have been held in check.

It exposes suppressed attitudes and ideas that the person has been concealing, and it encourages the individual to subject these to the light of critical reasoning.

It brings to the surface repudiated and fearsome impulses, with their attendant feeling of shame.

In this way, it takes the strain off autonomic channels that have been used to unload accumulated neurotic energy.

In the unburdening process, there is often a relief of guilt feelings in relation to past experiences, particularly sexual acting-out, hostile or aggressive outbursts, and competitive strivings.

Guilt is appeased as one examines presumably shameful fantasies as well as antisocial and unethical impulses.

Discussing these with an empathic person gives reassurance that one is not a helpless victim of uncontrollable strivings, that one has not been irreparably damaged by one's past.

Reviewing incidents in which one has been hurt, humiliated, or exploited also tends to put these into proper perspective.

Sharing one's fears of catastrophe and illness lightens their formidable quality.

Relinquished are conscious restraints that rob the person of spontaneity.

In short, the putting into words of diffuse and terrorizing feelings, and the acceptance by the listener of the pronouncements without expected condemnation and rejection, enables the person to gain greater control over emotions, to reconstitute new defenses, and to enforce a constructive action plan.

The incorporated image of harsh authority softens.

These developments may occur in the presence of any listener, whether this be a sympathetic friend or a respected authority, such as a physician, teacher, lawyer, minister, www.freepsychotherapybooks.org 54 or psychotherapist.

During psychotherapy emotional catharsis is especially prominent as imaginative conceptions and unconscious thoughts and feelings break through.

Training in interviewing and knowledge of how to encourage verbalizations, to direct them into productive channels, to give measured reassurance, or to challenge productions in a deliberately provocative way are important in facilitating the effect.

As the patient realizes that there is a consistent absence of vindictiveness from the listener, concepts of punitive authority tend to soften and some alteration of a harsh superego may possibly develop.

THE FACTOR OF SUGGESTION Human beings are constantly being influenced by various authoritative

formulations and directives.

Education is dependent on this process, students incorporating the ideas of their teachers whose wisdom and experience qualify them to instruct and indoctrinate.

In any helping relationship many forces are operative, including the need to identify oneself with the helping personage who serves as a model.

There is then an unqualified tendency to assimilate the precepts and injunctions of the helper purely on the basis of suggestion.

The influence of suggestion in one's daily life cannot be minimized.

Propaganda and advertising are dependent on it.

It is a factor that promotes thoughts, feelings, and behavior that operate for and against the individual.

It is the motor behind the placebo effect, fashion, vogue, and cultism.

It shapes many human activities.

It is an aspect of every helping situation.

Not only may it serve to bring about a relief of tension, but, if the helping agency happens to possess the proper values, it may, through the instrumentality of identification, register a reconstructive personality alteration.

Suggestion operates in all forms of psychotherapy even psychoanalysis (Bibring, 1954), and it has even been postulated that the suitability of an individual for treatment is dependent on the potential openness to the suggestive influence (Strupp, 1972b).

The exact nature of suggestion as a psychological phenomenon is not understood, and its eventual identification may supply the sought-for synthesis between the various schools of psychology (Winkelman & Saul, 1972).

www.freepsychotherapybooks.org 55 During psychotherapy the therapist deliberately or obliquely throws out ideas and directives that the patient often will pick up and utilize.

Sometimes these suggestions are helpful and constructive, at other times less so.

The therapist may not be aware that cues are constantly being released to the patient, not only from verbal statements, but also from nonverbal signals such as facial expressions of approval or disapproval, hesitations, silences, the accenting of some of the patient's comments, nodding, shaking the head, grunting, and various physical movements.

In this way, suggestive elements may come through, even where the therapist believes that little or nothing is being said.

A simple nod in relation to something the patient has said or done will give the patient the idea that what is being said or done is good and that one should continue in this approved vein.

But if the therapist says nothing or shakes the head, this may constitute an aversive suggestion for the patient and deliberately or unconsciously discourage certain types of activity.

The technique of paying no attention to a psychotic person's delusional ramblings or hallucinations, but expressing interest in the patient when acting reality-oriented will tend to reinforce constructive preoccupations.

All patients pick up from verbal and nonverbal cues of the therapist certain things they should do and believe in.

This includes insights that initially act as a placebo.

Even if an insight is wrong, if the person believes in it and imagines that it can help, he or she may start feeling better as a placebo consequence.

In dynamically oriented psychotherapy, direct suggestions are kept at a minimum, the patients being encouraged to think through their own solutions.

Nevertheless, the factor of prejudicial, inexpedient, or unwise suggestions unwittingly being

made must always be kept in mind.

On the other hand, if the suggestions are productive ones, the patient may benefit from pursuing them.

Nor is it essential in therapy always to abstain from direct suggestions or ego-building persuasive formulations.

Homework given to the patient is an example of the constructive use of suggestion.

The therapist will have to gauge the patient's readiness to experiment with any anxiety-provoking action before making a direct or indirect suggestion that the patient undertake it.

A premature exposure resulting in failure may merely intensify a phobia.

www.freepsychotherapybooks.org 56 A number of variables may be distinguished that appear to regulate the forcefulness of suggestion (Wolberg, 1962).

The first variable is concerned with the significance to the individual of the suggesting agency.

The anachronistic residue of need for a protective parent that is present in all persons makes the individual more suggestible in the presence of an authority symbol who approaches an ideal.

The higher the dependency level the more apt is the individual to design out of every relationship a child-parent tableau.

Behind the helplessness of such gestures is a drive to absorb the strength of the authoritative token through submissive identification.

By yielding to the authority's power the hope is to become powerful.

Where the dependency need is sufficiently intense, the individual may respond to instrumentalities that possess any kind of protective promise—including drugs, placebos, faith healing, yoga, hypnosis, psychotherapy, etc.

Another factor that heightens dependency, and hence intensifies suggestibility, is anxiety.

Individuals whose mastery is shattered will cling with desperation to any potential helping resource, responding dramatically to preferred injunctions and commands.

The intrusion on the individual of some catastrophic event that damages defensive integrity and diminishes security and self-confidence will thus tend to lower the suggestive threshold.

Soldiers after battle, and persons exposed to accidents and natural disasters, often verbalize the need for a loving, protective agency.

Under such circumstances the person may respond intensely to suggestions that are more easily resisted later.

A momentary regression to the actual or fantasied securities of childhood is the prompting that inspires the anxiety driven soul to aggrandize the expediency that is offered.

Qualities in the helping agency positively correlated with suggestibility are those that inspire confidence in the subject and raise expectations of spectacular performance from the agency.

Negatively correlated are characteristics that promote lack of confidence or that arouse resentment.

Appearance, diction, status, reputation, and fame usually enhance the authoritative image.

Group acceptance and acknowledgment of power of the authority as well as strong charisma of the authority augment the suggestive mystique.

The second variable is the significance to the subject of the specific content of the preferred suggestions.

The precise meaning of the communications to the individual is www.freepsychotherapybooks.org 57 what is of determining importance, rather than their true content.

Many vectors are operative, not the least of which is motivation.

If the recipient of a communication, verbal or nonverbal, is attuned to respond positively,

material will be plucked out of context that seems to justify a positive response. Utterances by prophets wedded to contemporary philosophical and religious movements may sound nonsensical to a casual and uninvolved observer, yet to a believer they are spell-binding and heavenly inspired.

Every individual is to some extent the victim of a mirage inspired by the projection of inner needs.

Thus the world is fashioned to personal improvisations.

If the helping authority is able to divine the incentives of the client, more suitably meaningful arguments and memoranda will be supplied.

The third variable that modulates suggestibility is the degree of anxiety that is mobilized in the subject by the acceptance of a specific suggestion or by the relationship itself.

Individuals who fear submitting themselves to others, who retreat from domination, who are reluctant to yield to their dependency drives, who are compulsively independent, who preserve a defensive detachment, or who are fiercely competitive may resist suggestions, even those that can be helpful to them, no matter how convincingly these are phrased.

Transfereential contaminants often enter into relationships, distorting their dimensions.

To some extent, transference may be controlled if the helping person is aware of its presence and restrains untoward countertransfereential impulses that neurotically interlock with those of the subject.

Nevertheless, even though the helping person attempts to keep the relationship reality-oriented, it may disintegrate through assignment to the agency of the most noxious characteristics of significant personages in the subject's past.

As a hated parent, the sentiments of the agency will be bitterly resisted.

In the absence of obstructive transference, suggestions may still be resisted if they challenge or run counter to vital defenses.

In the latter case, anxiety will tend to neutralize or modify the suggestion.

This was dramatically illustrated by a patient I was asked to see to help differentiate psychic from organic pain.

The surgeon who had advised a disc operation for severe back pain suspected a hysterical reaction; to rule this out, he requested a consultation.

To see if the patient's pain could be removed by suggestion www.freepsychotherapybooks.org 58 (which would help establish a functional basis for the pain), I induced hypnosis, during which I suggested that upon awakening the pain would disappear from his back, but would instead be transferred to his right arm.

The patient responded positively with considerable surprise.

Upon rehypnotizing him, I demonstrated to him that I could transfer the pain from his arm to his right leg.

Thereafter I attempted its displacement to his left arm.

To my consternation this suggestion was resisted, but instead the pain was again felt in the habitual back zone.

I reinduced hypnosis and repeated the transfer of pain to various areas, which proved successful except for the left upper extremity.

Questioned under hypnosis regarding this puzzling phenomenon, the patient replied that his father had died from a heart attack.

Prior to his death he had experienced angina pectoris, with referred pain to his left shoulder and arm.

The patient feared succumbing to the same illness.

Apparently the meaning to him of accepting my suggestion to feel pain in the left arm was to acknowledge succumbing to angina pectoris.

His anxiety blocked my suggestion.

Where suggestions upset the adaptive equilibrium, they will tend to be resisted.

Thus, where repression is threatened by a suggestion, or where positive gains and pleasure associations are in danger of renunciation, or where unconscious masochistic needs power the existing neurotic operations, the individual may be oblivious to the most sensible maxims or magisterial commands.

If prevailing values and philosophies are contradicted, suggestions may also be repelled.

Suggestions are best formulated, therefore, in terms that are congruent with the person's ideologies.

The fourth variable concerns the subject's critical judgment.

In spite of the fact that suggestion resides in the penumbra of illogic, critical reasoning and common sense are still posted as sentinels.

Periodically they will interfere with suggestion's automatic pursuits.

One may observe this in somnambulistic subjects during hypnosis, who, compulsively responding to irrational posthypnotic suggestions, force themselves to challenge, to inhibit, and, finally, to oppose acts that are foolish or without purpose.

Eventually, an interference with helping exhortations will probably occur should they not harmonize with the individual's intellectual understanding.

First accepted on faith, they www.freepsychotherapybooks.org 59 finally fall under the critique of logicity.

CONSTRUCTIVE GROUP EXPERIENCES (GROUP DYNAMICS) Groups exert a powerful influence on individual.

They may be responsible for significant changes among the constituent members.

The effect of Alcoholics Anonymous on victims of drink, of Synanon on drug addicts, and of Recovery on former hospitalized patients are examples of how even serious personality and emotional problems may be benefited through constructive group adventures.

In order for change to occur, however, a new and unique group experience is required, one that contributes to the individual's security and desires for belonging, at the same time that it challenges the customary patterns of thinking, feeling, and acting.

This generally occurs when the person is forced to participate in a problem-solving situation with an assemblage that is respected, the members of whom join their forces for some united purpose.

For example, a body of parents may meet at regular intervals for deliberation and action on problems affecting their children at school.

A special committee of a fraternal order convenes for a cooperative pooling of ideas and skills.

A council is elected in a neighborhood to devise tactics to fight racial discrimination.

A select crew of supervisors in an industrial plant organize to bolster the morale of the workers.

A small recreational or hobby club is developed that offers an exciting program.

Even though individuals have functioned in various groups during their life span with usual controls and defenses, the new group, if it is to be effective, will not sanction, indeed may challenge, their customary behavior if it runs contrary to the group's standards.

This creates conflict and may force them to interact in a different way.

Deviants—that is, persons whose standards are in conflict with the group norms—are particularly affected since their conduct upsets the members, who may turn on the deviants and try to force them to conform.

If the person has an investment in remaining with the group, the price may be a change in manner or ways of thinking.

As people collaborate in activities and relationships, they learn which of their traits and actions encourage rewards and which bring punishments.

Thus a person who seeks to www.freepsychotherapybooks.org 60 control the group will sooner or later realize that domineering tendencies are resented, but that acceptance depends on acting cooperatively.

The timid, submissive, and detached soul may be goaded into harmonious team work and assertiveness as security in the group becomes more firmly entrenched.

Identification with the group assuages the individual's helplessness and isolation.

Approval and disapproval cut deeply into personality defenses.

As the individual reacts repetitively with customary patterns to manipulate, monopolize, bully, undermine, lampoon, withdraw, reassure, comply, or collapse, oppositional responses of the group create a crisis.

Attacked, the person may then be tempted to leave the group or may be invited to leave.

It is at this point that change becomes possible, prompted by a need to remain with the group.

The individual may then learn that it is not necessary to shout to be heard or to flaunt one's virtues to be acknowledged.

There may be the realization that criticism can be constructive without offending and, conversely, that one can be criticized without being a victim of an evil design; that one may relate without becoming enslaved, accept help without loss of status, stand up for one's rights without being attacked, perform without being rejected, give without feeling exploited; and that one can be liked for what one is and not for what one does.

These experiences have a forceful effect, particularly where defenses are not too rigidly set. A remodeling of values, attitudes, and behavior follows upon a more accurate perception of one's role in the group.

Actually, while these happy effects are psychotherapeutic, they cannot be considered in the same light as those accomplished through a structured psychotherapeutic program, for instance, group psychotherapy.

Social or problem-solving groups are organized with an objective other than the treatment of its members; beneficial results in personality are a mere by-product of the existing group dynamics.

While the individual may retain new patterns, generalizing them toward situations other than the group in which they were evolved, the tendency is to slip back into old habits when pressures to conform are no longer present.

If, however, an understanding of disturbed ways of relating has been integrated, a permanent effect on character structure may be registered.

www.freepsychotherapybooks.org 61 DYADIC GROUP DYNAMICS In individual psychotherapy a dyadic group situation exists that inspires some of the identification phenomena observed in larger groups.

Various cues (e.g., the magazines and books in the waiting room, the pictures on the walls, the office furnishings and decor, the style of therapist's clothing and grooming, and the therapist's comments and behavior) will lead the patient to speculate on the value systems of the therapist and to attempt selfmodeling along similar lines, or at least to challenge some personal standards and patterns.

In short-term therapy interpersonal dynamics involving the therapist must be considered also as a factor in reeducation.

Karl Menninger (1952a) has emphasized that the value of the psychiatrist is greatest as a person rather than as a technician or scientist.

What one is has more effect upon the patient than anything one does.

Because of the intimate relationship between patient and psychiatrist, the value systems,

standards, interests and ideas of the doctor become important." A therapist cannot help but communicate personal values to patients.

This will occur no matter how passive, non-interfering and nonjudgmental the therapist tries to be.

An attempt may be made to suppress verbal valuations, but nonverbal prejudices will nevertheless come through.

Nods, grunts, groans, smiles, immobility, fidgetiness, pauses, choice of topics for questioning, emphasis, repetitions, and interpretations will soon convey to the patient the therapist's worldviews and tendencies toward deviance and conformity.

Subtle indications such as the kind of waiting room furniture, pictures and magazines, the therapist's hair style and clothing preferences, and the manner in which office routines, billing, and appointments are conducted are as eloquent in revealing standards as any direct verbal avowal of values.

It is useless to try to conceal the fact from the patient that the therapist has a definite point of view and possesses distinctive tastes and prejudices.

Indeed, the patient may even divine unconscious values and during transference confront the therapist with a bill of particulars, the validity of which may be staggering.

www.freepsychotherapybooks.org 62 If a therapist's values are apparent to the patient, the question may be asked whether to express them verbally as articles of personal conviction, at least those of which the therapist is aware?

It is obvious to most therapists who believe this that they must nevertheless restrain themselves from political and other declarations, and especially not force their values on patients, even when the therapist is convinced of their moral and pragmatic worth.

The patient's right to accept or reject the therapist's standards is usually respected.

Moreover, the therapist, assuming an ability to be objective, may subject personal value systems to soul-searching to discern which of these are warped.

A frank encounter with oneself, buttressed if necessary by personal therapy, may be a boon to both therapist and patient, since many patients will incorporate the therapist's theories and moral precepts more or less uncritically on the basis of a need to please, in order to learn from and amalgamate with the idealized authority figure who is rendering help.

This is probably allied to mechanisms that take place in any educational process.

EFFECTS OF EXTRATHERAPEUTIC AIDS ON PSYCHOTHERAPY How can we assay the effect of ad non-specific changes that have nothing to do with a formal psychotherapeutic situation?

It is doubtful if we can fully differentiate intercurrent from therapeutic factors.

Forces prevailing upon the individual outside of the psychotherapeutic process are often so elusive that they defy detection, let alone analysis.

For example, results of psychotherapy are undoubtedly influenced by the implicit assumption of the patient that one is expected to and will change for the better.

Under these circumstances, irrespective of the specific theories of the therapist and the tactics that the therapist employs, the patient may tend to show improvement.

Suffice it to say that the therapist accepts with gratitude the fact that there are supplementary "spontaneous" and "helping" healing aids to assist in therapeutic efforts.

The therapist tries not to interfere with these elements except insofar as they detract from treatment objectives.

For example, where the individual utilizes escape and control mechanisms to withdraw from relationships, or where existing philosophies negate a productive adjustment, the therapist will treat these devices as resistances to productive www.freepsychotherapybooks.org 63 change. But, by and large, the adventitious ameliorative agencies are accepted as unavoidable.

Some psychotherapeutic systems attempt to encourage them, exhorting the individual to seek out

spontaneous activities that help in growth; creating an atmosphere in therapy to inspire trust, faith, and confidence in order to enhance the placebo element; stimulating emotionally cathartic expressions of feeling; promoting a positive relationship situation by assuming the role of a benevolent, giving authority; suggesting participation in social or problem-solving groups; and fostering the development of philosophical formulas that take pressure off the individual and enhance a new way of looking at one's responsibilities.

Indeed some treatment schemes, adopting the tactic that if we cannot eradicate the coincidental therapeutic forces, we should join them, incorporate the extratherapeutic aids into the body of their systems.

From a practical point of view, the therapist may assume that whether he likes them or not, these elements are going to play some part in the patient's getting better.

The therapist would do well then to acknowledge their part in any results claimed.

Logic convinces that the effect of any psychotherapy actually is a summation of that psychotherapy plus intercurrent non-specific healing adjuncts.

Once we stop struggling against the impossible task of separating these two protagonists, we can apply ourselves to the task of utilizing both psychotherapeutic techniques and coincidental constituents that make for therapeutic gains.

Can we delineate the kind of psychotherapeutic climate that will expedite the nonspecific healing forces?

A considerable number of observers have attempted to do this in the past, among whom are Alexander (1948), Rogers (1951), Fromm-Reichmann (1952b), Raush and Bordin (1957), Strunk (1958), Halpern and Lesser (1960), Strupp (1960), Jourard (1959a), and Truax and Carkhuff (1964).

We may speculate that the climate of successful therapy depends on a number of factors that are operative in all good helping situations.

These include the following: 1.

Hope.

Patients approach the ministrations of the therapist with assured expectancy.

A sanguine anticipation of success is present.

Therapists have confidence in their methods and theories.

This activates the placebo effect.

2.

Trust.

Patients see their therapists as sincere, honest, reliable, guileless,

www.freepsychotherapybooks.org 64 trustworthy, undeceitful, unaffected, straightforward, and authentic beings.

Therapists display respect for their patient in spite of the latter's pathological behavior.

Therapists also reveal their own genuineness.

3.

Freedom to respond.

Patients permit themselves to think, feel, and act without restraint.

This permits them to verbalize freely and to unburden themselves.

Therapists encourage freedom in reactivity.

This sponsors emotional catharsis.

4.

Faith.

Patients have a conviction that therapists possess the percipience, sensitivity, wisdom,

experience, skill, and ingenuity to understand basic presenting problems and difficulties and to know what to do about them.

Therapists convey assuredness, positiveness, and empathic understanding. This facilitates the suggestive factor.

5.

Liking.

Patients conceive of their therapists as empathic individuals who relate to them personally and warmly; in turn, therapists feel a nonpossessive warmth toward the patients.

This releases dynamic ingredients present in all human encounters (dyadic and larger groups) that constructively influence attitude change.

www.freepsychotherapybooks.org 65 5 Significant Variables that Influence Psychotherapy Apart from extratherapeutic aids that can positively influence results in psychotherapy, multiple variables enter into the treatment gestalt that bear upon results for the good or bad.

At the outset it is necessary to recognize that no psychotherapeutic method exists today that is applicable to all patients or germane to the styles of all therapists.

As disparate as the various approaches to psychotherapy may seem, their impact on the patient is often registered in similar ways.

First, they offer a unique kind of interpersonal relationship in which one feels accepted for what one is and where judgements concerning attitudes and behavior do not agree with habitual expectations.

Second, there is an explicit and implicit reinforcement of selected responses with the object of overcoming important behavioral deficits and of extinguishing maladaptive habit patterns.

Third, there are direct or indirect attempts made at cognitive restructuring, through various instrumentalities, such as (1) persuasive arguments and proffering of philosophical precepts, (2) the exploration of conscious and unconscious conflicts aimed at the inculcation of insight, and (3) the provision of a corrective behavioral and emotional experience within the matrix of the patient-therapist interaction.

Irrespective of behavioral parameters that purportedly are selected for inquiry and rectification, the patient will respond to the therapeutic interventions being utilized in accordance with personal needs and readiness for change.

These facts have fathered a common idea among professionals that therapists of different theoretical orientations do essentially the same things.

The patient presents a problem; an attempt is made to establish a meaningful relationship; some formulation is presented to the patient as a working hypothesis; and special procedures are implemented to enhance the patient's mastery and eliminate disruptive elements in adjustment.

If this be true, then techniques are merely forms of communication secondary

www.freepsychotherapybooks.org 66 to a host of transactional processes that draw from many biological, intrapsychic, and interpersonal vectors.

A reasonable question that may be asked is whether current research can shed some light on how these vectors can be organized and manipulated to make our operations more effective.

Specifically, do we have reliable data that will permit us to match patients, therapists, and techniques?

Offhand we must admit that empirical studies to date have not settled this question.

In an extensive review of research on the assessment of psychosocial treatment of mental disorders by an NIMH working group functioning as an advisory committee on mental health to the Institute of Medicine of the National Academy of Sciences, it is concluded that we do not yet have the answers to the basic question of "what kinds of changes are effected by what kinds of techniques applied to what kinds of patients by what kinds of therapists under what kinds of conditions" (Parloff et al, 1978a).

We have insufficient data to date regarding the relative effectiveness of the different therapeutic modalities as well as the patient-therapist preferences to make a scientific

matching feasible.

When we consider the choice of the best kind of therapy, the report sums up the following way: "In summary, the data does not show that any of the tested forms of therapy are unconditionally superior to any other form." The report goes on with the statement that the existing single studies have proved inadequate to answer questions that we need to resolve.

What is required is a coordinated planning of wide-scale sophisticated research utilizing an agreed upon minimal set of standardized measures for describing and assessing the kinds of problems, the variety of treatment interventions, and the nature and degree of behavior change. Since help from present-day research is so problematic, we must depend on clinical hunches in sorting out the significance of the many variables in psychotherapy.

For convenience we may group these into: 1.

Patient variables 2.

Therapist variables 3.

Social and environmental variables www.freepsychotherapybooks.org 67 4.

Transfereential and countertransfereential variables 5.

Resistance variables PATIENT VARIABLES The Syndrome or Symptomatic Complaint Are there special techniques that coordinate best with selected symptom complexes, methods that are more rewarding with some types of complaints as compared to other types?

How really important are techniques in psychotherapy?

Can different therapists get the same results by utilizing various kinds of interventions with which they are individually expert?

By and large techniques do operate as a conduit through which a variety of healing and learning processes are liberated.

How the techniques are applied, the faith of therapists in their methods, and the confidence of the patient in the procedures being utilized will definitely determine the degree of effectiveness of a special technique.

But techniques are nevertheless important in themselves and experience over the years with the work of many therapists strongly indicates that certain methods score better results with special problems than other methods.

Thus symptoms associated with biochemical imbalances, as in schizophrenia or affective disorders, may be relieved with proper psychotropic medication, for example, neuroleptics in the case of schizophrenic thinking disorders, antidepressant drugs in depressive states, and lithium for manic excitement and bipolar depressions.

While thinking disorders in schizophrenia are helped by neuroleptics, problems in social adjustment are better mediated by behavior therapy, family therapy, and counseling.

Indeed, milieu therapy in the form of a therapeutic environment staffed by accepting, supportive persons or proper accepting, nonstressful surroundings in which to live and work (Warner, 1985) often permit an adequate adjustment without the use of drugs.

Such a congenial atmosphere must be continued indefinitely, in some cases over a lifelong period.

The presence of a hostile or nonaccepting family correlates with an increased www.freepsychotherapybooks.org 68 rehospitalization rate.

Research studies, i.e., controlled outcome studies for individual or group therapy in schizophrenia, provide no strong evidence for or against the value of psychotherapy.

Generally with continued aftercare following hospitalization in the form of a behaviorally oriented program and an adequate residential regime (halfway houses, partial hospitals, rehabilitation units and other community support systems), over 90 percent of patients can be discharged in the community with a 2-year rehospitalization rate of less than 5 percent (Paul & Lentz, 1977).

Our experience at the Postgraduate Center for Mental Health confirms the value of a rehabilitation program in lowering rehospitalization rates and, in rehospitalized patients, reducing the time of confinement.

Behavioral token economies are often valuable in hospitalized patients toward regulating ward behaviors in areas such as self-care, grooming, and social adjustment, and to control target behaviors such as job performance.

Behavioral extinction procedures have been utilized to manage gross pathological behavior, like violence and destructive tantrums in psychotic autistic children, especially when coupled with differential reinforcement of constructive behavior.

Such behavioral gains are consolidated when management techniques are taught to parents or other adults with whom the patient lives.

No cure of the basic condition is brought about by these methods, but definite improvement of behavioral repertoires may be achieved.

While drugs influence depressive symptoms, they have little effect on patterns of behaving and thinking, which are best mediated through dynamic interpersonal therapy, cognitive therapy, and behavior therapy aimed at reinforcing involvement with pleasant activities and changing target symptoms and behavior toward improvement of interpersonal relations and social functioning.

Combined drug and psychosocial therapy is thus best in most depressions.

In suicidal depression electroconvulsive therapy has proven to be a lifesaving measure.

In hyperkinetic children it is generally found that we may be able to reduce the overactivity with psychostimulant medications, like Ritalin® and Dexedrine®, plus behavioral forms of reinforcement of positive behavior or such mild aversive techniques

www.freepsychotherapybooks.org 69 as removing the child with disturbing behavior to a "time-out" room for a few minutes.

To influence neurophysiological dimensions, as reflected in severe tension states, simple relaxation procedures like meditation, relaxing hypnosis, or autogenous training, or emotive release therapy have been instituted.

Certain psychosomatic conditions, such as muscle spasms, migraine, hypertension and arrhythmia, may sometimes be helped by biofeedback.

Unfortunately, biofeedback has been oversold, its utility being limited to selected patients (Miller, 1978).

Behavioral advocates claim that best results with obesity are obtained with measures directed at self and stimulus control.

Behavioral operant reinforcement techniques and systematic desensitization are being used in anorexia nervosa, although the evidence of their effectiveness is still unclear.

More recently progress has been reported with anorexia and bulimia utilizing antidepressants.

Smoking control, it is claimed, is best handled by multicomponent treatment packages that program a reinforcement of nonsmoking behaviors and utilize smoking suppressive tactics (Bernstein & McAlister, 1976) supplemented by nicotine gum chewing.

Stimulus satiation, i.e., rapid smoking to satiation, is temporarily effective but the physical side effects must be considered.

Hypnosis is also useful in combination with follow-up behavioral methods.

In enuresis behavioral approaches (such as a urine alarm bell) are claimed to be superior to dynamic psychotherapy and imipramine.

Sexual disorders (voyeurism, fetishism, sadomasochism, exhibitionism, transvestism), though difficult to treat, are approached by some therapists with aversive therapy, such as electrical stimulation for transvestism (Marks, 1976) and with certain idiosyncratic methods like having an exhibitionist appear before a female audience to expose himself while talking about his condition, the extraordinary procedure being credited with some success on follow-up

(Wickramasekera, 1976).

Sexual dysfunctions (impotence, frigidity, dyspareunia, vaginismus, anorgasmia, premature ejaculation) do well with various behavior therapies, hypnosis, and dynamic psychotherapy. Of all of these programs, the Masters and Johnson techniques have proven most popular and have in the opinion of some authorities been made more effective when blended with www.freepsychotherapybooks.org 70 psychodynamic and interpersonal techniques (Sollod & Kaplan, 1976).

Phobias that have their origin in conditioning experiences (i.e., exposure to parental fears, like a mother who is in terror of mice or a father who shies away from heights), or to a catastrophic personal happening (i.e., an accident in an airplane in which a person was traveling, resulting in a fear of flying), or to an anxiety or physiologically distressing experience linking itself to a coincidental stimulus (i.e., nausea or gastric upset occurring at the same time that one is eating a certain food, resulting in a subsequent refusal to eat that food), seem to respond more rapidly to systematic desensitization, flooding or graded exposure to a phobic object than to any other kind of technique.

On the other hand, phobias that are the product of deep personality conflicts, the projected symbols of unconscious needs and fears, are in a different class from conditioned phobias and do not respond as well to behavioral methods.

In fact, they may stubbornly resist those techniques and are better suited for dynamic approaches.

Of all phobias, agoraphobia seems to be the most resistant to verbal therapies but may respond to antidepressant drugs.

About two-thirds of patients with anxiety reactions are helped by both dynamic psychotherapy and behavioral techniques, such as systematic desensitization, progressive relaxation, "participant modeling" (i.e., the therapist modeling how to master anxiety), and temporary drug therapy.

Behavioral therapy has moved toward recognizing the importance of cognitive factors (such as irrational self-statements and false attributes) and the need not only to modify environmental parameters, but also to consider the patient's interpretation of events and thoughts that mold reactions to the environment.

In line with this, applications of behavior therapy have expanded toward a wide spectrum of neuroses and personality disorders.

Some obsessive-compulsive disorders respond to such antidepressants as clomipramine (Anafranil®), others come under control with aversive behavioral techniques when they resist every other type of manipulation.

Prolonged exposure to thoughts or situations that provoke compulsive rituals combined with blocking of the www.freepsychotherapybooks.org 71 patient from engaging in such rituals has yielded some successes.

In vivo exposure is superior to exposure to fantasies.

We may minimize persuasion as a technique, but it can have a potent effect on some obsessive states, adjustment reactions, and related conditions (Truax & Carkhuff, 1967).

Stubborn as they are, personality problems and most neurotic disorders that are bracketed to personality problems are conditions most subject to the utilization of a dynamic orientation that probes for provocative conflicts and defenses.

Relationship difficulties are particularly suited for group therapy, marital therapy, and, especially where children are involved, family therapy.

Some addictions seem to respond best to certain inspirational groups (e.g.

drugs with Synanon, alcohol with Alcoholics Anonymous, food gorging with Weight Watchers, gambling with Gamblers Anonymous).

These groups are preferably led by a person who has gone through and has conquered a particular addiction and is willing to serve as a model for identification.

Accordingly, where a therapist works with such groups, an exaddict co-therapist may be a great asset.

Where repressions are extreme, classical psychoanalysis, intense confrontation, hypnoanalysis, narcoanalysis, and encounter groups have been employed in the attempt to blast the way through to the offensive pathogenic areas.

Understandably, patients with weak ego structures are not candidates for such active techniques, and therapists implementing these techniques must be stable and experienced. Antisocial personalities subjected to a directive, authoritarian approach with a firm but kindly therapist sometimes manage to restrain their acting-out, but require prolonged supervised overseeing.

Apart from the few selected approaches pointed out above that are preferred methods under certain circumstances, we are led to the conclusion that no one technique is suitable for all problems.

Given conditions of adequate patient motivation and proper therapist skills, many different modalities have yielded satisfactory results.

It is my feeling, however, that whatever techniques are employed, they must be adapted to the patient's needs and are most advantageously utilized within a dynamic framework.

www.freepsychotherapybooks.org 72 Transference reactions may come through with any of the techniques, even with biofeedback and the physical therapies.

Alerting oneself for transference, one must work with it when it operates as resistance to the working relationship.

Unless this is done our best alignment of method and syndrome will prove useless.

The fact that certain techniques have yielded good results with special syndromes and symptoms does not mean they will do so for all therapists or for every patient.

Interfering variables, such as will henceforth be described, will uniquely block results or will make the patient susceptible to other less popular methods.

Selective Response of the Patient to the Therapist At its core the patient's reaction to the therapist often represents how the patient feels about authority in general, such emotions and attitudes being projected onto the therapist even before the patient has had his first sessions.

The patient may rehearse in advance what to say and how to behave, setting up imaginary situations in the encounter to come.

Such a mental set will fashion feelings that can influence the direction of therapy.

Thus, if the patient believes that authority is bad or controlling, oppositional defenses may be apparent during the interview.

These global notions about authority and the reactions they sponsor are usually reinforced or neutralized by the response to the therapist as a symbol of an actual person important to the patient in the past (transference).

Some characteristic in the therapist may represent a quality in a father or mother or sibling and spark off a reaction akin to that which actually had occurred in past dealings with the person in question.

Or the reaction may be counteracted by a defense of gracious compliance or guarded formality. Confidence in one's therapist is enormously important—even when the therapist's ideas of the etiology or dynamics of the patient's difficulty are wrong.

The patient's acceptance of explanations proffered with conviction can have a determining influence on the patient.

They are incorporated into the patient's belief system and sponsor tension reduction and

restoration of habitual defenses.

Through what means this alchemy takes place is not clearly known, but suggestion, the placebo effect, and the impact of the www.freepsychotherapybooks.org 73 protecting relationship offered by the therapist undoubtedly play a part.

Sometimes unpredictable elements operate in the direction of cure.

This is illustrated by the case of a beautiful, poised female writer of 32 who was referred to me for hypnosis by a friend of mine after many attempts to stop smoking by herself, and after several unsuccessful tries at psychotherapy.

While there were no serious emotional problems that I could detect, and her present work adjustment, relationships with people, and sexual life seemed fairly satisfactory, one piece of data that she gave me put doubts in my mind as to how successful I would be in getting her off tobacco.

She confessed to me, somewhat shamefacedly, that she still used a baby pacifier that she stuck into her mouth alone at home (she was single) at times when a cigarette wasn't between her lips.

Often she went to sleep with it in her mouth.

I asked her if she knew what this meant.

"After all, doctor," she replied, "I'm no fool.

I'm just a plain oral character." Under the circumstances, I confessed to her that the chances of success with a symptom-oriented approach like suggestive hypnosis were poor, and that she had better get herself into analysis without wasting any more time or money.

"God forbid," she retorted.

Since I had promised to hypnotize her, I induced a trance with less than ardent enthusiasm, and went through my usual paces, hoping to demonstrate that my predictions of failure would surely come to pass.

Two days later she telephoned me to tell me she had quit smoking, and she asked if I would make a hypnotic tape for her.

We had three visits in all, and while she retained her discrete pacifier habit, she fooled me by staying off tobacco completely.

A year went by and she still had not resumed smoking, nor were there any substitute symptoms.

This abstinence was confirmed by several reliable friends who knew her well.

Now I have no idea of the dynamics of the symptomatic cure but it is possible that my pessimistic attitude was just the right tactic to use with her.

She may have decided to accept the challenge and to show me that I was wrong concerning my estimate of her.

Or perhaps the idea she would have to get into long-term analysis was more distasteful to her than giving up smoking.

Frankly, I was delighted that I was such a poor prognosticator, even though I still am convinced that a good analysis would have been a worthwhile investment.

The point I am trying to stress is that as thorough as we believe our initial interviewing may be as a way of estimating outcome, we still do not have all the variables at our fingertips.

We still can be right in our estimate, but sometimes, as in this case, we can be wrong, and to the patient's benefit.

Where the therapist becomes for the patient an idealized figure, the initial therapeutic impact may be enhanced.

Or if the transference is to an irrational authority, www.freepsychotherapybooks.org 74 resistance is more likely in evidence.

The degree of charisma possessed by the therapist also influences the patient's responses. Reputation, clothing, manner, and appearance all function to nurture the illusion of miracles

to come.

The various patterns that evolve have a powerful and often determining effect on any techniques the therapist may utilize.

Selective Response of the Patient to the Therapist's Techniques Patients occasionally have preconceptions and prejudices about certain techniques.

Hypnosis, for example, may be regarded as a magical device that can dissolve all encumbrances, or it may connote exposing oneself to Svengali-like dangers of control or seduction.

Misconceptions about psychoanalysis are rampant in relation to both its powers and its ineffectualities.

Frightening may be the idea that out of one's unconscious there will emerge monstrous devils who will take command—for example, the discovery that one is a potential rapist, pervert, or murderer.

Should the therapist have an inkling as to what is on the patient's mind, clarification will then be in order.

The manner of the therapist's style is also apt to influence reactions of rage at the therapist's passivity, balkiness at what is considered too intense activity, anger at aggressive confrontation.

Some patients are frustrated by having to talk about themselves and not being given the answers.

Moreover, responses to different methods will vary.

There is a story of a Gideon Bible (which may be apocryphal) that illustrates this.

As is known, practically all hotel rooms contain, as part of the general equipment, a Bible placed there by the Gideon Society, which in the front pages are suggested reading for the weary traveler.

For instance, if one is in need of salvation, the reference is to John 3:3; or desires peace to neutralize anxiety, Psalm 46; or relief in the time of suffering, Psalm 41; or consolation in conditions of loneliness, Psalm 23.

In a certain hotel Bible opposite Psalm 23, there was written in the margin this inscription:

"If after reading this Psalm you are still lonesome, upset, and feel life is not worthwhile, telephone 824-3921 and ask for Phyllis." This is eclecticism!

To help in the selection there are minimal techniques an eclectic therapist should know.

In my opinion, the following are most useful: dynamically oriented interview

www.freepsychotherapybooks.org 75 procedures, group therapy, marital therapy, family therapy, behavior modification, pharmacotherapy, cognitive therapy, relaxation methods, and hypnosis.

But irrespective of the techniques we select, we must be sure that they accord with the patient's belief systems.

As has been mentioned previously a patient who is convinced that spirit infestation is a cause of illness will do much better with a shaman or witch doctor than with a psychiatrist.

The patient's lack of faith in what the psychiatrist does will render worthless the most sophisticated treatment efforts.

Knowledge of the cultural concepts that mold a patient's ideas about what emotional suffering is all about may make necessary some preliminary education to prevent embarking on a futile therapeutic journey.

Some attempts have been made to assign patients to certain styles of therapeutic operation according to their unique characterological patterns (Horowitz, 1977).

For example, hysterical personalities are presumed to require a mode of therapist management that differs from that effective in obsessional personalities.

This distinction considers that certain optimal learning patterns correlate with identifiable personality typologies.

The relationship between character structure, diagnostic category, and learning abilities, however, never has been fully clarified.

Thus, among patients suffering from the same syndrome, let us say obsessive personality, there are great differences in the way they will respond to certain techniques and therapist styles. While some general principles may be applicable to all or most obsessives, the existing differences prevent our using a blanket approach.

People possess different modes, even within the same syndrome, of absorbing, processing, and responding cognitively, emotionally, and behaviorally to therapeutic interventions.

Since psychotherapy is in a way a form of reeducation, the learning characteristics of a patient should best correspond with the techniques that are to be used.

Problem-solving activities are often related to the kind of processes found successful in the past.

Some patients learn best by working through a challenge by themselves, depending to a large extent on experiment.

Some will solve their dilemmas by reasoning them out through www.freepsychotherapybooks.org 76 thinking of the best solution in advance.

Others learn more easily by following suggestions or incorporating precepts offered by a helpful authority figure.

Some are helped best by modeling themselves after an admired person, through identification with that person.

Some patients work well with free association, others do not.

Some are able to utilize dreams productively, or behavior modification, or sensitivity training, or other methods.

It would seem important to make the method fit the patient and not wedge the patient into the method.

The fact that learning patterns are so unique and modes of learning so varied lends justification to an eclectic approach.

It would be advantageous, of course, in the initial phases of therapy to find out how a patient might learn best, but no expedient is available today that can give us this information.

We usually settle for the fact that when a patient is first seen, learning capacities and styles are unknown and the therapist must proceed somewhat blindly.

We more or less shoot in the dark in order to coordinate a patient's specific problem and personality with our techniques.

We may get some help during interviewing in discovering how the patient has learned best in the past.

More pointedly, we get the most reliable data by actually exposing the patient to the interventions we have to offer and observing the response to what is being done.

A few other attempts have been made to identify variables that can make the selection of a therapeutic method more feasible.

Among these are the patient's response to hypnotic induction (Spiegel & Spiegel, 1978) and the isolation of core conflicts through the making of a developmental diagnosis (Burke et al, 1979).

Some interesting speculations may emerge in watching how patients react to the induction of hypnosis, but are these sufficiently reliable to pinpoint either the existing diagnosis or choices in therapeutic method?

More experimental substantiation is required.

Some therapists have attempted to utilize the area of developmental arrest as a way of selecting the ideal technique.

Where the patient's prime difficulty is centered around resolution of separation-individuation,

this is believed best accomplished through a technique such as described by Mann (1973) in which the struggle over short-term www.freepsychotherapybooks.org 77 termination of treatment threatens the patient's dependency, lights up the separation problem, and offers the opportunity to resolve the conflict in a favorable setting.

Where the oedipal conflict is primary, the confrontation styles of Sifneos (1972) and Malan (1964, 1976a) are believed to be most effective.

For problems originating in the latency period that precipitate out in the mid-life transition around issues of productivity and creativity, the "corrective action" approach of Alexander and French (1946) is recommended.

However, it is difficult to substantiate these views because of the interference of so many other variables that can vaporize our best choice-of-method intentions.

While empirical studies tell us little about factors that make for a good patient/therapist match, we may speculate that the personalities, values, and physical characteristics of both patient and therapist must be such that severe transference and unfavorable countertransference problems do not erupt to interfere with the working relationship.

A giving, accepting, warm, and active but not too interfering or obnoxiously confronting manner in the therapist is most conducive to good results.

Readiness for Change Another important factor is the individual's readiness for change.

This is a vast unexplored subject.

A person with a readiness for change will respond to almost any technique and take out of that technique what he or she is prepared to use.

What components enter into a satisfactory readiness for change have not been exactly defined, but they probably include a strong motivation for therapy, an expectation of success, an availability of flexible defenses, a willingness to tolerate a certain amount of anxiety and deprivation, the capacity to yield secondary gains accruing from indulgence of neurotic drives, and the ability either to adapt to or constructively change one's environment.

Patients come to therapy with different degrees of readiness to move ahead.

Some have worked out their problems within themselves to the extent that they need only a little clarification to make progress, perhaps only one or two sessions of therapy.

Others are scarcely prepared to proceed and they may require many sessions to prepare www.freepsychotherapybooks.org 78 themselves for some change.

We may compare this to climbing a ladder onto a platform.

Some people are at the bottom of the ladder and before getting to the top will need to climb many steps.

Others will be just one rung from the top, requiring only a little push to send them over to their destination.

In therapy we see people in different stages of readiness for change, and we often at the start are unable to determine exactly how far they have progressed.

One may arrive at an understanding of what is behind a patient's problem rapidly.

From this we may get an idea that benefits will occur with little delay.

Yet in relation to readiness for change, the patient may still be at the bottom of the ladder.

Others are at a point where almost any technique one happens to be using will score a miracle.

We may then overvalue the technique that seems to have worked so well and apply it to many different patients with such conviction that the placebo effect produces results.

We sometimes see patients who have been in therapy for a long time under the guidance of another therapist and who, on the surface, have made no progress at all.

Surprisingly, after a few sessions the patient will begin to progress remarkably, creating the impression in both participants that the therapist is a genius in being able to do in three sessions what the previous therapist was not able to accomplish in three years.

What actually may have happened is that the previous therapeutic effort succeeded in pushing the patient up the ladder toward the top, requiring only a bit more therapy to be able to take the last step that was accomplished by the second therapist taking advantage of the work achieved by the first.

I remember one patient who came to see me who advertised the fact that I had cured her in one session.

She had been under the care of physicians for years, suffering from a host of physical complaints, and her last few doctors, who were burdened by her incessant shifting symptoms, had given up on her as an obstinate hypochondriac.

No medicines seemed to help, and whenever psychiatric referral was mentioned she responded with an angry refusal and a host of new symptoms.

Finally, one day she announced in frustration to her doctor that she was ready to see a psychiatrist and the doctor then referred her to me.

At the appointed time a buxom, handsome, middle-aged lady walked into my office and from the moment the door opened started and continued talking without giving me a chance even to introduce myself.

My initial interview sheet in hand, I waited for a pause so that I could at least get some statistical data.

After what www.freepsychotherapybooks.org 79 seemed like an interminable span, she stopped for breath and I threw in an introductory: "By the way, how old are you?" Without pause she avoided the question and continued on her odyssey of voyages to doctors' offices around New York.

After several futile attempts to interrupt.

I gave up, put my sheet down, settled back in my chair and listened, interpolating an occasional "yes" and "hmm humm." I broke in at the end of the hour with: "Unfortunately our time is up." "Doctor," she said.

"I feel so much better—thank you very much," and she got up and walked out without making another appointment.

I was curious about what had happened, so that a week later, I telephoned her doctor.

His startling reply was: "I don't know what you did for her, but it was like a miracle.

For the first time since I've known her, she's lost her symptoms and is interested in getting out of the house and doing things.

And," he added humorously, "she says you are a brilliant conversationalist." Three months later the doctor called me and confirmed her continued improvement.

I do not know what happened but apparently my respecting her need to talk without interrupting was probably the best approach I could have used.

She was under such great tension that had I not permitted her to let off steam verbally I may have succeeded merely in frustrating and antagonizing her.

Grilling her with questions might have given me more information, but I am not sure what it would have done for her.

She apparently had climbed the readiness ladder by herself up to the top rung and what she needed to push her over the top was to have someone sit back and listen.

She undoubtedly wanted to talk freely to somebody who was not a member of her family, who was not going to judge her, who was passively objective, and who was, hopefully, nonjudgmental.

Whatever qualities she projected onto me, she utilized the relationship in line with her needs.

Obviously, while this passive stance worked well with this woman, it would not have served other patients who might have required more active confrontation over a more protracted period.

A person's readiness for change may, more than any other factor, be responsible for how rapidly movement proceeds in psychotherapy.

Degree and Persistence of Childish Distortions The distorted images of childhood, the

ungratified needs, the unwholesome defenses, may persist into adult life and influence the speed, direction, and goals of therapy.

These contaminations may obtrude themselves into the therapeutic situation irrespective of what kinds of technique are practiced.

Insidiously, they operate as www.freepsychotherapybooks.org 80 resistance and they can thwart movement toward a mature integration, no matter how persistent and dedicated the therapist may be.

For example, one patient, a successful businessman of 50, came to therapy in a crisis over abandonment by his mistress 25 years his junior who had run off with another man.

A deep depression and anxiety were the chief symptoms for which he sought help.

He blamed the young lady's defection partly on his inability to compete with his rival and partly on the ubiquitous duplicity of all females.

When he described his life, it became apparent that he had from the earliest days of adolescence looked for and pursued a certain physical type: blonde, fair skinned, long-legged, big-bosomed tarts whose teasing sexual provocativeness and irresponsibility added a fillip to his affairs.

He invariably would select young women who were unable to establish a meaningful relationship and who eventually, despite his wealth and generosity, would reject him and finally leave him.

This would make the relationship all the more precious in his mind and the rupture of the romance more disastrous.

During therapy he beat at himself, unmercifully proclaiming himself a worthless and destroyed person, a victim of the treachery of womankind.

No amount of reassurance, challenge, and interpretation could penetrate his overwhelming despair, and antidepressants proved of little benefit.

The fact that the great jewel in his life during the heyday of their relationship had led him a merry chase, deceiving and exploiting him mercilessly, made little difference.

Her destructive exploits constituted the main content of his discussions with me, but his depreciating her had little effect on his yearning.

After 12 sessions of gripes in therapy, the lady returned without warning, complaining that her most recent paramour was unworthy of her, unreliable and, most importantly, penniless.

She apologized for having hastily run off after knowing the man only several weeks and she begged the patient to take her back.

The effect on the patient was electric.

What common sense, medications, and psychiatry failed to do she accomplished in one evening. Depression, anxiety, and physical debility vanished.

To justify the reunion, the patient offered to bring the lady to me for an interview to prove that she was now reformed.

At the interview there was no question that she had been trading solely on her physical assets, which were indeed ample, but emotionally and intellectually there was much to be desired.

In fact, the most generous diagnosis one could bestow on her was that of a borderline case— and this was stretching a point.

It was certainly apparent insofar as my patient was concerned that the only motivation he had for therapy was a desire to dull the pain of deprivation, not to inquire into or eliminate its source.

My best efforts to halt the affair were of little avail since the lady could accomplish more with a casual pout than I could with all the armamentaria that Freud, Pavlov, and the other great pioneers had to offer.

And even though she soon again started her nonsense, he hung on desperately to the relationship at the same time that he bellowed like a wounded buffalo.

www.freepsychotherapybooks.org 81 In going into the history, the background for his enslavement became apparent.

The death of his mother when he was an infant and his placement with a series of relatives who provided him with a succession of nurses had failed to fulfill his need for real mothering. He was told, he said, that he was a colicky baby with respiratory trouble that was diagnosed as asthma.

He failed to see any connection between his childhood and what was happening to him in the present.

Interpretations of his orally frustrated dreams fell on deaf ears.

One day on inspiration I asked if he had ever seen a picture of his mother.

This he denied.

However, a later search by a relative through an old album yielded a startling picture that he excitedly handed me—a picture of a blonde beauty who presented an almost exact image of his present girlfriend.

It took no great work to convince him that he had practically all his life been searching for a physical duplicate of his mother.

This dramatic discovery had not the slightest effect on his futile mission to look for a symbolic breast, because after another abandonment, he started searching for a substitute blond, long-legged, big-breasted, unreliable paramour, whom unfortunately he found, starting a further round of exploitation, punishment, and anxiety.

Where severe traumas and deprivations are sustained in early infancy, especially prior to the acquisition of language, the damage may be so deep that all efforts to acquire that which never developed and to restore what never existed will fail.

Transference with the therapist may assume a disturbingly regressive form and, while the genetic discoveries may be dramatic, the patient, despite intellectual understanding, will not integrate any learning and will fail to abandon patterns that end only in disappointment and frustration.

Very little can be accomplished under such circumstances in short-term therapy, and even long-term depth therapy may lead to nothing except a transference neurosis that is difficult to manage or resolve.

Lest we be too pessimistic about what may be accomplished through psychotherapy, there are some patients who, though seriously traumatized, may when properly motivated be induced to yield the yearnings of childhood and to control if not reverse the impulses issuing from improper discipline and unsatisfied need gratification.

But this desirable achievement will require time, patience and, above all, perseverance.

APTITUDE FOR DYNAMICALLY ORIENTED PSYCHOTHERAPY Practically all people who apply for help in managing emotional problems can be approached successfully with supportive and educational therapies.

Eligibility for www.freepsychotherapybooks.org 82 dynamically oriented treatment, however, requires some special characteristics.

Some of the available research indicates that patients who respond best to psychodynamically oriented therapy need treatment the least.

What this would imply is that persons with good ego strength can somehow muddle along without requiring depth therapy.

That this is not always so becomes obvious when we examine the quality of adaptation of these near-to-healthy specimens.

In view of the shortage of trained manpower, we may want to look for characteristics in prospective candidates for therapy that have good prognostic value.

The following positive factors have been emphasized: 1.

strong motivation for therapy (actually coming to therapy represents some commitment); 2. existence of some past successes and positive achievements; 3. presence of at least one good relationship in the past; 4. a personality structure that has permitted adequate coping in the past; 5. symptomatic discomfort related more to anxiety and mild depression than to somatic complaints; 6. an ability to feel and express emotion; 7. a capacity for reflection; 8. desire for self-understanding; 9. adequate preparation for therapy prior to referral; and 10. belief systems that accord with the therapist's theories.

The patient's expectations, age, and socioeconomic status are not too significant, provided the therapist and patient are able to communicate adequately with each other.

CHOICE OF GOAL AND FOCUS www.freepsychotherapybooks.org 83 If a patient through therapy expects to be a Nobel Prize winner, the patient will be rudely disappointed and soon lose faith in the therapist.

There are certain realistic limits to how much we can accomplish through treatment, the boundaries largely being determined by the patient's dedication to the assigned task. Added to these are the curbs imposed by the many therapist variables soon to be considered. A great deal of tact will be required in dealing with inordinate expectations so as not to undermine further the already existent devalued self-image.

The selection by the patient of the area on which to concentrate during therapy is a legitimate and understandable theater around which initial interventions can be organized.

It may not be the most culpable area stirring up trouble for the patient.

But to push aside the patient's concerns with a symptom or a disturbing life situation and insist on attacking aspects of problems the patient does not understand or is not motivated to accept will lead to unnecessary complications and resistances.

It is far better to work on zones of the patient's interest at the same time that we make connections for the patient and educate the need to deal with additional dimensions.

Thus a man in the manic phase of a bipolar disorder may complain of a marital problem and press for its urgent resolution.

Should we attempt to bypass his complaint factor and merely press lithium on him, we may be rewarded with an abrupt termination of treatment.

His manic symptoms may subside, but his marital troubles may continue to plague him.

In attempting to choose the most productive arena for intervention we must keep in mind the fact that behavior is a complex integrate of biochemical, neuro-physiological, developmental, conditioning, intrapsychic, interpersonal, social, and spiritual elements intimately tied together like links in a chain.

Problems in one link cybernetically influence other links.

Elsewhere I have delineated the affiliation between the different links, contingent fields of inquiry, associated therapeutic modalities, and related syndromes.

Without denigrating the importance of the patient's chosen area of focus, we may most propitiously deal with a link in the behavioral chain that, in our opinion, needs the most urgent attention, that the patient is willing to work on, and that is realistically modifiable with the patient's current or potential resources.

Once we strengthen a pathogenic link www.freepsychotherapybooks.org 84 through therapy, the effects will usually reverberate through the entire behavioral chain.

THERAPIST VARIABLES Personality Factors Observations of the determining influence of personality traits in the therapist on outcome have been repeatedly made and reported by

Whitehorn and Betz (1960), Betz (1962), Rogers et al, (1967), Truax and Carkhuff (1967), and Truax and Mitchell (1972).

One finding is that a relatively untrained person with a concerned manner and empathic personality will get better immediate results, especially with sicker patients, than a highly trained therapist who manifests a “deadpan” detached professional attitude.

One should not assume from this that a therapist with a pleasing personality without adequate training will invariably get good results.

Some of the available research alerts us to the fact that the level of therapist expectations and the triad of empathy, warmth, and genuineness do not invariably represent the “necessary and sufficient” conditions of effective therapy (Parloff et al, 1978b).

A well-trained therapist, however, who also possesses the proper “therapeutic” personality is by far best qualified to do successful therapy.

A good deal of the flesh and blood of what happens in treatment, short-term or longterm, is provided by the relationship the therapist establishes with the patient, the quality of which is largely influenced by the therapist's personality in operation.

Personality expressions, good and bad, come through not only in the content of verbal communications but in nonverbal manifestations.

The latter are not merely the epiphenomena of interaction but are directly related to the outcome.

Nor does the factor of experience always operate to subdue damaging traits.

In some cases earnest, dedicated beginners may relate better to patients than do more experienced, scientifically oriented, passive practitioners.

Of course, we must consider some other variables also since we may be dealing here with different classes of patients, i.e., patients seeking a warm, giving authority as contrasted to those who want less personal involvement and greater ability to probe for and resolve defenses in quest of more extensive self-understanding.

www.freepsychotherapybooks.org 85 After many years of training students and observing their work, I would estimate that the most meritorious personality traits sponsoring a good relationship are objectivity, flexibility, empathy, and the absence of serious emotional pathology.

Successful therapists possess a bountiful blend of these attributes, unsuccessful therapists a dearth.

Where a student therapist in training possesses a healthy combination of such positive personality characteristics, we may anticipate a good career, although this is not guaranteed.

Where a candidate is less bountifully blessed, but cherishes rudiments of essential traits, these may be maximized by careful instruction and personal psychotherapy.

A few enter training with such rigid defenses that they scarcely budge even after years of intensive analysis.

How to find candidates who personality-wise have a good chance of becoming competent therapists is a challenge confronting all training institutions.

In the early days of existence of the Postgraduate Center for Mental Health, I once asked Paul Hoch, who was then Commissioner of Mental Health of New York State, what he believed the value to be of recommendations for admission to training from a candidate's personal analyst.

“From my own experience,” replied Hoch waggishly, “when you first start treating or supervising students, your immediate impression is that they are practically psychotic.

But shortly thereafter you develop a relationship with them and you believe that they are only neurotic.

And a while later you start endorsing them as either completely normal, or even better than normal.” Could it be, I inquired, “that therapy changed their personalities for the better?”

“More likely,” answered Hoch, “as a therapist works with a student, this changes the therapist’s judgment for the worse.” What he was referring to, of course, was the ubiquitous problem of countertransference that can mask or distort one’s appraisal of a partner in a relationship dyad.

Other criteria for selection are undoubtedly more reliable than endorsements from one’s personal analyst.

But the real test of how effective a candidate will be as a therapist is the actual performance with patients with varying syndromes and degrees of pathology, under the surveillance and tutelage of a competent supervisor.

Observation of a student’s performance behind a one-way mirror and the use of videotape recordings are also of substantial help.

Specifically, especially where the patient requires rapid stabilization, the therapist’s manner must convey empathy, confidence, and understanding of the patient’s turmoil and what is behind it.

The patient, no matter how upset, will usually discern these qualities in the therapist and react to them.

It is important also that therapists be able to control their own difficulties sufficiently so as to avoid the pitfalls of their own countertransferential problems interlocking with the problems of their patients.

Particularly important is sensitivity to and the ability to manage irrational projections of patients, hallmarks of transference neuroses.

Considering that desirable character traits, if absent, are difficult to acquire even with personal therapy, it would seem important in the selection of candidates for training that some criteria be available to spot in advance students who possess or will be able to develop appropriate personality characteristics.

This is more easily said than done.

When we first founded the Postgraduate Center we experimented with many devices, including projective psychological testing and structured and nonstructured interviews.

We failed to come up with any foolproof selection procedure.

This is probably because the role that a candidate plays with an interviewer or psychologist tester is different from that assumed with patients.

My own experience convinces me that two personality qualities are especially undesirable in a therapist doing therapy: First and most insidious is detachment.

A detached therapist will be unable, within the time span of treatment, to relate to the patient or to become involved in the essential transactions of therapeutic process.

A detached person finds it difficult to display empathy.

To put it simply, one cannot hatch an egg in a refrigerator.

A cold emotional relationship will not incubate much change in treatment.

The second quality that I believe is inimical to doing good therapy is excessive hostility.

Where therapists are angry people, they may utilize select patients as targets for their own irritations.

A patient has enough trouble with personal hostility and may not be able to handle that of the therapist.

A therapist, exposed to a restrictive childhood, having been reared by hostile parents, or forbidden to express indignation or rage, is apt to have difficulties with a patient who has similar problems.

Thus the patient may be prevented from working through crippling rage by the subtle tactic of the therapist changing the subject when the patient talks about feeling angry, or by excessive reassurance, or by a verbal attack on the patient, or by

making the patient feel guilty.

However, a therapist who is aware of personal hostile propensities, who can be objective about these and willing to back down under a patient's challenge, may be able to do fairly good therapy, provided the patient is permitted to fight back, and is not too frightened to challenge and stand up to the therapist.

But where the therapist refuses to allow the patient this freedom, and gets upset and vindictive, rejecting or punishing the patient, therapy has a good chance of coming to a halt. By the same token, a therapist who has serious problems with sexuality may not be able to handle a patient who also has certain sexual impediments.

For example, a therapist struggling with a homosexual impulse, may, when a patient with homosexual tendencies brings up the subject, become defensive, overmoralistic, punitive, or so intensely interested in the topic that the patient will be diverted from constructively dealing with other important concerns.

All in all, we hope that in doing therapy the therapist will be slightly less neurotic than the patient.

The least we may expect is that the therapist will have a reasonable capacity for maintaining objectivity.

Some neurosis or character disturbance is probably residual in all of us, but this need not interfere with doing good therapy provided that we have an awareness of our failings and do not permit them to contaminate the therapeutic atmosphere.

One of my teachers, an analysand of Freud, once remarked: "If there ever were such a monster as a completely 'normal' human being, he would very soon get psychotic trying to adjust to the rest of us neurotics." "Normal" probably embraces a host of minor neurotic vexations, but where a therapist finds that good results are not forthcoming, some personal psychotherapy or analysis would seem indicated.

Choice of Techniques Technical preferences by therapists are territories ruled by personal taste rather than by objective identifiable criteria.

As has been previously indicated, it matters little how scientifically based a system of psychotherapy may be or how skillfully it is www.freepsychotherapybooks.org 88 implemented—if a patient does not accept it, or if it does not deal directly or indirectly with the problems requiring correction, it will fail.

Because of the complex nature of human behavior, aspects that are pathologically implicated may require special interventions before any effect is registered.

Prescribing a psychotropic drug like lithium for an excited reaction in a psychopathic personality will not have the healing effect that it would have in violent outbursts of a manic-depressive disorder.

Unfortunately, some therapists still cling to a monolithic system into which they attempt to wedge all patients, crediting any failure of response to the patient's resistance.

A young analyst, for example, one year after having completed her training in a classical psychoanalytic school, wanted to get some further training in hypnosis.

It turned out that her entire patient load was exactly the same one she originally had when she started her personal analysis some years previously.

All nine patients were being seen four or five times weekly, associating freely in the couch position, analyzing their dreams, but showing little or no improvement, some even regressing.

The therapist, maintaining the traditional passive stance, tried to listen to what they were saying, but found herself getting more and more bored and increasingly discouraged at what was happening.

She wanted to learn hypnosis, she said, to be able to get at the early memories of her patients, since few significant infantile revelations had been forthcoming.

Her patients simply were not coming to grips with their unconscious. Hopefully, hypnosis might be able to break through their resistance and bring to the surface the noxious memories and conflicts that were responsible for their complaints. In reviewing her caseload, one could see three obsessive patients—one with a germ phobia who avoided contact with people; the second, a compulsive handwasher who spent most of the day in the bathroom; the third, a salesman with an obsession of death and killing, who, when he came across the number 23, had to engage in elaborate counting and other rituals to neutralize torturous preoccupations.

In all of these cases analysis had succeeded merely in providing some answers regarding the sources and meaning of these symptoms.

The other six patients ranged from severe borderline cases who went over the border periodically, to various species of schizophrenia, with one man, an engineer, www.freepsychotherapybooks.org 89 actively hallucinating.

There was not a single case, in my opinion, that could be considered a suitable candidate for classical psychoanalysis.

And yet here was an earnest, dedicated professional digging away at their unconscious in the hopes of uncovering some mnemonic treasures that would ransom them from the prison of their past.

My un verbalized hunch was that all hypnosis would do for her stockpile of patients would be to add more fascinating imaginative data to the huge store of information that the therapist had already accumulated, without budging their afflictions a single whit.

To expose all of these cases to one technique would be like a surgeon who, because he specializes in appendectomies, removes the appendix in every patient who comes to him with stomachaches, abdominal pains, and diarrhea.

No one technique can serve to ameliorate all maladies that burden humankind.

This is the best argument for a balanced and conservative eclecticism toward which modern psychotherapy has gainfully been moving in the past decades.

But there are still a few diehards who, loyal to the traditions of their chosen theoretical school, try to force a circumscribed method on all patients.

Such a tactic, following an analogy once proffered by Freud, usually proves no more effective than trying to appease victims of a famine by passing out menus of a French cuisine.

For reasons difficult to justify, some earnest students are unable to break away from the strictures of cherished theoretical systems and virtually become trapped along with their patients in its ineffectualities.

The only way some can escape from their stagnant caseloads is by moving out of town!

To return to my student, another dilemma confounded her.

When she started working with her patients, most of whom were therapeutic failures referred by other therapists only too happy to get them out of their offices, she had, as a beginner, accepted them at a low fee.

Considering that they came four and five times weekly, and could scarcely afford paying the accumulated sum each month, the therapist could not, despite the economic havoc inflation imposed on her, bring herself to raise their fees to even the standard minimal level charged by colleagues of her rank.

This added to her dilemma and undoubtedly created resentments that I felt enhanced depression. www.freepsychotherapybooks.org 90

My first task, prior to teaching her hypnosis, was to teach tactics of psychotherapy with sick patients.

We chose as our first prospect the obsessional patient with a germ phobia and a detached borderline case with masochistic fantasies.

She was to get them off the couch and allow them to sit up facing her.

She was to reduce their sessions to twice weekly and charge them the same fee monthly as before.

Most importantly, she was to stop acting like a “phantom therapist” by dropping her anonymity, with little digging and more relating.

Because she seemed bewildered at my unorthodoxy, I had to give her the exact words to say to her patients, which were to the effect that a point had been reached where it was no longer necessary to freely associate, where only two sessions weekly were needed, where only reality problems in the here-and-now were to be the focus.

She was enjoined to interest herself in what her patients were doing, smiling naturally, interchanging ideas and, if necessary, giving advice and support.

Curiously, the student showed no resistance to accepting my advice.

In two weeks a remarkable change developed in both the experimental patients and the therapist.

The patients, for the first time, spoke about how much better they felt.

And for good reason—they were relating to the therapist as a real person who was interested in them as people rather than as puppets of their unconscious.

The therapist found herself liking the patients, and her resentment resolved as the per session fee approximated that of her colleagues.

What occurred then was that she got all of her patients off the couch.

Sensing that she was losing her confidence in the analytic method, I had to work on her mistaken belief that the techniques I taught her were standard for all patients.

While they happened to be suited for the sick caseload she was currently carrying, they might not be right for other cases.

Indeed, classical psychoanalysis could be a boon for some patients carefully selected for the procedure.

Her supervision with me lasted one year, during which time she acquired new cases, one of which was a patient for classical analysis.

Skill and Experience in the Implementation of Techniques The history of science is replete with epic struggles between proponents of special conceptual systems.

Contemporary psychotherapists are no exception.

In a field as elusive www.freepsychotherapybooks.org 91 as mental health it is little wonder that we encounter a host of therapies, some old, some new, each of which proposes to provide all the answers to the manifold problems plaguing mankind.

A scrupulous choice of techniques requires that they be adapted to the needs and learning capacities of patients and be executed with skill and confidence.

Understandably, therapists do have predelections for certain approaches and they do vary in their facility for utilizing them.

Faith in and conviction about the value of their methods are vital to the greatest success.

Moreover, techniques must be implemented in an atmosphere of objectivity.

To function with greatest effectiveness, the therapist should ideally possess a good distribution of the following: 1.

Extensive training.

Training, in many parts of the country, has become parochial, therapists becoming wedded to special orientations that limit their use of techniques.

Accordingly, patients become wedged into restricted interventions and when they do not respond to these the therapeutic stalemate is credited to resistance.

Over and over, experience convinces that sophistication in a wide spectrum of techniques can be rewarding, especially if these are executed in a dynamic framework.

Whether a personal psychoanalysis is essential or not will depend on what anxieties and personal difficulties the therapist displays in working with patients.

The fact that the therapist does not resort to the discipline of formal analytic training does not imply being doomed to doing an inferior kind of therapy.

Indeed, in some programs, where the analytic design is promoted as the only acceptable therapy, training may be counterproductive.

Nevertheless, if a therapist does take advantage of a structured training program, which includes exposure to dynamic thinking and enough personal therapy to work out characterologic handicaps, this will open up rewarding dimensions, if solely to help resolve intrapsychic and interpersonal conflicts that could interfere with an effective therapeutic relationship.

Irrespective of training, there is no substitute for management under supervision of the wide variety of problems that potentially present themselves.

It is important that therapists try to recognize their strong and weak points in working with the various syndromes.

No matter how well adjusted therapists may be, there are some critical conditions that cannot be handled as well as others.

They may, when www.freepsychotherapybooks.org 92 recognizing which problems give the greatest difficulties, experiment with ways of buttressing shortcomings.

2.

Flexibility in approach.

A lack of personal investment in any one technique is advantageous.

This requires an understanding of the values and limitations of various procedures (differential therapeutics), experience in utilizing a selected technique as a preferred method, and the blending of a variety of approaches for their special combined effect.

Application of techniques to the specific needs of patients at certain times, and to particular situations that arise, will require inventiveness and willingness to utilize the important contributions to therapeutic process of the various behavioral sciences, accepting the dictum that no one school has the monopoly on therapeutic wisdom.

SOCIAL AND ENVIRONMENTAL VARIABLES Anyone who believes that the innate lenity of humankind transcend some of the abuses and indignities that society heaps on one is a victim of Utopian self-deception.

Social and environmental variables are probably the most neglected of factors in psychotherapy and sometimes among the most important.

If in doing therapy we do not consider the environment in which the patient will have to live and function, we will run the risk of annulling therapeutic gains.

An environment that does not support and encourage the patient's newly developed patterns or that punishes the patient for their constructive behavior will tend to reverse the gains brought about by the therapeutic process.

On the other hand, an environment that rewards for constructive behavior will reinforce therapeutic gains.

Treatment may be considered incomplete if it does not prepare the patient for contingencies that will have to be faced when treatment is over.

Adolescents who belong to gangs, for example, who learn to control delinquent behavior, may find themselves rejected by their peers for abiding by the law.

A young adult living at home under the yoke of domineering parents may not be permitted to assume an independent role after the therapeutic resolution of a pathological dependency drive.

An alcoholic helped to give up drink may not be able to remain dry so long as membership in a wine-tasting club continues.

www.freepsychotherapybooks.org 93 During therapy a thorough review of what the patient will be up against after termination will be urgently needed.

Either the patient will have to modify a destructive environment, if this is possible, or will

have to separate from it.

Thus the adolescent and the alcoholic will need to find new friends.

The young adult will have to get a job and take up residence in a more permissive atmosphere, that is, unless parents are willing to enter into family therapy and respond sufficiently to permit greater freedom.

Too frequently it is assumed that the patient will somehow get along once the treatment sessions have ended.

Because the environment will rarely take care of itself, its future impact on the patient must be studied as part of the treatment program.

TRANSFERENTIAL AND COUNTERTRANSFERENTIAL VARIABLES In therapy the initial positive relationship often serves during the first few sessions to quiet the patient's tensions and temporarily to restore a sense of mastery.

A good deal of the responsibility for this happening resides in the patient's need for an omnipotent idealized authority, which urgency is projected onto the therapist.

Some therapists advocate ending treatment abruptly when the patient has achieved a windfall of symptom relief, encouraging the patient to resume the customary threads of life and providing some awareness of the circumstances that contributed to this present disorder.

Where therapy is terminated after a few sessions the patient may continue to retain the initial image of the therapist as a powerful, benevolent and perhaps magical figure, having utilized this image, however unrealistic it may be, as a vehicle for restoring customary stability.

Where the therapist is more ambitious, or the patient requires more sessions to get well, around the eighth session a change often occurs in the image of the therapist that can precipitate a crisis in treatment.

The patient begins to realize that the therapist is not a god, does not have all the answers, and even possesses feet of clay.

This disillusionment may exhibit itself in a forceful return of symptoms, and a crediting to the therapist of ineffectual or evil qualities.

They draw their substance from a deep reservoir of fear and hate into which malevolent attitudes toward authority, some dating to childhood, have been stored.

This transference pollution may go on unconsciously and be manifested

www.freepsychotherapybooks.org 94 solely in dreams or acting-out away from the therapeutic situation.

The only sign the therapist may detect from the patient's manifest behavior is in the form of resistance to treatment.

The patient will complain about not getting well while breaking appointments or coming late for sessions and will delay the payment of bills for therapy.

A dynamically oriented therapist searches for transference signs realizing both the potential for helping the patient resolve some of the deepest problems through the insightful understanding of transference, and the destructive effect that unrecognized capricious transference can have on the therapeutic process.

A therapist who has no awareness of transference will be truly handicapped in managing patients whose reactions to treatment become paradoxical or inappropriate.

Understandably, the more serious the early problems with authority have been, the more likely will transference become apparent, and the more tumultuous its manifestations.

Where the therapist shares the initial illusions with the patient to the effect that he or she is a demigod, changing into a devil will be highly disturbing.

Such therapists may try to avoid trouble by confining themselves to the briefest forms of short-term therapy, terminating all treatments before the sixth session.

But sometimes this does not help, especially in vulnerable borderline or schizophrenic patients

who develop transference reactions toward the therapist even before seeing the therapist at the initial interview.

It is far better that professionals who wish to do good therapy work through their godlike image, if it is at all resolvable, by themselves or in personal therapy.

At any rate, a good degree of stability is required on the part of the therapist in order to handle transference reactions when they occur.

An unstable therapist finds it difficult, because of countertransference, to control responding negatively when challenged or unfairly accused by a patient in transference.

Impulsively discharging or furiously cowing the patient into submission will obviously rob the patient of an opportunity to work through problems with authority.

Whether personal psychotherapy is mandatory for all therapists in training as a www.freepsychotherapybooks.org 95 means of preventing obstructive countertransference is a question about which there is much debate.

If one is a good therapist, personal therapy will probably help make one a better therapist but will not accomplish miracles.

There are certain problems that are so deep that personal therapy may not be able to budge them.

For example, intense childish distortions developed in very early infancy may resist correction.

The therapist may get an awareness of these distortions through personal therapy, yet be unable at times to control their surfacing.

Nevertheless, therapy may have enabled the management of reactions sufficiently so that they do not interfere too much with functioning with patients.

Personal psychotherapy will accomplish its mission if it can control the therapist's use of the patient for personal designs and projections.

Destructive countertransference is probably present to some extent in all therapists, but this need not necessarily be hurtful if the therapist is aware of its presence, recognizes how it is manifesting itself, and takes steps to resolve it.

Countertransference is, of course, not always harmful.

It may alert therapists to traits and maneuvers of the patient that arouse important feelings in other people.

The important thing is how therapists utilize countertransference toward bringing out the unconscious needs and conflicts of the patient while describing the effect they can have personally as well as on others.

The patient may learn something important from this.

For example, a great deal of tolerance is required on the part of the therapist in adjusting to the habits of some patients.

Most therapists are punctual in appointments (as they should be) even though certain patients are lax in appearing promptly at the scheduled hour.

These patients are prone to subject the therapist to a bit of delinquency as a vehicle of testing or defiance.

The therapist may particularly be irritated by patients who come to clinics, paying little or nothing yet seeming unappreciative of what is being done for them.

There is no reason why the therapist should not focus on the patient's offensive behavior, not as a way of reprimanding the patient, but to clarify the meaning of what is going on.

There are certain patients we relish working with and others who are less than a joy to treat.

There are therapists who are completely unable to handle adolescents while others do their best work with young people.

Schizophrenic, violent, obsessional, paranoidal, psychopathic, hypochondriacal, suicidal, and

delinquent patients stimulate aversive www.freepsychotherapybooks.org 96 responses in many therapists.

Yet other therapists not only tolerate these syndromes but enjoy handling them.

Countertransference elements encourage therapists to project onto patients aspects of their own inner needs of which the therapists are partly or wholly unaware.

One therapist whom I was supervising in a class reported that his patient was getting progressively more depressed.

An audio cassette of a session brought out a repetitious theme voiced by the patient that nobody cared for her, nobody paid attention to her, nobody liked her; that she was the neglected child in the family whose destiny was to spend her life in misery as an isolate.

She insisted that she could never command respect or attention from anybody.

She went on and on in this depressive vein, and the therapist from his conversation seemed to be responding correctly to what the patient was saying.

We decided then that the therapist should interview the patient behind a one-way mirror with the class observing.

The therapist was instructed to set up the furniture similar to the arrangement in his office.

When the patient entered the room she seemed fairly animated, but as she talked she appeared to get progressively more and more depressed, the content of her verbalizations centering around feeling rejected by her family, by people, and by the world.

What was startling was that the therapist, without realizing it, was actually playing into her theme of rejection.

He had placed his chair at an angle so that he did not face her, and while he would from time to time fire an interpretation at her, he was constantly busy writing or looking away toward the opposite corner of the room.

Periodically the patient would glance at the therapist, who by all appearances was off in space.

One got the impression that she was being treated like a scientific specimen, not like a needy human being.

Having been reared by a schizoid mother who in later life was admitted to a mental institution, she undoubtedly interpreted the therapist's manner as rejection.

The interesting thing was that a sophisticated professional in his last year of postgraduate training did not realize that he was providing the patient with a stimulus that activated her habitual rejection theme.

On questioning, the therapist admitted that he was losing interest in the patient because she was beating at him constantly with her griping and complaining and getting no better.

He was unaware that the placement of his chair was a gesture that signaled his disinterest, nor did he realize that the sessions were as traumatic for the patient as they were for him.

He was sufficiently advanced in his training and personal analysis to explore his feelings toward the patient and affiliate them with attitudes toward a hypochondriacal mother who drove his own father to distraction.

In this way his countertransference interlocked with the transference projections of the patient.

www.freepsychotherapybooks.org 97 What the patient was doing with her therapist she did with all people with whom she became intimate.

She expected rejection so much that in testing their sincerity she did exactly the things that resulted in her being rejected.

People then responded by avoiding her.

I suggested that the next time the patient came for a session the interview be conducted face to face.

The therapist was to put aside his pad and just talk about the patient's interests and experiences without probing her feelings—in other words to work on building a relationship. In a very short time the whole nature of the therapy sessions changed. The patient became livelier, more interested in what she was doing, and more able to joke and smile.

And the therapist developed greater enthusiasm about the patient. Eventually the patient's depression lifted and the patient was able to manage the termination phases of therapy without too great difficulty.

Not all of a therapist's reactions are countertransferential. They may be prompted by deliberate, destructive, and outlandish conduct. I recall one patient whose behavior was so provocative as to challenge my capacity for disciplined objectivity.

It was all I could do during some sessions when she acted particularly nasty and insulting to stop myself from responding defensively or punitively.

The patient was a married woman in her late thirties who came to me not of her own free will but because of the pressures imposed on her by her friends and family, who realized that she was seriously depressed and disturbed.

She was the only child of a wealthy couple who adulated, pampered, and spoiled her so that she soon ruled the household like a tyrannical princess with an iron fist without the traditional velvet glove.

Screaming tantrums forced her parents and private tutors to yield to her slightest whim. When she grew up she transferred these tactics to people around her, responding to not getting her way with violence, headaches, and paranoid-like projections.

Her marriage, she revealed, started off sizzling on a King-sized bed. But soon, after she had succeeded in verbally whip-lashing her husband into partial impotence, the couple retreated into twin beds, and, following the birth of her two children, they sought refuge in separate rooms from which they sauntered out to combat.

Added to this the inanities of suburban life were more, she claimed, than human flesh could endure.

Her initial contacts with me were organized around exploratory maneuvers to determine how much she could win me over and manipulate me.

Interpretations fell on deaf ears.

She was certain that I was siding with the enemy at home who blamed her for the prevailing mess she was in.

Hostility was expressed in subtle and not so subtle ways.

On one occasion, she asked me to refer her to the best www.freepsychotherapybooks.org 98 dermatologist in town.

She appeared at his office with her dog whom she brought into the consulting room. It turned out that she wanted treatment not for herself but for her dog "who deserved the best." The dermatologist, who was a dear friend of mine, winked to his nurse and they proceeded to put the dog on the examining table and to treat him like a regular patient, right to the rendering of a prescription with the dog's name on it.

Fortunately, the doctor, a dermatologic authority, had a great sense of humor and he went along with the "gag." On another occasion, being more careful to explore her complaint of backaches, I referred her to an orthopedic surgeon who on walking through the waiting room found her sitting in a chair with her feet on a new expensive coffee table.

In not too gentle tones, he commanded her: "Won't you take your feet off my table?" Haughtily she turned on him with "It took me four months to hate Wolberg.

You I hate right away." With this as a background, I want to describe an incident where my loss

of objectivity resulted in a significant therapeutic gain.

During an interview, as she sat facing me, I confronted her with her responsibility in promoting a quarrel with her best friend.

Furiously the patient removed a diamond ring from her finger and fired it at me.

As the ring, a huge eight-carat gift from her father, whizzed by my ear, I tried to conceal my surprise and dismay by acting nonchalant and by not commenting on her behavior.

I could see that she was nonplussed and irritated by my lack of concern.

"Give me back my ring," she commanded imperiously.

"You threw it," I replied, "You find it." After several such exchanges, she stormed over to my chair and began to search for the ring.

It was nowhere in sight!

"You better find my ring," she shrieked, but she got no response from me.

However, after several minutes had passed in futile search, the ring remained undiscovered, and I leisurely proceeded to help her.

But the ring was nowhere to be seen.

By this time, I too was concerned.

Yet a minute search of the room produced nothing.

The patient burst into tears and I then tried to reassure her, utilizing the incident to accent my previous interpretations that her loss of temper hurt her more than it did other people.

After the patient left my office ringless, I went over the room minutely and finally I found the ring, which apparently had fallen on the couch and bounced off to the side, becoming wedged in between the mattress and the wooden side.

The patient was relieved at my telephone call and thanked me.

My victory, however, was short-lived.

The next day when the patient came for her ring she burst angrily into my office and slammed the door shut with her foot, registering a dirty footprint on my newly painted door.

I could feel my anger bubbling up.

"You wash that footprint off that door," I ordered.

"Ha, ha, ha," she retorted defiantly, "You make me." Reflexively, I grabbed her by the back of her neck and marched a frightened patient into the bathroom, stuck a wet soapy washcloth in her hand, and firmly marched her back to the door.

She obediently washed the door, then quietly sat down; then we had our first constructive talk.

She acted contented and even smiled.

What the patient seemed to have done was to force me to set limits on her behavior that her father had failed to do.

What she wanted and needed was some discipline.

The positive effect on our relationship was amazing and we were able to achieve changes in her life that earned for me the gratitude of the patient and her family.

If through the relationship the patient is able to modify the introjected image of authority, the therapeutic process will have scored a great gain.

Such modification comes www.freepsychotherapybooks.org 99 about by a replacement of the patient's imprinted authority figure, which is often harsh or overprotective, or negligent, or distorted, with a new, more rational and constructive figure as vested in the therapist.

An opportunity for this may come about through transference on the therapist of feelings or attitudes that relate to the authoritative introject.

Manifestations of the transference appear in direct or disguised form, in oppositional resistance to the therapist or to the techniques being employed, in unreasonable demands for favors or affection, in fantasies or dreams, or in acting-out away from treatment with persons other than the therapist.

The ability of the therapist to recognize transference when it appears, particularly in its disguised forms, and to deal with it through interpretation and proper management of the relationship will have a determining effect on the direction and results of treatment.

RESISTANCE VARIABLES Shorr (1972) cited Saul Bellow, who in *Herzog* describes the common resistance to normality so often encountered in therapy: "To tell the truth, I never had it so good ... but I lacked the strength of character to bear such joy. That was hardly a joke.

When a man's breast feels like a cage from which all the dark birds have flown—he is free, he is light.

And he longs to have his vultures back again.

He wants his customary struggles, his nameless empty works, his anger, his afflictions, and his sins." Paradoxically, some people are loath to give up the very chains that bind them to neurotic slavery.

Reluctance to accept normality is merely one of the many resistances that precipitate out in the course of therapy.

Some resistances, inspired by lack of motivation and refusal to give up a stereotype of a nonrealistic therapist, occur at the start of therapy.

Some, such as transference resistances, convert the therapeutic alliance into a battlefield of archaic projections and interfere with the treatment process itself.

Others, like regressive dependency, mobilize anger and grief, and obstruct proper termination of therapy.

Resistances can take many forms, often following defensive maneuvers customary for the individual.

Thus patients may become evasive or forgetful, breaking, cancelling or www.freepsychotherapybooks.org 100 coming late for appointments, or they may engage in prolonged silences during sessions.

They may indulge in superficial, rambling talk.

They may try to disarm the therapist with praise, or become aggressive, argumentative, and accusatory.

Women may become sexually seductive toward their male therapists and men toward their female therapists.

Destructive as they are, certain resistances protect the individual from catastrophic helplessness and anxiety.

They are means of preserving important neurotic coping mechanisms.

A phobia, for example, may disable the individual, but it still has a protective quality.

A hysterical arm paralysis can shield the individual from awareness of murderous impulses, while blandly protesting the inconvenience of being unable to utilize the limb.

Frigidity may mask overwhelming fear in a woman of assuming a feminine role.

In all of these cases the yielding of important defenses promises exposure to dangers far greater than the torments the patient already suffers.

Moreover, certain secondary gains of a positive nature may accrue to the indulgence of a neurosis.

In industrial accidents the victim who is on disability payments may in giving up pain and physical illness lose not only financial security, but also sympathy, freedom from responsibility, and the opportunity of occupying center stage with repetitive tales of what was endured at the hands of doctors.

Having been referred to a therapist by an insurance company, which insists on the victim getting treatment, or brought in by family who tire of complaints, the victim is exposed to the threat of health, which is a barren bounty compared to the advantages of disability.

Experience with large numbers of patients convinces that three common dynamic problems most often initiate emotional difficulties and also create resistance to psychotherapy.

They are: 1.

inadequate separation-individuation; 2.

a hypertrophied sadistic conscience; and 3.

devaluated self-esteem.

These are never isolated units.

Rather, they coexist and reinforce each other, and www.freepsychotherapybooks.org 101 they create needs to fasten onto and to distrust authority, to torment and punish oneself masochistically, and to wallow in a swamp of hopeless feelings of inferiority and ineffectuality.

They frequently sabotage a therapist's most skilled treatment interventions, and when they manifest themselves, unless dealt with deliberately and firmly, the treatment process will bog down in a stalemate.

The most the therapist may be able to do is to point out evidences of operation of resistance saboteurs, to delineate their origin in early life experience, to indicate their destructive impact on the achievement of reasonable adaptive goals, to warn that they may make a shambles out of the present treatment effort, and to encourage the patient to recognize personal responsibility in perpetuating their machinations.

The frightening hold a self-devaluating resistance can have on a patient is illustrated by the following fragment of an interview.

The patient, a writer, 42 years of age, who made a skimpy living as an editor in a publishing house came to therapy for depression and for help in working on a novel that had defied completion for years.

Anger, guilt, shame, and a host of other emotions bubbled over whenever he compared himself with his more successful colleagues.

He was in a customarily frustrated, despondent mood when he complained: Pt.

I just can't get my ass moving on anything.

I sit down and my mind goes blank.

Staring at a blank piece of paper for hours.

I finally give up.

Th.

This must be terribly frustrating to you.

Pt.

(angrily) Frustrating is a mild word, doctor.

I can kill myself for being such a shit.

Th.

You really think you are a shit?

Pt.

(angrily) Not only do I think I am a shit, I am a shit and nobody can convince me that I'm not.

Th.

Frankly, Fred, I'm not even going to try.

But you must have had some hope for yourself; otherwise you never would have come here.

Pt.

I figured you would get me out of this, but I know it's no use.

I've always been a tail ender.

Th.

(confronting the patient) You know, I get the impression that you've got an investment in holding on to the impression you are a shit.

What do you think you get out of this?

Pt.

Nothing, absolutely nothing.

Why should I need this?

Th.

You tell me.

(In his upbringing the patient was exposed to a rejecting father who demanded perfection from his son, who was never satisfied with his even better than average marks at school, who compared him unfavorably with boys in the neighborhood who were www.freepsychotherapybooks.org 102 prominent in athletics and received commendations for their schoolwork.

It seemed to me that the paternal introject was operating in the patient long after he left home, carrying the same belittling activities that had plagued his existence when he was growing up.) Pt.

(pause) There is no reason, (pause) Th.

You know I get the impression that you are doing the same job on yourself now that your father did on you when you were a boy.

It's like you've got him in your head.

(In the last session the patient had talked about the unreasonableness of his father and his inability to please his father.) Pt.

I am sure I do, but knowing this doesn't help.

Th.

Could it be that if you make yourself helpless somebody will come along and help you out: (I was convinced the patient was trying to foster a dependent relationship with me, one in which I would carry him to success that defied his own efforts.) Pt.

You mean, you?

Th.

Isn't that what you said at the beginning, that you came to me to get you out of this thing?

You see, if I let you get dependent on me it wouldn't really solve your problem.

What I want to do is help you help yourself.

This will strengthen you.

Pt.

But if I can't help myself, what then?

Th.

From what I see there isn't any reason why you can't get out of this thing—this self-sabotage. (The patient responds with a dubious expression on his face and then quickly tries to change the subject.) In the conduct of treatment one may not have to deal with conflicts such as those above so long as the patient is moving along and making progress.

It is only when therapy is in a stalemate that sources of resistance must be uncovered.

These, as has been indicated are usually rooted in the immature needs and defenses inspired by dependent, masochistic, self-devaluating promptings.

At some point an explanation of where such promptings originated and how they are now operating will have to be given the patient.

This explanation may at first fall on deaf ears, but as the therapist consistently demonstrates their existence from the patient's reactions and patterns, the patient may eventually grasp their significance.

The impulse to make oneself dependent and the destructiveness of this impulse, the connection of suffering and symptoms with a pervasive need for punishment, the masochistic desire to appease a sadistic conscience that derives from a bad parental introject, the operation of a devalued self-image, with the www.freepsychotherapybooks.org 103 subversive gains that accrue

from victimizing oneself, must be repeated at every opportunity, confronting the patient with questions as to why he or she continues to sponsor such activities.

When we consider the many patient, therapist, environmental, transference and resistance variables that have been described above, and that are parcels of all therapies, irrespective of type, it becomes apparent that empirical research into their effects may do a great deal in promoting more effective practice and in advancing psychotherapy to its rightful place in the family of scientific methodologies.

www.freepsychotherapybooks.org 104 6 Educational, Casework, and Counseling Approaches Versus Psychotherapy Educational, casework, and counseling approaches all aim at an expansion of the potentialities of the individual and a betterment of adjustment.

In this way they pursue some of the goals of psychotherapy.

They possess, however, certain distinctive features that should be differentiated from psychotherapy to avoid a confusion of role and function among the disciplines identified with these methods.

EDUCATIONAL APPROACHES AND PSYCHOTHERAPY Education has been defined as a process of “progressive or desirable changes in a person as a result of teaching and study” (English & English, 1958) and “the systematic instruction, schooling or training given to the young [and, by extension, to adults] in preparation for the work of life” (Oxford Univ. Dictionary, 1955).

Even though education has focused on the development of the intellectual capacities of the individual and the acquisition of knowledge, a new direction in education has been fostered by the recognition of the vital significance of human relationships in the learning process.

The purpose of education has been extended to include the emotional growth of the individual and more constructive relationships with people.

This progressive movement within the field of education, founded by John Dewey, “emphasizes the needs of the individual and the individual’s capacity for self-expression and self-direction” (Hinsie & Campbell, 1960).

The discipline of education has accordingly broadened its goals to include “the inculcation of social attitudes,” “the development of social sensitivity,” and the evolution of “better personal-social adjustment” (Smith & Tyler, 1942; Trecker, 1946).

Educational objectives have been widened to foster personal security, and to expand assertiveness and self-direction.

A principal aim in education “is to provide rich and significant experiences in the major aspects of living so directed as to www.freepsychotherapybooks.org 105 promote the fullest possible realization of personal potentialities” (Giles et al, 1942).

More and more schools are including in their curricula material “concerned with the total personality—not merely with the intellect but with emotions, habits, attitudes.

...

It [general education] consists of preparation for efficient living, no matter what one’s vocation” (N.

Central Assn., 1942).

Among the most interesting experiments along this line were those of the Bullis, Force, Ojemann and Forrest Hill Village projects (Comm.

Prev.

Psychiatry, 1951).

An additional factor has been recognized to the effect that emotional blocks may prevent the student from accepting or integrating educational offerings.

The traditional pedagogic techniques are usually unable to handle such impediments.

Interviewing and group work methods that are continually evolving in educational circles

(Cantor, 1946; Slavin, 1950) attempt to deal with learning blocks.

Both in objectives and techniques, some of these procedures resemble those in psychotherapy. The question, however, still has not been answered satisfactorily as to whether these techniques are forms of psychotherapy or pure aspects of educational intervention. Behavior modification, for example, dealing as it does with habit, learning, and adjustment and practiced in schools, hospitals, prisons, and clinical and community settings, appears to fall into both areas.

The confusion perhaps lies in the fact that therapeutic techniques often turn out to be educational for the patients in that they discover new things about themselves and learn different ways of behaving.

On the other hand, education can register a significant psychotherapeutic impact on some individuals, helping them to control their symptoms and fostering a better life adaptation.

The success of education in promoting the development of growing and plastic personality structures of children has aroused hopes in utilizing educational approaches toward reshaping attitudes, altering values, reorganizing feelings, and refashioning behavior.

Recent years have witnessed the popularization of mental health concepts in current periodicals, books, movies, radio, and television.

Under certain conditions the simple imparting of the proper kind of mental health information has geared the recipient www.freepsychotherapybooks.org 106 toward a more wholesome adjustment.

Such simple concepts as that an infant requires the loving presence of a mother and adequate stimulation during the first year of life, or that rebelliousness during adolescence is

universal and perhaps a necessary component of the child's reaching for independence, may soften parental anxiety and alter ways of handling problem situations in both parent and child.

Attempts are being made also to educate persons in more wholesome attitudes toward themselves, their families, and the community.

Materials are available that are intended to ameliorate the effects of emotional illness on adjustment.

This "mental health education" or "psychoeducation" in which there are promulgated precepts of normal personality growth, of psychodynamics and psychopathology, data on the health, adjustment, social and economic hazards of emotional illness, and details on the adoption of mental health information to various problems in the community has sponsored a profession of "Mental Health Educator." The impact on the public has in general been a questionable one, even though the substance of what is taught and the communication techniques have been correct.

The widespread preoccupation with psychological concepts has introduced a flood of writings and lectures formulating doctrines on how to regulate one's life most efficiently.

The intent is partly to educate people into the proper handling of their relationships, and partly to mediate their personal conflicts.

The effect of such writings and lectures is difficult to assess.

Where they do not oppose basic defenses, the person may integrate the teachings with a reeducational result.

Where they conflict with basic defenses, they may either wield no influence whatsoever or inspire guilt feelings induced by a realization that one is unable to abandon attitudes or patterns of behavior potentially hurtful to oneself or others.

Group seminars oriented around discussion and clarification are much more effective than writings, provided that they are headed by a skilled group leader.

Given the proper motivation, adequate educational media, a congenial setting, an intelligent educator who inspires respect and confidence, and a friendly group bent on www.freepsychotherapybooks.org 107 learning objectives with which the individual can identify, an individual may acquire a prodigious amount of knowledge.

Some change may also be expected in patterns of adjustment as a by-product of the learning experience, principally due to the relationship with the educator and the group.

There are limitations, however, as to how extensively attitudes, values, feelings, and behavior, in which the individual has a deep emotional investment, may be influenced.

Although these may seem senseless and are in opposition to a reasonable and happy adjustment, the individual continues to adhere to them with bullheaded persistence.

They are impervious to logic, common sense, and scientific argument.

The obese young woman, bloated with avoirdupois, may be better versed in the rationale and methods of calorie control than a trained dietician.

She knows every reason why she must diet, yet she is unable to stop stuffing herself with food.

The man with a recent coronary attack has read the latest antitobacco literature, yet he invites devastation by chain smoking.

The irate business executive, constantly disciplined by her superiors for her spleen, and in spite of a brush-up course in public relations, continues to attack her subordinates and to alienate her peers.

The promising young salesperson with a flying phobia has learned all about the safety of aircraft, yet prefers the risks of bus travel to the ease and comfort of a jet plane.

These are but a few examples of how limited an educational approach may be in dealing with patterns that reflect deep-seated fears and needs.

Educational techniques are valuable when the individual is able to countenance and to absorb the content they communicate.

They are a parcel of all good psychotherapies.

But they are unable in themselves to overcome resistances to change, nor can they readily handle the anxieties that inspire the resistances.

They usually deal with dimensions peripheral to those that have initiated the individual's neurotic disturbance and that continue to activate that person's mechanisms of defense.

In summary, while educational approaches are important and useful, they are no substitute for psychotherapy.

They have a growth potential, but one should not overestimate their impact on firmly conditioned and neurotically structuralized www.freepsychotherapybooks.org 108 behavioral patterns.

CASEWORK AND PSYCHOTHERAPY Psychiatric social work traditionally "is social work undertaken in psychiatric agencies and mental health programs." It has as its aim a contribution "to those services and activities within the community which promote mental health and are conducive to the restoration of the health of individuals who are suffering from mental and emotional disturbances." The social work process employed to promote this aim is "social casework," which is applied in the "identification, diagnosis, and treatment of persons with personal and social maladjustments caused or aggravated by mental and emotional problems" (Knee, 1957).

By means of a "person to person helping relationship through individual interviews or group process, the social worker can assist the individual to determine and resolve specific problems in the environment and interpersonal relationships which interfere with adequate functioning" (Am.

Assn of Psychiatr.

Soc.

Workers, 1955).

Among the varied services of the social worker are those that deal with family social work, family life education, adoption, child welfare, foster care for children, day care, homemaker activities, legal aid, public assistance, school social services, youth services, community organization and coordination, rehabilitation, protection of rights, and correctional work

involving the broadest aspects of law enforcement, detention, probation, parole, and crime control and prevention.

Problems in employment, housing, education, living arrangements, finances, recreation, and health come under the social worker's aegis.

In the course of work the social worker must make contact with a wide range of humanity, including the physically handicapped, mentally ill, mentally retarded, indigent, chronically ill, alien and foreign born, aged, unmarried mothers, alcoholics, juvenile delinquents, criminals, drug addicts, and generally unhappy and maladjusted individuals.

The enabling, problem-solving process the social worker performs to help individuals and their families resolve social difficulties they are unable to manage by themselves is "casework."

With the expansion of government responsibility for financial assistance, private family agencies particularly have employed casework in order to www.freepsychotherapybooks.org 109 counsel family members with reference to intrafamilial relations, so that marriage counseling, counseling of parents regarding their children, and counseling of adults regarding their aged parents constitute a substantial part of work in a family agency.

Conventionally, the trained worker has executed these functions through the rendering of services of which the client is in need.

The focus has been on the external problem or social situation and not, as in psychotherapy, on the individual's inner distress or illness.

In rendering social services, no deliberate attempt is made to alter the client's basic personality patterns, the object being to handle situational problems on a purely realistic level.

This classic role in casework has, during the past decades, undergone considerable modification.

Operationally, caseworkers have found it difficult or impossible to limit their area of work to the client's external life situation.

Indeed, a conviction has evolved that, unless certain capacities are mobilized or developed in clients, they will be unable to utilize the social services offered or the community resources made available to them toward a better life adjustment.

Consequently, much more extensive goals have developed in the practice of casework than are implicit in the early definition.

This broadening of objectives was largely the product of psychiatric influence, particularly that of Freud and Rank, and of better understanding of the use of sociologic theory and techniques.

Thus, Towle (1947) described the caseworker as one who handles persons "experiencing some breakdown in their capacity to cope unaided with their own affairs." This breakdown, she added, may be caused by external factors, "or it may be partially, largely, or entirely due to factors within the individual." Services may be geared toward reality needs or may be "oriented to feelings and to ways of responding." Bowers (1951), reviewing definitions of casework, adds this one: "Social casework is an art in which knowledge of the science of human relations and skill in relationship are used to mobilize capacities in the individual and resources in the community appropriate for better adjustment between the client and all or any part of his total environment." Other definitions are (1) "casework is a method of helping a troubled person to www.freepsychotherapybooks.org 110 understand what is causing his personal or family problems and to find inside himself, in the home, or in health and welfare agencies the resources to rebuild his or her family's life" (U.S.

Soc.

Sec.

Adm.

1949) and (2) casework is “the function of professional [social] workers who, through social services and personal counseling, attempt to help individuals and their families improve their personal and family adjustments” (Hinsie & Campbell, 1960).

When we examine these definitions carefully, we find that some of the goals toward which casework is directed are identical with those in our definition of psychotherapy.

Since the casework process involves an interpersonal relationship, similar emotional phenomena develop between the caseworker and client as occur between a psychotherapist and patient.

The relationship has often served to release forces within the individual that are essentially psychotherapeutic in effect.

But can we say the casework process is psychotherapy?

Some authorities insist that it is and that “when a psychiatric social worker says she is doing case work, within the interviewing room, she does psychotherapy” (Grinker, 1961).

Davidson cites Rennie’s account of the casework process: “People who have been interviewed by caseworkers ... say that they like to be talked with in this way; that they gained insight, came to understand some of the reasons for their problems, and got some inklings of the way out of their difficulty... .

The caseworker observes moods, hesitations, and emotions ... expressed in subtle ways.

He tries to trace problems to their roots.

He accepts the person as he finds him, without blame.... Often the client’s anxiety is drained off and his hostility and guilt are lessened.

He is freed from the blinding effects of these emotions and sees, for the first time, psychological connections of which he had formerly been unaware.” Davidson adds, “Now here, in the words of a distinguished psychiatrist, is a definition of casework— and it’s as good a definition of psychotherapy as you can find” (Perlman, 1960b).

When we examine the effects of casework, and scrutinize the dynamics of the caseworker-client relationship, we find a number of similarities to those of psychotherapy.

There are, however, differences.

First, the casework process is not geared toward the resolution of emotional problems as such, but rather toward a bringing of the www.freepsychotherapybooks.org 111 client to a recognition of those problems that interfere with the client’s social adjustment and utilization of services.

“The particular purpose of social casework is to help people who are suffering from some impairment or breakdown in their adequate social functioning and to restore, reinforce, or enhance the performance of their daily lifetasks” (Perlman, 1960b).

Second, resistances to the use of services, and fears and anxieties about changes that are occurring in the life situation and patterns of relatedness, are managed on a realistic level with active guidance and advice giving.

Interpretation of unconscious conflicts and a delineation of the origin and minute operations of mechanisms of defense are avoided.

Third, the usual education of the social worker provides counseling procedures, but not the more extensive psychotherapeutic techniques.

While there is general recognition of the fact that psychological disturbances are operating to initiate or reinforce problems in the milieu of the individual or family unit, these disturbances are handled on a different level than in psychotherapy.

The procedures are focused on enhancing the client’s own problem-solving capacities to cope more effectively with social difficulties or to execute actions to modify or resolve them.

The establishment of a working relationship with the patient, the making of a social diagnosis, the understanding of the client’s personality workings and resistances, the analysis of the client’s assets and liabilities, and the maintenance of an effective communicative climate are

important elements in the methodology of the caseworker.

Guidance, reassurance, emotional support, opportunity for emotional catharsis through verbalization, environmental manipulation, and interpretation of the client's feelings and resistances are freely employed in the counseling process that develops.

While the contributions of Freud and Rank are readily acknowledged, their influence on social casework practice has not been as vigorous as in previous years.

There are instead tendencies to employ the principles of ego psychology (Upham, 1973), systems theory (Hollis, 1974) and behavior modification (Fischer & Gochros, 1975), trends that are also preoccupying contemporary psychotherapy.

www.freepsychotherapybooks.org 112 Unfortunately, modern casework, for many years the dominant method of practice, is undergoing an unfair attack as too expensive, and that it focuses on getting people to conform to oppressive social conditions.

The field of social work itself is in a state of ferment as practitioners seek new, more lucrative roles and values to keep up with the temper of the times (Rein, 1970; Payne, 1972; Vigilante, 1972).

In the face of this many social workers are striving for changes in professional identification, moving closer to the clinical field.

The social worker, usually a psychiatric social worker, who has had specialized postgraduate training in psychotherapy, may, in addition to casework, function with clients on a psychotherapeutic level, dealing with their intrapsychic mechanisms as they influence the total adjustment of the individual.

When the trained caseworker does this, casework is no longer being practiced; psychotherapy is the modus operandi.

Toward this expanded function advanced clinical training is being offered to social workers in training centers around the country on a postgraduate level.

With such training, the designation of psychiatric social worker is being rejected by many social workers as a term connoting subservience to and imitation of the medical profession.

The title clinical social worker is favored, which better describes their practice while recognizing their independence of functioning.

COUNSELING AND PSYCHOTHERAPY Counseling is customarily defined as a form of interviewing in which clients are helped to understand themselves more completely in order that they may correct an environmental or adjustment difficulty.

Guiding and helping people to make rational decisions, to organize plans for constructive pursuits, to seek out the best available community resources to satisfy immediate and future needs, and to overcome reluctancies toward and fears of action are among the tasks of the counselor.

A wide variety of professional and paraprofessional paid and volunteer workers function in this way as counselors.

Clients seek help for a host of problems, some real, some projections of inner distress.

And it is the duty of the counselor to distinguish between the two.

The relationship between client and counselor, which is considered of prime

www.freepsychotherapybooks.org 113 importance in counseling, is used in different ways—from the offering of suggestions as to available resources to the interpretation of the client's attitudes and feelings.

The directiveness of the counselor varies.

In directive counseling (Thorne, 1950) the counselor assumes the role of an authority offering the client an evaluation of the particular problem and defining courses of action.

In nondirective counseling (Rogers, 1942) the counselor functions as an agent who encourages the client's expression of feelings, reflecting these and helping the client to assume

responsibility for them.

In this way the client thinks things out, develops goals, and plans the course of action.

Other forms of counseling draw from the field of dynamic psychology, seeking to utilize the counselor-client relationship to demonstrate the operations of the client's personality structure either in creating the situations for which help is being sought or in blocking the client from finding appropriate solutions.

Counseling programs have advanced rapidly, particularly in the educational, industrial, social work, health, and military fields.

Counselors in progressively larger numbers have been utilized for guidance activities and personal counseling in secondary schools, colleges, and universities, for "employment counseling" toward selection, placement, and morale building in industry, for "rehabilitation counseling" to enable handicapped persons to make a transition from disability to productiveness, for job relocation services for returning veterans, for "counseling psychology" in Veterans Administration hospitals and various community agencies.

Family agencies offer counseling services on matters of family relationship and social adjustment such as the following: 1.

Difficulties in interpersonal relationships manifest within the family, between the client and patient, siblings, spouse, children, and others, resulting in anxiety, symptoms, or deviant behavior.

2.

Difficulties in personal adjustment in relation to inner functions, such as sexuality, educational achievement and learning, and employment and work abilities.

3.

Environmental problems, such as economic hardships and housing deficiencies.

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Health problems, such as disabilities, physical handicaps, mental retardation, heredity illness (genetic counseling), etc.

The helping methods in the counseling services rendered include (1) advice giving, environmental manipulation, and ego support, such as reassurance and encouragement, (2) guidance in the client's roles in family and society and in what can be done about a specific situation, and (3) clarification regarding the client's feelings and attitudes and their deviations.

Services are given not only to individuals and couples (marital counseling), but also to the total family (family counseling) and to groups (group counseling).

Because of the increasing need for counselors, various training courses have been organized on different levels of sophistication.

They range from those that require only a few hours at an undergraduate level to those that lead to a doctor's degree.

No unified curriculum exists, but a body of information is gradually being organized from fields of psychology, psychiatry, sociology, social work, and anthropology that are helpful in understanding personality development and structure, human relationships, the vicissitudes of adjustment, and the interviewing process.

Goals in counseling have been expanded from simple testing and advice giving to managing the individual's general adjustment problems with the realization that a situational difficulty may be a mere surface manifestation of a more widespread disturbance.

For example, the attempt to broaden a person's occupational perspective or to outline a curriculum in line with the person's abilities may fail because of opposing personality forces.

This has tended to shift the emphasis of the interview in both individual and group discussions.

The client's perceptions, goals, and values as well as feelings about the self and the environment are reflected in behavioral choices.

Therefore, they need to be a part of the counseling operation (Cottle, 1973).

The counselor may thus act as a catalyzer to a growth process within the client.

The term counseling has become diffuse, covering information and advice giving, and merging imperceptibly into psychotherapy (Patterson, 1966; Truax & Carkhuff, 1967; Osipow & Walsh, 1970).

Training for a modern counseling role, accordingly, requires, in www.freepsychotherapybooks.org 115 addition to supplying the counselor with special knowledge and information about resources, a recognition of symptoms of emotional illness, some comprehension of dynamics, discernment of the forces of transference and resistance that are apt to be released during the counseling relationship, knowledge of how cultural factors influence value systems, and, finally, an understanding of one's own emotional shortcomings and prejudices (including destructive countertransference) that are apt to release themselves during counseling.

Particularly important is sufficient diagnostic skill to discriminate depression, paranoidal projections, and psychotic manifestations in their early stages.

The counselor should know when, how, and where to refer clients for psychotherapeutic help when their conditions require more than counseling and the counselor is not equipped to function in a psychotherapeutic role.

In summary, one may easily discern from the discussion how difficult it is to separate the goals of counseling from those of psychotherapy.

Some attempts have been made to distinguish the methodologies.

Counseling requires a relationship between a helping agent and a client.

In this relationship emotional intercurrents operate that may have a psychotherapeutic effect on the individual, with an influence far beyond the purposes for which help was sought.

But while personality change may be the outcome of the counseling relationship, as it is in psychotherapy, there are certain differences between the two processes.

The reason a counselee seeks help is because there is generally some situational difficulty for which specialized knowledge is required or because the individual is unable to cope with a problem through personal resources.

The counselor then executes specialized knowledge of the area of concern to aid the counselee.

Thus, the area may be educational, vocational, or behavioral, as in marital maladjustments.

Objective instruments, such as psychological tests, may be employed.

The counselee is then guided toward adequate courses of action.

The approach here is supportive, the counselor making suggestions, offering guidance, presenting to the counselee opportunities for ventilation of feelings, and encouraging the counselee toward the proper actions.

Sometimes the approach may, in addition, be reeducative, with an attempt at explaining www.freepsychotherapybooks.org 116 the meaning of destructive behavior patterns, helping the counselee to clarify feelings, and fostering an awareness of how the counselee behaves in relationships with family and other people.

If emotional factors seem to be responsible for the counselee's inhibitions, this may be pointed out to the counselee as well as ways in which the counselee may overcome blocks.

The counselor neither handles resistance and mechanisms of defense in terms of the total psychodynamic operations of the individual nor focuses on early conditionings and the unconscious forces that play upon the person.

However, where counselors have received appropriate postgraduate specialized training in psychotherapy, they may be qualified to add to the battery of counseling methods the operations of psychotherapy.

They will then function as psychotherapists rather than as counselors.

Although counseling borders closely on the domains of psychotherapy, unless some demarcation is made in boundaries and responsibilities, interprofessional communication and cooperation are apt to suffer.

www.freepsychotherapybooks.org 117 7 Basic Ingredients of Psychotherapy Emotionally upset people are constantly being assisted in achieving homeostatic equilibrium through a variety of approaches.

Taking a vacation, changing jobs, confiding in a concerned and wise friend, consulting a minister, swallowing tranquilizing substances, adopting a different philosophical outlook, and talking to a professional consultant all seem to bring relief.

Both informal approaches and formal psychotherapy are helpful.

The soothing embraces of a human relationship, the automatic arousal of a magical placebo element, and the releasing powers of emotional catharsis are parcels that may bring a person to an adaptive equilibrium.

The method is non-specific and diverse: it could be amulet, pill, environmental change, homely philosophy, systematized dogma, or scientific method.

But the fact that any contact between two human beings or that any device, appliance, or technique seems to bring relief does not justify our applying the label of psychotherapist to the healer and psychotherapy to the tactic.

People are abidingly achieving relief from symptoms in a propitious environment.

But only rarely—and this is most fortuitous—do they acquire a significant enrichment of their behavioral or creative potentials.

What we are concerned with is the studied manipulation of forces in a professional relationship that can, in addition to restoring homeostatic equilibrium, bring about behavioral and personality change with greater frequency than would occur by chance or through the activities of nonprofessional “helping” agencies or professional counseling.

Can we identify properties of psychotherapy that can bring about deeper and more permanent change than other forms of helping?

One of the great bewilderments in appraising the virtues of psychotherapy is the difficulty of assigning to it specific processes and effects apart from the non-specific instrumentalities of “helping” and the subsidies of casework and counseling.

In practice, the techniques of helping, counseling, and psychotherapy merge imperceptibly; as to www.freepsychotherapybooks.org 118 effects, it is generally impossible to apportion the degree of improvement brought about by non-specific and specific moieties.

Nevertheless, it is of more than heuristic value to attempt to distinguish aspects of relating to which we may affix the term “psychotherapeutic” as differentiated from counseling and helping.

Psychotherapeutic-like services are often rendered without intent by persons with no training whatsoever.

For example, what would one call the ministrations of an individual who is visited regularly—sometimes as often as six times weekly—by a steady “clientele” suffering from a wide range of psychiatric syndromes, a person who serves the purpose of relieving their emotional symptoms by (1) dispensing a tranquilizing substance more effective than the most powerful psychotropic drug and (2) relating with the clientele, variably reassuring, guiding, advising, and interpreting.

The “clients” in turn interact with this administrative individual as well as with the souls around them, participating in the boons associated with placebo influence, suggestion, and group dynamics.

Under the influence of the “tranquilizing drug” their resistances are softened, and they are

apt to express themselves volubly, often with free associations, experiencing emotional catharsis, exhibiting transference reactions (sometimes an actual transference neurosis), revealing aspects of their unconscious, and occasionally exhibiting acting-out tendencies that are usually dealt with firmly by the individual in charge.

No patients are more dedicated to their "sessions" than are these clients.

The individual to whom I refer practices the skills daily in every local bar in the country; his steady clients are among the sickest individuals in our society.

There are probably more bartenders functioning in the role of psychotherapists in this country than there are psychiatrists, psychologists, and psychiatric social workers combined.

Yet they dispense their medicaments without prescription, and they go through their interviewing maneuvers with no psychiatric supervision whatsoever.

To call such an individual a psychotherapist is obviously preposterous, and to dignify his activities as a form of psychotherapy would be a disservice to the art.

This is only one example (and there are many) of untutored helping agencies to whom an emotionally upset individual may turn who may serve a therapeuticlike function.

www.freepsychotherapybooks.org 119 Helpful intervention is also the intent of many professionals whose training equips them to deal with special segments of behavior and to enlist in this process community resources.

Human beings in trouble are constantly consulting such professionals to aid them in resolving their distress.

Generally the agency selected is authorized by social sanctions to manage a particular complaint; the assessment by the client of the problem will determine the choice of professional.

Thus, for marital difficulties a lawyer may be the counsel; for economic hardships, a social worker; for educational failings, a teacher or educational psychologist; for moral quandaries, a minister; and for physical troubles, a physician.

The goals of such consultations are (1) to assuage the prevailing tension, (2) to correct remediable disturbances responsible for the individual's present predicament, (3) to rectify deviant behavior, and (4) to prevent the outbreak of more serious disorders.

The responsibilities of both counselor and client are more or less explicitly defined, the role expectations of the counselor being structured by training and experience.

The effect of such counseling may be psychotherapeutic in essence, but the techniques employed and the objectives approached vary from those of psychotherapy.

Moreover, these counselors in other professional areas generally do not have the training or experience to deal definitively and correctly with emotional difficulties, although they may be able to mediate their effects.

Perhaps the most significant way psychotherapeutic relationships (other than supportive psychotherapy, which has a kinship with counseling) differ from nonpsychotherapeutic ones is that in the latter the helping agency or counselor enters into collusion with the neurotic forces to achieve an immediate objective.

In psychotherapy, there is an opposition to, and a direct attack made on, the neurotic forces in the hope of disposing of them and of reconstituting new and more adaptive defenses.

Table 7-1 attempts to distinguish helping, counseling, and psychotherapeutic situations in reference to a number of important variables.

Perhaps the main reason that psychotherapy has so often been considered affiliated with casework and counseling is that no attempt has been made to classify supportive psychotherapy in a category apart from re-educative and reconstructive psychotherapy.

Unless this is done we are unable to separate psychotherapy from other forms of helping.

www.freepsychotherapybooks.org 120 All helping, counseling, and psychotherapeutic situations

embrace automatic healing elements that are released during the relationship between client-patient and helper-counselor-psychotherapist.

These include the positive accruals of a projected idealized relationship along with the bounties of placebo influence, emotional catharsis, suggestion, and group dynamics.

A vital aspect of the prevailing interaction is identification with the helper-counselor-psychotherapist.

More or less, all clients-patients will regard the person to whom they relate as a model to pattern themselves by.

They will incorporate, consciously or unconsciously, that individual's ideas, attitudes, and values into their reality-testing and problem-solving activities.

The possession by the helper-counselor-psychotherapist of appropriate personality characteristics and attitudes will enhance this therapeutic dimension, while their absence may interfere with it.

Present also in all helping, counseling, and therapeutic relationships are elements of transference and countertransference, the understanding and management of which may constitute the difference between a successful and unsuccessful outcome.

In helping situations, counseling, and supportive psychotherapy, positive aspects of transference and countertransference are cultivated to enhance the operations being promoted; negative aspects, if recognized, are reasoned away or avoided.

In some reeducative and all reconstructive psychotherapies, positive and negative transference and countertransference are examined and analyzed as a means of understanding the patient's behavior and of aiding him or her in altering it.

COMMON ELEMENTS IN ALL PSYCHOTHERAPIES Let us now, then, attempt to delineate some processes that are inherent in effective psychotherapy, which while perhaps present to some extent in helping and counseling are not deliberately nurtured.

All good psychotherapeutic systems—irrespective of their theoretical underpinnings and while they manifest some differences—employ these processes to a greater or lesser degree, whether they involve conventional dyadic insight approaches, manipulations of the patient-therapist relationship, or selected reinforcement of special aspects of behavior.

www.freepsychotherapybooks.org 121 Interviewing Procedures Communication is the channel of interchange between patient and therapist.

Practitioners of different methods are usually taught principles of interviewing consonant with their theoretical systems.

In the main, the practitioner must be able to subject the patient's communications to selective scrutiny, directing comments toward facilitating and constructively utilizing verbalizations.

This involves an ability to employ language that is understandable to the patient.

It includes an awareness of nonverbal behavior, an index of some of the most important defensive operations.

It entails knowledge of techniques of maintaining the flow of significant verbalizations either toward free association or toward selective focusing on pertinent themes.

It embraces methods of understanding or inculcating insight by various techniques, including interpretation.

It encompasses an understanding of how to terminate the interview.

These formalities are often left to chance during training, and it is only through experience that the practitioner gains the interviewing skills that are most helpful to his patients.

Establishment of a Working Therapeutic Relationship Unless a cooperative empathic contact is established with the patient, the therapeutic process may come to naught.

An effective system must maintain this as a prime objective during the first part of therapy.

The techniques of achieving a relationship are rarely formalized, but usually they involve

gaining the patient's confidence, arousing expectations of help, accenting the conviction that the therapist wishes to work with the patient and is able to do so, motivating the patient to accept the conditions of therapy, and clarifying misconceptions.

Without a working relationship, in reconstructive therapy, there can be no movement into the exploratory and working through phases of therapy; the patient will be unable to handle anxieties associated with the recognition and facing of unconscious conflict.

In supportive and reeducative therapy, a good relationship expedites progress immeasurably.

Determination of the Sources and Dynamics of the Patient's Problem

www.freepsychotherapybooks.org 122 Cognitive learning is present in all therapies.

The different psychotherapies attempt to search for and to explain the patient's emotional difficulties in varying terms, such as discordant elements in the environment that mobilize stress, distorted interpersonal relationships that prevent the individual from self-fulfillment, conditionings that rigidly dragoon the patient to destructive behavior, and unconscious conflicts that mobilize anxiety and interfere with a realistic adjustment.

All psychotherapies attempt such explorations within the framework of special theories about human development and adaptation that include to a greater or lesser degree some of Freud's monumental discoveries and refinements of Pavlovian concepts.

It is generally considered essential in the resolution of a problem toward reconstructive change for the individual to become aware of the fact that one is being victimized by repetitive patterns that force one to actions opposed to a productive life.

These patterns are rigid and compulsive; they defy logic and common sense; they are both supported and opposed by ambivalent value systems that have been incorporated within the self; they make for an undermining of security and self-esteem, and for helpless expectations of injury that are registered in reactions of anxiety.

The physiologic and psychologic manifestations of anxiety, and the marshalling of defenses against anxiety create various symptoms of neurosis.

Much of this dynamic turmoil goes on below the level of awareness, and its recognition is opposed by the mechanisms of denial and repression that both safeguard the individual against anxiety and help to retain the neurotic gains residual in their preservation.

A variety of resistances operate to maintain this denial-repression.

The individual who is being treated under the aegis of this dynamic model is, through a number of techniques, taught to recognize personal offensive patterns and their consequences, the repudiated conflictual aspects of the psyche, and the origins of difficulties in destructive past conditionings.

Awareness of stress sources and conflicts hopefully enables the individual better to challenge current maladaptive patterns, to be liberated from old values, to rectify the disturbed life situation with new modes of relating to people, and to develop a more wholesome and realistic conception of the self.

Supportive and reeducative therapies deal more with provocative reality factors in the here and now, while attempting to correct faulty past learnings in line with goals of symptom alleviation and problem solving.

www.freepsychotherapybooks.org 123 Many contemporary psychotherapeutic systems utilize some of the fundamental principles of Freud, though they affix to these their own labels.

To a greater or lesser degree, concepts of the unconscious, repression, transference, and resistance are acknowledged.

The means by which the patient is brought to an awareness of problems and the extent of exploration of the unconscious, will depend on the type of theoretical orientation to which the therapist has been exposed.

The focused interview, free association, dream interpretation, analysis of the transference,

exploration of genetic material, and the buildup of a transference neurosis will thus be employed in varying degrees.

Behavioral therapies, rooted in conditioning theories, do not put much credence on insight acquisition; rather they focus more on tactics of relearning.

But inherent in the techniques employed is a relationship between therapist and patient and the inevitable derivation of some insight as part of the corrective therapeutic experience.

Utilization of Insight and Understanding in the Direction of Change Effective psychotherapies acknowledge that understanding is not enough, that conditioned patterns of behavior do not allow themselves to be displaced so easily, and that various techniques must be implemented to produce change.

Techniques, therefore, are put into effect to create incentives for change, to deal with forces that block action, to promote problem-solving and reality testing, to help the patient to master anxieties investing normal life goals, to correct remediable environmental distortions, to encourage adjustment to irremedial conditions, and to accept personal limitations and handicaps while fulfilling creative potential to the highest degree.

Behavior therapies focus on this relearning dimension almost exclusively.

Resistance and the Readiness for Change In all forms of therapy resistance will appear in stark or disguised forms and may block or destroy therapeutic progress.

Despite the fact that suffering is intense and symptoms disabling, the patient may resist changing a preferred way of life.

Toward this www.freepsychotherapybooks.org 124 end the efforts of even experienced therapists may be blocked.

The bounties derived from pursuing a course that must inevitably result in anxiety and turmoil may not be apparent on the surface.

The patient seems frozen into unreasonable bad habits that refuse to dissolve.

And credit for failure may be ascribed to the impotence of the therapist and the worthlessness of the latter's methods.

Resistance to change can paralyze all forms of therapy, and the capacity to recognize their subtleties and to deal with them constructively spell the difference in any psychotherapeutic endeavor between a therapeutic triumph or a debacle.

What we seem to be dealing with in all of our patients is their readiness for change, which apparently involves the degree to which they have spontaneously or with professional help resolved their resistance to change.

An individual with reasonable readiness to move forward will seem to benefit from almost any situation or tactic that can be used constructively.

For years there may have been silent building either through spontaneous insights and propitiously reinforcing life experiences or in formal therapy, with few apparent signs of improvement.

Should more psychotherapy later be sought, improvement or cure may then unjustifiably be entirely credited to the second treatment experience, however brief or coincidental it may be, or to some dramatic event in life that actually served as a convenience that was successfully manipulated.

Multiple obstructions in the form of resistance are apt to present themselves at every phase of treatment.

They may oppose the establishing of a working relationship, the acceptance of explanations of the therapist, the full cooperation with the therapist's techniques, the search for provocative conflicts, the probing into genetic material, the facing of reality, the abandoning of the pleasure values and secondary gains of neurotic tendencies, the acceptance of maturity, and the giving up of the treatment situation when termination is necessary.

Obstructions appear in various masquerades, as transference, as “acting out,” as forced “flight into health,” as self-devaluation, and as innumerable other disguises.

The skill of the therapist is revealed by dexterity in recognizing and managing the resistive maneuvers of the patient.

www.freepsychotherapybooks.org 125 Patient Variables There are an endless number of variables that the patient brings into therapy that will augment or negate the direction of psychotherapy.

The expectations of the patient, the kinds of symptoms possessed, the attitudes and reactions to the therapist as an authority as well as to the techniques being employed, and the intensity and persistence of childish distortions are among the most common factors that must be taken into account.

Perhaps of greatest importance is whether or not the patient will utilize the relationship with the therapist for objectives inimical to therapeutic goals.

Thus if residual dependency needs exist, the patient may overidealize the therapist and project personal aspirations for magic onto the therapist.

Basking in the sun of the therapist’s celestial power, the patient will establish a satellite position insisting that the therapist cure him or her even in the absence of any personal effort.

We are all victims of past conditionings and habit patterns, some of which interpose themselves subtly on our present-day adaptations.

If a person as a child has been able to maintain identity only by resisting or fighting parental authority, there is no reason why we should not suspect that the individual will attempt to treat the therapist with similar defensive tokens.

These may never interpose themselves in outright defiance; rather they may take the more subtle form of an inability to respond to remedial promptings.

An insidious pattern possessed by some patients who seek to enhance their independence and emancipation is a detachment that separates individuals from others and from themselves and accordingly tends to rob them of many of life’s pleasures.

The presenting complaint may be depression and generalized anhedonia.

We may find that a meticulous application of techniques fails to register marks on the patient’s indifference.

When we realize that our patient has an investment in maintaining detachment, that it has always served the patient as armor against being controlled and manipulated, that it dulls threatened anxiety and a thousand imagined hurts, we can see that efforts toward its maintenance have greater reinforcement value for the patient than the rewards we as therapists can proffer.

There may be nothing faulty in our techniques, but psychological obstructions act as impenetrable barriers to our efforts.

This is why a high level of motivation is so important in all therapies.

www.freepsychotherapybooks.org 126 Therapist Attitudes and Operant Conditioning The proper therapist attitudes are therefore crucial for effective psychotherapy as they are probably important for all kinds of learning.

They constitute powerful reinforcers that strongly influence the patient’s behavior.

Attitudes of empathy, warmth, and understanding tend to promote positive feelings in the patient; they relieve tension and lower the anxiety level.

In such an atmosphere learning is enhanced.

Interviewing, focused by the therapist on anxiety-laden content, may then prove rewarding.

Thus, if dynamically oriented, the therapist will pursue and encourage the patient to explore zones that are usually resisted or repressed.

Approbative responses, verbal and nonverbal, from the therapist reward the patient when repudiated material is prosecuted.

In addition to the temporary benefits of emotional catharsis, the patient learns that this material can be tolerated, and when placed in the context of the historical past, a reevaluation may occur.

Schedules of selected reinforcement foster the extinction of anxiety-provoking past experiences and their present-day associations.

In behavior therapy the patient is also exposed, in the medium of a rewarding emotional climate, to reinforcers that help extinguish certain reactions that have been self-defeating and accentuate others that have an adaptive potential.

Symptom relief and the acquisition of constructive behavior patterns occur without the formality of insight.

Apart from specific reinforcing maneuvers that are implemented in dynamic and behavioral approaches, the therapist-patient relationship itself serves as a relearning experience from which the patient may generalize constructive responses toward other relationships.

This gratuity may occur in any helping or therapeutic relationship.

Dynamic approaches have the advantage of working with transference contaminants that can effectively block this happening.

Where interfering transference is not bypassed, but dealt with firmly in terms of its genetic roots, and the patterns and defenses that it embraces are skillfully analyzed, it will tend to undergo negative reinforcement and extinction.

The therapist relationship will then become a powerful corrective experience for the patient.

This does not mean that cure is automatically guaranteed, since in some cases psychic damage is so profound, the secondary gain benefits so intense, the masochistic need so

www.freepsychotherapybooks.org 127 great, that inner rewards for the perpetuation of transference exceed those the therapist can supply by approving-disapproving tactics.

Nevertheless, in a considerable number of patients the developing and unravelment of transference can be most facilitating of extensive personality alterations.

Behavior therapies, while remarkably effective in promoting symptomatic improvement and behavior change, cannot approach the depth of reconstructive personality change possible in selected patients exposed to dynamic therapy with trained psychotherapists whose personality structures contain the proper ingredients of warmth and understanding, and who know how to deal with transference and countertransference.

A question immediately poses itself.

Is not the proposed climate for some types of psychotherapy, for instance classical psychoanalysis, a neutral, detached one, and, if so, would not the patient then respond in an antitherapeutic way to the traditional detached manner of the therapist?

The answer to this question lies in the simple fact that effective psychotherapists, including psychoanalysts, are not really neutral and unconcerned.

They communicate, in spite of practiced noninterference and passivity, an understanding of and empathy toward their patients.

The patient quickly discerns from nonverbal cues the underlying true emotional feeling of the therapist.

Noneffective therapists (including psychoanalysts), on the other hand, who personality-wise are detached, cold, uninvolved, or lacking in empathy will stimulate negative therapeutic reactions in their patients.

Extensive training and experience will not compensate for the absence of positive personality qualities, without which no technique can truly be productive.

Psychotherapists generally practice preferred methods that over the years have yielded enough

triumphs to reinforce faith in their powers.

What we do in therapy is tempered constantly by how we do it.

We have an affinity for some techniques and prejudices toward others.

Not all procedures make sense, nor will they work for all therapists.

A highly discriminating process generally takes place as therapists gain experience and find that certain theories and special techniques seem effective in their hands.

A problem that plagues our field, of course, is the tendency to apply one's personal experience to the world at large.

The fact that a therapist finds a particular approach of www.freepsychotherapybooks.org 128 great value for her or him does not mean that other therapists will do likewise.

Countertransference An effective psychotherapeutic system recognizes negative damaging consequences of countertransference.

The prejudiced responses of the therapist to the patient, positive or negative, may interfere with the latter's getting well.

The nature of countertransference projections onto the patient will depend upon the specific problems of the therapist that are being activated by the patient at the time.

These may be unique to a single case, or they may occur in different forms with various patients.

Where a therapist is victimized by feelings over which he or she has little control, such an individual may not be able to apply techniques with a proper measure of disciplined objectivity.

For instance, a therapist repulsed by homosexuality is not the preferred resource for an individual pursuing a nonheterosexual life style.

A therapist who is fearful of aggression may display anti-therapeutic behavior when verbally attacked by a disgruntled soul.

The passive, ingratiating, helpless patient may arouse overprotective attitudes in the therapist who may act as a crippling shield, isolating the patient from the realities of life.

Such attitudes will interfere with the working relationship, the womb in which personality change and other benefits are propagated.

In contrast, countertransference may be, if utilized correctly, an important indicator of nonverbalized attitudes and feelings that are being projected onto the therapist and thus prove helpful in understanding the patient's conflicts and needs.

Environmental Variables In considering what ingredients enter into psychotherapy, we cannot neglect social forces.

Existing cultural trends and the prevailing life style may motivate patients to seek out special types of therapy and actually influence their learning patterns.

In our presentday rock-loving, drug-dominated culture, members of the younger generation are especially attracted either to the expressive types of therapy characterized, on the one hand, by acting-out, screaming, and shedding superego restraints and, on the other hand, to an escape from tensions and responsibilities through meditation, psychotropic

www.freepsychotherapybooks.org 129 substances, and indulgence in Eastern philosophies.

It may be futile to try to impose variant therapeutic techniques on such individuals.

They may be more attuned to therapists with unconventional styles, particularly therapists labeling themselves as "avant-garde" who practice original and unorthodox methods that border on the irrational.

The environment itself in which the individual functions will influence therapeutic change both during and following treatment.

Thus, a milieu that reinforces destructive behavior will neutralize and one that rewards healthy behavior will encourage the success of the therapist's efforts.

Recognition of the environment in which the individual functions and will be forced to live in after therapy will permit the therapist to focus on elements that need to be altered or, if irremediable, adapted to without compromising the gains achieved in psychotherapy.

Termination of Therapy The termination of all types of therapy is best handled in a planned way and not left to chance.

An analysis of any obstructive dependency elements in the therapist-patient relationship is part of this process.

The patient is generally induced to shoulder the bulk of probings into personal problems and to take total responsibility for his or her plans and activities.

Independence and assertiveness are goals toward which the patient is encouraged.

The patient is prepared for possible relapses and reminded that should any symptoms return, the tool of self-understanding acquired in therapy should help him or her regain equilibrium.

SUMMARY Having delineated the important aspects of process, can we reasonably assume that these will bring good results?

In the main, yes; but, as has been indicated, there are important qualifications.

There are certain limitations to change in all people; there are certain potentialities for change in all people.

If the psychotherapist applies himself or herself to the task with disciplined process, he or she will be best equipped to foster in www.freepsychotherapybooks.org 130 patients a successful outcome.

www.freepsychotherapybooks.org 131 8 Is Psychotherapy Effective?

In some circles the idea still prevails that psychotherapy is a swamp of marshy theories imbedded in a quagmire of metapsychological slogans and convoluted methodologies.

This perhaps was the sentiment behind the Congressional queries several years ago regarding the effectiveness of psychotherapy while asking for demonstrated proof of its value.

Unfortunately, it has been extremely difficult to establish, without question, a causal relationship between techniques and methods of any psychotherapeutic system known today and the changes that have been brought about through the expedencies of that system.

Both the futility of all forms of therapy in altering neurotic processes (Eysenck, 1952, 1954, 1955, 1960a, 1964, 1965, 1966, 1967; Levitt, 1957, 1963) and arguments against these conclusions (DeCharms et al, 1954; Rosenzweig, 1954; Bergin, 1971) have been voiced.

Skeptics insist that neither clinical studies nor ordered observation and experiment have established beyond reasonable doubt the virtuosity of psychotherapy.

This does not mean that psychotherapy is unproductive; on the contrary, the experience of "effective" psychotherapists is testimony to its potentialities.

However, because present-day propositions that exist in the field of psychotherapy are not of a high order of empirically tested probability, it is difficult to demonstrate the consequences of treatment by any concrete methods and operations.

Attempts to apply probability theory to the events of psychotherapy are blocked by formidable difficulties that have up to the present time defied resolution.

This has encouraged some iconoclastic research psychologists to apply themselves to the evaluation of psychotherapy with the dedication of assassins.

Present-day outcome studies have yielded impressive statistics about the effectiveness of psychotherapy that contradict published negative reports (Smith et al, 1980; Andrews & Harvey, 1981; Epstein & Vlok, 1981; Am.

Psychiat.

Assn.

Com., 1982).

However, the skeptics insist that evidence from statistics is, upon close analysis, vastly

www.freepsychotherapybooks.org 132 misleading since we have few criteria upon which to gauge the quality of improvement or the specific parameters of personality that are being influenced by psychotherapy.

Thus it has been estimated that two-thirds of all patients suffering from emotional difficulties, who turn to and relate themselves with helping agencies other than psychotherapists, will, if the agencies are reasonably mature, experience "cure" or "improvement" purely as a product of the relationship.

It has been posited also by some observers that a similar proportion of cures or improvements will be registered should the same kinds of patients come under the care of psychotherapists or psychoanalysts.

With non-specific therapeutic measures, principally rest, sedation, and reassurance, Denker (1946) discovered a recovery rate of 70 percent, while Landis (1937) reported a recovery rate of 68 percent in patients who were not exposed to any therapy.

If these findings are true, psychotherapy would seem to be a fraud.

It would scarcely be worthwhile to expose oneself to the rigors and expense of psychotherapeutic treatments if at the end the results were no better than one could obtain with less elaborate procedures.

On the other hand, if one could demonstrate that the quality of the two-thirds cure or improvement was of a better grade, or if the total improvement rate with psychotherapy could be increased by at least 20 percent, the effort and financial outlay might be justified.

No matter how strong our conviction may be about the positive effectiveness of psychotherapy that is reinforced by some of the modern studies on outcome, we cannot, with a wave of hands, disregard the negative convictions of the skeptics nor the tenets of past published data pointing out the absence of irrefutable documentation that psychotherapy is more potent than spontaneous cure or counseling (Appel et al, 1953; Teuber & Powers, 1953; Barron & Leary, 1955; Frank, 1961; Eysenck, 1962).

Indeed, there are studies that seem to indicate that patients who apply for therapy and are merely put on waiting lists, receiving no further treatment, reveal after a six month's follow-up, a 40 percent rate of improvement (Endicott & Endicott, 1963), and five to six years after the initial evaluation, a spontaneous improvement rate of 65 percent (Schorer et al, 1966).

The latter rate is held to be superior to that following exposure of patients to an extensive and carefully designed treatment program.

www.freepsychotherapybooks.org 133 In the Cambridge-Somerville Youth Study (Powers, 1949; Powers & Witmer, 1951; Teuber & Powers, 1953) 2 equal groups of 325 boys were matched. The first group received therapy from adherents of both the psychoanalytic and Rogerian schools.

The second group served as controls.

Follow-up studies over a period of years disproved the expectation that the treatment group would be less delinquent than the other.

Indeed there was a slight difference in favor of the control group.

Brill and Beebe (1955), working with soldiers who had experienced a breakdown in the army, also found that no difference was scored in remission of neurosis between those who did and those who did not receive psychiatric treatment.

Barron and Leary (1955) treated a group of psychoneurotic patients and compared the end results with an untreated control group.

They discovered that "for the most part...the changes tended to be in the same direction for treatment and non-treatment groups, and of about equal magnitude." Three groups of patients were subjected to a follow-up study by Barendregt (1961).

The first division of 47 patients had been given psychoanalysis, the second of 79 patients received psychotherapy other than analysis, and the third of 74 was exposed to no form of psychotherapy.

The results showed little difference among the different groups.

Gliedman et al.

(1958), on the basis of their work, insist that placebos are as effective as psychotherapy in psychiatric cases exposed to both.

Walker and Kelley (1960), working with male schizophrenic patients, reported that short-term psychotherapy brought about no greater improvement than ordinary custodial care; indeed, it seemed to delay the discharge of patients.

The quoted results of psychotherapeutic treatment of over 70,000 cases reviewed by Eysenck (1952, 1965) concluded that these “fail to prove that psychotherapy, Freudian or otherwise, facilitates the recovery of neurotic patients.” Approximately two-thirds of patients will improve with or without psychotherapy.

Exposed to psychoanalytic treatment, the cure-improvement rates average 44 percent, variously being reported as 39 percent (Fenichel, 1920-1930), 62 percent (Kessell & Hyman, 1933), 47 percent (Jones, 1926—1936), 50 percent (Alexander, 1932-1937), and 67 percent (Knight, 1941). With “eclectic” psychotherapy the cure-improvement rate was cited as higher, averaging 64 percent.

This figure was drawn from the following: 46 percent (Huddleson, 1927), 41 percent (Matz, 1929), 55 percent (Luff & Garrod, 1935), 77 percent (Ross, 1936), 58

www.freepsychotherapybooks.org 134 percent (Yaskin, 1936), 61 percent (Curran, 1937), 54 percent (Masserman & Carmichael, 1938), 73 percent (Landis, 1938), 53 percent (Carmichael & Masserman, 1939), 63 percent (Schilder, 1939), 66 percent (Hamilton & Wall, 1941), 51 percent (Hamilton et al, 1942), 50 percent (Wilder, 1945), 58 percent (Miles et al, 1951).

The data of the Central Fact-finding Committee of the American Psychoanalytic Association, according to Brody (1962) and Masserman (1963), appear to reveal that of 210 “completely analyzed” cases, 126 were “cured” or “greatly improved.” Of the remaining 385 “incompletely analyzed patients,” it is estimated that about half achieved some improvement.

Levitt (1957), summarizing many studies on the psychotherapeutic treatment of children, arrived at a figure of 67.05 percent of cases who improve at the end of therapy and 78.22 percent at follow-up.

Eysenck (1965), in examining this mass of data, concluded that psychotherapy registers a small effect, if any, on patients, with the exception of therapies based on modern learning theory.

His conclusions, while upheld by Astrup (1965), Zubin (1965), Meehl (1965), Davidson (1965), and Wolpe (1965), have been vigorously challenged by Zetzel (1965), Frank (1965), Glover (1965), Barendregt (1965), Matte-Blanco (1965), Strupp (1965), Handlon (1965), and Bergin (1971).

To all of these criticisms, Eysenck (1973) has replied that there is not one single study that indisputably demonstrates that psychotherapy succeeds better than no treatment, or behavior therapy, or any other alternative.

Indeed, he avows that Rachman (1972) in his book, which exhaustively reviewed the literature, substantiates his own conclusions made in 1952.

He states, “I believe that psychotherapy of the usual interpretive kind is simply the premature crystallization of spurious orthodoxy, a verbal exercise without any proof of effectiveness.”

Bergin (1971) reviewing the data that Eysenck cites in his 1952 review comes up with a different statistic by taking another point of view.

For example, Eysenck quotes a study by Fenichel evaluating the work of the first ten years of operation of the Berlin psychoanalytic clinic and shows that the improvement rate was

two-thirds.

However, Eysenck obtains this rate by including those patients who dropped out of treatment after brief contact with the clinic.

If these patients are not included, then the improvement rate jumps to 91 percent.

Scientifically, however, one can validly argue for either including or

www.freepsychotherapybooks.org 135 excluding the dropouts in computing the percentage rates as improved.

Bergin has also demonstrated that the spontaneous improvement rate of two-thirds shown by control groups supposedly receiving no treatment is fallacious.

The reason that one cannot have an adequate control group in an outpatient psychotherapy study is because patients who are suffering and are refused help will usually look elsewhere for assistance.

They will go to friends, neighbors, or practitioners of various sorts (Gurin et al, 1960).

They thus do not constitute a scientific control group.

At best we must compare the results of professionally trained therapists with the results of nonprofessional operators in the community.

Some of the latter may influence patients toward improvement as or more effectively than professional therapists, largely perhaps because they are adept at impressing on their clients an unshakable optimism and thus stimulating non-specific elements of the helping relationship. Published reports of spontaneous recovery or improvement rates on emotionally ill persons in whom no formal therapy had been validated are below the two-thirds figure and range from zero upward: for instance, negligible (Orgel, 1958; Cappon, 1964; O'Conner et al, 1964; Koegler & Brill, 1967); 30 percent (Shore & Massimo, 1966); 25 percent (Kringlen, 1965a) and 18 to 22 percent (Paul, 1967).

These studies, which indicate a median rate of 30 percent, deal with varying patient populations with widely different syndromes and hardly satisfy rigorous empirical criteria (May, 1971).

Despite later writings disputing Eysenck's ideas (Bergin & Lambert, 1978; Vandembos & Pino, 1980), these continue to influence opinion and are upheld by some (Erwin, 1980).

To round out the statistical muddle, the different schools of psychotherapy cannot agree among themselves regarding the fruitfulness of their particular brands of therapy, published figures displaying a greater divergence among proponents of a special approach than between those of different schools.

There is general agreement, however, among non-analytic groups that results with their methods are superior to those of psychoanalysis.

Behavior therapists, for example, score their curative yield as roughly double that of psychoanalysts.

Psychoanalysts, on the other hand, label the results of nonanalytic therapy as "temporary" and "superficial." THERAPEUTIC IMPROVEMENT IN RELATION TO GOALS www.freepsychotherapybooks.org
Comparison of the effectiveness of competitive brands of psychotherapy is meaningless without considering the goals to which they direct themselves.

Symptom relief or cure, a legitimate and important target, is easier to achieve than attitude and behavior change, which, in turn, is more readily attainable than reconstructive personality change.

The reason for this is that habit patterns constitute for the individual a way of life.

They contain vital defenses and security operations, interference with which is bound to provoke anxiety.

The painful confrontations to which the ego must inevitably be exposed will promote resistance that may take diverse forms, one of which is flight from therapy.

Where a patient interrupts treatment before the goal of reconstruction has been reached, the initial symptoms, kept alive by resistance and transference, may still be present.

Therapy may then be graded as a failure.

Paradoxically, had treatment been discontinued during the early treatment phases when the salubrious glow of the placebo effect was still felt, and before resistance and transference had restored some symptoms, therapy may have been considered successful by the patient. However, more extensive goals would not have been reached where potentially achievable. Whereas an effective psychotherapist can obtain 80 to 90 percent of symptom cure or improvement, the positive results will be reduced where behavior change is the goal. In the event the therapist seeks reconstructive changes in patients, a figure of only 40 percent will probably be as high as can be attained, even where the therapist is highly trained.

There are many reasons why so small a percentage of patients can be influenced beyond the benefactions of symptom relief and attitude change.

With the best of intentions the therapist may succeed in tearing down, in sicker patients, their defensive structures, expectantly waiting for a new personality edifice to rear itself.

Helpless and deprived of customary neurotic resources, which have been "analyzed away," the patient will cling to the therapist with a dependent desperation that will confound both participants.

Hopeful expectations go unrewarded, the patient symptomatically being worse off than before treatment.

www.freepsychotherapybooks.org 137 As in major surgery, the risks are greater in reconstructive therapy than where palliative measures are employed.

The results will best justify the risks where cases are carefully selected.

This calls for diagnostic skills that enable the therapist to exclude patients who are least disposed toward extensive change (such as fragile psychotics, severe alcoholics, psychopathic personalities, drug addicts, borderline cases) profound characterologic dependencies, severe obsessive-compulsive neuroses, etc.

Reconstructive treatment necessitates sophisticated training and wide experience in the handling of stormy, long-term therapeutic relationships.

Nevertheless, with the skillful application of techniques, the rewards should more than justify its application in selected patients as the preferred treatment method.

THERAPEUTIC IMPROVEMENT IN RELATION TO TREATMENT PHASES At the beginning of any treatment effort, irrespective of type, extratherapeutic helping agencies operate to bring about improvement in symptoms.

Counteracting these positive non-specific influences during early treatment phases are (1) defective motivation, (2) continuing conflict that, sponsoring anxiety, revitalizes symptoms, and (3) the defensive dividends and secondary gains that make the retention of symptoms advantageous for the patient.

These interferences must be dealt with energetically by therapists as part of their technical pursuits.

Assuming that negative forces are not too prominent, or that they have been handled with proper therapeutic craftsmanship, the patient will register symptomatic improvement.

This boon is the conjoint product of such forces as the placebo influence, emotional catharsis, the projection of an idealized relationship, suggestion, and group dynamics (see Chapter 4).

Abatement of tension and anxiety and restoration of a sense of mastery will then promote a better outlook toward life.

If therapy is stopped at this period, the patient may be able to continue improvement, particularly if the sources of difficulty are dealt with constructively.

This can best be accomplished during the brief therapeutic effort after existing troubles have been explored and especially when a continuity has been established between the www.freepsychotherapybooks.org 138 immediate complaints, habitual personality operations, and determining childhood experiences and conditionings.

If, however, therapy continues beyond this early phase, initial benefits may soon expend themselves in the wake of the patient's realization that the idealized properties with which he or she has invested the therapist are truly nonexistent.

Faith, hope, and trust no longer will temper the therapeutic climate.

It is, of course, possible where the patient's need is sufficiently great—as in certain characterologically dependent personalities—for the patient to continue to endow the therapist with godlike qualities, particularly where the therapist narcissistically shares the patient's omnipotent delusions.

Under these circumstances the patient will bask in the therapist's sun, soaking up the power of celestial exposure, and feel protected and continue symptom free as a result.

On the other hand, neither the patient nor the therapist may be capable of keeping alive such a sainted image.

Indeed the therapist may purposefully retreat from playing a protective role and, particularly where striving for reconstructive goals, may even challenge the patient's defenses by pointing out existing behavioral improprieties.

An inescapable increment of the protracted therapeutic time period to which patients are exposed for purposes of extensive personality alteration is the furtive or dramatically explosive obtrusion of resistance and transference.

Where the patient's inherent personality strengths are sufficient to reconstitute personal defenses in a new climate of strife, and where the therapist is sufficiently skilled and by disposition equipped to handle the patient's insurgency, the patient will best be enabled to proceed toward a remodeling of relationships and toward values beyond the benefactions of symptom relief.

At any rate, we may expect that by-products of most therapeutic endeavors that extend themselves in time are an eruption of symptoms and a mobilization of tension and anxiety. Stopping treatment at this middle phase will usually expose a patient who insists that therapy has brought little benefit.

And yet, had treatment halted earlier, at the crest of the non-specific improvement wave, the effectiveness of the effort might have been endorsed.

In reconstructive therapy, therefore, we may expect a recrudescence of symptoms.

Challenge of resistance, maneuvers toward insight and working through, are aspects of www.freepsychotherapybooks.org 139 the therapeutic process, however painful they may be, that force patients to learn new modes of adjusting.

It is to be expected also that prolonged therapy will tend to promote dependency.

This silent saboteur may interfere with the patient's efforts toward self-actualization.

It may keep the patient reduced to an infantile status, violently promoting a return of original symptoms when the patient perceives a threat of termination of therapy.

As the therapist works through this "separation anxiety," the patient may be able to rely more and more on himself or herself and gradually to expand the feelings of assertiveness.

Table 8-1 outlines some of the foregoing processes.

THERAPEUTIC IMPROVEMENT IN RELATION TO THE THERAPIST'S PERSONALITY One of the most important variables in psychotherapy is the helping or therapeutic personage, whose character traits and technical skills are bound to influence results.

In Table 8-2 an attempt has been made to grade rates of cure or improvement that we expect in relation to desired areas of change (goals), the kinds of processes to which the individual is

exposed (techniques), and the quality of the agency administering help.

Starting with the baseline of what we might anticipate should the individual spontaneously exploit random avenues of help, we may then compare these rates with what could happen if the individual related himself or herself to some "helping" person, such as a minister, physician, teacher, friend, or authority who, while trained in a particular field, has had no special schooling in counseling or psychotherapy.

"Effective helpers" are those who possess personality qualities that inspire in the subject with whom they are working hope, faith, trust, liking, and freedom to respond.

Such helpers generally have characteristics of sincerity, honesty, a capacity to respect people, confidence in what they are doing, positiveness of approach, and what Rogers (1946) has called "genuineness," "empathic understanding," and "non-possessive warmth." Whitehorn and Betz (1960) and Betz (1962) substantiate the vital role the personality of the therapist plays in securing results with schizophrenics in www.freepsychotherapybooks.org 140 psychotherapy.

Effective therapists, they discovered, see a patient as a person and not a problem, stress assets of the patient not liabilities, challenge self-depreciatory attitudes, are reasonably permissive, behave naturally with the patient, and focus on securing a trusting relationship.

Ineffective therapists are too permissive, focus on the patient's mistakes, evince the qualities of an aloof teacher, and are more passive and permissive.

Betz (1967) suggests, however, from research studies that the outcome of therapy depends on the quality of the relationship that develops as a result of the blend of the personalities of therapist and patient.

Truax and Carkhuff (1967) have derived three scales of traits of effective therapists derived from ratings of typescripts: (1) positive regard for the patients, (2) accurate empathy, and (3) congruence.

The accurate empathy scale would seem to be related to the concept of accuracy of interpretation of what is going on within the patient, although no studies have been done to date to relate these two concepts.

Congruence refers to therapists being in touch with their own feelings.

If therapists assert that a patient is "liked," yet by tone of voice and previous statements show anger toward the patient, they are rated to be in low congruence.

Truax and Mitchell (1972) have reviewed the results of 10 years of such studies and have found that the results are very consistent across many studies employing different diagnostic groups, with therapists of different theoretical persuasions and with differing lengths of therapy.

Rogers et al.

(1967) reported an extensive controlled study of psychotherapy with schizophrenic patients who were hospitalized at a state hospital.

They found that patients whose therapists offered high levels of non-possessive warmth, genuineness or congruence, and accurate empathic understanding achieved significant positive personality and behavioral changes on a wide variety of indexes; while patients whose therapists offered relatively low levels of these interpersonal skills during therapy exhibited deterioration in personality and behavioral functioning.

These studies point to the existence of a "therapeutic personality" that effective therapists possess irrespective of their operational modes (techniques).

Ineffective www.freepsychotherapybooks.org 141 helpers do not possess therapeutic qualities and traits, and their absence, as may be seen from Table 8-2, will damage their capacity to render proper help.

Parenthetically, every helper (or counselor or psychotherapist for that matter) will relate differently to special persons.

Warmth and liking may be shown toward some individuals with whose personalities and problems

the helper identifies and with whom there is a feeling of security.

On the other hand, detachment and hostility may be manifested with other persons toward whom the helper feels alien and who tend to light up countertransference.

All helpers will consistently be more effective with certain individuals than with others.

Counselors are those who have been trained in casework and counseling techniques.

If they have the personality qualities outlined above, such training will enable them to function more expertly than non-trained helpers.

Effective counselors should accordingly score higher in results than effective helpers.

However, irrespective of how thoroughly trained, if counselors lack appropriate personality qualities, they may be classified as ineffective counselors, and the results will be no better than those of ineffective helpers.

When we consider trained psychotherapists, we view a somewhat complex picture since their theoretical and methodologic orientations vary so greatly.

We may expect that the average trained psychotherapist whose personality contains the positive units described above (i.e., an effective therapist) will be able to achieve symptom relief, as well as behavior change, above that brought about by an effective counselor.

Additionally, if the psychotherapist is trained psychoanalytically to do "depth therapy," we may expect that the most extensive and difficult to achieve goal of reconstructive personality change, potentially possible in patients selected for this approach, will at least be double that seen with therapists and counselors who do not employ insight techniques, and who depend on the adventitious operation of constructive relearning alone, which, sometimes, in a favorable atmosphere, may result in some reconstructive change.

Again, though a psychotherapist has had exhaustive training, should that therapist lack the proper personality qualities, the results will be no better than those of an ineffective counselor or an ineffective helper.

Patients may temporarily improve on the basis of non-specific extratherapeutic agencies, but soon these dividends will expend themselves as the www.freepsychotherapybooks.org 142 patients find themselves locked in a frustrating and unrewarding relationship situation.

Where a therapist with suitable traits is not too well trained or skilled in executing therapeutic maneuvers, the results will be no better than those of an effective helper who has had no training.

They will, however, eclipse those of the trained ineffective therapist who may have spent many years in exhaustive postgraduate studies that, though sharpening the cognitive processes, have made no dent on the individual's antitherapeutic personality.

The reason why the patient is impeded rather than benefited by an ineffective helping agency, counselor or psychotherapist is that the patient is blocked, by being trapped in an unrewarding situation, from spontaneously seeking out other helping resources that may bring homeostasis.

Moreover, the relationship with the ineffective helper-counselor-therapist will in all probability promote hostility and release transference distortions that will activate regressive defenses, like hostile dependency, lighting up new and galvanizing old symptoms.

We are inevitably drawn to the awesome and unpleasant conclusion that a person suffering from a psychological problem is better off with no treatment at all than if that person enters an emotionally inadequate helping or therapeutic situation, whether this involves an untrained helper, a trained counselor, or an intellectually sophisticated psychotherapist.

A crucial question is whether the appropriate personality ingredients essential for therapeutic change may be taught those who seek to administer help.

There is some evidence that helpers, counselors, and psychotherapists who do not inherently possess such traits, may be trained to communicate warmth, empathy, and genuineness without themselves entering into depth therapy.

However, this would probably apply only to those personality structures that were not too rigid, hostile, or detached—in short, to basically healthy individuals.

There is some evidence, too, that certain inflexible, hostile, and detached individuals may, with properly conducted personal reconstructive psychotherapy, work through extensive flaws in their personalities and acquire the www.freepsychotherapybooks.org 143 qualities important for functioning as effective psychotherapists.

This development, however, is not at all guaranteed, and we see evidences repeatedly of professional persons who have undergone extensive training, including personal didactic psychoanalysis, who are as arrested in their growth as when they first exposed themselves to treatment, and whose contributions to the ailing masses the world could very well do without. The consequences of their education and training have become the target of critics who are constantly sniping at the results of psychotherapy.

Available statistics do not convey an accurate picture of the potentialities of psychotherapy since they lump together the results of effective with those of ineffective therapists that because of the “deterioration” impact (Bergin, 1963, 1967; Truax & Carkhuff, 1967) cancel each other out and reduce the score to a figure not too much higher than that of the spontaneous cure.

Psychotherapy has vast potentials as a healing force, but it is the product of a complex equation, the elements of which require careful scrutiny, contemplation, and unravelment. It can be executed properly only by selected individuals who inherently possess or have acquired appropriate personality characteristics that will enable them to relate constructively with their patients.

It is enhanced by scholarly grounding and sophisticated postgraduate schooling best available through disciplined educational resources specialized to teach an extremely complex skill.

THE MEASUREMENT OF THERAPEUTIC IMPROVEMENT General procedures for the measurement of therapeutic improvement have been detailed by a number of authorities, including Waskow and Parloff (1975) and Gottman and Mark-man (1978).

Of primary concern is identifying the specific variables that are significant to measure and that give us reliable and valid data.

Of concern also are the research designs that can best provide answers to our questions about outcome.

The instruments that are used for the gauging of outcome must be selected carefully, recognizing that no one instrument is suitable for different patient populations and for varying forms of psychotherapy.

Rather, multiple outcome instruments are indicated.

Among the measures in use today are: self-reporting that deals with the patient's daily functioning (Cartwright, www.freepsychotherapybooks.org 144 1975; Imber, 1975); broad anamnestic material as in the popular Minnesota Multiphasic Personality Interview (Payne & Wiggins, 1972; data from family and friends (Hargreaves et al, 1975; Waskow & Parloff, 1975); a “Community Adjustment Scale” (Ellsworth, 1974); therapist assessment scales (Green et al, 1975; Endicott et al, 1976; Newman & Rinkus, 1978; Mintz et al, 1979); material from community agencies or members (Cummings & Follett, 1968; Halpern & Biner, 1972; Cummings, 1977); and changes in economic and creative output (Riess, 1967; Yates, 1980).

Insofar as research designs are concerned, a number of authorities have offered their ideas, for better or worse, including Glass et al.

(1973), Luborsky et al.

(1975), Bandura (1978), Cronbach (1978), Kazdin (1979), and Cook and Campbell (1979).

Formidable problems exist in any attempt to measure the results of psychotherapy or to verify its empirical propositions (Pumpian-Mindlin, 1956; Kubie, 1960).

Paradigms for psychotherapy evaluation that will identify and possibly quantify clinically relevant parameters that consider the uniqueness of each patient require further development (Glass, 1984).

Ideally, we should like to observe exhaustively what takes place in the course of treatment, to study the results by subjective and objective means, to erect comprehensive hypotheses concerning the relationships between events, to deduce the consequences of such relationships, and to elaborate methods of testing inferences under controlled conditions in order to construct scientifically valid concepts.

Most efforts in this direction, however, applied to the staggering uncertainties of the clinical situation, have merely accented the fallibility of our present research tools and techniques.

The methodologic problems encountered in studies on the outcome of psychotherapy are compounded by semantic befuddlements.

Hazy language and even hazier concepts permeate the field of psychotherapy.

When we attempt to put into words what treatment aims have been achieved, we are handicapped in translating the complex formulations of the different psychotherapeutic approaches into abstractions that possess reasonable unity.

We discover that each school has its particular way of talking about and emphasizing essentially similar elements in human behavior.

The nuances that are being stressed in these ideas are often not as important as the advocates of the particular www.freepsychotherapybooks.org 145 schools would make them out to be.

Another confusing thing is that the same labels may be employed to designate markedly diverse ideas, for instance the words relationship and ego mean entirely different things when used by select theoretical schools.

Furthermore, therapists may inject into words their own private meanings that may not at all resonate with the concepts under examination.

Another pitfall is that the bewildering number of variables, many of which are not manipulable, tempt one to grapple with only a few that can be handled with relative ease.

When these are lifted out of the context of the tremendously complex physiologicpsychologic-sociologic continuum that constitutes human adaptation, they often lose their significance.

We are rewarded with a catalogue of beautifully compiled categories that mean little, particularly when we try to generalize conclusions beyond the material with which we are dealing.

Before proceeding, let us review the outstanding problems in applying scientific method to evaluation studies: 1.

There is disagreement as to which observable phenomena are worthy of observation.

2.

The data available for study are difficult to manipulate and control, interfering with conditions ripe for experiment.

3.

It is cumbersome to qualify the quantitative data of psychotherapy due to the complexity of the variables involved.

4.

Available units of measurement are ill-defined, interfering with comparisons and with the synthetization of similarities and differences into a homogeneous unity.

5.

Theoretic prejudices and personal biases make for a loss of objectivity and an interference with the ability to utilize imagination in hypothetic structuring.

6.

The absence of an accepted conceptual framework that can act as a basis for communication obstructs the formulation of inferential judgments regarding order in the observed phenomena—blocking the deduction of www.freepsychotherapybooks.org 146 valid analogies justified by the available facts, and hindering the exploration of causal connections between antecedents and consequences.

7.

The reliability of our results is distorted by a variety of other difficulties that are related to special problems of the therapist, the resistances of the patient, the amorphous status of diagnosis, the prejudiced selection of the sample, the involvements of outside judges, coders, and raters, the inability to employ adequate controls, the interferences of adventitious non-specific changes, and certain complexes inherent in the psychotherapeutic process itself. With advances in computer technology, the development of treatment manuals to standardize therapeutic operations, the use of video tape recordings to enable independent assessors to monitor adherence to the prescribed techniques, the greater employment of adequate controls, the development of more sophisticated research designs, and improved follow-up studies on research, findings should become more reliable.

The question is how much will such findings enhance our techniques for the better.

The answer to this is still uncertain because individual interpretations of research findings can be different, and styles of operation vary so much that one therapist may completely foul up a prescribed method that another has utilized with great effectiveness.

This is one problem that a prepared “Manual of Operations” poses.

How the manual fits in with the preferred style of a therapist will determine its usefulness.

It may put some therapists into a straight jacket and may totally cripple their spontaneity and flexibility.

On the other hand, it may structure operations for some and make them more precise.

What a therapeutic manual may be able to do is to define more precisely what enters into a specific form of treatment (behavioral, psychoanalytic, eclectic, etc.).

When we examine what therapists do under the banner of a specific form of psychotherapy, we sometimes find that their operations deviate from what is commonly accepted as standard (Strupp, 1978).

Problems Related to the Therapist Not the least of the sources of error are the individuals who supply the data on www.freepsychotherapybooks.org 147 which we are dependent, particularly the therapists and the patients (Lambert & Utic, 1978; Parloff et al, 1978b).

A psychotherapist's assessment of what is accomplished in treating a patient is apt to tell us more about the therapist's narcissism than it does about what actually happened to the patient. One pointed example of how therapists may contaminate the data is their direct or indirect influencing of the verbal content.

This is significantly affected by comments and even indications of approval or disapproval as contained in nonverbal cues (Greenspoon, 1954a,b).

The material that is brought out may consequently be a facsimile of the therapist's ideas and values that have been subtly communicated and are now being regurgitated by the patient upon proper stimulation.

We repeatedly are confronted with productions from patients that seem to validate the specific theoretical systems espoused by the therapist.

Most psychotherapists have a tremendous investment in their theories and methods.

By the time they have completed training they have some firmly set ideas about human behavior. As students in training, affirmation of the approach in which they are being groomed offers rewards of approbation, of completion of training, of admission to the special graduate society

with status privileges, and of economic security in the form of patient referrals.

Denial of the verity of the system poses the hazards of being accused of hostile resistance, of exclusion from the ranks of the privileged, of dismissal from training, of excommunication and possible financial doom.

Against these odds, the preservation of the thinking integrity is put under a strain that few students can resist.

Glover (1952) has remarked, "There is a tendency in the training situation to perpetuate error." This error may be carried over to the patient who is the recipient of the therapist's values, no matter how passive and nondirective the therapist imagines himself or herself to be. The very nature of the psychotherapeutic process demands that the therapist have a conviction about what he or she is doing.

As.

F.

C.

Rhodes Chalke remarked in his presidential caveat at the Canadian Psychiatric Association (Psychiatric News 9:1, 1974): "It has been demonstrated that much of the therapeutic effectiveness of psychiatrists resides in an inherent faith in one's curative powers.

This can be significantly diminished if one were to be confronted in measured terms with one's failures or successes." www.freepsychotherapybooks.org 148 Dedication to one's system is an important constituent in the therapist's mental set.

If the therapist evinces a scientific attitude of skepticism, it may be interpreted by a patient as unsureness and lack of belief in the method.

This may influence adversely the patient's expectations of help and reduce faith in the therapist.

The therapist, therefore, may find that dogma tends to stabilize some patients as well as himself or herself.

While it may help one area of functioning, it does introduce errors that may be fatal in the experimental evaluation of therapeutic activities and results.

The therapist's bias also extends itself to patient selection, so that our examining the results may be handicapped by a restricted sample.

Therapists are inclined to select patients who will, according to their experience, respond best to their methods.

A not too inconspicuous screening process may be employed to eliminate a wide spectrum of patients and problems.

In intensive long-term therapy, an economic factor also enters into the picture, an important criterion of selection being, not the syndrome or personality, but the ability of the individual to afford the financial drain that will have been made.

Thus the patients who avail themselves of long-term treatment may belong to a different subcultural group than those who apply for short-term therapy, with value systems and social pressures that make for distinctive reactions and singular responses to psychotherapy.

Economic and status factors also interfere with the reliability of a therapist's accounting of results.

As a recording apparatus, the therapist possesses many defects.

Bias may divert from presenting factual data, leading the therapist to some erroneous conclusions.

Berg (1952) has humorously commented that when a patient loses his or her job, the therapist will find outside factors responsible; however, when the patient gets a salary increase, the therapist is willing to take credit for singular clinical competence.

A therapist is generally unwilling to stand by passively and offer an unprejudiced account of

why patients have failed to get well.

He or she has, as Greenacre et al.

(1948) pointed out, "too great a stake in the patient's recovery." When a researcher seeks to examine records or to listen to session recordings, the therapist most likely will refuse to deliver them.

Why become a sacrificial victim?

Why should one be a party to one's own discredit as a www.freepsychotherapybooks.org 149 therapist?

What constitutes a well-trained professional person who is qualified to do good psychotherapy is another delicate matter around which many heads have been shattered.

Avoiding the ever annoying topic of medical-non-medical, we can say that an individual who has been graduated from a reputable postgraduate school run by qualified and trained psychotherapists, which has an adequate screening process for candidates, a wellformulated program of didactic instruction, competent and elaborate supervision of a varied caseload, and provisions for personal psychotherapy or psychoanalysis of its students, can arbitrarily be considered a psychotherapist.

A study of cases handled psychotherapeutically by persons whose qualifications are questionable is certainly not a reliable index of the value of psychotherapeutic techniques.

After all, not every person who drives a nail can be considered a cabinetmaker.

Among the graduates of postgraduate schools for psychotherapy will be those with relatively fair, average, and excellent degrees of competence; there will be differences in the results of the individual practitioners with special kinds of patients and problems; there will be varying personality difficulties and aberrant characteristics among them that will determine, as has been indicated in an earlier part of this chapter, whether they will be effective or ineffective psychotherapists.

Of importance also is the ability to utilize methodologies that have proven valuable for special syndromes.

Many therapists attempt to squeeze all patients into the kinds of technique with which they are familiar.

This is understandable, but the patient may become a sacrificial victim as a consequence.

Problems Related to the Patient There are many sources of error in the patient's statements about results achieved in therapy.

Notoriously, many patients are poor judges of what actually is going on, but unfortunately, they may be the only witnesses at our disposal to testify.

They may, however, prove to be reluctant witnesses.

Patients are motivated to seek help for their suffering; their objective is not to offer themselves as research vehicles.

Asking them to subject themselves to psychological studies, to interviews with observers and research www.freepsychotherapybooks.org 150 workers, and to follow-up manipulations may mobilize attitudes and feelings that negate therapeutic aims.

It is always necessary to validate, by objective criteria, subjective reports by patients, good or bad, since their judgments are obviously colored by resistances.

These may operate either in the service of a false report of well-being, as may be the case where the patient conceives of further therapy as a threat; or they may evoke a depreciation of legitimate developmental progress, for instance, where such progress does not conform with personal value systems or those of the subcultural group to which he or she belongs.

The patient may have firm convictions of what is desired out of therapy that do not parallel the objectives of the therapist or meet the goals of reasonably good mental health.

For example, Mary Jones may be interested merely in resolving her tension, neutralizing her

anxiety, and reducing her symptoms so that she can function comfortably.

She may be willing to restrict specific life functions, adopt protective or precautionary defenses, or forego important measures of self-fulfillment in order to achieve peace and relief from suffering.

From her standpoint her therapy will be a success, if she achieves these goals, and if she symptomatically feels better she may consider herself cured.

From the standpoint of the therapist, Mary Jones may be only moderately improved, since she has not altered any of her basic life patterns.

From the yardstick of ideal mental health objectives, she may have retrogressed, in view of her abandonment of vital human functions.

The Problem of Diagnosis Another factor that interferes with good evaluative research is that our present-day psychiatric nosologic systems are still in need of reordering.

They embrace conglomerate labels that depict anxiety, symptomatic manifestations of anxiety, defenses against anxiety, character traits, and disintegrative phenomena.

Often a diagnosis is made of that symptom complex that is most disturbing to the individual even though coordinate pathologic elements exist that are more fundamental and more serious, but happen to be less annoying to the patient than the complaint factor.

Or the therapist may favor a special www.freepsychotherapybooks.org 151 group of manifestations and focus attention on these.

Even well-trained therapists examining the same case may, therefore, arrive at different diagnoses.

This is, to say the least, confusing to the researcher who attempts to establish some unity among the diagnostic categories.

The sharpening of diagnostic systems would certainly lead to a greater clarity about goals in therapy and help in our evaluative effort.

The authors of DSM-III and DSM-III R have made estimable progress toward this end.

New attempts are now in process, of working out more suitable classifications.

The problem is a most complex one since emotional difficulties influence every facet of functioning—physiologic, intrapsychic, interpersonal, and community relationships.

What distinguishes one individual from another is the unique configuration of tendencies and traits in the form of residues of faulty conditionings, fingerings of immature personality promptings, defective modes of dealing with aggression and sexuality, manifestations of anxiety and its neutralized derivatives, and defensive and characterologic distortions.

A person is qualitatively different from others because each individual is quantitatively constituted and disposed differently.

What we perhaps need to do in establishing a more serviceable classification is to determine which clusters of traits and problems, or combinations of the latter, respond best to special therapeutic approaches.

The ability to define significant clusters that have responded to special treatment attempts presupposes that we have a way of identifying them.

Of great handicap is improper, and even downright irresponsible, recording at the time treatment was begun.

It is manifestly impossible to gauge what has been accomplished for an individual if we have no starting point from which we can begin our measurements.

Too often we find ourselves desperately grasping for clues, attempting to reconstruct from memory or from the feeble jottings in our notebooks the approximate status at the beginning of treatment.

The fact that so many areas are pathologically afflicted in any emotional problem makes inadequate our simple designations of change.

Labels of “cured,” “much improved,” “moderately improved,” “unchanged,” and “worse” mean little in view of the complex systems with which we are dealing.

Rarely is the patient influenced similarly throughout the physio-psycho-sociologic spectrum. The patient may have experienced great www.freepsychotherapybooks.org 152 symptomatic improvement and even cure of a disabling condition, yet the patient’s interpersonal difficulties may continue in force.

The patient may have improved in work capacities and community relations, but marital and sexual adjustment may have deteriorated.

When we utilize ratings of change, it would seem important, therefore, to assign these to the areas to which they relate.

This would enable us better to appraise the specific zones of disturbance that respond to special approaches and techniques.

In group therapy, for example, we may find that the patient derives great help for interpersonal problems, while entrenched intrapsychic defenses have scarcely budged.

Among the areas we may consider for our ratings are the following: 1.

Changes in manifest symptoms 2.

Changes in interpersonal, family, work, educational, and community adjustment 3.

Changes in physical health 4.

Changes in feeding, eliminative, sexual, rest, sleep, and activity patterns 5.

Changes in consumption of alcohol, sedatives, tranquilizers, and other medications A fruitful area of research is in the development of scales by which we can assess the various outcome criteria.

We may then better be able to compare the results of varying kinds of therapy, performed by different therapists, with diverse types of problems and patients, and then contrast these results with what happens when no formal treatment has been administered.

Some years ago attempts in this direction were made by defining change in descriptive and operational terms without evaluating the changes (Witmer, 1935; Hunt & Kogan, 1950; Watterson, 1954).

How an evaluative scale may then be employed to utilize such descriptions has been described by Malamud (1946).

The Problem of the Sample In evaluating a psychotherapeutic system one may tend to neglect how the sample was selected.

Because many therapists screen their patients, choosing those whose www.freepsychotherapybooks.org 153 problems, in their opinion, are best suited to their methods and skills, a prejudiced weighting of the caseload is possible.

Our conclusions regarding the worth of a therapy, therefore, may have to be restricted to a special group of patients and problems.

For instance, an individual applying for formal psychoanalysis may be rejected if that individual does not possess a balance of characteristics that are lumped under the designation of “ego strength.” Other therapies may practice no partisanship in choosing candidates for their methods.

Attempts to contrast results between different therapies must obviously take into account the fact that we may be dealing with varying populations.

Thus, O’Connor and Stern (1972) studied the effects of psychoanalysis (4 sessions weekly for a minimum of two years) and psychotherapy (semiweekly for no longer than two years) on 96 patients with functional sexual disorders.

They reported an improvement rate of 77 percent with psychoanalysis and only 46 percent with psychotherapy.

This might sound significant except that the group selected for psychoanalysis “evidenced less

illness than those who received psychotherapy.” We are dealing then, at least in the area of this sample, with two dissimilar groups.

Some workers, in judging the results of treatment, include all individuals who have been interviewed and accepted for therapy, even though they do not continue beyond a few sessions. Others exclude persons who have discontinued therapy against advice.

Obviously this discrimination alters the percentage of individuals improved.

It is very easy to manipulate statistics to substantiate almost any bias, such partiality being not always unconscious.

Another error commonly overlooked is failure to mention the cultural and subcultural background of the persons composing the sample.

Therapeutic results are influenced to a marked degree by conditions in the social environment to which the patients must adapt themselves after they have completed their treatment.

Indeed, certain neurotic defenses may be mandatory if the individual is to survive in a predatory environment.

Value systems regarded as constructive from the standpoint of ideal mental health may be a source of victimization where one must adapt to groups perpetuating erratic conventions and folkways.

It is essential in doing comparative studies to include a www.freepsychotherapybooks.org 154 description of the individuals who are being evaluated, how they were selected, their background, their socioeconomic status, the length of treatment, and other information that might influence the treatment results.

What is important also is when in time evaluations are made “whether repeatedly, during the course of, or immediately afterwards, or at variable and increasing intervals after it is over” (Kubie, 1973).

These considerations will help reduce the error involved in generalizing beyond the sample to the population at large.

In our effort to select a homogeneous sample, we must keep in mind the fact that no two persons are alike even though their diagnosis is similar.

Thus individuals in a sample of agoraphobic patients will vary in their physiologic makeup, developmental conditionings, educational backgrounds, environmental experiences, intrapsychic structures, personality organizations, living milieu, values, and sundry other personal determinants, including age, severity of symptoms, and motivation.

These will make every patient a totally unique human being, who in a pool of other human beings, looks and behaves in certain ways distinctively from the rest.

In responses to the same kind of psychotherapy, we may expect differences.

In measuring change we need to know what area or areas are to be assessed.

The selection of different criteria by which to estimate improvement or cure has led to a great deal of confusion.

Thus the measure of successful problem-solving, symptom relief, or behavior change accepted by behavior therapists may not be acceptable to psychoanalysts who gauge improvement by reconstructive personality alterations.

Neither may be considered basic by therapists practicing humanistic approaches whose criterion would be inner peace, happiness, and creative self-fulfillment.

What has been suggested is the establishment of multiple measures that cut across the different kinds of psychotherapy.

Therapeutic changes involve many facets of an individual's functioning, not all of which are easily measurable.

Among these are symptoms, relationships with people, values, self-esteem, work capacity, self-image, economic status, creativity, etc.

No patient progresses equally along all dimensions of possible change.

To assess these we would www.freepsychotherapybooks.org 155 require a variety of instruments. The usual pronouncement of "improvement" or "cure" generally connotes merely a relief of symptoms, and does not indicate what sacrifices are being made to achieve this.

For example, a patient with depression initiated by loss of a loved person who served as a maternal object may find his or her depression cured when a new maternal companion is found. Should the patient coordinately be in therapy, the cure falsely may be ascribed by the patient to the effect of treatment.

In estimating change, we are confronted with the dilemma that the multiple change criteria with which we deal are not standard and that situation-specific behaviors (like efficiency at work) are more easily assessed than personality traits.

To bring some order to this muddle attempts have been made to establish a battery of measuring devices.

Waskow and Parloff (1975) have recommended as a standard test battery: the Minnesota Multiphasic Personality Inventory (Dahlstrom et al, 1972); the Hopkins Symptom Checklist (Derogatis et al, 1973); the Psychiatric Status Schedule (Spitzer et al, 1967, 1970); Target Complaints (Battle et al, 1966); and a choice of either the Personal Adjustment and Role Skills Scales (Ellsworth, 1975), or the Katz Adjustment Scales (Katz & Lyerly, 1963).

The value of these or any other proposed batteries will require further evaluation.

For the most part, outcome assessment will rely on the patient's divulgements, on disclosures of the therapist, as well as on the reports of family and friends, all of which may be highly biased.

Nevertheless, we may have no other way of assessing change than through these declarations and through the use of instruments such as the battery of tests cited above by Waskow and Parloff (1975).

Lambert (1979) has written an excellent review of measurement batteries, and useful measures have been described by Miles et al, 1951; Lorr and McNair, 1965; Strupp et al, 1969; Malan, 1976a; and Meldman et al, 1977.

Insofar as personality tests are concerned, they have not been found too useful (Mischel, 1977).

The Use of Outside Judges, Coders, and Raters Assuming that we are able to define concretely the variables that we wish to www.freepsychotherapybooks.org 156 observe, we would heighten the reliability of our observations to employ a number of competent and trained judges to go over the data on which we will base our estimates of therapeutic change.

A number of problems arise here related to the confidential nature of the material, the need on the part of the patient to retain anonymity, the belief that observational intrusions alter the behaviors of patients and therapists, and the fear of the therapist that his or her therapeutic competence may come under challenge.

During the training process in an outpatient clinical setting, the patient may be prepared to accept outside adjuncts as part of the treatment and the therapist may be motivated to work with raters and coders.

In private practice, however, this is practically impossible.

In the event we are able to employ qualified accessories, the reliability of the measuring and rating instruments that we have at our disposal at the present time is probably high enough to permit consistency in our results (Herzog, 1959).

It is essential, however, that we have dependable data, that we be explicit in the definitions of what is to be coded and rated, and that the accessory workers be properly qualified and trained in the use of the selected categories.

Whether or not accessories are used, once we have settled on the categories to be rated and are

satisfied that our methods of rating are reliable, we must still question the validity of what we are doing.

Let us say that we have accurate statistics about changes in symptoms and adaptive patterns; does this necessarily give us accurate information about the emotional status, whether the individual has been cured, is improved or unimproved?

An example may make this clear.

In going over the record of a patient who has completed therapy, the patient's work performance will come up for appraisal, work being one of the factors in evaluation.

We discover that shortly after leaving therapy, the patient was demoted.

This does not seem to be a good indication of the patient having made progress in treatment.

Yet when we examine the circumstances closely, we see that the basis for the patient's demotion is that he or she has become less obsequious and masochistic; hence the patient is more capable of resisting unfair demands.

This independence surely will not endear the patient to a domineering and exploitative foreman or employer.

On the other hand, it is conceivable that an increase in salary or job advancement may be the product of www.freepsychotherapybooks.org 157 a neurotic acquisition of overambitiousness and self-exploitative character traits that drive a person to be dedicated to the job with a merciless grimness that is so often rewarded by money and position.

A safeguard to the faulty assessment of environmental data is that each item is not viewed in itself, but rather is related to the totality of the patient's adaptation.

Not only must reports of environmental adjustment be questioned for their validity, but other indices of change must also be considered with caution, and only in relationship to the entire range of indices.

The relative weights to be given to each variable in the adjustment picture is a matter that needs to be decided individually after all of the elements have been put together.

Obviously, variables of change cannot be scored on an even basis, nor can the emphasis in rating extended to a set of criteria in one individual be transferred to any other individual.

The total constellation of forces that operate will determine the unique emphasis, if any, that is given to each variable.

Ratings of adjustment following a point scale system are consequently invalid unless the scores are considered of unequal weight, depending on special circumstances.

Because the weighting of so many of our items is based on the opinion of the observer, the element of prejudice cannot be eliminated from our results (Miles et al, 1951).

The sole safeguard we have is the background, training experience, and reputation of the researcher, which gives us some indication as to the researcher's reliability.

Yet we cannot be at all certain that a researcher for emotional and other reasons, consciously or unconsciously, may not abandon objectivity for the triumph of verifying a preconceived idea.

A good researcher is one who not merely can do an analysis of variance or compute a chi square, but more importantly, perhaps on the basis of personal experience, can blend common sense with the nuances of therapy.

The Problem of Controls The problem of controls in psychotherapy is perplexing.

Let us suppose that we have a clinic staffed by trained, competent psychotherapists who employ accepted psychotherapeutic techniques, and that we reject every other patient applying to the clinic for help and utilize the rejects as controls.

Assuming that we have developed proper www.freepsychotherapybooks.org 158 criteria for evaluating change in the direction of mental health, that we have evolved efficient ways of data gathering, and a methodology that enables us to deal expertly with this information, we still would not be able to say with reasonable certainty that psychotherapy was the only

dimension of difference between the experimental and the control groups.

Our quandary is that there is no such thing as exactly the same kind of an emotional problem. Subtle and oft undetectable cognitive, emotional and behavioral differences within the same syndrome may actually be the determining factor.

By no stretch of the imagination can we say that the environmental pressures, or the healing adventitious situational elements, that impinge themselves on any two people are the same, nor can we keep them anywhere near constant throughout the period of our differential study.

Experience with control groups shows that it is difficult to regulate the lives of human beings so that they behave the way a research sample should.

The matching of similar cases is, therefore, a haphazard process, based more upon hunches than facts.

Indeed any attempt to use controls may introduce new errors.

It is conceivable that if we followed thousands of cases that had been treated over a period of years, as well as a roughly similar number of control subjects, our errors would be reduced. However, this could not be guaranteed even though we could afford the formidable costs that such research would entail.

From a practical standpoint we are limited in using the sound experimental method of pairing individuals as a means of equating experimental and control groups with respect to relevant variables.

The proposals of utilizing each individual as his or her own control in the tactic of "wait" control (Dymond, 1955), the analysis of variance (Miller, 1954), the use of statistical control through covariance methods (Dressel, 1953), the dealing with control phenomena through "control in data" (Gordon et al, 1954), casual visits but no real treatment ("attention" or "placebo" control) (Kazdin & Wilson, 1978), treatment as needed ("PRN contact") (Weissman, 1979b), and the employing of the principles of "randomization" (McNemar, 1949) are ingenious, but they do not resolve all of the essential problems, no matter how we manipulate the sophisticated statistical devices we have at our disposal.

It is difficult to obtain a large number of homogenous patients who randomly can be assigned to special treatments for a set period of time.

www.freepsychotherapybooks.org 159 The use of dropouts ("terminator controls") of those who fail to keep any appointment or stop visits early in treatment also has many flaws (Gottman & Markman, 1978).

As a way out, it has been suggested that, instead of using a control group of untreated individuals, we employ two groups of the same population being treated by different methods. Our results would surely then be dependent upon the skill and experience of the therapists, accepting the contention that good therapists get approximately the same results, irrespective of their theoretical and methodologic differences (Fiedler, 1950a,b, 1951).

We would have to make sure that therapists practicing different methods be comparable in ability.

Zubin's (1953) idea of establishing a "standard control group" that would act as a basis for comparison with treated groups is interesting, but, as he puts it, still somewhat idealistic.

It is impossible to match patients on any more than a few personality variables.

Frequently a "no-therapy" group is set up as a control after an initial interview.

No therapy is an invalid concept.

An initial interview is a form of therapy and patients can benefit significantly from a single contact with a trained professional person.

After the initial interview, the patient who is not accepted for therapy does not exist in a vacuum.

That patient will exploit many measures to relieve his or her symptoms or to resolve problems.

These range from tranquilizers, to self-help measures, to relationships with sundry individuals through whom the patient may work through some of the difficulties.

The idea that the patient is receiving no therapy then is not true, even though the therapy is non-formal.

Other problems relate to informed consent in order to protect the rights of patients, and to the ethical issue of withholding treatment from a needy patient who happens to fall into the control group.

To disclose to a patient that he or she is being used as a guinea pig, and an untreated one at that, may not meet with universal acceptance and may thus compromise the research design.

To deny a patient treatment we know is needed constitutes a dilemma that the lofty principle of research for the sake of science cannot resolve.

www.freepsychotherapybooks.org 160 The Problem of Adventitious Change One of the bewilderments of evaluation is that we have little against which we can compare our results.

Statistics, as has been previously indicated, generally uphold the dictum that approximately two-thirds of all patients improve irrespective of the kind of psychotherapy to which they have been exposed.

They also attest to the fact that approximately two-thirds of persons with emotional problems also improve by arranging for their own destinies.

These figures mean next to nothing because we have no idea as to what is connoted by the words "improvement" or "psychotherapy" or "no treatment whatsoever." We have no data on the practitioners who have presumably rendered therapy, nor on the constituent problems and syndromes, as well as their severity, that have been treated in contrast with those that have not been treated.

We have no idea of the specific parameters of personality influenced in the patients who have received psychotherapy, as opposed to those who have achieved stabilization through the circumstance of extratherapeutic forces.

Are persons who apply for professional help those who have in greater proportion failed to achieve benefit through extratherapeutic elements?

It would seem more than coincidental that individuals who have exploited every device and resource in quest of relief from anxiety begin to improve with the institution of psychotherapy.

Problems in Dealing with the Statistical Data It is not irrelevant to point out that research studies in mental health can be flawed by an improper analysis and reporting of the quantitative results.

Spitzer and Cohen (1968) describe three common errors: inability to distinguish between statistical significance and magnitude of association, measurement of reliability, and neglect of statistical power analysis.

Because professional people in the field of therapy are generally untrained in quantitative techniques, it is essential that a researcher skilled in statistical methods be consulted whenever a serious research study is contemplated.

Before data is collected, it is important carefully to review the hypotheses to be tested, the kinds of data to be accumulated, and the statistical techniques to be employed.

This emphasis does not justify a deification of statistical methods as ends in themselves, but rather as tools that www.freepsychotherapybooks.org 161 can order massive data and lead to reasonably valid inferences.

CONCLUSION Concern with the rising costs of health care has focused the spotlight on the safety and cost-effectiveness of psychotherapy.

Governmental authorities and insurers are asking for proof regarding the usefulness of the various kinds of psychotherapy.

Can we verify the worthwhileness of an expensive project of psychiatric or psychological treatment?

The difficulty of supplying scientific evidence of merit is complicated by the fact that, no matter how good a species of psychotherapy may be, it will not prove costeffective in the hands of a bad therapist.

But even if we accept what the most dubious researchers now concede, that psychotherapy is at least minimally effective and better than no treatment or the use of a placebo, most impartial observers would have to consider it a beneficial enterprise.

But can we say it is cost-effective and that the benefits justify the expenditure of time, effort, and money?

This depends on how we rate the tangible and intangible costs of emotional disturbance and how much monetary value we put on human suffering and the misfortunes psychological illness foists on the community.

When we consider the misery wrought by neurotic symptoms—the awesome damage to families, the wrecked marriages, the derailed lives, and the shattered productivity that follow in the wake of a neurosis—and add to these calamities, crimes, delinquencies, rapes, arsons, murders, suicides, violence in the streets, and the ravages of alcoholism and drug addiction that are neurotically or psychotically inspired, we may ask: “How costly is it to society not to try to prevent these tragedies through some kind of corrective procedure?” Is not even a minimally effective solution better than no solution at all?

The motive on the part of governmental authorities for inquiring into the safety and cost-effectiveness of psychotherapy is understandable.

The various procedures employed today in treating mental and emotional problems, such as dynamically oriented psychotherapy, behavior therapy, family therapy, group therapy, marital therapy, pharmacotherapy, and others, are substantially safe and effective when executed by trained, experienced, and skilled professionals.

What makes a procedure unsafe and www.freepsychotherapybooks.org 162 ineffective is not the technique itself, but how it is applied.

A scalpel in the hands of an unskilled surgeon can be a dangerous and useless instrument.

Pardes, while Director of the National Institute of Mental Health, pointed out that the question of solid proof of treatment effectiveness extends across the entire health care field.

In a 1978 report from the Congressional Office of Technology Assessment, only 10 to 20 percent of all health care technology had been proven effective by formal methods.

Many of the commonly employed medical procedures have never been satisfactorily evaluated.

Controlled studies in the mental health field definitely show that psychological treatments rate at least no worse than treatments in medicine and surgery.

But further research in psychotherapy is necessary.

Research in psychotherapy is still burdened by many handicaps.

Yet the literature is replete with studies flaunting impressive statistics that “prove” the superiority of one brand of psychotherapy over others or that downgrade all forms of psychotherapy as worthless or limited at best.

We still do not possess a model of psychotherapy research that we can consider uniquely applicable to the special problems and conditions existing in psychotherapy (Frank, 1979; Karasu et al, 1984).

Nevertheless, in the opinion of the majority of practitioners, of patients who have received treatment, and of unprejudiced observers, psychotherapy, properly instituted, is the most effective measure available to us today for the treatment of emotional problems and for the liberation of potential adaptive and creative resources in the individual.

The fact that research in psychotherapy to this date has had surprisingly little impact on

contemporary clinical practice, should not discourage future attempts to substantiate the effect of psychotherapy by scientific means.

There is evidence that with recognition of the complexity of the variables involved and vitality and sophistication that is currently being manifested by researchers in the field, the outlook is an optimistic one.

Malan (1973) in a historical review terminates his paper with the prediction that we will eventually find "that there are particular techniques appropriate for particular types of patients, which give outcomes for which words written very long ago by Kessel and Hyman (1933) are appropriate: 'this patient was saved from an inferno, and we are

www.freepsychotherapybooks.org 163 convinced this could have been achieved by no other method.'"
" www.freepsychotherapybooks.org 164 9 Supportive Therapy The object in supportive therapy is to bring the patient to an emotional equilibrium as rapidly as possible, with amelioration of symptoms, so that the patient can function at approximately his or her norm.

An effort is made to strengthen existing defenses as well as to elaborate better "mechanisms of control." Coordinately, one attempts to remove or to reduce detrimental external factors that act as sources of stress.

There is no intent to change personality structure, although constructive characterologic alterations may develop serendipitously when mastery has been restored and successful new adaptations achieved.

There are times when supportive therapy is all that is needed to bring about adequate functioning.

This is the case where the basic ego structure is essentially sound, having broken down under the impact of extraordinary severe strains that sap the vitality of the individual.

A short period of supportive therapy will usually suffice to restore equilibrium.

On the opposite end of the pathological scale are those victims so deeply scarred by childhood experiences that the radical surgery of intensive psychotherapy can only increase the disfigurement.

The only practical thing we can do for some of these casualties is the topical cosmetic repair of symptom alleviation so that they can live more comfortably in spite of their handicaps.

While we do not yet possess the diagnostic tools to assess accurately which patients will benefit most from supportive expedients, some therapists rely on the pragmatic principal of resorting to supportive therapy where more extensive measures fail to produce an adequate response.

On the other hand, where case loads are overwhelming, therapists may be inclined to utilize supportive measures as a routine, reassigning patients for educational or reconstructive approaches where results are not satisfactory.

Since supportive therapy attempts the achievement of symptom relief or symptom removal, the question presents itself regarding the value of symptomatic cure.

Among the www.freepsychotherapybooks.org 165 most solecistic of legends is the notion that elimination of symptoms is shallow if not worthless.

This notion stems from the steam engine model of psychodynamics that conceives of energy in a closed system, bound by symptomatic defenses, which when removed, releases new, more dreadful troubles.

This is in spite of the fact that physicians from the time of Hippocrates have applied themselves to symptom removal in both organic and functional ailments with little or no baneful consequence.

Yet, legends survive from one generation to the next.

The presumed dangers of symptom removal are now as threatening to some younger therapists as they were to their teachers.

Little proof is offered of a causal relationship between the fact of symptom relief or removal and any pathological sequelae.

The evidence persuades that supposedly precipitated disasters are either coincidental or the product of inept therapeutic interventions (Spiegel, H. 1966).

The complaint, then, that symptom removal is an arbitrary, incomplete, irrational, and unsatisfactory approach in psychotherapy is apocryphal.

Yet, we cannot entirely dismiss the anecdotal accounts and experimental studies (Szasz, 1949; Browning & Houseworth, 1953; Seitz, 1953; Jones, HG, 1956; Paneth, 1959; Crisp, 1966; Bruch, 1974a) of infelicitous effects, but on the whole the empirical evidence for symptom substitution is not consistent.

I have, in my own consultative practice, seen occasional untoward results with supportive approaches.

However, I have also seen baneful consequences with reeducative and reconstructive approaches. My impression is that it is not the method that produces bad results; rather it is how the method is applied, as well as the fragility of the ego structure on whom it is applied.

A callous therapist who batters away with any technique without adapting tactics to the immediate reactions and sensitivities of patients may become a greater instrument of harm than of help.

The transference reaction can negate any therapeutic benefits; indeed, it may in itself be responsible for a therapeutic debacle.

Experience convinces that supportive measures, carefully and selectively applied, can, as a result of desirable symptom alleviation, promote substantive behavioral benefits.

In summary, symptom relief or removal is an essential goal in any useful psychotherapeutic program.

It may occur "spontaneously" or be brought about by a variety of methods, such as by drugs, by conditioning techniques, by faith and prayer, and by insight.

While one may not agree with Eysenck (1960a) that elimination of a symptom www.freepsychotherapybooks.org 166 cures the neurosis, there is evidence that it contributes to a better adjustment and to the elimination of auxiliary symptoms clustered around the original complaints.

Any therapy that leads to enlightenment and greater self-understanding without symptom relief may legitimately be regarded as a mediocre success if not a failure.

But this does not mean that symptom control should constitute the sole objective of therapy.

Undermining of the basis of the individual's symptoms, the resolution of past conditionings and current conflicts that nurture more symptoms expands one's potentials for happiness and creativity.

While such an objective may come about as a byproduct of symptomatic change, its studied achievement should be an important target.

In many patients receiving supportive therapy, consequently, an effort should be made to motivate them toward some kind of reeducative or reconstructive change in order to insure greater permanence of results.

Supportive measures thus may be utilized as the principal treatment or as adjuncts to reeducative or reconstructive psychotherapy.

They are employed as: 1.

A short-term exigency or expedient for basically sound personality structures, momentarily submerged by transient pressures that the individual cannot handle.

2.

A primary long-term means of keeping chronically sick patients in homeostasis.

3.

A way of “ego building” to encourage a dedication to more reintegrative psychotherapeutic tasks.

4.

A temporary expedient during insight therapy when anxiety becomes too strong for existing coping capacities.

An understanding of how and when to do supportive psychotherapy is, therefore, indispensable in the training of the psychotherapist.

Supportive therapy does not work in many cases where the problems with authority are so severe that the patient automatically goes into competition with the helping individual, depreciating, seeking to control, acting aggressive and hostile, detaching www.freepsychotherapybooks.org 167 himself or herself, or becoming inordinately helpless.

These reactions, appearing during therapy, may act as insurmountable resistances to the acceptance of even supportive help.

Therapies with designations of “palliative psychotherapy,” “social therapy,” “situational therapy,” and “milieu therapy” fall into the supportive category.

Many of the tactics utilized are similar to those in casework and counseling.

Among procedures employed in supportive therapy are guidance, tension control and release, environmental manipulation, externalization of interests, reassurance, prestige suggestion, pressure and coercion, persuasion, and inspirational group therapy.

GUIDANCE Guidance is the term given to a number of procedures that provide active help, in the form of fact giving and interpretation, in such matters as education (educational guidance), employment (vocational guidance), health, and social relationships.

Many casework, counseling, and educational operations come under the category of guidance.

The extent to which the patient’s life is manipulated varies with the patient’s condition and with the system of guidance employed.

Most of the guidance schemes are patterned after those described years ago by Payot (1909), Vitoz (1913), Walsh (1913a, b), Barrett (1915, 1925), Eymiew (1922), Traxler (1945), and, Erickson (1947).

The role that the therapist plays may be that of a directing authority who arranges for a planned daily regimen and allows no time for idleness and destructive rumination.

A balanced program may thus be organized, relating to the time of arising, bodily care and grooming, working schedule, diet, rest periods, recreation, sleep, and other activities that will account for every hour of the day.

Such complete control of the patient’s routines, however, is rarely necessary.

Usually, guidance is aimed at a specific disturbing problem that interferes with adjustment.

Instruction is given in ways of detecting, examining, and avoiding stressful situations.

Courses of action realistically suited to the problem are then outlined.

Recommendations may also be made toward specific adaptive goals, like enhancing one’s career, furthering one’s education, etc.

This advice is particularly helpful to individuals whose emotional problems are not too severe.

www.freepsychotherapybooks.org 168 Guidance is based on an authoritarian relationship established between therapist and patient.

One of the problems inherent in such a relationship is that a dependent patient may tend to overestimate the capacities and abilities of the therapist to a point where the patient’s reasoning abilities and rights to criticize are suspended.

Under these circumstances any doubt regarding the strength or wisdom of the authority will arouse strong insecurity.

Hostility and guilt feelings, if they develop at all, may be rigidly repressed for fear of

counterhostility or disapproval.

One may recognize in such irrational patterns the same attitudes that the child harbors toward an omnipotent parent.

Actually, the emotional helplessness of the neurotic individual resembles, to a strong degree, the helplessness of the immature child.

The neurotic person may project the original authority that was invested in parents, and may be seeking from the therapist extravagant evidences of support and love.

Characterologically dependent persons particularly demand demonstrations of infallibility.

Should the therapist display human frailties or appear to lack invincible qualities, the faith of the person may be shattered, precipitating helplessness and anxiety.

Mastery may then be sought by annexing oneself to another agency in whom magical and godlike features lacking in the previous host are anticipated.

The life history of such dependent individuals shows a flitting from one therapist to another, from clinic to clinic, from shrine to cult, in a ceaseless search for a parental figure who can guide them to paths of health and accomplishment.

Because they have so often been disappointed in this search, some persons will resent guidance, even though they feel too insecure within themselves to direct their own activities.

Others will reject guidance because of previous experiences with an authority who has been hostile or rejecting or who has made such demands on them for compliance as to thwart their impulses for self-growth.

Acceptance of advice may, to certain individuals, be tantamount to giving up their independent claims on life.

In spite of its disadvantages, guidance may be the only type of treatment to which some patients will respond.

Desperately helpless in the grip of their neurosis, such individuals have neither the motivation nor the strength to work with a technique that www.freepsychotherapybooks.org 169 requires self-direction.

Resistance to self-assertiveness is so strong that a parental figure must prod them into performing their daily tasks.

A guidance toward religion is sometimes deemed expedient (Holman, 1932; Blanton & Peale, 1940; Poole & Blanton, 1950).

An attempt is made here to convince the patient that health and self-fulfillment can be achieved by self-devotion to prayer.

In yielding to a stronger power, the patient can crush dread, overcome fear, and even achieve peace of mind.

Christian Science (Bates & Dittmore, 1932) and other "faith cures" draw on these principles.

Tormented by self-doubt and riddled by anxiety, the individual may reach for a solution through salvation.

By confessing his or her wickedness the patient becomes a candidate for forgiveness and a future blessed existence.

Attitudes of defeatism change to those of hopeful anticipation.

As one of God's chosen instruments, a sense of dignity is acquired.

Competitiveness may then be abandoned for compassion and hate for love.

Some patients, comforted by religious forms of therapy, benefit also through participation in church activities.

Within its limited orbit, therefore, guidance has utility in therapy.

One does not employ it under the illusion that any deep changes will occur in underlying conflicts or the dynamic structure of the personality.

Causal factors are usually whitewashed, and the person is encouraged to adjust to problems

rather than to recify them.

The patient may be taught many methods by which emotional blind alleys can be avoided. The patient may learn to correct certain defects or to adapt to circumstances that cannot be changed.

However, where guidance is not supplemented by other therapies calculated to render the person more self-sufficient and independent, fundamental difficulties in interpersonal relationships will probably not be altered.

TENSION CONTROL Tension activates many disturbing physiological and behavioral tendencies. More subdued than anxiety, of which it is undoubtedly a component, it often registers its effects subversively through the autonomic nervous system, influencing the functioning of various organs.

It is one of the earliest signs of emotional disturbance, and once mobilized www.freepsychotherapybooks.org 170 it may continue to torment the individual even after neurotic defenses have been established.

Tolerance of the effects of tension varies.

There are some persons whose repressive mechanisms work so well that they are unaware of how tense they are even though their physical health is affected by resultant physiological imbalances.

There are others whose sensitivity to tension is so extreme that they are in a constant state of uneasiness and discomfort.

Individuals with poor impulse control may release their tension in passionate outbursts even though this leads to violence, sexual acting-out, and sundry behavioral improprieties.

Because tension is so discomfiting, it is little wonder that escape from it constitutes a chief preoccupation of human beings, who will eagerly utilize assorted devices in pursuit of peace of mind.

Some of the contingencies exploited have a destructive potential.

Alcohol, for example, is often employed as a potent tranquilizer; its use is universal, a part of our social tradition.

It narcotizes the brain and easily, while its effects last, subdues awareness of tension.

Most people are capable of controlling the intake of drink, but for those who attempt to utilize it as a prime calmative resource, it poses serious hazards.

In recent years other tranquilizing substances (such as Valium and Xanax) have entered the scene.

Enthusiastically prescribed by physicians (by far, they constitute the most commonly sold drugstore medicaments), they are fervently utilized by patients, some of whom after several months may become addicted.

Barbiturates and nonbarbiturate hypnotics continue to enjoy a risky popularity.

This is not to deprecate the value of tranquilizers, sedatives, and hypnotics as temporary expedients, but caution must be exercised in their use since they cannot be employed as a way of life.

Among the less noxious modes of regulating tension are exercises in self-relaxation, self-hypnosis, meditation, and biofeedback.

There is no reason why these devices cannot be employed as part of a comprehensive program where tension is too unsettling.

Their influence is palliative and can be helpful, but their value is greatest where they are utilized with psychotherapy (see Chapter 56).

It is to be expected that many practitioners, with and without proper qualifications, www.freepsychotherapybooks.org 171 will offer unique schemes for tension release, including bodily massage, tapping, slapping, touching, intimate games, screaming exercises, spiritual

discipline, and self-induced altered states of consciousness.

Popular volumes flood the market and have varying virtues for individuals seeking self-help direction for tension control.

The professional person may derive some good ideas from a few of these volumes (e.g., Krippner & Kline, 1972) that can be adapted to one's own technique and style of operation.

Muscle relaxation exercises have been used for many years.

Many of these are founded on the system of the nurse, Annie Payson Call (1891), which combined muscle relaxation, rest, and "mind training" for purposes of repose.

The best known modern exercises are those of E.

Jacobson (1938) and Rippon and Fletcher (1940).

D.

H.

Yates (1946) and Neufeld (1951) have described a series of exercises that enables the individual to gain voluntary control over tension.

Massage enhances muscle tone in addition to encouraging relaxation (Jensen-Nelson, 1941; Mennell, 1945).

Enforced rest also has a relaxing effect on the individual's muscular system.

In part, this was an objective of the old Wier-Mitchell (1885) "rest cure," which combined isolation, diet, massage, and electrical stimulation.

These measures were often reinforced by prescribed isolation from relatives.

Weir-Mitchell's method gained wide repute, although beneficial effects were probably as much induced by psychologic as by physiologic factors.

Schultz's "autogenic training" (Schultz & Luthe, 1959) provides the dividend of muscle relaxation during self-hypnotic exercises.

The influence of muscular relaxation on the individual's tension is purely palliative and should be accompanied by some form of psychotherapy.

Unlearning of dysfunctional habits that escalate tension will require daily practice of a chosen method (Benson, H., et al, 1974).

This may involve nothing more than E.

Jacobson's simple progressive relaxation of muscle groups (1938) from scalp to toes or vice versa.

Such self-relaxation techniques can quiet major muscle groups and ultimately lead to substantial tension control.

It is estimated that on the average approximately 2 months of daily practice are required for satisfactory results.

As the subject experiences a sense of awareness about his or her bodily reactions, the subject may be able deliberately to reproduce at will an adequate state of relaxation without needing to go through all the www.freepsychotherapybooks.org 172 prescribed exercises.

Autogenic training (Schultz & Luthe, 1959) strives for a reorganization of subconscious thinking patterns through the use of a technique of "passive concentration," an unstructured relaxed form of cogitation and association.

No formal muscle relaxation maneuvers are utilized, yet the technique promotes a sense of warmth and lightness over the entire body.

Suggestions emphasize peacefulness and quietness and enjoin the subject to allow such feelings to develop without forcing them.

Meditation has a long history, most frequently being associated with the Buddhist religion (particularly the Zen cult) and with Yoga.

More recently Transcendental meditation has attracted large groups of people, particularly the young (Forem, 1973).

Experimental studies have shown that meditation can produce striking psychophysiological effects, including alleviation of tension, lowering of oxygen consumption and metabolic rate, and decrease of cardiac output (Wallace, RK, 1970; Glueck & Stroebel, 1975).

Many techniques exist for the production of meditation, ranging from practiced suspension of thinking to concentration on monotonous environmental stimuli, to repetition of certain sounds or words (mantras), to special forms of physical exercise.

The association of meditation with mysticism is understandable.

As in hypnosis, a change of body image and the evocation of weird fantasies may strike some as evidence of preternatural worlds.

But meditation may be practiced apart from mystical union with the absolute and without achievement of "enlightenment" and "universal wisdom," should these essences not suit the philosophical bent of the subject.

Faithful practice of meditation from 20 to 30 minutes daily is usually required for proper practice toward mastery of tension (Carrington & Ephron, 1975).

Biofeedback training is a recent entry into the arena of self-regulated tension control (Stroebel & Glueck, 1973; Glueck & Strobel, 1975).

There is considerable evidence that one may gain conscious command over involuntary bodily functions by receiving sensory information from visceral organs.

There gradually develops a type of operant conditioning that enables a person to monitor certain physiological functions.

The most common instruments employed are those that record changes in skin potential (psychogalvanic meter), brain-wave activity (EEG machine), muscular function (electromyograph www.freepsychotherapybooks.org 173 machine), and skin temperature (thermal machine).

By learning to diminish or increase the auditory signals or amplitudes on a visual scale, one may be able to produce greater quantities of alpha waves (a phenomenon associated with lessened anxiety), to reduce activity of the sympathetic nervous system, and to achieve profound muscle relaxation.

Lowering of blood pressure in hypertension, healing of stomach ulcers, and relief of migraine are among the conditions that have been successfully managed.

Relatively easily mastered instruments are available for biofeedback training through which most persons are able to develop an ability to regulate subconscious physiological activities. This kind of visceral learning holds out great promise for the future in treating psychosomatic ailments.

Therapists may profitably explore this field to see whether it may enhance their therapeutic repertoire, should their practice involve the treatment of many patients with psycho-physiological ailments.

In the event the therapist does not possess the instruments, the patient may be referred for adjunctive biofeedback therapy to a behavior therapist skilled in the method.

Most patients can be trained in about 10 sessions.

Some practitioners utilizing tension control as the primary treatment method employ a combination of techniques, for example, progressive relaxation, skin galvanometry, hypnosis, and meditation.

In my own experience, I have found that most patients do well with self-relaxation alone, utilizing an audiotape, the making of which is described later in the book.

Where this does not suffice, I have sometimes employed an inexpensive psychogalvanometer with electrodes that attach to the fingers as a way of measuring skin resistance.

Some patients do better if they possess a means of overtly gauging their progress, and a biofeedback apparatus in this way serves them well.

Patients with tension headaches often benefit from learning how to relax the frontalis muscle

with the electromyographic (EMG) machine, while migraine may respond to learning to direct the blood flow from the head to a hand, employing a thermal biofeedback apparatus.

MILIEU THERAPY (ENVIRONMENTAL ADJUSTMENT, SOCIOTHERAPY) A vast number of stimuli from outside impinge on the individual daily.

Some of www.freepsychotherapybooks.org 174 these are clearly responsible for an existing acute emotional upset.

Some are habitual aggravations that keep chronic troubles alive.

In milieu therapy we attempt to define and eliminate provocative environmental irritants or to remedy deficits in the living situation that create problems for the person.

Long considered an exclusive province of the social worker and rehabilitation counselor, many psychotherapists are recognizing the importance of dealing with defective environmental contingencies as part of the treatment process.

Broadening the scope of their concerns beyond their usual intrapsychic and interpersonal focus acknowledges that the patient is subject to influences away from the therapist's office that can sabotage the treatment effort.

Perhaps more important are the preventive aspects of a regulated environment that regard such things as nurseries as more than a dumping grounds for the children of working mothers, and hospitals as more than a warehouse for the storage of the mentally disabled.

The writings of Aichorn (1948), Bettelheim and Sylvester (1948), Main (1946), M.

Jones (1952, 1956, 1957, 1959), Stanton and Schwartz (1954), Cumming and Cumming (1962) have documented the growth of the movement outlining the history, rationale, and value of milieu therapy for preventive and therapeutic purposes.

Therapeutic nursery schools, residential treatment centers for children, psychiatric hospitals, halfway houses, daycare centers, rehabilitation centers, nursing homes, and other institutions have attempted to apply principles from education, psychoanalysis, and behavior therapy to the organization and management of therapeutic units (Greenblatt et al, 1955; Cumming & Cumming, 1962; Sherwood, 1975; Kramer & Kramer, 1976; Gunderson et al, 1983; Noshpitz et al, 1984).

Dominating the thinking has been the concept that supplying patients with the benefits of a scientifically organized therapeutic community has, in practice, displayed both benefits and limitations (Greenblatt et al, 1955; Knobloch, 1960, 1973; Rapaport, L, 1962; Karasu et al, 1977; Talbott, 1978; Gunderson, 1980; Oldham & Russakoff, 1982; Gunderson et al, 1983).

Great diversity of design are obviously possible varying with the specific facilities and resources available in a community.

Their efficacy, no matter how ingenious they may seem, is crucially dependent on (1) the dedication and skill of the involved personnel, (2) how completely the environment accords with the demands of the patient, and (3) the singular reactions of the patient to what is being done for and to him or her.

www.freepsychotherapybooks.org 175 No matter how benevolent a controlled environment may seem, the attitudes and reactions of administrative and clinical professionals will determine its therapeutic impact.

Many programs have been wrecked by negative countertransference reactions of the persons in charge.

The treatment and rehabilitative facilities must also be vigorous and varied to fit in with the requirements of different patients.

Unless a program is flexible and possesses many alternative resources, it will not be suitable for all assigned individuals.

Finally, some patients respond adversely to certain environments no matter how therapeutic they may seem.

For example, detached and schizoid patients may not be able to tolerate the overload of stimuli

characteristic of some programs (Van Putten, 1973).

Very often patients are so bound to an existing life situation, because of a sense of loyalty or because of a feeling that they have no right to express demands, that environmental distortions are tolerated as unalterable.

They may be unaware that tension and anxiety are generated by specific conditions, blaming their difficulties on things other than those actually responsible.

This may act as a deterrent to psychotherapy.

The therapist may have to interfere actively with environmental aspects that are grossly inimical to the best interests of the patient.

This may necessitate work with the patient's family, for it is rare that the patient's difficulties are self-limited.

The various family members may require some kind of help before the patient shows a maximal response to therapy (see Family Therapy, Chapter 52).

Social work more than any other discipline has evolved the most complete and best organized system of environmental manipulation (Atkinson et al, 1938; Bruno, 1948; French, LM, 1940; Jewish Board of Guardians, 1944; Lowrey, 1946; Menninger, WC, 1945; Pray, 1945; Ross & Johnson, 1946; Towle, 1946; U.S.

Children's Bureau, 1949; Van Ophuijsen, 1939).

Among services rendered are mediation of financial, housing, work, recreational, rehabilitative, marital, and family problems.

Many of the environmental manipulative techniques elaborated in social work may be employed by the therapist when situational difficulties are so disturbing to the individual that he or she is unable to live with them.

Or the therapist may utilize the services of a social worker as an adjuvant

www.freepsychotherapybooks.org 176 toward this end.

Home treatment is a modality through which inaccessible patients may be reached before a crisis makes hospitalization inevitable (Becker & Goldberg, 1970; Goldberg, HL, 1973).

It is especially applicable to disturbed individuals who are unable or unwilling to seek psychiatric help.

For such people a team effort is best.

That a visiting team sees the patient in the family and customary social setting facilitates diagnosis and treatment planning.

In many cases, the visit promotes greater confidence and trust and elicits far less anxiety than would be the case in a strange setting like a hospital clinic.

The interdisciplinary team with the different backgrounds of its constituent members can improve the quality of the total evaluation.

The interaction of the entire family at the initial home visit permits a better understanding of the dynamics that have led to the upset in the identified patient.

Although home treatment may seem an expensive process, the cost to the community is probably less than maintaining the patient in an institution and providing aftercare.

When several visits have been made, sufficient confidence may be inspired in the patient to be motivated to continue contacts with one or more members of the team in the central clinic.

Family therapy and drug treatment are particularly suitable in home treatment: paranoid patients have benefited from the use of long-acting injectible phenothiazines at intervals of 2 weeks (Goldberg, et al, 1970).

Sometimes the results of minor environmental adjustments are most gratifying, constituting all that the patient will need (Duncan, 1953; Suess, 1958; Murray & Cohen, 1959; Redl, 1959; Ytrehus, 1959; Lander & Schulman, 1960; Stanton, A, 1961; Wilkins et al 1963).

At other times, considerable psychotherapy will be required before the patient is able to take

advantage of improved environmental adjustments.

We should never lose sight of the fact that environmental difficulties, while accentuating the patient's problems, may merely be the precipitating factors.

The basis for the maladjustment exists largely in the personality structure.

As a matter of fact, most people have a tendency to objectify their problems by seeking out conditions in their environment that can justify feelings of upset.

For example, if a person has a problem associated with the fear of being taken advantage of by others, that individual will relate this fear to almost any situation in which
www.freepsychotherapybooks.org 177 the person is involved.

Environmental correction may, therefore, have little effect on the basic difficulty that has been structuralized in such widespread character disturbance that problems in interpersonal relationships appear to perpetuate themselves endlessly.

Indeed, the individual seems to create adversity toward which one is capable of reacting with customary defenses.

More confounding is the desire for martyrdom, a disturbed atmosphere actually being needed.

The individual may, for instance, seek to be victimized by others in order to justify feelings of hostility that could otherwise not be rationalized.

In cases such as this, the correction of environmental stress may produce depression or psycho-physiologic illness due to an internalization of aggression.

One of the most discouraging discoveries to the therapist is that in liberating a patient from a grossly distorted environment, the patient may promptly become involved in another situation equally as bad as the first.

The dynamic need for a disorganizing life circumstance will have to be remedied first before the patient responds adequately to environmental manipulation.

However, there may be no alternative in helping some patients (particularly nonmotivated, psychotic, psychopathic, or mentally defective persons) than to substitute for their habitual setting a radical change in the conditions of life even while they resist psychotherapy.

This may be the only practical means of management.

Even where there is an avoidance of intimate involvement, a milieu that taxes coping capacities minimally and supplies some gratifying experiences may bring about homeostasis and lead to greater self-fulfillment.

Hospitalization will occasionally be needed.

When considering hospitalization for an acutely disturbed, depressed, or psychotic patient, it is important to recognize the disadvantages and advantages of removing the individual from the situation to which that individual will have to adjust on returning home.

Most patients can be adequately cared for with partial hospitalization such as a well run day hospital.

Research studies show that this resource is to be preferred in the majority of cases and yields better results than full www.freepsychotherapybooks.org 178 hospitalization (Herz et al, 1971; Washburn et al, 1976).

However, if the patient is too disturbed or uncooperative, or if suicide is likely, admission to an institution may be advisable for the protection of that person and others.

What must be considered then is whether emotional stabilization is the preferred goal.

Under these instances, a short-term crisis-oriented period of patient care of 1 to 3 months may suffice.

However, where deeper structural personality changes are the objective for persons who have the capacity for such change but who resist altering their behavior, a longer period of hospitalization—1 year or more will be required.

In the latter case, the referring therapist should ascertain that the staff of the hospital

contains professionals trained in approaches that aim for deeper changes. Instead of hospitals, many rehabilitation units exist that may be ideally suited to some patients.

It may be productive for a therapist to visit some of the rehabilitation facilities in the area to see what services they render.

Some patients may be better suited for sheltered workshops than for other programs.

Still others will need halfway houses that act as a bridge to community living.

What is essential is the formulation of a plan both for immediate care and for therapy after the critical situation that necessitates referral has subsided.

Guidelines are suggested in the book by Frances et al, 1984.

A day hospital may, in many cases, serve as a substitute for hospitalization (Budson, 1973).

The cold impersonal institutional setting is replaced by a small intimate familylike atmosphere in which the individual may find a suitable identity.

In a controlled study, investigators at the Washington Heights Community Service of the New York State Psychiatric Institute discovered that "on virtually every measure used to evaluate outcome, there was clear evidence of the superiority of day treatment." Small communitybased residential facilities in the form of halfway houses or neighborhood residences have also been increasing in great numbers for alcoholics, drug addicts, criminal offenders, maladjusted youths, and persons variously handicapped physically and psychiatrically.

These facilities may serve as intermediate stations for the mentally ill patient after institutionalization, as a preliminary step to reentry into community life.

They may also function as a form of continuous aftercare (Axel, 1959; Bierer, 1951, 1959, 1961; Boag, 1960; Cameron, DE, 1947; Cameron, DE, et al; 1958; Carmichael, DM, 1960, 1961; Cosin, www.freepsychotherapybooks.org 179 1955; Craft, 1958, 1959; Fisher, SH, 1958; Goshen, 1959; Harrington & Mayer-Gross, 1959; Harris, A, 1957; Jones, CH, 1961; Kramer, BM, 1960; Kris, EB, 1959; Odenheimer, 1965; Rafferty, 1961; Robertson & Pitt, 1965; Steinman & Hunt, 1961; Winick, 1960; Zwerling & Wilder, 1964).

Obviously, emptying out mental hospitals and throwing sick patients onto the mercies of a community without adequate alternative rehabilitative facilities can be a disaster for both the patients and the community.

What is essential is that the environment to which patients are assigned be organized as a "therapeutic community," which is more easily said than done.

In hospital, day care, and other institutional settings occupational therapy plays a most important role [Fiddler, 1957; Schaefer & Smith, 1958; Subcomm.

Occup.

Therapy (no date on publication); Wittkower & LaTendresse, 1955; Conte, 1962], In this reference it may be possible to regulate the lives of patients so that a group work project becomes part of the daily routine.

The patient's responses and distortions of reaction to the group and work experiences may then be employed for counseling and psychotherapeutic purposes, in this way improving the patient's capacities for self-observation (Greenblatt et al, 1955; Mesnikoff, 1960).

Aftercare management of mental patients may also incorporate rehabilitation house programs (Tyhurst, 1957; Brooks, 1960), night care in a night hospital unit (Harris, A, 1957); neighborhood community centers (Kahn & Perlin, 1964), halfway houses (Williams, DB, 1956; Clark & Cooper, 1960; Wechsler, H, 1960b; Wayne, 1964; Landy & Greenblatt, 1965; Wilder & Caulfield, 1966), sheltered workshops (Black, 1959; Meyer & Borgatta, 1959; Hubbs, 1960; Olshansky, 1960), family care (Muth, 1957; Crutcher, 1959; Ullman & Berkman, 1959; Patton, 1961; Mason & Tarp, 1964), aftercare clinics (Muth, 1957), social clubs (Bierer, 1943; Palmer, MD, 1958; Wechsler, H, 1960b; Fleischl, 1962), and self-help groups, such as Recovery, Inc., Alcoholics Anonymous,

Gamblers Anonymous, and Synanon (Bromberg, 1961; Wayne, 1964).

In rehabilitative planning, the focus may be on the individual's physical work problems or social difficulties rather than on total functioning.

The skills brought into play here often involve a multidisciplinary approach, including methods and techniques from medicine, surgery, psychology, pedagogy, mechanics, sociology, etc.

(Greenblatt & Simon, www.freepsychotherapybooks.org 180 1959; Schwartz, CG, 1953; Bauman & Douthit, 1966; Eustace, 1966).

Some of the beneficial emotional effects of rehabilitation come from the sense of mastery that is restored to the individual when the handicap is overcome for which help was sought.

Programs organized around restoring the individual's social responsiveness help to eliminate isolation.

Recreational activities may constitute the bulk of services rendered in a day care center or in special units such as Fountain House (Fisher, SH, et al, 1960; Goertzel et al, 1960; Fisher & Beard, 1962), The Bridge, and the Social Therapy Club of the Postgraduate Center for Mental Health, in New York City (Fleischl, 1962).

Services may also include vocational training and placement, such as those of the Altro Workshop (Meyer & Borgatta, 1959).

There is much to be said for the development of comprehensive rehabilitation centers that contain a full range of services, including psychotherapy (Gelb, 1960).

The pioneer experiment of Maxwell Jones (1952, 1953, 1956, 1959) in establishing a therapeutic community has illustrated how useful this facility can be.

Since then, many comprehensive units have been erected, organized around social, recreational, work, and educational objectives, with and without formal programs of individual and group therapy (Agrin, 1960; Bennett, LR, 1961; Blackman, 1957; Brace-land, 1957; Caplan, 1961b; Clark, DH, 1971; Cohen, M, 1957; Denber, 1960; Edelson, 1964; Greenblatt, 1961; Irvine & Deery, 1961; Kepinski et al, 1960; Knobloch, 1960; Koltes & Jones, 1957; MacDonald & Daniels, 1956; Main, 1946; Meijering, 1960; Papanek, E, 1956, 1958; Rabiner et al, 1964; Rapaport, RN, 1960, 1961, 1963; Raskin, 1971; Rees, 1957; Siegel, 1964; Stainbrook, 1967; Talbot et al, 1961; Veomans, 1961; Visher & O'Sullivan, 1971; White, RB, et al, 1964; Wilmer, 1958).

An important aspect of the therapeutic community is that it removes patients from the irritants of their customary environment, theoretically giving them an opportunity 24 hours daily to build a new and viable personality.

The hope is that sufficient personality restructuring will occur to carry over into the patient's life situation after leaving the therapeutic facility.

There is some evidence that this can occur without formal psychotherapy.

In British Columbia Knobloch (1973) and a research team set up a www.freepsychotherapybooks.org 181 therapeutic community at Haney patterned after the Lobetch community of Czechoslovakia in which great behavioral changes were produced with dramatic economy in about 15 percent of the time required for an equivalent stay in a hospital.

Fifty-four patients were housed at Haney for 6 weeks and results compared with matched samples of patients treated in a day-care program and as inpatients.

Treatment efficacy was greatest at Haney at a fraction of the cost.

The results were credited to the use of a relatively closed socioecological system with a small group, assignment of work similar to the life work situation, the use of a variety of procedures including play, games, fantasy, psychodrama, kinesthetic therapy, and dramatic and pantomimic exercises, the homogeneity of the patients who all entered voluntarily into the program, and shared leadership.

Knobloch stresses that "this confusion about leadership makes some so-called therapeutic communities in North America lamentable examples of disorganization— social slums with

unnatural reinforcement contingencies which demoralize the patients." Follow-up studies have shown that the patients at Haney retained their low symptomatology.

The regressive impact of institutionalization has been consistently reported, but there is no reason why this has to be so.

There will always be patients who cannot adapt to any facility other than an institution.

The problem is not that institutionalization is antitherapeutic, but that the way institutions are run contributes to the illness and withdrawal tendencies of many patients.

"As long as we continue to view the purpose of institutions as lumber rooms for individuals with deviant behavior, we will continue to foster countertherapeutic atmospheres" (Jones, M, 1973).

It is not enough to load up patients with drugs and assign them to clean quarters.

There is no reason why enlightened administration cannot organize institutions as viable therapeutic communities, with activities to enhance latent growth factors in patients.

Describing one hospital organized around psychotherapeutic lines, Gralnick and D'Elia (1969) explain, "We began to believe we had created a society that had a positive effect on patients because it permitted them to discover, explore, and overcome their sick ways and learn healthier ways of relating to others—ways that became part of them for the rest of their lives." www.freepsychotherapybooks.org 182 We undoubtedly will find in the future that institutions will become less custodial than therapeutic because they will have flexible programs that consume the interests of patients, enabling them better to relate to one another and to the personnel, permitting them to develop new adaptive skills, and broadening their stress tolerance, which may carry over into the extrainstitutional environment.

EXTERNALIZATION OF INTERESTS Anxiety, depression, and excessive concern with symptoms may foster a withdrawal of the individual from interests and diversions that are a healthy component of living.

Where this has occurred, it may be expeditious to encourage such patients, as part of their psychotherapeutic program, to resume activities once meaningful to them or to help them to develop new diversions for leisure time (Davis, JE, 1938, 1945; Martin, AR, 1944, 1951; Slavson, 1946b).

Among the many outlets are sports (such as golf, tennis, swimming, handball, table tennis, boating, and riding), crafts (woodwork, needlecraft, weaving, metal work, and rug making), games (bridge, chess), photography and fine arts (painting, sketching, carving, sculpture, and mosaics), and various other activities (hiking, gardening, collecting—stamps, coins, etc.—music, and dancing).

Countless hobbies and recreations may be exploited where appropriate resources are available. Apart from physical exercise and the creative stimulation and temporary distraction from neurotic concerns, leisure time activities serve to bring the individual into contact with other persons, thus invading the individual's social isolation and exposing him or her to the influences of group dynamics.

They become a kind of bridge to a more adequate contact with reality.

Organized recreational programs may provide a consuming interest for patients that may be symbolically meaningful and perhaps serve as a way of relating themselves to the recreational workers and to their peers (Haun, 1967).

With this as a model they may then learn to act more constructively with other people.

Occupational Therapy Among the best organized activities, universally employed in mental institutions, is www.freepsychotherapybooks.org 183 occupational therapy, which has proved itself a vital means of restoring the individual through the relationship with the occupational therapy worker, the symbolic meaning to the therapy worker of the tools used, and the end products of his or her manipulations, toward a more adequate integration (Am.

Occup.

Therap.

Assn., 1972; Dunton, 1915, 1945; Fiddler, 1957; Haas, 1946; Linn, 1975; Linn et al, 1962; Meyer, 1922; Schaefer & Smith, 1958; Sub-comm.

Occup.

Therapy (no date on publication); Wittkower & La Tendresse, 1955).

Occupational therapy is more than merely recreational and diversionary.

It may be the sole means of entering into the inner world of the withdrawn and regressed patient.

The occupational therapy worker has become an important and respected member of the psychiatric team (Conte, 1962).

As such, the therapy worker must be capable of providing adequate support and understanding for patients whose stability is at best tenuous and who are hostile, defeated, and uncooperative.

At the same time the occupational therapy worker must be able to set limits and to apply adequate controls when necessary.

The ability to detect, understand, and help the patients to work through conflicts as they manifest themselves in the patient's performances and relationships with fellow patients requires a high degree of training, perceptiveness, and stability.

Facilitating these aims are an understanding of both rehabilitation and psychodynamic concepts.

Further objectives are improvement in work, socialization, and activities of daily living.

Accordingly prevocational guidance, work adjustment, self-care, home-making, child care, and other services are offered to patients of all ages in hospitals through sheltered workshops, public and private schools, group homes, correctional institutions, day-care centers, community mental health centers, and in private facilities with special "concern for the complexities of sensorimotor and cognitive skills, personal motivation, self-determination, and adaptation" (Fine, 1984).

Essentially what is attempted is restoration of a sense of mastery that has been eroded by the emotional illness.

Quality assurance is an objective of the American Occupational Therapy Association that has a membership of over 35,000 professionals.

The Association as well as the Committee on Allied Health Education and Accreditation set standards for training and www.freepsychotherapybooks.org 184 practice.

Certification as an occupational therapist requires completion of standard training and full-time field work.

Registration and licensure are necessary for practice in many states.

THE CREATIVE ARTS THERAPIES Among the creative arts therapies, art therapy, music therapy, drama therapy, and dance movement therapy have been developing rapidly as adjuncts to the more traditional psychotherapeutic procedures, in addition to serving as supportive and educational methods in their own right.

Recognizing the usefulness of these modalities especially with the chronically mentally ill and severely disturbed children and adolescents, legislation was formalized during the late 1970s in the Mental Health Systems Act that resulted in new federal job classifications in the creative arts therapies and thus vitalizing these fields of therapeutic activity.

Originally designed for use in hospitals and institutions, these methods are increasingly being utilized with individuals, their families, and other groups in outpatient clinics and even private practice.

Standards of training, qualification, certification, registration, and licensure have been developed by emerging national organizations that are related to each area of the creative therapies, and research and writings have been increasing steadily in the last few years as a

consequence of this ferment.

Art Therapy Art therapy has become more formalized over the past decades and has developed from a casual technique used with children in clinics and with withdrawn patients in mental institutions to a sophisticated vehicle, which in the words of the American Art Therapy Association, is capable of "reconciling emotional conflicts and fostering self-awareness and personal growth." In this way it may serve as a bridge to help an individual "find a more compatible relationship between his inner and outer worlds." Understandably, art therapists trained in different theoretical schools will fashion their methods and interpretations around their special orientations whether these be www.freepsychotherapybooks.org 185 behavioral, Gestalt, humanistic, psychoanalytic, etc.

They will also devise uniquely personal ways of conducting the art session and of relating themselves to a co-therapist or team, if there be one, in an adjunctive collaborative, or supervisory way.

Art therapy may be done individually, in groups, or with families.

With schizophrenic patients, art therapy helps to bring them out of their regressive state and to establish a relationship with a helping individual.

Kwiatkowska (1967) has developed a popular evaluation method for art therapy candidates that designates strengths, weaknesses, and family interactions.

The diagnostic value of children's drawings in learning disabilities has been pointed out and detailed by Levick et al. (1979).

Music Therapy There is an inherent quality in sound that tends to calm or disturb, contingent on its physiologic and psychological effects.

Thus, sudden loud noises may arouse fear and apprehension; coordinated musical resonance may evoke pleasure; rhythmic cadence stimulates motor activity and relieves tensions; dissonant and shrill reverberations promote tension and may actually be experienced as painful.

Sounds influence both cortical and subcortical areas, affecting the autonomic nervous system.

Harmonious and rhythmic tones can arouse emotional feelings, promoting happy, excited, and sad moods.

For this reason, increasing interest has been shown in the use of music in medicine (Gaston, 1968; Licht, 1946; Mathews, 1906; Nordoff & Robbins, 1977; Ruud, 1980; Reese, 1954; Schullian & Schoen, 1948; Stein & Euper, 1974; Walters, L, 1954; Zimney & Weidenfeller, 1978).

A Journal of Music Therapy exists with many interesting articles on the subject.

There is little doubt that music can stimulate, relax and sedate, depending upon past associations and present symbolic significances (Colbert, 1963).

Accordingly, it has been employed in various ways in both psychotic and neurotic patients (Altshuler, 1944; Blaine, 1957; Blair et al, 1960; Folsom, 1963; Gutheil, 1954; Ishiy-ama, 1963; Jenkins, 1955; Joseph & Heimlich, 1959; Masserman, 1954; Muscatenc, 1961; Pierce et al, 1964; Reinkes, 1952; Rogers, 1963; Rose et al, 1959; Simon, B, et al, 1951; Soibelman, 1948; Winick & www.freepsychotherapybooks.org 186 Holt, 1960b; Wortis, 1960).

In some cases comprehensive programs have been organized around music appreciation assemblies, rhythm bands, singing groups, and concert and community singing clubs (Van de Wall, 1936; Soibelman, 1948; Gilliland, 1961, 1962), providing the means for both personal achievement and socialization.

In hospitals, day hospitals, and other settings background music may help release tensions, allay fears, provide an escape from boredom, and encourage teamwork (JAMA, 1956).

Music may also serve as a means of opening up channels of communication among patients (Snell, 1965).

It becomes a stimulus for the verbalization of emotion and a vehicle for the encouragement of

interaction in a group (Heckel et al, 1963; Lucas et al, 1964).

From projection of feelings the individual is helped to assume responsibility for his or her feelings.

Initial comments about the musical composition and its sources are followed by verbalization concerning inner emotional stirrings.

These are at first dissociated from the self, but later are acknowledged as part of the person.

The patient talks increasingly about how different forms of music affect him or her.

In a group setting there is an opportunity to listen to others, to compare feelings, and to identify with members of the audience (Weiss & Margolin, 1953; Sterne, 1955; Shatin & Zimet, 1958).

Transference toward the music therapist and the group members is almost inevitable and provides opportunities for exploration, clarification, and interpretation.

Fultz (1966) contends that music therapy properly employed may serve the following rehabilitative goals: (1) to aid in diagnosis and treatment planning, (2) to establish and cultivate socialization, (3) to promote selfconfidence, (4) to control hyperactivity, (5) to foster the development of skills, (6) to assist in the correction of speech impairment, and (7) to facilitate transition from nonverbal to verbal codification systems.

In this way, music becomes an adjunctive therapeutic agent, and the music therapist who is properly trained may be employed constructively as part of a team in a treatment program.

The education and professionalism of music therapists is constantly being improved and monitored by the National Association of Music Therapy and the American Association of Music Therapy.

Drama Therapy www.freepsychotherapybooks.org 187 Drama therapy includes those "approaches that stress the appreciation of creative theatre as a medium for self-expression and playful group interaction and that base their techniques on improvisation and theatre exercises." (Fink, et al, 1984).

The field has developed during the past 10 years and has led to the organization in 1979 of the National Association of Drama Therapy, which has developed standards of training and competence as well as qualifications for functioning as registered drama therapists.

Theater techniques were introduced into mental health practices by a number of pioneers such as Jacob Moreno, Peter Slade, Winifred Ward, and Jerzy Grotowski, which focused on the inner experiences of the actor rather than the audience, utilizing fantasy and role playing.

Since then, creative theater methods have been introduced into hospitals, prisons, outpatient clinics, and schools and have been found helpful in disturbed and handicapped individuals as well as other populations who are not approachable by the usual psychotherapeutic methods (Jennings, 1974).

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Johnson (1982 a,b) has employed a developmental approach on the theory that a block in development sponsors mental illness, which may be resolved by an external organized environment with definite boundaries, expectations, and rules.

Such a structured environment is provided in drama therapy through specifying the roles of therapist and group members and designating physical and other material arrangements.

The use of movement, sound, and imagery attempt to reconstruct "sensorimotor," "symbolic," and "reflective" stages, such as depicted by Piaget and kindred developmentalists as a means of progressing from preverbal to verbal forms of expression.

Characters chosen for parts in the designed drama represent forces, objects, animals, people, etc., with which or whom the individual is neurotically fused and identified, or who are idealized or conflicted entities.

Acting out of feelings and attitudes is encouraged through a set script or by improvisations

and role playing.

The therapist participates actively, attempting to resolve any impasse and restraint in free expression.

Role playing brings out both stable patterns of behavior and styles of interaction, as well as fleeting undercurrent inhibitions and impulsive outbursts that are both constructive and destructive in their consequences.

What is especially interesting is the www.freepsychotherapybooks.org 188 relationship of the actor to the role that is being played, as well as the interactions with the other actors in the play (McReynolds & DeVoge, 1977) (see also Psychodrama, Chapter 52).

Dance and Movement Therapy Dance and movement therapists work in clinics, psychiatric hospitals, day-care centers, correctional centers, and special schools all over the world on an individual and group basis.

In this way they serve as adjuncts to primary psychotherapists.

Such technical terms as movement empathy, interactional synchrony, and replication refer to how body movements have symbolic meanings and reveal information about inner emotions and mental process.

An individual's movements—posture, carriage, gait, and muscular coordinations— reveal tensions and character traits (Birdwhistell, 1952, 1959).

The way people dance— their body direction, coordination, and use of space—communicates many of their conflicts.

Free dance improvisations often bring forth gestures and movements of special parts of the body.

Sequences of expression, hesitations, and aggressive motions have meaning for the dancer.

Thus, dance may be exploited, not only to secure emotional release, providing an outlet for energy, but also as a way of bringing out attitudes and conflicts.

Solo dance performance and improvisations, folk dancing, and ballroom dancing help externalize feelings and act as a bridge to greater social participation (Rosen, E, 1957).

As in music therapy, the greatest effect on the patient is the interpersonal involvement.

In a dancing relationship the patient has an opportunity to work through some of his or her shyness and embarrassment toward greater assertiveness and selfexpression.

Movement therapists also employ body-awareness techniques on an individual and group basis.

The techniques employed draw from various sources, ranging from Yoga and Tai Chi Chuan to Jacobson's "Progressive Relaxation." The object is not only to induce relaxation but also to promote emotional catharsis, mental and physical alertness, and greater awareness and acceptance of the body (Winston, 1966).

Awareness of muscle tension, of posture and body alignment and of freedom or inhibition in www.freepsychotherapybooks.org 189 breathing and body movements helps to focus attention on the self, its defenses, and its conflicts.

Exercises to improve muscle tone and posture help reduce anxiety, release energy and enhance self-confidence.

Focusing on select muscle groups may release an outpouring of memories and encourage the reexperiencing of affect related to past events.

As the patient is stimulated to move body parts that are held rigid, the patient often begins to work through the fantasies and fears that have sponsored the original immobilizations.

Obviously the relationship with the movement therapist, and the latter's ingenuity and sophistication, will have a determining influence on results (Smallwood, 1974).

Dance and movement therapy was endorsed by the President's Commission on Mental Health for use in federally assisted facilities.

A number of educational programs now are available leading to a master's degree, and the

American Dance Therapy Association coordinates activities in the field.

Also available is a journal, *The American Journal of Dance Therapy*.

Structural Integration A method that has been utilized in recent years is that of structural integration, developed and practiced for many years by Ida Rolf.

It has been given the name of Rolfing (Rolf, 1958, 1973; Sperber et al, 1969).

The object of this therapy, which is usually confined to no more than 10 sessions, is to regulate posture and motor control defects that are said to influence emotional factors adversely.

"Deep message" is systematically applied to different anatomical areas.

Since there is no formal interviewing, therapists need not be psychologically trained.

By producing proper muscle coordination, a psychological integration is said to be obtained, a development attested to by testimonials proffered by many clients who have undergone Rolfing.

The method is organized around a dubious and anatomical electromagnetic theory.

Carrera (1974) has stressed the need to develop a conceptual framework with clear observable referents before one is capable of judging the true value of this technique.

It is quite probable that potent factors in the helping process here are the placebo effect and the impact of the relationship with the therapist.

Poetry Therapy www.freepsychotherapybooks.org 190 A new ancillary technique, poetry therapy had its inception in Greenwich Village, New York, when Eli Greifer, a lawyer and poet, brought together groups of mentally ill persons and recited poems that he later published in many books and pamphlets, including "Philosophic Duels," "Rhymes for the Wretched," "Poems for What Ails You," "Lyrics for the Lovelorn," and "Psychic Ills and Poem-therapy" (Leedy, 1966).

Greifer was aided in his campaign by a number of Village poets, including Maxwell Bodenheim and John Rose Gildea.

Poems, according to Leedy, may incite patients to constructive action as well as contribute to the evolution of a life philosophy.

Various techniques may be employed to secure group participation and interaction, including the recitation by patients of poems from readings or through memorization, and the writing of original poems (Harrower, 1974).

Discussion of the emotional stirrings created by poems and their meaning for the individual serves to bring the individual to greater selfunderstanding.

Various techniques may be employed along with group reading, such as, if the therapist is dynamically oriented, or if acting as a co-therapist to a poetry therapist, the discussion of fantasies and the encouraging of free associations to the poems (Pietropinto, 1975).

The advocates of poetry therapy contend, with compelling examples, that poetry can be an aid in reconstructive therapy; repressed and unconscious feelings are accelerated by the reading, listening to, and writing of poetry.

Thus Arsula Mahlendorf contends that "Harmonious emotion, allows catharsis, reaches into the unconscious by rhythm, rhyme, and imagery, creates coherence, order, and insight into hidden relationships, promotes integration between the conscious and unconscious, and thereby increases selfunderstanding, self-esteem, and mastery." (*Psychiatric News*, Oct. 1, 1982).

A National Association for Poetry Therapy has drawn together therapists interested in this modality.

Social Therapy The protected atmosphere of an institution, clinic, or agency encourages the organization of groups that can participate in a full spectrum of social activities (Bierer, 1943, 1958; Palmer, MB, 1958; Wechsler, H, 1960b).

At the Postgraduate Center for www.freepsychotherapybooks.org 191 Mental Health, in New York City, a program has been in operation for years that offers the individual who is or has been

in psychotherapy expressive opportunities for emotional growth and development (Fleischl, 1962; Fleischl & Waxenberg, 1964; Waxenberg & Fleischl, 1965; Fleischl & Wolf, 1967).

An environment is provided that is favorable for the overcoming of detachment, aggression, and social isolation.

While patients may have gained insight through psychotherapy into their characterologic distortions, they may be unable to overcome these defects in their usual environment that customarily imposes harsh penalties for even minor deviations.

A therapeutic social club fosters real life experiences through interaction with other persons, directly or by means of such creative activities as arts, crafts, games, reading, dramatics, dancing, and music.

Parties, visits to museums, and attendance at plays and outings provide further possibilities for rich experiences.

The influence of group dynamics here cannot be discounted, many deviant patterns altering themselves in the relatively tolerant setting of the club.

Group psychotherapy and vocational rehabilitation are often coordinately employed.

Social therapy is particularly valuable where normal familial relationships, social activities, and work situations produce upsetting and self-defeating reactions in a patient in spite of exposure to psychotherapy.

The organization and variety of activities are limited only by the inventiveness of the staff and the creative planning of the director.

A screening of members is essential to eliminate hallucinating and delusional psychotics, chronic alcoholics, drug addicts, violent and assaultive persons, and criminal psychopaths.

While supportive in its intent, social therapy may become reconstructive in its effect as it enhances self-understanding and furnishes rewards for socially constructive behavior.

Changed attitudes and emotions lead to greater self-realization and capacities for less ambivalent relationships.

REASSURANCE Reassurance is a partner in all psychotherapies, even where there is purposeful avoidance of pacifying consolations.

The very presence of the therapist serves to conciliate the patient, apart from the auxiliary agencies of placebo and suggestion.

This is especially www.freepsychotherapybooks.org 192 the case in severely upset patients who, lacking the capacity to handle their anxiety through their own resources, seek comfort in the shadow of an idealized parental figure.

Coming to therapy, therefore, in itself constitutes for the patient an inspiration that he or she is not hopeless.

Verbalized reassurances are often given the patient in supportive therapy, particularly when doubts are voiced concerning the ability to get well.

The patient is also solaced whenever in the grip of fears conditioned by irrational thinking.

The therapist discusses such fears openly with the patient, offering explanations of how baseless they are, in the hope of diverting the patient from destructive thinking patterns.

The most common misconceptions nurtured by patients are those that are related to a fear of going insane, of being blemished by a hereditary mental taint, of harboring an undetected malignant disease or abnormality, of having sustained irreparable damage through early masturbatory excesses, or of being overwhelmed by murderous impulses or perverse sexual fantasies.

Such fears lead to brooding and self-recrimination.

Where the patient is convinced of the sincerity of the therapist and accepts the therapist's authority at face value, that patient may be helped, by verbal placating, to master some misconceptions.

Reassurance is least successful when it is directed at basic, egosyntonic personality difficulties, particularly devaluated self-esteem and its derivatives.

PRESTIGE SUGGESTION Among the oldest of techniques is prestige suggestion, which is still employed extensively throughout the world by witch doctors, religious healers, and even professional psychotherapists.

Reported results vary from unbounded enthusiasm to a discrediting of the method as an irrational form of influencing.

Because it deals with effects rather than with causes, the method has many limitations; nevertheless, it may in certain cases be the only tactic to which the patient will respond.

www.freepsychotherapybooks.org 193 Suggestion plays a part in every psychotherapeutic relationship even though the therapist seeks to avoid it.

It need not be direct.

A patient came to see me with the complaint that intercourse was getting to be burdensome. Her husband demanded it twice weekly, which in her opinion was too often.

She found it difficult to be aroused, and she had to strain mentally to achieve orgasm.

When she finally climaxed, she was exhausted from the effort, and it took hours to recover.

When I asked her how often she had orgasms, she replied: "Always." "Why must you have orgasms each time?" I asked.

A long pause was followed by, "Maybe I shouldn't." The effects of her restraint were immediately apparent.

She began to stop challenging herself, to relax, and after several weeks to enjoy sexual contacts with and without orgasm.

Generally, patients will tend to select from the content of what the therapist says or implies certain things they want to hear, to which they feel it important to respond.

However, suggestion may be used deliberately by some therapists in the form of directives delivered with authoritative emphasis to influence the patient in calculated ways.

A positive and optimistic bearing on the part of the therapist helps reduce anxiety that the situation is hopeless and encourages motivation to address the tasks of therapy.

Where the therapist occupies an omnipotent position in the mind of the patient, certain symptoms may be dissipated on command.

Symptoms that are removed by prestige suggestion probably disappear because the patient has an unconscious need to obey.

The motivation to comply is usually conditioned by a wish to gratify important security needs through archaic mechanisms of submission to and identification with an omnipotent authority.

So long as this motivation is greater than the gain the patient derives from the indulgence of symptoms, complaints may lessen or be abandoned by ordinance, and relative comfort will prevail, provided that faith in the power and omniscience of the therapist is continued.

Results are best where the symptom has minimal defensive purposes and where the need for symptom-free functioning constitutes a powerful incentive.

Certain physical symptoms and habit disorders, such as nail biting, insomnia, excessive eating, inordinate smoking, and drinking, are sometimes partially susceptible to prestige suggestion, especially when they are incapacitating to the person.

Results are most pronounced where the patient has no other motivation for therapy than to abandon his or her symptoms or to bring them under control.

www.freepsychotherapybooks.org 194 Contrary to prevailing opinion, the banishing of symptoms by suggestion need not be temporary.

Where the environment supports or rewards symptom-free functioning, benefits may persist indefinitely.

Only where the symptom is strongly anxiety binding or where it brings strong secondary gains

will it return or will substitutive symptomatic replacements appear.

Relief or removal of symptoms is often accompanied by a general reorientation in the patient's attitudes (Kraines, 1943).

The removal of a symptom may, then, as a byproduct, have an important effect on the total functioning.

Individuals handicapped by a disturbing symptom often lose self-respect, withdraw from people, and get more and more preoccupied with themselves.

The symptom becomes a chief concern, around which individuals organize their insecurity and inferiority feelings.

Here, the removal of a symptom may alter the person's whole pattern of adjustment.

Minimizing the individual's symptom, if this is at all possible, may start a process of personality growth.

An individual with an hysterical tic may isolate himself or herself because of the embarrassment caused by the symptom.

Abolishing the tic can influence the individual's general adjustment materially.

A patient with a paralytic limb of psychological origin may be restored to economic usefulness, and that patient may benefit emotionally to a point of satisfactory social rehabilitation.

Autosuggestion is regarded by many as a form of prestige suggestion directed at the self.

Here the individuals employ their internalized authoritative image as a dissociated surrogate who delivers commands to themselves.

It has been claimed by some that autosuggestion is one of the most powerful forces at the disposal of the person (Coub, 1936).

Thus, organs may sometimes be restored by self-suggestion to proper functioning and normal emotional attitudes may be vitalized.

These claims are, of course, open to challenge.

Among the techniques employed to reinforce prestige suggestion, hypnosis is paramount (Wolberg, LR, 1948).

The peculiar powers vested in the hypnotist make the subject absorb suggestions with greater readiness than in waking life.

But even with www.freepsychotherapybooks.org 195 hypnosis as a reinforcing agent, the permanence of suggestions may be limited where symptoms serve important coping purposes or where strong conditionings have been organized, as in certain phobias.

No symptom will be readily abandoned that neutralizes intense anxiety or possesses powerful pleasure values.

Another limitation of prestige suggestion is that a sizable group of patients fail to respond to symptom removal because they are unable to conceive of any authority as infallible (Wolberg, LR, 1962).

Where there is doubt as to the capacities and powers of the therapist, the patient will have limited motivation to comply, and may successfully resist the therapist's commands.

There are, nevertheless, occasional patients whose need for an invincible and protective authority is so strong that they will faithfully follow suggestions, even to the yielding of an important symptom that serves the psychic economy.

To compensate for this loss, in cases where there is a psychological need for the symptoms, other symptoms may develop that have the same significance to the patient as the original symptom, but are perhaps less incapacitating.

The relapse rate among disabled patients who have had their symptoms removed purely by suggestion is understandably substantial.

Needless to say, the chances of helping a patient permanently are much greater where one does a reintegrative kind of therapy that treats the problem source.

The concomitant rebuilding of ego strength and inner security gives the person the best chance of remaining symptom-free, even in the face of a disturbing precipitant.

In suggestive treatment, where no change has developed in the intrapsychic structure, there is always the possibility of a relapse.

This, however, is not inevitable; the patient's life situation may get less complicated, or the patient may, as a result of therapy, develop more adaptive ways of dealing with conflict and of getting along with people.

Here the loss of symptoms may be permanent.

Another objection that has been voiced to suggestive therapy, which is not entirely valid, is that it is apt to eliminate an important motivation for deeper treatment.

It is said that the inconvenience and discomfort of symptoms incites the individual to want to www.freepsychotherapybooks.org 196 inquire into their source.

If made too comfortable by removing symptoms, incentive may be lost.

Where the goal in therapy is to achieve reconstruction in the dynamic structure of the personality, suggestion or any other strongly supportive therapeutic method may thus act as a deterrent.

This contingency is balanced by the paralyzing effect of strong symptoms on faith, hope, and trust.

Modulating symptoms may actually be essential in keeping many patients in therapy since they will be prone to regard treatment as an unrewarding venture if they have no immediate proof regarding its efficacy toward reducing their suffering.

In spite of its limitations and disadvantages, there are instances when prestige suggestion has to be resorted to as an expedient.

A symptom may be so disabling and may cause such great distress that all psychotherapeutic efforts will be blocked until the person obtains some relief.

Furthermore, deficient motivation and minimal ego strength will destine to failure psychotherapeutic methods that are aimed at increasing selfknowledge.

The most that can be hoped for here is that the patient will respond to palliative or supportive devices, such as prestige suggestion.

As therapy succeeds, however, incentives for a more extensive treatment approach may evolve.

PRESSURE AND COERCION Pressure and coercion are authoritative measures that are calculated to bring to bear on the patient rewards or punishments that will stimulate the patient toward fruitful actions.

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Thorne (1950) has indicated that coercive measures are useful in some dependent personalities who refuse to face life under any other circumstances than to be forced to comply.

Injunctions may be of value for immature individuals who tend to act out their problems, for persons who habitually shy away from reality, and for those who refuse to take resolute actions by themselves and manifest discouraging, indecisive wavering.

In emergencies where the individual is endangering himself, or herself or others, and in uncontrolled emotionality where other methods fail, coercive devices may be mandatory.

Therapeutic pressure may be exerted in the form of assigned pursuits.

Thus www.freepsychotherapybooks.org 197 Herzberg (1945) advised that the patient be given tasks that are directed against (1) impulses that maintain the neurosis, attempting to remove them or lower their intensity below the critical threshold, (2) obstacles toward satisfaction of impulses, (3) neurotic gains, and (4) "delaying factors," which operate to prevent essential fulfillments.

Threats, prohibitions, exhortations, reproaches, and authoritative firmness lend weight to

repressive defenses.

Because such measures may reduplicate the disciplinary strictures of the parent-child relationship, the patient is apt to react to them with hostility, obstinacy, masochistic self punishment, and other responses characteristic of the ways the patient had adjusted to parental commands.

It is rare that a permanently good therapeutic effect issues out of the use of such authoritative procedures since the patient will resent being treated like a child, and will ultimately tend to defy the therapist, even to the point of leaving therapy.

Accordingly, pressure and coercion, if they are ever used, should be employed only as temporary instrumentalities in critical situations.

PERSUASION Playing an authoritarian role, the therapist may serve as a mentor to persuade the patient to revise values and life philosophies.

The object is to change the significance of habitual attitudes against which the patient is rebelling and to provide new goals and modes of adapting to reality.

The persuasive technique is based upon the belief that the patients have within themselves the power to modify their pathologic emotional processes by force of sheer will or by the utilization of common sense.

Appeals are made to the patients' reason and intelligence in order to convince them to abandon neurotic aims and symptoms and to help them gain self-respect.

They are enlightened as to the false nature of their own concepts regarding their illness, as well as the maladaptive habits they have formed.

By presenting them with all the facts in their case, an attempt is made to show them that there is no reason for them to be ill.

They are urged to ignore their symptoms by assuming a stoical attitude, by cultivating a new outlook aimed at facing their weaknesses, and by adopting an attitude of self-tolerance.

In this way they are brought into harmony with www.freepsychotherapybooks.org 198 their environment, and induced to think of the welfare of others.

A number of psychotherapists, in utilizing persuasion, strive to indoctrinate their patients with their own ideologies.

The therapist establishes a directive relationship with the patient, who, in turn, seeks approval by acknowledging that the therapeutic authority must know what is best for the patient.

The approach is a somewhat more mature one than that of guidance, since it presupposes active participation of the patient in the cure and aims for an expansion of personal powers and resources.

The majority of popular books on mental therapy are modified forms of persuasion.

The use of persuasion was first advocated by Paul DuBois (1909, 1911) of Switzerland, who held conversations with his patients and taught them a philosophy of life whereby they substituted in their minds thoughts of health for their customary preoccupations with disease and suffering.

Much of the success that DuBois achieved by his persuasive methods was due to his own vigorous personality, which exuded confidence and cheer.

DuBois recognized the importance of the interpersonal relationship, and he insisted that the physician treat the patient not merely as an interesting case, but as a friend.

He declared that the doctor must be inspired by a real sense of sympathy and affection for the patient and should manifest these sentiments so openly that the patient "would really be very ungrateful not to get well." The physician must be sincere in this conviction that the patient would get well because if he had any doubts, he could not help imparting them to the patient.

The aim of mental persuasion, according to DuBois, was to build up in the patient a feeling of

self-confidence, to make the patient his or her own master.

This was done by imbuing the patient with a belief in himself or herself by “education of the will, or, more exactly the reason.” The physician was enjoined to hammer the truth into the patient’s mind with the ardor of a barrister convinced of the justice of his or her plea.

In order to approach the patient’s problem rationally, it was first necessary to

www.freepsychotherapybooks.org 199 understand clearly the nature and sources of the disorder.

The therapist had to distinguish those symptoms of a physical nature from those of psychic origin.

The analysis with the patient of the symptoms, and the understanding by the patient of how these symptoms were debilitating and inconvenient were important, particularly because they made the person feel that the therapist was interested in him or her and was sympathetic to his or her suffering.

The patient had to be shown how one utilized the symptoms to escape responsibilities in life.

The patient had to be convinced that nervousness had crushed morale, that, even though that patient believed the trouble to be physical, it was really mental.

Urged to “chase his troubles from his mind.” The individual was promised that discomforts would vanish.

No medicine was needed, DuBois insisted, “for there is none to turn a pessimist into an optimist.” DuBois recommended prolonged discussions, during which it was necessary to convince the patient of his or her errors in reasoning.

The patient had to be shown that the symptoms were the product of emotional stress.

Though annoying, they were not serious in themselves.

The less one concentrated on symptoms, the less disturbing these would become.

If the heart palpitated, let it pound; if the intestines were active, let them grumble.

If one had insomnia, that person had best say, “If I sleep, all the better; if I don’t sleep, no matter, “Undue attention aggravated the difficulty.

The best way to overcome symptoms was to stop thinking about them.

Fatigue, tension, and fear were all exaggerated by attention.

It was necessary to stop thinking of pain and suffering and to dismiss petty ailments with a smile.

“The proper philosophy,” he said, “easily learned, can restore the mental balance.” DuBois contended that healthy people disregarded their bodily sensations.

The emotionally upset person, on the other hand, concentrated on them until they became the chief preoccupation.

For this reason one was upset by improper thinking habits.

Notions of happiness and health must then replace ideas of disease and suffering.

Happiness depended less on external circumstances than upon one’s inner state of mind.

One might be ill, or have some financial misfortunes, or have lost dear friends, but the intensity of suffering depended upon the spirit with which one accepted these calamities.

www.freepsychotherapybooks.org 200 The education of the self was the first step in securing real happiness, cultivating the belief that one would without question get well.

So long as the patient was convinced that pain, fatigue, or other symptoms were inevitable, they would be felt vividly.

If baneful thoughts were brought under control, problems would eventually be solved.

Every sign of progress was to be held up to the patient and even exaggerated.

Improvement was to be stressed as proof of the patient’s tenacity to get well.

As soon as ideas of health entered the mind, ideas of disease would vanish.

The patient was to be shown that he or she was not the only one with this trouble, that everyone had difficulties that varied only in their manifestations.

Concern with symptoms was understandable but the problem was deeper, involving basic convictions, alteration of which held forth the greatest chances for cure.

Above all, hopelessness about the outcome must be banished even though it required a long time to experience improvement.

And after cure had come about, one might anticipate a relapse if improper thinking habits returned; however, these would be easier to combat since formulas of bearing discomforts cheerfully and of leading a bold and active life had already been learned.

Soon confidence in one's own powers of resistance would be developed.

A questioning of the patient about standards was important.

False views were to be criticized, while logical viewpoints were to be encouraged.

An effort was to be made to discover qualities of superiority that would elevate the patient's stature.

Toward this end an optimistic inventory of one's good qualities was essential.

If difficulties had been brought about by tragic events, reassurance and sympathy were in order.

If irritability and emotional instability were pressing, the patient had to be taught the spirit of forbearance.

Therapeutic efforts were not confined to the patient, but also could be extended to those with whom one lived.

Among proper philosophic ideas to be imbibed were moral notions that could guide one's life and make for good relationships with others.

The best way to forget oneself was to devote more thought to other people.

The best road to happiness was altruism, a dedication toward making others happy.

Tolerance, sympathy, and kindness were the www.freepsychotherapybooks.org 201 keynotes of a serene life.

Religious sentiments were to be awakened and turned to good account.

Dejerine (1913), using the methods of DuBois, also emphasized the "reeducation of the reason." but he stressed emotion rather than the weakened will as the basis for neurosis.

He speculated that the emotions under certain conditions might overwhelm the intellect and cause illness.

Dejerine believed that therapy must, therefore, aim for a liberation of the personality from the effects of harmful emotions.

The emphasis in therapy was to get the patient to talk about traumatic incidents in his or her life, especially fears and sorrows in the present.

Unlike DuBois, Dejerine did not try to impose his philosophy; rather he worked toward developing an emotional relationship with the patient until a state of confidence developed.

When this was obtained, Dejerine practiced persuasion to encourage the patient to correct the "bad habits." He contended that to cure nervous illness one had to fight the deceptive systems of "monism, fatalism, skepticism, and determinism." Reason could overcome obsession once emotions were given a proper outlet.

It was necessary to keep salubrious ideas before the mind; to think thoughts of the noble, the just, and the beautiful; to learn to gain satisfactions by the fulfillment of duty, yet the brain had "always to be guided by the heart." Modern persuasive methods, even though coached in "scientific terms" and borrowing from psychoanalytic lore, draw largely for their inspiration on the works of DuBois and Dejerine.

Stress is laid on cultivation of the proper mental attitudes, on the facing of adversity, on the accepting of environmental hardships, and the tolerance of selflimitations.

Accenting existing assets, the patient is encouraged to expand positive personality qualities, to control overemotionality, to live with anxiety, to accept and endure deprivation,

frustration, and tension while acquiring proper controls for them.

Cognitive therapy and cognitive behavioral therapy employ some of the formulations and methods of old-time persuasive methods.

Psychocybernetics a term coined by Maxwell Maltz (1960), purports to change the self-image through a "teleological approach" designed after the "goal-oriented behavior of www.freepsychotherapybooks.org 202 mechanical systems." The approach is essentially persuasion coupled with relaxation exercises.

Change is best achieved by resourceful "experiencing." Results through diligent practice are said to be obtained in about 3 weeks.

Basic to the philosophy of psychocybernetics is that there is a creative guidance system in all people that can be used as a "Success Mechanism" rather than a "Failure Mechanism." New habits of thinking, imagining, and remembering are essential and obtainable through the built-in "servomechanism" of the brain and nervous system.

The reader is enjoined toward thinking about an attainable goal.

Mistakes and errors are to be expected as in all mechanical systems and should encourage a change of one's course.

Force through too much conscious effort is unnecessary—" 'Let it' work, rather than 'make it' work." One must employ one's imagination for 30 minutes daily, alone and undisturbed, relaxing, closing the eyes and picturing scenes of "acting and reacting appropriately, successfully, ideally." This practice usually later translates itself into "acting differently without trying" in real test situations.

Utilizing relaxation exercises also while visualizing a relaxed scene for at least 30 minutes daily, one may "dehypnotize himself" from the delusion that one is inferior or superior.

There is no reason why one cannot utilize one's God-given power of reasoning to change negative beliefs and behavior, even those unconsciously determined.

Errors that one makes in the present, and past mistakes, should be deliberately pushed out of the mind; successes are to be remembered and thought about.

Causes of remorse should be examined and their absurdity emphasized.

Foolish thoughts or feelings must be banished and rejected.

Wrestling with false beliefs and substituting reason is vital to adjustment.

Even during one's daily activities one will find time, though it be a few minutes lounging in a chair, to relax and remember briefly in detail the sensations of relaxation that one experienced during the 30-minute practice period.

One may repeat to oneself, "I feel more and more relaxed." One may acquire the habit of happiness by thinking pleasant thoughts.

Each morning, perhaps while tying one's shoes, one may give on-self suggestions to start the day with optimism, to act more cheerful and friendly, to anticipate success.

It is essential to accept oneself as is, tolerate one's imperfections and shortcomings while striving for a goal of self-betterment.

Maltz presents formulas of how to overcome failure, manage loneliness, overcome personality blemishes, and achieve www.freepsychotherapybooks.org 203 peace of mind.

Criticizing persuasion, we may say that most forms of persuasive therapy are, at best, very superficial and are often based on the acceptance by patients of banalities uttered by the therapist, who utilizes aphorisms and examples from the lives of the great to reinforce ideas that are scientifically unsound.

The dynamic basis for many persuasive cures lies in the repression of symptoms by appealing to the patients' sentiments of patriotism, family pride, altruism, and self-respect.

The therapist builds up in patients a desire to get well in order to indulge pleasures inherent in being healthy and sociable.

Patients are constantly reminded that if they regard themselves as better persons, others too will have a better opinion of them.

Their duties and responsibilities to get well are continuously emphasized.

Persuasive therapy, nevertheless, serves a purpose in that it provides certain people with a mental crutch where a psychologic analysis of their problem is impossible.

The substitution of persuasive philosophic precepts for destructive habit patterns is probably the lesser of two evils.

Some obsessive-compulsive personalities do remarkably well with persuasive methods.

Indeed, they respond better to persuasion than to psychoanalysis.

The greatest fallacy in persuasive therapy lies in the exaggerated value attached to the reasoning powers as potentially capable of diverting inner emotional processes.

There is, furthermore, an assumption that the patient is conscious of basic defects and is therefore capable of mastering them through concentrative effort.

Since unconscious conflicts and emotions are most important determinants of neurotic behavior, this explains why reason, knowledge, and will power often fail to bring about the mastery of symptoms.

The same effort should, therefore, be made in persuasion as in guidance where these are ineffective to bring about a change in the relationship with the therapist and to motivate the patient to work toward resolution of inner conflicts.

CONFESSION AND VENTILATION Confession, "talking things out," and "getting things off one's chest," in relation to a www.freepsychotherapybooks.org 204 friend or a professional person, such as a physician, minister, or teacher, are common methods of relieving emotional tension. Beneficial effects are due to the release of pent-up feelings and emotions and the subjection of inner painful elements to objective reappraisal.

The mere verbalization of aspects of the self of which the individual is ashamed or fearful helps to develop a more constructive attitude toward them.

Ventilation by the person of fears, hopes, ambitions, and demands often gives relief, particularly when the verbalizations are subjected to the uncritical and sympathetic appraisal of the listener.

Hitherto, the patient has covered up memories, conflicts, and impulses that could not be countenanced or admitted to others.

A growing confidence in the therapist secures an ally to help the patient tolerate and reveal inner secrets.

The ability to share troubles with a sympathetic and understanding person robs them of their frightening quality.

In addition, the patient may find that one's judgment as to the viciousness of one's experiences is distorted.

The very act of translating inner feelings into words helps to restore mastery.

The fact that revelation of shortcomings has not resulted in rejection by the therapist, encourages increasing self examination.

Many of the patient's disturbing concerns have their origin in fantasies or misinterpretations of early childhood.

Verbalization of faulty ideas gives the therapist an opportunity to correct misconceptions hitherto accepted without question.

The patient may need clarification on phases of life relating to the physical functions or interpersonal relationships.

Often one's preoccupations are interpreted as unique to oneself.

When assurance is gained that this is not so, that many impulses and needs are more or less universal, and that it is one's attitudes toward them that are faulty, a great deal of tension

may lift.

Discussion of the patient's problem is continued until there is no longer an emotional reaction to it.

Repeated verbalization of unpleasant and disagreeable attitudes and experiences permits the patient to face past fears and conflicts with diminished inner turmoil.

Many of the benefits that come about from confession and ventilation are contingent www.freepsychotherapybooks.org 205 on the fact that the patient becomes desensitized to those situations and conflicts that are disturbing but which reality demands endurance.

The tolerance of pain, disappointment, and frustration is inordinately low in neurotic persons, and it is necessary to build up the ability to deal with difficulties and painful experiences without collapse.

Because the individual conceives of certain memories, feelings, attitudes, and impulses as damaging to himself or herself and others, the individual tends to repudiate them.

Particularly traumatic are past sexual incidents, perverse fantasies, hostile strivings, and attitudes that brand the person as inferior, evil, or contemptible.

Some of these elements are fully known to the person.

Others are so frightening that they have been at least partially shunted out of awareness by the mechanism of repression.

The pathologic consequences of suppression and repression are legion.

The individual overreacts to incidents that threaten to bring the hidden material to awareness.

Symptoms may be elaborated, such as phobias, compulsions, and hysterical manifestations, that give vicarious expression to the repressed while shielding its direct manifestations.

Only by facing the disturbing experiences and forbidden impulses, by dissociating them from past misinterpretations, and by reevaluating them in the light of present-day reality is it possible for the person to gain true relief.

Methods of desensitization during therapy will vary with the extent of repression.

Conscious conflicts may be handled, as has been indicated, by open and unrestrained discussion.

Less conscious material, however, will require techniques discussed under reeducative and reconstruction therapy.

Hypnosis may be helpful as an adjunct in encouraging ventilation of feelings (Wolberg, LR, 1948).

Hypnotic drugs by intravenous injection have also been employed as a means of facilitating catharsis and desensitization.

This method, termed narcoanalysis by Horsley (1943) and narcosynthesis by Grinker and Spiegel (1945), produces a hypnotic-like state that resembles, but is not similar to, hypnosis.

Barbiturates, especially sodium amytal and sodium pentothal, are the most commonly used substances.

With hypnotic drugs the patient may find relief in a relatively short time divulging material that would have required prolonged interviewing.

www.freepsychotherapybooks.org 206 Repressed conflicts and traumatic memories are released, generally producing temporary palliation.

For this reason, narcosynthesis is particularly applicable to the acute neuroses of war, especially in dealing with functional amnesic states and conversion symptoms.

The released material is worked through in a waking state in the hope of insuring more permanent results.

Where this happens, the techniques employed are no longer supportive; rather, they embrace reeducative or reconstructive stratagems.

Narcosynthesis is not so effective for conflicts that date back in time, even though considerable emotional catharsis may be achieved.

What is required is a rather long-term working through of the accretions of defense that serve in the interests of repression.

One may classify under emotional catharsis the procedure that has been named "dianetic processing." Beneficial effects, if any, of dianetic processing are probably the product of a cathartic effect with the recounting, remembering, and reliving of past traumatic experiences or fantasies.

Originally introduced by L.

Ron Hubbard (1950), an American engineer, dianetics has been elaborated into a "church of Scientology" and has attracted a large group of "auditors" who practice "auditing." According to the theory of dianetics, painful, traumatic memories and experiences are recorded in the subconscious mind as "engrams," which invade the conscious mind and produce a variety of emotional ailments.

During dianetic processing restoration of the memory of subconscious experiences is said to eliminate the effect of engrams.

Search for engrams is often aided by a psychogalvanometer.

The subject, reclining on a couch, voluntarily associates freely, probing past incidents that have a painful import, this is called "taking the patient back along the time track." In the accepting atmosphere provided by the auditor, the patient often finds himself or herself dealing with material in the early past or speculating about past in an elaborate system of fantasies.

Although these fantasies are claimed to be actual memories, there is little question that a recounting of experiences prior to the age of two, and particularly in the prenatal stage, draws upon the vivid imagination of the person.

The subject is encouraged to "relive" painful past events and to reexperience sensations in the same form that the subject had them originally.

The determining effect of prenatal www.freepsychotherapybooks.org 207 impressions is an important aspect of dianetic theory.

Repetition of past traumatic events or fantasies is said to "take the charge out of an engram."

With its exaggerated claims, pseudoscientific theory, and theatrical methods many authorities contend, dianetics and Scientology do not belong among the accredited scientific therapies.

The method of confession has certain serious limitations.

Inasmuch as the most important sources of conflict are often unconscious, it is impossible to verbalize the basic sources of some anxieties.

Nevertheless, there are many conscious conflicts that plague a person, ventilation of which may have a beneficial effect.

The ability to express fearful memories helps to rebuild self-respect and neutralizes the damaging effects of guilt.

Furthermore, the very process of exploring incidents in one's past may lead to a more intelligent approach to the problems plaguing the person in the present.

Delving into controversial courses of action opens up possibilities that do not seem apparent at the moment.

Thus, Schwitzgebel (1961), experimenting with a "street-corner group" of delinquent youths, took advantage of their need for money by offering the boys a chance to talk to him directly and into tape recorders for a fee.

The boys, accepting the invitation as an easy way to gain some extra cash, verbalized their ideas and recounted their experiences with greater or lesser vehemence.

This process had a surprising effect on the participants.

Without their realizing it the youngsters gained a clearer picture of themselves and a greater sense of control that cut into and reduced their delinquent patterns.

In another study with nurses in training it was determined that engaging merely in a discussion group during the stress period of entering nursing training lowered the dropout rate from 24 percent to 6 percent (Rosenberg & Fuller, 1955).

Actually, confession and ventilation are ingredients of all psychotherapies and therefore need not be regarded as a special system.

SOMATIC THERAPIES While they cannot be considered forms of psychotherapy, the somatic therapies are useful adjuncts.

The studies of Sakel (1938) on insulin coma, of Meduna (1950) on convulsive therapy with cardiazol, of Cerletti and Bini (1938) on electroshock, and of www.freepsychotherapybooks.org 208 Delay and Hart (1952), and Delay and Deniker (1960, 1961) on chlorpromazine introduced a new era into uses of the somatic treatments (Kalinowsky & Hoch, 1961; Paterson, 1963; Sargant & Slater, 1963; Ruesch et al, 1964).

Through somatic measures mental ailments have yielded to a point where an adequate social adjustment for many patients previously considered hopeless has become possible.

Patients inaccessible to psychotherapy have also, because they have become more stable emotionally, been brought into reeducative and reconstructive treatment relationships that have proven to be productive.

Each year new chemical substances and special devices are introduced that hold forth revolutionary promises, but which need conservative testing over a period of time before they can be accepted as reliable and safe modalities in the armamentarium of the therapist.

Somatic therapies are no substitutes for psychotherapy; their effects are adjunctive and complementary to the influence, where it is indicated, of psychotherapy.

Their benefits must be balanced off against the side effects they produce.

PSYCHOACTIVE DRUGS Psychopharmacology essentially involves study of the action of drugs on neurotransmitters that regulate synaptic transmission in the brain.

Neurons that secrete norepinephrine, serotonin and dopamine, though relatively small quantitatively, greatly influence emotional behavior and reactions to stressful stimuli.

In recent years, there has been work on opioid peptides especially the enkephalins that are also involved in the control of stress reactions and pain.

Neurotransmitters are liberated into the synaptic clefts to foster nerve transmission.

They are then inactivated by reuptake back into the nerve terminals and broken down by enzymes such as monoamine oxidase.

According to the current hypothesis, some forms of depression, namely, the endogenous depressions, are produced by a relative diminution of the neurotransmitters norepinephrine, and serotonin in the synapses.

The antidepressant drugs are believed to enhance the synaptic actions of the neurotransmitters, the tricyclic drugs by inhibiting the reuptake process, the monoamine inhibitors by blocking the action of the enzyme monoamine oxidase thus increasing the concentration of the neurotransmitters.

Some www.freepsychotherapybooks.org 209 authorities believe that electroconvulsive therapy (ECT) similarly acts by enhancing the synaptic actions of norepinephrine.

It is believed that some of the symptoms of schizophrenia are due to overactivity of dopamine transmission, and there is evidence that neuroleptics such as the phenothiazines and butyrophenones exert their antipsychotic effects by blocking dopamine receptors.

The benzodiazepine anti-anxiety drugs are believed to act by facilitating the synaptic inhibitory actions of the neurotransmitter GABA.

Pharmacotherapy has advanced in the past decade to a point where it may be considered the primary treatment in some disorders and an important adjunctive supplement in others.

This does not preclude the concurrent use of psychosocial approaches that deal with behavioral,

interpersonal, intrapsychic, and social dimensions that are implicated in the illness. In the main, drugs are utilized to influence and rectify biochemical disturbances in the brain and nervous system, in order to suppress a variety of pathological symptoms, such as hyperactivity, agitation, excitement, violent rage, listlessness, social withdrawal, thinking disturbances (including hallucinations and delusions), depression, tension, and anxiety. Drug therapy has revolutionized the treatment of psychotic patients, dramatically reducing hospitalization, and making many disturbed individuals more amenable to treatment. It has changed the outlook for persons suffering from depression, reducing the suicide rate and halting the prolonged suffering characteristic of this condition. It has enabled many patients to make a more adequate social adjustment that would otherwise be impossible.

It has also proved its usefulness in the less serious emotional ailments by modulating anxiety and reducing symptoms that interfere with psychotherapy, thus helping in psychological exploration and working through.

The fear expressed by some professionals, especially those practicing reconstructive therapy, that an anomalous guidance-supportive element is introduced into the relationship by administering medications has not proven itself to be a valid contraindication.

On the contrary, there is growing evidence that the prescription of essential medicaments tends to enhance the working relationship, making the patient www.freepsychotherapybooks.org 210 more cooperative in therapy.

As with any other intervention, some patients may exhibit toward drugs psychological reactions in the form of neurotic defenses and disturbed personality responses.

Such reactions may reveal important aspects of the personality that productively may be examined and analyzed as part of the psychotherapeutic operation.

In reconstructive therapy this can provide grist for the analytic mill.

By their action on the brain, psychoactive drugs influence such provinces as perception, discrimination, conditioning, reasoning, learning, conflict, and motor behavior.

Drug effects depend on their depressant or stimulant impact on neural masses.

Thus, the barbiturates depress the neocortex, reticular formation, limbic system, and hypothalamus while stimulating the thalamus.

The substituted alkenediols (meprobamate) depress the thalamus and limbic system.

The Rauwolfia derivatives (reserpine) stimulate the reticular formation and limbic system, depress the sympathetic mechanisms of the hypothalamus (at the same time that they activate parasympathetic mechanisms), and deplete cerebral amines (serotonin, norepinephrine).

The phenothiazines depress the reticular formation and sympathetic mechanisms of the hypothalamus, stimulate the thalamus and limbic system and have antiadrenergic properties.

Table 9-1 illustrates some of the presumed effects of different drugs.

The use of psychoactive drugs to influence "target symptoms" is still in an expanding stage, although sufficient data have accumulated to indicate that drugs constitute an important, perhaps vital, adjunct in the management of emotionally ill persons.

More or less, drugs are employed on an empirical basis, the mechanisms by which they exert their beneficial effects being only partially clear.

They are generally divided into neuroleptic, anxiolytic, antidepressant, psychostimulant, and psychodysleptic groupings.

The neuroleptics have a calming effect in tension and anxiety as well as a controlling (antipsychotic) influence in schizophrenic and organic psychosis, producing what has been referred to as a "chemical lobotomy." The neuroleptics (phenothiazines, Rauwolfia derivatives, thioxanthenes, tricyclic antipsychotics, butyrophenones; dihydroindolones) suppress conditioned avoidance behavior and reduce aggressive activities in animals.

www.freepsychotherapybooks.org 211 Anxiolytics (meprobamate, diazepam, chlordiazepoxide, etc.) have an effect on mild to moderate anxiety.

Antidepressants (tricyclics, MAO inhibitors) reduce psychomotor retardation, whereas psychostimulants (amphetamines, methylphenidate, oxazolidine, theionized caffeine) increase alertness and enhance physical and mental activity.

Psychotomimetics (mescaline, psilocybin, [lysergic acid diethylamine] LSD) act as toxins to nerve tissue, inducing "model psychoses." Table 9-2 outlines the therapeutic uses of the more popular drugs.

Paradoxically, drugs do not affect all persons the same way.

Individuals vary in the constitutional sensitivity (elaborateness of neural circuits?) of their nervous systems and in their chemical structure (enzyme systems?).

It is to be expected that there will be varying responses to the array of substances that are available in the drug market.

This is borne out clinically by the highly selective reactions that all individuals display toward drugs.

Thus, some persons respond better to Ativan than to Valium and vice versa.

Some cannot tolerate Thorazine, yet do well with Mellaril.

Eysenck (1957) has posed the interesting idea that persons with excitatory and inhibitory personality dispositions behave differently with drugs not only in terms of speed of reaction, but also in strength of response.

The current status of one's metabolism ("law of initial values," Wilder, 1958) also influences how one reacts; thus a drug may have a pronounced effect at one time and a minimal effect at another.

One of the most important of intervening variables is the placebo factor, faith in and anticipated reactions to the drug determining the quality of response, even to a suggested action being diametrically opposite to the true chemical reaction.

Fluctuations in the environment of various kinds also register significantly on drug reactivity.

Perhaps even more important is the relationship with the individual administering the drug.

Psychostimulants that are clinically useful include the sympathomimetic amines.

Amphetamine and its derivatives (Benzedrine, Dexedrine, and Desoxyn) are employed to combat fatigability and lack of interest and drive (Myerson, 1936).

In doses of from 5 to 10 milligrams twice daily, amphetamine is used to relieve mild depression to produce a sense of well-being and vitality in patients who complain of lack of energy and a sense of www.freepsychotherapybooks.org 212 exhaustion.

It is used also temporarily in certain cases of alcoholism and drug addiction.

In children its effect is extraordinary in that hyperactivity and excitability are reduced in aggressive and disturbed youngsters (Bradley, 1950) and those suffering from attention deficit disorders.

This reaction is also observed in excited schizophrenics, helping to reduce the need for sedatives and hypnotics (Bischoff, 1951).

Great caution must be exercised in prescribing amphetamine drugs since more than temporary use may result in addiction and where large doses are employed, a paranoid psychosis may be produced (Connell, 1958; Leake, 1958).

These drugs have been placed on the list of compounds whose prescription is controlled by the federal government.

Among the antidepressants, imipramine (Tofranil) and its derivative, desipramine (Norpramin, Petrofrane), amitriptyline (Elavil), doxepin (Sinequan), protriptyline (Vivactil) and its derivative, nortriptyline (Aventyl), doxepin (Sinequan), amoxepin (Asendin), and trazodone

(Desyrel) are most commonly employed, and to a lesser extent the monoamine oxidase inhibitors (tranylcypromine [Parnate], phenelzine [Nardil], and isocarboxazid [Marplan]).

Drugs that reduce reactivity are sedatives and hypnotics, minor tranquilizers (anxiolytics) and major tranquilizers or neuroleptics.

Among the popular sedatives and hypnotics are the barbiturates (pheno-barbital, amytal, pentobarbital, secobarbital), which are employed in mild dosage to reduce agitation and anxiety and in larger dosage to induce sleep.

Most useful of these drugs are phenobarbital (taken in doses of 1 to 2 grains daily as a long-acting sedative) and sodium amytal (taken in doses of 1 to 2 grains as a short-acting sedative during the day and in doses of 3 to 6 grains at night as a hypnotic).

Because of the dangers of barbiturate addiction, the use of barbiturates must be carefully regulated (Fraser et al, 1958).

They have largely been replaced by the benzodiazepines (Dalmane, Halcion, Restoril, Xanax). Paraldehyde is, for many reasons, an ideal hypnotic, although the disadvantages inherent in its taste and residual odor have not given it the popularity it deserves.

Chloral hydrate (Noctec) is another hypnotic and sedative prescribed at times for various conditions.

Barbiturates are sometimes employed intravenously as an emergency measure in quelling intense excitement, sodium amytal www.freepsychotherapybooks.org 213 most often being prescribed for this purpose in doses of from 3 \ to 1\ grains.

Barbiturates should be employed for short periods only because of the possibility of habituation.

They should be avoided in borderline cases since they tend to lessen the patient's hold on reality.

The piperidine-diones (methyprylon-Noludar, glutethimide-Doriden) are sometimes employed as hypnotics in substitution for barbiturates, although habituation with them is possible if they are used over a long period.

The tertiary carbinol ethchlorvynol (Placidyl) is also used as a short-term (1 week) hypnotic, although its effect is diminished in the presence of anxiety.

Caution must be exercised in prescribing nonbarbiturate sedatives as well as the minor tranquilizers since physical dependence and addiction may result similar to the barbiturates.

Anxiolytics are employed to reduce muscle tonus, to combat anxiety, and to relieve stress.

Most important are: (1) the benzodiazepines (Librium, Valium, Xanax, Ativan), which, possessing both tranquilizing and antidepressant effects are employed alone in anxiety, tension, phobic, and agitated states, or in depression in combination with tricyclic and MAO inhibitor antidepressants; (2) the diphenylmethane derivatives (hydroxyzine [Vistaril] and benactyzine [Deprol]), which are sometimes prescribed for mild anxiety states (Jacobson, E, et al, 1955; Simms, 1958) possibly for their placebo value, and diphenhydramine (Benadryl), which is used in the behavior disorders of children (Fish, 1965); (3) the substituted propanediols (meprobamate-Equanil, Miltown), which, while decreasing conductivity along spinal interneuronal pathways, do not affect conditioned reflexes or the autonomic nervous system, and are occasionally used primarily for relief of tension, particularly in anxiety states of recent origin.

Minor tranquilizers, particularly meprobamate and the diazepines, should not be employed steadily for more than 3 months at a time.

Buspirone is a recent entry into the anxiolytic arena and has a relatively low profile of disagreeable side effects.

Antipsychotic drugs (neuroleptics) are noted for their influence on psychotic states.

Most prominent of these are the phenothiazines, which have not only effected a revolution in

the management of the disturbed mentally ill in institutions but have permitted a return of previously hopeless patients to the community.

Because of serious side effects and www.freepsychotherapybooks.org 214 sequelae, supervision is necessary.

Among the neuroleptics, thiothixene (Navane), haloperidol (Haldol), and molindone (Moban) have been enjoying increasing popularity.

Chlorpromazine (Thorazine) still continues to be used, with thioridazine (Mellaril) an adequate substitute.

Greater antipsychotic potency is possessed by phenothiazines with a piperazine side chain at position 2 in the phenothiazine ring system, for example, trifluoperazine (Stelazine), perphenazine (Trilafon), fluphenazine (Permitil, Prolixin).

However, side effects, such as Parkinsonian symptoms, akathisia, and paroxysmal dystonia, are also greater.

Such reactions are not serious since they can be controlled by an anti-Parkinsonian drug, such as benztropine (Cogentin) or trihexyphenidyl (Artane).

The problem of the choice of neuroleptics often arises.

Where patients are disturbed and agitated, an antipsychotic with sedative value, such as Thorazine or Mellaril, may be used.

Where patients are withdrawn and retarded, a drug with less sedative effect, such as Stelazine or Trilafon, may be employed.

Apart from these distinctions and the factor of individual idiosyncratic responses, there is little difference in the effect of antipsychotic drugs on psychotic processes.

If patients do not respond to one class of drugs, another class may be tried.

A trial period of 6 to 8 weeks with proper dosage of a drug is generally adequate.

If the response is satisfactory to one drug, there is little point in changing to others.

Insofar as combinations of drugs are concerned, there is no convincing evidence that they are any better than a single drug in the treatment of schizophrenia (Hollister, 1972).

The presence of extra-pyramidal syndromes may be expected with adequate dosage and is no cause for alarm.

Clonidine is another medication that has had an excellent effect on some cases of schizophrenia who have not responded to the usual antipsychotic drugs.

Because over 50 percent of chronic schizophrenic patients who require maintenance drug treatment fail to take psychotropic drugs, there has been a greater accent on the use of prolonged-action phenothiazines, such as fluphenazine enanthate and fluphenazine decanoate, which are given by injection.

The effects last from 2 to 4 weeks and patients are relieved of the responsibility of monitoring their own intake.

There are those who believe that the future treatment of psychosis lies in the prolonged-action drugs, particularly the depot preparations (Ayd, 1973).

In fractional doses side effects are www.freepsychotherapybooks.org 215 reduced for long-term maintenance.

At one time it was believed that treatment could continue for years and perhaps indefinitely.

However, because of the danger of tardive dyskinesia, attempts are now made both to avoid high dose rapid neuroleptization and to get patients on a reduced or drugfree regimen utilizing milieu and rehabilitative therapies to foster a better adaptation.

Experimental studies with psychodysleptics (psychotomimetic, hallucinogenic, or psychedelic drugs) particularly mescaline, LSD, and psilocybin are indicative of the contemporary efforts that are being made to correlate neurophysiologic, biochemical, and psychological processes that have both heuristic and practical value.

Psychodysleptic drugs effect the individual in a number of ways.

First, they encourage a psychotic-like experience with panic, grandiosity, paranoid delusions, impairment of reasoning and depression being most common.

Second, they introduce a unique, often lucid, mode of seeing problems from a unique perspective.

Third, there is a change and intensification in sensory perception.

Fourth, an upsurge of unconscious or preconscious ideation occurs with reliving of past incidents or symbolic portrayal of conflicts.

Fifth, distortion of ego boundaries is accompanied by peak cosmic transcendental or mystical experiences.

According to Pahnke et al., (1970) who worked with the drug at the Maryland Psychiatric Research Center, the effects of LSD "can be an enhancer of skilled psychotherapy when integrated with an intensive psychotherapeutic program of sufficient duration (30 to 50 hours)." This conclusion was generally endorsed by a sizable number of investigators who believed that the administration of the psychotomimetic drugs, usually LSD, was justified as an adjunct in psychotherapy since ego defenses become weakened and the patient is better able to reveal himself or herself (Savage et al, 1969; Kurland et al, 1971; McCabe et al, 1972; Savage & McCabe, 1973; Grinspoon & Bakalar, 1985).

The effects, however, cannot be predicted being dependent on the method of administration, the state of patient immediately prior to treatment, the surroundings, and the attitude and activity of the therapist.

There is little agreement regarding the conditions that may be benefitted.

Some observers believe anxious, upset, obsessional, and hysterical patients are most responsive (Sandison et al, 1954; Cutner, 1959).

Other therapists exclude such patients (Cohen, S, www.freepsychotherapybooks.org 216 1960).

Some are of the opinion that psychopathic personalities and alcoholics are helped most.

Others restrict the use of LSD to detached, inhibited, and uncooperative patients.

There is little question that channels are opened to forgotten or repressed memories and that the abreactive effect may be great.

How therapeutic this will be is another matter.

In patients who find it difficult to make contact with their inner feelings, and who have gained little from prolonged psychotherapy, hallucinogens may prove to be a means toward greater reexamination of the self.

Its specific effect cannot be dissociated from the placebo influence, emotional catharsis, suggestion, and other auxiliary healing forces.

The coordinate use of psychotherapy is mandatory and the manner of its employment will have a determining effect on the results (Crocket et al, 1963).

Against the possibility of some therapeutic gain is the real danger, in vulnerable patients whose hold on reality is tenuous, that hallucinogens may undermine defenses and throw the patient into a state of psychological decompensation.

The upset may be temporary ("the bad trip") lasting 8 to 12 hours, or may last longer with periodic flashbacks of frightening images or thoughts or more prolonged reactions requiring appropriate therapy.

The empathy, understanding, and skill of the therapist are probably the key factors as to whether an experience with hallucinogens will prove destructive or rewarding.

Psilocybin, the active agent of the intoxicating mushroom used by Indians in Mexico during religious ceremonies, produces effects similar to but less intensive than those of LSD.

Other hallucinogens in use are 2,5-dimethoxy-4-methylamphetamine (DOM) or (STP), hashish, myristicene (Nutmeg), dimethyltryptamine (DMT), and a number of other natural and synthetic

substances.

Psychedelic drugs are now rarely employed in therapy except by therapists skilled in their use for nonresponsive alcoholics, sociopaths, and obsessive compulsives unresponsive to other forms of treatment.

On the whole the use of these drugs is questionable.

Miscellaneous Drugs Tonics and vitamins are indicated in instances of dietary deficiency.

Where there is evidence of specific glandular impairment, appropriate hormones may be employed www.freepsychotherapybooks.org 217 (Hoagland, 1957; Paredes et al, 1961).

The most commonly utilized products are thyroid and estrogenic and androgenic hormones.

During physiologic upsets in the involutional period, natural estrins (Premarin), and stilbestrol and other synthetic estrogenic hormones, have been found useful.

Androgens, such as testosterone (Oreton), are prescribed in males who show a deficiency in this hormonal substance, manifested in waning libido, muscular weakness, and atony.

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Cade's report in 1949 lithium carbonate (Eskalith, Lithobid, Lithonate) have been employed in bipolar affective disorder both during the active manic psychosis and as a form of maintenance therapy in cases subject to recurrent attacks.

A constant check on the blood-level of lithium is essential to insure the therapeutic effect.

In addition to being an impressive remedy for manic and associated depressed states, lithium does not dull feelings or reduce clarity of thinking as is so common with the large dosage of phenothiazines needed to calm excited patients.

Lithium has also been tried with other syndromes, such as borderline conditions, aggressive episodes, intermittent explosive disorders, pyromania, and schizoaffective disorders, but its value here is not as yet as fully established as with the bipolar disorders.

Continuous sleep treatment with drugs (Klaesi, 1922; Palmer & Braceland, 1937; Walsh, J, 1947; Williams & Webb, 1966) is sometimes used where other forms of therapy have failed.

The patient is narcotized for at least 7 to 10 days with intervals for feeding, urination, and defecation.

Drugs are gradually withdrawn, and sleep terminated slowly.

Since patients must be monitored over a 24-hour day period, hospitalization is essential.

The technique is utilized mostly in Europe and Russia, rarely in the United States.

Chlorpromazine is the chief drug employed along with a short-acting hypnotic, amobarbital sodium (sodium amytal).

Coordinately, depending on the symptoms, antidepressant drugs and ECT (2 or 3 times weekly) may be used.

On this regime more than 70 percent of patients suffering from obdurate chronic tension states, depression and phobic anxiety states, 43 percent from obsessional neuroses, and 41 percent from longstanding schizophrenia or schizoaffective states are said to have become symptom-free or much improved.

www.freepsychotherapybooks.org 218 Antabuse, originally used in Denmark (Hald et al, 1948), is still widely employed as a means of controlling alcoholism (Bowman et al, 1951).

Although the drug is generally nontoxic, symptoms of a frightening nature occur when a patient under antabuse medication imbibes alcohol.

The reaction is so disagreeable that the individual willingly abstains from drink.

The individual's self-confidence soon is restored and self-esteem increases as a result of his or her ability to remain sober.

Psychotherapy should always be administered jointly with antabuse (Child et al, 1951), to help

prevent a relapse, and the patient should be encouraged to join Alcoholics Anonymous. In drug addiction, methadone (Dole & Nyswander, 1965) and cyclazocine (Martin, WR, et al, 1965; Jaffe & Brill, 1966) are being given to control the ravages of this disorder. Carbon dioxide, introduced by Meduna (1950), was used as a method of treating neurotic and addiction problems alone and in combination with psychotherapy. Its use continues to be controversial. Hargrove, Bennett, and Steele (1953), for example, challenge the findings of Meduna and present evidence that most patients subjected to carbon dioxide show no improvement in their emotional condition. It is difficult to judge how much of the beneficial effect is due to the placebo influence or to psychotherapy that is conjunctively employed (Tibbetts & Hawkings, 1956).

Orthomolecular psychiatry.
Utilizing large doses of B & C vitamins and various minerals and hormones, attempts have been made to reestablish chemical imbalances of the brain presumed to be responsible for schizophrenia. While megavitamin therapy has not been proven effective nor the theory on which it is based substantiated, there is growing evidence that nutritional factors do play some part in neurotransmitter synthesis and brain metabolism, the specifics of which will undoubtedly be empirically explored in the forthcoming years.

Miscellaneous somatic therapies.
As might be expected a host of somatic therapies have been introduced that have not proven themselves to be of lasting value. Among these are implantation of brain pacemakers, detoxification procedures as with hemodialysis, desensitization with histamine, administration of acroagone suspensions and www.freepsychotherapybooks.org 219 acetylcholine, cerebral oxidation procedures, cerebral pneumotherapy, use of a hyperbaric chamber, and acupuncture. Where a patient has faith in any of the above or other procedures, and the therapist applies them with conviction and enthusiasm at least temporary improvement may be expected purely through the placebo and relationship dimension.

CONVULSIVE THERAPY In severe depressions, and in some cases of mania and acute catatonia, particularly where there is danger of suicide, ECT is the treatment of choice (Kalinowsky & Hoch, 1961; Paterson, 1963; Kalinowsky, 1965; APA Task Force, 1978; NIMH, 1978; Fink, M, 1979; Abrams & Esman, 1982; Kalinowsky et al, 1982). Approximately 80 to 90 percent of all patients suffering from involutional melancholia or major depression psychosis recover with 6 to 10 ECTs. In schizoaffective disorders a somewhat lower recovery rate (about 60 percent) is scored, and more treatments are needed. Neurotic depressions are generally treated by psychotherapy alone or in conjunction with antidepressants, except where the depression is leading to exhaustion or poses the risk of suicide. Under these circumstances four to six treatments may suffice to restore patients to their predepression state. In excited reactions that cannot be controlled and in anxieties that become so overwhelming that a psychosis is threatened, as in obsessive-compulsive and borderline patients whose defenses are crumbling, ECT may serve to restore the psychological equilibrium. Periodic maintenance ECT may be helpful in recurrent depressions and some chronic schizophrenic and very unstable borderline patients, who totter on the verge of disintegrative reactions. An anesthetic agent (Pentothal, Surital, Brevital), atropine, and a muscle relaxant (succinylcholine chloride—Anectine) administered prior to the electrical stimulus, reduce

discomfort and eliminate skeletal complications.

To minimize the memory loss, unilateral ECT rather than bilateral ECT may be employed, though in the opinion of some, a somewhat attenuated antidepressive effect results (Abrams, 1972; Abrams & Taylor, 1976).

This is disputed by other unilateral ECT advocates who claim equal antidepressant effects provided the induced seizure is adequate. Indokolon (hexafluorodiethyl ether) administered by inhalation is an alternative method of inducing convulsions (Krantz et al, 1957; Esquibel et al, 1958; Karliner, 1966).

It is employed by some therapists when regular ECT treatments are unacceptable or have failed. Since it is not manufactured in the United States it may be difficult to obtain.

A task force in Massachusetts (Psychiatric News, 7, [22] 1972) investigating the use of electroconvulsive therapy made comments that are still pertinent today on the ever widening gulf between doctors who seek a biological approach to psychiatric illness and those concerned primarily with human emotions.

The task force, however, did discover abuses and that there were some psychiatrists who were "so enthusiastic about shock treatment that they recommend it for almost all patients, believing, with only their personal clinical experience to support their opinions, in the relative omnipotence of ECT." The study, as we would expect, indicated that there was no evidence that shock was able to modify character or personality structure in any way. According to the task force the study revealed that in severely depressed patients with suicidal tendencies the procedure could be life saving.

ELECTRONARCOSIS AND ELECTROSLEEP Subconvulsive electrical stimulation (electronarcosis) has occasionally been employed for its sedative effect in prolonged anxiety or panic, and in excited states that do not yield to psychotherapy or drugs.

Subconvulsive electric current of high intensity has also been recommended to produce abreactions to supplement the abreactive methods of hypnosis and narcosynthesis.

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Paterson (1963) reported that good results may be anticipated in traumatic neurosis, antisocial personality disorder, and other patients with a poor prognosis.

The general experience has been that electronarcosis is less effective than full ECT.

The electrical induction of sleep has also been attempted, which may be followed by abreaction and dissociative reliving of traumatic experiences.

Utilization of weak electrical currents have been employed to produce behavioral and subjective relaxation without concurrent changes in heart rate, blood pressure, and respiratory rate (Rosenthal, SH, 1971).

Improvement in anxiety, depression, and sleep disturbance has been claimed (Rosenthal, SH, 1972).

The best results have been obtained with chronic anxiety and tension states.

However, Frankel, BL, et al.

(1973) reported an ineffectiveness of electrosleep in chronic insomnia.

Treatments are usually given from 3 to 5 times weekly, each session lasting 1 hour, treatment current ranging from 0.5 to 1.5 milliamps.

In this age of electronics we may expect that the mysterious equipment trappings have great placebo value for some patients.

The relaxation induced, which may proceed to a hypnotic state, increases the suggestive dimension.

A rapid positive relationship with the therapist is also established.

Whether there are specific therapeutic effects beyond these intercurrent influences is

difficult to say.

INSULIN COMA Because of the expense of administration, the relative danger, the long time contraindications, and the effectiveness of the neuroleptics, the use of insulin coma has gradually diminished.

However, it may still have some utility in early schizophrenia, deep, long, and frequent comas being of value in interrupting a process that, neglected, may proceed to chronicity (Sakel, 1938; Hoch & Kalinowsky, 1961; Paterson, 1963; Sargant & Slater, 1963; Kalinowsky et al, 1982). Between 50 and 150 1-hour-long comas may be given to selected patients on a 6-day-a-week basis. Subcoma insulin treatment is of little service in schizophrenia, although it may sometimes be helpful in acute anxiety and toxic confusional states.

In such conditions the patient may quiet down sufficiently so that psychotherapy may be used. All in all, we may consider insulin coma of little practical utility today.

Among the coma treatments that are now only rarely employed are atropine coma, nitrogen and nitrous oxide inhalation, and ether and trichloroethylene inhalation.

PSYCHOSURGERY Psychosurgery is only rarely done today being restricted to long-term, disabling, severe psychiatric illness after all medicinal, somatic, and psychosocial procedures have failed after being tried for at least one year (Freeman & Watts, 1942).

The most frequent www.freepsychotherapybooks.org 222 condition for such a radical intervention is endogenous depression that has not responded to antidepressant drug treatments as well as ECT.

Sometimes psychosurgery is done to eliminate long-term unmanageable pain not helped by any other method.

A much less frequent indication has been intractable obsessive-compulsive neurosis and, even more rarely, schizophrenia with affective symptomatology.

Through cuttings made in the frontal lobes of the brain and the cingulate bundle (cingulotomy), an attempt is made to interrupt impulses traversing the limbic system.

This operation is modest compared to earlier surgery that had resulted in untoward personality and physical sequelae.

Psychosurgery, while having a minimal influence on the thinking process, does relieve or eliminate strong emotional undercurrents that may be creating severe problems for the patient. It is important, prior to making arrangements for surgery, that the patient and responsible relatives, are informed of the risks and possible benefits of the operation and that the informed consent or refusal is noted carefully in the case record.

Since a number of states set up rules to be followed for or have statutes prohibiting the operation, the legality of the procedure in a certain locality should be checked.

INSPIRATIONAL GROUP THERAPY Inspirational group therapy continues to be of value for a large number of people who may not be willing to accept more formal therapy, or are unable to afford it, or are unmotivated for any other type of intervention.

It is a time-honored method that has brought relief to many distraught people and whose influence is unfairly underrated.

An example of important inspirational groups are those run by Alcoholics Anonymous, Recovery, Inc., and Synanon.

There are other helpful groups in operation, some of the most successful being those organized for addiction problems, e.g., drug abuse, chronic gambling, cigarette smoking, and obesity.

There are groups oriented around objectives of education and of social rehabilitation dealing with various adjustment problems.

They are composed of members seeking support, companionship, and opportunities for discussion as in parents anonymous, widows clubs, etc.

(Greenblatt, 1985b).

Indeed, such groups, www.freepsychotherapybooks.org 223 often led by leaders who have experienced and successfully conquered the same problems for which help is being sought by the group members are more successful with certain problems in some situations than formal psychotherapy.

In order to deal with underlying personality difficulties, some participants may become motivated as a result of the group experience to seek formal psychotherapy.

Some inspirational groups are oriented around a charismatic leader who is established as an omniscient personage whom the patient is expected to obey.

To a great extent, these groups exist as an appendage of the leader, and beneficial results are maintained as long as the followers are capable of supporting an image of the leader as powerful and protective.

Symptom relief is largely brought about as

a result of repression of conflict and a desire on the part of the individual to identify with and gain status in the eyes of the leader.

Such inspirational groups are particularly tempting to dependent souls whose inner will to develop is diminutive, and who flourish in a setting in which they are able to establish a submissive relationship to a stronger individual.

They then become interminably attached to the leader, the group, or authoritative individuals within the group.

The group, nevertheless, continues to exert an important independent effect on the individual, being composed of members suffering from problems as severe as, or more severe than one's own. In a group of normal persons, the individual often feels handicapped and inferior and may succumb to defenses of self-justification or building up self-esteem or of striking out aggressively in order to avoid fancied hurt.

In a supportive therapeutic group, the individual is not subjected to the same pressures and cannot help but feel a sense of unity in the course of identifying with many of the problems of fellow members.

Inspirational group therapy is also employed as part of a psychotherapy program by some therapists who claim beneficial effects for it, including the mastery of symptoms, the institution of self-discipline, the tolerance of anxiety and tension, and the repression of antisocial impulses and drives (Pratt, 1934; Rhoades, 1935; Harris, 1939; Wender, 1940; Blackman, 1942).

The patient feels an acceptance in the group that could not be experienced elsewhere.

The patient finds that he or she can be self-expressive and that www.freepsychotherapybooks.org 224 admitting to certain strivings does not make one "bad" or worthless in the eyes of others.

The patient may eventually discover that status can be gained within the group.

Emphasis is upon day-to-day victories, self-control, finding new social outlets, and strengthening will power.

Herschelman and Freundlich (1972) have described an inspirational group experience in working with a large group of patients utilizing multiple therapists.

Group therapy meetings were held at the Philadelphia Naval Hospital inpatient psychiatric service attended by the entire patient population of 35 to 45 patients and staff (5 psychiatrists, 2 or 3 psychiatric residents, 2 hospital corpsmen, and 1 psychiatric nurse).

Meetings were of one hour's duration on a weekly basis.

Most of the patients were poorly motivated for formal psychotherapy, and the staff at first was resistant, passive, and nonparticipant.

As the latter gradually resolved their anxiety, they became more enthusiastic and participant.

Staff-patient conference techniques were utilized such as those proposed by Berne (1968) in the spontaneous meetings held immediately after large group sessions.

The results exceeded expectations and point to the advantage of utilizing multiple therapists rather than single therapists with large groups.

Many persons benefit chiefly through the social contacts that they make in a group setting, particularly when they find other persons with whom they can share experiences.

The relationships established in the group help to ease social tensions and to promote self-confidence.

After the formal group experience the participants may continue on their own in self-help groups.

The self-help movement has been gaining momentum over the years, many taking the form of "therapeutic clubs" of members suffering from various habit, behavior, and psychiatric disorders.

Curiously, some of the most stubborn problems that resist the ministrations of professional personnel appear to be benefitted by group experiences without the presence of a therapist, in which the patients relate to each other and share their past experiences, present difficulties, and progress with peers suffering from similar ordeals and backgrounds.

Perhaps the oldest group of a self-help type is Recovery, Inc., www.freepsychotherapybooks.org 225 consisting of former mental patients.

Founded in 1937 by Abraham Low, Recovery, Inc., has rendered service to many thousands of patients.

At present there are hundreds of such groups in the United States.

They are generally headed by nonprofessionals drawn from the membership.

At their meetings a panel discussion format is employed with the leader selected from the group and trained along lines of Low's book *Mental Health Through Will Training* (1952).

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Grosz (*Psychiatric News*, 7, [14], 1972, 1973; and 8, [12], 1973) has studied and reported positively on the results of Recovery, Inc., as a resource that is available to many patients who otherwise would have no other form of help available to them.

Many other self-help groups have flourished because they provide a helping resource that for considerable numbers of individuals eclipses that provided by trained professionals.

Why this is so is still shadowy, but such factors undoubtedly operate as fear of and subtle antagonism against authority as vested in traditional psychiatry and psychology, the presence in the group of individuals with whom the person can identify, the opportunity for modeling by group leaders who have overcome destructive habits the person seeks to relinquish, opportunities for constructive group interaction and postgroup companionship, the force of peer pressure to maintain improvement, and other bounties of group dynamics.

Economic factors also play a part especially for those on meager incomes since attendance is usually free or at a low cost.

To a large extent the success of these movements is related to the leaders who run them.

Intelligent, empathic yet firm direction is essential in running a group of this kind due to the inherent personality problems of members who seek help, particularly those who have difficulty with authority and with interpersonal relationships in general, borderline cases, severe obsessive-compulsives, paranoid individuals, and psychotics.

Many have tried outpatient resources and private therapists without success.

They require special reassurance and the installation of hope.

Members are usually most responsive to a warm, non-authoritarian yet structured environment, led by leaders who have experienced and conquered the devastation the members themselves have gone through.

The leader must consequently be relatively free from emotional illness and

www.freepsychotherapybooks.org 226 capable of dealing with transference and personal

countertransference.

This will usually require training and some personal therapy that some leaders refuse to accept.

Self-help groups that run into problems generally are victims of poor or destructive leadership.

In leaderless self-help groups, the most aggressive individuals with needs for control usually take charge, which may cause difficulties unless the other members are strong enough to prevent the leader from becoming too dictatorial.

Alcoholics Anonymous (see Chapter 60) particularly has inspired many self-help groups.

These include Narcotics Anonymous (NA) for persons seeking to overcome narcotic addiction, as well as Nar-Anon for their families; Pills Anonymous (PA) for those habituated to various sedatives, tranquilizers, hypnotics, and stimulants, and Pil-Anon for their families; and Overeaters Anonymous.

All of these organizations allege themselves to be nonprofit societies with no initiation fees or dues and are self-supporting through their own contributions, with no affiliations with any sect, denomination, politics, organization, or institution.

In all of these habit controlling groups 12 steps toward recovery are presented.

The individual is enjoined (1) to admit and accept one's inability to manage one's life single-handedly, (2) to recognize that only a Power, an ultimate authority greater than oneself, can restore one to health (this Power can be God, a force in the Universe, the group in itself, or nature), (3) to decide unequivocally to turn one's life and will over to this Power; (4) to make a searching and fearless moral inventory of oneself; (5) to admit the exact nature of one's wrong to oneself, to God (or the Power) in order to remedy one's defects of character, (7) to ask this Power to remove one's short-comings, (8) to list the people one has harmed and be willing to make amends to them all, (9) to see that such amends are made if possible except where some injury may result, (10) to continue to take a personal inventory and when wrong to admit it, (11) to improve, through meditation and prayer, one's contact with this Power as one understands it, praying for knowledge of God's (or the Power's) will and the ability to carry out what is necessary, and (12) after having a spiritual awakening, to carry this message to others in need and to practice these principles in all of one's affairs.

We may see in the 12 steps the devotional help and protection one expects of www.freepsychotherapybooks.org 227 religion or other guidance resources, as well as the need to alter one's values and meaning systems such as occurs in cognitive therapy.

Not insignificant also in these groups are the benefits of emotional catharsis and group dynamics.

The principles and practices of groups organized for addictions and habit disorders have also been extended to general emotional problems in a self-help group calling itself Emotions Anonymous (P.O.

Box 4245, St.

Paul, Minnesota 55104).

Other self-help groups following to some extent the format of Alcoholics Anonymous are Gambler's Anonymous and Weight Watchers.

Excellent pamphlets, articles, booklets, audio tapes, films, and video cassettes dealing with self-help groups are sold by Hazelden Educational Materials (Box 176, Center City, Minnesota 55012).

Branches of self-help groups, where they exist, are listed in local telephone directories.

The accomplishments of self-help groups point to the need for a new psychiatric task force that can combine with adjunctive disciplines to help shift the emphasis from institutional based therapy to domiciliary and community involvement (Dean, 1970/1971).

This is all the more advisable in view of the fact that results in some self-help groups have been found wanting, particularly where encounter and confrontation techniques are utilized, as with ex-drug addicts who are unable to relinquish their defenses or where they are suffering from borderline conditions (Shick & Freedman, 1975).

Acting-out, paranoid, and psychopathic patients do not do well in self-help groups.

The involvement of trained professional people for screening purposes and to manage emergencies serve as an important safeguard.

www.freepsychotherapybooks.org 228 10 Reeducative Therapy The relationship between patient and therapist in reeducative therapy has as its object the achievement of more extensive goals than in supportive therapy, namely, an actual remodeling of the patient's attitudes and behavior in line with more adaptive life integration.

The therapist here attempts either (1) to influence the processes directly between the patient and his or her neurotic behavior, rewarding healthy responses, or (2) to release in the patient self-actualizing tendencies by utilizing the relationship as a corrective emotional experience. There is less emphasis on searching for causes than on promoting new and better forms of behavior.

It is posited that individuals with help from a therapist have within themselves the ability to reorganize their values and behavioral patterns.

Such approaches are more or less reeducative in nature and, therefore, may be designated as "reeducative therapy." The objective in reeducative therapy, thus, is the modification of behavior directly through positive and negative reinforcers, and/or interpersonal relationships, with deliberate efforts at environmental readjustment, goal modification, liberation of existing creative potentialities, and, it is hoped, promotion of greater self-growth.

No deliberate attempt is made to probe for unconscious conflict.

Under these circumstances individuals may achieve sufficient command of their problems to enable them (1) to check acting-out tendencies, to rectify remediable environmental distortions, or to adjust to irremediable ones; (2) to organize life goals more rationally and to execute them in a facile manner; and (3) to consolidate some adaptive defenses and to alter others that are less adaptive.

These are eminently worthwhile objectives, and, for reasons that will be considered later, are often as far as many patients can progress, even with the most intensive reconstructive approaches.

Indeed, in many instances, reeducative therapy is the treatment of choice.

A fundamental assumption in reeducative therapy is that if one succeeds in altering a significant pattern in one's life, the restored sense of mastery will generalize over a
www.freepsychotherapybooks.org 229 broad spectrum of behavior.

If substantial improvement is scored in one dimension of functioning, this may importantly influence other parameters of personality operation.

Reeducative therapy is conducted through (1) the implementation of a variety of techniques aimed at reconditioning behavior or (2) an examination by the patient and the therapist of ways that the patient relates to people and to himself or herself.

In the latter, manifestations of tension and anxiety are explored, and the patient is helped to recognize certain aspects of his or her behavior that are destructive to adjustment.

The patient is then encouraged to experiment with new interpersonal attitudes and additionally stimulated to utilize his or her assets to best advantage so as to expand positive qualities within.

While interview procedures are employed, little or no use is made of dream material, transference manifestations, and free association.

Sometimes reconstructive changes occur as a consequence of reeducative therapy, although these are not specifically the objectives toward which treatment is directed.

The application of reeducative therapy requires specialized training that sensitizes the therapist both to aspects of behavior that require and will be amenable to alteration and to the recognition of gross interferences to the therapeutic process of transference and resistance.

While personal psychoanalysis or personal reconstructive therapy for the therapist is helpful, it is not absolutely essential in executing this approach, provided the therapist does not have severe neurotic difficulties and can control countertransference if this begins to project itself harmfully into relationships with patients.

Among reeducative therapeutic measures are "behavior therapy," therapeutic counseling, directive psychotherapy, casework therapy, "relationship therapy," "attitude therapy," distributive analysis and synthesis, interview psychotherapy, semantic therapy, reeducative group therapy, and certain philosophical approaches.

BEHAVIOR THERAPY Behavior therapy continues to spread its influence over the entire field of human afflictions and disabilities.

This is no fortuitous event since its methods have proven valuable in the hands of skilled practitioners.

Books and articles on the subject and www.freepsychotherapybooks.org 230 membership in behavior therapy organizations have increased in the past decade.

To the traditional zone of behavioral distortions have been added internal mental processes and psycho-physiological ailments and habits.

Maintaining the original dedication to the principles and findings of experimental psychology, behavior therapy has elaborated a plethora of techniques and a diversity of views that go far beyond learning theory and that are dedicated to the alleviation of all aspects of human suffering and the general enhancement of functioning.

More specifically, behavior therapy is said to address "clinical problems using (a) a testable conceptual framework, (b) treatment methods that can be objectively measured and replicated, (c) outcome criteria that can be validated, and (d) evaluative procedures for determining the effectiveness of specific methods applied to particular problems" (Lazarus & Fay, 1984).

Lazarus (1984) has stated that "there is scarcely a clinical entity for which a behavioral intervention strategy cannot be proposed and implemented." The literature includes discussion of behavior therapy in the treatment of these and other problems: affective disorders (De Rubeis & Hollon, 1981), alcoholism (Chaney et al, 1978), asthma (Creer & Kotses, 1983; Dekker et al, 1957; King, 1980; Philander, 1979; Richter & Dahme, 1982), back pain (Cairns et al, 1980; Gottlieb et al, 1977; Newman, et al 1978), cardiac arrhythmia (Benson et al, 1975), cardiac problems (Matarazzo et al, 1982), dental problems (Ingersoll et al, 1977), depression (Lewinsohn & Hoberman, 1982; McLean & Hakstian, 1979), geriatrics (Patterson & Jackson, 1980; Patterson, 1982), headache (Blanchard et al, 1979), insomnia (Ascher & Efran, 1978; Ascher & Turner, 1980; Bootzen, 1972, 1977; Borkovec & Boundewyns, 1976; Jason, 1975; NaHauri, 1979; Turner & Ascher, 1982), obesity (Stunkard, 1982), obsessions (Emmelkamp & Kwee, 1977; Foa et al, 1980; Marks et al, 1975, 1980; Marks, 1981; Rachman & Hodgson, 1980; Steketee et al, 1982), overeating (Stuart, 1967); pain (Fordyce et al, 1973; Fordyce, 1976; Roberts & Rheinhardt, 1980; Sanders, 1979; Stegar & Fordyce, 1982; Swanson et al, 1979), phobias (Ost et al, 1981, 1982), problems of children (Benson 1979; Brownell et al, 1977; Drabman et al, 1978), schizophrenia (Curran et al, 1982), smoking (Litchenstein & Brown, 1982), and urological disorders (Doleys & Meredith, 1982).

www.freepsychotherapybooks.org 231 The many varieties of behavior therapy are commonly related to a behavioral learning model of psychopathology that focuses on "observable behavior" instead

of on “hypothesized personality structures or presumptive subjective experiences” (Phillips & Kanfer, 1969).

Speculation, appraisal, and interpretation are in terms of environmental stimuli and behavioral acts rather than inner conflicts and other “conjectural constructs.” Modification of behavior is presumably bracketed to research findings and studies in experimental psychology laboratories.

Being data-oriented, behavior therapy attempts to avoid speculative inferences about the meaning of events.

This is not to deny the importance of such inferences or the usefulness of the reports of subjects about what is happening to them.

But these concepts are not employed to explain behavior.

The historical genesis of behaviors selected for modification is not considered material for diagnosis and treatment, even though there is recognition that abnormal behaviors have historical origins.

Rather, the circumstances that control and sustain these behaviors in the here-and-now are the central targets of treatment.

It is a common observable fact that encouraging a patient to behave in emotionally constructive ways and helping him or her to derive pleasure or to secure rewards for such behavior may reinforce the behavior to a point where it becomes an established pattern.

Such relearning often results in the evolvment of new attitudes and values and sometimes even in cognitive restructuring.

Though the auxiliary healing agencies (placebo, emotional catharsis, idealized relationship, suggestion, and group dynamics) play a role in the interaction between therapist and patient and transference experiences and the working through of resistance undoubtedly take place, the principal therapeutic instrumentalities are directive stratagems and maneuvers designed to provide the individual with a corrective relearning experience.

While not always successful (as with any therapy) such “behavior” or “conditioning” therapy has become a valuable instrument with which to approach certain emotional problems, and it has proved successful in many cases that have not yielded in the least to insight therapies.

Particularly dramatic have been results in retarded, schizophrenic, psychopathic, addictive, and other patients who do not respond to the traditional interviewing and insight approaches.

www.freepsychotherapybooks.org 232 Unlike conventional proponents of psychopathology who regard symptoms as surface manifestations of unconscious disease processes, behavior therapists base their operations on a social-learning model (Bandura, 1965b; O’Leary & Wilson, 1965; Franks, 1969; Rimm & Masters, 1974).

This conceives of most maladaptive behavior as primarily learned response patterns that may be altered directly by manipulating stimulus variables of which the behavior distortions are said to be a function.

Since it is believed that the distortions can be dealt with directly and adequately, it is unnecessary, in the opinion of behavior therapists, to introduce the concept of underlying pathology into their model on either practical or theoretical levels.

Behavior therapy is thus organized around the conception of neuroses as learned maladaptive patterns of thinking, feeling, and behaving, conditioned by aversive experiences and sustained by reinforcements that prevent their extinction.

It is also recognized that these learned patterns of behavior are superimposed upon biological determinants.

Learned habits become fixed and refractory to elimination.

Neither postulated unobservable events nor purposive and teleological principles, the bedrock of psychoanalytic theories, are employed as the basis for therapeutic maneuvers.

Behavioral deficits or excesses are considered to issue out of the patient's immediate historical environment.

Consequently, a search is organized for pathogenic environmental variables in order to manipulate and modify them.

Implicit in behavior therapy is the precept that whatever has been learned can be unlearned by reversing the learning process (Eysenck, 1960a&b; Staats & Staats, 1963; Reyna, 1964).

But, by the same token, behavior therapists recognize that learned behavior can also be modified by nonbehavioral techniques (e.g., drugs) and, similarly, that changes brought about other than by learning (e.g., physical trauma) can be modified by some form of learning process.

A number of modes of learning reversal have been suggested.

Originally, behavior therapists focused exclusively upon modification of overt behavior and the presenting complaints or "symptoms" of the patients.

Gradually, behavior therapy is said to have evolved into its present sophisticated multimodal framework, which— while by no means ignoring presenting complaints—focuses upon all maladaptive systems within the patient, as determined by an intensive behavioral assessment conducted as a cooperative project by the patient and therapist in concert.

Cognitions, affects, drives, and certain www.freepsychotherapybooks.org 233 physiological processes are all considered modifiable in accordance with the principles of classical and operant conditioning, modeling, and related concepts.

Classic is the historic case reported by M.

Jones (1924a&b) of a child who had developed a great fear of animals in general after being bitten by a rabbit.

Jones formulated the hypothesis that if he could get the child to associate rabbits with a pleasant emotion, fear of rabbits—and perhaps of all animals—might disappear.

Upon complaints of hunger, he fed the child appetizing foods in sight of the rabbit held at a secure distance.

Gradually he diminished the distance until the child was able to tolerate the close presence of the rabbit.

This eventually extinguished the fear.

D.

Yates (1939), treating a girl who was upset emotionally by the presence of men, had her repeat the word "calm" while associating with it at the same time ideas of security, well-being, and peace.

She gradually learned that constant repetition of the word in the presence of men sufficed to maintain her emotional composure.

The use of aversive stimuli to correct untoward behavior was reported by Max (1935) and Mowrer and Mowrer (1938).

Max treated a homosexual patient who was obsessed with homosexual thoughts whenever he came into contact with a certain inanimate object.

Presenting this object to the patient and giving him an electric shock at the same time sufficed to terminate the power of the object to excite homosexual thoughts.

Mowrer and Mowrer treated enuresis by constructing an apparatus that was placed in the bed of the enuretic child.

The bed, when wet, caused a circuit in the apparatus to close and to ring the bell, thus awakening the child.

They discovered that after three or four such experiences the impulse to urinate in itself sufficed to arouse the child.

Modern forms of behavior therapy follow these early reconditioning tactics, but they base their

rationale on current ideas about learning theory, particularly classical (Pavlovian) and operant (Skinnerian) conditioning (Hilgard, 1956; Kimble, 1961; Wolpe & Lazarus 1966). Self-knowledge and efforts toward reconstruction of personality, as the psychodynamic therapist views these concepts, are felt to be too time-consuming, productive of untoward side effects, unscientifically founded, and, above all, ineffectual.

Accordingly, the reliving of the original formative experiences and knowledge of historical precursors of present problems are deemphasized both diagnostically and therapeutically. www.freepsychotherapybooks.org 234 Instead, the individual is exposed to stimuli and to autonomic responses that neutralize fear and anxiety (classical or respondent conditioning). Where the repertoire of learned behavior must be expanded, principles of operant conditioning are employed, new behavioral tendencies being shaped by reinforcements toward specific goals. In the past decade, formulations derived solely from learning theory have been considered too restrictive and concepts from experimental and social psychology have been employed to evolve useful interventions.

The resulting techniques are relatively directive and aimed at manipulating the forces of behavior, which are believed to be recognizable, contemporary stimuli.

Identifying the direction of the treatment, however, is a collaborative process arrived at by the patient, therapist, and involved "others." Treatment is an ongoing process that is subject to change and periodic review.

The aim is always to maximize the individual patient's control of and responsibility for determining the nature and course of what is to be changed.

Behavior therapy that imposes directives for change upon a patient is bad behavior therapy. Objectives are explicitly defined by mutual agreement with the patient, and conditions are deliberately controlled in order to change disturbing emotional reactions to provocative situations.

Among the procedures used are the following: 1.

Desensitization by exposing the patient to a rapid repetition of the stimulus and/or the response, with or without reinforcement.

2.

Extinction of old responses by omitting either reinforcement or the original unconditioned stimulus that followed the eliciting stimulus or the instrumental responses.

3.

Punishment by subjecting the patient to an aversive stimulus whenever the patient makes an undesirable response.

4.

Counterconditioning by affiliating a different and opposing response with the conditional stimulus, followed by direct reinforcement of the opposing response.

5.

Cognitive restructuring by dealing with attitudes and faulty ideas that influence behavior.

www.freepsychotherapybooks.org 235 Positive Counterconditioning In counterconditioning, the quality of the anxiety-inhibitory response utilized is related to the ultimately desired reaction.

Thus, "assertive responses" are encouraged by the therapist, even to the point of "acting-out," in patients who have neurotic anxieties about asserting themselves with people.

This idea is the basis of Salter's Conditioned Reflex Therapy (1961), in which, through authoritarian direction, the patient is enjoined to abandon destructive patterns of behavior and to practice new habits that will be of value to him or her.

Postulated is the theory that healthy biological organisms are in a state of free emotional expressiveness (excitatory state).

On the other hand, unhealthy organisms with emotional illness are said to be in a state of pathological inhibition, which has become a conditioned response and which blocks normal excitation.

Therapy must be directed toward unlearning conditioned inhibitory reflexes and replacing them with conditioned excitatory reflexes.

This is accomplished by deliberately practicing excitatory emotional reactions until they become established as conditioned reflexes.

Thus, patients are encouraged to express their feelings openly, accompanying these by appropriate or exaggerated motor reactions.

Whenever they are in a situation where they disagree with others, they must deliberately and forcefully express themselves instead of inhibiting their feelings.

Spontaneous reactions to new situations are preferred to assumed or conventionalized behavior. Expressing self-praise and promulgating one's own opinions and values are urged as a means of increasing self-confidence.

By making them repeat such "positive" and "excitatory" acts, patients are said to be liberated from the harmful effects of inhibition.

Thorne (1950) describes a method somewhat along these lines whereby fear reactions may be dissipated through a conditioning process of verbally admitting the presence of fear, of appreciating that it is a common reaction, of anticipating its arousal, and of training oneself to avoid inappropriate responses.

Tasks are assigned that enhance emotional control and self-control, and the patient is reminded that many months will be required before success is complete.

"Relaxation responses" through "systematic desensitization" are recommended for fears associated with almost every aspect of behavior.

By presenting anxiety-provoking www.freepsychotherapybooks.org 236 cues in a climate of pleasure or relaxation, aversive stimuli are, one hopes, mastered in progressively stronger forms.

Amenable to this tactic are said to be a variety of emotional conditions, including tension states, anxiety reactions, behavioral inhibitions, phobias, and some psycho-physiological disorders.

Among positive stimuli jointly presented with and calculated to neutralize the aversive stimuli are muscular relaxation (Wolpe, 1958; Eysenck, 1960a), pleasurable associations to interpersonal situations (English, 1924; Bentler, 1962), imagery of an emotive nature (Lazarus & Abramovitz, 1962), pharmacological agents that reduce sympathetic reactivity (Walton & Mather, 1963b), and food and sexual fantasies.

A form of this method known as "reciprocal inhibition" was developed by Wolpe (1958, 1961, 1969), who exposed animals with an experimental neurosis, while feeding them, to stimuli at first remotely related to and then more closely approximating the conditioned aversive stimulus.

Increasing increments of anxiety were thus mastered until the original conditioned stimulus brought forth no reaction and the experimental neurosis was relieved.

Around this finding Wolpe developed his theory of "reciprocal inhibition" to the effect that "if a response inhibitory of anxiety can be made to occur in the presence of anxiety-evoking stimuli it will weaken the bond between these two stimuli and the anxiety." In his technique Wolpe enjoins the patient to identify and to list in rank order a number (say, 20) of categories of upsetting stimuli.

The patient is taught how to relax (hypnosis and self-hypnosis may be employed) for several sessions, and then in a state of relaxation is asked to bring to mind, for a few seconds, the the weakest of the anxietyevoking situations in the anxiety hierarchy.

When the anxiety response is zero, the next stimulus in the hierarchy is envisioned. Since relaxation inhibits anxiety, the offensive stimuli are progressively mastered until the most provocative and disturbing stimulus can be countenanced without upset. A simple illustration of classic conditioning technique may be found in the account of Rubenstein (1964), who describes the conditioned reflex treatment of tubercular patients addicted to morphine.

The first was a man of 38 who consumed 8.5 grains of morphine daily.

The method employed consisted of the ringing of a bell at the start of each

www.freepsychotherapybooks.org 237 hypodermic injection.

This was abandoned after a few days, massage of the dorsal surface of the forearm being substituted for one minute after each injection.

Gradually the dosage of morphine was lowered by substituting sterile injections until, after 4 weeks, the amount was 2.5 grains.

At the end of 6 weeks, injections were discontinued.

There were no withdrawal symptoms.

In the second case, a woman of 38 who was getting 2 grains of morphine daily, the conditioning stimulus was a tuning fork held close to the ear, and the patient counted until the vibration ceased.

Replacement of the morphine by sterile water was accomplished without withdrawal symptoms in 10 days, the vibrations of the tuning fork apparently substituting for the morphine.

Should Rubenstein's findings be duplicated by other therapists, this would constitute a most revolutionary approach to the treatment of drug addiction.

The report lacks detail regarding any conversation between the patients and therapist so that it is difficult to judge what other factors besides conditioning entered into the picture.

Experimental corroboration for counterconditioning is reported in studies by Lazarus (1961) and Lang and Lazovik (1963).

Claimed for the desensitization techniques, in contrast to interpretive techniques, are quicker, more effective and more lasting elimination of phobic symptoms.

In the study by Lazarus patients with acrophobia, claustrophobia, and impotence were matched in pairs according to age, sex, form, and severity of phobic symptoms.

They were then randomly assigned to desensitization and interpretive treatments.

Patients delegated to group desensitization were exposed to items of a common stimulus hierarchy in a deep state of relaxation and were asked to signal by raising their hands when an item was disturbing.

Treatment was terminated when no stimulus item in the hierarchy upset any of the group members.

Traditional group psychotherapy was employed in the control group.

One month following therapy, the acrophobic patients were required to climb a fire escape 50 feet high and to take an elevator to the roof garden of an eight-story building from which they had to count the number of automobiles passing by for 2 minutes.

The claustrophobic patients were subjected to an equally rigorous task, being required to remain undisturbed for 5 minutes in a tiny enclosed space.

Of the 18 patients treated by group desensitization, 13 were completely recovered, as compared to only 2 out of 17 patients in the interpretive group.

www.freepsychotherapybooks.org 238 Of the unsuccessful cases in the latter body, 10 were then treated successfully by group desensitization within 10 sessions.

On follow-up, 80 percent of the successfully treated patients had experienced no return of fear.

Employing an apparatus to measure psychogalvanic responses as an index of autonomic reactivity, MacKay and Laverty (1963) demonstrated a progressive reduction of anxiety responses with

desensitization procedures.

Paul (1964) subjected four groups of college students with stage fright (1) to insight-oriented psychotherapy, (2) to an attention-placebo situation, (3) to counterconditioning associated with fantasies of public speaking, and (4) to mere assessment procedures (the control group). After 6 weeks, exposure to a stressful speech test and measurement of physiological variables, behavior dysfunctioning, and self-reported distress showed that all members of the first three groups showed improvement as compared to the control group, with the counterconditioning group rating far above the others.

Aversive Counterconditioning Pairing certain behavioral deviations with aversive stimuli sometimes has succeeded in controlling or abating them.

Among the earliest treatments of this type was the conditioned reflex therapy for alcoholism using a pharmacological nauseant (Lemere et al, 1942; Lemere & Voegtlin, 1950; Ruck, 1958; Miller et al, 1960).

Treatment along similar lines for homosexuality, transvestism, and fetishism has been reported by K.

Freund (1960), Glynn and Harper (1961), and A. Cooper (1963).

Because of the difficulty in controlling the effects of drugs, several experiments in aversive conditioning have employed faradic stimulation as an aversive stimulus.

This presentation of an unpleasant stimulus in close temporal relationship to an undesirable behavior, with the object of extinguishing it, is not as popular or successful as other forms of therapy.

The dedication to the behavior, conscious or unconscious, may be too great to abandon it.

Or a masochistic need for punishment may enable the patient to endure the painful consequences in order to appease a demanding sense of guilt.

Moreover, for some patients aversive methods serve to reproduce the parental precedent of punishment for www.freepsychotherapybooks.org 239 infractions; the patient will then rebel against the therapist or passively resist getting well.

In certain cases, however, when nothing else seems to work and an obnoxious habit or behavior must be controlled, aversive therapy may, surprisingly, be the only method to which a patient will respond.

Currently, several types of aversive schemes are occasionally used, principally those that follow the Pavlovian model and those that are patterned after the operant model.

In the former group of therapies are the conditioning of alcoholics with such nauseating drugs as Emetine and Antabuse and, more rarely, with electric shock.

Painful shock has also been used in the treatment of cases of self-injurious behavior as headbanging, selfbiting, and face-slapping in retarded children and of self-induced vomiting.

Aversive or punishing sequelae are employed less often than withdrawing positive rewards or reinforcements, such as shutting off a TV set during an argument between children over the choice of a program.

A more severe disciplinary action is to penalize the individual for reprehensible behavior by levying a fine.

Some therapists still use aversive stimulation for delivering an unpleasant stimulus like an electric shock from a small battery-operated unit or snapping a rubber band placed around their wrist whenever they indulge in certain behaviors they wish to control.

In cases of hair plucking and skin mutilation that have not responded to other methods, a painful stimulus may replace the masochistic need to torment oneself.

Rapid inhalation of cigarette smoke to a point where the mucous membrane hurts or burns for the purpose of overcoming the smoking habit is another example of aversive control.

The use of an alerting system to eliminate bed wetting and of delayed auditory feedback in stuttering has elements of both Pavlovian and operant methods.

Compulsive overeating, gambling, and sexual deviations (fetishism, exhibitionism, voyeurism) and obsessive-compulsive symptoms have also been treated with aversive control methods, with undocumented claims of success.

A substantial literature has accumulated detailing aversive techniques for the reversal of homosexuality in cases eager to change to heterosexuality (Freund K, 1960; Glynn & Harper, 1961; Cooper A, 1963).

Virtually all behavior therapists now question the ethics involved in using aversive measures to treat www.freepsychotherapybooks.org 240 severe sexual deviations, even at the request of the patient.

The emphasis today is on positive treatment strategies, cognitive involvement, and sophisticated evaluation of complex situations, leading to multimodel interventions.

While not strictly an aversive technique, use of a diary in which the patient simply charts the frequency of undesirable behaviors for which control is sought appears to lessen the incidence of such behaviors.

Extinction Procedures Exposure of the individual to graduated anxiety-provoking cues, and the mastery of such stimuli, are said to support an increased ability to tolerate and overcome certain avoidance reactions.

Thus a patient fearful of entering elevators may be encouraged to walk into small rooms with the door open, then closed, following which the size of the room used is decreased, until the patient is able to tolerate remaining in a closet for a relatively long period.

Thereafter, entering an elevator and stepping out before the door closes, then going up one flight, increasing the trip floor by floor, may enable the individual to extinguish the elevator fear.

Unfortunately, this practice by itself has not proved to be altogether successful.

Patients refuse to follow suggestions for a variety of reasons, including the secondary gain dividends they derive from their illness.

Nevertheless, stratagems can be devised, tailor-made for each patient, which may on occasion prove successful (Herzberg, 1941; Saul et al, 1946; Kimble & Kendall, 1953; Walton & Mather, 1963a&b).

Psychotherapeutic interviewing also embodies gradual mastery of anxiety.

At first patients respond with anxiety to certain content, but repetition with increase of the depth of probing, along with permissive responses from the therapist, gradually extinguishes aversive responses as measured by the galvanic skin reflex (Dittes, 1957a, b), generalizing to related forms of behavior (Dollard & Miller, 1950).

In general, extinction procedures require frequent, even massive exposure to extinction trials with not enough interval between trials to build up resistance (Edmonson & Amsel, 1954; Calvin et al, 1956).

Because behavior deficits and neurotic symptoms often become reinforced when they break out against the will of the patient, certain procedures have been introduced www.freepsychotherapybooks.org 241 that purposefully and deliberately encourage the patient to engage in the disturbed behavior or to produce a symptom in the absence of the usual reinforcement contingencies (Dunlap, 1932; Lehner, 1954).

Thus, if an individual's hand shakes while lifting a glass or holding a fork resulting in avoidance of eating with people, he or she is requested to eat or drink in the presence of the therapist and ordered to produce shaking in as exaggerated a form as possible.

If he or she has a tic, he or she is encouraged to practice reproducing it voluntarily, then to engage in prolonged rest.

This is claimed as the best method of extinguishing tics (Yates, 1958).

Stuttering, too, is often benefited by the stutterer admitting to the speech defect to others and calculatedly trying to bring it on, or at least not struggling to hold it back (Fishman, 1937; Rutherford, 1940; Meisner, 1946; Sheehan, 1951; Sheehan & Voas, 1957).

Very often deviant behavior is reinforced by the reaction of individuals in the environment. Schedules in which the person is not reinforced for this behavior sometimes lead to its extinction.

Thus, in children, screaming spells, tantrums, refusal to eat, and other forms of misbehavior calculated to attract attention, positive or negative, may be handled by ignoring such performances (Williams C, 1959).

These principles have also been adopted in psychotics with some positive results (Ayllon & Michael, 1959; Ayllon & Houghton, 1962).

Operant Conditioning Operant behavioral approaches offer a prolific group of behavior change methodologies, as well as guidelines for their evaluation.

A contingency relationship exists between operants and the environmental events that follow them.

Thus, behavioral responses may be set up, accelerated, and strengthened (reinforced) or diminished and eliminated (extinguished) by their succeeding environmental consequences.

In applying operant methods, schedules of positive or aversive reinforcement are developed at fixed or variable intervals or ratios, which will gradually shape the desired behavior.

This design has been used with variable success for overcoming behavioral deficits, eliminating maladaptive activities, and continuing therapeutic gains.

Success is not always possible for www.freepsychotherapybooks.org 242 the same reasons as in any other therapy: lack of skill in the therapist, secondary gains that reward illness, inner conflictual resistances that obstruct progress, a masochistic need that supports suffering, anxiety that accompanies the achievement of health, and transference reactions that sidetrack therapeutic aims.

Operant conditioning probably plays a part in all therapies, in that the therapist reinforces certain verbal and behavioral responses that are in accord with his or her theoretical convictions and the goals toward which treatment is directed.

A vast bibliography on operant conditioning is available.

A brief review is contained in the article by Karoly (1980).

Behavior deficits or behavioral impoverishment, in which traits that make for a productive adjustment are in default, may sometimes be overcome by developing adaptive responses through operant conditioning.

Dramatic examples of what can be done with autistic children who are dissociated from reality and are unable to engage in gratifying interpersonal relationships have been reported by Goldfarb (1943), Freud and Burlingham (1944), Ayllon and Michael (1959), Lindsley (1960), Ferster (1961), Ferster and De Myer (1961), and Gerwitz (1961).

As Bandura (1965b) has pointed out, the existence of inadequate social-reinforcer systems inhibit the use of positive attention and approval in these sicker patients.

Indeed, other human beings have weak or negative reinforcing value for such patients (Bandura & Walters, 1959; Cairns, 1961; Ferster, 1961).

To develop adequate behavior repertoires, ingenious schemes of operant conditioning have been elaborated.

Selection of adequate primary reinforcers that are contingent on the execution of desired behavioral sequences and proper programming of rewards without setting criteria for initial reinforcement too high often result in a gradual shaping of behavior toward an adaptive objective.

For instance, in the study of King et al.

(1960) on withdrawn schizophrenics, the performance of simple motor tasks was rewarded with verbal approval, pictures, candy, and cigarettes.

Gradually these bounties were made contingent on more complicated behavior, such as communication and cooperation with the therapist and other patients in solving simple problems. The result was an emergence from their private worlds toward relationships with others.

Working with nursery school children, Baer et al.

(1963) concluded that attention www.freepsychotherapybooks.org 243 paid to withdrawn children (the conventional approach) merely tended to reinforce detached behavior.

The investigators consequently avoided attending to this behavior when it occurred, but immediately rewarded the appearance of any evidence of social interaction.

Social responsiveness immediately increased.

To test this further, they again consoled and paid attention to the same children when they relapsed into solitary play, with the result that withdrawal tendencies rapidly returned.

Reversing this process for the second time and rewarding social interaction, they readily brought the children back to cooperative relationships.

As soon as patterns were consolidated, the reinforcers were slowly withdrawn, reinforcement being made contingent on the usual rewards of everyday life.

Follow-up studies confirmed the stability of the acquired patterns.

Other observers have validated the efficacy of these procedures in both behavior disorders and behavior deficits (Allen E, et al, 1964; Wolf M, et al, 1964).

The use of operant conditioning in unmotivated patients, such as delinquents, has resulted in the elimination of some antisocial patterns.

When "hard-core" delinquents were compensated with money, candy, and cigarettes for their service as subjects in talking about themselves and their ideas, constructive activities soon were observed and their delinquent behavior diminished (Slack, 1960; Schwitzgebel & Kolb, 1964).

Response-correlated aversive stimulation is an example of how operant conditioning may be employed.

Here unacceptable forms of behavior result in removal of positive reinforcers (such as music) or the delivery to the individual of a disagreeable stimulus (faradic electrical stimulation) (Liversedge & Sylvester, 1955; Goldiamond, 1965).

Results depend on the intensity, frequency, and temporal sequence of the stimulus in relation to the response to be eliminated (Church, 1963).

By manipulating the social consequences of certain forms of behavior, modifications in behavior may be secured.

Thus Wolf et al.

(1964) successfully treated a refractory 3-year-old child whose violent temper tantrums could not be controlled with drugs and physical restraint simply by removing him to another room until his tantrums ceased.

In some compulsive-obsessive patients, unapproachable by conventional methods, who are torturing themselves with destructive thoughts and impulses, success had been obtained by use of a small toy (a battery www.freepsychotherapybooks.org 244 operated shocking apparatus, activated by closing a switch when the container of a card box is opened) that provides the aversive stimulation.

The patients, engaging in practice sessions at home, deliberately bring on their fantasies, then turn on the current until the fantasies disappear.

Eventually some are able to divert their thinking from obsessional concerns.

Bandura (1962b) aptly cautions that aversive stimuli may through classical conditioning serve

to motivate and reinforce undesirable patterns.

Social Learning through Identification (Modeling) Modification of social responses through patterning of the self after observed models has been described as a usual means of acquiring new behavior repertoires and of changing existing maladaptive ones (Bandura, 1962a, 1965a, 1977; Bandura & McDonald, 1963; Bandura & Walters, 1963; Rachman & Wilson, 1980) By perceiving the behavior of the models, including associated rewards or punishments for this behavior, marked changes in the observer may be scored in terms of (1) moral judgments (Bandura & McDonald, 1963), (2) control of aggression (Bandura et al, 1961, 1963a, b; Feshbach, 1961; Walters & Llewellyn, 1963; Berkowitz, 1964; Bandura, 1965b), (3) tendencies toward violation of prohibition (Blake, 1958; Walters et al, 1963), and (4) various other responses (Bandura & Huston, 1961; Bandura et al, 1963c).

Conditioned emotional reactions may be acquired by observing pain and fear in models (Berger S, 1962; Bandura & Rosenthal, 1965).

Thus, Chittenden (1942) exposed hyperaggressive children to a series of plays in which dolls exhibited alternating aggressive and then cooperative activity.

The consequences of aggressive behavior (two dolls fighting over a wagon) were shown to be destructive (the breaking of the wagon with unhappiness in the children).

On the other hand, constructive interaction resulted in the boys enjoying themselves (taking turns playing in the wagon).

An impressive decrease in aggression resulted in the audience in comparison with a matched control group.

Other examples of modeling procedures have been described by Levy (1939), Slavson (1950), and Kelly (1955), who essentially assign to the therapist the task of executing in real life or symbolically the desired behavior.

www.freepsychotherapybooks.org 245 The efficacy of the therapist in serving as a model for the patient will depend upon the role the therapist plays and how this is conceptualized by the patient.

If in a position to mete out rewards or occupy a prestige role, the therapist will have the greatest chance of being accepted for the desired modeling (Lippitt et al, 1952; Bandura & Huston, 1961; Bandura et al, 1963c).

Once social response patterns have been obtained, they may be secured by reinforcers.

Cognitive Behavior Therapy Emphasis on cognitive processes represents a shift from stimulus-response and drive models to the dynamics of systems and subsystems of thought. Psychotherapy, following this paradigm, is organized around the direct influencing of thought systems and the interpretation of events.

Behaviorally oriented methods that have taken on a cognitive dimension have been classified under the label of "cognitive behavior therapy." The distinctive quality of these techniques is that they do not attempt to force ideas on the patient but rather seek cooperation by providing the patient with graded tasks and assignments calculated to instill new ways of thinking.

Thus, through instruction and modeling patients are taught to replace negative thoughts with thoughts that are more relevant to a proper adjustment (Meichenbaum & Cameron, 1974).

Messages may be written out and given the patient for study and reflection.

The aim is to soften the projected consequences of worrisome or destructive ideas.

Eventually, it is hoped, negative self-statements and irrelevant cognitions that provoke untoward behavior may be eliminated.

Disturbed thoughts and ideas are believed capable of setting off chain reactions ranging from emotional outbursts to behavioral aberrations to physiological upheavals.

Such thoughts and ideas, which we may call cognitions, are often the product of faulty belief

systems acquired through improper upbringing and false cultural values.

The end result is interference with a satisfactory personal and social adjustment.

Therapeutic efforts are therefore directed toward the forthright and immediate correction of erroneous ideas.

www.freepsychotherapybooks.org 246 In contrast to dynamic therapy, which tends to alter cognitions through insight into how past conditionings mold attitudes and behavior, cognitive therapy deals directly with present-day thoughts, irrational assumptions, destructive self-statements, and selfdefeating ideas.

Their influence on feelings and behavior is explored with the object of regulating a more harmonious adjustment and helping the patient reduce or eliminate anxiety, depression, anger, and accompanying physiological residues.

The original work in this field was done by Ellis (1962).

Several elaborations of Ellis's ideas have occurred, for example, the work of Meichenbaum and Cameron (1974), who have concentrated on the determining effect of the patient's negative self-statements and other irrelevant cognitives on behavior.

A.

Beck (1976) and Rush (1978) have also contributed substantially to the field through their stress on faulty thinking patterns, particularly in depressive states.

When rudiments of adaptive skills are present and anxiety is not too paralyzing, the individual with proper therapy along cognitive lines may be able in a relatively brief period to reorganize his or her thinking skills and to find alternative, constructive solutions for difficulties in living.

Intervention programs of this type have been designed for application in a variety of clinical and educational settings (Spivack et al, 1976).

Behavioral Medicine Behavioral medicine is a term used to describe health and illness as they relate to the biological and social sciences, as well as the techniques that lead to diagnosis, prevention, treatment, and rehabilitation.

A large number of publications have attempted to define this important but rather amorphous behavioral field (Berk, 1973; Collins, 1981; Davidson & Davidson, 1980; Doleys et al, 1982; Epstein & Cluss, 1982; Melamed & Siegel, 1980; Miller N, 1983; Pomerleau & Brady, 1979; Swartz & Weiss, 1978; West & Stein, 1982; Williams & Gentry, 1977).

A number of subspecialties have appeared that attempt to divide the field into pediatric, adolescent, gerontologic, ophthalmologic, urologic, dental, and other areas of behavioral medicine, some authorities consider psychological methods that are used adjunctive to medical techniques, as in the treatment of asthma, insomnia, www.freepsychotherapybooks.org 247 obesity, pain, cardiovascular disease, and habit disorders (smoking, overeating), within the parameters of the field.

Hypertension, Raynaud's disease, arrhythmias, and coronary artery disease have been shown to respond to methods related to behavioral medicine (Agras & Jacob, 1979; Benson et al, 1975; Brady et al, 1974; Berk, 1973).

An excellent bibliography of behavioral approaches to pain, asthma, insomnia, headache, alcoholism, and other problems may be found in the article by Lazarus (1984).

Conditioning techniques have been used to modify behavior and to control physical symptoms in every age category.

In children, problems ranging from nail biting to colic have been addressed.

Parents have sometimes been trained in behavior management interventions as primary therapists or adjunctive helpers (Patterson & Gullion, 1968).

In adults, there is scarcely a syndrome or symptom complex that has not been approached with behavioral methods.

In those elderly patients in whom the deficits of aging have been amplified by environmental impoverishment and relative lack of reinforcements (Patterson, 1982), behavior therapy can help restore a good deal of constructive functioning (Berger & Rose, 1977).

It may be valuable in terminal care problems.

Behavioral Prostheses Several mechanical or electronic devices have been developed that can be carried by patients to help them acquire more adaptive behavioral patterns.

Severe stuttering, for example, has been helped by an auditory metronome with a volume and rate control that the patient wears behind the ear like a hearing aid and that delivers rhythmic beats, permitting a pacing of speech and leading to greater fluency (Burns & Brady, 1980).

A coordinated behavioral program, which may be used unaided by the prosthesis, is employed to decondition anxiety related to speaking (Brady, 1985).

Social Skills Training Behavioral approaches are indispensable in the rehabilitation of persons who have failed to acquire or have lost social and self-care skills because of faulty upbringing or the development of psychiatric illness.

Deficits and ineptitudes in human relationships and www.freepsychotherapybooks.org 248 communicative skills are reflected in awkwardness, poor impulse control, and offensive habits and behavior, resulting in rejection and social isolation.

Programs designed to overcome social and self-care deficits have been developed around behavior therapy principles and are used primarily in day hospitals and rehabilitation centers, as well as private practice.

Individual and group therapy, using role playing and the modeling of appropriate behavior in simulated social situations, have proven singularly effective.

Homework assignments are an important part of the program to encourage the generalization of learnings to the specific environment in which the individual will function (Hersen & Bellack, 1976; Trower et al, 1978; Wallace et al, 1980).

The Practice of Behavior Therapy No two behavior therapists function alike, but all have a common respect for the methodology of the behavioral sciences and an emphasis on some form of stimulus response or social learning theory as a potential but not exclusive conceptual idea.

It is this focus that characterizes modern behavior therapy, together with its emphasis on accountability and openness to scrutiny from within and without.

The debatable clinical advantages and short-term therapeutic "successes" in the long run are probably of less significance.

As in traditional psychotherapy, considerable flexibility is exercised in the stratagems that are employed, which generally incorporate both interviewing and conditioning tactics.

Therapy is usually started with a "behavioral analysis" to describe the maladaptive behavior pattern in objective and explicit terms, to review environmental and other variables that initiate and sustain the behavior, and to outline the tentative treatment plan.

Each step of the treatment as it proceeds is explained to the patient, including what will be expected of him or her.

"the patient's behavior toward the therapist is examined only when it points up important "maladaptive responses" (Storow, 1965).

In the opening phases of treatment, the main tasks are diagnosis, structuring of the therapeutic situation, designation of goals, and permitting the patient to "adapt" to the experimental situation by avoiding sensitive areas.

The circumstances that led to www.freepsychotherapybooks.org 249 treatment are reviewed, with discussion of the patient's general reactions to these.

The present is considered rather than the past.

Patterns to be changed and the methods to be used in changing them are delineated.

To become an adequate agency of reinforcement, it must be established that the therapist is a

source of rewards.

This is done by developing a relationship with the patient and giving him or her some immediate relief through medication or practical assistance with a pressing problem.

Once the therapist understands the causal relationship between the patient's behavior and other variables, the true therapeutic effort begins when the therapist uses available stimuli to extinguish the maladaptive behavior (Meehl, 1962) and to strengthen adaptive responses.

New patterns are most effectively shaped through the principle of "successive approximations" (Skinner, 1953), during which the therapist expects only small steps of improvement, never pushing the patient beyond his or her capacities.

"The basic principle is to start with some response pattern remotely similar to the missing adaptive behavior and to shape the new behavior by differentially reinforcing new responses as they approach the goal." For instance, if a woman is fearful of aggressive behavior, she is encouraged to grouse at her husband or coworkers when she feels like it, and she is complimented when she reports success.

She gradually is encouraged to stand her ground when criticized and to assert herself with authority, always being sure that her behavior is not destructively designed.

Should the patient be unable to express herself openly, fantasy and role-playing techniques are employed, such as those suggested by Cameron (1951).

For instance, she can first imagine herself in a certain role; then she may act it out with the therapist, who plays the part of the threatening figure.

Should the reinforcements gained from her environment for her behavior prove unsubstantial, tangible rewards may be given, like those described by I.

Stevenson (1962), such as special attention being shown her outside of the interview or reduction or cancellation of the fee for one or more sessions.

Leads toward social reinforcement in difficult problems have been suggested by R.

Lundin (1961), Storrow (1962), Staats and Staats (1963).

The quickest way of overcoming a response is "to condition an incompatible response in its place" in the form of counterconditioning (Kimble, 1961; Lundin, 1961).

www.freepsychotherapybooks.org 250 Thus, an adaptive response may be chosen to replace an avoidance response.

Verbal selfstimulation may be helpful, words and thoughts serving as conditioned stimuli and acting as reinforcers.

First "the erroneous assumptions underlying the patient's anxiety and avoidance behavior" are pointed out, "arguing with him about this, if necessary." A cognitive behavioral approach may be indispensable.

The patient is then instructed "to bombard himself with opposing thoughts when he next confronts the threatening situation." This approach is similar to that of Albert Ellis (1962).

Mild punishment may be used as a means of suppressing maladaptive behavior.

This may be in the form of verbal disapproval or may consist of such methods as those suggested by Stevenson (1962), for example, "increasing the fee for one or more interviews or postponing an appointment when a patient fails to complete an assigned task." In the closing phases of treatment, liberal approval during the interview is sometimes yielded in favor of rewards available in the environment.

Advantages that will accrue to the patient as a result of the new behavior are pointed out.

Little difficulty is usually experienced in terminating therapy.

HOW EFFECTIVE IS BEHAVIOR THERAPY?

COMPARISON WITH PSYCHOANALYSIS Some attempts have been made to compare the results of behavior therapy with those of conventional therapy.

Reporting on three series of unselected patients (psychotics and psychopaths were, however,

excluded from treatment), Wolpe (1958) states that of 210 neurotic patients, 90 percent were cured or much improved after an average of 30 therapeutic interviews.

This figure is topped by a claim by Hussain (1965) of 95.2 percent in his treatment of 105 patients.

Lazarus (1961) argues that while insight therapy brought satisfactory results in 1 out of 15 cases suffering from phobic reactions, behavior therapy cured 13 out of 18 cases.

In a later study of 321 cases, he found that 78 percent had achieved a strikingly good outcome.

Follow-up observations show that the results by conditioning methods are durable.

www.freepsychotherapybooks.org 251 On the basis of these findings, in the formative years,

behavior therapists have launched a frontal attack on both the precepts and results of psychoanalysis, branding it as an inferior treatment method that may contain elements that not only do not foster but actually retard improvement or cure.

Among the charges are the following: 1.

Psychoanalytic treatment is founded on the unsubstantiated premise that emotional problems are the product of repression of unconscious conflicts.

The objective in cure, therefore, is to resolve resistances, thus restoring repudiated impulses to consciousness.

2.

Statistical reports prove that results with psychoanalytic approaches are inferior to those of other forms of therapy, and particularly to behavior therapy.

3.

On any comparison study, behavior therapy scores higher than other types of treatment; indeed, conditioning frequently overcome neuroses that have been treated for years unsuccessfully through psychoanalysis.

4.

The evidence justifies substituting behavior therapy for psychoanalysis in the training of therapists.

5.

It confirms a community interest in behavior therapy, since it permits the therapist to see large numbers of cases over a short period of time, producing "a higher proportion of lasting recoveries from the distress and disability of neurosis than does psychoanalysis," at a fraction of the cost.

Insight Versus Reinforcement Behavior therapy conceives of neuroses as residues of faulty learning and conditioning that are amenable to tactics of relearning and reconditioning.

In such retraining, "insight," as we conventionally conceive of it, plays a minor role or none at all.

There is little evidence, contend behavior therapists, that "insights" gained during interview psychotherapy or psychoanalysis have any influence in modifying social behavior.

For the behavior therapist the term "insight," if it is used at all, pertains to the appreciation gained by the patient of the relevant contingencies in his or her life and their consequences as well as the consequences of responding to available options.

Generally, www.freepsychotherapybooks.org 252 "insights" are the regurgitated sentiments of the therapist, rather than true discoveries of the patient.

Studies of verbal conditioning show that almost any response can be elicited from a subject under interview by positive reinforcement from the interviewer and that it may be reduced by withholding reinforcement (Salzinger, 1959; Krasner, 1958, 1962).

The content of a patient's stream of thought is thus definitively influenced by suggestive probings on the part of the therapist (Bandura et al, 1960; Murray, 1956).

It is perhaps for this reason that patients are constantly feeding back to the therapist the

latter's psychodynamic schemes (Marmor, 1962).

Rather than being a precursor to change, insight is often a consequence of change, during which, due to a strengthening of the personality, certain ideas are allowed access to consciousness (Alexander & French, 1946).

Awareness of the responses made, and of the reinforcements, expedites learning.

Awareness alone, however, does not guarantee behavioral change.

Rather than waste time exploring hypothetical unconscious conflicts that turn out merely to reflect the therapist's theoretical predilections, is it not more practical as well as scientific to apply oneself to modifying responses through methods that have proved themselves efficacious?

Indeed, where patients improve with insight therapies, it is probable that they do so not because of cognitive factors but because they have been subjected by the therapist to differential reinforcements, to counterconditioning, to extinction and to modeling—in other words to principles of learning rather than to the divulgence of unconscious content.

Is it not then reasonable that insight therapists give up their esoteric rituals and forays into the unconscious and deliberately apply themselves to effective social learning procedures (Bandura, 1965b)?

Criticism of Behavior Therapy Modern clinical and experimental research has contributed to the sophistication of behavioral interventions and has extended their usefulness for dealing with many syndromes, ranging from adjustment and habit disorders, to neurotic symptomatology (e.g., phobias, obsessions, compulsions, and depressions), to problems of retardates and psychotics. Questions continue to be asked about the utilities and dangers of behavior therapy, many based on misconceptions, for example, the ideas that the method is too www.freepsychotherapybooks.org 253 authoritarian, coercive, controlling, and punitive; that behavior therapists avoid historytaking and focus extensive attention on data-gathering; that the removal of symptoms often results in symptom substitution or in only temporary benefits; and that the therapist-patient relationship is not considered very important.

Accusations are still levied at behavior therapists to the effect that they disregard such nonmeasurable aspects of inner experience as feelings and fantasies, and even that some therapists are so tied to a simple stimulus-response ideology that they consider the human brain “an irrelevant and unnecessary intervening variable.” These ideas are largely erroneous and contribute to the existing climate of misunderstanding.

Although analysts and behavior therapists act as if they were more tolerant of each other's ideas, there is still a good deal of distance and distrust between them that, one hopes, time and constructive dialogue will resolve.

Franks (1984) believes that it is important to recognize the pervasive differences between behavior therapy and the Freudian system.

Fundamental philosophical, conceptual, methodological, and practical distinctions make meaningful comparisons (other than for certain circumscribed situations) virtually impossible. For example, the two models have incompatible notions about the nature of acceptable data and the goals of therapy, and both the criteria and process of outcome evaluation are different. In the early days of behavior therapy, rivalries resulted in potentially destructive thinking in terms of “better” or “worse.” The prevailing climate for many individuals in either “camp” seems now to be that the two systems are fundamentally incompatible and that in the long run advancement will best be served by encouraging each to work within its own framework.

A perhaps smaller but vociferous and responsible group believes that some form of integration is both possible and beginning to occur (see Goldfried, 1982).

A great deal of concern has recently been expressed about the ethics of behavior modification and the infringement of rights of individuals on whom behavior change is practiced.

This is particularly directed at work with populations in institutions and prisons where motivation for change may be lacking.

It is also pointed out that follow-up studies are not particularly impressive regarding the permanency of change once the individual leaves the milieu in which appropriate reinforcements exist.

These criticisms www.freepsychotherapybooks.org 254 are, however, not unique for behavior modification and can equally be applied to all mental health programs.

It is assumed that professionals are sufficiently ethical to weigh advantages against risks, to gain informal consent if possible for programs, and to impose proper safeguards so that individual rights are not abrogated.

Monitoring routines by a review committee, supplemented by reports to guardians or advocates of the person whose behavior is to be modified, have been proposed.

Preoccupation with the issue of control has been overemphasized.

As W.

Brown et al.

(1975) have stated, "On the whole, the goal of behavior modification, as generally practiced, is not to force people to conform or to behave in some mindless, automaton-like way.

Rather, the goals generally include providing new skills and individual options and developing creativity and spontaneity." Contemporary behavior therapists are highly sensitive to ethical considerations and the potential for infringement upon human dignity, rights, and privacy.

Historians and behavioral architects stress that the saga of behavior therapy is an evolution from simplistic beginnings to the present form of the discipline (Kazdin, 1978).

One major strength of behavior therapy is its willingness to accept modification in the light of new data and thinking and its sensitivity to personal and social needs.

Effective behavior therapists recognize both their limitations and the need to grapple with scientific, ethical, and philosophical issues (Erwin, 1978).

One of the drawbacks of any symptom-oriented short-term method of therapy, including behavior therapy, contend dynamically oriented therapists, is that there is little or no time spent on evaluating the individual's personality needs, and particularly the meaning to the patient of his or her symptoms.

To impose behavior modification on a patient without thorough cooperation and the resolution of resistances to change will impose, in the least, barriers to ready progress and in some cases will inflict serious dangers.

This is not to say that alterations may not be forthcoming, as one may witness in the token economies practiced in a state institutions with unmotivated patients.

But when a symptom serves as a defense, initial improvements with forced behavioral techniques, unless they resolve the need for the defense, are apt to be short-lived and may even precipitate anxiety, with further disabling compensations.

Admittedly, this is not a usual phenomenon, but it does occur as Bruch (1974b) reported in her follow-up studies of the www.freepsychotherapybooks.org 255 effect of behavior therapy on a group of patients with anorexia nervosa.

The clinician will obviously be at an advantage if he or she recognizes that an individual's needs for autonomy and self-determination are sacrosanct and that imposing a regimen on any unwilling patient poses hazards that have to be evaluated carefully before undertaking treatment.

It is here that dynamically oriented therapists have an advantage over their colleagues who dissociate themselves from approaches that acknowledge the determining power of existing personality drive and conflicts.

There is little question that the techniques of behavior therapy constitute important additions

to the armamentarium of the psychotherapist.

Employed with skill, they offer effective forms of treatment for the relief of certain inhibitory coping mechanisms and symptoms, such as phobias.

They are applicable to a broad spectrum of the population, including less verbal and less educated patients.

An improved total adjustment often comes about as a by-product of the control or resolution of symptoms.

Proficiency in the use of behavioral therapy techniques may be acquired without extensive or expensive training, an advantage in the face of the current shortage of psychotherapists.

Fears and other symptoms resolved through behavior therapy may be permanently extinguished and, instead of symptom substitution, may be followed by constructive influence on many other elements of the personality.

Behavior therapists strongly stress, however, that proficiency in newly acquired behavioral techniques does not make one a behavior therapist; nor does it mean one is practicing behavior therapy.

Behavior therapy is a conceptual and methodological approach that is both complex and time consuming to master.

Techniques may be readily acquired and easily applied by anyone, but this is not behavior therapy.

On the debit side of the ledger we find that, in their zeal, unsophisticated behavior therapists have tended (in the past but less so now) to downgrade other forms of treatment, claiming, first, that theirs is a more scientifically based therapy, grounded in substantial learning theory rather than esoteric personality theory, and, second, that results are statistically better than those reported by therapists of other schools.

Such signal successes are credited to the singular corrective powers of conditioning.

www.freepsychotherapybooks.org 256 Actually, as modern behavior therapy takes pains to stress (Franks & Wilson, 1975), there is little basis for any autocratic assumptions about learning theory.

It possesses no special magic, nor does it occupy a preferred place in the vestibule of science.

Indeed, there is as much confusion about how complex learning takes place, and as much controversy about which of the prevailing learning theories is sovereign, as there is in the field of personality theory.

It is better to ascribe the effects of behavior therapy to the disciplined use of learning principles and the methodology of the behavioral scientist than to the dubious application of learning theory.

The beneficial effects scored by any of the psychotherapies are probably the consequence of the unlearning of old and the acquisition of new patterns.

Even psychoanalysis owes many of its gains to constructive relearning.

How learning principles enter into the methodologies, circuitously or by design, has been described elsewhere (Wolberg LR, 1966).

Learning principles are not the monopoly of any of the psychotherapeutic schools.

Nevertheless, it cannot be denied that behavior therapy is one of the few systems of therapy that explicitly endeavors to unravel and then systematically and consistently apply these principles of learning in the therapeutic situation.

Ideally, speculation and subjective intuition—while having their place as an occasional starting-off point—yield to evidence, data, controlled study, and the rigorous application as an ongoing process of the methodology of the behavioral scientist.

It is this that really gives behavior therapy its unique position and its advantage, if it be

an advantage, rather than any grandiose claims to extraordinary successes.

A well-deserved criticism is that early behavior therapists tended to regard conditioning as the cardinal if not exclusive agency in the therapeutic process.

Little attention was paid to the factor of spontaneous remission and to the non-specific benefits of the helping process, such as the placebo effect, the relationship dimension, emotional catharsis, suggestion, and group dynamics, which in themselves may account for a certain percentage of recoveries.

Nor was the enthusiasm with which therapists apply themselves, and the faith inspired in their patients, considered to be too significant.

Neglected also were the factors of transference and resistance that operate in all relationships in which one human being is influenced by another.

This, of course, is of primarily historical relevance at this stage.

Behavior therapy has matured since these stormy and dogmatic days of infancy and early adolescence.

At this stage it is essential to differentiate between the techniques of behavior therapy that any therapist may acquire and apply as the occasion demand, but which do not make the therapist into a behavior therapist, and the consistent application of a conceptual model or framework with which all problems are approached regardless of technique employed or maladaptive function to be altered.

Technical eclecticism (the use of any technique that has been empirically demonstrated to work) is justifiable and perhaps the hallmark of the good therapist.

Theoretical eclecticism—the switching from one conceptual framework or theoretical set of constructs to another, from one patient to another—leads the therapist to a conceptual disarray that is akin to philosophical schizophrenia.

Lazarus (1971) has elaborated on this important point.

A case described by J.

White (1964), as an illustration of the application of learning theory to the treatment of anorexia in a child, brings up some of the problems in accounting for therapeutic results.

A child who had become conditioned to eating while on her father's lap stopped eating regularly at the age of 5 when her father died.

Various devices were utilized to incite her appetite; however, her eating steadily deteriorated.

Therapy consisted of a reconditioning process, the therapist, then other members of the family playing the father's role.

Play therapy, four times weekly, was also employed.

It was soon apparent that the child, in her contrariness, was using food rejection as a way of frustrating her mother.

On one occasion when the child refused to stay in the playroom with both her mother and the psychologist, she was sent home without the cup of coffee promised her.

Thereafter, she was rewarded only when she did useful little jobs around the clinic.

In 5 months the patient began showing an interest in food.

Whether learning theory, relationship therapy, or pure suggestion operated here is quite pertinent to our concern as to what is important in the healing process.

The author has successfully treated a number of anorexic children with hypnosis, employing pure suggestive techniques.

In one instance cure took place after one session (Wolberg LR, 1965, p. 290).

www.freepsychotherapybooks.org 258 The most biting criticisms of behavior therapy are voiced by

adherents of psychodynamic models of psychotherapy, who consider behavior deviations the symptomatic manifestations of underlying pathological processes.

Such criticisms contend that the behavior therapist operates falsely on the assumption that it is possible, from the patient's behavior or expressed complaints, to discern all the variables that require correction.

Actually, the aspects of personality that require greatest alteration may be repressed or shielded by manifest defenses or by symptoms that mask more fundamental problems.

Applying oneself faithfully to the extinguishing of such symptoms may prove futile, unless one discloses and makes available to the patient the conflicts behind his or her symptoms.

This is not to say that many symptoms have not outlived their usefulness as defenses against anxiety, persisting as conditioned responses.

In such cases they will yield to tactics of symptom removal, such as are practiced in some forms of behavior therapy.

When they serve some anxiety-binding or pleasure-producing purpose in the psychic economy, however, their origins and function will have to be explored and exposed, and resistances to giving them up "worked through." Only then will conditioning procedures be effective.

To this, of course, the sophisticated behavior therapist would reply that there are no hard data demonstrating the need to introduce superfluous intermediary concepts, such as the unconscious or psyche.

They would also point out that behavior therapists go to great lengths to explore with the patient the whole range of presenting complaints and pertinent issues, some of which the patient may not be aware of.

For example, treating a lawyer by systematic desensitization for the presenting complaint of difficulty in speaking in public may be useless if the real or only partly recognized problem is fear of losing a case in court and of losing face.

It is this, among other things, that must be modified rather than the face-saving presenting complaint.

Another challenged assumption in early behavior therapy was that the therapist always can function as an adequate source of reinforcement—approval and rewards meted out by acting as positive reinforcer and disapproval as an aversive stimulus.

This is not always true.

When the patient has a problem in accepting the therapist as an authority, he or she will not be influenced too much by the therapist's approval or disapproval.

In patients who have a need to frustrate, compete with, defy, and fight authority, resistance www.freepsychotherapybooks.org 259 to commands and blandishments may obstruct reinforcement.

Moreover, an unconscious masochistic need in the patient may dictate suffering as a way of life.

Relinquishing a disturbing symptom may then constitute too much of a threat.

Where there is resistance to accepting social rewards for any reason, reinforcements from the environment will have little effect on the patient.

It is necessary in such cases to deal with transference projections, differentiating the therapist as a real person from the image of authority residual in the patient's past experiences.

To do this, reconstructive techniques may have to be employed.

Contemporary behavior therapy, then, is both interperson- and intraperson-oriented.

Emphasis is on those contingencies within the environment that modify the patient's life style.

For example, it is of little use to help an alcoholic reduce drinking or to change a basic life pattern in the clinic if there is return home to the same unfortunate set of contingencies that contributed to the distress (or foibles) in the first instance.

Behavior therapists have, therefore, developed techniques such as contingency contracting and cognitive restructuring for modification of the various systems within which the patient has to operate.

Such a systems approach necessitates concern with virtually every aspect of the patient's social, economic, biological, and physical environments.

Flexibility and ingenuity, coupled with scientific rigor and attention to detail, are essential, if hard to maintain, ingredients of this approach (Franks, 1974; Franks & Wilson, 1975; Lazarus, 1976).

At the same time, contemporary behavior therapy no longer neglects the "inner being." Cognition and self-control rather than direction from without are increasingly important in the behavioral approach (Mahoney, 1974).

Selfinstruction and self-monitoring, utilizing advances in behavioral biofeedback, are considered to be focal goals.

Lazarus (1976), with his acronym BASIC ID, provides an outstanding example of the direction in which forward-looking behavior therapy is going, a direction shown by the current interest in differential therapeutics (Frances et al, 1984).

Two major problem areas remain for this new breed of behavior therapist to contend with.

By definition, the strength of behavior therapy and its identity lies primarily in its unswerving adherence to the methodology and rigor of the behavioral scientist within a learning theory framework and its unique combination of this with the flexibility and acumen of the patient-oriented clinician.

The problem is that the more broad www.freepsychotherapybooks.org 260 spectrum or multimodal the behavior therapists become, the more they are in danger of losing the behaviorally oriented foundations upon which their system of therapy is founded.

From the patient's stance, this, of course, is irrelevant.

From the point of view of behavior therapy, which may, of course, be of little consequence in the long run, this is very important.

The other problem area—with which behavior therapists are contending more successfully (Franks, 1974; Franks & Wilson, 1975)—pertain to such vital and timely issues as ethics, accountability, licensing, and its public and its private images.

Now that behavior therapy is demonstrably viable and highly visible to both mental health professionals and the public at large (witness the not-altogether unmerited furor about aversive control and token economies in the prison system in the popular press), it is essential that such matters be given top priority along with matters of further research.

Behavior therapists insist that schedules of reinforcement rather than internal psychodynamic factors account for the genesis and continuance of disturbed behavioral patterns.

By removing positive reinforcers, deviant patterns are said to be reversible.

The origin of these reinforcement schedules may well lie in early parental conditioning and be determined in part by constitutional, biochemical, and even genetic variables.

This, all but the most radical of behavior therapists recognize, sets an inevitable limitation upon what reinforcement alone can accomplish.

Within these limiting factors, the emphasis is on the contingencies that maintain the behavior now.

Behavior therapy has developed techniques for working with these contingencies, whether they be operating primarily in the external environment or largely (or even exclusively) in the imagination of the patient.

The reinforcing agencies are sometimes not in the environment but in the residues of parental conditionings imprinted on the psyche that carry on the re-ward-punishing activities irrespective of reality.

The individual's superego may be a more powerful reinforcing agency than any external personage including the therapist.

Because the force and even existence of this intrapsychic body is usually unknown to the individual, the patient cannot deal with it until, through releasing measures, he or she becomes aware of its nature and manifestations.

Liberation of such forces from the unconscious is an www.freepsychotherapybooks.org 261 important objective in the insight therapies, such as psychoanalysis.

The fact that not all patients are cured by psychoanalysis and that behavior therapy yields symptomatic relief more readily does not invalidate psychoanalysis as a method.

Freud himself considered psychoanalysis as a technique applicable to a limited number of patients in whom the objective was extensive reconstruction of personality.

By rendering repressed impulses conscious through the resolution of resistances, by working through the infantile neurosis in the more readily resolvable transference neurosis, he attempted to release the arrested personality growth.

Rapid symptom relief was sacrificed in favor of future, more widespread change.

The goals in psychoanalysis are thus more extensive than those in behavior therapy, even though its application is limited to a selected group of patients who are properly motivated and who possess sufficient ego strength to endure the rigors of depth therapy.

It is recognized that release of repressed material may for a considerable period aggravate a patient's symptoms instead of relieving them.

It is acknowledged, too, that greater self-awareness does not guarantee that change will take place; it merely motivates the individual to approach life from a different perspective, a contingency that may or may not bear fruit.

Behavior therapists are themselves now among the forefront of clinicians prepared to accept and make constructive use of their failures.

In this respect the emergence of a searching volume entitled *Failure in Behavior Therapy* (Foa & Emmelkamp, 1983) is a signal event.

To attack a theory simply because therapeutic results are not always positive constitutes an error in logic.

For example we have some established theories of kidney functioning and pathology, and we try to design therapies in line with these theories.

The reality that we are unable to cure some types of kidney disease, or even to halt its steady retrogression, means that our methods are still too unrefined, not that our theories are necessarily wrong.

In many fields we know much more about what creates derangements than what to do about them. This is the case in the relatively new field of psychoanalysis.

Early behavior therapists, with the insecurity and messianic fervor that tends to accompany most new movements of significance, focused upon the "evils" of psychoanalysis rather than the "good" of behavior therapy.

Those forward-looking www.freepsychotherapybooks.org 262 behavior therapists acknowledge the futility and dangers of such a position.

They recognize that true science is open-minded and that techniques and modes of thinking that are acceptable strategies for either practice or valid research in one paradigm may not be viable in the other.

To the extent that this is recognized both psychodynamics and behavioral approaches are continuing and acquiring adult perspectives.

By the same token, psychoanalysts increasingly recognize that all known methods of extinction may not budge some habits and that this does not disprove conditioning as a basic process in learning.

It merely points up that we still do not possess sufficiently refined procedures of extinction and new response replacement.

The minimization of insight as an important force in behavior change constitutes a vital shortcoming of most behavior theories.

By insight we mean an awareness of forces within the self and of motives and values that sponsor maladjustment.

This is not to be confused with hypothetical, often mythological formulations that upheld a therapist's biased doctrines and credendas.

Unfortunately, a good deal of folklore is extant in the psychological field, particularly as it applies to personality theory and psychotherapeutic process.

Such "insight," fashioned by mimicry, has little other than a placebo effect and, as such, serves a limited purpose.

But the human person as a thinking animal operates on the basis of capacity for insight and understanding.

A recognition of the irrationality of impulses, a realization of their purpose and origins, and apprehension of one's reality situation structure a person's field and foster the extinguishing of old destructive patterns and the reinforcement of new adaptive ones.

It is hardly conceivable that any type of complex problem solving can take place in our entangled social environment without the mediation of insight.

Another point of controversy concerns the statistical reporting through which behavior therapists attempt to verify their claim as the most effective professionals in the psychotherapeutic field.

Statistical results reported by any school of psychotherapy must be looked upon with skepticism.

The evaluation of results of psychotherapy pose insuperable problems, including, among other things, definitions of change.

For example, in the reports of some groups utilizing insight approaches, symptom relief, for some www.freepsychotherapybooks.org 263 reason, is not considered to be an acceptable sign of improvement.

What is believed to be important is "character change," whatever this may mean in terms of orientation of the particular school.

As a short-term method geared toward symptom relief, behavior therapy probably can effectuate palliation in larger groups of patients than can psychoanalysis.

Whether it can surpass other forms of short-term therapy in speed, effectiveness, and permanence of improvement is a moot point.

A pioneer pilot project of Group Health Insurance, Inc., in which a large group of patients treated by 1200 participating psychiatrists, most of whom were analytically and long-term oriented, limited to 15 sessions per patient, revealed that cure or improvement of 76 percent was reported by the psychiatrists (Avnet, 1965).

A follow-up study after an average of 2 ½ years following termination revealed that 81 percent of patients reported sustained recovery or improvement.

With a caseload this size, and with psychiatrists employing a host of varied procedures, the rate of improvement is impressive.

The fact that a few skilled behavior therapists report a high percentage of improved patients may be ascribed to their skill and experience as well as to dedication to their method, rather than to the specific virtues of the method itself, effective as it appears to be.

nondirective and clientcentered therapists, transactional analysts, psychodramatists.

"Gestalt therapists," directive counselors, casework practitioners, and therapists of a great many other schools report improvement rates of approximately 80 percent and, in some cases, as

high as 95 percent.

When we consider the diversity of methods employed, the similarity of results is puzzling. Apparently there is release in all psychotherapies of processes of a healing nature that are not described or that are subordinated to the specific procedures heralded as the effective therapeutic agencies.

A properly controlled comparison series would, of course, help solve this question, but whether it is possible to arrange for such controls in psychotherapy, where the variables are so great that no two cases are even remotely alike, is doubtful.

Not all behavior therapists are convinced of the infallibility of their conditioning, counterconditioning, and extinction instruments.

While there is general agreement that phobic patients are helped significantly, there is some question as to whether other kinds of problems respond better to behavior therapy than to other treatment methods.

Franks www.freepsychotherapybooks.org 264 (1965), in summing up many reports, stated what a good number of experienced clinicians are now emphasizing, namely, that we do not yet know for whom or when behavior therapy had best be applied or whether or not it is shorter and the remission rate in its employment lower than that of conventional therapy.

He commented on the potential advantages of behavior therapy in its being more goal-directed toward specific symptoms and more adaptable at all levels of intellectual and linguistic sophistication than psychoanalytic therapy.

Selection of patients, therefore, need not be so discriminating.

A study by E.

J.

Ends and C.

W.

Page (1957), who employed group therapy based on learning theory on alcoholics, concludes that this approach was not only not helpful but deleterious, in contrast with approaches that employed client-centered and psychoanalytically oriented approaches.

What is lacking in all the statistical and research reports is a description of the quality of the agencies administering treatment, their education, experience, sophistication, bias, and, above all, their personality structures—their capacity to relate, to empathize, to understand, and to control and utilize countertransference to good advantage.

These ingredients are probably much more important to results in psychotherapy than are the theories or some of the methods that are being practiced.

Actually, there is little to be gained from the polemic that is taking place between the groups deifying insight and those worshiping behavior therapy.

Psychoanalytically oriented therapists, while deriding behavioral approaches as superficial, are constantly though unwittingly utilizing learning principles, including extinction and counterconditioning.

Deliberate avoidance of behavioral techniques in patients who cannot benefit from insight approaches alone constitutes an unfortunate defection.

On the other hand, not all of the forces operating in any interpersonal relationship, including psychotherapy, are immediately identifiable and manipulatable.

The refusal to acknowledge that a patient may unrealistically project negative attitudes that have nothing to do with the immediate situation but owe their force to generalizations toward authority, the original stimulus of which dates back in the patient's life history, will handicap the behavior therapist greatly.

Similarly, refusal to believe that reinforcements that sustain symptoms are internally inspired by such disguised rewards as sexualization www.freepsychotherapybooks.org 265 and masochism

will confound the therapist, since he or she will have done everything obvious to foster an extinction that simply does not come to pass.

Cognizance of such forces may not always improve the results obtained with behavior therapy, but they enable us better to explain the phenomena taking place within the conditioning situation.

The willingness of behavior therapists to abandon their defensiveness, to share the throne of empiricism with other therapists whom they consider unscientifically steeped in the mystique of their arts, and to amalgamate their findings with those of schools different than their own could lead to an enrichment of psychotherapy as a whole.

Out of this unity may come the wedding of the insight and behavior therapies, dynamic explorations helping the individual to structure and refine his or her learning field, and conditioning techniques expediting the unlearning of neurotic and the incorporation of healthy patterns.

A dynamic orientation to behavioral and other problem-solving approaches is taken not for the purpose of expanding the goals of treatment beyond symptom cure or problem solving but for dealing with some of the most powerful resistances that impede progress.

These have to do with unconscious needs to perpetuate a childish dependent adaptation, to assuage guilt through masochistic self-punishment, and to project onto the therapist needs and attitudes originating in early relationships with important parental and sibling figures (transference).

These drives can distract the patient from aims congenial with the objectives of treatment and make a shambles out of the most dedicated and skilled efforts of the therapist.

In part, behavioral therapists have come to recognize the importance of cognitive factors that operate as resistance and in some cases have adopted recent approaches used in cognitive behavior therapy.

What is still lacking is recognition of the importance of unconscious conflict as one of the significant determinants of behavior.

Of course, a dynamic orientation is not always necessary, since considerable numbers of patients do not resist behavioral and other symptom-oriented approaches.

For those who do not respond, however, the understanding of unconscious motivational deterrents, as well as application of this understanding toward their resolution, can mean the difference www.freepsychotherapybooks.org 266 between success and failure.

COGNITIVE LEARNING Cognitive learning approaches are designed to deal as rapidly as possible with symptoms and adjustment problems without probing for causes or attempting diagnosis.

This is done by assigning to the patient (who is called a “student” since the medical model is assiduously avoided) a series of problem-solving activities calculated to alter customary attitudinal and behavior patterns.

The student and therapist (who is called the “teacher” or “instructor”) jointly agree on goals that have a reasonable chance for alteration, and the format (individual, couples, or group) is specifically organized to deal with the problem areas in a set number of sessions.

Students are encouraged to utilize all existing constructive forces in their environment and in themselves and to accept greater responsibility for their past actions and present learning enterprise.

As in behavior therapy, the student’s difficulties are conceived of as problems in learning.

The value of other behavioral approaches is acknowledged and their methods even utilized, such as operant learning and the proper timing of reinforcements, imitative learning, and the observation of models (e.g., programmed instruction, tape recordings, videotapes, television presentations, movies, books, and actual live plays and demonstrations), and emotional learning (e.g., systematic desensitization).

Concentration, however, is on information giving, verbal instructions, role playing, and the proper timing of cues.

An example of how cognitive learning operates is provided by an Adult Development Program of the Department of Psychiatry and Behavioral School of Medicine, University of Washington, which was established in 1970 by Cornelis B.

Bakker and Hubert E.

Armstrong, Jr.

Students in the program are accepted who have a desire to make changes in themselves and their situation, are willing and able to assume responsibility for their behavior, and are ready to approach new ways of dealing with their problems.

As many courses as are desired may be chosen from a few hours a day to an entire day.

5 days a week.

Some courses are arranged in "pathway" groups of six units of four sessions (3-hour evening sessions, once weekly for 4 weeks).

Couples sign up for one unit at a time, which www.freepsychotherapybooks.org 267 while taken in sequence can be completed in 6 months.

The Marital Enhancement Pathway, for instance, enables couples to set goals and master skills to achieve these goals in a graded series.

The classes are as follows: 1.

Human territoriality (learning to bargain to resolve conflicts).

2.

Assertiveness training (learning to substitute more constructive modes of communication for nonproductive interchanges).

3.

Precision behavior change (learning to bring mutually agreed upon changes in oneself and one's partner).

4.

Marital myths (recognizing and learning to overcome false ideas about oneself and the marital relationship that block change).

5.

Fixed role (practicing through role playing, alteration of behavioral patterns to overcome defects in the marital relationship).

6.

Sensuality and sexuality (enhancing pleasure in sexual relations in marriage).

Small groups are available for specific goals.

Thus an Assertiveness Laboratory of two 2-hour sessions each week for 3 weeks teaches people to role play to learn to accept responsibility for permitting others to take advantage of them, to learn to say "no," and to recognize and "do what one wishes" rather than "what one should do."

A Couples Communication course of 2 hours once weekly for 8 weeks teaches how to negotiate disagreements "without making each other feel miserable." Here each couple is encouraged to practice fighting about an unresolved issue while other couples referee.

Each member has a "second," a classmate who helps define gripes, feelings, and demands.

An Effective Communication course of two 75-minute classes each week for 4 weeks focuses on learning through structured exercises, such as how to say things in different ways to avoid hurting others, how to exchange compliments, and how to ask for and receive positive comments.

A Fixed Role course of two 2-hour sessions each week teaches how in changing one's behavior other people's behavior changes and how to overcome resistance to change.

This is done through following a detailed script written jointly with the instructor that

permits practice through role www.freepsychotherapybooks.org 268 playing of a new way of

dressing, talking, and behaving.

A Human Relations Laboratory of three sessions a month for 2 hours each presents two models: one following the scheme of transactional analysis, i.e., parental injunctions and childish demands operating simultaneously within oneself, mediated by one's inner adult self in relation to specific situations; the other, patterned after Ellis's A Guide to Rational Living, explores how interpretations of events rather than the events themselves are the keynote to one's irrationalities.

A Human Sexuality Seminar of one 2-hour session for one month teaches how to break down myths about sexuality and to replace them with information to enhance sexual functioning.

No attempt is made to deal directly with actual sexual dysfunctions.

The Human Territoriality Seminar teaches bargaining skills to avoid being exploited, to gain and defend legitimate demands by standing up for one's own rights, to resolve conflicts, and to increase self-confidence and enjoyment in dealing with people.

Another example has been provided by B.

L.

Greene (1975), who, working with a more or less homogeneous group of "hard science" professionals at the Midland Community Mental Health Center, who were resistive to traditional psychoanalytic and conditioning cognitive approaches, related to the proposition that behavior is determined by thinking patterns that may be analyzed and modified.

Explored within the therapy session and assigned as homework were goals, attitudes, expectancies, and beliefs with the object of both modifying handicapping perspectives and thought processes and the adoption of constructive alternatives.

A wide variety of therapeutic techniques were then exploited to facilitate the achievement of this object.

The method is not designed to change basic characterologic defects, which usually will require long-term dynamically oriented interventions.

It is a short-term course geared toward enabling a better reality adaptation.

Criticisms of Cognitive Learning Cognitive learning bases its techniques on the premise that people are capable of changing their behavior by altering the immediate environmental circumstances in which they function.

It is claimed that many forms of psychiatric intervention rather than fostering change tend to stabilize undesirable personality patterns.

By labeling a person www.freepsychotherapybooks.org 269 with a diagnosis, we tend to increase the probability that that individual will respond in a way consistent with the diagnosis.

Psychological testing, while its validity and reliability are notoriously low (Little & Shneidman 1959; Chapman & Chapman, 1967), stamps the individual with a pathological blemish. By concentrating the attack on the inner psychic structure to the neglect of situational alteration, psychotherapists are said to become counterproductive.

The results of attacking the environment instead of the intrapsychic structure, of avoiding diagnosis and psychological testing, and of focusing on methods that directly influence behavior, it is furthermore avowed, tend to shatter the myth of unchangeability of human nature.

In view of the savings of time and costs, cognitive learning offers the motivated patient who is able and willing to learn an opportunity for substantive benefits.

In a community mental health program its short-term nature will greatly reduce the patient load.

In private practice it may well serve as a way of initiating therapy in many patients in combination if necessary with behavioral and other educational approaches.

In some cases, as a result of tension relief, restoration of mastery, and a more wholesome

adjustment, patients may show in addition to symptom relief and enhanced adaptation considerable personality growth.

Like any other therapy, however, it is not universally applicable.

Blocks to learning fostered by inner conflicts, self-destructive masochistic impulses, lack of motivation, a need for failure, irremediable environmental stress circumstances, intellectual inhibitions, and attention failures may obstruct progress.

Here other long-term psychotherapeutic approaches are better applied, although their success obviously is not guaranteed.

It goes without saying that a well-designed program geared toward specifically oriented learning and executed by a well-trained, dedicated, and empathic professional is mandatory for progress.

The experience with many cognitive learning programs has been most encouraging, pointing to the inherent flexibility within people that enables them to learn from shortterm educational techniques.

There is obviously nothing new about the cognitive educational model.

It was www.freepsychotherapybooks.org 270 popularly utilized in the mid-nineteenth century.

It fell into discard with the rise of the psychoanalytic model in the first half of the twentieth century.

Its revival is predicated on better information that we now have that we can communicate to clients, much of which comes from dynamic psychology, and on work in experimental psychology laboratories.

Persons suffering from severe mental problems or from extraordinary stress situations may not be able to take advantage of the learning techniques offered and may require additional specialized psychotherapeutic and psychiatric help.

Success is also dependent on the acceptance of the role that the teacher plays in providing information and guiding the learning process.

A psychotherapist who wishes to exploit the value of educational techniques may well benefit from taking a course in executing techniques, such as helping the student to define attainable goals, establishing successful criteria for change, designing courses of study, acquiring essential information for acquisition of skills and changed attitudes, selecting the proper format of studies, and ensuring that motivation in the student continues.

An advantage is that staff can be recruited from a wide variety of disciplines, thus reducing time and cost.

THERAPEUTIC COUNSELING (PSYCHOLOGICAL THERAPY) In Chapter 6 the rationale and the principles of counseling were described and reference was made to the fact that some forms of counseling were reeducative in nature.

In recent years formal training in reeducative counseling techniques has been extended to individuals such as ministers, rehabilitation workers, police officers, and paraprofessionals who in their work come into contact with persons with problems.

The focus for which help is sought often involves specialized information.

The focus also labels the kind of counseling rendered.

Among the more common forms are vocational counseling, premarital counseling, marriage counseling, genetic counseling, spiritual counseling, and educational counseling.

In actual practice, irrespective of the specific focus, common principles obtain in relation to such important areas as setting up a client-counselor relationship and managing it toward productive objectives.

Some of the older readings in the field are still valuable, including those of Williamson and Darley (1937); Williamson (1939); Rogers (1943); S.

E.

Goldstein (1945); Garrett (1945); New York State www.freepsychotherapybooks.org 271 Counselors Association (1945); and Snyder (1947a&b).

More recent literature is contained in the section on counseling in Chapter 6 and in the Selected Texts section (page 1397).

Audiovisual aids are also described in the designated sections.

Some forms of psychological therapy downgrade technical and intellectual methods, contending that they are non-meaningful in the treatment process.

What is considered important is to bring up and express feelings.

It is believed that only through emotional arousal can one's potentials be actualized.

The traditional therapeutic authority-patient alliance is rejected as a proper basis for personality change.

Instead of manipulateness on the one hand and anonymity or detached noninterference on the other, the climate of the therapeutic relationship is one of empathic warmth, friendliness, and expressed regard for the patient.

Neither knowledge of the developmental past nor promulgation of present insights is considered important.

The emotional encounter between client and therapist is the basis for significant movement.

Defenses against proper relating are dealt with by confrontation and the assignment of tasks.

Experiencing the encounter as a different mode of relating eventually is said to lead to self-actualization.

Among the different types of treatment that operate on this philosophy are client-centered therapy, psychodrama, Gestalt therapy, experimental groups, EST, and existential therapy.

Client-centered Therapy The client-centered methods of Carl Rogers (1942, 1944, 1946, 1951, 1959, 1961a&b, 1980, 1983) have long been a favorite with psychologists, although in recent years the impact of psychoanalytic concepts and behavior therapy have made inroads into their popularity.

Nevertheless, they are still employed, and aspects of them are utilized in group work, play therapy, community development, and educational, religious, marital and industrial counseling (Snyder WU, 1943, 1947b; Hart & Tomlinson, 1970; Meador, 1975; Rogers CR, 1973).

Essentially, the philosophy around which client-centered approaches are oriented is a humanistic one, embracing the idea that a human being is possessed of innate goodness, actualizing tendencies, and capacities for evaluative judgments leading to "balanced, www.freepsychotherapybooks.org 272 realistic, self-enhancing, other-enhancing behavior." A human being becomes ineffective, hateful and self-centered, and then incapable of making proper judgments and responses as the result of faulty learning.

This leads to incongruence between what is being experienced and the concept of self.

Through the medium of special kinds of relationships, it is possible for the individual to rectify improper learning and to acquire new and productive patterns.

Release of the self-actualizing potential will lead a client to emotional growth.

It is essential, however, for the therapist or facilitator to be genuine or real (congruent) in the relationship and to possess a caring or an acceptant attitude in order to experience empathically a sensitive understanding of the client.

The client, in turn, must perceive these qualities in the therapist (Rogers CR, 1975).

Over the years the theories and methods of client-centered therapy have undergone some revision as a result of experience and research (Rogers & Sanford, 1985), but on the whole the original formulations are still the basic underpinnings of the method.

Rogers, drawing to an extent from Rankian methods, based his personality theories on research findings and on his own observations in psychotherapy.

These contend that subjectively observable responses of individuals are potentially available

to their awareness and constitute the body of their “experience.” There are, first, non-symbolic correlates of experience of an organismic nature that exist in the form of sensory, visceral, and emotional or “feeling” responses, through which the raw data of the environment can potentially become available to the individual’s awareness. Second, there are symbolic correlates, composed of attending responses accompanied by “free and undistorted awareness” of external and internal events, clearly perceived and faithfully symbolized.

The innate human capacity to symbolize permits the individual to register events accurately unless faulty learning has produced distortions.

The driving force of life is innately governed by an “actualizing tendency” around which all other motivations revolve (such as tension reduction, the drive for autonomy, etc.).

All behavior is holistically organized around this innate tendency and is patterned by the capacity to differentiate between effective (pleasure-producing) and ineffective (pain-producing) responses, a capacity that Rogers terms the “organismic valuing process.” If responses are positively evaluated as effective and actualizing, they continue or recur; if they are www.freepsychotherapybooks.org 273 negatively evaluated as ineffective or nonactualizing, they are avoided or terminated.

Some of Rogers’s early concepts draw from the theories of Rank, Goldstein, Sullivan, Maslow, Angyal, and Lecky.

They embrace ideas from existentialism, Gestalt therapy, Eastern philosophies, information theory processing, and group dynamics.

They reflect a phenomenological viewpoint, contending that each individual has a “phenomenal field” that determines behavior, and that shifts from time to time according to needs.

Aspects of the phenomenal field that one recognizes as part of oneself, such as perceptions about one’s physical self and one’s relationships with the world, make up one’s “self-concept.” Significant experiences may exist that the individual does not admit to awareness and that are therefore not a part of his or her self-concept.

Thus, certain needs, such as organic experiences and drives, may not be symbolized, rather being “disowned,” since they are not consistent with the self-concept.

As such, they can create tensions.

Values that may become incorporated in the self-concept are those derived from personal experiences and those absorbed from other individuals (“learned evaluative thoughts”).

A clash may result between these two sets of values (“incongruence between self and experience”).

The fundamental urge in all persons is to preserve their phenomenal selves; impulses and experiences that do not coordinate with the self-concept are perceived as a threat.

The individual responds to the “basic estrangement” that results with contradictory behavior and rigidity in the self-structure.

If the self-structure cannot meet the demands of the reality situation, if there is a great disparity between ambitions and accomplishments, if responses elicit both positive and negative self-evaluative thoughts, the individual will experience tension and anxiety.

He or she will then react with two types of defensive mechanisms to avoid awareness of the contradictory responses: first, by ignoring important responses (“denial to awareness”) and, second, by distorting thinking about the responses (“distortion in awareness”).

Where defenses that sponsor neurotic symptoms fail the individual, he or she may “break down” and manifest grossly disordered behavior such as psychosis.

The object in treating neurotic victims of faulty learning is to provide them with an atmosphere that does not threaten their self-structure but that enables them to examine, www.freepsychotherapybooks.org 274 recognize, and reorganize it, thus permitting an integration

of their organism and their self and acceptance of experiences previously excluded as alien. The phenomenal fields of both therapist and client are usually sufficiently related to allow intelligible communication and meaningful interchange of feeling.

During therapy the client, obsessed with self-criticism and self-devaluing because of failure to live up to the idealized image, soon becomes aware of contradictory attitudes.

In the accepting and approving atmosphere of a therapy that is totally devoid of threat and imbued with empathy, the client becomes more tolerant of oneself and one's failings.

Tension abates and a reintegration develops as discordance resolves between the ideal image and the actual self-perception.

Constructive personality change is thus contingent on a number of stipulations.

First, it is essential that the client be motivated to seek help.

This is generally in the form of some anxiety produced by an awareness of a state of "incongruence," in that a disparity exists between the client's self-picture and actual experiences.

Second, a special kind of human contact is required.

In a client-therapist relationship, it demands (1) that the therapist be both empathic of the client's awareness of his or her own experiences and able to communicate what is going on in the client's inner world on the basis of this empathy, (2) that the therapist has ample self-awareness, is honest about personal feelings, and is capable of "being oneself" in the relationship in order to function congruently, and (3) that the therapist possesses a positive regard for the client, accepting fully every aspect of the client's experience and right to be and feel as he or she is.

This does not mean that the therapist must be completely free of deviant response patterns as long as they do not force him or her to be authoritative and evaluating.

Third, it is important that the client perceive, by the therapist's behavior and verbalizations, that he or she is fully accepted, understood, respected, and "cared about," irrespective of experiences, problems, and feelings.

Entering into a relationship situation with a client-centered therapist provides the client with a unique encounter in which he or she is neither challenged nor condemned, every aspect of the client is respected and accepted, and the client can yield defenses

www.freepsychotherapybooks.org 275 without hurt.

Out of this adventure, a new conceptualization of the self evolves, with greater capacities to symbolize more accurately sensory and visceral experiences and with reconstitution of one's system to values in concert with the perceived self.

The guiding principle of client-centered therapy is oriented around the fact that the client or patient is the one responsible for his or her own destiny: the client possesses the right of choice of solution for his or her problems, irrespective of the choice of the therapist.

Residual in each individual, it is contended, are resources for growth that need merely be released to enable the person to achieve maturity.

The therapist strives to unleash growth forces by refraining from imposing patterns and values on the client and by promoting in the relationship the free expression of feeling.

Among the activities of the client-centered therapist are (1) attentive listening to the client's communications for content and feeling, (2) responding by a friendly non-punitive, empathic attitude and by occasional verbal comments that neither approve nor disapprove, (3) pointing out the client's feelings, (4) structuring the extensions and limitations of the therapeutic relationship, (5) encouraging the client in his or her efforts to manage problems, (6) engaging in answering questions and giving information only when it is essential to do so to help a client work through problems, yet avoiding this directive role at the slightest threat of emerging dependency, and (7) refraining from insight offerings, advice giving,

environmental manipulation, censure, commendation, or the posing of questions and suggestions regarding areas of exploration.

The client is given complete responsibility for the choice of topic, the extent of concern with it, and the interpretation of the meanings of reactions.

The therapist's responses are chiefly in relation to the evaluative ideas that the client verbalizes about himself and other people, and the feelings associated with such ideas.

The role of the therapist is solely to direct the client's attention to his or her ideas and not to interpret or clarify.

The feelings of the individual are always accepted in a tolerant, nonjudgmental way and are reflected back to the person in order to bring to the client's consciousness the full pattern of his or her emotional attitudes.

At times the rephrasing of the client's utterances www.freepsychotherapybooks.org 276 helps the client to clarify facts.

The catharsis involved in the process, as well as the therapist's activity in reflecting feeling, are believed to lead to genuine self-understanding and insight in the individual's own terms.

The release of normal growth potentials helps the client to gain control over discordant forces in the self.

The role of the therapist, thus, is to act as a catalyst of growth, a ("change agent") not to impose growth on the client.

Treatment, as can be seen, is oriented around the idea that the individual has the capacity to deal effectively with those aspects of his or her personality of which he or she becomes conscious during the relationship with the therapist.

It is assumed that the client can achieve insight in the relationship and can accept and make constructive use of responsibility.

Because efforts to interpret, evaluate, or guide the individual are felt to hamper the emerging sense of self-direction and self-growth, a passive role on the part of the therapist is mandatory.

Rogers insists that a nondirective approach is not to be confused with a laissez-faire policy, which the patient is apt to regard as evidence of rejection or indifference.

A truly nondirective attitude avoids clarification of the individual's attitudes, since this is a form of subtle directiveness.

The function of the therapist is to perceive empathically the feelings of the client and to communicate this understanding.

It is essential for the therapist to discard a preoccupation with diagnosis, to stop making professional evaluations, to eliminate estimates of accurate prognosis, to abandon all attempts to guide the individual, and to concentrate solely on accepting and understanding the attitudes of which the client permits himself or herself to become conscious.

During therapy there is a gradual shift in the content of the material discussed— from symptoms to explorations involving the self.

Changes develop in the perception of and attitude toward the self, with more positive appraisals and a more realistic consideration of oneself and one's environment.

Judgments are recognized as originating in values residing within, rather than outside, the self.

Perceptions shift from wide generalizations to more limited ones rooted in primary experiences. Symbolizations become increasingly adequate and differentiated.

Movement is registered in awareness of denied or repressed experiences and feelings.

Changes occur in personality structure and www.freepsychotherapybooks.org 277 organization toward expanded unification and integration.

Anxiety abates; neurotic tendencies decrease with greater acceptance of the self. There is heightened objectivity in the handling of reality, a more constructive mediation of stress, a harmonious expression of attitudes and feelings, and better intellectual functioning. Developing changes in behavior are in line with improved adjustment and maturity. Decreased tension, lessening of defensive tendencies, and greater tolerance of frustration are concomitant.

As has been indicated, diagnosis in client-centered therapy is not felt to be essential to the treatment process.

Indeed it is conceived of as a hindrance.

Making a diagnosis is also felt to involve the dangers of subordinating the individual to an evaluation by authority, putting the therapist in a godlike role.

This opposes an atmosphere of equality.

Psychometric tests similarly are not encouraged, for the therapist's attitudes toward the client may be colored by the test findings and the client is ill equipped to handle most of the information revealed by testing.

The medical model, with its focus on pathology, diagnosis, and specificity of cure as a basis of understanding or working with emotional problems, is considered inaccurate.

Not only is it likely to enhance dependency but it avoids the basis of the prevailing difficulty.

Rogers revised upward his original idea of client-centered therapy as being most useful in essentially normal people who have sufficient personality integrity to solve their problems with a minimum of help from the therapist.

His contention is that his method is universally applicable from childhood to old age, from mild adjustment difficulties to severe psychoses, from "normal" to deeply neurotic situations, from immature dependent people to those with strong ego development, from lower-class to upper-class citizens, from lowly to highly intelligent persons, from physically healthy souls to those with psychosomatic ailments.

It is claimed that transference does not develop too intensely in client-centered therapy since the individual is not evaluated nor held subject to specific rules.

Self-esteem consequently is prevented from crumbling, thereby avoiding a dependent relationship.

Where transference erupts, it is more or less disregarded; it is simply accepted, not explored or interpreted.

In a climate of empathic understanding transference attitudes are said to disappear or to express themselves so minimally that they do not interfere with progress.

Past experiences and future projections are subordinated to the reality of the present, even though it is acknowledged that they might not be symbolized and thus operate to provoke tension.

Conscious material is felt to be sufficient, and probings into unconscious motivation are not considered of too great importance.

In practice it has been found by many present-day client-centered counselors that a purely nondirective method does not suffice.

Reflecting back to the client his or her words often failed to release the spontaneous growth forces postulated in the original client-centered approach.

To be successful, therapeutic interventions needed to be conducted in a participant climate, provided by the operator, of accurate empathy, unconditional positive regard, and congruence. This constituted a base for "experiential therapy." Empathy requires that the therapist perceive expressed and unexpressed feelings of the patient, experiencing them as if they were his or her own.

Unconditional positive regard necessitates respecting and valuing the patient's self-components.

Congruence means manifesting authenticity and sincerity of feelings for the patient, whatever these may be, avoiding phony facades and revealing true responses rather than playing games. The popularity of the client-centered approach is contingent on the fact that its theories are uncomplicated and relatively easy to understand; its techniques are simple to master, requiring little clinical experience for satisfactory results and involving few dangers to the patient, who actually works out his or her own problems.

Criticisms of Client-centered Therapy Client-centered psychotherapy is tailor-made for persons who need and respond to a kind, caring, nonjudgmental atmosphere and who are ready for and possess a strong motivation for change.

It is helpful to individuals with a relatively sound personality structure who require aid in clarifying their ideas about a current life difficulty or www.freepsychotherapybooks.org 279 situational impasse and who may be responsive to a "helping process." It is less helpful in the treatment of emotional problems that contain strong anxiety elements.

Anxiety is the greatest motivator of all human behavior; it impedes and even blocks the emergence of positive growth potentials no matter how tolerant and permissive the therapist may be.

Anxiety nurtures resistance that can effectively prevent thinking about, or spontaneously focusing on, significant conflictual patterns.

Left to one's own devices, the individual will usually avoid coming to grips with deep anxieties.

He or she will even choose to retain neurotic defenses, warding off inherent impulses for growth.

It is doubtful that anxieties rooted in unconscious conflict can be dealt with effectively by approaches that deal exclusively with conscious ideation.

It is essential, then, that a therapist enter actively, at times, into a neurotic cycle.

Resistances may have to be dealt with in a blunt and even forceful way.

Similarly, even though the person may acquire self-understanding, anxiety may prevent his or her utilizing insight in the direction of change.

Here, directive measures of challenge and confrontation may be required before inertia yields.

Thus choice or rejection of a traditional client-centered therapy will depend on whether or not the individual is deeply disturbed emotionally, has existing ego strength, and the nature of the problem for which he or she seeks help.

For some patients, a completely accepting atmosphere will do little to interrupt repetitive, destructive defensive operations.

Without arguing the advantages of a humanistic over a medical model, traits of genuineness and empathy, such as described by Rogers, are of distinct advantage in any kind of therapy and are indispensable aspects of a therapeutic personality.

Such traits and the conduct of therapy along caring, nonjudgmental lines may reduce the severity of introjected authority imprints and help soften a harsh image of the self.

They will not suffice, however, in dealing with pathology residual in unconscious variables that are provoking problems in adaptation.

Dealing with such variables will require interventions other than or supplemental to the recommended client-centered techniques.

The field of client-centered therapy has, nevertheless, made an important contribution to psychotherapy by pointing out basic elements in the "helping process," by

www.freepsychotherapybooks.org 280 emphasizing some essential principles in interviewing, by encouraging research into process and outcome (Seeman & Raskin, 1953, Cartwright, 1957; Shlien

& Zimring, 1970), by stimulating a vast amount of professional literature, and by elucidating the management of certain phases of treatment, especially termination.

Directive Approaches Directive approaches put the therapist in an active role in determining which of the basic problems of the patient to attack, the immediate and remote objectives, and the promotion of a plan of action.

Persuasive and commanding tactics are often employed, the therapist exerting strong pressures on the patient, even purposefully mobilizing tension or reducing it with supportive techniques if tension becomes unbearable.

Goals are thus more or less vested in the therapist, who makes an effort to dissect, tear down, rebuild, and resynthesize the personality.

A number of techniques aimed at these goals were originally delineated by F.

C.

Thorne, (1944, 1945, 1946, 1950) under the title directive psychotherapy.

For instance a patient might be given information with the object of reorienting his Weltanschauung.

This was done by confronting the patient with factual information about himself in an effort to get the patient to reevaluate his or her attitudes.

The case history was used by Thorne both as a diagnostic and as a therapeutic aid, helping in establishing rapport, promoting catharsis, giving reassurance, and fostering insight.

A more active technique was the therapeutic use of conflict.

Where certain maladjustments were sponsored by too little concern, conflicts might deliberately be induced.

Thus, the patient was provoked to reconsider attitudes in the direction of reality.

The patient was then presented with his or her inconsistent behavior in a strong confrontational way in an effort to motivate him or her to resolve it.

The extent of directiveness was gauged in each patient and varied from forceful coerciveness to a relatively participating relationship.

Thorne, however, recognized that the term "directive psychotherapy" was too limited and nondescriptive of his method, which was of an eclectic nature, and he advocated a comprehensive integrative approach.

www.freepsychotherapybooks.org 281 Concern with the past, issues of transference, and other dynamic principles are not considered vital or necessary for behavior change in directive approaches.

Principles of cognitive behavior therapy are apparently operative in certain forms of directive treatment and may be important in some individuals who are seeking a strong authority who can influence and regulate their lives.

Rational emotive psychotherapy is the name given by A.

Ellis (1957, 1958b, 1965, 1973) to cognitive therapy procedures that combine formulations from semantic, persuasive, directive, and behavior therapies.

Treatment tactics are organized around the hypothesis that aberrant emotions are controlled by faulty thinking processes, such as illogical "self-talk and internalized sentences." A search is consequently instituted for irrationalities in the patient's ideational stream that result in negative, unrealistic, and self-defeating behavior.

Generally, this exploration will reveal the origin of illogical ideas in the attitudes of parents, teachers, peers, and, above all, the credos supported by the culture.

The effective therapist must bring the patient's self-defeating verbalizations to his or her attention, show how they cause and maintain disturbance, reveal the irrational links in the internalized sentences, and teach the patient how to rethink and reverbitalize these in a "more logical, self-helping way." For example, a commonly accepted misconception is that the

individual must depend on, be loved by, and approved of at all times by others. In the patient's upbringing he or she is taught that to be self-sufficient is to be selfish. To reverse the behavior that this distortion produces, one must substitute the idea that universal approval is impossible and that dependency is self-annihilating. Self-sufficiency is by far a much more wholesome way of life, and it is in this direction that the individual must move.

There are countless other illogical ideas to which the person falsely subscribes that operate to cause and to sustain neurosis.

Release from these ideas can be brought about only by rethinking and reconceptualization. Toward this end, the therapist may employ many relationship-building and expressive-emotive techniques—analytic, persuasive, and behavioral—as long as the patient's self-defeating propaganda is ultimately defeated and a new rational philosophy of living substituted.

A number of other cognitive rational reeducative approaches, such as the www.freepsychotherapybooks.org 282 “assumption-centered” psychotherapy of Anderson and the “problem-centered” method of Wertz, have been proposed.

In each, the patient is confronted with his or her ideas and behavior that are maladaptive and is offered corrective solutions.

According to Frank (1984) one of the basic features shared by all forms of psychotherapy is that when they are effective they correct the demoralization produced by symptoms.

Patients seek help only after their morale has been shattered by symptoms.

Such patients are often victimized by an “assumptive world” with distorted ideas about themselves, their future, and the world around them, which leads to symptomatic distress.

By altering the meaning or expounding on the function of a symptom through psychotherapy, the “assumptive world” can sometimes be altered or rectified.

Corrections in the “assumptive world” can result in changes in thinking and behavior which reinforce the gains produced by any form of psychotherapy.

In programmed psychotherapy (Young H, 1974) the patient is given a series of significant problems and appropriate solutions to illuminate and revise assumptions.

These problems are related to events in the patient's past history and are also taken from the histories of patients with similar problems.

Psychotherapy as a form of instruction aimed at reorganization of “assumptive worlds” is considered in the section on cognitive learning, which is focused more on external than internal variables.

Concerned with the latter (i.e., with identification of inner forces that create assumptive distortions and impede problem-solving techniques) are approaches such as those described by Brunner (1966) and M.

T.

McGuire (1968).

Combining the ideas and methods of psychoanalysis, individual psychology, rational emotive therapy, and behavior modification within an existential framework, Harold Greenwald (1974, 1980) has described a useful treatment in individual, group, and family therapy that he calls Direct Decision Therapy and that he dates back to 1970.

The patient is asked to state as clearly as possible the nature of his or her problems and the goals he or she wishes to achieve.

The patient and therapist jointly examine past decisions that created the problems that prevented the patient from reaching the goals.

The therapist helps the patient become aware of these faulty decisions as they are expressed in the patient's activities, attitudes, and philosophy of life.

The gains (“payoffs”) for the decisions www.freepsychotherapybooks.org 283 are examined.

The original basis for the decisions is explored and is contrasted with its utility in the here and now.

Next the patient is invited to examine options or alternatives that could enable him or her to function in a different, more constructive way.

Once the patient has made a decision to change, methods are discussed regarding how to do this. Often the patient describes the method that he or she would like to try—Gestalt, hypnosis, rational emotive therapy, behavior therapy, and so on.

The therapist obviously participates in making and implementing this choice.

Reality therapy is another directive method organized around the central theme of “identity as a core problem” (Glasser, 1965; Glasser & Zunin, 1972).

In therapy the patient is guided away from methods by which he or she avoids reality and is motivated toward success.

The first principle is “involvement” by the therapist with the patient to demonstrate “that he cares, that he is warm and friendly ... willing, if indicated and appropriate, to discuss his own experiences and to have his values challenged and discussed.” The therapist reassures the patient of confidence in him or her and communicates the belief that the patient can become happier and can function in a more responsible, effective, and self-fulfilling manner.

Treatment is organized around the relationship with the therapist rather than the content of verbal exchange.

The second principle of reality therapy is the focusing “on behavior rather than feelings.”

Doing is more important than feeling, and the therapist orients the conversation around the patient’s daily actions.

The third principle is concentration on the present and future rather than the past.

If past critical events are mentioned, the patient is asked what was learned from them.

Past character-building experiences are reviewed in relation to present success-oriented strivings.

Constructive alternatives are explored.

The fourth principle is helping the patient arrive at a value judgment about what he or she is doing to contribute to his or her own failure.

The fifth principle is evolving a realistic life plan (perhaps put in writing in the form of a contract between patient and therapist) and assuming responsibility for seeing it through.

The sixth principle is making it clear to the patient that no excuses for failure to execute the plan are acceptable.

If failure occurs, a new plan is jointly made or the old one modified.

The seventh principle is avoiding punishment, sarcasm, ridicule, or hostile statements; transference, when it occurs, is www.freepsychotherapybooks.org 284 immediately dealt with and dissipated rather than encouraged.

Some therapists have used a form of treatment called Releasing to counter a potent source of stress that stems from the frustration that results when one urgently wants to change something that seems immediately difficult or impossible to change.

The target of the desired change may be a current or anticipated irritating happening, a past event or memory, a physical need that cannot be fulfilled, indeed almost any stimulus, impulse, emotion, striving, or situation that is upsetting to the person.

The consequence of this compelling urge or “wanting” is an overinvolvement with the target and an overpush to rectify it, resulting in further tension and anxiety.

Awareness of what is going on, even insight into its dynamics, may not succeed in removing the existing distress.

What is sometimes helpful is a cognitive shift that diverts the individual from engaging in selfdefeating overpush maneuvers.

Recognizing that some people inescapably become locked into cycles of stress, techniques such as the "Sedona Method" have been used in workshops to help people "let go" of their investment in "wanting" (overpushing) to change things, which results in their stirring up troubles rather than resolving them.

In this way, one hopes, they are "released" from overinvolvement and achieve peace of mind and freedom from tension.

Releasing techniques have been elaborated by a number of writers and lecturers.

Patricia Carrington (1984) has written a textbook on "releasing" and has developed audiotapes that provide step-by-step instructions on how to use this method to reduce stress and enhance the quality of one's experiences.

Included in the book are clinical examples of how to use the technique.

According to Carrington, the procedures of "releasing" are not designed to replace psychotherapy, which deals with different psychological dimensions.

In some cases, however, "releasing" techniques can operate as an adjunct to psychotherapy, particularly during the difficult phase in which insight is translated into action to alter unproductive patterns.

There are some similarities between the "releasing" method and cognitive behavior therapy, persuasion, and desensitization; all work toward the same goals in a somewhat different way. www.freepsychotherapybooks.org 285 The goal of "releasing" exercises is to "let go" of trying to change whatever it is one wants to change, such as the behavior of another person, unalterable events, or past happenings.

Effort or force (overpush) are eschewed.

"As we let go of wanting to change any one thing (however insignificant it may seem to us at the moment)," says Carrington, "we simultaneously let go of whole networks of related events which are stored in our memory banks." "Letting go" of "wanting to change something" does not make a person passive or take away the motivation to overcome a problem.

This is an important point because some people assume that letting go of the "wanting to change something" means that they have to detach and abandon the desire for constructive change.

The "wanting" in releasing methods refers to the compulsive overpushing that does nothing to solve a problem.

"Releasing," on the contrary is said to clear the mind so that a person can make constructive choices.

What may confuse some persons is the language used in "releasing." The word releasing simply means "letting go or yielding compulsive thoughts and behavior and not dredging up buried emotions." Explosive outbursts of emotional catharsis may occur in the course of practice sessions, but this is serendipitous.

A good deal of the success of "releasing" is based on the suggestibility of the subject and faith in the guide or teacher who is promoting a different way of thinking.

As with any other technique, not all persons are susceptible to "releasing." It is designed to deal with the effects of conflict and with conditioned behavioral patterns and not with their sources.

It does not propose to reconstruct personality but merely to redirect patterns of thinking.

Understandably, it is not intended as a cure for serious emotional problems that call for more intensive treatment approaches.

It may occasionally reduce stress in a subject who accepts its precepts and lacks a masochistic dedication to self-defeating behavior and who does not submit to other obdurate resistances to change.

Jay Haley (1963b) has described a type of directive therapy for the short-term management of patients, as well as for strategic interventions during long-term therapy, during which the

therapist enters into an alliance with the patient's symptoms.

Assuming that the dynamic function of symptoms is essentially power-motivated, Haley believes that www.freepsychotherapybooks.org 286 the operations of the therapist should be pointed toward wresting control from the patient.

Once the symptoms are clearly delineated, the therapist instructs the patient to do what he or she would ordinarily do anyway, such as indulge in the symptoms and withhold information. The object is to encourage the resistances of the patient, but at the same time to bring them under the control of the therapist.

Then it is suggested that symptoms will appear, intensify, and lessen under certain circumstances.

Finally, the patient is enjoined to execute the symptoms in such a way that he or she cannot possibly continued them or until he or she begs to give them up.

For instance, compulsive symptoms may be encouraged by the therapist until the patient requests a change, whereupon "permission" is granted to relinquish them.

An obese woman with a yen for rich desserts may be ordered to restrict herself to only this item of food, which she may incorporate to her heart's desire.

When she has surfeited herself to the point where sweets repulse her, she may then be given permission to eliminate them from her diet.

A compulsive patient who is punishing himself with a distressing symptom is asked to discipline himself whenever the symptom appears—in other words, to castigate himself for treating himself so badly with his symptom.

Ways of self-punishment may be worked out that will benefit rather than hurt the patient.

Thus, a man who is convinced he must exercise more for his health is instructed that whenever his symptoms are intense, he will get up in the middle of the night and do deep knee bends.

The relationship of these tactics to counterconditioning is apparent.

Criticism of Directive Therapy Criticism of directive psychotherapy is voiced, particularly by psychoanalysts and client-centered therapists, in terms of the imposition on the patient of the therapist's goals and sense of values and the reestablishment of the disciplinary atmosphere of the childparent relationship.

Under these circumstances, liberation from the yoke of one's authoritarian conscience may be impeded, leading to an interference with growth toward assertiveness and independence.

A disadvantage claimed is that the patient is kept on a www.freepsychotherapybooks.org 287 dependency level longer than is necessary.

Where the aim in therapy is to make the patient self-sufficient and capable of finding security within himself or herself, directiveness may inhibit this aim.

In looking for security or support from the outside, the patient may be inhibited in developing an autonomous stand in life and in becoming a stronger being through his or her own resources.

Another criticism of directive therapy is that hostility is often mobilized by the very nature of the magisterial relationship.

This prevents the patient from becoming liberated from a punitive and severe conscience fostered by an irrational attitude toward authority.

While the new ways of conduct that are developed may be better than those the patient has followed most of his or her life, they do not alter the intrapsychic structure.

The patient virtually remains a child who has merely incorporated the mandates of the new parent-therapist.

Critics of cognitive therapy attack the "guiding" influence of the therapist, whose own values and assumptions may not necessarily be pristine and accurate.

In this way the therapist functions like a clergyman or educator whose philosophies or viewpoints are sometimes suspect.

Not all of these criticisms are justified.

Actually, at certain points in all therapies a cognitive position is taken, as in psychoanalytic interpretations, in which bias is not too unusual.

All good psychotherapists probably do a species of cognitive therapy after gaining an understanding of the existing fallacious ideas harbored by the patient.

The therapist's attitude and mode of dealing with a patient's rebelliousness at directive mandates will determine the direction of the behavioral change.

Unless their dependency needs are too great, many patients will learn to cope with the directiveness of the therapist and to evolve new ego strengths out of their own spontaneous needs.

If the therapist acknowledges the patient's right to criticize and shows respect for the patient's autonomy, the patient may be able to effect some change in concepts toward authority as universally obstinate and punitive.

A value of directive therapy is in the techniques that have been evolved, which may be adapted to some aspects of a therapeutic program, particularly during the resistive phase of translating insight into action.

Freud himself acknowledged that some patients require forceful mandates when their resistance blocks www.freepsychotherapybooks.org 288 constructive action.

CASEWORK THERAPY Traditionally, social casework is a guidance or counseling technique based on the casework interview and employed by social workers.

The aim is to help individuals find a solution to problems of social adjustment that they are unable to handle in a satisfactory way through their own resources.

This process is usually divided into three parts: the case study, the plan, and the working out of the plan with the client.

Casework, employed in this way, can be either preventive or remedial.

In recent years the casework method has been enriched by knowledge from psychoanalysis, psychology, sociology, and other behavioral sciences.

This has fostered an interest in broadening the function of the caseworker from management of manifest social problems and the utilization of community resources toward increasing the inherent capacities within the individual to cope more effectively with his or her environment.

Interpersonal and social problems are resolved both to help clients meet unfulfilled needs and to adapt more adequately in their relationships with people and society.

Toward this end, inner psychological difficulties are dealt with in addition to external vexations.

The expanding role of the social worker is dealt with in greater detail in chapter (16) "Who Can Do Psychotherapy?" In examining modern casework procedure we find that the process of helping is remarkably similar to that of psychotherapy with educational goals.

The complaint factor is usually a social or relationship problem, and the motivations that prompt consultation are focused on external rather than internal problems.

The ultimate objective is to enhance social functioning, but the means to this usually necessitates some change within the client's attitudinal and value systems as well as the customary modes of problem solving.

To reach this goal, the client is encouraged to express feelings about accepting and utilizing help while the caseworker responds empathically to communicate understanding.

Basic is a proper diagnosis to determine the nature of the problem, the changes that can or cannot realistically be implemented, and the most expedient means of bringing about these changes.

The initial inquiry (the social study process) secures www.freepsychotherapybooks.org 289 important information about the client, the social situation confronting the client, and the

difficulties that bring him or her to the agency.

To help in the survey, as complete information as possible is obtained regarding the client's assessments, feelings, and judgments.

Information other than that revealed by the client is secured from authoritative sources, such as hospitals, schools, courts, and agencies.

Skilled interviewing is mandatory to obtain reliable data that will contribute to a workable diagnosis and the estimate of possibilities of change.

Helpful also are planned visits to the client in the family setting.

The social treatment process is geared toward strengthening the client's adjustment skills as well as reducing destructive environmental pressures.

The practitioner must be on the alert to obstacles that block the client's coping capacities and ability to utilize available environmental resources.

The amount of active help and support given by the worker varies with the existing strengths of the client.

As the latter gains in understanding and problem-solving talents, less and less active help is extended.

Reasonable short-term goals are designed with the object of giving the client greater confidence in reaching long-term, more difficult objectives.

It is obvious that what is being described in this process is a treatment that focuses on social change; however, it is one that inherently and inescapably aims for enhancement of growth processes within the individual, a diminution of regressive tendencies, and a more realistic appraisal of inner capacities.

Focus on the individual's capacity for self-direction expands ideal goals of casework from mere solution of the complaint factor to actual intrapsychic change.

The means toward this reconstructive end will vary with the theoretical orientation of the practitioner.

In the past, social workers, increasing their operations, concentrated largely on the field of psychoanalysis and drew their inspiration from two sources, the theories of Otto Rank and Sigmund Freud.

The school of functional casework (Kasius, 1950), oriented around the teachings of Otto Rank, attempted to help people seeking specific services in social agencies in such a way that the use of the services became psychologically constructive for the individual.

www.freepsychotherapybooks.org 290 The relationship with the caseworker therapist was considered a "helping process," in which the individual experienced a new, constructive way of observing himself or herself and of relating to another person.

The relationship served as a kind of laboratory in which the client acted out, with a representative of authority (the therapist), the full range of attitudes and patterns such as belligerency, detachment, ingratiation, and the need to control or to be controlled, that were habitually cherished toward authoritative persons.

The therapist handled these projections as a necessary part of the client's accepting help and, in tolerant recipience, reflected back to the difference between the projections and the existing realities.

This enabled the client to become aware of his or her characteristic ways of relating and permitted the client to accept in a more realistic manner the kind of help being offered.

The client was charged with direction for processes of change; choices and goals were conceived of as his or her right and responsibility.

Within the limitations of the structure of the agency, the client was free to move toward self-responsibility and selfacceptance.

The relationship between therapist and client was thus believed to serve two aims: first, the

solution of a specific problem and, second, the evolvment of a more mature personality. The school of diagnostic casework with theories, drawing largely from the teachings of Freud and his contemporaries, tried to provide a corrective emotional experience for the individual through the medium of a positive relationship with the caseworkertherapist.

The relationship was not the central core of treatment as in functional casework, but rather it was a vehicle toward increasing the client's ability to solve problems on a more adult level.

The attitudes of the client and the kind of relationship that he or she sought with the therapist were used diagnostically, but they did not determine the direction of therapy.

This was geared toward dissipating inner conflict and expanding ego strength as well as reducing, by social planning, existing environmental pressures.

Objectives were variably graded to meet requirements of the diagnosis and the specific needs of the client.

Techniques, which included emotional catharsis, reassurance, guidance, clarification, and interpretation, were used in whatever ways best served to increase the capacities of the ego.

The focus of discussion was on the client's problems, particularly on ways of overcoming obstacles to their solution imposed by inner feelings www.freepsychotherapybooks.org 291 and social reality.

There was a continued evaluation and reevaluation of the problem from cues supplied by behavior patterns and attitudes, from the client's responses to the therapeutic situation, and from reactions to interpretations and suggestions made by the therapist.

Projected irrational feelings and attitudes were recognized and discussed, leading to greater awareness of current patterns of behavior and of their origins in earlier relationships.

The client gradually was led to see connections between present attitudes and conflicts and those harbored toward parental and other important personages in the past.

The goal was a strengthening of ego capacities, a reappraising of reality issues divorced from anachronistic expectations, a developing of a sense of being valued, and an incorporating of acceptable social concepts and standards.

As in functional casework, dreams and other derivatives of the unconscious were accepted, but they were not handled or interpreted unless the caseworker was trained to do reconstructive therapy.

These differential points between the functional and diagnostic schools are not as rigidly drawn now as they were in years past.

This is because both systems have not fit into the contemporary practice scene, which is branching off into many areas, including the behavioral field and group work.

Since casework practitioners must be concerned with both intrapsychic and social systems (the family, groups, community), they need several models to understand their interactions, for example, psychoanalytic theory and social system and role theories.

Perhaps it was inevitable that analytically oriented practitioners would move from classical theory to that of ego psychology.

According to this, the ego is conceived of as the mind's regulating mechanism toward management of needs and feelings.

The ultimate aim of casework is toward enlarging the capacities of the ego to select more rewarding aims and objects.

Stressed is an assessment of the client's motivations causing resistance to taking help and of utilizing help effectively in the interest of improving healthful adaptive patterns.

Immediate and long-term goals are particularized, including situational changes to enhance improved client functioning.

Frances Upham (1973) has outlined a method of ego analysis to influence the www.freepsychotherapybooks.org 292 developmental process that has been interrupted by pressures

with which the individual in the past was unable to cope.

The practitioner serves as an identification model, as a supportive ally to enhance existing strengths, and as an educator to teach new skills in modifying the environment and evolving more constructive coping patterns.

Once a helping relationship is established between client and practitioner, a search is made for any resistances.

To resolve these, the practitioner attempts to clarify misunderstandings, supply information, outline alternatives, and negotiate differences between expectations and possibilities of fulfillment in the medium of empathic understanding.

Positive transference is not interfered with, but negative transference is handled actively by discussion and interpretation.

An agreement is reached regarding the kind of help that will be administered and the goals to be attained.

The practitioner in working with the client attempts to assess the available coping strengths and weaknesses that will help or hinder the treatment plan.

This estimate is based on a developmental model during which more elaborate ego patterns are progressively evolved on a biological timetable.

The practitioner gauges modes of perception that help orient the individual to reality, enabling him or her to relate to other human beings and to play essential roles in the service of adaptation.

The practitioner makes an assessment of cognitive capacities, of adaptive and maladaptive patterns, of adequacy in managing drives, needs, and feelings, of maturity in handling relationships with others, of abilities in utilizing executive competence in carrying out goal-directed activities, of sophistication in integrating patterns toward appropriate functioning, and of the sense of identity.

Such assessment acts as a basis for interventions to increase the client's capacities and lessen limitations.

It enables the recognition of adaptive patterns that require strengthening as well as the recognition of maladaptive ones that necessitate change.

As it will be seen, such a method draws from psychoanalytic, social, and role theories and flexibly accepts supportive, reeducative, and reconstructive goals, recognizing that these must be graded to the motivations, resistances, and adaptive plasticities of the individual.

While unconscious motivation and conflict are accepted as key sources of behavior, and transference is acknowledged as perhaps inevitable, free association and dream analysis are not employed.

The focus is on resistances and defenses that operate currently, and there is greater emphasis on the present than on the past.

Behavior theory is also influencing the current dimensions of casework.

Some practitioners find in behavioral approaches shortcuts to dealing with specific symptoms and complaints, and some practitioners concentrate on group approaches that they believe are more practically attuned to the needs of clients.

Casework practices are not static but relate to continued advances in the field of the helping processes.

In recent years, for economic and other motives, caseworkers have been increasingly entering the field of "clinical social work" and moving into private practice as clinicians, offering increasing competition to psychiatrists and clinical psychologists.

RELATIONSHIP THERAPY AND "ATTITUDE THERAPY" Relationship therapy was the name given a half-century ago by John Levy (1938) to a process in which the focus of treatment was organized around the patient-therapist relationship.

While admitting projection into the relationship of many attitudes and feelings related to the past, interpretations were made in terms of feelings experienced in the present, F. H.

Allen (1934) also described a system of relationship psychotherapy with children that acted as a positive growth experience, releasing forces that make for more complete development. The work of Taft (1933) on the relationship aspects of casework may during this epoch be considered in this category.

As the name "relationship therapy" implies, the relationship is the vehicle that both promotes change and serves as a target for an inquiry into basic interpersonal patterns.

The therapeutic encounter constitutes for the patient a new experience with a human being, which permits full expression of needs and strivings without retaliatory injury or rejection.

The awareness gained in this unique setting helps patients readapt to their situations and achieve some of their potentials.

Actually, what has been described as www.freepsychotherapybooks.org 294 relationship therapy has been recognized as constituting a vital aspect of all therapies.

In "relationship therapy" the therapist provides a corrective emotional experience for the patient by absorbing the patient's neurotic behavior and not responding to it with expected anger or indignation.

Instead, a non-critical interpretation of the behavior is offered, with suggestions of an alternative style for handling oneself.

Sometimes the therapist will model a more constructive way of managing the environmental provocations that beset the patient.

Relationship therapy may be helpful for individuals who are unable to utilize expressive insight-oriented therapy and yet need to change destructive patterns.

This method is embodied, without labeling it as such, in many counseling, educational, and behavioral approaches in which the therapist adopts the stance of a non-punitive, helpful authority figure guiding the patient toward a more productive way of life.

It can be helpful in developmental and other crises.

Whether it can replace a punitive parental introject with a more tolerant figure and thus lessen hostility and guilt is difficult to say, but over a long-term period we may anticipate a softening of a harsh superego if countertransference is kept under control while the patient doggedly and sometimes brutally tests the sincerity of the therapist.

The term attitude therapy was used originally by David Levy (1937a) to describe a process of treating children by working with the disturbed attitudes of their parents.

At present, the term is sometimes employed to describe a reeducative procedure focused on the current dispositions of the patient.

Distortions in attitudes are examined, their origins discussed, and their present purpose appraised.

Following this, attitudes that make for harmonious relationships are introduced as topics for discussion, and the patient is helped to incorporate these as substitutes for morbid habitudes.

Thorne (1950), for example, suggests a reeducative method that identifies "core attitudes" that cause varied maladaptive secondary traits and characteristics.

The core attitudes, once identified, are neutralized by presenting opposite healthy sentiments to promote a reorganization of attitudinal constellations.

Since the patient will not yield old ways of thinking readily, repeated emphasis on a new point of view is essential to achieve desired results.

A systematic reconstitution is attempted of the patient's outlook as it affects various areas of www.freepsychotherapybooks.org 295 the patient's life.

Reinforcement of new attitudes is achieved by providing the patient with a corrective emotional

experience within and outside of therapy.

Some of these techniques have been utilized in the methods of cognitive therapy.

DISTRIBUTIVE ANALYSIS AND SYNTHESIS (PSYCHOBIOLOGIC THERAPY) Adolf Meyer (1915, 1948) of the psychobiologic school, emphasized that the human being was an “experiment in nature,” the product of an integrated fusion of somatic, neurological, and psychological organizations fashioned by social conditionings.

It was impossible, he claimed, to isolate any one of the many structures that made up the totality of a human being.

They all had to be considered in relation to the living individual and not regarded as detached units.

Furthermore, Meyer urged an empirical viewpoint, utilizing the contributions of any of the various branches of science that could shed light on the total behavior of the individual.

A person was to be studied as a functioning unit in society and as a part of nature by means of the scientific method.

Because a human being evolved from lower forms, it was necessary to investigate human physics, chemistry, biology, embryology, anatomy, and anthropology.

Because a human being differed from lower forms, it was essential to examine the more highly developed qualities that distinguished a human from the lower species.

The most intricate function was that of mental activity, which Meyer termed “mentation” or “the minding function.” This included the ability to sense, imagine, discriminate, communicate ideas, learn, recall, think, and reason.

In studying human behavior it was necessary to take into account everything significant in the life history of the person, including heredity, body build, temperament, developmental history, illnesses, traumatic experiences, and the interaction of the individual with parents, siblings, and other important personages.

It was necessary to consider intellectual, spiritual, and sexual development, school, vocational, marital, and community adjustments as well as interests, ambitions, moods, habits, and life goals.

Toward this end the therapist was enjoined to exploit all methods of diagnosis.

It was vital to understand that each individual had a different capacity for bearing stress.

Hence, it was necessary to evaluate the strength of the personality through an assay of one’s assets and liabilities.

The psychobiologic approach, with its emphasis on eclecticism and its consideration of every facet of the individual’s functioning as material for inquiry, had a great influence on the mental hygiene movement in this country.

It gave rise also to an eclectic therapy known as “distributive analysis and synthesis,” or “psychobiologic therapy.” In psychobiologic therapy (Billings, 1939; Kraines, 1943; Muncie, 1948), therapist and patient cooperatively engage in a systematic examination of all forces that go into the shaping of the patterns of the patient, namely, hereditary and constitutional elements, early childhood conditionings, and later experiential influences, including educational, economic, work, marital, interpersonal, and social factors.

An initial workup consists of a thorough exploration of both the patient’s problem and personality.

This usually involves an inquiry into the individual’s life history, a physical examination, and psychological testing.

A psychiatric social worker, clinical psychologist, internist, and psychiatrist can function together as a team here in making a proper study.

The character of the complaint, the history of its development, and the patient’s past and current attitudes toward it are discussed thoroughly.

All available sources are explored, with the help of a psychiatric caseworker if necessary, to determine hereditary, constitutional, and experiential elements of importance in explaining the patient's reactions.

The patient's social, sexual, work, educational, and recreational adjustments are considered, as are interests, ambitions, habits, cravings, and conflicts.

Since the therapeutic objective is the retraining of unhealthful attitudes and the elimination of immature reaction patterns, it is essential to obtain as clear an idea of the patient's personality in operation as is possible.

A physical examination is important not only to detect existing organic conditions, but also to reassure the patient and to inspire confidence in the competence of the therapist.

X-ray and laboratory facilities are utilized where indicated.

Psychological testing, particularly projection tests, yield data as to personality resources and liabilities, www.freepsychotherapybooks.org 297 existing anxieties and conflicts, and the nature of the patient's defenses against anxiety.

The initial workup is invaluable in estimating the best type of therapy to utilize at the start, the prognosis, and the possible duration of treatment.

It allows for a much more scientific approach to the patient's problems.

Factors divulged in the initial study, which have operated to mold the individual's personality and to produce the present disorder, are examined systematically in a series of interviews.

The past is considered important in providing an understanding of the makeup of the individual.

The past is translated, however, in terms of the patient's present attitudes.

The formulation of difficulties is couched in concepts consonant with the patient's current capacities for understanding.

The therapist always attempts to avoid leading the patient into material that he or she is unable to face, and the therapist tries to circumvent the stirring up of guilt or resentment that may interfere with progress.

The manner of thinking displayed by the patient, and general reaction tendencies, are carefully studied.

A life history chart may be constructed, detailing important facts in the patient's case, including family background, socioeconomic influences, and significant childhood and adult experiences.

Once a blueprint is obtained of the formative and presently operating influences in the patient's neurosis, the therapist strives to help the patient make constructive use of what has been discovered.

This is not done in a mechanistic and authoritative way.

Treatment following Meyer's inspiration is a matter of negotiation between patient and therapist for the most favorable solution.

In this process the relationship between therapist and patient is important to understand, always implicitly and at times for explicit analysis.

To the greatest degree possible I rely on the spontaneous contributions of the patient to keep the process going, but I do not exclude my right to bring up new or unfinished topics pertinent to the problem.

And always the historical roots of the problem in overt or covert material from the past are used to illuminate the points where education in living went astray.

Then comes the effort to discover unused learning opportunities— unused because of emotional blocks.

Dependence on the therapist is not denied the patient, but encouragement to assume the risks involved in the reeducation process is given.

There is constant feedback—as there must be in www.freepsychotherapybooks.org 298 meaningful

negotiation.

(Muncie.

1976) Discussions cover in great detail those facts that have influenced the patient and his or her problems.

The patient, one hopes, through these discussions, gains insight into the difficulty.

During each interview positive and constructive elements are accented, successes are emphasized, and hopeful elements are brought to the fore.

This is to counterbalance and to counteract negative, destructive forces and liabilities.

The patient's assets are constantly considered against liabilities.

A "synthesis" is then made of those factors that can help in adjustment, and the patient is encouraged to deal with life through a healthier perspective.

The patient is encouraged to correct disturbing environmental situations and to avoid or to control traits that adversely influence adjustment.

The patient is also inspired to develop adequate compensations.

As treatment goes on, and more material becomes available, the therapist's initial hypotheses may have to be reformulated.

A number of techniques (such as suggestion, guidance, reassurance, persuasion, and confession and ventilation) may be used jointly toward the goal of a more constructive adaptation.

If analytically trained, the therapist may deal with the more deeply repressed material, utilizing psychoanalytic techniques, such as free association and dream interpretation.

While the clinical material of psychoanalysis is accepted in psychobiology, psychoanalytic theoretical explanations are considered intuitive and metapsychologic rather than scientific.

Therefore, a broader biosociological explanation is attempted for which there is experimental evidence or reasonably assured probability.

Pointed questions are asked and areas of discussion are delineated, but the patient is stimulated to think things through for himself or herself.

The focus in therapy is usually on present situations and symptoms of which the patient is aware rather than on unconscious attitudes and mechanisms.

The relationship between the patient and therapist is explicitly analyzed only when found to be necessary, and no attempt is made to induce the patient to relieve past experiences, although an understanding of present reactions in the light of past conditioning can be helpful.

www.freepsychotherapybooks.org 299 These techniques may, in an incredibly short time, help to restore a patient to an emotional equilibrium, symptom free and capable (because of the knowledge that has been gained) of avoiding pitfalls that have hitherto created anxiety.

Additionally, the patient may learn to utilize his or her assets to best advantage and to get along in life far better than before.

Correcting difficulties in relationships with people may relieve much anxiety, tension, and hostility.

Knowledge of his or her character weaknesses may make life more tolerable.

Discovering positive qualities within himself or herself help raise the patient's pathologically low self-esteem.

The dynamic sources of the individual's emotional problem are sought to the degree necessary to achieve understanding.

The relationship with the therapist may inspire curative forces that influence personality growth.

The philosophy and methods of psychobiologic therapy have been incorporated into the body of many, perhaps most, of the current psychotherapies without giving credit to the rich contributions of Adolph Meyer, Wendell Muncie, and other pioneers in this field.

INTERVIEW PSYCHOTHERAPY (PSYCHIATRIC INTERVIEWING) "Psychiatric interviewing" is the

appellation attached to certain interview procedures that attempt to bring the patient to a state of awareness by focusing the interview on pertinent problems.

Most types of psychiatric interviewing are forms of insight therapy with limited goals—focused or conscious stresses and conflicts—since deeply repressed elements of personality are not exposed to awareness.

Transference material and dreams are utilized rarely, if at all, as foci for interviewing.

What is essential is an effective doctor-patient relationship, the employment of goal-directed planning and management, the focusing of content on specific aspects of behavior, and use of minimal activity (Fine-singer, 1948).

The ultimate goals in treatment are formulated in advance, considering the preliminary diagnosis, the patient's needs, and the therapist's clinical experience.

Intermediate goals are determined by what is happening in the therapist-patient relationship and by the kind of material that is brought up during discussions.

Focusing of www.freepsychotherapybooks.org 300 the interview is generally, at first, along lines of an inquiry into existing symptoms, attitudes, and problems.

Next, there is a search for repetitive patterns in the patient's behavior.

Then, the effect of habitual patterns of current behavior is explored with an investigation of the meaning and function of such patterns as well as their historical origin.

Flexibility must be observed in intermediate goals, and there is usually a shift from one to another, determined by the needs of the situation.

Interpretations are made for the patient, as required, and, from time to time, there are summarizing statements.

A focusing of the patient's attention on relevant topics is essential to avoid rambling.

This may be achieved by exhibiting interest or by displaying disinterest in specific topics brought up by the patient.

The therapist employs as minimal amounts of activity as are consistent with therapeutic plans and goals.

Only when the patient fails to respond does the therapist become more active.

A careful regulation of the relationship is essential, being balanced between emotional support and the stimulation of tension to activate therapeutic progress.

Interviewing procedures consist of studied nonverbal and verbal responses.

Included in the former are facial expressions, nodding, glances, gestures, postural changes, and vocal inflections and intonations.

Low-activity verbal responses are preferred; they consist of articulate syllables with rising inflection, repetition of the patient's last uttered word, elaboration of a phrase, mild commands, and questions of a general or specific nature aimed at exploring a certain topic.

Greater activity is required only where material is not readily obtained.

Technical procedures here include a repetition of the patient's statements with special emphasis, rearrangement, or juxtaposition, statements of a descriptive, elaborative, or summarizing nature, and direct questions in relation to associations.

Difficulties in communication may be handled by questions related to the manifest problem and by mild or more active encouragement to talk.

Methods of marked activity are used only when absolutely essential.

These consist of suggesting reasons for reticence, interpreting the reasons, provoking emotional reactions through rapid probing, focusing on the relationship,

www.freepsychotherapybooks.org 301 forcing the patient to verbalize material, or displaying affect.

Extreme active measures are rarely indicated, such as active reassurance, shared experiences, gratifying the demands made by the patient, and shifting the relationship in the direction of a

social experience.

A less structured method of therapeutic interviewing, directed toward goals of insight, has been described by Stanley Law (1948), which he believes can be learned and practiced by general practitioners.

SEMANTIC APPROACHES TO THERAPY J.

Ruesch (1957, 1959) and R.

Spiegel (1959) have hypothesized that abnormal behavior is accompanied by physical and social interference with communication.

The more intense and repetitious the interference, the more extensive the traces that exist to plague the person.

Therapy, focused on improving the patient's communication process, may lead to a better ability to relate interpersonally, and also to acquire a more cogent understanding of oneself, in regard to one's inner life, past experiences, and current interactions.

Formal study of human evaluative processes, particularly in relation to signs and symbols, is the goal of the field of "general semantics." Semantics deals with systems of symbols, including language structures, and the uses made of these systems by individuals and social groups, as well as the influence on the systems of existing social values and individual behavior tendencies (Korzybski, 1941).

Therapeutically employed, semantics tries to teach the individual the principles of scientific thinking through a more valid use of communication.

The rare employment of semantics as a therapeutic method necessitates a high degree of intelligence in the patient and requires special semantic training for the therapist.

During treatment, a mutual examination of verbal forms used by both therapist and patient is conducted to see whether exact meanings are being communicated.

There follows a detailed evaluation of the patient in relation to language efficiency and the prevalence of semantic problems.

Finally, semantic retraining is employed to teach the www.freepsychotherapybooks.org 302 patient the more exact use of symbols and terms.

Adequate problem solving presupposes a sufficient mastery of language and use of symbols to enable the person to state difficulties, differentiate them, make inferences, and draw conclusions from facts.

The person must know something about the uses and abuses of symbols.

These are the foci of investigation in semantic therapy.

W.

Johnson (1946) has outlined the effect of semantic difficulties on adjustment.

One syndrome of maladjustment is the product of unrealistic ideals in life which bring about disappointment and frustration.

Part of the problem is due to the doubt that governs the individual's ideas, making it impossible for the person to conceptualize clearly what he or she wants from life.

The person is, therefore, inevitably disappointed in efforts to gain pleasure in living.

Another source of maladjustment is the inability to identify or even state problems coherently because language is poorly organized and lacking in clarity.

This vagueness in thinking, embracing faulty notions about oneself and fallacious concepts of the meaning of life, interfere with the formulation of realistic goals.

Semantic distortions include oververbalization, which may serve the function of avoiding silence, of concealing the truth, or of searching frantically for meanings.

Underverbalization is another response, which is often motivated by a fear of failure, demoralization, or perfectionistic strivings.

There may be great verbal rigidity involving content, modes of phrasing, and moods.

There may also be a defect in the capacity to discriminate levels of verbal and nonverbal abstracting.

Other disturbances of communication occur that require careful analysis and diagnosis.

In therapy, once a good relationship with the therapist is established, semantic training and reorganization are practiced.

Interpretations are given the patient as the semantic defects become apparent.

Training proceeds as rapidly as the patient's resistances will permit.

As the patient becomes capable of formulating the problem cogently, he or she greatly expands the ability to communicate with freedom and clarity.

Verbal rigidities relaxing, the patient becomes more conscious of the real difficulties, feelings about oneself, and life goals.

By being able to put the problem into meaningful www.freepsychotherapybooks.org 303 words, the person is helped to achieve greater volitional control over emotional processes.

When we evaluate the semantic approach to therapy, we have to consider that all emotional ailments are associated with problems in communication.

These involve, among other things, vagueness in phrasing and defects in conceptualization.

As long as there is an unclearness in the use of symbols, the individual is unable to define, differentiate, or think critically about his or her attitudes, values, and life goals.

It is likely that a good part of the existing communication difficulty in emotional illness is the product of repression—a purposive attempt to cloud issues in order to keep painful aspects of the personality outside the range of awareness.

During any kind of psychotherapy patients, by mastering anxiety, become capable of expressing themselves more and more clearly and of verbalizing in increasingly explicit terms that which they had hitherto merely been able to feel.

This restores the patients' ability to deal more adequately with the sources of their problems.

In summary, semantic approaches to therapy put the emphasis on the patient's difficulties in communication.

Emotional illness is believed to arise from problems of symbolic functioning.

The focus of the therapeutic effort is on defining and clarifying symbols, making the patient's use of them more precise.

As the individual formulates ideas more clearly, interactions with others become more meaningful.

REEDUCATIVE GROUP THERAPY Group therapy may have the reeducative goal of altering attitudinal and behavioral patterns (Pratt, 1907; Slavson, 1944, 1946, 1947; Solomon & Axelrod, 1944; Rome, 1945; Luchins, 1947).

The group setting offers the patient a splendid opportunity to relate to others and to observe distortions that develop in such relationships.

Benefits accrue to the patient from experiences with other members within the group.

A protective as well as challenging milieu fosters the growth and expansion of cooperative attitudes that may eventually replace neurotic strivings.

Dynamics derived from a study of the process of individual psychotherapy do not www.freepsychotherapybooks.org 304 seem to suffice for such group experiences.

In searching for a comprehensive and operational theory of reeducative groups, we are obliged to enter the field of sociological research.

Some efforts here have been made to apply research in group dynamics to therapeutic groups (Bales, 1950; Cartwright & Zander, 1960).

In recent years communication theory and systems theory have added some important dimensions (Durkin, 1975).

Group therapists contend that the group experience can liberate and give direction to what is

socially significant in the individual.

A person is by nature a group creature and needs to associate closely with others in order to operate adequately as a human being.

Unfortunately, disturbances in relationships of emotionally ill persons eventually alienate them in the group, i.e., society.

The therapy group provides an atmosphere for resolution of some of these distortions.

An overcoming of neurotic impulses may follow experiences of becoming less defensive toward others.

This may lead to greater tolerance of personal lacks and failings.

During the past few decades a great deal of work has been done, particularly by social psychologists and sociologists, on how people relate in groups.

Description of the observable variables in a group and delineation of the dynamics of interaction are the contribution of the field which has come to be known as group dynamics.

This field is distinctive from that of personality dynamics, of which psychoanalysis is the best known representative, which, applying itself to the why of human behavior, deals with the motivational determinants.

Some of the data from studies of group dynamics are applicable to therapeutic groups.

Unfortunately, there is still very little communication between group therapists and social psychologists.

Perhaps the major contribution to group dynamics was made by Kurt Lewin, who, utilizing field therapy, focused attention on the complex and shifting nature of group life (1947, 1948, 1951). Lewin emphasized that in a group there is an interdependence of the individuals, which characterizes the "dynamical whole," and that a change in one subpart can effect change in the state of any other subpart.

A group was thus more than a sum of www.freepsychotherapybooks.org 305 its parts; it had unique characteristics of its own.

Individual psychodynamics could not explain behavior since the individual was subject to social forces in the form of pressures by the group.

Such group pressure could effectuate changes in behavior.

Under Lewin's leadership, experimental studies attempted to delineate the internal structure, processes, phenomena, and laws of group life as well as to apply these data to such practical problems as group productivity, leadership, and cohesiveness, which occurred in industry, education, correctional work, and other fields.

The research and bibliography in group dynamics have been substantial, including among others the work of French (1941, 1944), Festinger (1942, 1947), Bavelas (1948, 1952), F.

Deutsch (1949, 1951), Cartwright (1950), Homans (1950), Bales (1958), and Hare (1962).

The contributions of Bion (1951) have been especially noteworthy.

Bion pointed out that members of a group at first look for a leader on whom to depend but soon realize that this is a fantasy and that one must look to oneself for survival.

They then form pairing couples, which also leads to disappointment when one realizes that no one can be fully depended on to fill a hoped-for role.

This encourages impulses toward fight (quarreling among themselves) or flight (leaving the group).

Working through these impulses can have a therapeutic effect.

The concern of psychotherapists with group dynamics is predicated on the basis that behavioral changes are constantly being consummated through the individual's interactions with family, peer, occupational, religious, and other groups of which he or she is a member.

Processes may thus be observed taking place within the individual in his or her relationship with others, in the course of which certain kinds of change come about.

A study of the dynamics of change illuminates some of the allied operations in psychotherapy. Moreover, it helps differentiate the contingencies that precipitate out in all groups, irrespective of structure, from those that are specifically parcels of the psychotherapeutic experience.

Common to all groups are a number of phenomena: (1) all groups possess some kind of structure, (2) the members assume or are assigned special roles, (3) goals toward which the group strives are implicitly accepted or explicitly defined, (4) a communication network mediates the interactions among members, (5) group norms are applied with varying pressure to each individual (social control), and (6) both cohesive and disorganizing forces are at all times operative.

Dynamic interaction is the essence of group activity.

Never static, the group constellations alter themselves as new fusions, enmities, and alliances allocate different roles for the members.

A status hierarchy emerges, which determines the nature and direction of communication.

Interacting patterns are evolved that reflect role expectancies.

Observation of different groups in operation discloses a number of consistent processes (Wolberg LR, 1966): 1.

Individuals upon entering a new group bring into it all of the distortions and expectancies that are parcels of their personality structure, while attitudes directed toward them by the other members similarly reflect their prejudices.

These immediate impressions are rectified as interaction continues, perceptions tending to become more reality-oriented ("consensual validation").

2.

A status struggle often occurs at the beginning of group formation during the establishing of leader-follower hierarchies.

Members reach out for leadership on the basis of a number of inner needs, such as identification and dependency.

Leadership characteristics are not the same for all groups; they depend on the culture and needs of the group.

Intelligence, dominance, self-confidence, vitality, the ability to relate to the goals of the group, and the capacity to participate socially have been found to be important in all groups.

3.

The norms developed by the group represent rules of behavior designed by the group to achieve its expressed or implied goals.

Group norms applied to an individual define his or her role expectations.

The individual also possesses norms that are related to personal values and goals, which may or may not conflict with group norms.

4.

All groups approach one or more of four goals: (a) the group goal of problem solving in relation to some area of concern involving all members (as in a parent-teacher or industrial group), (b) the group goal of resolving expressed or undefined social-emotional problems, for example, shared anxieties (as in an executive-training group where group dynamics are emphasized), (c) the goal of supplying an individual in the group with a solution to a personal need (as in a social or educational group), and (d) the goal of solving emotional difficulties associated with personal and relationship disturbances (as in a therapeutic group).

5.

The structure of the group, and the activities that it sponsors, will, more or less, be modeled by the goals toward which the group directs itself.

Generally, goals relate to resolution of an explicitly defined external problem that concerns the group or to a less well-defined social-emotional problem within the group that is reflected in shared anxieties.

6.

Group size influences the responses and movement of the group.

As numbers in the group increase, information and suggestion giving become more pronounced. This is accompanied by a diminished request for and expression of opinions and by a lessened agreement among members.

Groups of two (dyad) show high tension, avoid antagonism and disputation, ask for opinions (but shy away from giving opinions), and focus on the exchange of information and on reconciliation. The optimal size of a group is five, a smaller or greater number being less satisfactory to members.

Discontentment is expressed if the group gets too large.

Each additional member expands the potential relationships among the individuals and subgroups. As the group increases in size, members feel inhibited and threatened, and the leader becomes more removed from the members.

Greater difficulty exists in reaching a consensus.

7.

The superiority of group over individual performance has been tested.

It has been found that manual productivity is greatest in groups, but intellectual productivity is not necessarily increased.

Problem solving within a group framework is enhanced in some individuals but retarded in others.

Group discussion, however, makes individual judgment more accurate.

Recall of information is expanded in a group setting.

8.

A variety of situations make for greater group productivity.

Productive groups are composed of members whose skills are appropriate for the tasks.

The most resourceful and accomplished groups are small, are cohesive, are of the same sex, have a satisfactory communication network and feedback, and are led by a skillful leader.

Autocratic leadership encourages greater quantitative productivity, while democratic leadership results in optimal morale.

In authoritarian settings authoritarian groups achieve greater output; in democratic settings egalitarian groups are more efficient.

www.freepsychotherapybooks.org 308 Personality characteristics that result in compatibility of individuals in a group and in free communication make for expanded group productivity.

Tasks are best accomplished by a group in which rules are appropriate for the tasks.

Where cooperation is expected, there is greater individual incentive, friendliness, communication, and productivity; self-oriented needs in a member here tend to disrupt the group.

Application of stress, if not too strong, encourages greater productivity; if too strong, the group yield diminishes.

Productivity is retarded by any conflicts that develop in the group.

9.

Conflict often appears when the roles of two members clash ("role collision"), where an individual plays two opposing roles ("role incompatibility"), and where group members cannot agree as to expected roles ("role confusion").

It also develops where the personality structure of an individual does not coordinate with the role expectation that is dictated by his or her position in the group.

Ethnic, status, intellectual, and educational differences among the group members may act as sources of tension and conflict.

10.

Cohesiveness is expanded and disruptive forces are minimized where the members know and like each other, where the prestige of the group is stressed, where members are rewarded on a cooperative rather than competitive basis, where they possess strong mutual interests and are democratically led, and where communication processes are facilitated.

11.

Both deviance and conformity are present in members of a group.

The more highly the individual regards the group, the more he or she needs its prestige, its output, or the friendship of its members—the more he or she will want to conform.

Interactive processes in the group are released by a deviant.

This has an effect on group morale, group decisions, and group cohesiveness.

Every group exerts pressures on its members to conform to accepted norms.

Where a member deviates in behavior from the norm, that person is subject to one of four choices: conform with group norms, change norms to align them with those of the group, operate as a deviant, or retire from the group.

Deviant members are pressured to yield to majority opinion, which some will accept.

Conformity is encouraged where the majority maintaining a contrary view is large, where membership in the group is valued, and where one's opinion must be stated publicly.

Yet a minority viewpoint is possible where the minority possesses high status, is considered expert, or is especially popular.

Thus, a leader may promote his or her ideas in the group and find them

www.freepsychotherapybooks.org 309 accepted even though they clash with sentiments of the group majority; yet the leader will also have to abide by group norms once they are solidified.

Membership in a group influences the way an individual perceives reality, his or her decision making, and the nature of his or her values and prejudices.

Often the individual will suppress a correct perception or judgment in favor of the majority opinion, particularly from respected sources, firmly convinced that conclusions have been independently reached.

Where a person is highly motivated to remain in a group, he or she is more prone to resist arguments, however logical they may be, that are opposed to the group norms.

This principle has been found empirically useful and probably accounts for certain dramatic changes, hardly possible in individual therapy, that come about in therapeutic and some social groups (Alcoholics Anonymous, Synanon, etc.).

Group dynamics share with psychotherapy the common goal of altering norms in the individual.

The manner in which this change is implemented differs in the two disciplines.

Psychotherapy approaches the problem of change by dealing directly with the forces of emotional motivation and conflict; group dynamics operate more peripherally through the influences of group interaction and pressure.

Each of the disciplines complements the other.

During individual psychotherapy the patient continues to be permeated by pressures from family and peer and secondary groups.

In group psychotherapy the individual is in addition subjected to group dynamic vectors.

The pressures of the group for individual conformity are great.

A problem posed here is that if group standards are abnormal, it will be difficult for an individual to change a pattern supported or sponsored by the new group.

Should behavior be altered in isolation, the deviancy will come under attack when returning to the old group, and one may be forced to assume a previous role.

Thus it would seem, as Lewin pointed out, a group change may have to precede transformation in attitudes in a group-rooted individual.

One may, however, retain the gains by leaving the customary group for a group whose norms are more consonant with the newly developed ideas.

Or one may assume a leadership role and change the norms of the habitual group to coordinate with one's own.

www.freepsychotherapybooks.org 310 R.

Dreikurs (1950) speaks of the "unique social climate" experienced in group psychotherapy.

Among its distinctive qualities is the fact that the therapy group acts as a natural testing ground for observation and experimentation with the phenomenon of group values.

It has been discovered that the values operating in a therapy group geared toward reeducational objectives are best democratic or egalitarian, as each member, regardless of achievements or deficiencies, is granted equal status and rights in the group.

Another value stressed in the social climate of the group is honesty and frankness, the ability to reveal oneself freely without putting up a front.

The therapist, too, functions as an equal among equals, who can assume the responsibility of leadership because he or she knows more about human relationships than the other members of the group.

Binding the group members together is the need for mutual trust, for belongingness, and for mutual identification.

In addition to the phenomena of group dynamics, which operate in all groups and on which the therapeutic results of educational groups are to a large extent dependent, Slavson (1957) has pointed out that "in therapy groups there constantly occurs verbalized and nonverbalized interpersonal action and reaction, partly as a result of therapeutic projections and partly because of the inevitable effect persons have upon one another, such as contagion, mutual induction, interstimulation, intensification of emotions, sympathy, empathy, and others.

These cannot be considered, however, as group dynamics but rather as interpersonal interactions." Interpersonal factors are constantly operative in groups and complement the effect of group dynamics as well as the more irrational transference phenomena that are exploited in analytic groups.

As each patient interacts with other members and behavior is discussed frankly, and often with affect, he or she gains greater self-understanding by allowing feelings to be harbored and expressed in an accepting climate.

The group may stimulate destructive patterns in interpersonal relationships, but eventually the members may be able to participate without exploiting their neurotic character drives.

For instance, a powerhungry person may learn in time to merge closely with the group without being controlled or hurt.

A negativistic individual may discover that yielding to others does not mean

www.freepsychotherapybooks.org 311 abandoning one's personal rights and freedom.

A detached patient may derive positive pleasures from slowly entering into group activities.

As group members relate to each other freely, they learn to compromise as well as to give and receive.

Thus they begin to develop cooperative participation, helpfulness, and friendliness, which may in time replace destructive interpersonal feelings.

H.

T.

Glatzer and Pederson-Krag (1947) have emphasized the importance of these interpersonal patterns.

The group setting provides individuals with a theater in which to play out their impulses and

fantasies.

It is not uncommon for group members to create surrogates (father, mother, siblings) and to project transferentially onto them archaic needs and defenses.

These, when detected, may be interpreted to the patient by the therapist.

Immediate emotions are dealt with and handled, particularly those the patient can face without too much anxiety.

In this way, the patient gradually becomes aware of his or her feelings and even of some of the origins of neurotic conflicts.

The object is to help people come to terms with themselves.

Activity Group Therapy Slavson (1943) has described a type of reeducative therapy called "activity group therapy," which is used with children in the medium of a social club.

Complete permissiveness prevails and the youngsters are free to express all types of behavior. This usually results in a cathartic ventilation of feelings.

Arts and crafts materials are supplied, refreshments are served, and outings are arranged.

Children's activity groups are usually composed of eight to ten members whose ages are relatively comparable; for instance, children from 6 to 9 may constitute one group, those from 9 to 13 another.

The participants are encouraged to be freely emotive and expressive.

There is no special focusing on problems, although aggression, destructiveness, withdrawal, and other disturbed kinds of behavior that endanger the child or others are dealt with in a firm but kindly manner.

A permissive and understanding environment is the keynote.

The setting of limits is essential, for instance, in regard to destroying property, taking things from one another, and persistent fighting.

The group functions as a secondary family, with the therapist as substitute parent.

Relationships established with the therapist and with others in the group may lead the members to a more realistic conception of the world and www.freepsychotherapybooks.org 312 of themselves.

This awareness is gained within the action setting, and not through interpretation.

Activity groups with adults have also been developed around special interests and needs of the participants.

Thus, sports, crafts, games, photography, painting, sketching, carving, mosaics, and other hobbies and recreations may form the focus around which the group is organized.

In hospitals, day hospitals, and therapeutic communities, occupational therapy (see Occupational Therapy), special programs of exercise (Coulter, 1966), and social clubs (Bierer, 1944, 1948) may be the means of bringing together persons who eventually begin to relate to each other.

In outpatient set-ups, dramatics, music (see Music Therapy), dance and movement (see Dance and Movement Therapy), poetry (see Poetry Therapy), and social activities (see Social Therapy) may constitute the central diversion that enables the audience to communicate and to release their feelings.

Caseworkers have developed a social group work process around the rehabilitative needs of their clients (Hendry, 1948; Konopka, 1947, 1960, 1963; Kunstler, 1955; Lindeman, 1939; M. Murphy, 1959; E.

Phillips, 1957; Schwartz, 1959; Trecker, 1955; G.

Wilson, 1941; Wilson & Ryland, 1949).

This provides enrichment of school-day and after-school programs as well as activities for the handicapped and the aged.

Health education, recreation, camping, athletics, dramatics, arts and crafts, social dancing,

forums, lectures, and other constructive measures are combined with skilled guidance to meet the various needs of clients.

The introduction of an activity provides structure to individuals who otherwise would be too inhibited to interact.

Directive-Didactic Behavioral Group Approaches Bringing small groups together to discuss matters of special interest to the members constitutes an important reeducative measure. Such topics may concern health, marital relationships, child-parent difficulties, and social and community problems of the widest range.

During the sessions the participants bring up subjects for discussion which may then become the central theme.

In less active groups the therapist may deliver prepared talks or lectures around which the deliberations may proceed.

Even Bible-reading classes www.freepsychotherapybooks.org 313 have formed the basis for therapeutic assemblages (Marsh, 1931).

There is some advantage to organized presentations in settings where personnel changes are the rule, and relatively little disruption is experienced when the regular group leader is replaced by a new leader who continues with mimeographed or printed material.

Books and readings may be assigned as part of the proceedings.

A tendering by individual group members of their problems and histories serve as periodic interesting diversions (Pratt, 1953).

Sometimes group relaxation (Jacobson, 1938) and group hypnosis (Wolberg LR, 1948, p. 180) precede talks and discussions.

These methods have the advantage of calming the group by enhancing the placebo effect and putting the therapist in the role of a benevolent healer.

Persuasive arguments may better be received under these circumstances.

Some therapists who work in hospitals have stressed the importance of "will" therapy, which emphasizes self-help and encourages the continuance of group discussions after patients leave the institution.

An example of such a self-help group movement is Recovery, Inc. (Low, 1950).

In groups led by a therapist dedicated to behavior therapy, group hypnosis with desensitization techniques may be employed, particularly with phobic patients (Lazarus, 1961).

Behavioral groups are usually most effective with patients sharing common symptoms, such as agoraphobia, fear of flying, a smoking habit, obesity, sexual problems, or deficiencies in assertiveness.

Behavioral techniques may then be appropriate for all the patients.

Subgroups are usually organized on a short-term basis for up to 6 months.

Among the group therapy approaches incorporating principles of learning theory (Mowrer OH, 1950, 1953) are those developed by Saslow and Matarazzo (1962) and Small et al. (1963), which regard emotional difficulties as learned disturbances subject to extinction, reinforcement, and reeducation.

In hospital setups didactic groups have been operating for many years, providing special topics and guided discussions for patients who are too detached, regressed, or deteriorated to enter into spontaneous interactions (Marsh LC, 1931, 1935; Klapman, 1946; Low, 1950; Klapman & Lundin, 1952).

Persons who have been apprehended or www.freepsychotherapybooks.org 314 imprisoned for antisocial tendencies, such as sex offenders, juvenile delinquents, prisoners and paroled convicts, often benefit from guided group conferences.

Alcoholics and drug addicts profit from didactic approaches combined with

repressive-inspirational measures, such as practiced in some Alcoholics Anonymous and Synanon groups.

nondirective (Client-centered) Group Therapy Therapists espousing the philosophy that inherent self-actualizing tendencies may be released in an atmosphere of permissiveness, "genuineness," and empathy have applied their concepts to work with groups (Hobbs & Rogers, 1951; Hobbs, 1969; Gordon, 1955; Lifton, 1961).

In accordance with client-centered theory, the therapist operates to confront, guide, and clarify feelings and distortions that prevent the group from realizing itself as a constructive body that can release the growth potentials of each member.

In an atmosphere in which the free exploration of feelings and communication to other members is encouraged, self-understanding and self-acceptance emerge.

The activities of the therapist are geared toward conveying to the clients "through gesture, posture, facial expression as well as by verbal means, the therapist's congruence, his sense of acceptance and of confidence in the ability of the client, with the help of the group, to resolve his problems... .

The therapist does not interpret, probe, evaluate or reassess... (Smith et al, 1963).

Family therapy It has been said that a family is an autocracy ruled by its sickest member.

Often this member is not the "primary patient" who is sent out for help.

What we generally observe within the family structure is an unstable homeostasis maintained through interlocking neurotic roles that the various members play with one another.

Such roles are usually concealed from the awareness of the participants by perceptual blocks, and their resolution is staunchly resisted, particularly in families with either a psychotic or borderline member.

The evolving network of interlacing patterns is remarkably stable, and sometimes contagious, being passed along in almost pure culture from one generation to the next.

Other patterns are constantly being influenced and modified by defenses and
www.freepsychotherapybooks.org 315 reaction formations.

Affectionate, supportive, domineering, submissive, exploitative, punitive, aggressive, indulgent, cooperative, sadistic, masochistic, detached, seductive, inhibited—these and other traits are adopted through identification with parents or are compounded in the crucible of family life in response to the pressures, deprivations, and indulgences brought to bear on the individual.

Constantly in play are configurations that operate in the service of both healthy and neurotic adaptation.

The permutations of traits among the parents and siblings become diverse as controllingness is met by compliance or rebellion, overprotection by helplessness or compulsive independence, neglect by defiance or distrust.

Incompatibility between father and mother, between parental and social values, between intrinsic family standards and the laws of the prevailing subcultural group generate complex subpatterns in all of the inhabitants in the home.

These, derived from the incorporation of and resistance against attitudes present within the family, often act as sources of conflict.

Neurosis or psychosis that breaks out in one or another of the members is merely an event in the family drama and may actually constitute a means of helping the constituent members achieve a kind of tenuous balance.

Should the developing problems precipitate intolerable symptoms in a member, he or she may be the first emissary (the "primary patient") from the disturbed family group.

It is apparent from this that disabling emotional problems in any single individual must be approached within the context of relations with the family group.

The patient's difficulty is usually a symptomatic manifestation of a disorganizing family unit. Only rarely do we find that only one member of a family is emotionally ill; generally several and even all members share the disturbance that manifests itself most dramatically in the "primary patient."

"The emotional equilibrium of a subgroup may be maintained by the sacrifice of one member or another subgroup in the home.

The "scapegoating" of selected persons is a common phenomenon in establishing a balance, which easily becomes disrupted should the victims begin to emancipate themselves.

Thus mother and daughter may war against father, who seeks his peace in alcoholic overindulgence, the latter becoming the grievance around which battles are fought.

A father's competitiveness with his son, with subtle www.freepsychotherapybooks.org 316 imposition of expectations beyond the resources of the child, may drive the latter into a schizoid retreat, which then marshals further aggression from the father and detachment by the child.

Subgroup splits may be along several lines, varying roles being assumed as the alliances are joined or relinquished by changing needs.

New alignments are encouraged by a host of forces, including the maturational demands of growing children as they proceed from the helplessness of infancy through the defiant turmoil of adolescence into adulthood—as well as disintegrative trends in contemporary society itself, expressed in pressures toward conformity, impingement on personal freedom, and social upheavals that interfere with the sense of identity, feelings of security, and the need for belongingness essential for the requirements of a stable personality.

Manifestations of alienation and conflict may take the form of minor engagements between parents and siblings, with dogfights and cursory clashes; or they may be expressed in violent prolonged eruptions.

Generally, however, a cold war of attrition is waged between family subgroups, open combat being avoided because of guilt feelings or fear of hostility.

This subversive warfare is most wearing on the belligerents; eventually the weakest members break down.

Encouraging such collapse is a precipitating event, such as the imposition on the individual of demands that require a new role, for example, a change in occupation, death of a family member, sexual maturation, marriage, or the birth of a child.

The family pathology may manifest itself either in outright neurotic or psychotic disorganization of one or more members.

Or the family itself may fail to fulfill its traditional function of child rearing and responsibility to the community because of such disruptions as abandonment, unwarranted divorce, and delinquency.

The family then, as the cradle of health and illness, must be considered in any appraisal of individual emotional illness.

It is both the fount of neurotic contagion and a potential resource for therapeutic intervention.

Acceptance of this doctrine has sponsored a systematic family approach, which has proved to be productive in both diagnosis and treatment of individuals and groups.

Even though therapy ultimately may be focused on one person, a recognition of the family vectors operating on that individual in the past and www.freepsychotherapybooks.org 317 present is vital.

Bringing the family together and working with various members who need help has, in many instances, resulted in a better understanding of one another and in an improvement of their mutual relationships.

The origins of family therapy are vague.

The field of social work for years dealt with individual problems in a family setting.

It was not a coincidence that Nathan Ackerman, who worked with a child guidance social agency over fifty years ago, began exploring the possibilities of the family as a vehicle for growth as well as pathology.

J.

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Bell (1953) and Jackson (1959) published some of the earliest ideas on the family as a focus for treatment, and in 1958 Ackerman made a seminal contribution with his book on the psychodynamics of family life.

In the short time of its existence, family therapy has witnessed the evolvement of a surprising number of models (Ackerman, 1954, 1957, 1957b, 1958a, b, 1960, 1961, 1965; Albert, 1960; Bateson, 1959, 1960, 1961; Bateson and Jackson, 1963; Bell JE, 1961; Bell & Vogel, 1960a, b; Boszormenyi-Nagy, 1962; Boszormenyi-Nagy & Framo, 1965; Bott, 1957; Brim, 1960; Carroll, 1960; Ehrenwald, 1963; Elkin, 1962; Ferreira, 1960; Fleck, 1960; Framo, 1962; Friedman, 1962; Fry & Heersema, 1965; Goodman JA, 1962; Grotjahn, 1960; Haley, 1962a, b, 1963a; Jackson & Satir, 1961; Jackson & Weakland, 1961; Kadis & Markowitz, 1963; Lidz, 1958; Masserman, 1959; Parloff, 1961; Rubinstein, 1964; Satir, 1963, 1964; Spiegel, 1958; Watzlawick, 1963; Weakland, 1962).

While no specific methodology has emerged from current work in the field, various strategies of relationship and communication have been developed, such as split family, intergenerational, couple, individual as well as conjoint family sessions (Stein, 1966).

Flexibility in approach is most helpful (Deutsch D, 1966).

Treatment of family members or a spouse may follow individual therapy ("consecutive therapy").

Treatment may be done by different therapists ("collaborative therapy") or by the same therapist who utilizes both group and individual treatment ("concurrent therapy").

Treatment may involve the entire family in the same room ("conjoint therapy") as well as individual treatment for one or more members ("combined therapy").

It may involve the treatment in the same room of more than one family ("multiple family therapy").

It may employ co-therapists or www.freepsychotherapybooks.org 318 multiple therapists when necessary.

Procedures are improvised in accordance with the clinical perceptiveness, theoretical orientation, inventiveness, and idiosyncratic working methods of the different therapists.

For instance, some approaches draw their substance from social and role theory (Murphy G, 1947); communications theory (Watzlawick et al, 1967); general systems theory (Bertalanffy, 1968); intergenerational systems theory (Bowen, 1978); social learning (Weathers & Liberman, 1978); interpersonal perception (Liang et al, 1966); behavior theory (Patterson et al, 1975); psychoanalytic theory, with an accent on Freud, Adler, Horney, or Sullivan, contingent on the favored school of the therapist; transactional analysis (Berne, 1963); or Gestalt therapy (Perls et al, 1951).

Irrespective of orientation, the family therapist seeks to eliminate destructive symbiotic bonds that tie family members together.

The aims of family therapy are not only to resolve current problems and difficulties but to teach the family to solve further impasses by itself, to communicate more clearly with one another, to develop strategies for avoidance of conflicts, to accept one another's handicaps and differences without blaming and scapegoating, and to foster "functional processes for appropriate balancing of competing values such as family cohesiveness and individuation, balanced separateness and togetherness, stability and flexibility, to name only a few" (Bodin, 1984).

Family therapy is of prime importance in crisis intervention (Everstine & Everstine, 1983). Indeed there is no limit to the conditions for which family therapy may be suited since family relationships affect and are affected by the entire host of afflictions to which people are subject.

The essential separation on which maturity rests is usually impeded unless there is a working through of aggressive feelings the members have toward each other (Rubenstein et al, 1966). It is to be expected that the family at first will resist altering its fixed patterns of interaction.

Attempts will be made to coerce the therapist into supporting the neurotic cohesiveness and divisiveness that fosters the family neurosis.

Improvement in one member of a family may be resisted and sabotaged by the others who need the identified patient to be ill to maintain their own and the family's homeostatic balance (Lewis JM, 1984).

The resistance will be especially great in motherwww.freepsychotherapybooks.org 319 child symbiotic patterns, and particularly in overcoming "double-bind" verbal and nonverbal communicative links (Bateson et al, 1956; Jackson DD, 1959).

It may be possible to detect in the family relationships how parents communicate their unconscious demands to their children.

The therapist will, at first, be enlisted to serve as judge and to allocate blame.

As patterns are challenged, new alliances and fusions are sought.

The therapist is then promoted to the role of omniscient adviser who can provide answers and gratifications.

When these are not forthcoming, new subgroups and pairings organize spontaneously, around which different patterns of conflict develop.

The working through of these collusions eventuates in more realistic attitudes.

Establishing better communication among the members helps them to allay guilt, to resolve defensiveness, and perhaps to realize that the pressures to which they are exposed are motivated by forces that have little to do with them.

More tolerance of and compassion with the problems of the aggressor alleviate resentment and hopelessness.

Rebelliousness and other forms of acting-out may be replaced by more wholesome ways of standing up to and relating with the aggressor.

Eventually, new values and healthy modes emerge.

There is a tendency to overemphasize the existing family pathology without giving due recognition to healthy elements that are present and that may be mobilized even in the sickest family constellations.

Family therapy may activate latent resources not apparent during phases of family crisis when members collide with and withdraw from each other.

How effective this can be is illustrated by the experiment of family-oriented therapy in 50 cases of acute decompensation applying for admission to the Colorado Psychopathic Hospital (Pittman et al, 1966): On the assumption that the suicidal act or psychotic collapse was one manifestation of family disorganization, the therapeutic goal was as rapid restoration of the family equilibrium as was possible while developing new roles among the members.

An immediate evaluation of the family problem was made, and absent relatives were summoned to the hospital.

The patient's symptoms were considered as efforts at communication and were interpreted to the patient as such.

Guilt feelings and reluctance to cooperate on the part of relatives were dealt with firmly.

The involvement in the www.freepsychotherapybooks.org 320 patient's problem of each family

member was demonstrated, responsibilities were outlined, and tasks assigned. The patient and family were faced with the grim fact that the presenting symptoms of the patient were escape mechanisms and that a modification of family roles and rules was the best way to proceed. Role assignments were considered important, and tasks were allotted to each family member. This ended the functional paralysis in the household. A team visited the home in 24 hours to see that the tasks were being carried out. Medications were prescribed when necessary. Any social agencies already servicing the family were included in the planning. Responsibility for the patient's symptoms was imperatively placed in the family to reinforce the need for cooperative task performances. As new rules were made and role assignments set up, daily visits were sometimes necessary to help meet the needs of the different members and to see to it that the treatment plan was being followed. Pressure was exerted on the patient and family to change, their responsibility to change was preemptorily assumed. The success of this experiment was demonstrated by the fact that hospitalization was avoided completely in 42 of the 50 cases; 6 cases required temporary care for an average of 17 days, and only 2 cases needed long-term hospitalization. An average of 6 home or office visits per family was required, and some member in 75 percent of the families was referred to another agency for long-term outpatient services. Ten of the 50 families called again about a subsequent crisis, but this usually required only telephone handling. In patients who have been hospitalized, conjoint family group therapy has been done with four or five patients and their families meeting together as a group led by a therapist and co-therapist (Laqueur & LaBurt, 1966). The objective of such meetings is not to solve problems or give advice, but to improve communications within the family and to explore attitudes likely to form sources of difficulty in the family that may lead to a relapse of illness in the patient when returning home. The families have an opportunity to learn from each other by analogy, indirect interpretation, mimicking, identification, and objective description. (Techniques in family therapy are described in Chapter 52.) Conjoint Treatment of Married Couples (Marital Therapy) www.freepsychotherapybooks.org 321 The realization that no psychopathological condition exists in isolation, that it is an appendage of more extensive psycho-pathological processes within the family structure and social matrix, has resulted in a recasting of individual psychodynamics in the mold of family and social dynamics. A consequence of this orientation has been to see both husband and wife together as well as individually, or in groups with other married couples, even though symptoms occur in only one partner. Experience persuades that a feedback of pathological processes goes on from one to the other partner and that these must be dealt with definitively before either one can get well. In studying marital alliances, it becomes apparent that in many cases the choice of a marital partner rests on the hope that the prospective spouse will fulfill urgent unconscious needs that have been consciously rejected and repudiated. The most compelling of these needs are transferred by the mechanism of projective identification to a spouse who one hopes will introject and act them out. Reciprocal needs in the spouse follow the same dynamic and result in an interlocking projection-introjection collusion that makes psychotherapy difficult unless both partners are involved in the therapeutic plan.

Otherwise, improvement in one spouse can result in disruption in the homeostatic balance of the other, encouraging a sabotage of the partner's treatment.

A variety of treatment tactics have been used in this conjunctive effort, including counseling, casework, small group process, group therapy, and intensive psychoanalysis with a Freudian or neo-Freudian accent.

Among the most important intervention strategies, in addition to the psychoanalytic (Abies & Brandsma, 1977; Sager, 1976), are systems therapy (Haley, 1976; Minuchin, 1974a&b), client-centered approaches (Guerney, 1977), behavior therapy (Jacobson & Margolin, 1979), and combined therapies (Paolino & McCrady, 1978).

Formats include crisis counseling, collaborative therapy, concurrent therapy, conjoint marital therapy, conjoint family therapy, combined-collaborative therapy, and multiple group therapy.

The extensive early writings include Bird and Martin (1956), Boas (1962), Bruyn and deJong (1959), Dicks (1953), Dreikurs (1950), Eisenstein (1956), Ellis (1958a&b), Faucett (1962), Green S.

(1954), Greene & Solomon (1963), Grotjahn (1960), Gomberg (1956), Goodwin and Mudd (1961), Harrower (1956a), Hastings and Runkle (1963), Harper (1960), Herbert and Jarvis (1959), Jackson and www.freepsychotherapybooks.org 322 Grotjahn (1958b), Kadis (1963), Kohl (1962), Katz (1965), Laidlaw (1950), Leichter (1962), H.

E.

Mitchell (1963), Mittelman (1944a, 1948, 1956), Oberndorf (1938), Perelman (1960), Sager (1966a&b), Satir (1965), Saul et al.

(1953), Sarwer-Foner (1963), Tharp (1963), A.

Thomas (1956), A.

S.

Watson (1963), and Whitaker (1958).

Later contributions include those of Alger (1967a, b), Berman and Lief (1975), Bolte (1970), Fitzgerald (1969), Framo (1973), Greene (1972), Hurvitz (1970), Martin and Lief (1973), Minuchin (1974a), Sager et al.

(1968), Watzlawick et al.

(1967).

The extensive bibliography in the field indicates the wide prevalence of marital pathology.

Couples in trouble often consult marriage counselors, ministers, lawyers, and even friendly people of goodwill to advise them on proper steps to straighten out their entanglements.

Where the partners are not too seriously plagued by neurotic drives, where their defenses are flexible and reasonably intact, and where they essentially love and respect each other, they may at least temporarily be held together in marriage by such consultations.

Usually, however, the sources of serious marital problems are rooted in inner conflicts and stem from the operation of personality disturbances that resist pressure, education, convention, and the lessons of morality and proper decorum.

The prescription of tasks and exercises that are intended to influence couples to be less abrasive toward each other, to communicate more constructively, and to foster a balanced relationship will therefore not succeed in those couples whose behavior is intractably motivated by urgent unconscious needs and impelling inner conflicts.

For example, if a wife transferentially relates to a husband as if he represents a hateful brother with whom she was in competition during early childhood, she may resent being nice to him and continuously fail in her therapeutic assignments.

A husband who is struggling with a dependency need, idealizing his wife as a mother figure who must love, nurture and take care of him, may be unable to give up acting irresponsibly, resisting the independent role his wife insists he must assume as a condition for more fruitful

living together.

We should not minimize the utility of the various persuasive, behavioral, and cognitive techniques practiced by counselors to expedite marital congeniality.

They can be www.freepsychotherapybooks.org 323 valuable, but they will miss their mark if one utilizes them while ignoring the enormously important developmentally inspired motivational forces that are constantly maneuvering marital partners to act against their best interests.

These more insistently dictate the terms of conduct than any injunctions, maxims, precepts, recipes, prohibitions, and interpretations presented by the most skilled and dedicated marital counselors.

Most therapists dealing with marital difficulties agree that the complex triangular transferential problems inherent in working with a couple make treatment difficult.

As Ackerman (1964) has pointed out, the therapist's role is that of a participant-observer, who must be forthright and at times even blunt.

As the therapist moves alternately in and out of the pool of marital conflict, it is necessary to make free and undefensive use of oneself.

Sager (1964) has suggested five levels of involvement in conjoint sessions: 1.

Elucidation of the current source of irritation between the spouses.

2.

The working through of role, function, rule setting, and who is in charge.

3.

Consideration of how roles intermesh or clash (complementarity).

4.

Recognition and analysis of defensive behavior.

5.

Understanding and dealing with transferential reactions.

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Satir (1964a&b, 1965) encourages the couple to be authentic and spontaneous and to risk committing themselves to reporting all feelings and thoughts without inhibition.

Through what she calls "communication analysis," she attempts to help each partner change so that self-maintenance is substituted for "parasitic operations." An analysis of the respective ideas of the interaction of parents as marital partners is encouraged ("model integration analysis").

Often one finds in such analysis that marital partners attempt to accomplish in their relations with one another what was lacking in the families of their origin.

What they assume with each other is not a husband-wife association but a parent-child or sibling relationship ("role-function discrepancy").

A number of processes become apparent in the working through of role discrepancies.

First, the individual attempts to perpetuate his or her style of matching current experiences www.freepsychotherapybooks.org 324 with previous expectations ("manifesting self").

Second, there is recognition that the individual has separate needs and modes of operation apart from the spouse ("separating self from the other").

Third, accepting the uniqueness of the separate selves, there are attempts at decision making, considering the needs of the partner ("making room for the other").

Fourth, a search is conducted for compromise settlements of life needs, negotiating for what is suitable ("ways in which differentness is acknowledged").

To achieve these goals best, the therapist must report to the couple fully and clearly what he or she observes in the marital interaction.

The therapist acts as a "model of communication," asking questions, intervening, giving

answers, and demonstrating how to pose questions, and to negotiate for meaning. The therapist serves as a resource person, presenting a delineated structure that encourages change in a way that permits the marital pair to identify and use its own resources. The advantage of treatment of two individuals connected by blood, marriage, or friendship, together or separately, according to Grinker (1966), are many, such as revelation of new areas of information, a more precise clarification of issues, stimulation of intense affect, mobilization of repressed content, provocation of transference and countertransference reactions, increased reality testing, and reduced acting out, all of which provide for a corrective emotional experience and help shorten therapy.

On the other hand, there are disadvantages in that the released heightened emotions can be upsetting to both patient and therapist: The divulged material of one or the other partner puts a strain on confidentiality and trust; the atmosphere becomes provocative of intense transference and countertransference, leading to acting out on the parts of therapist and patients; the trap doors to repressed or suppressed feelings and impulses are lifted too rapidly to permit proper control and working through of the material; and the therapist is more easily led into a sadomasochistic triangle.

The actual techniques of marital therapy are detailed in Chapter 52.

Criticism of Educational Group Therapy There is little question that an educational group, chiefly through group dynamics, www.freepsychotherapybooks.org 325 can influence values and behavior significantly.

It may provide the individual with opportunities for constructive learning and serve as a means of working through some distortions in the basic personality structure.

The benefits of reeducation, however, are too often registered as an overlay, superimposed on unchanged underlying impulses and attitudes that will, in situations of stress, tend to displace the newly acquired characteristics.

Unless intrapsychic restructuring has occurred, results may be impermanent.

This is not to depreciate group dynamics, nor to say that true cognitive alterations are not effectuated through reeducational groups.

They do occur, but they cannot always be relied upon.

Wolf and Schwartz (1962) have pointed out the difficulty of relying on group dynamics as a therapeutic force: "The group dynamic emphasis brings with it a failure to differentiate one patient from the next and occupies itself with phases of group development... .

The use of group dynamics in interpretation and as a means for affecting change in the individuals that constitute the therapy group is a misperception and a rejection of both intrapsychic and interpersonal processes." The authors consider that in a group directed toward personality change, one should consider group dynamics as they emerge as resistance phenomena that must be resolved.

"A group therapist who occupies himself with group dynamics is catering to the patient's resistance.

To contemplate the seeming homogeneity of response is to give undue importance to the superficial and obvious reaction and to disregard the covert and unconscious individual responses." There are some group therapists, such as Bion (1961), who attempt to circumvent the limitations of reeducational therapy by utilizing group dynamic phenomena, when they appear in a group, as material for analysis of the group as a whole on the basis that resolution of the "group neurosis" will effectuate reconstructive changes in the individual members.

This approach is described by Bach (1954) in these words: "The group is a state or projection screen onto which the total repertoire of conscious, pre-conscious, and unconscious needs are internalized, projected, acted out, recognized, and corrected through a group-center therapeutic work process." H.

Durkin (1957) has stressed that group dynamic and psychotherapeutic processes are on the whole mutually reinforcing.

www.freepsychotherapybooks.org 326 Both group dynamics and interpersonal reactions are present in all groups.

They may be destructive as well as constructive for the individual.

If therapists apply themselves to support the beneficial elements, with analysis and resolution of destructive aspects as they come up, a reeducative group may serve the individual remarkably well in fostering a more wholesome adjustment and in liberating tendencies toward greater self-realization.

PHILOSOPHICAL AND RELIGIOUS APPROACHES Essential in human adjustment is a salutary philosophy that gives direction to one's needs and substances to life's goals.

Philosophical and religious systems abound that lend purpose and meaning to existence.

Embedded in these ideologies are ways of adjusting to inequities, of executing obligations to others, of finding peace and contentment.

More remotely, they seek to alter the individual's sense of values.

A fundamental goal of psychotherapy is to effectuate a change in value systems through an altered perception of one's inner self.

Philosophical and religious approaches aim for and sometimes accomplish the same purpose through a different means, namely, by direct attempts to influence modes of looking at things through entreaty, logic, argument, or authoritative mandate.

Such approaches are often embraced in an effort to bring about peace of mind, to bolster personality defenses, and to open up outlets for promptings that could otherwise neither be countenanced nor expressed.

Since psychological needs are diverse, no single religious or philosophical credo appeals to all persons.

Variant drives and defenses make for endless protocols that are organized to fit specific designs.

Thus, ungratified mothering will open the door to the acceptance of philosophical or religious systems that promise protection, love, and pleasure from some "God-mother" or "earthmother," if not immediately, then in the hereafter.

The price of submission, condign obedience, and indulgence in ritual and prayer, is small indeed for the size of the bounty promised.

Illustrative of how philosophies may substantiate coping mechanisms are the mystical cults that reinforce detachment and introspection and the stoical systems that put a premium on masochism and self-punishment.

Credendas and codes can neutralize a severe conscience that puts an embargo on impulse expression.

For example, acceptance of hedonistic philosophies will in guilt-ridden persons help promote pleasure as a prime purpose.

It is interesting that, in organizing values, modern humankind still exploits philosophical systems that parallel those developed hundreds and even thousands of years ago.

Thus, hedonism may be commandeered as a credo to give sanction to impulses for unbridled pleasure seeking.

In conditions such as psychopathic personality, alcoholism, drug addiction, and sexual perversion, a doctrine that endorses pleasure as a mainstay of life may lend patronage to an irresponsibility that is prompted by uncontrollable inner needs.

Epicurianism may be espoused as a means of balancing an incubus of guilt in persons with consciences that refuse to lend sanction to life's delectations without restraint.

Worries are purged from the mind as wanton waste, with encouragement of (1) regulation of one's

life to anticipate unpleasant events so that they may be avoided, (2) arrangement of matters so that as much pleasure as possible may be crowded into each day, (3) elimination of those pleasures for which one has to pay too dearly, (4) the judicious cutting off of thoughts that mobilize pain or create tension, and (5) the banishment of profitless recriminations about the past or anticipated fears of the future.

In the canons of stoicism the detached soul often finds refuge.

Skepticism, the philosophy of doubting, serves some obsessive-compulsive personalities in their quest to achieve tranquility.

By scrupulous doubting and withdrawing into the self, the skeptic attempts to escape unhappiness.

This suspension of judgment ("isosthenia") results in a state of mental balance ("epoche").

We might speculate that the basis for the skeptical maneuver is a defense against a severe, dogmatic, and authoritarian conscience.

The skeptic is fired by a desire to avoid the controlling hand of fate.

By doubting, the person negates it thus gaining liberation from the shackles of cosmic doom.

In this way the person neutralizes anxiety.

At the same time the refusal to arrive at definite judgments perhaps falls in with the coexistence of ambivalent value systems often a product of inconsistent disciplines in childhood.

Diametrically opposed to the values of skepticism are those of dogmatism, which, as a philosophy, may be exploited for inner harmony and peace of mind.

This is the sanctuary of a severe and authoritarian conscience that seeks constant control because of a devastating fear of uncertainty and the unknown.

The dogmatist www.freepsychotherapybooks.org 328 seeks to mold fate to his or her own controlling prospectus.

It may be seen, then, that the values of certainty and truth—and the need for a precise structuring of reality in the dogmatist and the value of doubting in the skeptic that negates certainty and structure—are both manifestations of obsessive-compulsive personality structures and constitute different modes of coping with precisely the same kind of anxiety.

Individuals reared in a religious atmosphere, or achieving a decision through a need for personal salvation, may at times become dedicated to the adoration of God, either as a symbol of power and protectiveness or in the form of ethical self-devotion—for instance, as a Christian in the morality embodied in Jesus Christ.

They may then conceive that what appeals to their souls as deserving of worship is what they feel their souls may trust, which has the ability to deliver them from evil, sin, fear, and death.

In this way they will, according to their interpretation, attempt to conform to the purpose of the life of their Savior, with expressions of devotion to God and love for humanity in accordance with the theological creeds and sacraments and in the church, synagogue, temple, or mosque within which they find their identity.

Those aspects of religion that possess greatest meaning will be cherished whether they involve ritual, sacrament, logic, belief, faith, or the privileges of church membership.

The values for the individuals will be contingent on their needs and range from the comforts of group belongingness to the enhancement of the spiritual aspects of the self and the enrichment of moral goals.

This is not to say that neurotic objectives may not be sought through the exploitation of religion's instrumentalities; for religion, with its Divinities, ceremonials, dogma, and taboos offers the neurotic individual rich resources for the projection of strivings, demands, and defenses that keep him or her in psychological homeostasis.

Whatever the motives, belief in God and reliance on prayer, according to a recent Gallup poll, are on the upswing, and more people in the United States today than in the past three decades believe that religion is having an increasing influence on U.S. life.

Sixty-one percent of those interviewed feel that religion can answer all or most of today's problems.

Even in countries that have tried to abolish religion as "the opiate of the people" efforts have not been too successful.

On the other hand, the values of science may become for some individuals, at least www.freepsychotherapybooks.org 329 temporarily, of supreme importance in supplying answers to the ultimate meaning of reality.

They will then extrapolate from the data of their observations the functional relationships between pertinent variables in the hope of bringing some order to the phenomenalistic chaos that invests their lives.

Through the doorway of empiricism, rather than faith, they will approach purpose and meaning. And yet they will, realizing the limits of reason, attempt from time to time to liberate themselves from the tyranny of their senses by introspection and subjectivism.

Transcending the empirical sphere into the hazardous world of speculation, some people in their quest for meaning, take a journey into arcane zones.

Accepting the dictum that "things may not be quite other in themselves than that which by the laws of our thought they necessarily appear," people weld together concepts by the powerful force of faith.

In search of experience beyond knowledge, they penetrate into the penumbra of mysticism. Justifying their illusions of reality, they argue that humans are limited by the testimony of their senses.

Perspectives of reality are always relative to the percipient.

No universal standard exists or can exist except that which each person discovers by retreating into the self.

Liberated from the fetters of reason, the individual can blend with the Absolute and then perceive the ultimate nature of reality.

His or her mind, the individual may believe, is directly linked to the Cosmic Mind, which All-seeing and Allknowing can fashion his or her destiny and direct his or her journey through infinity from gloomy evil toward the luminous fields of spirit, love, and truth.

It is to be expected that the mystical path to tranquility is the refuse of some who are unable to brook the harsh realities of their existence, whose coping mechanisms have failed them, or who have lost their faith in the dignity of humanity and the virtue and safety of the material world.

Organization of one's life around philosophical religious, or, for that matter, "scientific" values is an inescapable aspect of mental functioning.

These centralizing points are woven into the complex tapestry of adjustment.

They may be adapted, changed, or discarded as the shifting needs of the individual dictate modification.

Mysticism www.freepsychotherapybooks.org 330 Mysticism possesses properties that have led a number of psychotherapists to explore its therapeutic potentials, some even attempting to blend mystical formulations into their treatment systems, as, for instance, Zen Buddhism (Suzuki, 1947; Watts, 1957; Ben-Avi, 1959).

The mystical striving is twofold; first, there is an attempt to achieve communion with the Absolute (the Highest, the One, God, Brahma, the Order of Heaven, Being of beings) and, second, a desire to grasp through introspection the ultimate nature of reality.

The Absolute is conceived of in various symbolic forms as an encompassing, irresistible, indwelling power that can overcome the temporal, the changing, the relative, the impermanent, and other aspects of existential anxiety.

The Absolute establishes stability, methodical arrangement, and permanence in the universe. Since it is impossible to approach it through the senses, the establishment of the Absolute is attempted by epistemological arguments, by mandates in sacred writings presumably divinely inspired and enforced through religious discipline, and by mystical experience.

Approach to the Absolute through mystical experience has an old and elaborate history. In such experience there is no need for intermediaries, such as oracles, priests, historical revelations, or prayers; rather direct and personal contact is made, resulting in an identification and fusion with the Absolute substance.

Description of the austerity of mortifications suffered, of the rapture of the visions, and of the intoxication of the senses as the soul finds its resting place with the Absolute have produced some of the world's most florid and poetic literature.

The mystical experience may become a part of multiform religious or philosophical systems. Historically, it was organized into bodies of practice among dissatisfied adherents of the inflexible, formalistic, and legalistic religions who sought to detach themselves from the ossification of formulas and ceremonies toward a liberating union with a divine spirit.

A search for the Absolute has preoccupied philosophers and theologians throughout the ages. Its most prominent forms have been found in Eastern systems, for example, the Brahmanic and Buddhist religions, which promote the illusory nature of reality and the goal of absorption in mystical essences toward ecstatic enlightenment ("nirvana," "satori").

Reeducation of the self to plastic passivity as a precursor to the transcendental experience is encouraged by sets of rules outlining contemplative and ascetic rituals that are said to result in an expansion of consciousness and in the promotion of self-control.

In all mystical systems direct personal experience is approached through states of absorption in which the individual becomes aware of oneness with the Absolute.

By various stages the mystic achieves this union.

Through meditation the student invokes reason, memory, and will, concentrating on some scene or subject.

In Western religious systems, the student may employ a preparatory prayer for Grace, focusing on several points in the image he or she creates, then pouring devotion out freely into the Colloquy.

In Eastern systems an attempt may be to suspend reason, employing certain aids, such as breathing and body control, as in Yoga; or practice of the arts, like archery or flower arrangement in Zen Buddhism; or indulge asceticism to bring the body under regulation to extinguish desire and reduce the self to submission.

Gradually, reason gives way to inner contemplation, and even to hallucination.

Excitement, rapture, despair, and varied other emotions may develop within the matrix of a delicious solitude.

A symbolic dialect unites mystics of all persuasions.

In different words there is described the same adventure: the "paradox" of existence, the "journey" into the unknown within, the periods of "darkness" and "light," the excitement and enlightenment at achieving the strange and wonderful world of inner reality, and the "marriage" with the Absolute or Divine.

What patients derive from mysticism is illustrated in a letter to me by a professional man who had immersed himself in it.

For me there is nothing "mysterious" about mysticism.

Rather, it gives a clear view of how to best live my life for myself and others.

“Mysticism” is a commitment to life and reality, nothing less than a seeking of total conscious awareness and identity with life itself.

Not a merging of the ego beyond the cognitive, sensory, and emotional structures which, though they are so necessary for our day-to-day productivity and happiness, nevertheless, can be seen as limits when we set our sights on our relationship to God and the cosmos.

(The questions, “Who are we?

Why are we here?

Where are we going?”) The ego and awareness naturally expands as we

www.freepsychotherapybooks.org 332 progress from infancy to adulthood and finally obtain the perspective of old age.

For some, this process occurs experientially, for others—they crawl their way through life still dragging their childhood fears and fantasies behind, creating for themselves in their lives an ensnaring chaos, from which the consciousness longs only for escape, not greater understanding.

For me, once or twice a day, meditation, or rather, contemplation, and active fixing of attention, has gone hand-in-hand with my psychotherapy in accelerating my awareness.

It gives me the conscious experience of my larger self, or the whole knowing and loving the whole at every point—my awareness being just a point in this sense.

It is the realization that I am a lot more than I thought I was.

Eastern Systems Hindu Philosophical Approaches: Yoga Historically, Hinduism preceded Buddhism. It was a polytheistic system, with sacred writings (Vedas and Brahamanas) that developed into an elaborate sacrificial and ritualistic movement.

Essentially, it conceived of an absolute spirit, the Brahma, as the source and goal of all things.

Creative, conservative, and destructive principles were reflected in the divine personalities of Brahma, Vishnu, and Siva, who respectively represented creation, preservation, and destruction.

While many forms of Hindu religion have evolved from its early origins, there is generally accepted the doctrine of Karma, which makes humans accountable in the present or in their reincarnations (transmigration of souls is a principal belief) for their good and bad actions.

Karma of the spirit can drive humans to evil, toward a concern with matter and selfishness, or toward selfless good and love.

Modern Hindu systems of philosophy are employed to help the distressed.

These, according to Vahia (1962), conceive of the basis sources of conflict or disturbed peace of mind as threefold: 1.

Too intense emotional attachment to external things, such as possessions, status and power, which inevitably produces destructive competitiveness.

2.

A delusion that humans with their finite mind can understand cosmic reality, which is actually beyond human comprehension.

3.

Inharmonious interactions with other people, such as (a) excessive attachments, www.freepsychotherapybooks.org 333 (b) too powerful emotional yearnings, including love, (c) inordinate jealousy, and (d) unreasonable anger.

While these conflicts are universal, the special way in which they operate is unique for each individual.

Measures, valuable for the achievement of peace of mind, will vary with the personality.

Three main categories of people exist, and the approach will differ in each.

First, there are those who operate on the basis of intellectual understanding; the second are swayed primarily by sentiment and emotion; the third are insistent on a practical solution without analyzing the problem or bothering about feelings.

Those persons who wish to learn the essential techniques may seek the guidance of an expert. No attempt is made at the start to understand the problem.

Rather, the person is taught the principles of relaxation.

Various postures are tried until the best one is discovered.

Voluntary muscles are relaxed; then an attempt is made to bring the involuntary nervous system under control.

Once respiration is regulated, heart action and management of other aspects of the cardiovascular system are attempted.

By special exercises, the stomach and intestines are also relaxed; the autonomic nervous system is brought under voluntary management.

This will require a long period of work under supervision.

Complete muscle laxity enables the individual to proceed to the practice of meditation in order to achieve a void where the intellect is suspended.

What is desired is a new objective kind of understanding.

Concentration is shifted from the sources of problems to an object of faith and love, focusing on the latter as a way of removing the investment in the conflict.

For example, one may focus on the idea of God or a prophet.

Hinduism is an ideal religion on which to meditate.

The concept of God becomes a concept of “everlasting unchanging cosmic reality... . This, the only Reality, is free from life and death, or pleasure or pain ... from love and hatred, pride and prejudice.” By identifying oneself with this reality, one may eschew emotional involvement and best be able to study the forces that have led to one’s difficulties.

A state of mind is achieved “where peace www.freepsychotherapybooks.org 334 prevails and turbulence, if any, is only transitory.” In this way one can eventually “remain unaffected by any event of stress and strain.” Eastern philosophical systems were not designed as forms of psychotherapy.

They were oriented around promoting spiritual growth and achieving a oneness with the Universal.

The resulting serenity and tranquility, however, may palliate a mind in turmoil and shift values toward harmony with the world, thus indirectly registering a therapeutic impact.

Appraising what may be accomplished in this approach, we observe a combination of supportive and reeducational modalities: 1.

Relaxation helps to resolve tension and to relieve anxiety.

2.

Meditation redirects thoughts from anxiety-laden sources toward those that sponsor a more constructive life orientation.

3.

Guidance from the helping agency helps to buttress defenses and to insulate oneself from sources of anxiety by (a) encouraging detachment from emotional situations, with avoidance of competitive strivings and a removing of oneself from involvement with other persons, (b) making oneself dependent on a power stronger than oneself through mystical cosmic unity, and (c) promoting independence by means of introspection and self-realization.

Yoga, a form of Hindu mysticism, had its origins in an idealistic monism, the Vedanta philosophy, which conceived of the world as a conscious spiritual principle permeating all things, the Atman.

“The wise who perceive him as being within their own Self, to them belongs eternal peace, not

to others" (Katha-Upanishad, ii, 5, 12).

A modification of this philosophy in the Sankhya system assumed "the eternal coexistence of a material first cause and a plurality of spiritual elements or Selves, Puru-sha." A schism in the Sankhya movement developed that added the concept of God.

This, the Yoga system, advocated a mortification of the senses, brought about by such measures as prolonged fixation of the eyes on the nose, protracted assumption of rigid postures, to liberate energies, and a new awareness.

The purpose was to bring about an ecstatic vision of the Deity and perhaps the acquisition of miraculous powers.

Hoped for was a permanent union with the Supreme Being.

To achieve the concentration essential for this, eight stages are deemed necessary: 1.

Self-control (yama) obtained by such devices as chastity, nonstealing, nonviolence, truthfulness, and avoidance of greed.

2.

Religious observance (niyama) through chanting of the Vedic hymns, austerity, purity, and contentment.

3.

Assumption of certain postures (asana).

4.

Regulation of the breath (pranayama) with controlled rhythmic exhalation, inhalation, and temporary suspension of breathing.

5.

Restraint of the senses (pratyahara) by withdrawing of the senses from their objects.

6.

Steadying of the mind (dharana) through fixation on some part of the body, such as the nose or navel.

7.

Meditation (dhyana) on the true object of knowledge, the Supreme Spirit, to the exclusion of other thoughts.

8.

Profound contemplation (samadhi) with such complete absorption and detachment that there is insensitivity to heat and cold, pain and pleasure.

The "Yogin" who arrives at perfecting the last three stages (samadhi) achieves an ecstatic mystical state that stimulates ability to have an awareness of the past and future, of what has happened in previous births, and knowledge of "everything that exists in the world and in his own body." Proper application of the samadhi promises "complete control over everything in the universe," "traversing anywhere at will," and other miraculous powers, including, finally, a complete release of the intellect from the self, purity, and eternal liberation in the form of separation of matter from spirit (Raja-yoga or Kaivalya).

www.freepsychotherapybooks.org 336 Modern practices of Yoga essentially avoid asceticism in quest of the mystical experience of "awakening." The ultimate goal is fusion with the Absolute, during which the person experiences the highest wisdom and most profound truth of being, thus gaining possession of his or her real self through expanded comprehension and self-realization (Bagchi, 1936; Behanan, 1937; Yesudian & Haich, 1956; Yeats-Brown, 1958; Wood, 1959; Malhotra, 1963; Majumdar, 1964).

Buddhism and Zen Buddhism A young prince, Gautama Siddhartha, at the age of 29, during the sixth century B.C., struck with the suffering of humankind, deserted his family to search for the true meaning of life.

For years he wandered, attempting in vain to reach the "truth" by fasting, selftorture, and

meditation, which were prescribed by the traditional Brahmanistic teachings.

One day under a fig tree, known later as the Bo Tree or tree of enlightenment, he suddenly achieved a glimpse of the truth.

This was to the effect that the cure for unhappiness was a renunciation of selfish desires and a mastery of the self.

He then taught others his discovery of how to find true happiness, and he became known as Buddha, the Enlightened One.

The great popularity of Buddhism was due to its making salvation available to all through personal efforts without the self-torture that existed in the older Brahmanism.

Among the teachings of Buddha were three basic concepts: 1.

The soul of a person passes after death to higher or lower animals depending on whether the person performed good or bad deeds in his or her lifetime.

The purpose of this transmigration is to overcome all desire.

Then the soul no longer has individuality and may enter "Nirvana," a peaceful state of oblivion.

2.

Release from conscious existence is hastened by pursuing the Eightfold Path of Right Faith, Right Intention, Right Speech, Right Conduct, Right Livelihood, Right Effort, Right Thinking, and Right Meditation.

3.

There is no supreme God.

www.freepsychotherapybooks.org 337 Over the centuries the teachings of Buddha underwent revision.

Buddha, himself, became considered a god who had miraculously come into the world.

Sacrifices of flowers and images were made in his honor.

Elaborate rites and ceremonies were evolved.

Legends were developed about Buddha and his might.

Prayers to Buddha began to replace personal effort.

Self-mastery gradually became forgotten as the most important goal.

The principles of Buddhism include the following four main ideas: (1) life is imbued with suffering; (2) suffering is the product of desires seeking fulfillment; (3) control of all desire and self-mastery can end suffering, hence celibacy is sometimes encouraged; and (4) pursuance of certain rules of living and of proper attitudes and knowledge leads to the overcoming of desire.

The final objective in life is Nirvana, a blissful contemplation and peaceful state of oblivion, characterized by an extinction of the material personality and achievement of enlightenment.

Among the Buddhist sects, that of Zen Buddhism, a kind of introspective mysticism difficult to reduce to words, has most attracted Western psychotherapists.

It is difficult for the Western mind, oriented as it is around logic, intellectualism, and determinism, to conceive of Zen.

Satori ("enlightenment and awakening") experience is predicated on a suspension of the reasoning facilities, which are held to be a hindrance to the release of the intuitive essence of being.

Zen is dedicated to the idea that one may solve all problems through intuition and enlightenment rather than through logical abstractions such as constitute the methods of the West.

Zen cannot be taught easily since each person finds his or her own path through search and struggle.

A number of excellent books have been written about Zen (Blofeld, 1959; Conze, E., 1951, 1954, 1958; McGovern, 1922, 1923; Rhys-Davids, 1899-1938; Sasaki, 1960; Suzuki, DT, 1949, 1952, 1953, 1957a, b, 1959; Takakusu, 1947; Thomas EJ, 1931, 1933, 1935; Watts, 1957; Yama-kami, 1912). Practically all advocate the achievement of the extraordinary Satori experience (a oneness with Buddha) "which is devoid of emotion and intellectual content" through study with a Zen Master. One method commonly employed by the master is meditation on a Koan, a riddle for which there is no set solution.

The Zen www.freepsychotherapybooks.org 338 Master operates by posing paradoxes, such as, "What is the sound of one hand clapping?" At the same time the Zen Master puts the student in an untenable situation.

For instance, the master may threaten the student with a stick, saying, "If you call this a stick, I will beat you.

If you say this is not really a stick I will beat you.

And if you say nothing I will beat you." This establishes for the student a paradox that cannot be unraveled through customary modes of problem solving.

Somehow the student must work out a reply that will satisfy the Zen Master.

In the process of doing this, the student will have developed a unique mode of conceptualizing reality that dissociates him or her from habitual points of reference.

Another means to Satori may be through random activities such as flower arrangement or by practicing archery, as described by Herrigel (1953).

What one strives for is an experience of being that cannot be described except in such terms as "freemovement of the spirit" and "an original and nameless essence." This is achieved by "methodical immersion in oneself" leading "to one's becoming aware, in the deepest ground of the soul, of the unmanageable Groundlessness and Qualitylessness—nay more, to one's becoming one with it." The experience cannot be reduced to words ("one knows it by not knowing it"), but it is said to liberate and change the human being.

Only by true detachment can one become completely empty of the self and then "become one" with the "Transcendent Deity." In breathing exercises, in practicing archery, "I learned to lose myself so effortlessly in the breathing that I sometimes had the feeling that I myself was not breathing, but strange as this may sound—being breathed." What Herrigel is particularizing are psychological phenomena characteristic of all forms of mystical experience, which is, as has been indicated, the essential substance of Zen Buddhism.

Cultist Movements A growing number of movements have developed that appeal to alienated young people seeking to dissociate themselves from the values and life styles of their parents and groups with which they are customarily identified.

These cults appeal to school dropouts, drug abusers, borderline cases, youngsters with severe personality disorders, and www.freepsychotherapybooks.org 339 individuals in emotional turmoil. Many of these youngsters are simply unhappy souls who have been unable to complete their developmental growth to reasonable separation/individuation.

On the surface they appear and act normal (Ross, 1983).

Rejecting the usual religious and political tenets that are accepted in their families, they seek self-fulfillment through mysticism and a faith in an esoteric leadership onto which they can project their infantile needs and hopes.

Heads of such movements are usually charismatic and grandiose gurus, ministers, or exploitative laymen whose pronouncements are sufficiently ambiguous so that the devotees can interpret them however they wish.

Participants come to these movements of their own free will, recruited by acquaintances or friends who have become ardent supporters and salesmen.

A brief exposure to such dedicated groups convinces the new recruits that they have finally

found a resource that alleviates their anxieties and satisfies their search for purpose and meaning.

Perhaps for the first time, these individuals feel accepted by their peers and find their dependency needs gratified by submitting to the rules of the leader (with whose grandiosity they may inwardly identify) and the mores of the group members, who become for them a new and more accepting family.

Needless to say, these developments are often viewed with horror by a recruit's parents, who may, in their turmoil, seek ways of extricating their child from what they consider a most destructive and bizarre way of life.

Should the parents succeed in removing their deviant from the group by kidnapping or legal means, an attempt will often be made to deprogram the presumably brainwashed victim.

Such efforts usually prove unsuccessful, unless the deviant is sufficiently motivated to seek help from a professional who has had experience in dealing with his or her problems.

Coordinate family therapy is usually essential.

Among the most common cults are the Hare Krishna movement, Arica, and the Unification Church of Reverend Sun Myung Moon.

In 1965, A.C.

Bhaktivedanta Phapupadha came to the United States to bring to the needy the benefits of his transcendental teachings, including Bhakti yoga, which endorses dietary and sexual abstemiousness and various taboos against intoxication, gambling, and aggression.

The Hare Krishna movement sponsors community living devoted to the worship of Krishna, one of the incarnations of the god Vishnu, "to help man in distress." The followers of this www.freepsychotherapybooks.org 340 sect in the Western world assume the dress and life style of the ancient Vedic society and devote themselves to a simple religious existence, chanting the glories of Krishna as a way of perfecting their lives.

Many other movements, such as the Self-Realization Fellowship of Paramhansa Yogananda, have attracted enthusiastic followers who claim that the Vedic teachings have brought purpose to their lives and harmonized their physical, mental, and spiritual natures.

The Arica Institute was founded in 1971 by Oscar Ichazo a Bolivian.

Presented as a course of training, it teaches meditation to obtain a state of liberated consciousness; deep body massage to release mental stress; "protoanalysis" to understand the destructive ways the body seeks to express stress; "psychoalchemy" to transfer physical into psychic energy; "Kinerhythm," in which slow movements help evoke ecstasy and "yantras"; "Pneumorhythm," which combines breathing with rhythmic counting to expand consciousness; "Opening the Rainbow Eye," which provides a state of "satori" in which all internal and external experiences are seen clearly; the "Alpha Heat" ritual to free the mind of worry, regret, rivalry, hatred, envy, pretense, fear, jealousy, and prejudice; "Trialectics," which teaches the logic of unity to permit the analysis of reality; the "Cutting the Diamond" ritual to complete the mystical union; "Hypergnostic Analysis" of internal meanings and connections of things; the "Golden Eye" ritual to observe the voyage of consciousness inside the body and to transfer consciousness; and the "Psychic Shapes" ritual, which sponsors the internal recognition of the Unity of God.

These practices and rituals have attracted hundreds of thousands of followers who often gather in communes where Arica is taught.

The Unification Church, founded by Reverend Sun Myung Moon and which has received a great deal of publicity in the press, is intended to rectify the "moral confusion" and "the absence of a clear sense of values" that threaten America's spiritual heritage.

Presented as signs of such corrosion are child abuse, prostitution, abortions in girls below the age of 18, fatherless children, juvenile crime, drug deaths, suicide, murder, robbery,

assaults, burglary, and divorce.

More generally, followers believe there is a universal lack of well-being and a sense of unfulfillment and unhappiness.

All these woes stem from www.freepsychotherapybooks.org 341 "confusion in the hearts of individuals." What is necessary is a revolution within men and women that only God can bring about through a "new revelation." Reverend Moon claims that this truth, which current religion, philosophy, and ethics have been unable to supply, was personally revealed to him by God, who appeared and selected him for the mission of revelation.

Thereafter, Reverend Moon journeyed into the spirit world many times, and God instructed him to teach the revelation, the Divine Principle, in every corner on earth.

It is now taught in all languages in 127 nations.

Through this revelation, says Reverend Moon, alcoholism and mental illness can be cured, physical healing occurs, and racism and nationalistic prejudice disappear.

Fresh hope, love for one's fellow brothers and sisters, and happy marriages are said to become a way of life.

These bounties are available to those who join the Unification Church.

And thousands upon thousands of people do in order to live the prescribed life and to receive the important revelation (Galanter et al, 1979).

To detail the specific beliefs and rituals of all the different cultist movements would encompass a textbook in itself and would merely repeat what has been written elsewhere (Patrick, 1976; Edward, 1979; Levine & Salter, 1976; Ungerleider & Wellisch, 1979; Levin & Zegans, 1974; Marmor, 1984).

Suffice it to say that the psychopathology in some of the leadership and followers is relatively high.

Nonetheless, the movements seem to serve some purpose in stabilizing disturbed individuals who otherwise would become drug addicts, delinquents, psychotics, and victims of suicide.

It is an indictment of society that more rational group resources have not been made available.

It may be that the successes of experiential groups (see Chapter 52) serve the same purpose as religious cults by supplying some populations with avenues for gratification of their personality needs, albeit in more socially accepted and less peculiar ways.

Western Religious Approaches In the Book of St.

Matthew (17:14-18) there is recorded the following miracle: ...there came to Him a certain man, kneeling down to Him and saying, Lord have mercy on my son: for he is a lunatick, and sore vexed: for oftentimes www.freepsychotherapybooks.org 342 he falleth in the fire, and oft into the water.

And I brought him to thy disciples, and they could not cure him.

Then Jesus answered and said O faithless and perverse generation, how long shall I be with you? how long shall I suffer you?

bring him hither to me.

And Jesus rebuked the devil; and he departed out of him: and the child was cured from that very hour.

Religious healing, recorded in the successes of Jesus, is legendary.

And great multitudes came unto him.

having with them those that were lame, blind, dumb, maimed, and many others, and cast them down at Jesus' feet; and he healed them.

Insomuch that the multitude wondered, when they saw the dumb to speak, the maimed to be whole, the lame to walk, and the blind to see: and they gloried the God of Israel.

[St.

Matthew 15:30-31] The inspiring effects of divine influence are testimony to the fact that a

human being has the potentiality for extraordinary response to the proper authoritative mandates.

Faith cures are irrefutable.

Under favorable circumstances the ravages of physical and psychological illness may be palliated.

Some may even be permanently resolved.

The relief of inner turmoil through faith is dramatically described by St.

Augustine: How then do I seek Thee, O Lord?

For when I seek Thee, my God, I seek a happy life.

I will seek Thee, that my soul may live.

For my body liveth by my soul; and my soul by Thee....

When I shall with my whole self cleave to Thee, I shall nowhere have sorrow or labour; and my life shall wholly live, as wholly full of Thee.

Thou fillest, Thou liftest up, because I am not full of Thee I am a burthen to myself....

Woe is me!

Lord, have pity on me.

Woe is me!

Io!

I hide not my wounds; Thou art the Physician, I the sick; Thou merciful, I miserable....

And all my hope is nowhere but in Thy exceeding great mercy.

A study of comparative religions elucidates the vital role that religion has historically played in the emotional adjustment of people.

Its survival as one of the most potent of institutions for the supplying of some vital human needs is manifest; in the face of the most devastating attacks by science on the authenticity of its sacred documents, it continues to function unabated.

This, Montaigne claimed, was because religion is "man's only succour from his native state of helplessness and uncertainty." Carl Jung (1933), www.freepsychotherapybooks.org 343 denouncing Freud's antireligious attitude, wrote that "man has never yet been able singlehanded to hold his own against the powers of darkness—that is, of the unconscious.

Man has always stood in need of spiritual help which each individual's own religion held out to him." The survey of the Joint Commission on Mental Illness and Health (1960) indicated that, on the whole, persons who regularly attend church have less discomfort than those who do not. Whether religion can be serviceable for a specific patient will be contingent on the person's special needs and his or her propensity to employ religious principles for healthy or neurotic gains.

There is no doubt that dramatic relief from neurotic suffering is possible after religious conversion.

Through conversion the individual may harmonize with the order of the universe and feel a oneness with the world or a union with God.

Riddled by anxiety, tortured by self-doubt, the person in anguish is susceptible to the help held out through salvation by the evangelist.

A reeducational experience develops as the person moves in the conversion from defeatism to hope.

By confessing wickedness, the convert attains both forgiveness and the means to a blessed existence.

Feelings of insignificance are replaced with a sense of distinction as one of God's instruments.

Competitive strivings are abandoned; hate changes to love, tolerance, and compassion.

Remarkable shifts in attitudes and behavior may follow.

Such experiences have been reported in detail by William James (1941) in his *Varieties of Religious Experience*.

The consequence of these transformations is another matter, since psychic stability is sometimes restored at the expense of general psychological integrity.

Many patients come to psychotherapy after extensive efforts of religious devotion have failed them.

Other patients who have received psychotherapy find themselves more fulfilled by affiliating themselves with a religious movement.

Religion in some form is employed by many people to bring happiness and solace to an otherwise turbulent existence.

It is conscripted as a vehicle for values that fill the person's life with purpose and meaning.

It is also utilized to substantiate neurotic www.freepsychotherapybooks.org 344 defenses, justifying these by a blind adherence to codes and authoritarian mandates (Conigliaro, 1965).

If religious credos serve the individual's personality needs, they may facilitate adaptation and contribute to the social good.

If they conflict with the individual's needs, they will interfere with adjustment and militate against social good.

A number of notable attempts have been made to reconcile psychiatry, psychotherapy, and psychoanalysis with religion (Ryle, 1945; Santayana, 1948; Tillich, 1952; Moore, 1953; Toulmin, 1953; Wisdom, 1955; Niebuhr, 1955; McLean, 1959; Zilboorg, 1956, 1962; Lee, 1957; Bartemeier, 1965).

Implied in most of these contributions is the idea that even a psychotherapist in search of values may find these in religious experience.

Psychotherapy, it is alleged, releases the individual to choose constructive values, while religion is a suitable supply depot for such values.

Conflict between psychiatry and religion, however, still continues, sustained in part by an inability to define religion.

If by religion we mean the striving for worthy ideals and values, most scientists would concede its merits.

On the other hand, if we mean the acceptance on faith of fixed creeds and beliefs regarding the origin of the world and the ultimate purpose of life and the abiding by ethical standards as absolute in relation to an eternal law, most scientists would register their opposition.

Amidst the conflicts of science and religion many individuals search within themselves for meanings of faith and attempt to define God in their own terms shorn of the dogma and ritual of the church.

In the systems that they evolve, ancient ideas of logic and cosmology are discarded and scientific concepts of the material world are accepted in a realm of their own.

To some, God continues to be accepted as the essential element, the embodiment of the highest ideal of self-sacrificing love, which can lead humanity to peace and fulfillment.

For others, a revision of religious belief prevails that has gone so far as to challenge the essential doctrines of the church of their origin and to make out of the Savior a noble mortal rather than an incarnation of God.

God is accepted as a principle of order and goodness, rather than as an extramundane Creator who rules the universe.

The highest religious achievement is identification with the principle of goodness and the www.freepsychotherapybooks.org 345 achievement of ethical self-realization.

Actually, we have witnessed in official circles of the church a reinterpretation of some of its doctrines and a revision of standard documents.

There are many now who are capable, without conflict, of maintaining communion while accepting

a scientific conception of the world.

We have seen also a softening of the stand of science in the form of recognition that religious fulfillment satisfies a fundamental human need.

Psychotherapy and the newer direction in religion thus share a number of common goals. They both strive to bring about better self-understanding and a full utilization of latent creative potentials.

Psychotherapy attempts to do this by bringing the person to a fuller realization in terms of past experiences and the residual distortions that hamper his or her present interpersonal relationships, while encouraging new and more productive behavioral patterns.

Religion encourages the search for new meanings by affiliation with the Divine Being and by worship and prayer that can suffuse the human spirit with hope, with strength to resist deviant drives, and with new directions that will lead to selffulfillment.

Both psychotherapy and religion seek to alter destructive values in the individual and to lead one toward humane values that will accomplish the greatest social good, such as honesty, loyalty, charity, love, courage, and compassion for suffering.

Both psychotherapy and religion promote salutary family and community relationships as virtuous to the highest degree.

Existentialism Existentialism is a philosophy that has attracted many psychotherapists in Europe and the United States, who have, on its premises, developed "ontological" or "existential" treatment procedures that attempt to combine existentialist doctrines with reeducative and reconstructive methodologies (Binswanger, 1947, 1956; Boss, 1957, 1963; Frankl, 1948; Havens, 1967; Holt, 1975; Husserl, 1931; Jaspers, 1963; Laing, 1967; May R, 1950, 1959; Minkowski, 1970; Tillich, 1952).

More or less, existentialism is oriented around the writings of Søren Kierkegaard www.freepsychotherapybooks.org 346 (1813-1855), who revolted against the impotence of Hegel's philosophy of "pure thought" as a means of coping with the paradoxes and contradictions of human existence (Kierkegaard, 1951).

Fruitless, contended Kierkegaard, were faith and coercive divine grace, proffered as a means of salvation; Christ was no substitute for true experience in the world.

Nor was science of any greater use, for the human search for facts as an escape from moral decisions was not possible.

Haunted by perpetual despair and dread (Angst, anxiety), which "eats away all the things of the finite world and lays bare all illusions," a human could not remain a mere spectator, finding refuge in evanescent comforts.

He or she was forced to assume responsibility and to make a choice.

Duty might dull the person's consciousness; it might, in a romantically optimistic way, enable the person to evade responsibility.

It could not, however, eliminate responsibility.

Essential was a free choice to which the individual committed as a whole in the recognition that human values were insignificant indeed.

In the crisis of existence, the only true refuge was a leap into religion in which a person related to the infinite.

Freedom of choice was a fount of anguish because individuals had a limited time in which to act.

They needed courage to be.

When the immortal soul was at stake, the choice would crucially determine which way to turn.

The phenomenological descriptions of inner turmoil, such as have so vividly been described by Kierkegaard, are of a different order of conceptualization than psychological ideas of drive and defense.

Kierkegaard, being a religious man, stressed the choice of religion as a means of surcease from anguish.

Later existentialists, however, made other choices, such as agnosticism (as in the writings of Jean Paul Sartre) or aestheticism.

Karl Jaspers, in *Man in the Modern Age*, considered that the chief threat to modern humankind lay in our complex technology.

Philosophie, published in 1932, spoke of the importance of the persistent quest for knowledge as a means by which humans actualized themselves.

These ideas were enhanced by the phenomenological concepts of Edmund Husserl, a German philosopher, who contended that human experience was corrupted by attempting its analysis through the physical sciences.

Only through reflection ("transcendental phenomenology") could one best realize essential structures, vital relationships, authentic existence, and "the experiences of phenomena."

Science with its focus on causation www.freepsychotherapybooks.org 347 contributed to human alienation by crushing individuals' creativity and intuition.

Human beings were not puppets upon whom reality could be thrust.

They were free agents with freedom of choice and responsibility for their own values.

In 1927, Martin Heidegger, a pupil of Edmund Husserl, wrote *Sein und Zeit*, which followed Husserl's feeling, as expressed in his *Logische Untersuchungen*, that human beings needed to focus on their inner experience as a way of apprehending the outer world.

Heidegger detailed some of the important inner experiences underlying our scientific understanding.

His description of humans' preoccupation with the inevitability of death had a profound influence on existential formulations.

Periods of crisis in world history, such as during and after war, bring forth the philosophy of protest against the world.

Various interpretations of existentialism have been made by different devotees of this philosophy.

Jean Paul Sartre stresses the need to preserve human loneliness from the encroachment of others.

Karl Jaspers and Gabriel Marcel emphasize the interpersonal communication of "loving conflict," during which each participant retains his or her uniqueness.

Other prominent existentialists who have contributed to the theory are Karl Barth, Martin Buber, Emil Brunner, Paul Tillich, Reinhold Niebuhr, H.

Spiegelberg (1960), and E.

Straus (1963).

Drawing from Kierkegaard's original conceptions, psychiatrists such as Karl Jaspers (1947), Ludwig Binswanger (1942, 1947, 1955, 1956), Viktor Frankl (1955, 1967), and Medard Boss (1963) adapted existentialist ideas within a framework of psychotherapy.

Incorporated was Heidegger's conception that concern with others was the key to the optimal development of the self and that endurance of anxiety was more essential than attempts to deny or evade it.

According to Heidegger, the basis of existence is a sense of nothingness that vitalizes the self with meaningfulness.

This idea borders on mysticism, although existentialism is not a mystical philosophy.

Heidegger insisted that the self reached its highest stature in its struggle with anxiety, guilt, and the ubiquitous threat of death.

To this idea Binswanger added the concept of the self reaching its fullest development in the experience of love: indeed the keystone of existence, he believed, was

www.freepsychotherapybooks.org 348 the achievement of “we-ness” through love.

But love is threatened and broken down by anxiety and isolated by excessive independence and overidealization of the love object.

With the shattering of love, the self suffers damage and returns to nothingness.

Frankl introduced the entity of the “existential neurosis,” derived from the inability to see meaning in life.

Through “logotherapy,” he avowed, a search could be organized into the patient’s world view with the object of helping the patient to adjust it to an “authentic existential modality.”

Novel approaches to existential therapy reflect varying interpretations of existential philosophy, blended with the specialized psychotherapeutic techniques of the authors.

Since the conceptual model of existentialism is ontological, it is difficult to understand it in mechanistic terms.

We are dealing with frames of reference unlike those that the average therapist has known.

It seems futile to describe such phenomena as “being in the world” and “I am” in purely verbal terms.

Inherent in these ideas are selfacceptance, tolerance of limitations, the ability to scale down ambitions, acknowledgment of prevailing creativity, self-realization, and self-determination, shorn of neurotic cultural values and demands.

These goals are achieved and achievable only in a special kind of empathic relationship, which presupposes a willingness on the part of the therapist to enter into the world of the patient.

While the therapist utilizes habitual psychotherapeutic tools, for example, the various reeducational techniques, they are employed from a perspective of “reverent love” toward the human being “encountered” as a patient, sharing with him or her a common plane of existence.

The therapist moves beyond drives and mechanisms in dealing with the person as he or she is.

In fundamental “being-togetherness,” the patient’s and therapist’s modes of being are related.

In such a climate of “care” and understanding, the patient gets the courage to emulate the

therapist’s healthy style, and so moves on to “be” in the matrix of a uniqueness of self.

There is much in this conception of the therapeutic relationship that resembles the

“genuineness” and “empathic understanding” advocated for the “client-centered” therapist.

The explication of the phenomenon of self relating to self, however, is much more embellished and particularized.

www.freepsychotherapybooks.org 349 Logotherapy is a special existential approach, elaborated by

Viktor Frankl (1955, 1961a, b, 1962, 1963, 1966), which includes two distinctive reeducative procedures, “paradoxical intention” and “de-reflection.” It proposes to fulfill “the innate

spiritual drive in man” by exploring the “meaning of human existence.” The “will to meaning,”

according to Frankl, is one of the deepest motivating forces of humanity, even more fundamental than Freud’s “pleasure principle” or Adler’s “will to power.” When human beings no longer

question the meaning of their existence, they become ill.

This is because they cannot function in an “existential vacuum.” A basic conflict precipitates

as a result of this inability to “emerge spiritually above the level of his own psychic and

physical conditions.” This conflict is not rooted in psychological complexes; it is focused in

spiritual and ethical issues and produces neuroses of a unique quality, which Frankl calls the

“noogenic neuroses.” Approximately 12 percent of all neuroses are said by him to be of the

“noogenic” variety.

Such “noogenic, spiritually rooted” neuroses require for their solution the existential

approach of logotherapy “in contrast to psychotherapy in the narrower sense of the word.” In

logotherapy, the patient is helped to find new values and to develop a constructive philosophy in life.

“The logotherapist is not primarily concerned with treating the individual symptom or the

disease as such.

Rather, he sets out to transform the neurotic's attitude toward his neurosis." Responsibility is put firmly on the shoulders of the patient to "push forward independently toward the concrete meaning of his personal existence." The patient "must choose it on his own, search for it, and find it." Logotherapy helps the patient evolve three kinds of values which will lend meaning to existence: "creative values," "experiential values," and "attitudinal values." The patient is shown how to make life meaningful by "the experience of love," which enables the patient to enjoy "truth, beauty and kindness ... and human beings in their uniqueness and individuality." The patient is shown that suffering may be useful in helping to change his or her attitudes.

For instance, an irremediable situation may have to be endured.

"Where we can no longer change our fate by action, what matters is the right attitude toward fate ...

we must be able to accept it." The goal of the logotherapist, then, is to evoke in the individual the "will-to-meaning," which is specific and personal for each person.

Only when this goal is fulfilled can one survive the most unfavorable conditions.

The dimension of helping to find "meaning" is above and beyond what may be done for the individual through exploration of the dynamics of his or her intrapsychic processes and by probing his or her interpersonal relationships through the traditional psychotherapeutic and psychoanalytic techniques.

The latter tactics become futile or only partially successful unless there is restored to the person the spark in his or her being that gives life purpose and meaning.

In the words of Nietzsche, "He who knows a Why of living surmounts almost every How." The explicit answer to the question of what constitutes the "meaning of his existence" can be answered only by the patient under the guidance of the therapist.

Even in dire distress or under depriving circumstances in which activity and creativity are blocked, a person "can still give his life meaning by the way and manner in which he faces his fate, in which he takes his suffering upon himself.

Precisely in this he has been given a last chance to realize values." Frankl agrees with Goethe, who emphasized that "there is no condition which cannot be ennobled either by a deed or by suffering." Thus, "the right kind of suffering is itself a deed, nay the highest achievement which has been granted to man." In this way the patient may be aided in utilizing daily vexations and sufferings as a means toward finding purpose in living.

By putting a positive value on suffering, the victim may overcome "trends in our present-day civilization where the incurable sufferer is given very little opportunity to be proud of his unavoidable and inescapable suffering and to consider it ennobling rather than degrading."

Persuasive tactics are employed in logotherapy to get the patient to adopt a more constructive attitude toward his or her difficulties.

Frankl illustrates this in a case of a nurse who suffered from an inoperable tumor and who experienced despair centered about her inability to work in her cherished profession.

"I tried to help her understand that to work eight hours or ten hours or God knows how many hours a day, is no great thing.

Many people can do that.

But to be as eager to work as she was and to be incapable of working yet not despairing would be an achievement few could attain." An appeal was then made to her sense of fairness to the patients to whom she was dedicated in her work, since she was depreciating them when she assumed that a sick person's life had no meaning.

"In so doing you take away from all sick and incurable people the right to life and

justification for their existence.” A shift in point of view effectuated by the therapist’s promptings may interrupt a neurosis.

Frankl illustrates this point: I should like to quote another case of a colleague, an old general practitioner who turned to me because he still could not get over the loss of his wife who had died two years earlier.

His marriage had been very happy and he was very depressed.

I asked him quite simply, “Tell me, what would have happened if you had died instead of your wife, if she had survived you?” “That would have been terrible,” he said.

“Quite unthinkable.

How my wife would have suffered.” “Well, you see,” I answered, “your wife has been spared that, and it was you who spared her, though, of course, you must now pay by surviving and mourning her.” In that very moment his mourning had been given a meaning— the meaning of sacrifice.

The depression was overcome.

What Frankl brings out is that he had succeeded in giving a new meaning to the experience of his patient’s suffering.

What precise dynamics were involved in the patient’s attitudinal shift, other than appeasement of his sense of guilt, we do not know; however, the tremendous impact that a respected authority may have on a susceptible patient by a timed interpretation is clearly brought out, particularly in helping the patient face death, suffering, and other disasters in life.

Because happiness can rarely be achieved intentionally, the patient is enjoined by the logotherapist to avoid striving for its achievement.

Rather, the patient is shown that happiness will come as a byproduct to the attainment of other values.

Nor should “unpleasure” be evaded.

Logotherapy contends that intention may prevent the occurrence of an event.

This principle may be applied to fighting neurotic symptoms.

By the technique that Frankl calls “paradoxical intention,” the patient is sometimes “encouraged not only to accept the neurotic symptoms but even to try to exaggerate them.

Thus the patient increasingly learns to put himself ‘above’ the symptom.” Paradoxical intention, which mobilizes the “capacity of self-detachment,” is particularly suited for the short-term treatment of obsessive-compulsive and phobic patients.

The fearful anticipation of an www.freepsychotherapybooks.org 352 event often succeeds in bringing on the reactions developing from the event.

Patients with strong obsessions tend to fight off their obsessions and compulsions.

If they stop fighting their symptoms, and even joke about them, the symptoms may diminish and disappear.

The patient is requested to think about or to experience in his or her mind that which is unpleasant, terrifying, or embarrassing.

In this way the patient develops, as in behavior therapy, an ability to counteract his or her fears.

Paradoxical intention, says Frankl, is contraindicated in psychotic depressions.

It is a basis around which more modern ideas of paradoxical therapy have been organized (Borkovec & Boudewyns (1976), Ascher & Efran (1978), Fay (1976).

Another logotherapeutic technique is “de-reflection,” which enhances the “capacity for self-transcendence.” Here the patient is first helped to an awareness of unused or forgotten capabilities and potentialities.

“It is a kind of appeal to the patient’s deeply buried values.

Once they are uncovered, they assert themselves and give the patient a feeling of uniqueness, of usefulness, and a sense of life.” De-reflection is said to be valuable in somatic

preoccupations, neurotic sleep disturbances, and such sexual disorders as impotence and frigidity.

Frankl supports eclecticism in method and advocates, when necessary, a combination of logotherapeutic techniques with hypnosis, Schultz's autogenic training, behavior therapy, and pharmacotherapy.

The concept of "responsibility" is a cornerstone in logotherapy, as it is in other forms of existential therapy.

This purports that individuals are responsible for making of their lives what they will—a thing of joy or a living hell.

The interpretation imparted to his or her experiences is the arbiter; one possesses the means of changing one's destiny by altering one's values.

The leading maxim of existential therapy, in the words of Frankl, may be phrased in this way: Live as if you were living for the second time and you had acted as wrongly the first time as you are about to act now.

Once one really puts oneself into this imagined situation, one will instantaneously become conscious of the full gravity of the responsibility that www.freepsychotherapybooks.org 353 every human bears throughout every moment of his or her life, the responsibility for what he or she will make of the next hour, or how he or she will shape the next day.

Criticism of Philosophical and Religious Approaches The utility of philosophical and religious approaches in promoting mental tranquility is evidenced by their universal employment.

As a means toward providing institutionalized outlets for inner needs, of bolstering psychological defenses, and of facilitating more congenial relationships among people, they have served humankind from its earliest origins.

Where they are supported by cultural sanctions, where environmental stress and inner conflict are not beyond coping capacities, they may sponsor a healthful adaptation.

They have serious limitations, however, where they contradict the disposition of a culture, buttress crippling repressions, or endorse neurotic mechanisms, such as retreat from reality.

Once accepted, ideologies tend to be defended by a captious logic or transcendental dialectic that seeks to deny their inherent contradictions.

Rationalizations suffuse the individual with the reasonableness of his or her own arguments and attempt to weld together into a completely coherent body that which even superficial examination shows to be spurious.

Many philosophies are predicated on the principle that the human mind, as it is constituted, is unable to understand the ultimate nature of things.

Unique attitudes, values, and modes of feeling and behaving are then evolved to provide new meanings for existence.

If all thinking, feeling, and striving can be channelized into a unified and unifying system—whether this be mystical, existential, hedonistic, skeptical, or other— emotional stabilization may be achieved, at least so long as the system remains intact.

Should the system fail to secure inner peace, however, a search for new and more stabilizing prescripts will then ensue.

Psychotherapists are attracted to philosophical and religious helping practices partly in appreciation of the limitations of psychotherapy and partly because of their www.freepsychotherapybooks.org 354 personal emotional needs.

Because they have found their own lives enriched by certain points of view, they may adopt these as a basis for an approach that, in their hands, will probably prove to be fruitful.

Yet the sponsoring of philosophies will not be useful for all patients; special needs and resistances will enjoin them to modify or reject the therapist's best efforts at indoctrination.

Philosophical promptings in themselves are not sufficient for the struggle with severe personality immaturities and deep neurotic conflicts.

Indeed, they are too often used to reinforce neurotic strivings, for instance, to bolster fears of authority, to negate sexual and aggressive impulses, to avert anxiety through obsessive rituals, to pander to self-punitive and masochistic leanings, to remove the individual from the demands of reality, and to identify with omnipotent extramundane forces with the intent of incorporating them within the self.

Such ideologies may negate rather than help a constructive adjustment.

Arguments posed by scientists against the serviceability of religious practices are even more stringent (Freud, 1949; Reik, 1951).

A good number of devotees of science admit that there are aspects beyond observable experience, but they are not willing to elevate these to a deistic substance or being in whose transcendent vision all phenomena are clear.

Religion resents the implication of some scientists that the craving for God is an infantile or neurotic prompting, and it challenges the denial of the validity of an Almighty Being, since science can offer no experimental proof that there is none.

What better evidence is there for God, protests religion, than the miracle of creation of living things, the source of which no scientist has been able to qualify, let alone quantify.

On more specific grounds, a point of conflict between science and religion is that the former regards moral deviation as a symptom, the latter as a sin.

Science is apt to look on a human being as an irrational entity and on deviations as unwilled and manipulatory of the individual without his or her desire or awareness.

Religion considers a human being a rational being and holds deviation to be an act against humankind and God to be judged in moralistic terms.

It suspects science of belittling the individual's responsibility for his or her behavior, appeasing guilt feelings and encouraging the acting out of errant impulses and drives that are morally reprehensible.

Thus, science is seen as acquitting anger, covetousness, envy, gluttony, lust, pride, and sloth—the seven deadly sins—as byproducts of an individual's past conditionings, the liability for their present manifestations falling on the shoulders of parents and not patients.

Moreover, science is considered by some representatives of religion to encourage a closer relationship to the psychotherapist than to the pastor, hence diverting patients from seeking spiritual guidance.

Science counters by pointing out that certain religious precepts contradict the biological nature of a human being and that a literal interpretation of the Bible will cast a shadow over healthy promptings that stir in all persons in the course of personality development, for instance, in relation to sexual and aggressive feelings.

Relegating the individual to eternal damnation for sexual drives out of wedlock, for adulterous thoughts and fantasies, for interest in prurient materials, for retaliatory aggression in the face of humiliation or exploitation, for rebellious impulses toward parental agencies—all of this serves merely to mobilize pathological guilt and shame, to sponsor masochistic, sadistic, and other neurotic responses, and even to cripple healthy drives for reproduction and self-defense.

The meek, the poor, the sad, the peaceful are not necessarily blessed.

Temporary anger, resentment, and bitterness may be justified and need not irreparably violate the spirit of Love.

Mutilation is not a worthy punishment for fantasies that oppose monogamy and fidelity.

Passive compliance cannot always be the response to those who promote evil, nor can one in the spirit of tolerance always love one's enemies.

There are some scientists who believe that any morality that proposes an absolute code, violation of which brings supernatural punishment, contains within it seeds of conflict, since the definition of what constitutes the proper morality is open to some question. Thus, obedience to authority and unquestioning submission conflicts with selfdetermination and independence.

Un-worldiness, with its abandonment of natural pursuits, leads to passive alienation; shame and guilt in relation to bodily desires may inspire monastic self-torment and degradation of sexual desire with a repression of rage and aggression.

Many of the sacrificial rituals in religion stem from this conflict.

Religious systems, it is also claimed, may sponsor conflict and bad values, representing the bad in www.freepsychotherapybooks.org 356 the form of an inhabitant of the underworld—devil, satan, Baal-Zebub—an evil representative who opposes God for the mastery of the universe. This struggle, recorded vividly in the Talmud and New Testament, and especially in the Apocryphal and Apocalyptic Books, is literally incorporated in the conviction that the devil is constantly on the alert to occupy an individual's body and to pervert that individual to his satanic purposes.

This is a potent fount of anxiety, particularly when the religious individual feels incapable of living a perpetually sainted life.

More in the form of an abstraction than in a pictorialized image of a cloven-footed, horned monster, the notion of an invasive devil may preoccupy those whose fundamentalist notions are rooted in medieval conceptions that continue to be a means through which they control and discipline themselves.

A conflict in moral values may furthermore be sponsored in religious systems that conceive of the right and ability on the part of the individual to do and think as he or she wishes, as theologically opposed to the idea of predestination.

If we conceive of all humankind as “free,” we must assume an autonomous morality with self-determinism and unpredictability dissociated from Divine foreknowledge.

Science claims that sources of behavior are too often considered by religion to be the product of moral judgments of which the individual is fully aware and which he or she may freely “will” and “choose.” Very often, however, the determining factors in behavior are unconscious in nature, outside the awareness and control of the individual.

One is frequently driven in one's choices by forces that are not apparent at moments of decision.

A view reconciling these disputations, and one that psychotherapy endorses, is that moral choice is still the individual's in his or her freedom to exercise it; however, each person has the nature to misuse it.

Awareness of one's unconscious enables the individual better to exercise moral control.

Freedom of will does not imply capriciousness or irresponsibility.

Another conflict brews over the matter of fundamentalism.

A believer must accept the Divine authorship of the Scripture, since the authors were merely the agencies for the transcription of sacred and unalterable doctrines.

Deviation from the sentence and word, in the mind of fundamentalists, tends to discredit God. Science contends that the Bible is a human document, subject to the same kind of study and analysis as any other human www.freepsychotherapybooks.org 357 document.

Fundamentalists insist that biblical criticism, and the pointing out of discrepancies in the Scripture, are illusions of the devil.

Reconciliation of opposing viewpoints is often attempted by the judgment that while the ideas in the Bible are Divine, the canons of Scripture are the product of the church on earth.

A strong undercurrent of fundamentalism still exists that may disturb those who have no

investment in orthodoxy.

Fundamentalist conflict, incidentally, is not confined to the Bible; it invests many fields of thought, even scientific, where allegiance to the omniscience of founders of a movement tends to circumscribe one's thinking to the letter of published text or to interpretations of the text by self-appointed prophets.

We are currently witnessing a softening of the stand of science and religion toward each other. By integrating the findings of science, religion may be able to achieve its highest aims in encouraging values that make for a better world.

By studying the forces activated by faith, scientists may be able to release the same healing elements more precisely in the medium of a scientific methodology.

In summary, philosophical and religious systems in the themselves do not suffice to control or rectify severe neurotic distortions.

In an eclectic therapeutic framework, philosophical precepts may sometimes be incorporated to develop socially useful values helpful in promoting adaptation.

www.freepsychotherapybooks.org 358 11 Reconstructive Therapy An ultimate goal of psychotherapy is to reduce the force of irrational impulses and strivings and bring them under control, to increase the repertoire of defenses and make them more flexible, and to lessen the severity of the conscience, altering value systems so as to enable the patients to adapt to reality and their inner needs.

These aims are formidable because the various components of personality are so forged into a conditioned system as to be almost impervious to outside influence.

Homeostatic balances are maintained to safeguard neurotic adjustment.

Resistances block attempts to interfere with coping mechanisms and defenses.

To cut into the neurotic system in order to modify the structure of personality and to expand the potentials of the individual in all required dimensions are difficult and frequently unrewarding undertakings.

Reconstructive psychotherapy is aimed at these objectives.

Reconstructive psychotherapy is more or less traditionally rooted in the theoretical soil of a genetic-dynamic model of personality.

This purports that past inimical experiences and conditionings have retarded the normal psychosocial growth process and are now promoting in the individual immature strivings and emotions that come into conflict with reality, on the one hand, and, on the other, with the person's own incorporated system of ideals and standards.

Resultant are tensions, catastrophic feelings of helplessness, and expectations of injury that in turn invoke protective devices, most common of which is repression, a sealing-off process that blankets offending impulses, attitudes, and memories from awareness.

However noble the attempt, repression of unacceptable strivings rarely succeeds in annihilating them, for their expression is sought, from time to time, by powerful motivations of impelling need.

The filtering of offensive impulses into conscious life promotes bouts of anxiety and whips up the defenses of the ego, which, while ameliorating anxiety, may be destructive of adjustment. Additionally, repressed strivings may express themselves as symptoms.

The direct or disguised operations of repudiated strivings, and the defenses that are mobilized against them, www.freepsychotherapybooks.org 359 promote attitudes and values that disorganize interpersonal relationships.

Reactions develop that are opposed to judgment and common sense.

While individuals may assume they are acting like adults, emotionally they are behaving like children, projecting into their present life the same kinds of fears, misinterpretations, and expectations of hurt that confronted them in their early years, as if neither time nor reality

considerations have altered materially the patterns learned in the past.

As long as individuals protect themselves from fancied hurt by circumscribing their activities, they may manage to get along; but should they venture beyond their habitual zone of safety, the precarious balances they have erected will be upset.

Were we to treat a patient according to this hypothesis, we would consider that his or her symptoms were manifestations of a general collapse in adaptation, and our therapeutic effort would be directed toward correcting disorganizing drives that were destructive to the patient's total adjustment.

The objective, therefore, would be expanded toward personality growth and maturation, toward heightened assertiveness and greater self-esteem, and toward more harmonious interpersonal relationships.

In quest of these objectives, we would strive for a strengthening of the patient's ego, which, involved hitherto in warding off anxiety through the marshalling of neurotic defenses, has been unable to attend to the individual's essential needs.

Instrumentalities toward ego strengthening are, first, greater self-understanding and, second, the living through with a new kind of authority, as vested in the therapist, of experiences that rectify residual distortions in attitudes, feelings, values, and behavior.

Increasingly, psychotherapists, directing their efforts toward reconstructive changes, have drawn for inspiration on psychoanalytic theories and methods, although some have tended to label the ideas and tactics that they employ with tags that mask their origin.

Accordingly, during the past half century, many ingredients of psychoanalytic thinking have permeated into the very fiber of American psychiatry and psychology and have fashioned a good number of its trends.

In turn, psychoanalysis has been influenced by contemporary developments in the psychiatric and psychological fields.

It has consequently lost many of its esoteric qualities while acquiring a firmer anchoring in www.freepsychotherapybooks.org 360 scientific experiment.

Reconstructive psychotherapy is distinguished from supportive and reeducative therapy by the degree and quality of insight mobilized.

In supportive therapy efforts at insight are minimal.

In reeducative therapy they are more extensive, but they are focused on relatively conscious problems.

The traditional objective in reconstructive therapy is to bring the individual to an awareness of crucial unconscious conflicts and their derivatives.

Reconstructive psychotherapy strives not only to bring about a restoration of the individual to effective life functioning, through the resolution of disabling symptoms and disturbed interpersonal relationships, but also to promote maturation of emotional development with the creation of new adaptive potentialities.

The methods employed in bringing unconscious aspects to awareness were originally developed and described by Sigmund Freud, who had the happy faculty of illuminating the most obscure concepts with a refreshing verbal simplicity.

Included are such techniques as free association, dream interpretation, the analysis of the evolving transference, the use of strategically timed interpretations, and dealing with resistances to the content of unconscious material.

An understanding of the genetic determinants of the individual's personality and of the relationship of these determinants to the operative present-day character structure are component aspects of the therapeutic process.

To do reconstructive therapy, the therapist must have received special training, which ideally includes a personal psychoanalysis and the successful treatment of a number of patients under

the supervision of an experienced psychoanalyst.

As has been indicated previously, reconstructive personality changes sometimes occur spontaneously during the course of supportive and reeducative therapies or upon completion of these treatments as a consequence of more congenial relationships with people.

For instance, the individual may work out serendipitously, in the medium of a relationship with a helping agency or reeducative therapist, such archaic strivings as infantile dependency needs, unyielding fears of rejection or overprotection, intense detachment, and untoward aggression.

The patient may even spontaneously connect the www.freepsychotherapybooks.org 361 origin of such impulses with unfortunate childhood conditionings and experiences.

Any changes developing in this way, however, are more or less fortuitous.

In reconstructive psychotherapy, the treatment situation is deliberately planned to encourage change by a living through, with insight, of the deepest fears and conflicts.

There are four main "types" of insight therapy with reconstructive goals: (1) "Freudian psychoanalysis," (2) "ego analysis," (3) "non-Freudian" or "neo-Freudian psychoanalysis," and (4) "psychoanalytically oriented psychotherapy."

All of these therapies aim at reconstructive alterations in the personality.

They differ, however, as to the methods by which this objective is realized.

Freudian psychoanalysis is, more or less, the original technique of Sigmund Freud.

Ego analysis, while retaining the classical therapeutic form, focuses on the adaptive functions of the ego.

Neo-Freudian psychoanalysis, which includes the approaches of Horney, Sullivan, Rank, Jung, and Adler, is a modified, more active technique.

Psychoanalytically oriented psychotherapy is the most active of the reconstructive therapies.

In addition to these main types, there are a number of modifications, such as Kleinian and transactional analysis.

Derived from Kleinian theory are object relations therapy, therapy oriented around self-psychology, and some forms of transactional analysis.

FREUDIAN PSYCHOANALYSIS From recorded history it is obvious that there were people in every age and in almost every culture who had some understanding of the significance of people's psychic life in the scheme of things.

The Bible and the writings of the early philosophers contain a wealth of psychological wisdom.

But none of the insights was integrated into an organized form until a man from Moravia.

Sigmund Freud, through his genius for understanding people's inner motivations and his unflinching determination in the face of violent opposition, broke through and laid a foundation for the science of the psychic processes in what is known today as psychoanalysis.

In 1880 Joseph Breuer discovered that when a hysterical girl under hypnosis was induced to speak freely, she expressed profound emotion and experienced relief from her www.freepsychotherapybooks.org 362 symptoms.

Under the impression that her hysteria originated in certain painful experiences while caring for her sick father, Breuer enjoined her, while she was in an hypnotic state, to remember and to relive the traumatic scenes in her past.

This seemed to produce a cure for her hysteria.

Ten years later, in conjunction with Freud, Breuer continued his research, and in 1895 the two men published their observations in a book, *Studien Uber Hysteria* (1936).

Their conclusions were that hysterical symptoms developed as a result of experiences so traumatic to the individual that they were repressed.

The mental energy associated with the experiences was blocked off, and not being able to reach consciousness was converted into bodily innervations.

The discharge of strangulated emotions (abreaction), through normal channels during hypnosis, would relieve the need to divert the energy into symptoms.

This method was termed “catharsis.” Freud soon found that equally good therapeutic results could be obtained without hypnosis by permitting the patient to talk freely, expressing whatever ideas came to mind.

Freud invented the term “psychoanalysis” for the process of uncovering and permitting the verbal expression of hidden traumatic experiences.

Freud found that there were forces that kept memories from invading consciousness, and he discovered that it was necessary to neutralize the repressing forces before recall was possible.

An effective way to overcome resistances was to permit the patient to relax and to talk freely about any ideas or fantasy that entered his or her mind, no matter how trivial or absurd.

Freud could observe in this “free association” a sequential theme that gave clues to the nature of the repressed material.

Mainly through an introspective analysis of his own dreams, Freud (1938) was able to show how dreams were expressions of unconscious wishes and fears that evaded the barriers of repression through the assumption of symbolic disguises.

He perfected a technique of arriving at the meaning of the unconscious material through the translation of symbols.

Freud also observed that when patients were encouraged to say whatever came to mind, irrational attitudes toward the therapist—such as deep love, fear, hate, overvaluation, expectancy, disappointment, and other strivings that were not justified by the reality situation—were verbalized.

He noted, too, that patients identified the therapist with significant personages in their past, particularly their parents, and that this identification motivated the transfer over to the therapist of attitudes similar to those that they originally had toward their parents.

This phenomenon Freud called “transference.” For example, a male patient with a phobia of being subject to imminent but indefinable injury might, at a certain phase in his analysis, begin to develop an aversion toward and dread of the analyst, expressed in fears of being mutilated.

At the same time incestuous wishes for the mother might appear in dreams.

Analysis of the relationship with the analyst (transference) would then possibly reveal an identification of the analyst with the patient’s father.

It would then become apparent that the patient secretly feared injury by the father for his forbidden wish to possess the mother and that his phobia was an expression of this fear of mutilation that had been dissociated from awareness by repression.

The hope was that bringing patients’ attention to the sources of their fears, and their realization of its irrational nature, would result in an amelioration or cure of their neurosis.

The material uncovered by Freud from his studies of free association, dream interpretation, and analysis of the transference suggested to him that there was a dynamic portion of the psyche, closely associated with the emotional disorder, that did not follow the normal laws of mental functioning.

Freud called this aspect of the mind the “unconscious,” and he set about to determine the unique laws that dominated the repressed psychic component.² To aid him in this task he formulated a topography of the mind by dividing it into three zones or systems: the unconscious (Ucs.), the pre-conscious (Pcs.), and the conscious (Cs.).

The preconscious contained thoughts that could be revived into consciousness with some effort, in contrast to the unconscious, which contained material barricaded from consciousness by an obstructive force.

In studying the symbols issuing from the unconscious, Freud noted that they were concerned chiefly with sexual material, and he concluded from this that the unconscious www.freepsychotherapybooks.org 364 was preoccupied for the most part with sexual wishes and fears.

Consequently, he assumed that the most important traumatic events that had been repressed were sexual in nature.

It was largely on this evidence that he evolved his “theory of instincts” or the “libido theory.” In his theory of instincts Freud (1930a) postulated that all energy had its origin in instincts that persistently expressed themselves (repetition compulsion) and were represented mentally as ideas with an emotional charge (cathexis).

A fundamental instinct was that of eros, the sexual or life instinct, manifesting itself in a force called “libido.” Freud hypothesized a permeation of the body by this vital instinctual force, the “libido,” which powered the individual’s development toward mature sexuality. Libido was, however, subject to many developmental vicissitudes in its destined course to adult genitality.

During the first year of life it concentrated itself around the oral zone—the mouth and lips—the child gaining a kind of erotic pleasure by sucking and later by biting.

At the end of the first year there was a partial shift in libido to the anal zone, and intense pleasures were derived from the retention and expulsion of feces.

During this period the child’s interests were more or less concentrated on oneself (narcissism), and satisfactions were localized primarily within his or her own body (autoerotism).

Relationships with people were primitive, being circumscribed to only part of the parent (part-object relationships), like the nipple or breast instead of the entire parent.

Around the age of 3, libido was centered around the phallic zone—the penis or clitoris.

“Object relationships” were less primitive and were extended to a more complete relatedness with the parent.

Yet, fundamentally, the child was ambivalent, responding to parents and other people with a mixture of love and hate.

This stage of psychosexual growth continued into the Oedipal period, during which the little boy developed toward his mother a profound interest, with strong sexual overtones and desires for exclusive ownership.

The little girl, envying men for their possession of a penis, created in part by a desire to repudiate her femininity and to become a male (penis envy), and resenting the fact that she had no penis, accused the www.freepsychotherapybooks.org 365 mother of responsibility for this deprivation and turned to the father with an intensified sexual interest.

In the case of the boy, hostility toward the father, due to a desire to eliminate him as a rival, generated a fear of counterhostility, and particularly a fear of castration, which inspired such anxiety as to induce him to give up his interest in the mother and to make friends with his father.

The intensity of fear became so overwhelming and so unendurable that the boy was forced to yield to his more powerful competitor by renouncing, repudiating, and repressing sexual feelings toward the maternal love object.

He was obliged also to repress concomitant hostile impulses toward the father.

The little girl similarly resolved her enmity toward her mother as well as her sexual interest in her father.

This drama, known as the Oedipus complex, was to Freud the crucial nuclear conflict in the development of the personality contributing to both character formation and neurotic symptoms. The incorporation of parental injunctions and prohibitions and the repudiation of sexual and

hostile aims as related to the parents resulted in the crystallization of an aspect of the psyche that took over the judging, prohibiting, and punitive functions hitherto vested in the parents.

This aspect became the conscience or superego.

The adequate resolution of the Oedipus complex was associated with channelization of libido into the genital zone, with capacities for complete, mature, unambivalent, "whole-object" relationships.

After the Oedipal period was an era characterized by the neutralization of sexual impulses, which Freud called the "latency period." With the advent of puberty, however, increased libido, due to the heightened activity of the genital glands, reactivated the old Oedipal interests.

The person then lived through the revived early Oedipal conflict, and the capacity to solve this anew was determined by the extent of previous vicissitudes and the adequacy with which the conflict had formerly been resolved.

In "normal" solutions the child transferred his or her sexual interest to extrafamilial persons of the opposite sex.

In Freud's pattern of behavior the little girl renounced her boyish interests and accepted a passive female role.

www.freepsychotherapybooks.org 366 Under certain conditions normal psychosexual development was impeded by a "fixation" of libido onto oral, anal, and phallic zones.

The libido, bound down in this way, was unable to participate in the development of full genitality.

Freud believed that both constitutional and experiential factors were responsible for this.

Most prominent were excessive gratifications or inordinate frustrations experienced at an early stage of growth.

Not only did libidinal fixations interfere with the development of mature sexuality, but they constituted stations to which the individual might return when confronted with overwhelming stress or frustration.

Under these circumstances the libidinal stream was said to undergo "regression" to pregenital fixation points.

When this happened, there were revived attitudes and interests characteristic of childhood, with immature sexual strivings, interest in "part objects," and narcissism.³ In addition to the libido theory described above, Freud elaborated the theory of the death instinct to account for phenomena not explicable in terms of libido.

He postulated that it was this second instinct that prompted aggressive and destructive drives.

This instinct manifested itself in a "repetition compulsion" to undo the forward evolutionary development of the organism and to return it to its primordial inorganic state.

The death instinct, though sometimes libidinized (sadism), was totally different from the sexual instinct.

Largely because of his studies on sadism and masochism, Freud came to the conclusion that the sexual and aggressive drives could be fused to foster both normal and pathological activities.

The aggressive drive was also subject to vicissitudes in its development through oral (biting), anal (soiling and retention), and phallic phases, during which fixations could occur to which regression was later possible.

Freud conceived of the mental apparatus as an organ that prevented the damming up of energy. Pain was related to an increase of energy and pleasure to a decrease.

To help understand the operations of the mental apparatus, Freud elaborated, apart from his topographical theory, a structural theory of the psyche.

According to this theory, the psyche involved (1) a reservoir of instinctual energy, the id;

(2) a supervisory area serving a censoring and sanctioning function, the superego; and (3) a

body that mediated internal and external adjustments, the ego.

Although recognized by Freud as arbitrary, empiric, www.freepsychotherapybooks.org 367 and metapsychologic, these subdivisions were retained by him as a conceptual necessity (Freud, 1930a).

Freud classified the id as the original undifferentiated mind, the repository of inherited urges and instinctual energy.

It contained the instincts of Eros—the life or sexual instinct—and Thanatos—the death instinct. It provided the individual with dynamic energy (libido), which vitalized every organ and tissue and sought expression in response to a “pleasure principle,” along whatever channels were available for it.

Through impressions received by the perceptual organs, the id underwent modifications immediately after birth.

Differentiation by the child of himself or herself as an entity apart from the world was in keeping with the evolution of the ego, which increasingly assumed the function of an executive organ, harnessing the id to the demands of reality (reality principle).

Important impressions, particularly those related to experiences with parents or their surrogates, and frustrations created by prohibitions of pleasure strivings, registered themselves on the child's psyche and stimulated primitive mechanisms of projection and introjection.

In projection, aggression was discharged outward and directed toward parents; in introjection, the frustrating parental agencies were “incorporated” within the child's psychic apparatus. Through these mechanisms, rudiments of a superego developed that later, with the resolution of the Oedipus complex, crystallized and took over the guiding and prohibitive functions of the parents.

One aspect of the superego contained constructive ideals toward which the individual felt driven (ego ideals).

The superego, oriented as it was around childish concepts of reality, evaluated actions in terms of whether they had approbatory or punitive potentials.

It was the guardian of a primitive morality attended at all times by a custodian of guilt.

Unconscious disapproval by the superego gave rise to a host of disturbances, including a need for punishment, feelings of inadequacy, and low self-esteem.

Under the lash of the superego the ego created repressions against libidinal strivings and their ideational representatives.

Such repressions served to avoid conflict.

When, however, for any reason, repression relaxed or proved insufficient, the ego was invaded with some of the content of the repressed.

This threat to the individual's security inspired www.freepsychotherapybooks.org 368 anxiety, a danger signal that indicated a breakthrough of the repressed material.

As Freud continued his work, he laid less and less stress on strangled emotions due to early traumatic experiences as the primary cause of neurosis.

More and more he became cognizant of the purposeful nature of symptoms, and in 1926 he revised his theory of neurosis drastically, claiming that symptoms were not only manifestations of repressed instinctive strivings but also represented defenses against these strivings (Freud, 1936).

Freud contended, however, that the essence of a neurosis was a repression of infantile fears and experiences that continually forced the individual to act in the present as if living in the past.

The neurotic seemed to be dominated by past anxieties that, split off, operated autonomously and served no further function in reality.

The accent on the ego and its responses to anxiety moved the emphasis away from the id. Anxiety, a biologically inherited pattern, appeared under two sets of circumstances: “traumatic situations” and “danger situations.” The prototype of the former was the birth trauma. During infancy the influx of stimuli often was too great for the individual to handle and sponsored the development of a stimulus barrier.

As the ego developed, recognition of danger situations enabled the child to anticipate them with anxiety (“signal anxiety”).

The ego mobilized its defenses in response to such signals.

Typical danger situations during infancy were a threatened loss of the love object (breast-mother); around 18 months, loss of love and good will from the love object (parents and other significant persons); around the end of age 3, injury to the genitals (castration anxiety) and deprivation of its gratifications; and after 5 years of age, disapproval and punishment from the superego.

These dangers persisted unconsciously throughout the life of the individual.

Three types of anxiety could be distinguished: real or objective anxiety (in which the danger was external), neurotic anxiety (the danger here was the eruption of id impulses that the ego considered harmful), and moral anxiety (in which punishment from the superego was threatened).

The sources of anxiety were unconscious in nature and were www.freepsychotherapybooks.org 369 usually highly disguised, rationalized, and shielded from awareness.

Neurotic anxiety manifested itself in free-floating anxiety (vague and unfixed), phobic anxiety (irrationally displaced to symbolic situations), and panic anxiety (acting out of id impulses impossible to control—for instance, aggressive and sexual drives).

The latter could activate moral anxiety with needs for punishment.

The examination of defenses that the ego evolved to cope with anxiety revealed a variety of mechanisms, including: 1.

Repression—neutralization of cathexis of psychic energy from the id by counter cathexis, thus barricading the ego from unacceptable impulses.

2.

Projection—transferring responsibility for one’s own inner impulses to others.

3.

Introjection—incorporating qualities of others into the self.

4.

Rationalization—seeking amnesty by disguising motives.

5.

Regression—reverting to earlier forms of fulfillment and defense.

6.

Isolation—splitting emotion or thought associations off from a memory, fantasy, or impulse.

7.

Turning against the self—redirecting aggressive wishes toward others through self-castigation.

8.

Reaction formation—neutralization of one drive by conscious expression of a diametrically opposite one.

9.

Denial of reality—distortion of facts to support inner wishes.

A weak ego, due to constitutional defects and/or severe overindulgence or frustration in childhood, could exhaust the psychic energy and stimulate defenses, such as repression, depriving the id of essential gratifications and diverting the ego from an adaptive dealing with reality.

This was often complicated by problems in superego formation due to an inability to resolve the

Oedipal struggle.

Thus, the superego could be excessively harsh and destructive, sponsoring excessive guilt and inferiority feelings, or it www.freepsychotherapybooks.org 370 might be underdeveloped, leading to impulsive, psychopathic, amoralistic, and delinquent behavior.

A harmonious balance of superego-ego-id led to a character structure that permitted constructive relationships and a proper adaptation.

Imbalance led to psychopathology.

Internal dangers were constantly threatened by the efforts of the id to discharge accumulated tension.

Such discharge was opposed by the mental force of the superego in the form of repression to prevent the release of tension.

Repression was a dynamic force that attempted to seal off internal dangers.

The maintenance of repression, however, required an enormous expenditure of energy.

The ego derived this energy from the id in a subversive manner.

Thus, an idea or tendency invested with libido (cathexis) would be stripped of libido and this energy used to oppose the idea or tendency (anticathexis).

Subtle mechanisms, such as symbolization, condensation, distortion, and displacement, were employed to evade repressive forces and to provide a substitutive discharge of repressed energy and a consequent relief of tension.

Fantasies, dreams, and symptoms were expressions of such mechanisms.

Where the substitutive expression was in harmony with social values and superego ideals, it provided a suitable means of relief (sublimation).

Where it was not in harmony, conflict resulted and repressive mechanisms were again invoked.

If repression proved ineffective in mediating tension, a return to earlier modes of adaptation was possible.

This happened particularly where the individual was confronted by experiences similar to, or representative of, those that initiated anxiety in childhood.

The ego reacted automatically to these experiences, as if the reality conditionings of later years had had no corrective effect on the original danger situation.

It responded with essentially the same defenses of childhood, even though these were now inappropriate.

At puberty and at menopause certain instincts became powerfully reinforced and flooded the capacities of the ego.

Even normal people could become neurotic at these times when, for physiological reasons, the instincts were too strong.

Accidental influences at any other time might also make them too powerful.

All repressions developed in www.freepsychotherapybooks.org 371 childhood as “primitive defensive measures adopted by the immature, feeble ego.” In later years no fresh repressions occurred, but the old ones persisted and were employed by the ego to master instinct.

A retention of a relationship to reality at the expense of an intrapsychic balance produced a psychoneurotic disturbance.

The existing conflict here was between the ego and the id.

If an intrapsychic balance developed at the expense of reality relationships, the consequence was psychosis.

The latter resulted when the ego was overwhelmed by id forces, the conflict being between the ego and the environment.

Freudian psychoanalytic therapy is based on the libido theory described above.

It rests on the hypothesis that neurotic illness is nurtured by the repression of vital aspects of the self and its experiences— particularly oral, anal, and sexual (including Oedipal)

experiences in relation to important parental agencies.

This repression is sponsored by fear of the loss of love or of punishment from the parents, which has been internalized in the superego.

Repressed feelings, attitudes, and fears, and the early experiences associated with them, continue to strive for conscious recognition, but they are kept from awareness by dread of repetition of parental loss of love or punishment now invested in the superego.

Their removal from the mainstream of consciousness makes it impossible for the individual to come to grips with basic conflicts.

They remain in their pristine state, uncorrected by reality and by later experiences.

The energy required to maintain repression, as well as to sustain other defenses against anxiety, robs the individual of energy that could be used to nurture psychosexual development. In his last clinical paper, "Analysis Terminable and Interminable," (1952), Freud emphasized that the essence of the analytic situation was the entry by the analyst into an alliance with the ego of the patient in order to subdue certain uncontrolled parts of the id and ultimately to include them in the synthesis of the ego.

Reviewing this, Freud contended that the ego acted as an intermediary between the id and the external world "to protect the id from the dangers of the external world." In the process the ego tended to "adopt a defensive attitude towards its own id and to treat the instinctual demands of the www.freepsychotherapybooks.org 372 latter like external dangers...." The child's ego thus learned "to shift the scene of the battle from outside to inside and to master the inner danger before it becomes external" by elaborating defensive mechanisms, such as repression, to avoid danger, anxiety, and unpleasure.

Moreover, a defensive falsification of inner perceptions rendered "an imperfect and travestied picture of the id." Unfortunately, defensive mechanisms, while averting danger, could themselves become dangerous.

They consumed enormous expenditures of energy, and they imposed ego restrictions that burdened the psychic economy.

The services rendered by the defenses thus entailed too high a price.

Mechanisms of defense were varied, and each individual selected those that satisfied his or her needs.

They became fixated in the ego and reappeared as characteristic reactions under situations similar to those that originally invoked them.

Even though they had outlived their usefulness, they persistently survived "like outmoded institutions which contaminate a modern society." The adult ego "continues to defend itself against dangers which no longer exist in reality and even finds itself impelled to seek out real situations which may serve as a substitute for the original danger, so as to be able to justify its clinging to its habitual modes of reaction." Thus the defensive mechanisms distorted perceptions of reality and undermined the ego to a point where neurosis finally broke out.

During analysis the individual exhibited customary defense mechanisms, the dealing with which constituted half of the analytic task.

The other half "is the revelation of what is hidden in the id.

Our therapeutic work swings to and fro during the treatment like a pendulum, analyzing now a fragment of the id and now a fragment of the ego.

The mechanisms of defense recur in analysis in the shape of resistances to cure.

It follows that the ego treats recovery itself as a new danger." The resistances, unconscious in nature, constantly blocked the therapeutic task of making conscious what was repressed within the id.

The effect of unpleasurable impulses in the patient "when his defensive conflicts are once more

roused may be that negative transferences gain the upper hand and break up the whole analytic situation." The patient regarded the analyst as an alien personality in whom he had no confidence.

www.freepsychotherapybooks.org 373 Some of the resistances were peculiarities of the ego determined by heredity.

Some were acquired in defensive conflicts.

Some, such as the ease or stubbornness with which libidinal cathexes were released from one object and displaced to another, were due to "changes in some rhythm in the development of psychical life which we have not yet apprehended." Some of the resistances that prevented a release from illness were manifestations of "the behavior of the two primal instincts, Eros and the death instinct, their distribution, fusion and defusion," which permeated all provinces of the mental apparatus—id, ego, and superego.

The sense of guilt and need for punishment, a product of the ego's relation to the superego, which accounted for the phenomena of masochism and negative therapeutic reaction, was only one expression of this resistance.

Freud considered that the etiology of all neurosis "was a mixed one; either the patient's instincts are excessively strong and refuse to submit to the taming influence of his ego or else he is suffering from the effects of premature traumas, by which I mean traumas which his immature ego was unable to surmount." A relationship existed between these two factors; i.e., the stronger the constitutional strength of instincts, the more easily could a trauma lead to fixation and a disturbance in development.

If the instinctual life was normal, severe trauma alone might suffice to create neurosis.

Analysis had a better chance of strengthening the ego where the traumatic factor predominated.

Prejudicial to analysis and fostering its interminability was "a constitutional strength of instinct and an unfavourable modification of the ego in the defensive conflict." Also a reinforcement of instinctual energy at some later period in life made demands on the ego it could not tolerate.

It was then difficult to achieve the goal of analysis, the taming of instincts and bringing them into harmony with the ego.

The two themes that gave the analyst the greatest difficulty were penis envy in women and in men the struggle against their passive or feminine attitude toward other men.

In women, normally, large portions of the striving for masculinity underwent a transformation. The unsatisfied wish for a penis became converted into a wish for a child and for a man who possessed a penis.

Often, however, the masculine wish persisted, repressed in the unconscious, exercising itself disturbingly.

In men the passive attitude www.freepsychotherapybooks.org 374 often continued to signify castration.

Where these drives had not been satisfactorily resolved, they could persist doggedly throughout analysis, frustrating the analyst's most dedicated attempts at resolution.

Many resistances arose out of a female patient's refusal to give up the wish for a penis and the masculine protest.

When these drives were elucidated in analysis, "we have penetrated all the psychological strata and reached 'bedrock.'" Then the analytic task was accomplished, and analysts could console themselves "that everything possible has been done to encourage the patient to examine and to change his attitude to the question." All people were bisexual since "their libido is distributed between objects of both sexes, either in a manifest or a latent form." Why conflict developed with a restriction of the love object.

Freud attributed to the "intervention of an element of aggressiveness...a manifestation of the

destructive or aggressive instinct.” Quite modestly, Freud admitted the reason for “the variable results of analytic therapy might be that our success in replacing insecure repressions by reliable and egosyntonic controls is not always complete, i.e., is not radical enough.

A change does occur, but it is often only partial: parts of the old mechanisms remain untouched by analysis.” While “analysis is always right in theory in its claim to cure neurosis by ensuring control over instinct ...

in practice its claim is not always justified.” This is because the instincts continue to be so strong that the ego fails in its tasks, “for the power of analysis is not infinite; it is limited, and the final result always depends on the relative strength of the conflicting psychological agencies.” Adequate criteria for the termination of psychoanalysis were, first, symptom relief with overcoming of various anxieties and inhibitions, and, second, the opinion on the part of the analyst that so much repressed material had been made conscious, with sufficient evidence that that which was inexplicable is now elucidated, and enough inner resistance eliminated, that we need fear no repetition of the patient’s specific pathological processes.

The latter criterion for the “end” of analysis was ambitious in its scope.

“According to it we have to answer the question whether the effect upon the patient has been so profound www.freepsychotherapybooks.org 375 that no further change would take place in him if his analysis were continued.” This, Freud added, implied that through analysis one could achieve absolute psychological normality, with a lifting of all the patient’s repressions and a filling of every gap in the memory.

“If the patient who has made such a good recovery never produces any more symptoms calling for analysis, it still, of course, remains an open question how much of this immunity is due to a benevolent fate which spares him too searching a test.” In analytic practice treatment had to be carried out “in a state of abstinence” to bring the “conflict to a head” by increasing the “instinctual energy available for its solution.” Yet, “a prophylactic treatment of instinctual conflicts” was not to be fostered by subjecting the patient to cruel experiments, since “in conditions of acute crisis it is, to all intents and purposes, impossible to use analysis.”

Fresh conflicts “will only make the analysis longer and more difficult.” During analysis the strengthening of the ego permits a review of old repressions “with the result that some are lifted, while others are accepted but reconstructed from more solid material ... they will not so easily give way before the floodtide of instinct.” Sometimes analysis counteracted an increase in the instinctual strength; sometimes it raised the resistances so they could take a greater instinctual strain.

The Technique of Classical Psychoanalysis It is generally accepted that classical psychoanalysis is useful only in persons who have reached a relatively mature state of personality development.

They must have experienced the traditional triangular (Oedipal) conflict that in resolution has enabled them to sustain a reasonably meaningful relationship with both their parents.

If the preoedipal relationship with the mother was too severely ambivalent, the regression during psychoanalysis will provoke anxiety and depression so great as to interfere with the management of the transference neurosis.

Not all persons are capable of establishing an initial therapeutic alliance, which is the preliminary requisite for psychoanalysis.

Defensive mistrust and frustration at the lack of immediate gratification of needs and demands may block progress.

The therapist’s www.freepsychotherapybooks.org 376 seemingly cruel and unfeeling behavior may be looked upon as a sign of a harsh perversity, and the patient may be unwilling to give up his

or her detachment and need for control.

The analytic situation may therefore never get off the ground.

Those patients who are capable of establishing a therapeutic alliance have an opportunity to be helped through the next phase of regression, in which infantile conflicts are revived and a transference neurosis develops that enables the patient to work through and resolve the conflicts.

The techniques of free association, lying on the couch, passivity on the part of the analyst, are purposely designed to encourage passive dependence and regression.

The inevitable resistance of the patient is managed by timed interpretations.

Unresolved conflicts emerge in the transference, and their understanding is helped by proper interpretation.

Slowly the original infantile conflict emerges and is definitively resolved.

The termination phase involves an ability to separate from therapy and the analyst and to assume independence and autonomy.

Freud's formulations are more or less accepted as the basis for present-day classical psychoanalysis.

It is believed essential that the patient recognize the derivatives of the repressed since these represent, in an attenuated form, the warded-off material.

To minimize the distortion of these derivatives, the obtrusion of current situations and other reality influences must be kept at a minimum.

The core of Freudian psychoanalysis lies in what is perhaps Freud's most vital discovery—that of transference.

As has previously been indicated, Freud found that patients, if not interfered with, inevitably projected into the therapeutic situation feelings and attitudes that were parcels of their past.

Sometimes transference manifestations became so intense that they actually reproduced and reenacted with the therapist important conflictual situations and traumatic experiences (transference neurosis) that had been subject to infantile amnesia.

By recovering and recognizing these repressed experiences and conflictual situations that had never been resolved and by living through them with a new, less neurotic, and non-punitive parental agency, the superego was believed to undergo modification.

The individual became tolerant of his or her id and more capable of altering ego defenses that had crippled adaptation.

There occurred, www.freepsychotherapybooks.org 377 finally, a mastery of early conflicts and a liberation of fixated libido that could then enter into the development of a mature personality.

Since the Oedipus complex is considered to be the nucleus of every neurosis, its analysis and resolution in transference constitutes a primary focus.

Where the Oedipus complex is not revealed, where its pathologic manifestations are not thoroughly analyzed and worked through, and where forgotten memories of early childhood experiences are not restored, treatment is considered incomplete.

Because Freudian psychoanalysis is transference analysis, all means of facilitating transference are employed.

These include the assumption by the therapist of an extremely passive role, the verbalization by the patient of a special kind of communication—"free association"—the analysis of dream material, the maintenance of an intense contact with the patient during no less than four or five visits weekly, and the employment of the recumbent couch position.

Passivity on the part of the therapist is judiciously maintained even through long periods of silence.

The therapist also refrains from reacting emotionally or responding positively or negatively to any verbalized or nonverbalized attitude or feeling expressed by the patient.

Strict anonymity is observed, and no personal information is supplied to the patient irrespective of how importunate he or she may become.

A nonjudgmental, non-punitive, non-condoning attitude by the therapist is adhered to, dogmatic utterances of any kind being forbidden.

The chief "rule" the patient is asked to obey is the "basic rule" or "fundamental rule" of verbalizing whatever comes to mind, however fleeting, repulsive, or seemingly inconsequential it may seem (free association).

This undirected kind of thinking is a most important means of tapping the unconscious and of reviving unconscious conflicts and the memories that are related to their origin.

Most important, free association, like passivity, enhances the evolution of transference.

As long as the patient continues to associate freely, the therapist keeps silent, even though much time may pass without a comment.

The therapist fights off all temptations toward "small talk" or impulses to expound on www.freepsychotherapybooks.org 378 theory.

The therapist interferes only when resistances to free association develop and until the patient proceeds with undirected verbalizations.

Dream analysis is used constantly as another means of penetrating the unconscious.

By activating repressed material and working on defenses as they are revealed in dream structure, the therapist aids the development of transference.

The frequency of visits in Freudian psychoanalysis is important, to encourage transference, no fewer than four or five visits weekly are required.

Fewer visits than this encourage "acting-out" and other resistances to transference.

The use of the recumbent couch position enables the patient to concentrate on the task of free association with as few encumbrances of reality as possible.

It helps the therapist, also, to focus on the unconscious content underlying the patient's verbalizations without having to adjust to the demands such as would exist in a face-to-face position.

Concentrating on the patient's inner life, rather than on external reality, helps to bring on the phenomenon of transference.

During the early stages of analysis the main task is to observe—from free associations and dreams—manifestations of conflicts and the types of defenses employed, which form a kind of blueprint of the unconscious problems of the patient.

This blueprint is used later at the stage of transference.

Since repression is threatened by the operation of exploring the unconscious, anxiety is apt to appear, stimulating defensive mechanisms.

These function as resistances to productivity, and even to verbalization.

Free association may consequently cease, and the patient may exhibit other manifestations that oppose cooperation with the treatment endeavor.

Such resistances are dealt with by interpretation.

Through interpretation the patient is brought to an awareness of how and why he or she is resisting and the conflicts that make resistance necessary.

Sooner or later the patient will "transfer" past attitudes and feelings into the present relationship with the analyst.

Observance of the "basic rule," the attack on resistances through interpretation, and the consideration of unconscious material in dreams and free www.freepsychotherapybooks.org 379 associations remove habitual protective devices and facades that permit the patient to maintain a conventional relationship.

The patient is most apt to express strivings toward the therapist rooted in past experiences, perhaps even reproducing the past in the present.

Thus, a revival of pathogenic past conflicts is instituted.

Unlike supportive and reeducative therapy, in which transference may be used as a therapeutic vehicle, the transference is interpreted to the patient in order to expose its nature.

This is the chief means of resolving resistance, of bringing the individual to an awareness of the warded-off content, and of realizing the historical origin of conflicts.

The development of transference may occur insidiously and manifest itself indirectly, or it may suddenly break out in stark form.

It often shows itself in changes in the content of free associations, from inner feelings and past relationships with parents to more innocuous topics, like current events and situations.

This shift is evidence of resistance to deeper material activated by the erupting transference feelings.

Sometimes free association may cease entirely, with long stubborn silences prevailing that are engendered by an inability to talk about feelings in relation to the therapist.

The purpose of superficial talk or silence is to keep from awareness repressed emotions and forgotten memories associated with early childhood, particularly the Oedipus complex.

Until these can be brought out into the open, the emotions relating to them discharged, and the associated memories reclaimed, the conflictual base of neurosis will survive.

The transference neurosis offers an opportunity for this revivification since, in the relationship with the therapist, the patient will expose loves, fears, and hates that were characteristic of his or her own experiences during the Oedipal period.

Transference, however, acts as a source of powerful resistances that impede therapeutic progress.

Once the patient is in the grip of such resistances, he or she is usually determined to cling to them at the expense of any other motivation, including that of getting well.

On the positive side, transference is important diagnostically since it reveals a most accurate picture of the patient's inner conflicts.

Additionally, it induces a coming to grips with and a working through in a much more favorable setting of those unresolved conflicts that have blocked maturation.

The resolution of transference is felt www.freepsychotherapybooks.org 380 by Freudian psychoanalysts to be the most powerful vehicle known today for producing structural alterations in the personality.

Active interpretations of the transference are essential to its resolution.

These include the interpretation of its manifestations, its source, and its original and present purposes.

The working through of transference is accompanied by a recollection of forgotten infantile and childhood experiences—a recounting of distortions in relationships with parents or parental surrogates.

The accuracy of interpretations will usually be denied at first as part of the resistance manifestation.

Acknowledgment of the unreal nature of transference is usually opposed by the patient because this either constitutes too great a threat or because the patient does not want to relinquish transference gratifications that are deemed essential to life itself.

As long as he or she continues to deny transference, the analysis will remain interminable, unless forcefully terminated by either participant.

With persistence on the part of the therapist, interpretations usually take hold and the patient is rewarded with greater insight, an increased sense of mastery, liberation from neurotic symptoms, and genuine growth in maturity.

The therapist must also constantly guard against manifestations of destructive countertransference, which, disguised and varied, are mobilized by unresolved problems and pressing needs within the therapist.

Common forms of countertransference are subtle sadistic attacks on the patient, impulses to be pompous and omnipotent, or desires to reject the patient or to detach oneself from the relationship.

Because of countertransference, a personal analysis is considered essential for analysts so that they can deal with their own unconscious tendencies and resistances precipitated by contact with their patients.

As the ego of the patient is strengthened by an alliance with the therapist, it becomes more and more capable of tolerating less and less distorted derivatives of unconscious conflict.

The continued interpretation by the therapist of the patient's unconscious feelings and attitudes, as well as the defensive devices that the patient employs against

www.freepsychotherapybooks.org 381 them, enables the patient to work through problems by seeing how they condition every aspect of his or her life.

In the medium of the therapeutic relationship the individual is helped to come to grips with early fears and misconceptions, resolving these by living them through in the transference.

The patient is finally able to resolve libidinal fixations and to liberate energy that should originally have gone into the formation of a mature sexual organization (Freud, 1920, 1924, 1930b, 1933, 1936, 1937, 1938a, b; Jones E, 1924; Glover E, 1927; Strachey, 1934; Balint, 1936; Bibring-Lehner, 1936; Glover et al, 1937; LaForgue, 1937; Schmideberg, 1938; Zilboorg, 1939; Sterba, 1940; Fenichel, 1941; Lorand, 1946; Berg, C, 1948; Nunberg, 1948; Kubie, 1950b).

A number of additions and modifications of Freudian theory have been introduced, especially as they relate to sicker patients.

Most notable is the focus on object relations during the pregenital period and residues of existing distortions in adult life.

Interferences with instinctual development brought about by difficulties with the parenting objects have been especially explicated by Anna Freud and Margeret Mahler.

According to Mahler (1971), children who fail to resolve the rapprochement aspect of the separation-individuation process may incorporate "bad" introjects laden with derivatives of aggressive drive that encourage a splitting of the object world into "good" and "bad" objects. This splitting is characteristic of the transference experience in most borderline cases.

Mahler (1958) contends that during the first three months of the life of the infant, in the "presymbiotic" or "normal-autistic" phase, the mother acts as an "external executive ego." At the end of this stage the infant enters the "phase of symbiosis," during which the mother is dimly perceived as a "need-satisfying-quasi-extension-of-the-self." The infant's and mother's body-ego boundaries seem fused.

Mother and infant are "an omnipotent symbiotic dual unity" (Mahler et al, 1959).

Toward the end of the first year, coincident with locomotion and the beginning of language, the process of separation-individuation takes place; at 18 months ego boundaries begin to crystallize; and during the second year reality testing consolidates toward a "clear distinction between the self and the object world." A disturbed mother-child relationship during these early phases disrupts the normal progression toward separation-individuation and reality testing.

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Jacobson (1967) has further expanded on the dynamics of the psychotic process.

She states that whereas both the neurotic and psychotic possess pre-Oedipal narcissistic fixations and are subject to pregenital and ambivalence conflicts, the relative stability of these defenses in the neurotic (particularly the repressive capacity and the intactness of

boundaries between self and object representations) help prevent not only drive defusion and drive deneutralization but also flooding of the ego with sexual, destructive, and selfdestructive forces.

These forces in the poorly defended psychotic lead to a regressive dissolution of the psychic structures.

In the latter case, an increase of free aggression weakens the ego's ability to resist the assault of the instinctual forces, particularly outward destructive and self-destructive impulses.

The basic intrapsychic conflict in the psychotic relates to the "struggle between active and passive, sadistic and masochistic, destructive and self-destructive tendencies, and in general between sexual and aggressive impulses," which may at varying times be employed as defenses against each other.

Many psychoanalysts today have attempted to incorporate in their thinking and practices some of the new advances in the behavioral sciences.

A survey conducted by Hofling and Meyers (1972) of the opinions of 90 American psychoanalysts of the most important information and technical advances made in their field since Freud's death indicated that, although there was some doubt as to the value and originality of most of the work, substantive contributions had been made by Hartmann (on ego autonomy), Erikson (on identity and psychosocial development), Mahler (on separationindividuation), Kohut (on narcissism), Anna Freud (on early infancy and lines of development), and Jacobson (on object-relations theory).

Kernberg (1973) has written extensively about problems of the borderline patient.

Disagreement with certain psychoanalytic concepts is common among contemporary psychoanalysts. Even those analysts who consider themselves to be "orthodox" Freudians are not in complete accord with Freud in theory and method.

Many analysts challenge the death instinct hypothesis, for instance.

Insofar as technique is concerned, practically every analyst implements psychoanalytic methods in his or her own specific way.

Many years ago an extensive questionnaire distributed by Glover (1940)

www.freepsychotherapybooks.org 383 to a representative group of practicing psychoanalysts demonstrated that deviations from orthodox techniques were extensive even then.

There were differences in the form, timing, and amount and depth of interpretation.

The degree of adherence to free association varied, as did the assumption of passivity and anonymity, the use of reassurance, and the management of transference.

Variation in methods of doing psychoanalysis was indicated by the fact that out of 82 questions, there was general agreement on only 6, and even here there was not complete conformity.

Glover (1964) has emphasized repeatedly that established principles of psychoanalysis are difficult to delineate even for avowed classical analysts.

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Mitchell's (1927) contention that a psychoanalyst can be defined as one who accepts the ideas of the unconscious, infantile sexuality, repression, conflict, and transference would not be considered adequate for a British Kleinian, who would insist on explorations and analysis in depth through the 3-to6-month-old "depressive position" and the later infantile "paranoid-schizoid" phases.

Drawing inspiration from Levi-Strauss's structuralism, Lacan (1953, 1964, 1977, 1978; Leavy, 1977; Miel, 1966; Wilden, 1973; Muller, 1985) attempted a mating of linguistics with psychoanalysis.

The amalgam resulted in writings notable for their obscurity of meaning.

Nonetheless, Lacan's dedication and energy resulted in the development of a popular school of analysis.

According to Lacan, psychopathology is rooted in developmental defects, particularly around the "mirror stage," which occurs between the ages of 8 to 18 months and consists of visual perceptions of one's body form as seen in a mirror at an early stage in life (Lacan, 1949).

Disturbances in this perception may result in a false recognition of the self and the assumption that there is another being shadowing the self (the "other"), which is principally responsible for self-alienation.

An ego structure develops that is defensive and maladaptive, and the self becomes increasingly identified with the "other." Therapy consists, through language, of clarifying this distortion and liberating the true self from the "other," which manifests itself in many ways, including identification with the analyst.

Lacan resigned from the Psychoanalytic Society of Paris to found his own school and popular training institute, the "Freudian Cause." His methods, which include shortening www.freepsychotherapybooks.org 384 the traditional 50-minute session, and his writings, which are difficult to understand, have made him a controversial figure.

Some consider him a genius; others, such as members of the International Psychoanalytic Society, which expelled him for "deviant practices," believe that he is grossly incompetent.

Except for using the couch position in the treatment of a majority of his patients, Lacan deviates from practically all other accepted Freudian methodological principles, although he pays homage to the greatness of the master and calls himself a true Freudian.

The shortening of the session is perhaps the key issue around which clinicians organize their criticism.

Lacan provides a rationale for the abbreviated therapeutic session (which is sometimes as short as 3 minutes) by avowing that dismissing a patient before the end of the hour is an effective way of dealing with verbal resistances and that such dismissal actually solidifies the therapeutic relationship.

It is difficult to say how much of Lacan's success was due to his charisma, or to disagreement of his followers with classical psychoanalytic theory and method, or to the validity of his developmental theory of the "mirror stage." Because Lacan's explanations are too obscure for one to be certain of their true meaning, his concepts have been subject to many interpretations.

Many of his statements are revolutionary, such as that good therapy should strengthen not overcome the ego, since it is a distorting organization.

It will be interesting to observe how many of his theories and practices survive Lacan's death.

Some analysts still practice classical psychoanalytic technique, but, by and large, many, perhaps most, analysts employ a modified psychoanalytic technique that differs from the orthodox form.

Questionnaires sent out by the Research Committee on Psychoanalytic Practice of the American Academy of Psychoanalysis (Tabachnick, 1973) to a sizable number of therapists indicated that only 6 percent of the patients being treated by the respondees were seen four times a week and a mere 1 percent were seen five times a week.

The great majority of the patients were treated in the sitting-up rather than the recumbent position.

Group, family, and marital therapy were occasionally used, and 94 percent of those polled sometimes prescribed drugs.

We might speculate from these findings that either the majority of patients were not suited for intensive psychoanalytic work, or that the analysts in the survey believed that psychoanalysis

could be done with www.freepsychotherapybooks.org 385 less than the classical four to five sessions per week, or that most patients could not afford to pay for more frequent visits, or that the members of the American Academy of Psychoanalysis were less rigorous than their colleagues in the more orthodox American Psychoanalytic Association.

If this survey truly reflects what is going on in contemporary psychoanalysis, most therapists who call themselves psychoanalysts are not using the classical technique for most of their patients.

One would conclude that a sizable group of psychoanalysts of various schools are providing a psychoanalytically oriented treatment rather than the classical type, irrespective of what the analysts themselves say they are doing.

Whether we can apply the term “psychoanalysis” to these modifications is a matter of opinion.

Criticism of Freudian Psychoanalysis Perhaps no branch of psychotherapy has created so great a furor among professionals and laity, or is as responsible for so great a polemic, as Freudian psychoanalysis.

“Superlative,” “finest,” “unique,” “peerless,” “incomparable,” “insipid,” “senseless,” “absurd,” “inconsistent,” “prejudiced”—in these words, from the mouths and pens of its devotees and critics, psychoanalysis is qualified in both transcendent and opprobrious terms.

While the brilliance of Freud’s clinical observations is acknowledged by even his sternest critics, the theoretical cement that binds his concepts together is considered by many to be fragile (Bieber, 1958).

Such concepts are considered by some to be rooted in nineteenth-century physics and biology.

Objection is voiced to the thesis (1) that a human being, frustrated by the sexual impasse imposed by inimical events in childhood, spends the rest of his or her life fruitlessly trying to repair this damage, (2) that, blocked in this quest by a false friend, the mechanism of repression, the individual aimlessly wanders through the circuitous by-paths of life, seeking vicarious and infantile satisfactions, and (3) that, while constantly casting in the present the shadows of the past, the individual defends himself or herself with capricious maneuvers, relentlessly marching on to inevitable psychological doom.

A person’s destiny, it is claimed, is not so dismal.

Neo-Freudians and existential analysts particularly, stress one’s “responsibility” for one’s own fate and future.

They have also accused Freud of minimizing the social www.freepsychotherapybooks.org 386 determinants of psychic phenomena.

A scientific approach to the study of personality must consider that the basic biological forces of humanity are shaped and fashioned through the social environment.

Freudians counter with the explanation that although Freud did not elaborate too fully on them, he did not neglect cultural influences while stressing instinctual processes.

Fenichel (1945, p.

6), for example, has pointed out that Freud was fully aware of how instinctual attitudes, objects, and aims were constantly being altered as a result of social experience.

A search through Freud’s voluminous writings easily reveals passages that can sustain almost any variant version of psychodynamics.

As is so often done with Shakespeare and the Bible, the practice is common of plucking of statements out of context and exploiting them for prejudiced purposes.

One may in various volumes find contradictory passages among Freud’s contributions, in the form of contrasting sentiments and revisions.

Such amendments are in the highest traditions of science, which demand alteration of old conclusions as new hypotheses are advanced.

In the richness of his contributions Freud developed a variety of theoretical configurations.

In some quarters, however, obsolescent models have been retained even as more functional ones evolved.

This superimposition of model upon model makes for a certain grotesqueness of concepts. In the main, Freud's dynamic theoretical premises have been accepted more enthusiastically than either his topological constructs of the mental apparatus or his economic theories, which relate intrapsychic and interpersonal transactions to energy distributions issuing from libidinal and aggressive instincts.

Dealing with so many disparate aspects, psychoanalysis has suffered by the tendency to judge its whole by the weakness of some of its parts.

For instance, its theoretical inconsistencies have led to a minimization of its therapeutic potential.

Conversely, the fact that it fails as a treatment procedure in certain problems has tended to devalue psychoanalysis as reflecting a reasonable theory of behavior.

Its limitations as a model for the interpretation of anthropologic, historical, educational, religious, and literary data have invited the repudiation of psychoanalysis as a whole.

www.freepsychotherapybooks.org 387 Some of Freud's ideas have gained a wide acceptance and continue to ring as true today as they did when he originally formulated them.

Others are outmoded, and like any other human concepts have undergone revision and reformulation.

Freud framed many of his notions in a tentative and speculative mood, and he preferred them with the cautious warning that they would undoubtedly later be altered or discarded.

"Psycho-analysis," he wrote, "has never set itself up as a panacea and has never claimed to perform miracles" (Freud, S, 1959).

One of the problems in the evolution of a school is the resistance of the founder to countenance changes in his formulations by any person other than himself.

Freud was no exception.

While he took great liberties with his own postulates, transforming and discarding many of his pronouncements, he was not so generous with innovations introduced by his contemporaries.

The abandonment of anonymity and passivity by Ferenczi with assumption of a "caring" and loving attitude toward his patients, the active involvement in therapeutic maneuvers by Stekel, the concern with mystical philosophy by Jung, the drift away from infantile sexuality toward inferiority feelings by Adler, the elaboration of the birth trauma and separation anxiety by Rank all brought forth the sternest criticism from Freud.

This tendency of denying validity to approaches that deviate from formal doctrine has continued in psychoanalysis to the present day.

It sponsors the commonly voiced criticism of Freudian method that analysts insist upon wedging their patients into a preconceived theoretic structure.

When the patient does not produce appropriate material that substantiates accepted notions of dynamics or refuses to accept interpretations, he or she is credited with being in an obstinate state of resistance.

A sentiment expressed by non-Freudians is that, in their eagerness to smuggle "deep" insights into patients, certain "orthodox" analysts make dogmatic interpretations that patients feel obliged to accept.

This practice may mobilize intense anxiety that can disorganize patients with weak ego structures.

Another criticism is that many classical analysts are intolerant toward those who practice therapies other than Freudian psychoanalysis, considering these to be superficial and of little real value.

Accordingly, they are inclined to depreciate treatment by non-analysts as well as by analysts

of nonwww.freepsychotherapybooks.org 388 Freudian orientation.

What is interpreted as a lack of flexibility, however, is actually a staunch defense of a theoretical position that "psychoanalysis" is a specific mode of treatment in which the focus is on the unconscious, revealed through the transference, uncovered by resolution of resistance, and centered on the bringing to the surface and working through of buried childhood conflicts.

Obstinacy in retaining this position is derived not from cantankerousness but from a passionate conviction that this is the most effective way of achieving extensive restructuring of the psychic apparatus.

Opinions vary about the effectiveness of classical psychoanalysis because countless characteristics of patients and therapists influence outcome.

It is often avowed that current pessimistic impressions of the potential of psychoanalysis as a therapy result from the inclusion of reports of outcome made by inexperienced practitioners (Gedo, 1979).

Are there any reliable reports of experienced analysts?

Summarizing his own work of two decades as an experienced full-time analyst, Gedo is convinced of the effectiveness of the method.

His data are drawn from a total caseload of 36 people treated over 20 years of practice, 28 of whom terminated analysis with a "consensus about the satisfactory outcome of the enterprise." The patients were primarily in the higher socioeconomic class.

The technique was classical psychoanalysis four or five times weekly, for 600 to 1000 sessions in three to seven years.

There was no systematic follow-up study, but the author estimates that failure to contact him "argues for the probability that those who have not been heard from are not simply withholding unfavorable tidings." The author cryptically concludes with the statement that an analyst should not approach clinical work with a personal need to be a healer since "to require patients to improve is an illegitimate infringement on their autonomy." In reviewing the presented statistics of 28 out of 36 patients improved over a period of 20 years of practice, one would estimate that, irrespective of whether the results justify the conclusion of a "matchless usefulness of the analytic method as a means for personal growth," the classical technique, for economic reasons, is definitely not designed for the www.freepsychotherapybooks.org 389 great majority of patients seeking help for emotional problems.

This does not in any way invalidate the incorporation of analytic principles in a lesser than long-term program.

My own impression is that, given a carefully screened patient and a well-trained analyst, the results with classical analysis can justify the expense and effort.

On the other hand, the great majority of patients can be effectively helped by less intensive and costly methods.

Where poor results have been obtained with classical psychoanalysis, the chances are that it was employed with patients who were unsuited for the technique or that the therapist was by training or personality not capable of working with the method.

A discreet appraisal of its therapeutic merit indicates that classical psychoanalysis constitutes a serviceable, though by no means exclusive, insight-oriented approach to problems of an emotional nature.

In some cases, and in the hands of qualified and experienced psychoanalysts, it may be the most consummate of all treatments.

In other cases it may fail miserably to benefit patients in the least.

It is useful in individuals who are not too sick emotionally, whose conflicts are severely and

obstinately repressed, who are capable of understanding abstract concepts, who have the time and money to invest in long-term therapy, and who are willing to expose themselves to the rigors of the method.

This limits the number of patients to a small percentage of those who require psychotherapy. Accordingly, therapists trained in the classical method often adopt techniques from other approaches (somatic, group, family, active interviewing, etc.) when they are obliged to treat patients who are not suited for the formal analytic procedure.

Its qualification in transcendent terms is not only undeserved, but contrary to the spirit of its founder, who cautiously wrote in one of his last depositions: "The future will probably attribute far greater importance to psychoanalysis as the science of the unconscious than as a therapeutic procedure" (Freud, 1926).

The homage paid Freud is, nevertheless, truly deserving.

Despite his intolerance of nonconformity, his was a truly great mind, reflected in his pioneer writings by his sparkling prose, his brilliant wit, and his unique and penetrating insights into human behavior.

Humanity owes this extraordinary man an enormous debt for opening up new www.freepsychotherapybooks.org 390 psychological vistas and for bringing to science of the mind the same dignity and grandeur that Darwin brought to biology and Harvey to physiology.

The concept of the unconscious, the dynamic nature of repression, the importance of psychic determinism, the goal-directed nature of behavior, the relation of psychosocial development to personality evolution, the consociation of abnormal mental symptoms and normal mental processes, the meaning of anxiety, the role of symbolism, the significance of dreams, and the phenomena of transference and resistance—these have proved themselves to be of revolutionary importance and have provided us with tools for the investigation of intrapsychic and interpersonal activities.

One may easily understand the esteem in which his contemporaries and students have held Freud and why those who have known him personally and from his writings talk about him with such love and reverence.

While many of Freud's findings have been challenged and some altered, his discoveries have paved the way to a better understanding of human beings' psychological nature.

They have stimulated independent and productive research for the benefit of all humanity.

Freudian psychoanalysis is practiced extensively throughout most of the world, especially in the United States and England.

The parent organization is the International Psychoanalytic Association, which publishes the International Journal for Psychoanalysis.

The American Psychoanalytic Association, which sponsors the American Journal of Psychoanalysis, is composed of a number of local societies throughout the country.

KLEINIAN PSYCHOANALYSIS By analyzing the free-associational play of young children in the 1920s, Melanie Klein developed a biological theory that has gained a great deal of prominence among English psychoanalysts.

She has a large following in South America, Spain, and Switzerland.

Some authorities believe that an understanding of Kleinian principles is basic to a dynamic conception of child psychology as well as adult psychopathology, particularly the phenomena of schizophrenia and manic-depressive psychosis.

Freud contended that the superego of children was evolved by taking over (introjecting) the parental attitude at the time of the resolution of the Oedipus complex,

www.freepsychotherapybooks.org 391 around the age of 4 years.

According to Melanie Klein's theory (1932, 1946, 1948, 1952, 1955, 1957, 1960, 1961, 1963), the Oedipus complex emerges during the first year of life.

The superego, too, is organized soon after birth.

In the initial contacts with reality, the child fuses emotional feelings in relation to objects (mother, breast, nipple, etc.) with the objects themselves.

Thus objects that inspire pleasure are “good objects”; those that provoke pain are “bad objects.” The first object within the infant’s perceptual range is the mother’s breast (“breast” is used as an inclusive term—for breast, nipple of a bottle, mother’s body, and her closeness).

Since hunger is an unbearable tension state for the infant, the breast and everything associated with it become a chief means of allaying the child’s discomfort.

But the breast is not always forthcoming on demand.

The infant does not have the knowledge “that loss, frustration, pain and discomfort are usually temporary and will be followed by relief.” The slightest change in the tension-alleviating state, such as a less easy grasp of the nipple or diminution in the flow of milk, will suffice to change the pleasant stimulus into an unpleasant, dissatisfying one.

The child will then love and hate the breast and mother simultaneously on the basis of an all-or-nothing principle.

The world for the child is peopled with good objects that satisfy and bring relief and with bad objects that frustrate and provoke suffering.

Among the child’s earliest concepts, incorporation (taking in milk) and expulsion (giving out feces) are most prominent; these processes come to play a vital role in ideas of oneself and the world.

A rudimentary sense of the world and objects (breast of mother, bottle and nipple, mother’s body, his or her own feces and urine, genital organs, the closeness and absence of mother) surrounds the child, along with dim phylogenetically determined images of the penis, vagina, coitus, and childbirth.

Reacting to the Oedipus complex, the infant fears destruction by the Oedipal rival.

The infant conceives of coitus as an act of biting, with oral incorporation of the penis by the mother.

Penis and vagina are regarded as dangerous weapons; the penis equated with breast, the vagina with a biting mouth.

The infant desires to take in (incorporate, introject) “good objects,” such as the pleasurable and satisfying breast, and in doing so conceives of himself or herself as good and whole.

The assault on the breast (around the sixth month passive sucking is replaced in part by active biting) enables the infant to react to anxiety with attack.

This oral sadism www.freepsychotherapybooks.org 392 is vital to personality development, and rage issuing from frustration serves to strengthen the sadistic instincts.

Operation of the death instinct inspires active aggression against the self; in self-protection there is a projection outward against the breast, or “persecutors.” Introjection of the breast is also a means of controlling and destroying “bad objects.” This leads to guilt and to efforts to deal with aggression by projection or destruction.

The infant’s notion of aggression is at first centered around the oral function.

Fantasies of sucking and biting tend to be externalized, as are the hatred and “envy” of the mother.

These return as fear of the mother destroying, tearing, and eviscerating the child.

A persecutory “paranoid-schizoid position” prevails when the bad objects, the product of the child’s own aggression, in being projected, come back to torment the child (“projective identification”).

The “paranoid position” that occupies the first 4 months of life makes for the first active relationship with the world.

A primitive form of the adult paranoid persecutory delusion, it is usually outgrown, though remnants remain in the psyche to be affiliated with a later developing sense of guilt. Such conceptual formulations are concocted during a stage in the child's mental life when ideas are diffuse and undifferentiated.

The incorporated desires toward the part object "good breast" and aggressive wishes toward the part object "bad breast" are soon directed toward the whole object, the mother, the person the child needs and loves.

The mother is conceived of as being both good and bad; if one attacks the bad mother, one also destroys the good mother.

The "whole object" is at this time introjected.

The fear that the child has destroyed or may destroy the mother leads to guilt, fear of loss, and feelings of depression.

This "depressive position," which occurs at about 6 months to a year, is so painful that the child tends to regress to the "paranoid position" of separate good and bad objects.

Eventually the child realizes that the aggressive wishes have not and will not destroy the mother, and the depression ceases, though residues of the depressive position remain in the form of guilt feelings.

Defenses against the depressive conflict bring about a return of paranoid-schizoid phenomena, and the individual may fluctuate between two states.

The paranoid-schizoid position gives way to the depressive position, particularly when external reality provides a preponderance of good experiences.

This enables the ego of the child to acquire belief in the prevalence of ideal objects over persecutory objects.

www.freepsychotherapybooks.org 393 The introjection of these ideal objects also helps to modify the internal persecutory objects and death instinct.

As the ego of the child strengthens, it can cope with internal and external anxieties more easily without recourse to violent mechanisms of defense and extreme splitting.

Under favorable conditions, the infant feels that his or her ideal object and libidinal impulses are stronger than the bad object and bad impulses, and he or she will be able more and more to identify with the ideal object.

The child is less frightened of bad impulses and less driven to project them outside.

Integration proceeds, and finally a phase of development takes place during which the infant recognizes and relates to the whole object.

Originally this is the mother; later there is cognizance of other people in the environment.

In contrast to the infant's perceiving the mother only as bits and pieces—breast, hands, and eyes—she is conceived of in her entirety.

She is also accepted as the same mother who can at times be good, at times bad; present and absent; loved and hated.

The infant begins to see that good and bad experiences do not proceed from two sources but rather from the same mother.

Thus, ambivalence develops.

The child will also realize at this time that the mother is an individual with a life of her own and with other relationships.

This leads to feelings of jealousy and painful feelings of dependency.

As the mother becomes a whole object, the infant's ego becomes whole and fragmentation diminishes.

In the paranoid-schizoid position the main anxiety is that the ego will be destroyed by bad objects; in the depressive position, anxieties spring from ambivalence and from the fear that the child's destructive impulses will destroy or have destroyed the object he or she loves and

depends on.

Mourning occurs, despair is felt, and guilt arises connected to destructive feelings toward the good object.

As the depressive position is worked through by the child, the relationship to objects alters and the child acquires the capacity to love and to respect people as separate and differentiated individuals.

The child has concern for objects, which helps to control impulses and to regulate activities toward self and objects.

Regression supersedes splitting, and neurotic mechanisms rather than psychotic ones take over.

Symbol formation also begins as the infant inhibits the instincts and displaces

www.freepsychotherapybooks.org 394 them onto substitutes.

The development of the child is influenced by both internal and external factors.

External deprivations, physical or mental, clearly influence the child in various ways, but even when the environment is bountiful and should be conducive to satisfactory growth, development may still be modified or prevented by internal factors.

Melanie Klein describes one such factor as “envy,” which she believes operates from birth and affects the infant’s earliest experiences.

While Freud recognized and paid a great deal of attention to penis envy in women, other needs of envy, for example man’s envy of females and both sexes’ envy of one another, were not so specifically recognized and described.

There has been a strong tendency, both in the literature and in everyday practice, to confuse “envy” with jealousy.

In her book *Envy and Gratitude*, Melanie Klein (1957b) makes a distinction between the emotions of “envy” and jealousy.

She considers “envy” the earlier of the two, and she believes it to be one of the most primitive and fundamental emotions.

She differentiates it clearly from jealousy and greed: Jealousy is based on love; it aims at the possession of the loved object and the removal of the rival.

It always exists in a triangular relationship and therefore occurs at a time when objects can be clearly recognized and differentiated from one another.

Envy is a two-person relationship in which the subject begrudges the object for some possession or quality.

It may be experienced in terms of part-object as well as whole-object relationships.

Greed, on the one hand, aims at the possession of all the goodness that can be extracted from the object.

It may result in the destruction and “spoiling” of the object, but this destruction is not directed at, and is only incidental to, ruthless acquisition.

Envy, on the other hand, aims at being as good as the object, and, if this is not possible, at blemishing the goodness of the object.

It is this spoiling aspect of “envy” that is so destructive to development, since the www.freepsychotherapybooks.org 395 very source of goodness that the infant depends on, i.e., the breast in the first instance, and the good things to be achieved from it, is turned bad by envious attacks.

As the source of good things is spoiled, so the capacity to obtain good things from the source, i.e., by introjection, is seriously interfered with.

The infant is really attacking the very source of life, and this phenomenon can be considered to be the earliest direct externalization of the death instinct.

It starts as soon as the infant becomes aware of the breast, and, paradoxically, the better the good experience has been, the more powerful the envious attacks that may be levied at the

breast.

Sometimes the infant will attack the “goodness” after it has been incorporated, envying what has been obtained.

Envy operates mainly by projection, and the fantasy of attacking the breast is supported by such physiological processes as spitting, urinating, defecating, the passage of wind, and penetrating looking.

Even the Oedipal situation may be dominated by envy rather than jealousy, the child attacking the parents and their relationship out of envy of what they have rather than the love that the child feels has been lost (jealousy).

Strong feelings of envy prevent the child from differentiating good and bad things because the good things are attacked and turned bad.

This leads to confusion between good and bad and often results in despair.

Because no ideal object can be found, there appears to be no hope of love or help.

It is apparent from Melanie Klein's theories about child development that aggression is regarded in large measure as hereditary and that mistrust, fear of attack, and depression are inevitable, irrespective of the upbringing the child has experienced.

“The repeated attempts,” wrote Melanie Klein, “that have been made to improve humanity—in particular to make it more peaceful—have failed, because nobody has understood the full depth and vigour of the instincts of aggression innate in each individual.” The only means of resolving infantile anxiety, and of socially modifying the aggressive impulses, she concludes, is by universal child analysis.

Any adult analysis, she insists, that does not deal with and resolve infantile anxiety and aggressiveness is incomplete.

Clinical Implications Kleinian theory holds that neurotic difficulties that occur during and beyond www.freepsychotherapybooks.org 396 childhood are manifestations of paranoid-schizoid or depressive patterns.

Neurotic defenses are evolved around paranoid-schizoid or depressive personalities, i.e., the determining way in which object relations are integrated, part or whole.

An important mechanism in Kleinian theory is called “projective identification,” in which parts of the self and internal objects are split off and projected into an important external object for the purpose of possessing and controlling it.

Projective identification is also aimed at avoiding separation from the ideal object by uniting with it.

Bad parts of the self may be projected to get rid of them and good parts to keep them safe or to avoid separation.

At certain times when normal mechanisms fail to protect the ego from anxiety, the ego may disintegrate as a defensive measure and then project the fragmented pieces.

Extensive use of this mechanism is made in more severely disturbed children and in psychotics.

The use of projective identification produces its own anxieties, i.e., the fear that the object will retaliate and also that the parts of the self that are projected will be imprisoned and trapped inside the object, thus leading to claustrophobic anxieties.

One of the main features of the paranoid-schizoid position is “splitting,” which allows the ego to order its experiences and begin to integrate.

This process usually starts by differentiating objects into good and bad.

Splitting is the basis of what later is called repression; providing it is not excessive and rigid but an important mechanism of defense it functions in a modified form throughout life.

An example of projective identification is provided by Albert Mason (1966), a Kleinian psychoanalyst.

The patient was a 25-year-old nurse who had recovered from two schizophrenic breakdowns.

Though she was not acutely psychotic at the moment, her dreams and fantasy material showed marked psychotic mechanisms.

For example, one dream was of her being at a party when she caught sight of an Arum Lily. She screamed and fainted and a tall, slim carpenter with a pot-belly came out of the water to comfort her.

The dream then changed to her seeing a baby sitting at one end of a carriage.

The baby had a huge penis.

The mother of the baby, an Indian lady, was wrapped in a white sari.

The breast of the mother had a purple engorged look like a genital.

It appeared that the mother had just finished feeding the baby.

The patient's feeling in the dream was of overwhelming frustration.

Her associations made it fairly easy to interpret confidently that the tall slim

www.freepsychotherapybooks.org 397 pot-bellied man stood for the penis of her father that she extracts from the parental intercourse (Arum Lily—associated with marriage) by her scream.

The scream, which is an evacuation of part of herself, is the equivalent of the child's angry evacuation of excreta, which is felt to get into the mother and push out the father.

The man comes out of the water.

The baby (baby part of herself) then feels it possesses the father's genital and can push it into the mother's breast (Indian woman in sari = pigmented nipple surrounded by white breast) in an infantile representation of intercourse.

She screams with frustration because although she fantasizes that she possesses the mother sexually, the baby part of herself gets frustrated because it will not be fed by this kind of relationship.

One can see clearly in this dream how father is really a part object, a tall, thin man with a pot belly representing a penis and scrotum.

Mother likewise is a breast, merely a pigmented head surrounded by a white sari.

During the session with me the patient felt that her stomach was distended (confusion with pot-bellied man) and felt on occasion that I was cutting her sentences short and thrusting my opinions down her throat.

She cuts off the penis and appropriates it (baby with penis) and now feels that I cut off her speech.

She feels I push my analytical penis into her violently, just as she pushed the penis into the mother's breast in the dream.

At one moment in the session my interpretations excited her and made her lose track of an association that she thought was helpful to me.

Here she is confused with [i.e., identifies with] the breast that she excited by thrusting sexuality into it and seducing it away from the feeding situation, i.e., I am felt to be putting things into her which excite and seduce her mind away from the thought which could feed the analysis.

One sees over and over in this kind of relationship how she becomes confused with her objects and how she feels the feeling that her objects—breast and penis in this instance—are experiencing in her dreams and phantasies.

At one point in the session she remembered testing urine for sugar and noticing the doctor looking at her, as she thought, sexily.

Again the confusion with me is clear.

She becomes the analyst of patient's material who is being penetrated by a doctor's sexy look. (In the dream she penetrates me as the mother, with the genital).

Another patient, a paranoid homosexual of twenty four, continually feels that his words are dirty and excite me, i.e., dirty my mind.

This is then followed in the session by intense irritation and preoccupation with his anus with a mixture of pain and excitement.

This he feels I cause by interpretations which he feels humiliate him, penetrate to his core, and get on top of him.

He continually makes jokes and quips in the session, i.e., he buggers the analysis about and immediately feels my interpretations are aimed at making him small, exhibiting my skill (penis) and in fact bugging him rather than nourishing and feeding his mind.

He often gets claustrophobic on these occasions and feels in despair because he is not sure whether he is not telling me the truth of whether my interpretations are true or just a load of "phoney crap" as he puts it.

One can see here how he projects parts of himself into me and immediately becomes confused with me and feels penetrated and projected into by me.

He also feels trapped inside me and the mess he produces which he pretends is exciting (faeces presented as something sexual or good to eat) become hopelessly confused

www.freepsychotherapybooks.org 398 with my analytical food, so that that is suspected of being phoney, i.e.

not true food but disguised "crap." This kind of confusion which is based upon the infant's envy of the breast and feeding qualities of the mother, and which are then confused with its own excretory activities, is a common and persistent characteristic of many homosexuals and needs careful analysis for any resolution of the homosexuality to become possible.

The primitive emotion of envy is often said to be the root of negative therapeutic reactions and interminable treatment; the patient is unable to tolerate help from the analysis and destroys it outside or inside his or her own mind.

Good interpretations, felt to be like the nipple putting good food in to nourish the patient's mind, or like the potent father's penis, which can satisfy or create, are both attacked and destroyed when envy is strong.

A frequent defense against envy and one that makes itself felt very commonly in the analytic situation is contempt, which is an effort to defend the patient against unbearable envy and hostility.

Envy is very often unconscious, and considerable working through of it is needed before it appears in awareness.

"The making of the unconscious envy conscious," says Mason, "will usually result in the mobilization of more love and concern for the attacked object and therefore some diminution of destructive envious attacks." The further understanding of early mechanisms in the mental life of the child brought about by Melanie Klein's work has encouraged several of her followers to apply her findings to the treatment of psychotics.

Notable among these is Herbert Rosenfeld, whose book *Psychotic States* (1965) outlines some of his work.

What emerges most clearly is Rosenfeld's strict adherence to classical analytic procedure and principles.

The understanding of the psychotic transference is considered essential to maintain contact with schizophrenics and to analyze them without resorting to artificial means.

Freud (1953) and Abraham believed that schizophrenics were incapable of forming a transference to the autoerotic level of development because they were regressed.

Nurnberg (1955), O'Malley (1931), Barkas (1925), LaFargue (1937), and later Sullivan (1962) (1962) (1939) (1954), Fromm-Reichmann, Knight, and their collaborators have, however, described the importance of the transference in schizophrenia.

www.freepsychotherapybooks.org 399 Rosenfeld demonstrates that the acute schizophrenic patient is capable of forming both a positive and negative transference, which may be interpreted and

can produce clear responses from the patient.

He states that in all schizophrenics whom he has observed one particular form of object relationship appears very clearly: "As soon as the schizophrenic approaches any object in love or hate, he becomes confused with this object (projective identification).

This is due not only to identification by introjection, but to impulses and fantasies of entering the object with the whole or part of the self in order to control it." Agreeing with Melanie Klein, Rosenfeld believes that this is the most primitive type of object relationship and that it begins at birth.

He contends that the withdrawal of the schizophrenic is not simply an autoerotic regression; it may be a defense against external persecutors, or it may be due to identification with an object involving both introjection and projection.

In a state of projective identification, which may be experienced by the patient as confusion, the patient is aware of being mixed up with someone else (his or her object).

Rosenfeld believes that in the analysis of acute psychosis the psychotic manifestations attach themselves to the transference and a transference psychosis develops.

The transference phenomena can be interpreted to the patient only when they are comprehended.

The full understanding of projective identification enables one to do this.

In his book Rosenfeld also has interesting chapters on drug addiction and hypochondriasis.

He believes that drug addiction is closely related to manic-depressive states but is not identical with them.

Drug addicts use manic and depressive mechanisms that are reinforced and consequently altered by the drugs.

Drugs have both a symbolic meaning, relating to unconscious fantasies, and a pharmacotoxic effect, which increases the omnipotence of the prevailing impulses and mechanisms.

The use of such mechanisms as idealization, identification with ideal objects, and denial of persecutory and depressive anxieties is associated with a positive or defensive aspect of mania.

Also, the destructive phases in drug addiction are closely allied to the destructive aspects of mania.

An important feature of drug addiction is the projection of good and bad parts of the self.

Rosenfeld reveals that crises of severe drugging may occur when a drug addict is making progress in analysis and a splitting of the ego diminishes.

This leads to aggressive acting out and can be regarded as an envious attack on the therapist.

On the surface, the Oedipal conflict and homosexuality play an important part in the psychopathology of the drug addict, but the overwhelming force of these conflicts can be understood only by examining their basis in the very earliest conflicts and mechanisms of the infant.

Rosenfeld considers the hypochondriac state as constituting a defense against schizophrenic or paranoid conditions and confusional anxieties.

The ego seems to be unable to work through the confusional state that it projects, including internal objects and parts of the self, into external objects.

These are then immediately reintrojected into the body and body organs.

A characteristic of the chronic hypochondriac is the inability to obtain proper oral gratification.

This frustration is displaced into and complicates the genital sphere of activity.

The anxiety created by the patient's genital frustration increases the tendency to regression and mobilizes early confusional anxieties.

As a defense against this, the hypochondriac state becomes manifest.

Another of Klein's followers is W.

R.

Bion (1962, 1963, 1965, 1967, 1970).

He has made certain contributions, especially in the field of thinking and thought disorder.

His first book, *Experiences in Groups* (1961), has become a standard work among group therapists and demonstrates his use of Kleinian theory in the group process.

He notes in detail the use of omnipotent fantasies present in groups and how these fantasies are used to deal with the frustrations concomitant with contact with reality.

These omnipotent fantasies are similar to those Klein described that she observed in the infant and its responses to the outside world.

Bion also made important observations on the effect of the "genius" or "Messiah" in a group and described how this genius could enlarge or improve the group and be enlarged and improved by it or, conversely, how they could destroy each other.

Bion's work on psychoses essentially supported that of Klein and Rosenfeld, and he demonstrated that a strict analytic and interpretive technique could be utilized successfully with psychotics, as Rosenfeld had also done.

He linked the phenomenon of projective identification with the development of thinking.

He also described certain www.freepsychotherapybooks.org 401 pathological forms of projective identification and postulated that these produced a failure of development of verbal thought and thus played a part in the development of schizophrenic illnesses.

Klein's concept "that the infant treats the absence of the good breast as the presence of a bad breast to be evacuated" was explored and expanded by Bion and was the central theme in many of his papers.

He also believed that this fantasy plays an important part in the development of psychosis.

Bion examines the nature of thoughts and thinking in several of his books and has written extensively about the nature of hallucinations and hallucinosis.

He has described the problem of the patient who cannot learn from experience and, therefore, cannot grow, and he has linked this phenomenon with Klein's work on envy.

Donald Meltzer has worked extensively with children and, from his experience, has written several books (1973a & b, 1975).

He sets out to revise the psychoanalytic theory of sexuality as formulated by Freud.

In so doing, he contends that the original nebulous formulation of the infantile polymorphous-perverse disposition (and genetic concepts of stages of psycho-sexual development and erogenous zone primacies, eventuating in a genital primacy) is shown to resolve itself.

Through delineation of the distinction between adult and infantile structures of the mind, criteria of a purely psychoanalytic variety are evolved for assessing the significance of sexual states of mind and consequent behavior.

Meltzer discusses the nature of autism and the specific technique of obsessional dismantling of the sensory apparatus of the ego that brings it about.

He also examines the origins of mutism as part of the natural history of the disorder and the implications of all these findings for psychoanalytic theory and practice in general.

His understanding and clarification of the perversions and their link to perverse states of mind are believed by some to be an important synthesis of the work of Freud, Abraham, Klein, and Bion and a major contribution to the understanding and treatment of psychoses, perversions, and addictions.

Elliot Jaques has written eight books on the implications of Melanie Klein's work to the fields of industry, sociology, and law.

In addition, he has written papers about social justice, art, and creativity and is one of the leading applicants of psychoanalytic theory to www.freepsychotherapybooks.org 402 everyday

life.

Other Kleinians whose works have dealt with art and anthropology, as well as the treatment of patients, are Roger Money-Kyrle and Hanna Segal.

Bion, Rosenfeld, Meltzer, Jaques, Money-Kyrle, and Segal were all analyzed by Klein.

Criticism of Kleinian Analysis Both Freudians and non-Freudians believe that the concept of phylogenetically determined images of the genitals, breasts, and primal scene and the description of incorporative and projective symbolic thought processes in infancy are fanciful projections of a theoretically biased therapist.

They insist also that focusing all psychopathology on distortions developed during the first year of life is too limiting.

There are some who accept aspects of Kleinian theory, for example, the basic paranoid and depressive positions, while rejecting such aspects as pregenital Oedipal conflict and formulations related to the death instinct.

In spite of the criticisms levied against Melanie Klein, her contributions have had a great impact on analytic theory and practice.

She was instrumental in showing how the superego formed itself from environmental rather than biological sources, the vitality of intrapsychic struggles between introjected good and bad objects, how object relations were prejudiced by projections from these objects, and the impact of aggression on pathological development, the infantile precursor of structural entities.

Most important, her work acted as a basis for present-day ego analysis and object relations theory.

EGO ANALYSIS Fairbairn (1946a & b; 1954) was one of the pioneers in emphasizing the importance of the ego.

He developed an object-relations theory, patterned after some of the conceptions of Melanie Klein, which posited a splitting of the ego in early infancy as a consequence of reciprocal action of introjected good and bad objects.

While the ego was whole at birth (rather than incomplete as in classical theory), unfortunate relationships with the mother fostered this splitting.

The function of the instincts (libido) was to seek good objects.

This was needed to promote ego growth.

Aggression was a reaction to www.freepsychotherapybooks.org 403 frustration of the libidinal drive in this quest.

Aggression, therefore, was a defense rather than an instinct (as in classical theory).

Loss of ego integrity through the process of splitting and evolution of internal ego-object relations created pathology.

The developmental process was thus bracketed to vicissitudes of objects rather than vicissitudes of instincts.

Early interactions of mother and child as a basis for ego development was also emphasized by Winnicott (1958).

Among the contentions of these early adherents of ego psychology such as Fairbairn was the idea that objects were not casual figures who served merely as conduits for infantile need gratification, but rather an integral part of the infant's nature and instinctually sought out from the start.

In this way Fairbairn anticipated what later researchers in human development demonstrated, namely, that the newborn child was object-related from the moment of birth.

Instead of considering structure (the ego) and energy (the id) as distinct entities, Fairbairn contended that they were intimately bound together.

Libido was not pleasure-seeking as much as object-seeking, and this was a biological survival mechanism.

A so-called primary narcissistic objectless initial stage of development was an empty abstraction, because object-seeking, albeit disorganized at first, was the primary aim of the infant.

Relations with an object were the key to survival of the individual (mouth to breast) and later to the survival of the race (genitals to genitals).

Adaptation was the product of a good relationship between individual and object; psychopathology, the consequence of a poor relationship.

The conviction that human behavior is too complex to be accounted for purely in terms of instinctual processes gradually turned a body of Freudians toward the focal consideration of dimensions of personality other than the id, particularly the ego, while retaining fidelity to the dynamic, structural, economic, topographic, and other basic psychoanalytic concepts, including the libido theory.

Among the first of these “ego analysts” were Anna Freud (1946), Erikson (1946, 1950), Hartmann (1950a & b, 1951, 1958), Rapaport (1950, 1951, 1958, 1960), Kris (1951), and Loewenstein (1953).

The direction of the ego analysts has been less introspective and speculative than it has been empirical, based on factual investigations, the systematic gathering of data, and organized www.freepsychotherapybooks.org 404 experiments.

Attempts have been made to avoid philosophical issues and implications in order to deal more scientifically with facts.

This has led to intensive studies of the child and particularly responses to various child-rearing practices, interactions within the family, as well as the influence of the community.

Sociological and anthropological vectors have accordingly entered into some of the emerging formulations, although the orientation is definitely a biological one.

Although primary psychological drives are considered basic and important, these are dealt with in the context of the molding and modifying influence of the environment, which is believed to play a decisive role in eliciting behavior independent of instinctual forces.

Building on Freud’s conceptions of instinct, ego analysts regard behavior as undifferentiated at birth, at which time the infant possesses certain response potentials, innately derived but requiring the influence of environment to arouse and consolidate them into adaptive sequences.

Among the groupings of responses are certain internal and external elicitors of behavior that are distinctive from instincts— those that deal with responses to perceptual stimuli and those that serve organizing, integrating, and controlling functions.

Response patterns serve to adjust the individual to the particular environment.

Responses such as awareness and thought, which serve to control and direct behavior, are also innately determined.

Learned responses soon displace instinctual and automatic reactions.

Behavior is more than a means of reduction of sexual and aggressive energies.

Ego functions can be pleasurable in their own right.

Among the most important ego functions are those that mediate perceptions and sensation and support operations that maintain contact with the external and internal environment; there are those that deal with awareness and attention (which can help delay or inhibit impulses), those that govern thinking and communicating (verbal response), and others that control action and motility, enabling management of one’s environment.

The ego in its synthetic, integrating, and organizing operations fosters a controlled, thoughtful, planned, and efficient mediation of behavior directed at consciously selected goals.

Though psychosocial development is crucial during the first five years of life, laying down

patterns that will determine behavior throughout the remainder of an individual's existence, these patterns are not as completely fixed and unmodifiable as the earlier Freudian theorists supposed.

Nevertheless, at certain stages of growth environmental experiences can have a decisive influence on the total personality structure.

Ego development occurs immediately after birth as the child discriminates between inner responses and the influences of the environment, for instance, in feeding.

Gradually the child differentiates self from the environment and anticipates future events. Frustrations encourage self-control.

The child develops the ability to recall past situations in which delay in gratification was followed by fulfillment.

Habitual response patterns are developed in relation to surrounding objects enabling the child to win their approval and to control feelings from within.

A sense of personal continuity and identity emerge.

Problem solving and coping are aided by imitation (identification).

The social milieu becomes incorporated within the individual, seducing the child, as Erikson has put it, to its particular life style.

Self-esteem is built from exercise of different skills and the fulfilling of interpersonal experiences.

Defense mechanisms are evolved to control fear and the situations in which it becomes conditioned.

The signal of anxiety serves to mobilize defenses in the repertory of the child, and although the early conditions that fostered them no longer exist, the individual may continue to employ them throughout life.

Learned patterns of behavior are established as "hierarchical structures" from the base of the earliest patterns to the apex of the latest responses, the original ones never being completely ablated but merely replaced by the later ones.

This applies also to thoughts, at the foundation of which are primitive "primary-process" thought patterns concerned with instinctual drive reduction; these are replaced gradually by logical thought.

Furthermore, the mechanisms of defense are in hierarchical arrangements.

Their antecedents reside in physiological responses, and their latest representations are in the form of creative thought.

Behavior is considered to be neither the by-product of instinctual energies nor the result of situational events.

Rather, it is a mode that reflects and yet gradually achieves relative independence from both through the development of autonomous stable response structures.

Healthy behavior is under conscious control.

When the ego loses its autonomy from the id or from reality, behavior is no longer under conscious control and pathology may ensue.

This is particularly the case when residual stable behavior patterns are insufficient to deal with an existing stress situation.

A variety of circumstances contribute to the formation and maintenance of learned adaptive structures, and a consideration of these is vital to the understanding of behavior pathology.

In therapy that is conducted under orthodox Freudian rules, an added goal is an attempt at expansion of the repertory of learned patterns to enhance conscious control of behavior in relation to both inner impulses and environmental pressures.

Hartmann has speculated that eventually a technique system will be evolved that can keep abreast of new theoretical developments.

Under such a system an effort would be made to understand not only pathological but also adaptive behavior patterns and to examine the interrelationship between the conflict and nonconflict aspects of the ego while tracing the antecedents of neurotic anxiety.

There is an implication in some of the writings of ego analysts that therapy should embody more active procedures than the orthodox techniques employ.

For instance, interpretations should be couched in terms of specific events rather than in abstract concepts.

A focus on immediate problems in the current life situation and on character defenses that influence interpersonal relationships can be productive.

In summary, the basic contributions of the "ego analysts," as these analysts have become known, are the following: 1.

Behavior is determined by forces other than instinct in the form of response sequences encompassed under the classification of "ego." 2.

The ego as an entity has an autonomy separate from both instinct and reality.

3.

The ego supports drives for environmental mastery and adaptive learning that are divorced from sexual and aggressive instincts.

4.

The adaptive aspects of learned behavior are as important as instinctual behavior and lead to important gratifications in their own right.

5.

A greater emphasis must be put on the environment and on healthy, as opposed to pathological, behavior than is found in orthodox Freudian approaches.

An understanding of pathological behavior in relation to normal behavior is vital.

6.

Personality is more plastic and modifiable, even beyond the period of childhood, than is traditionally supposed.

A more hopeful prognosis is consequently forecast.

7.

The human being is the master of his or her destiny and can control and select behavioral patterns to achieve differentiated goals.

8.

Society is a force that does not necessarily emerge from a human being's expressions of instinct; nor does it always thwart the biologic nature of a human being.

It can exert a constructive influence on the individual while modifying primitive instinctual drives.

9.

Conscious and learned responses are basic to a person's adjustment.

10.

Technical innovations in the direction of greater activity are sometimes necessary.

Criticisms of Ego Analysis Many non-Freudians contend that ego analysts essentially have not really abandoned the archaic classical model.

They have merely altered it to fit into a more palatable framework of development and learning. While the structural hypothesis of id, ego, and superego conceptualizes some aspects of behavioral function, by no means does it encompass all aspects.

The mechanisms of adaptation are so complex and involve so many facets of behavior that simplistic models cover only limited areas of operation.

There are also flaws in the related epigenetic theory that identifies sequential stages of

learning in the developmental process and organizes a diagnostic system around these stages. Of course, assessment of development levels and existing fixations is a convenient way of looking at pathology, but inferences drawn from this regarding coping capacities are not always correct, and in fact may be misleading.

Freudians, on the other hand, insist that Freud in no way minimized the importance of external objects and their function in molding the inner organization.

They believe that www.freepsychotherapybooks.org 408 putting the ego at the helm of the entire psychic structure neglects other important and perhaps more determining components.

The ego should be considered a substructure, a part of the total apparatus, emerging from the original primary narcissism, gradually evolving as an aggregate of functions.

NEO-FREUDIAN AND NON-FREUDIAN PSYCHOANALYSIS Classical psychoanalysis as a specialized psychotherapy, devised by Sigmund Freud, consists, as indicated previously, of a group of organized procedures whose objective is to activate in the relationship with the therapist (transference neurosis) the conflicts experienced during the early formative years (infantile neurosis) that, sealed off in the unconscious by repression, dissipate energy and foster an unrealistic adaptation.

By interpreting and “working through” the regressive transference neurosis (that embodies distortions in relationships with parental agencies), an attempt is made to resolve the infantile neurosis, liberating the individual from fixations in personality growth toward greater self-actualization.

As a technique, classical (Freudian) psychoanalysis is applicable to a class of patients who are able to devote themselves to and tolerate the rigors of a long-term intensive exploratory process.

In essence, therapeutic results depend upon the replacement of unconscious mental acts by conscious ones through the overcoming of internal resistances in the patient's mind.

The vanquishing of resistances is brought about by the use of special techniques that are employed with the object: first, reviving memories of past experiences, particularly those of childhood, which, due to their painful content, have been repudiated and repressed; and, second, activating and transferring over to the analyst the early “infantile neurosis” responsible for the unconscious conflicts that continue to blight the patient in the here and now.

These conventional characteristics of therapeutic psychoanalysis have been disputed by some factions of the psychoanalytic fraternity.

Operating under the rubric of “neo-Freudian psychoanalysis,” these analysts challenge the economic, dynamic and topographic formulations of Freud, including the instinct theory, the significance of www.freepsychotherapybooks.org 409 infantile sexuality, and the ubiquity of the Oedipus complex.

The very existence of the unconscious itself is questioned by some.

These deviations have sponsored combinations of orthodox techniques with more active and supportive measures, such as shifting the focus from the past to the present, reducing the number of weekly sessions, veering from free association to more structured interviewing, and minimizing the need for transference neurosis.

Understandably, some consecrated Freudians have responded to these adulterations with consternation, insisting that neo-Freudians, in removing the pillars of psychoanalytic structure and in altering the orthodox procedure, have no right to call their therapeutic maneuvers “psychoanalysis.” Rigorously speaking, psychoanalysis in its “pure” form consists of the creation, by a professional person thoroughly trained in the method, of a transference neurosis through the maintenance of neutrality, anonymity, and passivity in the relationship with the patient.

As already noted, this method includes frequent sessions (four to five times weekly), the use of the couch, the adoption of the “fundamental rule” of free association, the employment of dreams, the focus on early significant memories, and the detection and interpretation of resistance.

Patients are thus afforded an opportunity to experience, to understand, and to resolve in a dynamic, protected atmosphere conflicts of which they were previously unaware.

Contaminations of the purity of psychoanalysis have been fomented by an increasingly large body of psychoanalysts who have introduced ideas from social theory, role theory, group dynamics, cultural anthropology, field theory, and even philosophy.

Deviations from Freudian psychoanalysis vary from minor divergences related to one aspect of theory to major disagreements in which substitute hypotheses or therapeutic methods appear to depart radically from commonly accepted definitions of psychoanalysis.

Among the propositions of the neo-Freudians are the following: 1.

Personality is fashioned principally by cultural rather than instinctual forces; the value systems of society are incorporated in the individual's character structure and determine the individual's action tendencies; www.freepsychotherapybooks.org 410 conflict is a product of diverse factors within and outside of the person and involves both conscious and unconscious factors.

2.

The myriad elements—social, interpersonal, intrapsychic—entering into character organization necessitate a comprehensive and holistic view of personality theory, together with a concern for healthy as well as abnormal adaptation.

3.

The libido and death-instinct theories are formulations that cannot explain either normal or abnormal behavior; infantile sexuality alone cannot account for an individual's basic conflicts or for the lines along which character structure develops.

4.

Female sexuality is an entity on a parity with, rather than inferior to, male sexuality.

5.

The classical topography does not explain the structure of the psychic apparatus.

6.

The therapeutic encounter is more than a means of repeating and working through early traumatic experiences; it is an experience in a relationship, containing positive growth potentials that can lead to greater selfactualization.

7.

Activity and flexibility in the therapeutic approach are essential; this encourages eclecticism in method.

8.

An optimistic, rather than pessimistic, viewpoint is justified regarding a human being's potentials as a creative, loving, and peaceful being.

Primacy of Cultural Factors in Personality Development The concept, in the words of Geza Roheim, that “culture is the creation of a substitute object” embodying solutions for pre-Oedipal and Oedipal conflicts has not gained too wide acceptance among sociologists and anthropologists.

While Freudian ideas about individual development and the unconscious are considered credible, those that apply to cultural theories and social process are more or less rejected.

Interesting accounts of how society, particularly through its child-rearing practices, molds personality structure are found in Kardiner's writings (1939).

Elaborated is a delineation from www.freepsychotherapybooks.org 411 comparative studies of a

number of different societies that reveals personality characteristics as reflective of, and congenial with, the total range of institutions within a culture, although an individual character structure may show certain variations in response to personal experience. Ruth Benedict (1953) and Margaret Mead (1939, 1952) in extensive field studies, have shown that stages of personality development that have been accepted as universal are not applicable to certain cultures.

For example.

Mead (1939, 1952) in studying adolescent girls in Samoa observed that the traditional tumultuous adolescent upheaval did not exist, possibly because few restraints were put on self-expression and sexuality.

Personality characteristics usually credited to male and female are shown to be more the product of social role than biological forces.

Disturbances of society, such as delinquency, crime, drug addition, and sex offenses, as L. K.

Frank (1957) has pointed out, should not be regarded as distortions of the instincts or signs of human wickedness but as manifestations of disintegrating forces in the environment.

Stressing concepts from field theory, neo-Freudians do not regard the environment as a projection vehicle that is molded by instinctual needs and demands.

Rather, they look on it as a cardinal force in itself that shapes personality.

Behavior is conceived as the product of many vectors, both biologic and social.

Constitutional and hereditary elements, while present, do not determine an individual's destiny.

This is mediated principally by experiences in life.

Attention must be focused, therefore, not on the unconscious—on instincts and their vicissitudes—but on the relationship of the individual with the significant persons in the early and later development who are carriers of the value systems of his or her culture.

Personal values reflect these systems.

Distortions in relationships and in values are registered in the character structure, disparate operations of which act as a potent source of conflict.

Though there is general agreement among the various neo-Freudian groups who hold these ideas, there are discrepancies as to the most important cultural determinants, the specific effects that are registered on character structure, and the consequences of such impacts.

The existence, role, and content of the unconscious are also matters about which there is disagreement.

www.freepsychotherapybooks.org 412 The Holistic Viewpoint in Abnormal and Healthy Adaptation

The concept of character as fashioned by the culture has directed attention to the institutions and values of society that foster healthy and unhealthy adaptation.

Instead of viewing healthy aspects of personality through the lens of pathological distortion, neoFreudians have tended to investigate what goes on in the "normal" or healthy individual, drawing inferences from data dealing with adaptive rather than sick behavior.

Abnormal behavior is considered within the framework of understanding "normal" behavior rather than the reverse.

This has widened the horizons of progressive personality research to include pertinent areas of sociology, social psychology, ecology, anthropology, ethology, and philosophy.

The focus of inquiry is on the intricate network of organizational units— interpersonal, familial, group, national, and international—as well as the sum total of institutions that constitute society as a whole.

Behavioral studies are encouraged that scrutinize the interaction of individuals and groups in a variety of settings.

The habits, manners, mores, and customs of people in primitive and civilized organizations are surveyed, and cross-cultural data on child-rearing practices are analyzed.

Modes of reacting to aspects of the environment that constitute the individual's life space and ecological patterns of such phenomena as crime, delinquency, poverty, and insanity are examined.

The behavior of animals in their natural habitat is observed to determine the relative roles of instincts and social learning.

Finally, studies of aesthetic, moral, ethical, and spiritual promptings, of social values and how they are internalized and influence behavior, bring the behavioral scientist into the fields of philosophy and religion.

Scientists from diverse fields, cooperating together in interdisciplinary research, foster a better integration of the biological and social sciences.

By considering the human being as a totality, neo-Freudian approaches support a holistic and Gestalt point of view.

The individual is considered to be a tapestry of biochemical, physiological, psychological, sociological, and spiritual systems, each of which has a feedback onto the others.

Shortcomings of the Instinct Theory The libido theory as a developmental as well as therapeutic model, ingenious as it is, is considered by neo-Freudians to be inadequate in explaining what goes on in all www.freepsychotherapybooks.org 413 personality operations.

It is believed to be highly overgeneralized, extending itself into zones of energy exchange that cannot possibly deal with the complexities of human relationships.

For instance, pregenital instinctual drives are not considered to be the basic elements involved in character organization.

Parental attitudes and practices, on the other hand, do have a determining effect upon feeding, excretory, assertive, aggressive, and sexual patterns—indeed a greater impact than forces of maturation.

Experiences in the family are the cradle of faulty conditionings.

Personality functions are best conceived of in an interactional or transactional framework.

This calls for a different perspective on such phenomena as infantile sexuality.

All bodily activities should not be regarded as manifestations of sexuality.

For example, pleasure in feeding and excreting cannot conceivably be sexual even in the broadest sense of the word.

Evidence of the sexualization of certain bodily activities during analysis does not necessarily prove the case for pansexual development in childhood.

Even frank genital exploration and manipulation in childhood may be less a sign of true sexuality than a mark of curiosity and the seeking of knowledge of how the body is constituted. Where childhood sexual aberrancies develop and persist, this is evidence of a disturbed upbringing, precocious erotic stimulation, and response to anxiety rather than anarchical sexual instincts.

The presence of the Oedipus complex during psychosocial development is not disputed, but its universality and ultimate destiny are questioned.

Some neo-Freudians accept firmly the ubiquitous biological nature of the Oedipus complex, but they contend that its form is influenced by the particular culture in which the child is reared.

Others believe that the Oedipus complex is not a biological phenomenon but rather the product of provocative conditioning, particularly in families in which strong dependent attitudes are encouraged in the children or in which the child encounters excessive sexual stimulation through the overfondling activities of the parents.

Cultures in which such dependency or sexually seductive attitudes do not exist do not foster

the Oedipus complex in children.

When the Oedipus complex develops, it may be a manifestation of discord and distrust between the parents, resulting in their utilizing the child as a vehicle for frustrated love needs.

Emotions of jealousy and hostility emerge in the child as a by-product of the conflict that is engendered in the child.

The child selects the “strong” and dominant parent www.freepsychotherapybooks.org 414 —mother or father—as an identification vehicle, and when mothers play the dominant role, the child will, for security’s sake, tend to identify with the mother.

This can create problems in sexual identification for the boy.

The Oedipus complex may accordingly be understood in cultural terms as a reaction to anxiety, and not necessarily as a manifestation of the libido.

The death instinct also is labeled a metaphysical concept rather than a plausible theory to account for aggression, masochism, and sadism.

It has little theoretical or clinical usefulness.

Aggression is not a primary drive but, as has been repeatedly pointed out, a secondary reaction to drive frustration.

Masochism is a special kind of defensive response to anxiety marshalled by certain interpersonal conflicts.

Dualistic formulations regarding sexuality and aggressiveness are not considered adequate in explaining the complexity of these drives.

For instance, bisexuality is not universal as some authorities insist.

Though physical rudiments of the opposite sex are present in an individual, this does not justify the notion that they must influence the individual’s mental life (Lillie, 1931; Rado, 1956).

The idea of universal latent homosexuality has led to therapeutic nihilism in treating homosexual problems.

Placing a higher biological value on the aggressive quality of the male genital is also unjustified; the crediting of superiority to the penis as compared to the female genital is a cultural phenomenon that can easily shift with a change in social values.

A Positive Approach to Female Psychology Male-oriented concepts of female psychology that regard women as arrested males, frustrated by their biological inferiority, are alleged to be a product of the cultural consideration of women as an inferior species.

Under these circumstances penis envy, when it develops, is a manifestation of the underdog philosophy foisted on females by the dominant males.

By possessing a fantasized penis and masculine strivings, a woman compensates for her feelings of social inferiority.

Similarly, the designation of passivity, dependency, and masochism as female characteristics does not reveal their true nonsexual dynamic qualities.

Female psychology, therefore, can be explained purely in www.freepsychotherapybooks.org 415 cultural rather than biological terms.

On the other hand, there are biological differences between males and females, and there are variant social roles that they must assume.

For instance, motherhood imposes certain demands on women that makes for characteristics distinct from those of males.

The Structure of the Psychic Apparatus While convenient for categorizing broad groups of mental activity, the structural conception of the mental apparatus in terms of superego, ego, and id is not believed to be adequate for explaining psychic functions.

Overweighting of the superego and id in conventional analytic formulations has left the ego a barren area.

Moreover, breathing life into the id-ego-super-ego trinity, giving it substance and location, and charging it with human lusts, fears, hostilities and jealousies are both animistic and clinically untenable.

The id is not the core of all human energy and activity.

The concept of the superego is a confusing one since it incorporates both the healthy elements of the conscience and the neurotic, compulsive qualities.

The values embraced by the superego are not a mere facsimile of standards incorporated from parental agencies, but reflect a variety of other conditionings.

The Therapeutic Encounter as a Positive Growth Experience The therapeutic relationship is a two-way transaction in which there is a feedback of feeling between therapist and patient.

What is effective in therapy is not the expulsion from the unconscious of material that results in startling insights, but the emotional experience of two individuals relating to each other in a productive way.

The therapeutic interpersonal relationship has a healing effect that mobilizes the patient's capacities to solve his or her own problems.

The therapist is never neutral during this process.

Values and prejudices filter through irrespective of how much the therapist tries to act as a neutral screen.

Nonverbal responses, the emphasis on certain kinds of content, and the nature of interpretations all reflect personal standards that will influence the lines along which the patient thinks, the kinds of ideas that will be retained, and the direction the patient will follow in revising his or her life style.

This has led to a deliberate www.freepsychotherapybooks.org 416 abandonment of anonymity on the part of the neo-Freudian analyst, interpersonal spontaneity, and the ability to reveal personal values that are proffered as potential contingencies rather than as absolute mandates.

The expression of hostility by the patient during analysis is not always considered necessarily an index of good therapy, marking the release of repressed, transferential energy.

It may be a manifestation of provocations inspired by the therapist's rejecting and detached attitude.

Aggression is regarded as a secondary reaction to frustration and a defense against anxiety. When it occurs in therapy, it is usually being mobilized as a response to conflict within the immediate interpersonal relationship.

Rather than helping, it may be a detriment to therapeutic progress.

There is a tendency among neo-Freudians to regard conscious aspects of experience as important, if not more important, than unconscious operations in the formation of conflict.

Indeed, some neo-Freudians depreciate the value of probing into unconscious ideation.

Also important to many neoFreudian theorists is the question of "responsibility" of a patient for his or her own actions.

"Eclecticism" in Therapeutic Method The realization that most patients cannot avail themselves of the opportunity of coming to sessions four to five times weekly, and that, of those who can, many are not suited for intensive probing and the rigors of a transference neurosis, has led to a reduction in the number of sessions, greater activity and flexibility in the therapist's tactics, relaxation of the fundamental rule of free association, and the introduction of a variety of adjunctive procedures within the framework of treatment.

Without arguing the points as to whether such stratagems convert the gold of psychoanalysis into a baser metal, whether results of this "eclecticism" are more superficial because they are based on suggestion, or whether reconstruction of personality is bypassed in favor of symptom relief and the expediency of environmental adjustment, the amalgam has, it is claimed, proven helpful to more patients than could otherwise be reached (Abroms, 1969).

A Constructive Philosophy Toward Humankind www.freepsychotherapybooks.org 417 In minimizing the fixity of behavior in instincts, a different philosophy toward humankind is encouraged.

A human being is more than an animal whose biological heritage chains him or her to the limitations of inner strivings.

A human being is not basically lecherous or destructive.

These characteristics, if they occur, are environmentally nurtured.

Emphasis on the essential goodness of humankind, not as a reaction formation to aggressive and destructive instincts but as a quality in its own right, emphasizes positive values as determining forces in the creation and molding of personality.

It substantiates the human being as a creature who has needs to receive and to extend "tenderness," "care," and "love" for others.

Discussion Little uniformity of opinion exists among the different neo-Freudian groups and even among members of the same school regarding the above points.

Ideas range from a practically complete acceptance of the basic Freudian tenets, differing only slightly in how they are formulated, to extensive deviations, which include denying the existence of the Oedipus complex and even of the unconscious.

Some analysts believe in a treatment process focused around the mobilization of a transference neurosis (although the explanation of the nature of the infantile neurosis is more inclusive than the Oedipus complex), and hence they employ the traditional tactics of frequent sessions, therapist anonymity and passivity, free association, concentration on infantile and early experiences, dream analysis, and interpretive focus on transference and resistance.

Others depreciate the rationale of transference neurosis and contend that dealing with conscious material focused on the present is more effective than delving into the past with discursive explorations of the unconscious.

The introduction of supportive procedures and activity in the relationship, the abandonment of free association and the couch position, the breaking up of transference before it gets out of hand, and the violation of the most traditional rules of psychoanalytic technique constitute the methodology employed.

To the consternation of orthodox analysts, there is insistence on the part of some apostates that their instituted www.freepsychotherapybooks.org 418 modifications be accepted as legitimate variations of psychoanalytic techniques.

In some instances, even substitution of philosophic precepts for analytic techniques are presented under the title of "psychoanalysis." A number of neo-Freudian groups have dedicated themselves to "exploring more scientific approaches to psychoanalysis." To support an interchange of ideas they have organized a rival group to the Freudian American Psychoanalytic Association called the American Academy of Psychoanalysis and publish their own journal, The Journal of the American Academy of Psychoanalysis.

The technical modifications employed by many of those who have deviated from orthodox Freudian theory and method include the following: 1.

Passivity in the relationship is superseded by activity in order to deal more adequately with resistance and to subdue the development of neurotic transference.

Anonymity of the therapist is, for the same reason, not completely observed.

2.

Free association is abandoned as a "fundamental rule," and the interview is focused in nature.

3.

The couch position is replaced completely or partly by a sitting-up, face-to-face position.

4.

The number of visits may be reduced to as few as three sessions weekly, sometimes to even two or one.

5.

The therapeutic relationship is handled in a manner so as to resolve transference as soon as it begins to operate as resistance.

An attempt is made to minimize the development of a transference neurosis.

Positive elements in the relationship may be encouraged as a catalyst to therapy.

The relationship is regarded not only as a mirror that reflects unconscious strivings, but also as a vehicle that has values in itself as a growth experience.

6.

The focus in therapy is on both unconscious and conscious aspects of personality.

Current problems and situations are stressed as much as past experiences.

7.

There is a blending of analytic techniques with methods derived from supportive and reeducative approaches.

www.freepsychotherapybooks.org 419 The most notable non-Freudian contributions to dynamic psychiatry have been made by Adler, Jung, Ferenczi, Rank, Stekel, Reich, Rado, and members of the "dynamiccultural school," including Fromm, Horney, and Sullivan.

Though the followers of these schools generally subscribe to the basic principles of the movement with which they affiliate themselves, wide degrees of difference exist among members of the same school as to interpretation of these principles.

This is to be expected since each therapist will introduce individual unique ideas, many of which may deviate from the classical tenets of the founders of his or her school.

More significant, each therapist will practice with his or her own style, often blending orthodox techniques with those of other schools of thought, sometimes to a point where an impartial observer may not be able to identify the professed ideology.

THE "INDIVIDUAL PSYCHOLOGY" OF ALFRED ADLER Alfred Adler was the earliest contributor of ego psychology.

He is rarely given credit for his pioneer work in child guidance, group therapy, family therapy, community psychiatry, social therapy, and a host of original theoretical ideas that have been widely adopted by the various schools.

About 1910 Adler broke with Freud over the importance of infantile sexuality and the validity of the libido theory.

He insisted that human development was conditioned by the social environment rather than by biological forces.

Because each person was unique, his or her psychology was an "individual psychology." Adler propounded a theory of neurosis based on the idea that behavior must be examined, both historically from the viewpoint of past causes and teleologically from the standpoint of goals. Body and mind were a unity, indivisible and goal-directed, without separation of id, ego, and superego.

This biosocial-psychological approach did not ignore the importance of cause but viewed it in the service of life's objectives.

According to Adler's theory (1917a & b, 1929, 1930, 1938), the basic helplessness of the human infant, magnified by existing body or organ defects, by the child's interpretation of his or her inadvantageous ordinal position in the family, or by parental neglect or rejection, creates feelings of inferiority.

Since inferiority feelings oppose www.freepsychotherapybooks.org 420 security and a sense of well-being, the individual attempts to cope with them by elaborating compensatory attitudes and patterns of behavior.

One extreme neurotic compensation is a "will to power" characterized by irrational strivings for power, dominance, and superiority.

This, the “aggressive” way of dealing with inferiority feelings, in the minds of both males and females is equated with “masculinity.” A “submissive” way of handling inferiority is to conceal, deny, or escape from it through fantasy and rationalization. Unable to gain self-esteem or power through other means, the individual may attempt to achieve objectives by a “flight from reality” and the development of neurotic symptoms. Since the feminine role is associated with inferiority, both men and women exhibit a “masculine protest” to compensate for their feminine characteristics, either by trying to subdue the other or by denying their own sociosexual roles. Sexual symptoms, including the Oedipus complex, are comprehensible only as manifestations of an inferiority-superiority continuum. Adler rejected the concept of “penis envy.” He regarded women’s “masculine protest” often as objecting to their subordinate economic and social roles. The present-day feminist movement is in accord with Adler’s view that cultural rather than biological trends have created inferiority feelings in women. Adler acknowledged the importance but not the primacy of sex roles. The neo-Freudians are in agreement with Adler in the lesser emphasis on the libido. The constellation of impulses, attitudes, and strivings marshalled to overcome inferiority and to achieve power, originally elaborated in relation to significant persons in the environment, are organized into an elaborate “life style” or “life plan” that influences the individual in every dimension. In accordance with the “concept of unity and purpose” there is a total involvement in pursuit of one’s “life style.” This “personal purposive pattern” has for its goal power and social significance that may be crystallized in (1) successful compensation and a good adjustment, (2) overcompensation with various kinds of faulty adjustment, and (3) “Active goals,” such as retreat into illness as a means to power. The improper operation of one’s “life style” may interfere with the healthy growth of the person and with good social and community relationships. www.freepsychotherapybooks.org 421 Adler fostered the idea that the school is the extended arm of the family, and he founded child guidance clinics in the schools of Vienna, instituting a type of family therapy. His pioneer ideas on child guidance and family therapy have not been fully acknowledged. The technique of Adlerian therapy is organized around the exploration and detection of the “life style” of the individual, including the aims, motivations, and compensatory strivings that operate in both negative and positive ways. The past, while considered of historical interest is relegated to a position less important than the present or the future, except for the “earliest recollections,” which are used to identify the source of the “life style.” Once the “life style” is identified, the patient is guided into more effective ways of functioning in order to aspire to greater potentialities through “normal” means. In the course of this educational process, strivings for power diminish and are replaced by social feelings and interests that lead to healthier attitudes toward the self and the community. Social integration is one of the most important facets of Adlerian theory (Gemeinschaftsgefuehl). The basic tools are interpretation of early recollections, the family constellation, and recurrent and recent dreams (Deutsch, D, 1966). Early remembrances help to detect the point at which the biased apperception originated. Family constellation offers a clue to the individual’s perception of his or her role within the family and other social settings.

Recurrent dreams expose and reinforce the patient's self-doubts and vulnerabilities as well as strengths; recent dreams serve as indicators of the amount of insight gained and the degree of therapeutic progress.

The healthy aspects of the patient's personality and strivings toward improvement are emphasized.

The relationship of the therapist to the patient is considered to be of great importance.

The therapist ideally should represent to the patient a benevolent, empathic, trustworthy person, making up for real or imaginary disappointments in early childhood.

A cooperative exploration into the past is encouraged, during which the therapist endeavors to correct the patient's distorted views of past happenings and relationships.

Adler warned against fostering too intense a relationship, finding it more beneficial for the www.freepsychotherapybooks.org 422 patient to keep the interaction on a reality level.

Adlerian concepts can be applied to short-term or long-term therapy, depending on the nature of the disturbance.

The therapist may utilize analytic, active, passive, directive, and nondirective methods.

The course of therapy can extend from counseling and the solving of acute situational problems to a more extensive reconstruction of personality that is intended to lead to self-actualization.

The functioning of the individual in major life areas is used as the yardstick of improvement and readiness for termination.

Termination by the patient, however, even if seemingly premature, is not necessarily interpreted as resistance.

It is accepted that some individuals can change without continued treatment by utilizing on their own the therapeutic insights that they have already gained.

The door is left open, however, so that they can return at any time they feel it is necessary.

In this way people may return to therapy at different critical periods in their lives, beginning with problems in nursery school and continuing through marriage and parenthood. Thus, therapy serves as a support for life, avoiding the danger of becoming a substitute for life.

Adler's theory, according to H.

Papanek (1966), "is based on carefully observed clinical data from which he abstracted broad generalizations.

He was less interested in constructing a tightly fitting system of speculative thinking than Freud.

His aim was to bring together, to achieve a synthesis of the multitude and variety of psychological facts.

This creative thinking and intuitive understanding of the living organism has resulted in a theory which anticipates many viewpoints and hypotheses of present-day psychology: to mention among others Gestalt and Field psychology." Criticisms of Adler's theory and method are organized around the following contentions: first, that only one of manifold human strivings is stressed—that of feelings of inferiority; second, that not enough credence is given to deep unconscious forces; and, third, that goals in therapy tend to be reeducative rather than reconstructive in nature.

Adlerians answer these criticisms by insisting that Adler used terms like inferiority in the broadest sense, including connotations of insecurity and anxiety.

Moreover, results of www.freepsychotherapybooks.org 423 therapy are often reconstructive.

At present, "individual psychology" is practiced by a multidisciplinary group, who, like the Freudian group, are strongly loyal to their founder.

Adlerian associations in New York, New Jersey, Washington, D.C., Chicago, and Los Angeles

maintain training institutes and mental hygiene centers.

There are also important Adlerian groups in Oregon and Wilmington, Delaware.

The parent organization is the American Society of Adlerian Psychology, which conducts annual meetings and publishes *The Journal of Individual Psychology* as well as *The Individual Psychologist*.

Adlerian groups in Austria, Italy, England, France, Holland, Switzerland, and Israel, together with the American Society, form the International Association of Individual Psychology.

Alexandra Adler and Kurt Adler, both psychiatrists and the children of Alfred Adler, contributed to the growth of "individual psychology" through teaching, writing, and leadership in the above organizations, as had their associates Danica Deutsch and Helene Papanek.

Among the better-known contributions to the Adlerian literature are those by Alexandra Adler (1948), K.

Adler and Deutsch (1959), Ansbacher and Rowena (1956), Bottome (1957), Dreikurs (1957), Farau (1962), Orgler (1963), E.

Papanek and H.

Papanek (1961), and H.

Papanek (1965).

Rudolf Dreikurs made a major contribution introducing Adlerian psychology to American educational institutions.

THE "ANALYTICAL PSYCHOLOGY" OF CARL JUNG Freud contended that each individual in childhood repeated in an abbreviated form the whole course of evolution of the human race.

In psychological development each person acted out the racial tragedy of incest and retaliation.

This conflict, being biologically ordained, occurred universally.

It constituted a psychological hurdle that suspended the individual inexorably between passion and reason.

Primary congenital variations, Freud explained, existed in the ego.

"Indeed, analytic experience convinces us that particular psychical contents, such as symbolism, have no other source than hereditary transmission...there are other, no less specialized, deposits from primitive human development present in our archaic heritage."

www.freepsychotherapybooks.org 424 Carl Jung (1916, 1923) expounded these areas in great detail, while rejecting the general phases of Freud's biological and genetic approach for a teleological point of view.

Although he acknowledged the existence of bodily libido, he contended that it is issued, not from the sexual instinct, but from a universal force or "life urge." He recognized that neurotic parents promoted neurosis in their offspring, but he minimized the effect of sexual intimidation as well as the general importance of infantile sexuality.

Observing that the symbolic productions of neurotics and psychotics bore a resemblance to those of primitive people, Jung speculated on the existence of a collective unconscious—a hereditary portion of the mind that contained the imprints of ancestral experience.

A study by Jung of associations, dreams, fantasies, and drawings seemed to substantiate the presence of instinctive thought processes.

These appeared in the form of primordial images, which Jung called "archetypes." Prominent, for instance, was the quadruple "mandala" symbolism, which throughout recorded history appeared as a "magic circle" in legend, art, literature, and religion.

Repetitive configurations of a circle in a square, or a square in a circle, containing groupings of radial or spherical components, the mandala symbol was viewed by Jung as an archetypal manifestation of the collective unconscious that, not based on tradition or model, was determined by archetypal ideas unknown to their creators: "Mandalas are symbols of order,

unity, totality.

As magic circles they bind and subdue the lawless powers belonging to the world of darkness, depict or create an order that transforms the chaos into a cosmos." Among the archetypes that emerge in myths, legends, dreams, and other symbolisms are the earth-mother or witch-mother; the old wise man; the hero figure (such as Hercules, Siegfried, and St.

George); the night journey under the sea (appearing in such forms as Jonah and the whale); the Anima, the mate ideal of the male psyche, and the Animus, the mate ideal of the female psyche. Human beings are influenced not only by values and motives acquired through personal experience (residing in their personal unconscious) but also by the collective experience of the human race embedded in their neural structure.

Jung compartmentalized the psyche in different terms than Freud.

He divided it into a superficial part, the persona, which was a social mask assumed by the individual, made www.freepsychotherapybooks.org 425 up of social interests and sanctions; a less superficial aspect, the ego, which was only to some extent conscious and reflected past personal experiences; and a deeply unconscious part, which had within it the collective unconscious and contained archetypes.

Difficulties developed when an improper balance of masculine Animus elements and feminine Anima elements prevailed.

Difficulties also occurred when there was a lack of harmony among the persona, the ego, and the collective unconscious.

Jung conceived of the idea that baser elements of the soul were present in the collective unconscious, and he characterized these as "the Shadow." He believed also that the collective unconscious contained creative founts of energy.

Primitive fears and other untoward manifestations of the unconscious invaded the patient's conscious mind and created tensions and various neurotic symptoms that were attempts at self-cure.

The collective unconscious, unleashed, constituted a source of danger for the person.

Another area of conflict was residual in the way personality structure functioned.

Jung evolved a theory of character, dividing people into two types: introverts and extroverts.

The introvert's interests centered on himself or herself; the extrovert's interests were on the external world.

Each type was further subdivided into feeling, thinking, intuition, and sensation subtypes.

Problems developed when an individual pursued his or her own personality type or subtype too thoroughly, with extreme inhibition of other reactions.

Complexes were formed by a blending of innate dispositions with external circumstances.

Charged with affect, they influenced the individual for the good or bad.

Only when they became autonomous and operated like separate egos did they threaten the ego-supremacy.

A human being, said Jung, had an innate religious craving which powered the need for self-realization.

There was, he explained, no retreat from life's burdens other than to find refuge in spiritual strivings with "acceptance of the irrational and unbelievable." By experiencing the collective unconscious, an individual no longer would experience personal sorrow "but the sorrow of the world, no longer a personal isolating pain, but pain without bitterness, binding all human beings together." We are brought "back to ourselves www.freepsychotherapybooks.org 426 as an existing, living something, stretched as it were between two worlds of images, from which forces proceed that are only dimly discerned but are all the more clearly felt.

This something, though strange to us, is yet so near, it is altogether ourselves and yet unrecognizable, a virtual middlepoint of such a mysterious constitution that it can demand

anything, relationship with animals and with gods, with crystals and with stars.” These ideas were elaborated by Jung (1961) in his last book, *Memories, Dreams, Reflections*, a fascinating autobiographical document that explains his relationship with Freud and gives us a broader understanding of the essence and meaning of his work.

Jungian psychoanalytic therapy presupposes a basic attitude toward neurosis that is regarded as an attempt by the organism to promote growth as well as illness.

The Jungian process is essentially nonsystematic, although Jung stated that analysis consisted of four stages: catharsis, explanation, education, and transformation.

Therapy involves an exploration, with the help of dream interpretation and art analysis, of various aspects of the psyche, including elements of the personal and collective unconscious. Particularly, an effort is made to explore “archetypes” in order to determine how these imprints contaminate the patient’s present life and interfere with self-development and selfrealization (individuation).

Bringing the individual into contact with his or her collective unconscious is said to help liberate creative forces that will have a constructive effect on adjustment.

Once nonconscious elements are recognized, an attempt is made to guide the patient actively into a productive relationship with the unconscious.

In this way, a balance of masculine and feminine components is restored within the personality. Regressive impulses, such as desires for return to the womb and impulses for rebirth, become dissipated.

For instance, Jung cites the case of a man with panic attacks who produced mandalas in dreams and waking fantasies.

Jung consulted a 400-year-old book which contained a woodcut that was an exact duplication of the patient’s symbol.

Explained Jung, “You see, your dream is no secret.

You are not...separated from mankind by an inexplicable psychosis.

You are merely ignorant of certain experiences well within human knowledge and understanding.”

It may be necessary to help patients work through a long series of mandalas until they stop following an ideal from the past and move toward the world of reality.

Neurosis is related to the hold on the individual of archetype processes.

www.freepsychotherapybooks.org 427 As individuals grope for wholeness and fulfillment, they are said invariably to come upon the ancient geometrical designs and symbols of their evolving selves.

Emphasis in treatment is not only on the unconscious but also on current difficulties.

Dreams, for instance, are regarded as reflecting present strivings as well as future plans of action.

Activity is the keynote in therapy, and the relationship is kept on a positive level, transference neurosis being avoided as much as possible.

Free association is secondary to a focusing of the interview along specific lines.

The therapeutic approach varies with the personality type.

The introvert is presumed to need elaborate coherent interpretations, while the extrovert is said to achieve adjustment on a much more pragmatic basis.

The development of an intellectually satisfying religion is often considered an essential part of therapy, since religion is believed capable of reconciling existing “archetypes” with an ethical system.

In their actual working with patients, modern Jungians proceed along transactional lines, minimizing theory.

A great deal of variability reigns in the way modern Jungian analysis proceeds, with session frequency ranging from one to six times weekly.

Group therapy is commonly employed.

Some Jungians follow the format of classical Freudian analysis and use the couch.

Others use face-to-face interviews.

Artistic productions, sometimes via a sandtray, paintings, drawings, sculptures, and especially dreams are freely utilized for exploration of the unconscious.

The dream is regarded as a vital means not only of approaching the past but also of understanding complexes in the present through a synthesis of the inherent images and associations.

Archetypal motifs are discerned through transference as well as through “amplification” of personal productions with mythological, religious, and anthropological themes.

Countertransference is often considered a positive force that can help in the transformation process, which ultimately is directed toward greater individuation.

The abandoning of therapist authoritarianism and the use of the vis-à-vis position, as opposed to the couch, expedite rapport and even reactivity on the part of the analyst.

The effectiveness of therapy stems “from the reality-based interpersonal exchange between www.freepsychotherapybooks.org 428 analysand and therapist.

This also means that the two people in the transaction are partners, jointly engaged in a struggle to explore and dispel the neurosis that affects the patient, including the resolution of the transference.

It involves making an alliance with the healthy aspects of the patient ... [which] need to be activated and encouraged throughout the therapeutic procedure.

...

It seems to me important to hold theory in abeyance so that nothing will stand between patient and therapist, and interfere with the immediacy of contract” (Wheelwright, 1956).

Criticisms of the Jungian approach relate to its metaphysical content, to the religious-like elements with which therapy is imbued, and to a tendency for some patients to become preoccupied with a mystic philosophy toward life and with speculations of “archetypes” and other manifestations of their racial past.

On the whole, however, Jungian therapy achieves results comparable to those of other analytic treatments.

Jungian principles have recently gained great popularity in the United States, possibly because of the enhanced interest in mysticism.

Jungian centers exist in New York City and on the West Coast.

Jungian therapy is practiced in various parts of the world, drawing its inspiration from the writings of its founder (Groesback, 1983).

THERAPEUTIC MODIFICATIONS OF SANDOR FERENCZI Sandor Ferenczi (1950a, b, c, & d, 1952), v remaining loyal to Freud’s theories, introduced certain modifications of method.

Finding that transference did not develop readily in many patients, Ferenczi advocated “active” therapy in the form of an embargo on physical and sexual gratifications.

He believed that this restriction would block libido and make it available for projection into the transference.

The patient was consequently enjoined to abstain from sexual satisfactions of all kinds, and even to limit toilet activities.

The release of resentment and aggression directed toward the therapist was felt to be of therapeutic importance.

Experience soon proved the method to be of little value, however, and Ferenczi himself abandoned it.

Instead, he substituted a completely permissive atmosphere by acting as a tolerant “good” parental figure who acceded to many of the patient’s wishes and demands.

He urged that the therapist admit his or her own faults and www.freepsychotherapybooks.org 429 shortcomings to the patient in order to convince the latter that all authority was not harsh, intolerant, nor incapable of admitting to failings.

A mutual analysis often resulted.

To help patients to an awareness of their past, Ferenczi also encouraged them to relive it by dramatizing childhood situations, while he remained tolerant to the patients' "acting-out."

Thus patients would relive their childhood, play with dolls, and engage in baby talk, while Ferenczi joined in the play.

He regarded relationships as a two-way interaction, and his writings stimulated interest in what later was expanded by Sullivan in the concept of the interpersonal relationship.

Ferenczi recognized that the analyst's personal feelings about the patient and attitudes toward the patient often determined the nature of the latter's reactions.

He advocated that the therapist consider the patient's responses as being conditioned both by transference and by reality provocations for which the therapist should admit responsibility.

Collaborating with Rank (1925), Ferenczi advised setting a time limit to analysis as a means of accelerating the end of treatment.

Ferenczi's technical innovations are still employed by some therapists, some of whom chance upon identical methods and then claim originality for them.

THE "WILL THERAPY" OF OTTO RANK Otto Rank was responsible for a number of important technical innovations, and, with Stekel, may be considered an innovator of short-term psychoanalytic therapy.

He advocated a flexible, active, patient-oriented treatment process, with the patient determining the particular mode of reaching self-direction and self-determination.

Some of the present-day transactional approaches draw their tactics from Rank, while clientcentered therapy appears to have absorbed much of his philosophy regarding "selfrealization." The functional school of casework was also oriented around dynamic concepts of what Rank called the "helping process" (Taft, 1933, 1948; Allen, F. 1942; Kasius, 1950; Karpf, 1953).

To Rank (1929, 1947), Freud's original idea that the process of birth, with forceful separation of the child from the mother, constituted a trauma from which the individual www.freepsychotherapybooks.org 430 never recovered was of crucial importance.

Two sets of strivings resulted from the "birth trauma": the first, an impulse to return to the womb in order to restore prenatal conditionings of security; the second, an impulse for rebirth or separation from a maternal object so as to achieve independence.

The first group of impulses stimulated the establishment of relationships of a dependent, infantile, and clinging nature.

The second group appeared as a "will" to grow, to achieve "individuation," and to separate oneself from confining relationships.

The life of the person was governed by these contradictory strivings to unite and to separate. The primordial anxiety of separation from the mother, rooted in the original birth trauma, was revived at all subsequent experiences of separation, such as weaning, castration threats, and removal from close relationships with people.

The need to restore unity with the maternal figure was contained in a desire to submit oneself in human relationships, including sexual relationships, while the need for assertive individuality was residual in an impulse to fight off the desire to unite with another person.

Separation anxiety manifested itself (1) as the "life fear," when "the person recognized creative capacities within himself which would threaten to separate him from others lighting up the fear of having to live as an isolated person," and (2) the "death fear," manifested by terror of losing one's individuality and being swallowed by others.

Rank classified personality into “normal,” “neurotic,” and “creative artist” types.

“Normal” people subordinated their own will to that of the group in contrast to “neurotics” who refused to yield their will to the group.

This reluctance, coupled with an inability to break out of the trap between dependency and individual autonomy, made it impossible to utilize their will in the direction of developing into “creative artists.” Thus, unable to achieve latent creative aspirations and the will to be themselves, they were forced to evolve their own standards and to stand alone if necessary. Rank emphasized that the analytic hour offered the patient a unique opportunity to live through with the therapist past experiences, particularly the birth trauma, and to move toward a more complete individuation.

Psychotherapy was thus essentially a www.freepsychotherapybooks.org 431 therapist-patient relationship during which the therapist as a helper mobilized the patient’s “creative will impulse” for positive self-realization.

The relationship itself served as a corrective experience for the patient, revelation of unconscious material and insight being secondary.

Patterns of reaction rather than specific content were to be studied.

The therapeutic situation was to be adapted to the unique and individual needs of each patient.

Rankian analysis, according to Karpf (1957), “must be patient-centered, not therapist-centered; it must be flexible, adaptable, alert to the new and unexpected—a genuinely creative experience for both patient and therapist.

...

In essence it is a view which has a special appeal to non-authoritarian therapists who respect patients as self-reliant and self-responsible persons... .” The process of therapy was entirely centered in the patient-therapist relationship, the focus being on the patient’s feelings toward the therapist.

The analysis was an experience from which the patient would eventually separate and then go on to a new experience.

The reactions of the patient to the inevitable circumstance of separation were studied carefully, with the object of working through fear of, and guilt toward, the separation as well as needs to control and to be controlled.

The struggle of the will was also studied as it was reflected in the desire to continue therapy and to be dependent as well as to discontinue treatment and to separate oneself from a dependent relationship.

The emphasis in therapy, thus, was on the present rather than the past.

Activity in treatment was the keynote, the patient being encouraged to assert himself or herself in order to develop and strengthen his or her own will.

An effort was made to mobilize constructive elements in the personality and to transfer the negative expression of will into positive and creative will.

A time limit to therapy was usually set.

This was believed to act as a catalyst to the union-separation conflict.

No effort was made to bring out sexual material.

Resistance was accepted as an inevitable expression of the will.

It was not met with counterresistance or explanatory interpretation.

Transference was also accepted as an aspect of the growth process in which there was a strengthening of the “will” to be oneself.

www.freepsychotherapybooks.org 432 Guilt and fear were gradually resolved through this experience in the relationship, which liberated the will from its one-sided expression and resulted in a “utilization of its own contrariness.” The acceptance of responsibility for one’s ambivalence reduced guilt and fear to a point compatible with living.

The patient eventually learned to tolerate separation from the therapist and with strengthened will achieved independence and growth.

Certain Rankian principles have been incorporated into a number of psychotherapeutic approaches, including psychobiological therapy, client-centered therapy, neo-Freudian analysis, short-term dynamic psychotherapy (Mann, 1973), and psychoanalytically oriented psychotherapy. The theory of the birth trauma is not as generally accepted as are Rank's concepts of therapy, particularly those that deal with the importance of the patient-therapist relationship as a positive growth experience.

An Otto Rank Association exists in Doylestown, Pennsylvania, under leadership of Virginia Robinson and Anita Faatz, and a biannual Journal of the Otto Rank Association is published. As social work has shifted from clinical to community work, the once prominent functional casework method, drawing its inspiration from Rank, has lost its appeal.

His ideas on short-term therapy, however, are undergoing a revival.

Criticisms of Rankian analysis are expressed by those who object to the activity involved and to the focus on union and separation to the neglect of other personality aspects.

The abrupt insistence that the patient stand on his or her own feet under any circumstances is said to have an unfortunate effect on persons with weak ego structures who may require a great deal of support in early phases of treatment.

THE "ACTIVE PSYCHOANALYSIS" OF WILHELM STEKEL Wilhelm Stekel (1950) is responsible for a number of original techniques in analytic psychotherapy that have been assimilated in many current systems of short-term psychotherapy (Lowy & Gutheil, 1956).

Possessed of an uncanny intuition as well as an active personality, Stekel related himself perceptively and rapidly to the immediate problems of his patients, and he utilized himself forcefully to bring them to an awareness of their conflicts.

He was particularly interested in sexual disorders ("paraphilias"), and his volumes on sadomasochism (1929), fetishism (1949), impotence (1927), frigidity (1926), and exhibitionism (1952) are best known.

His book on psychopathic behavior, which he called "impulsions" (1924), is also popular.

While Stekel retained Freud's basic concepts of unconscious conflict, transference, and resistance, he believed that the libido theory did not explain the multiform conflicts of the human mind, and he felt that the castration complex was not nearly as ubiquitous as Freud had assumed.

He declared that current life conflicts were as important as past conflicts and that absolute unconscious elements were not the only foci of neurosis; rather, aspects of the conscious mind might be repressed and transformed by a wide variety of symbols.

Stekel emphasized certain formulations, such as the "central idea of the neurosis," which varied in each person, and the inevitable anchoring of mental conflict in the immediate life situation.

Emotional disturbance was often a product of competition of inharmonious "motives." Glimpses of these might be captured from dreams and free associations.

Anticipating later characterologic approaches, Stekel remarked that the future of analysis was residual in an analysis of character.

The chief contributions of Stekel, however, were in the field of technique.

Perhaps the greatest of these was in the area of dream interpretation, his studies on symbolism being acknowledged by Freud as unique.

In employing dreams, Stekel enjoined his patients to bring them recorded to each session.

At first, resistance elements were discussed in terms of the patient's desire to remain ill or to avoid facing reality.

Then the therapist applied himself to the patient's past involvements and unconscious designs

for the future.

His flair for symbolism enabled him to pinpoint important conflicts that, sooner or later, were brought to the attention of the patient.

Stekel often remarked that it was not the particular method that cured but rather the personality of the therapist employing it.

He stressed what we now call "countertransference." The general therapeutic formula propounded was this: "Recall www.freepsychotherapybooks.org 434 what originated your trouble, recognize your morbid attitude, and surmount it." Stekel alleged that resistance prevented the patient from recognizing morbid attitudes, from recalling their origins, and above all, from surmounting difficulty.

Resistance, however, could not be resolved by the orthodox analyst's manner of remaining a passive spectator to the patient's free associations and dreams.

This was the fallacy of passive psychoanalysis.

The therapist must actively interfere in breaking up repression.

It is necessary to collaborate actively with the patient in the interpretation of free associations and dreams.

Through "sympathy" and "imaginative insight" (qualities Stekel subsumed under the term "intuition") the therapist must be alerted to repressed complexes and must intervene actively to make the patient aware of them.

The intuitive facility with which the symbolic meaning of the neurosis was determined and the skill with which interpretation was offered influenced the speed of therapy.

The therapist's main function, then, was as an intuitive artist probing his or her way into the psyche.

Stekel admitted that active analysis presupposed that the analyst was endowed with an intuitive faculty, but he avowed that this faculty was more widespread than had been presumed.

Abandonment of the analyst's passive role was associated with activity and directiveness, even to the point of advice giving and exhortation.

While free association was utilized, the patient could not be permitted to ramble along into blind channels; instead, selection of pertinent topics for discussion was in order.

The use of the face-to-face, sitting-up position was also advocated.

Emphasis was put on the interpretation of dreams, but the therapist's intuition had to be relied on in order to divine what eluded free associations to the latent content.

The use of adjuvants in therapy was also indicated; Stekel insisted that it was essential to adopt methods to the particular case rather than to force the patient to abide by a particular method.

With active methods Stekel believed that it was rare for more than 6 months to be required for analysis.

This short period avoided interminable analyses and prevented the development of untoward, ill-fated, unmanageable reactions.

The importance of transference was recognized by Stekel as an essential part of every analysis. Indeed, he felt that analysis was impossible without transference.

www.freepsychotherapybooks.org 435 Transference, however, could serve two functions—that of expediting therapy and that of acting as resistance to therapy.

Only when transference functioned as resistance was its handling justified.

A frequently heard criticism of Stekel's system of psychotherapy is that the activity of the therapist may sponsor an excessively disciplinary, prohibitive, and punitive attitude toward the patient.

Objections are also expressed in regard to the maximal 6-month time limit to therapy on the basis that character reconstruction is long-term.

The few followers of Stekel who remain have revised his temporal limitation and often do long-term therapy.

The “intuitive” aspects of active analysis may also come under questioning.

Many observers would contend that what makes a therapist intuitive is a high degree of sensitivity that enables the therapist rapidly to perceive nuances in the interpersonal process and, on the basis of extensive experience, to translate these into valid deductions.

Not all persons, however, would be capable of doing this, irrespective of the extent of training and experience.

The intuitive aspects of the Stekelian system, therefore, would be limited to a restricted number of “intuitive” analysts, who could hypothesize constructs with a high degree of probability.

A less intuitive analyst who tried this would indulge in guesswork that might be disastrous to the therapeutic objective because it would foist faulty interpretations on the patient.

To use Stekel's system effectively, then, the therapist required an extremely high degree of analytic training, an extraordinary flexibility in personality, a deep sensitivity that would enable the perception of nuances, and, above all, good judgment, which would permit the judicious employment of active procedures.

At the present time, Stekelian analysts are not organized into a special analytic school.

There have been many modifications of the Stekelian method as Stekelian analysts have introduced into their work contributions from various other psychiatric groups.

THE "CHARACTER ANALYSIS" AND "ORGONE THERAPY" OF WILHELM REICH The basic theoretic of Wilhelm Reich at first followed along the lines of Freud's earliest formulations of the libido theory.

This contended that neurosis was due to www.freepsychotherapybooks.org 436 a conflict between repressed instinctual desires—usually infantile sexual desires—and ego-repressing forces.

The resulting conflict produced a stagnation of libido that was converted into anxiety and that subsequently engendered neurotic symptoms or neurotic character traits.

Therapy involved the making conscious of unconscious conflict in an effort to liberate strangulated libido.

Defensive forces of the ego, however, acted as resistance to the return of the repressed.

Before unconscious elements could be restored to awareness, it was essential to eliminate resistances.

Through interpretation, patients were helped to see how their resistances operated, their nature and purpose.

Character formation was conceived of by Reich as a kind of psychic armor that protected the individual from the disturbing stimuli of the outside world and from inner libidinal strivings (Reich, 1927, 1928).

During psychoanalysis the patient's character served the interests of resistance against the repressed.

Before one could tap the unconscious, therefore, it was essential to break down “character resistances” until the individual was denuded of defenses that barricaded repressed material. Character resistance revealed itself in attitudes and behavior toward the analyst and the analytic situation.

Reich, in his book, *Character Analysis* (1949), described the analysis of resistance, including character resistance, and he insisted that this was necessary before the patient could accept and integrate the content of the unconscious.

Four main types of character defenses were apparent.

First, the hysterical character, fashioned by a defense against incest, was manifested by a passive-feminine orientation with subversive expressions of sexuality at the same time that an

open avowal of sexual interest was avoided.

Second, the compulsive character, determined by a defense against sadistic and aggressive impulses, displayed a penchant for orderliness, cleanliness, and thriftiness.

Third, the phallic-narcissistic character, conditioned by a defense against passive-feminine tendencies, exhibited a cold, arrogant, and derisively aggressive manner; active homosexuality and schizophrenia could develop in such individuals.

Fourth, the masochistic character, which was sponsored by a defense (substitutive punishment) against fantasied recriminations from the conscience, avoided anxiety by forcing others to treat him or her badly so as to be able to reproach them while experiencing self-damage www.freepsychotherapybooks.org 437 and self-depreciation.

The masochist wanted to be loved but disguised this in grandiose provocations of the love object.

Reich also classified character types according to how they managed to resolve the problems existing during earlier stages of their development.

In terms of libido, an oralreceptive character existed as a sublimation of the earliest sucking stage; he or she was friendly and optimistic and looked upon the world and people as mothering objects.

The oral-aggressive character, a sublimation of the oral biting stage, manifested itself in aggressiveness, envy, ambition, and a need to exploit others.

The anal character, a sublimation of the anal period, was pedantic, overly clean, and miserly.

The phallic character sublimated the phallic stage through cold, arrogant behavior.

The urethral character, often showed symptoms of bed wetting and a burning ambition and was boastful about his or her achievements.

The genital character was mature and capable of relating in an adult way to others.

Reich, like Trigant Burrow, pointed out that body tensions were evidences of emotional states and reflected characteristic ways of reacting.

Posture, gait, facial expression, and other muscular manifestations revealed certain resistances and had to be attacked by calling attention to them before formal analytic procedures could be effective.

Although the theoretical basis for Reich's method has been discounted by many analysts, the techniques of character analysis have proven themselves to be invaluable.

Followers of the "dynamic-cultural" school, particularly, find the analysis of character vital, apart from its resistance-disintegrating virtues.

Emphasis is placed on the neurotic nature of character trends, analysis of which constitutes a chief objective rather than a means to deeper repressed material.

Reich, however, coincident with his reported discovery of a "cosmic substance, orgone" tended later to depreciate his contributions on character in favor of a newer "physiological" orientation to therapy (1942).

The establishment of orgasmic potency was felt by Reich to be the most important goal in therapy.

He described the therapeutic process as a consecutive loosening of the character armor, a breakthrough of repressed www.freepsychotherapybooks.org 438 and affect-laden material released by activation of infantile sexual conflicts, a working through of infantile genital anxieties, a dealing with orgasm anxiety, and, finally, a developing of full orgasmic potency.

Character analysis in itself, however, he claimed was incapable of achieving the desired goal of orgasmic potency.

This was because another form of armor besides character armor shielded the unconscious.

This was "muscular armor," which, in the form of chronically fixed muscular attitudes, increased tonus and rigidity and shielded components of sexuality and aggression from awareness

Therapy, to be effective, had to provide for a loosening of the muscular armor.

These new ideas made necessary a reformulation of his hypothesis.

Character was developed from a binding of "bio-energy." Therapy remobilized "bio-energy" from character armor through character analysis, and from muscular armor through "vegetotherapy." The resulting liberation of emotions produced a mobilization of "orgone energy," which vitalized orgasmic potency.

This was "orgone therapy," and it reached the biological depths of the human being, bringing an awareness of both organ sensations and muscular armoring, with an eventual destruction of the armor, a reestablishment of "plasma motility," and an appearance of "orgasm reflex." In orgone therapy the muscles of the patient's back, chest, jaw, abdomen, and extremities are pressed firmly by the therapist to elicit emotional reactions and to liberate associations and memories.

The patient's reactions are then interpreted.

Sometimes the therapist imitates the patient's mannerisms of behavior or encourages "acting-out" tendencies.

Verbalization of fantasies, memories, and feelings associated with the "muscular armor" is said to dissipate the armor and to allow the patient to deal with direct impulses, of which the muscular manifestations are defenses and resistances.

Although character analysis has gained wide acceptance among many analysts, the validity of the theory and technique of orgone therapy is generally considered controversial; it is currently practiced under the term "vegetotherapy" (Konia, 1975).

Alexander Lowen (1958), while breaking away from some formulations of Reich, has elaborated his own techniques of releasing energy through muscle activity ("biowww.freepsychotherapybooks.org 439 energetics") and thus relating character disturbances to muscle armoring.

ADAPTATIONAL PSYCHODYNAMICS AND PSYCHOANALYSIS A biologically oriented theory of human has been developed principally by Rado (1939, 1949, 1950, 1956, 1962), Rado and Daniels (1956), Kardiner et al.

(1945), and D.

Levy (1956).

It deals with the functions of the ego and the mechanisms that are involved in the process of adaptation.

Essentially the theory purports that the individual is constantly in a state of shifting homeostasis.

Physiological drives and social needs must constantly be satisfied in order for the organism to survive.

Unfulfilled strivings generate tensions and upset the homeostatic balance.

This gives rise to a series of mechanisms, which Kardiner calls "action systems," conditioned by past successful activities toward satisfying the same drives and needs.

As gratification takes place, emotionally charged memory traces are recorded that will later be activated when homeostasis is again upset.

At the same time, feelings of mastery bolster the self-image.

"Perception of the sources of stimulation changes according to the degree of gratification and comfort which has been achieved" (Karush, 1961).

Rado's structural model of adaptational psychodynamics introduces several gradations of psychological defense associated with different levels of nerve integration.

The most primitive, the "hedonic" level, which phylogenetically antedates all other levels, deals with pain and pleasure and is activated by physiological and instinctual tension states.

The second division is the "emotional" level, which registers itself in four basic effects: rage, fear, grief, and love.

This is a more advanced phylogenetic level and is associated with greater capacities in the organism to alter his or her environment.

The next stage of integration incorporates the cerebral cortex and replaces emotion and “emotional thought” with higher-thought “self-attributive” processes.

This supports the ability to anticipate and organize means of attack or withdrawal.

The emotional and thought levels constantly intermingle.

Feedback occurs among all three levels of defensive integration.

Stimulation of pain thus arouses the hedonic level, but it also activates the

www.freepsychotherapybooks.org 440 emotional and thought levels.

This permits anticipatory and adaptive reactions.

According to Rado, neurosis is the product of faulty responses of the organism to danger registered as “failures in emergency adjustment.” A signaling arrangement, evoked by any kind of pain threatening the organism or by the anticipation of pain, is the basis for the development of “emergency behavior.” Of all motivations, emergency behavior is the strongest: it takes precedence over any other motivation.

In disturbed emotional states, however, this rule may be violated.

On the “hedonic” level of organization, pain evokes the “riddance response” in the form of physiological reactions to rid oneself of offensive agents.

Vomiting, diarrhea, spitting, sneezing, and coughing are manifestations of these reactions.

The psychic correlate of the “riddance principle” is the mechanism of repression.

Fear and rage are conditioned by an anticipation of pain.

They warn of impending damage, and they inspire protective responses of flight (in fear) and of fight (in rage).

On a social level, flight may be into dependency relationships with, and submission to, authority; fight may be expressed in terms of defiance of authority.

On the level of “emotional thought,” emotions are tempered to some degree with ensuing apprehensive and angry thought patterns.

Discrimination and analysis promote greater flexibility of performance.

The person is still subject to escape and combat mechanisms, however, in response to painful stimuli.

Basic emergency reactions on the level of “unemotional thought” are controlled to some extent by the intellect.

There is an advance detection of threats to the organism with an appraisal of its powers to cope with the threats.

On the highest or “self-attributive” level, heightened pride accompanies rage responses or awareness of self-strength, while diminished pride follows fear reactions or awareness of self-weakness.

The development of the individual’s conscience issues out of disciplinary rewards and punishments in relation to parents.

Fear of punishment and the restraints it inspires become automatized, and in adult life continue in force.

Obedience and the moral pride consequent to it also persist as adaptive patterns.

Fear of one’s conscience is a residue of fear of parental punishment.

www.freepsychotherapybooks.org 441 Temptation may release rage and defiance, however, which, by overwhelming the fear of one’s conscience, may drive the individual to disobedience.

A fear of condign punishment may then eventuate for such defiance.

This may lead to a desire to reinstate oneself.

Defiant rage, consequently, may be turned inward with self-reproach, confession, remorse, and pleas for forgiveness.

The hope is to be restored to the good graces of the parents.

This expiatory pattern may become fixed so that the individual seeks forgiveness by self-punishment.

More pernicious are the phenomena of self-punishment for imagined guilt and of advance painful punishment as a release for gratification of forbidden desires.

Sometimes rage breaks loose with an abandonment of self-reproach and an attack on the person who is feared.

Failures of emergency control may be caused by an overproduction of fear, rage, and pain.

Resultant are overreactions to existing danger and emergency responses in the absence of real danger.

An overproduction of emergency emotions may express itself in an outflow of emotions.

To stop the overproduction, the organism may have to resort to repression and other automatic "riddance" mechanisms.

All disordered behavior is the consequence of such failures in emergency adjustment.

Rado, in his later contributions, has continued to stress that the human being is by nature equipped with a survival mechanism that operates through patterns of adaptation.

These change with age in both strength and form.

Schematically, the period of youth is characterized by dependent patterns; the adult period, by patterns of self-reliance; the period of aging, by patterns of declining adaptation.

In early life survival dictates subordination of inner desires to parental demands.

Tactics to mollify or to deceive or to coerce, and expiatory and aggressive modes of coping with actual or anticipated rejection, may be evolved.

Cravings for magical fulfillment of dependent needs occur with refusal to accept the reality of limitations in parental powers.

Civilization dictates a taming of the child's rage response, and disciplines are imposed by the parent to contain the child's reactions and to bring them under control.

The mechanism of conscience is built up on the basis of these conditionings, and it helps in fostering the development of responsible independence.

This sponsors emergency control, realistic thinking, and behavior replacing

www.freepsychotherapybooks.org 442 emotional thinking and activity; self-reliance displaces dependency.

The implications of adaptational psychodynamic theory for treatment stem from its stress on ego functions.

This supports the contention that all material uncovered in analysis must be related to current problems of adaptation.

In an emotional matrix of "welfare emotions" (love, compassion), controlled by reason ("adaptive insight"), therapy is directed toward supportive, reeducative (reparative), or reconstructive goals depending on the patient's level of motivation.

Four categories of motivation are apparent that will determine what kind of technical processes are best adapted to the individual.

These motivational forms are allied to stages of personality development (Rado, 1965).

The first level, "magical craving" for an idealized parent, will make possible such suggestive techniques as hypnosis.

The second level, characteristic of a somewhat higher stage of development, "parental invocation," also addresses itself to limited goals, permitting persuasive and educational measures.

The third level, that of "cooperative striving," and the fourth level, of "realistic self-reliance," permit techniques geared toward reconstructive objectives.

It is possible through therapy to bring patients from a limiting motivation to one that is more

mature.

When the patient is motivated to develop, reconstructive therapy, which deals with inner conflict, may be employed with the objective of bringing the patient to the highest degree of self-reliance.

A prime focus here is recognition and resolution of the patient's rage, expressed or repressed, as well as excessive fear of punishment.

Awareness of how rooted present-day behavior is in childish experience and misconceptions is an essential ingredient in reconstructive therapy, although the value of memory probing and the focus on insight is questioned.

During treatment the therapist constantly must be on the alert for regressive thrusts toward dependency, helping the patient return to a self-reliant motivation.

Indeed, advantage is taken of the patient's dependency needs to promote emotional learning through educational tactics.

The therapist constantly interprets the difference between realistic and infantile aspects of the patient's behavior in and out of the treatment situation.

Rather than considering transference as a manifestation of the repetition compulsion as in classical theory, it is regarded as arising www.freepsychotherapybooks.org 443 from the patient's reacting regressively (through a parentified relationship) to faulty emergency responses as a result of a present failure in adaptation (Ovesey, 1954, 1965).

The tracing of the origins of faulty emergency reactions and the "emotional redefinition" of memories are aided by proper interpretations.

Transference is handled by helping the patient to recognize that helplessness and anger in relation to the therapist are really directed toward past personages.

The patient is encouraged to reproduce in memory the actual rage-provoking scenes "with the original cast," to be self-reliant, and to expand a realistic adaptation.

Dreams are considered to be compensatory reactions to unresolved conflicts from the previous day, a "pressure gauge of the patient's latent emotional tensions." Activity by the therapist is encouraged, particularly in an educational role.

Critics of adaptational psychodynamics contend that, in their attempt to reinterpret Freudian formulations, adaptational theorists have succeeded merely in introducing further metaphysical concepts into the literature rather than in contributing to a greater understanding of psychodynamics.

The active handling of transference is exprobrated with the contention that self-reliance can be brought about more readily by a noncontributory and nonstructuring therapist.

"Priming" the patient's reactions toward operating on a more mature motivational level is also disputed as more ego reinforcing than analytic and hence more in keeping with reeducative rather than reconstructive goals.

On the whole, however, there is a general consensus that adaptational psychodynamics has introduced a fresh way of looking at the phenomenon of adaptation.

It has also evolved some special techniques for helping the patient arrive at distortions in ways of relating.

Particularly interesting is its approach to sexual problems such as homosexuality, cravings for omnipotence, and strivings for power and dependency (Ovesey, 1954, 1965).

Rado's adaptational theories have been utilized as a means of approaching studies on the brain physiology of behavior (Heath et al, 1974).

The theoretical concepts of the adaptational school of Rado have been taught at the Psychoanalytic Clinic for Training and Research at Columbia University.

THE "DYNAMIC-CULTURAL" SCHOOL OF PSYCHOANALYSIS www.freepsychotherapybooks.org 444
theoretical speculations were originally based on the investigation of symptoms and other

“ego-alien” phenomena.

During psychoanalytic treatment the consideration of resistances and the mechanisms of defense focused investigations on certain “ego-syntonic” transpirations, such as character manifestations.

Concentration on dynamisms employed by the ego in its adjustment led to the development of “ego psychology.” The contributions of Reich (1949), Fromm (1932, 1936), and Anna Freud (1937) have been utilized by sociologically minded professionals, who, forming what has come to be known as the “dynamic-cultural” school, have made a significant contribution to ego psychology.

This school is characterized by a shift in theoretical emphasis from biological to sociological events, from concern with past experiences to the patient’s present-day contacts with people, from consideration of the vicissitudes in sexual development to character patterns that, though of early origin, influence current interpersonal relationships, from preoccupation with fixations of libido to concentration on growth and maturation.

It is the contention of the “dynamic-cultural” school that Freud (1947) confused cultural phenomena with biological instinctual manifestations.

Challenged also is the sexual nature of infantile urges.

Orally centered activities of the newborn infant, for instance, are believed to stem not from an urge for erotic satisfaction, but rather from the fact that the mouth and the cortical area governing the mouth are more highly developed at birth than any of the other bodily areas. The oral zone, consequently, serves as a primary means of contact with the world.

The shift of interest to the anal area is regarded not as a biological transfer of libido to this locality, but as a pattern characteristic of the emphasis put on toilet training in Western civilization.

Instead of pleasures in fecal retention or excretion, the focus is put on struggles with parental disciplines.

The phenomena of the latency period are also said to be culturally determined, in that field studies of anthropologists demonstrate the absence of a latency period in some societies.

The Oedipus complex is regarded as a neurotic reaction, a consequence of incongruities in a monogamous patriarchal society.

Sexual feelings in the child for the parent are excited www.freepsychotherapybooks.org 445 when neurotic needs cause the parents to overstimulate and overfondle the child.

The child’s responses are provoked by attitudes of the parents, punitive or disapproving reactions precipitating exaggerated fear of the loss of love and terror of castration.

Penis envy is explained not by a craving on the part of the girl for a penis, but rather by the desire for privileges that masculinity awards the individual in our culture.

The child’s reactions during puberty are also determined by cultural factors.

For example, homosexual interests are much less where boys and girls are freely allowed to relate to each other.

The resentment a girl feels at accepting her femininity is stimulated not by a need to renounce interest in being a girl or in transferring clitoral to vaginal pleasures, but rather by pressures and demands put on her by the environment because she is a girl.

Experiential and sociological factors, rather than biological influences, are thus regarded of prime etiological importance in conflict formation.

Fundamental to a human being is an inner urge to fulfill potentialities and thus achieve “real self.” The individual is blocked, however, in reaching his or her goal, both by destructive factors in personal upbringing and by repressive elements in the environment that put a taboo on certain traits, such as lovingness and tenderness, and that encourage other traits, such as hostility and aggression, that may alienate the individual from others and from self.

The “dynamic-cultural” school accents character structure above all other aspects of

personality.

Character, the fusion of conditionings with the constitutional makeup, is organized in complex behavior tendencies that regulate one's relationships with other people and the environment. Most character strivings pattern themselves around the demands of the culture as vested in the disciplines, prohibitions, and commands of the parents.

Among their aims is propitiating needs for security and self-esteem.

Tension and anxiety may result should a character drive fail to function or should one important drive conflict with another.

Distorted character drives make for defects in interpersonal relationships and oppose normal biological and social needs.

In this way, they are considered by the dynamic-cultural school to be the core of the neurotic process.

Erich Fromm (1941, 1947, 1950, 1955, 1959a, b, & c) contributed to the dynamiccultural school a view of human behavior from the perspective of social psychology.

www.freepsychotherapybooks.org 446 Unlike Freud, he did not believe that there was an essential disparity between the individual and society, that human nature was evil in essence, or that civilization was the product of the sublimation of instincts.

Personality, though circumscribed by human biology, is not created by it.

Individuals, according to Fromm, reflect the values of their society.

Since their needs are economically dependent for fulfillment on interaction with others, individuals must pattern themselves by, and their character structure then reflects, the conditions under which they have to live.

They must give and take, "buy and sell" by the rules of their social system.

Modern industrial society resists any attempt to impose on it a rational order.

This mobilizes aloneness and helplessness in the individual that forces the adoption of such psychic mechanisms as destructiveness, sadism, masochism, and conformity, through which the individual attempts to relate to people and to the world.

A person's relationship with society, the basis of one's conflicts, is never a static one: it is constantly changing with the social process.

Traits, such as drives for power and puritanism reflect this change.

This is why "human nature" has shifted in its form at different historical epochs.

A person, Fromm insisted, is not an innocent victim of sociological forces: his or her energies are constantly molding society to the special needs of one's personality.

Inherited behavior patterns are present in lower animals, but such behavior has been replaced in higher animals by learned patterns.

Inherited drives, such as hunger and sex, are present, but the extent and means of their satisfaction is determined by the culture.

Moreover, the culture creates new needs that may be more powerful than biological needs, for example, loyalty to one's country or religious piety.

Unlike other animals, a human being is both blessed and burdened by a sense of awareness.

The capacity for symbolization enables a person to store past experience and to project beyond the range of the senses into the future.

The environment is so complex that one cannot be bound by the fixed solutions of instinct; reason and imagination are the chief means of adaptation.

Yet awareness of one's helplessness in the massive universe and the inevitability of death imposes on existence a sense of futility and adds an "existential dichotomy" to the "historical dichotomy" (war, poverty, disease, etc.).

One may rebel against contradictions of our society, or one may try to adapt by rationalizing or denying www.freepsychotherapybooks.org 447 them, soaking up the ideologies of the ruling

classes who have a stake in maintaining the status quo.

To establish one's equilibrium in the face of conflict stirred up by this inescapable dilemma, the individual reaches for a frame of orientation in some philosophy or for devotion to some religion, supernatural or secular.

This enables the individual to relate more easily to the world, to people, and to self.

Instead of religion being a universal neurosis, as Freud believed, neurosis becomes a kind of personally designed, disorganized, and potentially disorganizing religion.

Social forces thus encourage irrational mechanisms in a person's relationships with the group and promote the isolation of the person from others.

A primary need is for closeness with, and approval from, a significant individual.

Fear of disapproval from this individual, as originally was the case with the parental agency causes the person to deny or to repress any feeling, impulse, attitude, or reaction that inspires disapproval, no matter how constructive or important it may be.

A number of character strivings are elaborated to cope with the reactions of the significant parental figure.

Fromm conceives of character types as rooted in certain fundamental attitudes, such as dependent, masochistic, exploitative, (aggressive, sadistic, power-driven), hoarding (meticulous, pedantic), marketing (opportunistic), and productive (loving, mature), conditioned by experiences with parents who exerted a specific influence over the child that led one to develop these attitudes as security mechanisms.

Basic anxiety issues form a conflict between a need for approval from a parental figure and a need for independence.

Fromm emphasizes the value in therapy of discovering what healthy aspects of self have been eliminated as a result of environmental restriction or condemnation.

The therapist helps the patient to understand and to rectify the need to cling to irrational authority, with the end result of encouraging a character organization that permits the patient to relate to the group in a healthy and productive way.

Learning the difference between rational and irrational authority through the relationship with the therapist is the essence of the therapeutic process.

This releases the patient's potentialities for fulfillment through relationships with other human beings; it resolves neurotic strictures on productivity and creativity.

The goal of psychotherapy is not too different from that of a www.freepsychotherapybooks.org 448 constructive religion.

The latter, an affirmation of a human's faith, which is a basic requirement for human existence, like psychotherapy encourages self-actualizing tendencies.

Psychotherapy and religion are thus mutually compatible rather than antagonistic, since they both strive to remove blocks to loving and to being loved.

School of Karen Horney Karen Horney played a unique role from many standpoints in the history of psychoanalysis.

While appreciating Freud's contributions, she was among the first psychoanalysts to organize and present her theoretical objections based on her dissatisfaction with clinical results.

She led the way for the first expansion of American psychoanalysis beyond the orthodox view into other schools of psychoanalytic thought.

Horney viewed neurosis as a special form of human development that, because of the resulting waste of constructive energies, made it antithetical to human growth.

Growing in a healthy way meant to Horney the liberating of those evolutionary constructive forces inherent in every human being that urge each one forward to realize given potentialities (the "Real Self").

No matter how impressed we may be with evidence of pathology, we must never forget, however

buried or inactive they may be, these potentials for healthier growing.

This optimistic, positive, life-affirming approach led to Horney's concept of "a morality of evolution." Rather than seeing humans as by nature sinful or ridden by primitive instincts that must be tamed, each person is seen as neither good nor evil but with the moral obligation and privilege of evolving toward self-realization by everincreasing awareness and understanding of oneself, by being truthful to oneself, active and productive, relating to others in the spirit of mutuality, and assuming responsibility for oneself (Horney, 1950).

In her early emphasis on the role of culture, Horney felt that neurosis must be viewed sociologically as well as psychologically since each culture determines its norms, which vary within the culture temporarily and according to sex and status.

People are www.freepsychotherapybooks.org 449 considered neurotic to the degree to which they deviate from the pattern common to their culture.

At the dynamic center of neurosis is anxiety, much of which is generated by the culture.

The four principal devices that we use to escape anxiety are rationalization, denial, use of narcotics and the avoidance of thoughts, feelings, and impulses that produce anxiety.

Since the latter are so frequent and dangerous, they must be repressed, which leads to more anxiety (Horney, 1937).

The evolution of the neurotic process begins early in infancy as children begin to sense that they are not being accepted for themselves.

The actions and attitudes of the parents arouse a basic hostility that has to be repressed.

This leads to an insidiously increasing all-pervading feeling of being isolated and helpless in a hostile world.

This "Basic Anxiety," one of the essential concepts in Horney theory, is inseparably interwoven with the basic hostility and lies unconsciously at the core of the neurotic process (Cantor, 1967).

The child's normal experience of immediacy leads the child to organize behavior by moving toward others, against others, or away from others.

How these functions are used depends on the shifting way the child feels about himself or herself.

Healthy children who feel loved and accepted are able to move flexibly toward another person when they want contact or support.

They are also able to oppose others when they feel their attitudes are respected.

And they can move away to be alone (but not lonely) when they feel they can depend on themselves while alone, while depending on others being there for them when they return.

These fundamental ways of relating can be spontaneous and interchangeable.

They allow for a sense of integration and genuine satisfaction within oneself and in relation to one's environment.

To the extent that the child is operating under the lash of basic anxiety satisfaction based on inner needs, wishes, and feelings cannot be freely fulfilled.

Directions for safety are pursued in ways that are compulsive, indiscriminate, and insatiable.

These become rigidified into characterological attitudes (trends), each related to a specific aspect of the basic anxiety that is being overemphasized.

Healthy moving toward people now becomes www.freepsychotherapybooks.org 450 "compulsive compliancy" as the child accepts helplessness and tries to cope with it by clinging, submitting, and obeying.

Healthy moving against others now becomes "compulsive aggressiveness" as the surrounding hostility is taken for granted and the child has to defy, attack, and rebel against others.

Healthy moving away from others becomes "compulsive detachment" in which isolation is accepted as fact and the child tries to avoid either belonging or fighting by secrecy, uninvolvedness,

and distancing.

Since these ways of relating are no longer spontaneous or freely interchangeable, the child is now caught up in an interpersonal “Basic Conflict” between mutually exclusive, contradictory, compulsive drives, each absolute in their demand for fulfillment.

Horney (1945) saw four kinds of automatic, unconscious attempts at solution of this conflict. First, the child may repress two of the drives while streamlining the third in order to become predominantly organized around this one set of needs, qualities, sensitivities, and inhibitions in an attempt to achieve a sense of value and identity.

This entails radical changes in personality, the development of a Gestalt, which, although Horney described it in specific detail, for purposes of simplicity, she cautioned was not an attempt at typology since there are no “pure forms.” There are differing unique degrees to which one trend may be in the foreground.

The other two, although in the background, subversively exert their influence.

While the child attempts to deny the repressed trends, they continue to affect operating, manifested by contradictory thoughts, confusion, acting-out, physical, or psychological symptoms.

A second way of dealing with the “basic conflict” is to withdraw as much as possible from relating to others so as not to have to confront the conflict.

To do away with the feelings of being divided, weak, and confused, a third attempted solution is unconsciously to create an “Idealized Image” of oneself.

Through imagination the child becomes a conglomerate of an idealized person (the predominant trend).

Compulsive submissiveness may become saintliness; compulsive disparaging of others becomes strength and honesty; compulsive detachment becomes independence.

Glorifying the aspects of interpersonal conflict makes it temporarily possible for the child to combine the incompatibles into one apparently harmonious whole so that he or she can feel spuriously self-confident, www.freepsychotherapybooks.org 451 superior, meaningful, and, above all, unconflicted: In her last book.

Neurosis and Human Growth (1950), Horney expanded on the consequences of the “idealized image” as the essential intrapsychic process within the development of the neurotic; this will be discussed shortly.

The fourth attempted solution of the “Basic Conflict” is “externalization,” the experiencing of inner processes as if they were external ones.

This is more comprehensive than the term “projection,” which refers to shifting blame and responsibility for subjectively rejected qualities.

In externalizing, all feelings, including positive ones, are seen as externally derived.

Externalizing is essentially an active process of self-elimination that creates further conflicts between the individual and the external world.

This unstable equilibrium designates what Horney called “auxiliary approaches to artificial harmony.” These include 1.

“Blindspots” to avoid seeing obvious contradictions 2.

“Compartmentalization or psychic fragmentation” so that the differences in how the person lives simply seem to be parts of himself or herself that are not contradictory 3.

“Rationalization” 4.

“Excessive self-control” to hold back being flooded by contradictory emotions 5.

“Arbitrary rightness” to eliminate doubt from within and influence from without 6.

“Elusiveness” 7.

“Cynicism” to deny and deride moral values so that nothing counts but appearances and not getting caught Since conflicts can be resolved only by changing the conditions within the

person which brought them into being, all the aforementioned attempts at developing a protective structure ultimately lead to further unsatisfactory consequences, for instance, to a multitude of fears that the tenuous equilibrium of the protective structure will be disturbed. www.freepsychotherapybooks.org 452

These fears manifest themselves in fears of exposure, ridicule, humiliation, and, above all, of changing things within oneself.

Further consequences of unresolved conflict are the impoverishment of personality through the waste of human energies, indecisiveness, ineffectualness, and inertia.

In addition, there is moral impairment as the person becomes less sincere and more egocentric. Substituting for authentic ideals are unconscious pretenses of love, goodness, interest, knowledge, honesty, and suffering.

Along with this is unconscious arrogance in that the person may not be aware that he or she feels entitled to act demanding and derogatory toward others.

Moral impairment makes it difficult for the person to take a definite stand.

The person may become undependable since he or she has no appreciation of genuine responsibility.

An ultimate product of unresolved conflicts is hopelessness, deeply rooted in the despair of ever being wholehearted and undivided.

A person without hope may become destructive and at the same time make an attempt at restitution by living vicariously through others.

This Horney saw as the basis of sadistic trends.

Such a view of moral problems in neurosis and sadism is quite different from Freud's views.

The culmination of all Horney's previous works was contained in her volume *Neurosis and Human Growth* (1950).

She focused on the "Real Self" as that central inner force common to all human beings, with a uniqueness in each that is a deep source of growth, sponsoring free healthy development in accordance with the potentials of one's individual nature.

Homeostatic equilibrium is avoided by maintaining the imbalance of an open system that moves the person toward higher forms of order and organization.

The "Real Self" is not a concrete entity but an abstract approximation, a direction toward which one can move to varying degrees were it not for the obstructive internal and external forces, past and present, affecting the individual.

In an attempt to find a sense of wholeness and identity, the person's idealized image becomes an "Idealized Self," which becomes more real to the individual than the "Real Self," not primarily because it is more appealing, but because it seems to answer all the person's stringent needs.

The energies driving toward self-realization are shifted to the www.freepsychotherapybooks.org 453 aims of actualizing the "Idealized Self," which molds the whole personality and may be entirely unconscious.

Because it becomes a total plan of behavior around which a person organized all functions of life compulsively, Horney referred to it as the "comprehensive solution of the search for glory." With self-idealization as its nucleus, the search for glory has three essential elements: (1) the need for perfection according to the special features of one's "Idealized Image," (2) neurotic ambition, the drive toward external success, and (3) the drive for vindictive triumph to put others to shame or defeat them through one's success.

The motivating force for the vindictiveness stems from impulses to take revenge for humiliations suffered in childhood that are augmented during later neurotic development.

To reinforce the search for glory, needs become "neurotic claims" that are irrational because they assume an entitlement that does not exist in reality.

Demands are made without regard for the possibility of their fulfillment (e.g., claims to be exempt from illness, old age, and death).

These claims are expected to be fulfilled without the individual making adequate efforts and must be asserted as one's guarantee for future glory.

Vindictiveness is the response to their frustration.

Whereas neurotic claims delineate the search for glory in terms of the outer world, the person is simultaneously trying to become an image of perfection by a system of "shoulds" and "should nots," which Horney called "the tyranny of the shoulds." For all these efforts, the neurotic does not get what is needed most—self-confidence and self-respect—and is left with "neurotic pride," which is very vulnerable as it is based on spurious premises.

Reality (the individual's "Actual Self" and how that person is empirically existing in the world) is always threatening to prove the falseness of the neurotic's "Idealized Self." The inevitable consequence here is "self-hate" with merciless self-accusations, self-contempt, self-frustration, self-torment, and self-destructiveness.

Neurotic pride and self-hate belong inseparably together as one process, which Horney called the "Pride System." Everything involved in the neurotic search for glory involves "alienation from self," www.freepsychotherapybooks.org 454 something much more pervasive than feelings of unreality and states of depersonalization.

All that is compulsive in neurosis moves the person further away from his or her "Actual Self," the material self (the body, and possessions), and especially the "Real Self." As long as neurotic pride dictates what one should and should not feel, one loses the capacity to be aware of real feelings, to direct life rather than be driven, and to assume responsibility for oneself.

Alienation from self is also an active process to relieve the tensions created by the disruptive conflicts and unbearable tensions.

Alienation and the previously discussed approaches for artificial harmony are still only partial solutions.

Something of an encompassing character is necessary to give form and direction to the whole personality, to deal with the intrapsychic conflict of how the person identifies with his or her glorified self or despised self.

There are three major solutions to this conflict.

These are to be differentiated from the three trends (compulsive compliancy, aggressiveness, or detachment), which attempt to solve the interpersonal "Basic Conflict." Here again Horney cautioned against the rigid usage of "types" since what we see are mainly mixed types.

The major solutions are more properly directions of development that determine the kinds of satisfactions attainable, what is to be avoided, the hierarchy of values established, and how the person will relate to himself or herself and others.

Horney felt that curative forces are as inherent in the mind as they are in the body.

The analyst's task is to give the patient a helping hand in sanctioning constructive forces of the "Real Self" that support an opportunity to grow.

It is essential to help the patient identify on an experiential level the neurotic character structure, the complex defensive mechanisms, and underlying psychodynamic conflicts, as well as talents, capacities, and assets that have evidenced themselves in past and present life, including in dreams.

In the early phase of analysis the Horney analyst first addresses the patient's original motivations for therapy, which may be to remove a symptom, to dissolve the anxiety resulting from a conflict the patient is unable to resolve and that thus prevents living comfortably.

www.freepsychotherapybooks.org 455 In quest of bolstering the Pride System and perfecting the Idealized Self the patient may have to be helped to build up healthy pride and to manage

self-hate.

Constant reminders enjoin that there is no magic cure, that intellectual “knowing” will not automatically dissolve difficulties, and that immediate relief cannot be obtained through “answers”.

Efforts are made to promote basic trust in the therapist in order to facilitate progress.

The immediate objective is to deal with what is of most immediate concern to the patient.

The ultimate objective is to consolidate emotional security, a functioning without pretense, and the putting of the whole of oneself into feelings, work beliefs, and relationships.

The therapist engages the patient in diagnosing, prognosticating, sensing and measuring motivation and availability of resources for moving forward (Kelman 1971).

How much one can do depends on the patient’s ability to “let go” of some externalizing, rigid self-idealizing, and self-hate (M.B.

Cantor 1976).

At the time of her death in 1952 Horney was still in the process of further formulating and extending her concepts about psychoanalytic therapy.

Her colleagues and students at the American Institute for Psychoanalysis, which she founded, have continued her work along her basic lines, adding more current developments from recent findings in the biological, social, and physical sciences.

Noteworthy have been contributions by Muriel Ivimey, Harry Gersham, Frederick Weiss, Alexander Martin, Jack Rubins, Harold Kelman, David Shainberg, Joseph Vollmerhausen, Ralph Slater, Morton Cantor, and Sara Sheiner.

School of Harry Stack Sullivan By reporting on observable events in human interactions, Harry Stack Sullivan (1947, 1948, 1949, 1953, 1954, 1956) attempted to take psychoanalysis out of the realm of fanciful speculation and establish it in the empirical sciences.

In fulfillment of this goal, Sullivan contributed original ideas about normal personality development, a theory of psychodynamics, and a body of technical procedures, particularly in the field of interviewing.

Emotional illness, Sullivan stresses, is both nurtured by and manifested through www.freepsychotherapybooks.org 456 disturbances in interpersonal relationships.

These develop out of frustrating early experiences with parents and other significant adults. Their effect is registered in interference with proper assessment of reality and suitable communication patterns.

Aberrant reactions (parataxic distortions), which are elaborated to maintain security, bear upon all later reactions and lead to other accumulated distortions.

Rejecting the libido theory, Sullivan tends to regard sexual disturbances as one aspect of interpersonal disorganization.

A basic factor that molds personality is the need to maintain a sense of well-being or “euphoria,” loss of which is associated with tension and anxiety.

Early in life the continuance or loss of “euphoria” becomes conditioned to approval or disapproval from parental agencies.

To preserve “euphoria,” the child imbibes the attitudes, values, and standards dictated by parental sanctions (these become personified as the “good me”), while inhibiting and dissociating traits and tendencies that meet with parental disapproval and punishment (which become personified as the “bad me” and especially an anxiety-provoking “not me”).

Demands for social conformity cause the child to respond compliantly to avoid hurt, even though the child is confused as to the meaning of such demands.

The “self-system” that is eventually established contains special anxietyprovoking aspects that, repudiated as alien to the self, are either tentatively suppressed or actually barricaded from awareness by repression.

Reverberations of the “good me,” “bad me,” and “not me” influence everyday behavior and become exaggerated in neurosis.

Startling personifications of these entities often become apparent in the psychoses.

An important function of the self-system is to reduce anxiety.

This would presuppose a repudiation of the “bad me.” When an activity charged with pleasure becomes equated with being “bad,” however, the individual, to retain gratification, may become resigned to accepting the “bad me” as a worthy penalty for indulgence.

The “not-me” personification is, however, too devastating to permit acknowledgment; experiences that relate to it are dissociated or repressed, appearing only in dreams or during the dislodging of repression, as in psychosis.

In our culture sexual behavior is especially forbidden in childhood and becomes a “not-me” experience, producing inhibitions in functioning.

A vulnerable self www.freepsychotherapybooks.org 457 system hinges on opinions of persons significant to the individual (“reflected appraisals”) or what the individual imagines their attitudes to be.

While productive relationships later in life may alter some attitudes toward the self, a constricted self-system does not permit such rectification readily.

The various stages of development are characterized by the emergence of new aptitudes that increase the repertoire of the child’s capacities, particularly those that regulate relationships with other human beings.

Maturation, contingent on the inherent ripening of motor, sensory, and physiological functions, is stimulated by the need to avoid anxiety.

Provided it is not too strong, anxiety, consequently, is to some extent a positive stimulant that encourages personality growth.

Intense anxiety, on the other hand, paralyzes learning and fosters neurotic defense mechanisms.

While a human being is an animal at the mercy of biological needs, human development is shaped almost exclusively by cultural forces.

Unlike other animals, humans participate in cultural interchange with their fellow creatures.

The vehicle of such interchange is communication.

The communicative process, to a large extent nonverbal, begins immediately following birth when the child experiences the closeness and tenderness of the mother.

A milieu of empathy embraces both mother and child.

Being both a biological and cultural creature, the child is motivated to pursue biological goals in gratifying such demands as for food and freedom from pain as well as cultural requirements and satisfaction of personal security needs.

These become increasingly complex with the growth of the child.

They have to do not only with disposing of anxiety, but also with maintaining self-esteem, status, and approval.

Sexual needs are secondary to the need for acceptance.

Need tensions give rise to goal-directed strivings that, if successful, result in “integrating” or “conjunctive” activities (“dynamisms”); if they are not successful, they result in “disintegrating” and “disjunctive” solutions that augment tension and create conflict and anxiety, particularly when security needs are frustrated.

Integrating dynamisms may employ bodily zones (eyes, mouth, ears, anus, urethra, genitals) through which the individual interacts with the environment.

During infancy the child, not recognizing himself or herself as separate from the environment, experiences a www.freepsychotherapybooks.org 458 kind of cosmic identification (“the prototaxic mode”).

Later the child relates events serially rather than logically (“the parataxic mode”).

For instance, thunder occurring simultaneously with the closing of a door creates the idea that the door caused the thunder.

Parataxic reasoning, while present to some extent in adjusted people, may become pathological, as in autistic and paranoid thinking.

The last stage of development is that of logical and rational thinking ("the syntactic mode").

Residues of the prototaxic and parataxic modes persist in all persons, contaminating rational thinking.

When anxiety is not adequately controlled, it may disrupt the learning process.

The self-system becomes constricted through concentrating on reducing anxiety rather than on expanding the potentials of the individual.

Anxiety is always generated in an interpersonal frame, usually because of feelings of low self-esteem and fear of disapproval from real or imagined personages.

It may not be reality determined but be conditioned by prototaxis or parataxic modes of thinking.

When it occurs, it sets into motion operations toward its reduction or elimination.

To forestall anxiety, the individual develops techniques of gaining approval and avoiding disapproval, for instance, by recognizing and responding to cues such as "forbidding gestures" as anticipatory to disapproval.

Anticipatory cues, in neurotic conditions, may be generated from within when prototaxic or parataxic modes come into play.

The individual may consequently react as if self-esteem is vitiated or as though he or she has incurred disapproval, even when apparent reasons are lacking.

An aura of disapproval often invests certain childish exploratory and pleasure activities, for instance, thumb sucking (an important means of gaining self-satisfaction as a substitute for the nipple) and genital and anal exploration.

Disapproving gestures from the parent may imbue these operations with anxiety and lead to their inhibition.

The self-system operates to reduce anxiety by defensive processes of selective inattention and dissociation, distorting awareness and blunting reactions to events and people.

In this way the self-system circumscribes the creative potential of the individual and fosters the neurotic process.

As the child matures and is expected to adapt to the demands of society, the integrity of self-system and the degree of anxiety harbored will determine the measure of how he www.freepsychotherapybooks.org 459 or she adjusts, sublimates, subdues fear, manages anger, and handles contradictory standards in the culture.

When anxiety is not too great and opportunities are propitious, much constructive learning and relearning can occur.

This is particularly the case during the juvenile period (5 to 9 years of age) when the child is exposed to compeers, with whom one competes, and teachers whose attention he or she must learn to share.

Experiences of cooperation, compromise, belonging to an in-group and out-group, and competition expand the self-system, increase facilities for dealing with people, and promote greater introspectiveness and self-criticism.

Around the preadolescent period (8½ to 12 years) there is a blossoming of the capacity to relate and love.

Nonsexual intimacies with members of the same sex are the rule, and an interchange of ideas, experiences, and values in the medium of a warm relationship expands the child's horizons.

With the maturing of the sexual organs, minor discursive explorations of the opposite sex occur, but the child returns to same-sex friends for safety and companionship.

Toward the end of the preadolescent period there is characteristically experienced a strong emotion of loneliness as feelings of separation and distance develop toward friends who are also in the process of breaking away from their preadolescent ties.

With genital maturity the stormy period of adolescence begins.

New and complicated adjustments are required.

For one thing, the upsurge of sexual feelings ("lust") may drive the youth to express them with or without true interpersonal intimacy.

Emergent are a host of responses— fulfilling, frustrating, productive, neurotic—that reflect residual patterns.

Masturbation with all of its attending associations becomes a way of relief, indeed, it may become a chief means of dispelling tension.

When relationships with the opposite sex are invested with anxieties that are too strong for resolution by the self-system, the youth may retreat to the safety of same-sex intimacies, releasing sexual drives in homosexual affinities.

A chief reason for such anxieties is conditioned disgust and shame related to one's own genitals, which produces loathing and fear of opposite-sex genital organs.

Homosexuality is thus more than a sexual problem: it is a disturbance that invests the total personality, making relationships with the less frightening genital equipment of the same-sex members more palatable.

Other sexual deviations may similarly be regarded as selective responses to anxiety.

www.freepsychotherapybooks.org 460 Complementing the need for sexual adjustment is the ubiquitous adolescent struggle for independence in an economic world that necessitates dependency on parents.

Conformity and a need to please alternates with rebellion and a need to disobey.

Insurgency, though punished, serves the end of imagined personal freedom.

Obsequiousness, however humiliating, fulfills security needs.

Caught in this conflict, the youth begins to question the values and standards of society to which he or she has been dedicated.

Bursts of religious and philosophical interests, concern with ethics and morality, persistent questioning and self-searching constitute the stormy process of growing up, a goal unfortunately that many adolescents never reach, although physically they may mature into adults.

Such "chronic adolescents" travel through life as discontented rebels, constantly trying to find peace and the meaning of existence in a world of frustration that provokes feelings of being misunderstood.

Adolescents who are successful in attaining maturity display harmony between their inner selves and the demands of their group.

Self-respect and the ability to relate intimately, lovingly, and cooperatively with others are the end result of successful group interactions.

Interpersonal relationships are determined by the many characterological traits that the individual has elaborated as consonant with or alien to the self.

Some "parataxic distortions," reflecting attitudes toward significant past persons, are automatically projected into all interpersonal relationships.

The psychotherapeutic interview is an interpersonal experience in which the therapist as a "participant observer" engages with the patient in a joint effort to examine difficulties in relating as they are reflected in the therapist-patient encounter and in the patient's interactions in general.

In his volume *The Psychiatric Interview* (1954) Sullivan delineates a variety of techniques. Essentially what is important is to help resolve the patient's anxieties in order to overcome

avoidance patterns and to release learning potentials.

Learning requires that the patient understand his or her behavior in conceptual terms in order to evaluate it accurately.

Patients come into therapy with a number of assumptions that may block their effective use of the therapeutic situation; for example, they may assume that they should not need help, that they should be at all times www.freepsychotherapybooks.org 461 governed by logic, that they should not be ruled by the difficulties of their early life; and that they must at all times be self-sufficient and independent.

These assumptions will require clarification.

A wide range of problems may be reviewed: areas of “selective inattention,” dissociations, parataxic distortions, false personifications, and inaccurate perceptions of one’s behavior or anxiety.

The area of focus will be the particular complaint that concerns the patient most.

The therapist is alert to inappropriate self-evaluations, inaccurate identification of events, perceptual distortions, and other neurotic patterns that come out during the interview.

Once patients commit to therapy and agree to report on their thoughts, they are encouraged to talk about their perceptions, ideas, feelings, and acts in relation to people and to themselves.

Areas of difficulty (anxiety and anxiety avoidance) are spotted by the therapist and are bracketed to customary interpersonal reactions.

An attempt is made to establish a consociation of events in order to conceptualize them as accurately as possible.

On the basis of this understanding, the patient eventually will discuss plans and make appropriate changes in behavioral patterns outside of therapy.

The initial phases of therapy are concerned with the establishment of a diagnosis and the estimate of the tentative prognosis to determine whether therapy can have a constructive influence.

Rather than establishing a sterile diagnosis, the therapist appraises the existing problems in living and the assets of the patient in contrast with liabilities.

An estimate is then made of what may be accomplished by psychotherapy.

Once the probable effectiveness of psychotherapy is decided, a systematic inquiry follows.

The developmental history is examined in relation to current areas of anxiety, the prevailing anxiety-avoidance responses and security operations, and the circumstances that bring these about.

The therapist is sensitive to manifestations that appear within the therapeutic interpersonal relationship.

When convinced that these reflect basic patterns, the therapist focuses the patient’s attention on their nature and origins.

A variety of questions probe pockets of anxiety.

Hypothetical situations are posed, such as “How would you feel if your employer depreciated your services?”; and tentative interpretations are made, such as “I wonder if this doesn’t make you want to run away?” www.freepsychotherapybooks.org 462 In the therapeutic encounter, as in any other situation, the patient acts out “parataxic distortions.” Bringing these to the awareness of the patient helps the patient separate the present from the past and appreciate the attitudes and values that are a part of a self that he or she tends to repudiate.

The origins, manifestations, and consequences of the patient’s defenses are actively explored.

The patient experiences emotionally what has been dissociated, and in this way is enabled to evaluate, toward their possible reacceptance, aspects of himself or herself that have been split off from awareness.

While the current situation is actively considered, childhood experiences and conditionings are also constantly prospected in order to expedite separation of the past from the present. In these ways new and healthier interpersonal relationships may be constituted. The management of patients in therapy is more flexible and active than in Freudian psychoanalysis.

Thus, patients may assume a sitting or recumbent position; may use free associations or deal with specific aspects of their experience; and may work with present reality problems as well as early childhood memories and productions from the unconscious, such as dreams. The emphasis is on the character structure and problems in interpersonal relationships, although genetic origins are not neglected.

Relaxation of the "basic rule" permits a focusing of the interview on significant material. The therapeutic situation is considered a real relationship that has values in itself in addition to serving as an arena for transference.

The manner of the therapist, Sullivan believes, should never be stilted or blank; it should reflect the therapist's inner feelings, even though annoyance, irritation, or pleasure are mobilized.

The skillful use of satire may be employed, where indicated.

Even provocatively inaccurate statements may be presented to stimulate corrective responses from the patient.

Exploited also may be a strategic shifting of the stream of thought into an unexpected area before the patient's defenses are mobilized.

Skill in interviewing, Sullivan insists, is dependent on the therapist's drawing the patient out by pointed questions, identifying parataxic distortions in the relationship, demonstrating the unreasonableness of the patient's reactions in view of what actually has been going on in therapy, speculating on whether feelings of a similar nature occurred in the past, examining the assumptions the patient is making through transference www.freepsychotherapybooks.org 463 reactions, and exploring important past relationships associated with the origins of the patient's trends.

Particularly important is the uncovering of dissociated systems that have been repressed because of their anxiety content.

Here a frontal attack is useless since the patient's defenses can easily ward this off.

Rather, slow and indirect interviewing may eventually uncover these systems.

Skill is required in dealing with and circumventing the patient's verbal defenses, such as rambling about irrelevances.

Sullivan recommends only brief questions and brief comments by the therapist while encouraging lengthy responses from the patient; long dissertations by the therapist are not in order.

Clarification of communication is mandatory as is activity as a "participant observer, the therapist utilizing his countertransference in a constructive way" (Crowley, 1971).

Much of the theory and method of Sullivan are at present adopted by the William Alanson White School for Psychiatry, Psychoanalysis and Psychology, which is located in New York.

Criticism of the "Dynamic-Cultural" School Criticism of the "dynamic-cultural" school stems mostly from Freudian psychoanalysts who regard the diversion into the sociological field a form of resistance against the biological-sexual hypotheses of Freud.

This revolt against classical thinking is considered by some to be a manifestation either of unresolved transference in the "renegade" analyst, a product of an incomplete personal analysis, or a manifestation of an entrenched narcissism that spawns the evolvment of ambitious, albeit faulty, new systems.

Others attack neo-Freudian concepts along conceptual lines.

For example, Franz Alexander questioned Horney's attack on the libido theory on the basis that

it substituted for the mystic biological substance of libido an equally empty sociological slogan of culture (Alexander F, 1940).

There is a general feeling among Freudians that sociological-cultural approaches are loose and myopic preconscious generalizations that deal with only one facet of human experience and do not consider the basic infantile sexual conflicts, which are the nuclei of neuroses.

While analysis of character is said to be helpful, it does not eradicate the deepest
www.freepsychotherapybooks.org 464 sources of conflict.

Freudians contend that distortions in interpersonal relationships and defense mechanisms as a whole cannot be understood or helped unless we deal with their unconscious roots, a formality often overlooked or not considered essential in the methodologies of some of the neo-Freudians. Therapy conducted in accordance with principles of the dynamic-cultural school is, therefore, considered "superficial" and of reeducative rather than reconstructive influence.

Particularly deplored is the discrediting of sexuality as a vital force in personality development by assigning it to its traditional place as a manifestation appearing with the maturation of the genitals.

This retrogression to an atavistic conception of sexuality denies clinical findings that indicate that infantile sexual drives are present, powerful, and persistent, shaping not only the expression of adult sexuality, but the individual's personality as a whole.

Divergencies, it is claimed, are often more apparent than real; certainly this is so in technique.

But even in theory, once the semantics are clarified, differences are more a matter of emphasis than of explicit contrast.

A fine line of distinction may reside in the elaborateness of description by the neo-Freudians of the phenomenological nature and consequences of defenses and character traits.

The generalizations drawn from these variances, however, are often expanded into global doctrines that seek to account for the whole of life.

But what is even more confounding, it is claimed, is that some deviants from Freud tend to utilize the hypotheses and terminology of Freud whenever it suits their convenience.

Indeed, there is a tendency to recast Freudian formulations in novel, excruciatingly complicated neologisms and to concentrate on nuances, torturing these into broad theorems that cannot possibly explain the complexities of behavior, while ignoring those psychopathological areas that do not fit into the renovated scheme of things.

There is also a tendency on the part of some neo-Freudians to attack an orthodoxy that no longer exists, particularly among the younger psychoanalysts.

Criticism is also leveled at those, such as Harry Stack Sullivan, who decry the delving into the inaccessible and unobservable, insisting that the material for study be purely that which is apparent and demonstrable, such as interpersonal relationships.

Yet such concepts as "self-fulfillment" and "self-actualization" border on the metaphysical and
www.freepsychotherapybooks.org 465 require postulates that remove them from the zone of science.

To the charge of the neoFreudians that classical hypotheses are farfetched, it is obvious in studying them, say the Freudians, that some of the concepts of the neo-Freudians vie for absurdity with the most fanciful contentions of the orthodox school.

To the neo-Freudians these charges are considered as further evidence of the basic intolerance of the Freudians.

They are also branded as rationalizations for clinging to an outmoded theory that has lost its clinical usefulness.

On the contrary, they insist, neoFreudian theories support methodologies that expand the therapeutic spectrum to a wide group of patients not suited for classical techniques (Wolman,

1967; Marmor, 1968).

EXISTENTIAL ANALYSIS Existential “analysis,” a more depth-oriented approach than existential “therapy,” is, as is the latter, rooted in the precepts of existentialism.

This philosophy attempts to solve a human’s apparently insoluble quandary through a search for the meaning of one’s existence.

It posits that only by finding purpose in one’s life, even in the face of catastrophe, can one experience that true sense of “being” that fuses the physical, psychological, and spiritual natures.

Such elements operating individually isolate humans from their selves and from the world; they sponsor a profound contradiction.

For the spiritual self demands freedom, unlimited choice, and responsibility, while the psychological and physical selves dictate finite restrictions, restricting one to one’s own endowment, past history, and conditionings.

Although anxiety and despair are inescapable products of this “existential predicament,” a human being has the capacity, responsibility, and the freedom of choice to deal with the disruptive forces that tend to unbalance an individual.

The crisis in one’s existence urges the individual onward toward some solution that is generally registered in terms of greater “self-realization.” A human being cannot approach problems of existence solely through objective means, for example, the methods of science; what is required is a search for ultimate values that may be achieved only through moral, ethical, and spiritual means.

Only then can one cope with one’s ever present anxiety or resolve some of the riddles of one’s identity, adapt to life’s purpose, and www.freepsychotherapybooks.org 466 accept the inevitability of death.

These concepts have been expressed in various ways over and over again as far back as the first philosophical speculative conceptions about the meaning of life.

They were most clearly organized into a system by such philosophers as Søren Kierkegaard, Edmund Husserl, Martin Heidegger, Karl Jaspers, Jean-Paul Sartre, Martin Buber, and Paul Tillich, each contributing his own unique mode of viewing the nature of existence and the means by which a human being may solve its ambiguities.

It was Husserl (1962) who was principally responsible for the “phenomenological method” of viewing the behavior of a person without preconceived theories or notions of causation while describing phenomena as they were observed.

This, with Heidegger’s contribution (1962), served as foundations for the existential analytic approach which rapidly advanced in Europe largely because of the influence and writings of Ludwig Binswanger (1942, 1947, 1956) and Medard Boss (1957, 1963) and in this country Rollo May (1950, 1960).

These authorities concede that, while biochemical, neurophysiological, and psychological mechanisms are important in describing the functions of the human organism, they cannot be dissociated from the existence of the experiencing individual and from the understanding of his or her ontology.

Nor can one, as Buber (1937) has emphasized in his “I-thou” duality, conceive of humans as separated from other humans or apart from their world.

Humans are shaped by their world and in turn fashion their world to bring it into focus with their needs.

By applying a phenomenological approach to the technical problems of psychotherapy, existential analysts claim to go beyond conventional ways of managing neurotic problems.

This is not because their methods are different from, or superior to, those of other psychotherapists (since the techniques employed are essentially the same), but rather because

they feel that they approach their patients from a more constructive and comprehensive standpoint.

Patients are generally encouraged to perceive of their behavior in the context of the “what” it is that one is experiencing rather than the “why.” While the genesis of the www.freepsychotherapybooks.org 467 individual’s problem is considered important, as are the mechanisms of defense, they are regarded as only one aspect of the patient’s “being-in-the-world.” The crux is the functioning, experiencing person in the present.

“Therapy, then, is not primarily an uncovering process, but a creative one.

It is the self-making aspects of being that are in the foreground—one’s decisions, commitments, and responsibility.

The existential therapist is not content with the elimination of undesirable aspects of functioning, such as inhibitions, frustrations, and symptoms, but has the larger goal of the creation of positive values” (Basescu, 1963).

The patient-therapist relationship, unlike that of Freudian psychoanalysis, is predicated on activity.

It is in essence an “encounter” in which both participants extend their “full being in the world of the other, without treating the other as an object subordinated to some purpose of one’s own.” This means complete freedom in mutual self-revelation and an openness in the therapist’s approach to the patient, in which the therapist exposes himself or herself as a real person, not as the traditionally tolerant authority.

Unconditional acceptance does not preclude criticizing the patient for “selfimposed limitations on being.

Patients are not innocent victims of circumstances.

They have a responsibility for their own destinies, even to overcoming self-limitations.

This requires resolution and fortitude, for if one loses one’s courage, one loses one’s being (Tillich, 1952).

The active “encounter,” in which the therapist engages with the patient without assuming an artificial and studied role, releases the “courage to be.” As Tillich has remarked, “A person becomes a person in the encounter with other persons, and in no other way.

This interdependence of man and man in the process of becoming human is a judgment against a psychotherapeutic method in which the patient is a mere object for the analyst as a subject.”

In the conduct of therapy no preconceived theories are admissible.

“There is nothing other than a person’s definition of self for which that person alone is responsible.

The work of therapy is the illumination of the person’s view of the way things are (the personal myth) and the way in which that person’s suffering incarnates that cosmology” (Ofman, 1985).

The patient describes his or her “world of being” and the therapist shows

www.freepsychotherapybooks.org 468 nonjudgmental interest.

What the patient does as a consequence of gaining increasing understanding is entirely left up to him or her.

No pressure or manipulation is admissible.

As the patient gains an awareness of personal myths that control his or her existence, modes of self-deception, and means of denial of personal autonomy and freedom, in short of the “self-created world of perpetual crises,” he or she is helped by authentic relating with the therapist to avoid self-rebuke and spurious ploys and to assume complete responsibility for the distortions.

Essential, then, is a complete acceptance of the patient who is in the process of “becoming,” irrespective of his or her behavior and problems.

In the words of Martin Buber (1948), "conforming the other" means accepting one's potentialities as well as the person that he has been and is now, in the light of the capacity to develop, for "when something, no matter how imperceptible, happens between two men so that each becomes aware of the other and his world is related to him in such a way that he does not use him as his object but as his partner in a living event, the fact of this encounter can never be entirely eliminated" (Buber, 1957).

In the medium of such unadulterated acceptance various techniques may be employed, including psychoanalytic techniques.

What is necessary at all times, however, is a unique kind of phenomenological observation during which the therapist comes to terms with the patient's (as well as the therapist's own) problems of being human without preconceived and structured formulations.

The therapist goes beyond what patients believe and perceive about themselves to how they experience themselves in the world.

The neurotic patient, sick as he or she is, first needs to be "cared for" by the therapist.

Only as one grows strong in the relationship does one develop the courage to move toward ontological self-realization.

The patient is helped by the therapist's extending to the patient a "humanness" in the relationship, a "being-togetherness," and a "standing-on-the-same-level-of-existence" through meaningful communication.

During this process repressions blockading the sense of being are gradually lifted.

The existential system of analysis is oriented around the uniqueness of each individual and is not impeded by adherence to scientific determinism or the search for systematic regularity (Holt H, 1965).

An understanding of human experience, including the human commitment to independence, freedom, and "being-in-the-world," is acquired most fruitfully by recognizing three types of basic experience: the biological body world ("Umwelt"), interpersonal relationships ("Mitwelt"), and self-recognition ("Eigenwelt").

A unification of these three is essential for a complete feeling of authentic existence (May et al, 1958).

Unity in these three modes of existence is lost as a result of inescapable basic "ontological" anxiety (Burton, 1965).

Integration of the different elements of self, essential for what Iago Galdston (1963) has described as a "healthy ontological thrust toward maturity," is achieved as the therapist moves into the patient's life and helps one experience oneself in terms of the universe around one, including the biological substance and instincts, the world of meaning in relation to others, and the private area of self.

Binswanger implies that psychoanalysis deals with the first dimension, partly with the second, but not with the third, i.e., how the self relates to the self.

On the other hand, existential analysis applies itself to all three modes and, in their liberation and harmonization, brings to the individual a true alteration in the sense of values and more positive purpose in relationships with people.

Essentially, existential analysis is an approach to how the therapist applies himself or herself to therapy rather than a technique in itself.

The therapist conceives of the goal in treatment as the restoration of the patient to "bodily-togetherness-with-others-in-the-world."

The cornerstone of existential constructs are embedded in concepts of will and decision (May, 1960).

The patient is at all times aware of free choice in perceiving wishes and in "willing" their fulfillment toward doing something about them.

"This is the level of accepting one's self as having a world.

If I experience the fact that my wishes are not simply blind pushes toward someone or something, that I am the one who stands in the world where touch, nourishment, sexual pleasure and relatedness may be possible between me and other persons, I can begin to see how I may do something about these wishes" (May & Van Kaam, 1963).

In moving toward a better integration, the patient is www.freepsychotherapybooks.org 470 helped to appreciate the forces of decision and responsibility "within a nexus of relationships upon which the individual himself depends not only for his fulfillment but for his existence." The application of the existential philosophy to therapeutic process will vary with the therapist (Holt H, 1968, 1972b; Havens, 1972; Wenkart, 1972).

In a number of important ways, however, we can detect a similarity in operation.

First, there is a focus on the inner experience of the patient.

Second, there is recognition that the therapist cannot remain aloof and detached from this experience, that the therapist is privileged to emote and to feel torments and dilemmas, many similar to those of the patient.

In the ensuing encounter the therapist's feelings are bound to be communicated.

This will foster engagement in sympathy, challenge, and confrontation at the same time that the relationship is exposed to investigation.

What eventuates is the humanness of the therapist, the therapist's

"being-where-the-patient-is," and empathy sharing the inner experiences and desperation of the patient.

An emotional clash often occurs as empathy and understanding temporarily recede, and in the ensuing struggle new understandings occur and change is brought about both in the patient and the therapist.

Entering the patient's world and communicating with him or her deeply and humanely help to penetrate the defenses of sicker patients, such as schizophrenics.

Experiencing the dissociated elements of the patient's personality is a potent force in their reuniting.

"Because the existential therapist tries to stay with the patient, he must be willing to 'encounter' or confront aspects of the patient not so comfortably assimilated" (Havens, 1972).

Existential analysis has not been formalized into a special school, but it has attracted a number of followers, particularly those oriented toward the theories of Jung, Horney, Sullivan, and Fromm.

The "third-force" movement of humanistic psychology is oriented around existential analytic ideas.

Criticism of Existential Analysis www.freepsychotherapybooks.org 471 A rift is inevitable between existentialism and traditional Western psychotherapeutic approaches that promote both objective scientific naturalism and subjective human psychology.

Existential analysis "tries for insightful, intuitive and crosssectional descriptions of the experiencing self," a kind of ontological unfolding (Opler, 1963a) sponsored by intuition, which is foreign to the training and ideologies particularly of the American psychiatrist and psychologist.

Criticisms are consequently directed at existential analysis as being a "subjectivist melange" that avoids regularities in process and hence does not help us in arriving at a scientific understanding of how human beings function, get ill, and get well again.

Western psychotherapists express irritation at the language of existentialism and accuse its practitioners of diffuseness in concepts and of depending for their therapeutic effects on faith and the profits of a helping relationship.

Existential analysis is not considered to be "analysis," but merely a descriptive exploration

of problems in self-realization.

There is a feeling that existential approaches constitute a regressive movement, a throwback to a prescientific era from which the mental health field has painstakingly tried to liberate itself.

To an extent this intolerance is stimulated by an inability on the part of existential analysts to communicate adequately; the coining of new words and phrases without appropriate definition and clarification has been particularly confounding.

To an extent the prejudice is due to sectarianism and a need to defend personal theories against all external assaults.

We must remember that most of the therapists who have become attracted to existential analysis have been trained in dynamic and other systems with a scientific pretension.

Ludwig Binswanger, for example, was a psychoanalyst and a close friend of Freud.

A principal reason why therapists have turned to approaches different from those in which they have been schooled has been that they were disappointed with therapeutic results.

In exploiting the existential method, many therapists claim to have found their work more successful and rewarding.

This, of course, may be due to the affinity they feel with existentialism because of personal needs.

A methodology that has significant meanings for a therapist will enable the therapist to relate more sincerely to patients, who will sense genuineness and then respond appreciatively to the therapist's efforts.

www.freepsychotherapybooks.org 472 Therapists who practice existential analysis continue to utilize their old techniques on the basis that traditional and existential methods are not mutually exclusive: they influence different aspects of the psyche, hence they actually reinforce each other.

Whether or not therapists need to introduce into their modes of working stratagems from different systems is contingent on the systems' helping or hindering their functioning.

All progressive psychotherapists incorporate into their operations techniques that yield for them the best results.

Somatic therapies, tactics that expose the unconscious, maneuvers in interpersonal relationships, and devices that influence spiritual strivings and alter moral values are often blended into an eclectic framework that is applied flexibly to patients in accordance with their needs.

If any approach enhances results, therapists may pragmatically incorporate it into their therapeutic armamentarium.

Danger arises, however, from the tendency to overvalue any of the systems, including existential analysis.

Many therapists cannot be taught maneuvers and attitudes essential for their functioning as existential analysts.

They will then do better with approaches with which they feel more comfortable.

Actually, when we cut through the semantic persiflage and consider the goals and methods of existential analysis, we find that they differ little from those of other forms of psychoanalytic psychotherapy in which patients are encouraged to observe thoughts, feelings, and behavior in the context of their immediate experiences, not merely in terms of whence and why they arise.

There is a focus on the present, which is regarded of primary importance in serving as a repository of the past.

The patient's mechanisms of defense are considered to be aspects of the total behavior and meaningless in themselves.

The objectives are not solely the relief of symptoms and intellectual insight into one's problems, but the utilization of understanding in the direction of change, particularly toward the altering of value systems to promote the greatest possible self-actualization.

The relationship with the patient is predicated on measured activity.

The therapist on occasion may reveal that he or she has some personal difficulties or that the therapist has made mistakes.

Like other therapists, however, most existential analysts do not purposefully display their neurotic distortions or act out destructive feelings that emerge from countertransference.

They do not consider their patients helpless puppets www.freepsychotherapybooks.org 473 dragooned to everlasting suffering by an inimical past, but rather as active participants in perpetrating their neuroses.

Patients have a responsibility in making constructive choices and in getting well.

The presence of some anxiety is an inevitable consequence of personal and social conflict, even in healthy people, but existential analysts and seasoned therapists of other schools differentiate between grades of anxiety— between productive and neurotic reactions to anxiety.

In short, a properly working psychotherapist incorporates into his or her system of therapy essentially the same essences of “meaning” and of method as a good existential analyst, or a good Freudian psychoanalyst, or a good neo-Freudian analyst.

This presupposes flexibility in approach and full intelligence in the use of oneself in the therapeutic relationship.

The language that therapists employ in describing what they do, and the theories they invent to explain why they do it, do not alter the fact that essentially the same healing processes will have to be implemented, or will spontaneously come into operation, if a patient is to get well.

OBJECT RELATIONS APPROACHES Freud's early drive theory viewed psychopathology as revolving around the nebulous orbit of psychic energy.

This explained some of the phenomena encountered in clinical practice, particularly in hysterical and phobic disorders.

In working with preOedipal problems, however, the limitations of the drive model became apparent.

Attempts were then made to fill in the gap by appending to the instinct hypothesis theories of interpersonal relations.

Finding this combination troublesome, some authorities dissociated themselves from Freud's early formulations while retaining his dynamic view that a multitude of motivational forces, originating in prior experiences and conditionings, operating harmoniously or in conflict, consciously or outside of awareness, express themselves in relationships with other human beings.

The direction of psychoanalysis shifted from the capriciousness of instincts to the regulatory operations of the ego and its defenses.

The early work of Nurnberg (1955) on the synthetic functions of the ego, of Waelder (1936) on its multiple operations, of Anna www.freepsychotherapybooks.org 474 Freud (1937) on its ingenious maneuvers to contain primitive drives, and of Heinz Hartmann (1958, 1964) on its transactions with the total personality, as well as of Fairbairn and other ego analysts, led to the enhancement of ideas about the ego and to what has become known as “object relations theory.” Fairbairn (1954), for example, evolved a developmental model that included many of the components of present-day object relations theory.

His contention was that during the first two months of life a child was emotionally fused with the mother.

This “primary identification” with the internalized mother figure (the “object”) had to be resolved and renunciated in the course of development if mature growth was to be achieved.

Otherwise the ego (or “self”) would not evolve from infantile dependence to normal conditional dependence with differentiated objects.

The relationship with the mother had gratifying and enticing aspects, as well as ungratifying components.

Internalization of the relationship into the “ideal object” (gratifying), the “exciting or libidinal object” (promising and enticing), and the “rejecting anti-libidinal object” (depriving and withholding) was accompanied by a threefold split of the ego from its binding to each aspect.

Ego and object became inseparable.

If too much was incorporated of the “bad” exciting or rejecting mother, efforts to control and preserve relations with the real mother were prevented and the ego was unable to bind satisfactorily with the “ideal object.” There was thus a splitting and fragmentation of the ego, interference in relationships with people, and tendencies toward psychopathology. The battle with the rejecting, depriving antilibidinal mother and the enticing, exciting libidinal object was carried on internally.

The frustrated antilibidinal ego hated and attacked the libidinal ego as well as the exciting object, in this way perpetuating self-destructive, self-punitive behavior.

As a result of this battle, hateful attitudes were produced that through transference were extended outward toward maternal representatives, including the analyst.

Adjustment was thus compromised by the conflict between object-related libidinal desires, object-related antilibidinal, hateful impulses, and object-related idealizations.

The threefold splitting of the ego helped the child maintain a good relationship with the mother but created a disunity that continued throughout the www.freepsychotherapybooks.org 475 life of the individual.

The “splitting” paradigm established with the mother followed through also with the father, and to complicate matters the resulting internalized paternal object was fused with that of the mother and the combination was then projected onto both parents.

The continued need for nurturance fostered persistent dependency, which might be sexualized as a defense.

The basic conflict that underlined all psychopathology, therefore, was dependency and not through the classical Oedipal situation.

Failure in object relations in the early oral phase produced frustrated love and schizoid withdrawal; failure later during the oral biting phase contributed to one’s hating one’s parents and resulted in depressive manifestations.

Somewhat later, Fairbairn modified his ideas about how internalized objects were developed. He added the notion that since children had to maintain an illusion of the “goodness” of parents for their own security needs, realization of parental seductiveness, rejection, nongivingness, or cruelty was repudiated or repressed by blaming inherent “badness” on themselves.

The resulting “bad” internal objects, with which the ego identified (primary identification) to a greater or lesser degree, were repressed with the elaboration of such defenses as splitting and guilt to protect the ego.

Attempts to work through painful memories and past experiences with the parents fostered a choice of love objects with similar bad qualities, encouraging disappointing outcomes.

The individual through his or her own character distortions provoked rejecting and sadistic behavior on the part of parental surrogates, further aggravating the distress and convictions of hopelessness and despair.

The contributions of Winnicott (1965) revolve about the evolution of the self with speculations of what must be on the infant’s mind during this process.

The mother is conceived of as a physical and emotional “holding” environment for the child, who, if she has sufficient devotion and provides demonstrable caring, serves as a means of satisfying the child’s illusions of power and sense of omnipotence, which in being gratified become the foundation for building of the healthy self.

The mother also acts as a mirror, reflecting the child’s own “personalizations.” Inability of the mother to provide a reasonably “perfect” (“good-enough-mothering”) environment usually interferes with the growth of www.freepsychotherapybooks.org 476 self.

Once a nucleus of “hallucinatory omnipotence” is established, further development of the self necessitates coping with limitations and frustrations in the environment and adapting to a loss of control.

This is the second stage of development and encourages separateness from the mother.

The use of “transitional objects” (blanket, teddy bear, etc.) over which the child can execute control and that allows the child to experience other people for what they are proceeding from “object relating” to “object usage,” helps to ease the transition from total involvement with the mother to separation from her.

Serious failures in this process of evolution encourage fragmentation and splitting of the self into a “true self” that becomes impoverished and a “false self” that is artificially compliant and provides an illusion of security.

The essential shifting from dependence to independence, from omnipotent fantasy to realistic thinking, is thwarted.

Life thereafter is spent in searching for the missing parental provisions that should have existed in one’s early development.

The curative factor in psychoanalysis is that it can be conducted in an environment that can supply these missing emotional ingredients.

Unlike classical analysis, in which the analyst does not allow regressive gratifications, Winnicott alleges that to liberate the self the therapist must function as a maternal caretaker.

In these early excursions in object relations theory there was no intention of abandoning the biological orientation of drive theory.

The ego, though predominant, was considered a derivative of fundamental biological forces concerned with the maintenance of homeostasis.

The Freudian dynamic, economic, and typographic models were in large degree retained while crediting to the ego greater autonomy from the id and better control over forces in reality (Hartmann & Kris, 1964).

This swing toward the ego was abetted by such developmental researchers as Margaret Mahler (1941, 1975).

Early developmental theorists conceived of the infant as a helpless blob of palpitating flesh totally bent on needs gratification (“autoerotism,” “primary narcissism,” “absolute dependence”) through an emotional umbilical cord.

Severance of this cord was resisted desperately, and, because of maternal immaturity and faulty child care practices, sometimes never occurred.

This accounted for the survival into adulthood of chaotic traits that seriously interfered with realistic adaptation.

The road www.freepsychotherapybooks.org 477 from autoerotism to full genital maturity was paved with many stumbling blocks, not the least of which was failure to resolve the Oedipal complex.

This anchored the individual symbiotically to archaic objects and blocked proper separation-individuation (Mahler et al, 1975).

According to Mahler, the first weeks of the infant’s life are occupied autistically with hallucinatory wish fulfillment and obliviousness to external stimulation.

At three to four weeks there is some response to external stimulation (e.g., smiling in response to the mother's face), indicating some awareness of the environment.

This constitutes a normal symbiotic phase when there is no differentiation between the self and the object (the mother).

There is some registration of "good" and "bad" environmental happenings.

From 4 or 5 months to 10 months a "hatching phase" initiates early differentiation, marked by discrimination between self and object and between perceptual and inner sensations.

At the end of this phase a "practicing" subphase is initiated, characterized by bodily movements such as crawling and thrusts into the environment away from the object.

This aids initial separation-individuation.

There is interest in one's body ("secondary narcissism") as well as in the world around.

This subphase is encouraged when the mother is willing to accept the child's independence; it is discouraged when the parent refuses to grant her child autonomy.

During this period there are indications that the child does not consider the mother a distinctive entity.

Between 15 and 18 months, the child begins to realize his or her own helplessness as the child sees the mother as a person separate from himself or herself.

This is a "rapprochement" subphase that around 18 to 24 months of age leads to a crisis in separation and individuation with ambivalent clinging to the mother (symbiosis) and resistance of her efforts at separation.

Resolution of this ambivalence is crucial to the child's development since failure leads to splitting of "good" and "bad" object representations.

The mother's role during this period is vital, and how she deals with the child's shifting between symbiosis and separation will crucially influence what happens.

Coincident with this individuation struggle is the refinement of verbal communication, cognizance of sexual differences (eventually leading to gender identity), and greater awareness of the father.

During the third year of life, self and object become more separate and their internalized representations become more unified www.freepsychotherapybooks.org 478 ("libidinal object constancy").

Differences in the degree of resolution of symbiosis and separateness can occur so that the one is much more advanced than the other, complicating adaptation.

A derivative of object relations theory has emphasized self-representations as distinguished from object representations (Hartmann, 1964; Mahler & McDevitt, 1982).

This has encouraged explorations into "self-psychology," of which the most prominent contributors have been Edith Jacobson, Otto Kernberg, Heinz Kohut, and Joseph Sandler.

According to Jacobson (1964), the desire of the infant and child to relieve tension (unpleasure) and to secure gratification of needs with resulting pleasure leads to the internalization of the object in the form of bad (frustrating) and good (gratifying) images toward which the child responds as variably and as intensely as if they existed in reality.

Frustrated and aggressive feelings are liberated toward the bad object; a desire to possess and merge with it toward the good.

The representational worlds of the self and the object are intricately intertwined and lead to the establishment of a sense of identity and the building of the intrapsychic structure, whereas the mature ego resists fantasies of merging with the object.

Situations within the representational world may weaken reality testing and tempt a return to earlier, less differentiated ego states.

Retaining some of the elements of drive theory, Jacobson has fused them with a phenomenological approach that makes for a rather complicated structure.

Drawing some substance from the ideas of both Mahler and Jacobson, Otto Kernberg (1976, 1980) has contributed some interesting ideas to contemporary object relations theory while retaining a fidelity to classical metapsychology.

From the stormy transferences of narcissistic and borderline personality disorders, the chaotic nature of self and object representations, the use of primitive defense mechanisms, such as splitting, projective identification, and archaic ambivalences, assumptions are made by Kernberg that early object configurations have not been properly "metabolized." This is the reason why they precipitate out rapidly in the transference.

The original self and object images as well as the affective colorations of drive derivatives constitute the "internalization system." The representational world consists of bad and good self and object images, as www.freepsychotherapybooks.org 479 well as components of the ideal self and object.

As the ego consolidates through the employment of such devices as introjection, the defense of repression relegates unacceptable representations and identifications along with their affective charges to the unconscious.

Splitting separates ambivalent introjects.

These processes are accompanied by strengthening of the superego, formed from hostile object internalization, the ego ideal (combined ideal self and object representations), and realistic parental introjections (commands, values, etc.).

The "self," an intrapsychic structure originating from the ego, consists of the total of self-representations that connect with the total of object representations (Kernberg, 1982).

In working with sicker patients during psychoanalysis, one consistently has to deal with transference paradigms, the product of unmetabolized early relationships, as well as with such surviving primitive mechanisms as splitting and ambivalent self-object configurations, with the hope of achieving a better integration.

Heinz Kohut (1971, 1977), a former president of the American Psychoanalytic Association, avowed that severe narcissistic personality disorders, which ordinarily have not been considered susceptible to psychoanalysis, could be treated analytically.

What was required, he insisted, was a revision of classical theoretical and methodological concepts, while retaining fundamental premises of the drive theory.

Leaning toward a relationship model, Kohut promoted a "self-psychology" that subordinated the structures of id, ego, and superego to the active manipulations of the self.

The self evolves out of participation with others ("selfobjects") with whom the child merges and whose feeling states (empathic responsiveness) the child incorporates.

The child's fundamental narcissistic needs (omnipotence and grandiosity) require that the child be admired for his or her capabilities by the idealized selfobject (a kind of "mirroring" phenomenon), which enhances fusion of the self-image with the idealized selfobject ("you are perfect, and I am part of you.").

Disappointments in "mirroring" by the selfobject hamper idealization but contribute to the development of the self structure ("transmitting internalization").

In one line (pole) of self-development the grandiose exhibitionistic anlage becomes expressed as healthy assertiveness and ambition; in a second alternative line (pole) the idealization of the selfobject results in healthy ideals and values.

Failure to develop adequately along either pole impairs self-development, with resulting damage to the self-image and self-esteem. www.freepsychotherapybooks.org 480

If, because of parental pathology, proper empathy is not displayed, self-development in the child is thwarted, with psychopathological consequences.

The theoretical premises of Kohut have provided a rationale for empathic activity in treatment. Since deficits exist in the constitution of the self, transference in either mirroring or

idealizing modes provides an opportunity to rectify these deficits.

By acting empathically, the therapist gives the patient a second chance to incorporate healthy structures and thus overcome the original selfobject failures.

Gradually, the narcissistic elements are resolved as the therapist slowly adopts a decreasing empathic role and the patient needs the therapist less and less as a selfobject.

What is responsible for character reconstruction, according to Kohut, is a fruitful experience with an empathic and idealized therapist (the transferential selfobject) rather than interpretation, the touted classical agency that presumably brings about change.

Instead of conceiving of the development of the psyche in terms of the conflict between primary drives within the individual and the socializing demands of parental authority, self-psychology thus focuses on the vicissitudes of the self and object representations in their archaic and mature forms.

Agreeing with Kohut, Ornstein (1984) conceives of the first pole of the bipolar self as the grandiose exhibitionistic self, which becomes the nucleus of later self-assertive tendencies, regulation of self-esteem, enjoyment of mental and physical activities, and successful pursuit of goals and purposes.

The second pole involves the idealized parental image incorporated in the selfobject matrix, which becomes the basis for values and guiding ideals, helps regulate inner tension, contains and channels drive needs and affects, and fosters enthusiasm and idealization of one's values and edifying principles.

Psychopathology is the product of developmental arrests or derailments that produce disturbances in the bipolar self and failure of maturation.

Resultant is a continued dependence on archaic selfobjects and an inability to harmonize one's directive values and ideals with innate skills and talents toward satisfaction of one's self-assertive ambitions.

The psychoanalytic cure consists of mobilizing the regressive transference neurosis in the usual analytic way and of employing it to "break the bondage that formerly tied the archaic self to the archaic selfobject" and then establishing "empathic in-tuneness between self and selfobject on www.freepsychotherapybooks.org 481 adult, mature levels." A growing number of analysts have climbed onto the bandwagon of object relations theory.

Utilizing some of the formulations of Sullivan, Klein, Mahler, Fairbairn, Winnicott, Kernberg, Kohut, and others, they have expanded on and enriched certain elements of the theory.

Some contemporary theorists have seemingly forgotten the contributions of the earlier workers who presented the original formulations.

Others have paid proper tribute to the innovators who preceded them.

Among these contemporaries are Joseph Sandler, who has elaborated on how the child's representational world evolves, develops, and forms the basis for the organization of later experience (Sandler & Rosenblatt, 1962) and (Sandler & Sandler, 1978).

His formulations are clearly expressed and possess a clinical utility.

The phenomenological viewpoint of Stolorow and Atwood (1979) and the work on developmental arrests by Stolorow and Lachman (1980) are of interest.

Greenberg and Mitchell (1983) have done an excellent job of showing how object relations theory evolved from ego psychology and of integrating the contributions of the various authorities in the field.

There is some evidence that an object relations orientation adds a dimension to classical structural analytic theory, particularly in relation to self and object representations and to the identification in borderline cases of such pathological defenses as "splitting" and "projective identification." The value of modern object-relations theory is obscured, however, by the use of confusing language that lends ambiguity to its inherent concepts.

Some, but by no means unanimous, agreement exists that an object relations approach is better designed for problems that date back to early ("pre-Oedipal") life and that classical methods are more applicable to difficulties originating around a later ("Oedipal") period.

There is a difference of opinion as to whether the improvement achieved with borderline, narcissistic, and other serious personality problems may not be due to the greater activity and empathic involvement of the therapist with the patient rather than to the insights emerging from analysis of the transference.

Moreover, conflicting claims continue to be made that narcissistic problems are actually variants of the Oedipal neurosis.

Thus the introduction of object relations and other theories of www.freepsychotherapybooks.org 482 personality beyond classical metapsychology is felt by many orthodox analysts to be unnecessary.

In clinical practice most object relations analysts play a more active role than do their classical counterparts.

Instead of operating as a blank screen onto which displacements from the past are projected, they actively participate in the relationship, and some actually play the role of the patient's previous objects or of aspects of the patient's self.

In this way they influence the nature of the transference.

Countertransference is actively watched and used, providing a keen view of the projective activities of the patient.

Except for the repetition of old patterns, the relationship in object relations therapy possesses a momentum of its own.

There is agreement that a regression occurs so that patients live over early developmental stages.

At first there is repetition in the transference of the state of fusion with the protective mother figure in the form of projection onto the therapist of the need for total need fulfillment.

Separation anxiety is apt to emerge.

As therapy progresses, ambivalence develops and the "good breast"- "bad breast" duality emerges toward the therapist or, as in group therapy, a dissociation of the opposing attitudes, one focused on the therapist-leader, the other on the group.

The therapist must have the ability to tolerate and "contain" the products of the patient's "projective identification." The hope is that the patient will eventually be strong enough to reabsorb and tolerate disavowed aspects into the self-image.

As the "good-bad" split becomes absorbed into the self, the "depressive position" develops. Soon concern about how the patient affects others and guilt feelings emerge, and jealousy replaces envy.

Earlier identifications become integrated into the self, which in becoming consolidated enable the person to perceive others more clearly.

A number of attempts have been made to apply object relations theory to group work with children (Soo, 1985) and adults (Alonso & Rutan, 1984).

Criticism of Object Relations Approaches Some therapists consider the contributions of object relations theorists advanced www.freepsychotherapybooks.org 483 and brilliant.

On the other hand, a gathering group of dissenters, while admitting the ingeniousness of the developmental theories, question their validity.

Perhaps the most extensive criticisms have come from Heimann (1966) and Calef and Weinshel (1979), the latter expressing reservations about the clarity of the theoretical conceptualizations and the usefulness of the advocated technical procedures.

Others have branded current ideas about object relations pejoratively as a jerrybuilt structure

of metaphor upon metaphor compounded out of a pseudo-synthesis of Kleinian, Bionian, ego-psychological, object relations, and other theories.

These criticisms, it seems to me, are altogether harsh, for if the authors under criticism have done nothing else, they have challenged the sanctity of some existing metapsychological credendas and opened the way to a reassessment of their value.

This does not certify concepts that modern researchers on development would consider questionable, such as some ideas about infantile development and ideation.

Object relations theory is perhaps more tenable than tripartite structural theory in accounting for some phenomena of the borderline state, but it still does not embrace the multiple variables that enter into borderline personality formation.

Although some object relations theorists continue to pay homage to classical drive theory, there is a feeling among classical analysts that most tend to replace fundamental concepts with an overlay of secondary and derivative issues.

The focus on "good" and "bad" maternal qualities, on basic dependency, and on developmental deprivation as the principal if not exclusive dynamic is considered myopic since it minimizes other crucial factors in the evolution of personality.

Substitution of an innate need for attachment to an object (Bowlby, 1964, 1969; Bulent, 1968) for an instinctual libidinal drive is merely another way of saying the same thing with different linguistic terms.

Similarly, Melanie Klein's challenge of primary narcissism, that the infant is involved with inner objects rather than oriented toward the self, is felt to be merely tautological.

Repudiation of instinctual aggression by such theorists as Fairbairn evades the findings of neurophysiologists of subcortical centers for aggression.

Reinterpretation of the Oedipus complex as less instinctually derived but rather rooted in loving and hateful attitudes toward objects, or due to conflict between warring aspects of the self, are labeled exercises in circumlocution and operations of verbal legerdemain.

Kohut's emphasis on www.freepsychotherapybooks.org 484 infantile grandiosity and idealization as developmentally normal characteristics, rather than regressive and defensive operations, and his justification of playing an active empathic role to make up developmental deficits have been considered by classical analysts as an abandonment of some basic psychoanalytic tenets. The clinical data are interesting, but some therapists find the language and descriptions of self-psychology complex and confusing.

They contend that Kohut has evolved a dynamic to justify his use of activity in his relationship with his patients.

Since activity and empathy are very helpful in the treatment of sicker patients, such as those with borderline and narcissistic personality disorders, modifying the conventional passive psychoanalytic stance is helpful but does not require a modification of the classical developmental theory.

Doing this has not endeared Kohut to the hearts of many orthodox believers.

The severest criticism has been leveled at the additions to developmental theory of ideas related to representational differentiation and integration of self and external objects.

Here it is presumed that neonates cannot differentiate themselves from others nor discriminate between their own and their object's sensations.

The first task of development, according to this idea, is to distinguish and separate self from object representations (usually the mother).

This common notion is believed by certain authorities to be speculative and not in keeping with their opinions of child development, decidedly substantiated by modern developmental research (Kagan, 1971, 1978a, 1978b; Kagan et al, 1978) who point out that object relations theory comes from observations of psychotics and of patients in deep regression (as during a transference

psychosis) who are presumed to be repeating what the normal person goes through in infancy. Studies of development, they contend, indicate that neonates are highly capable of differentiating themselves from objects.

They downgrade the notion that infants are unable to integrate contrasting emotions as related to object representations (i.e., positive affects associated with the “all good mother” and negative affects associated with the “bad mother”) or that infants are at first unable to synthesize “good” and “bad” affects into a combined representation of the total mother figure. The second developmental task, according to object relations theorists, is to effect synthesis of the “good” and “bad” mother, as well as the self-representations that issue from the developmental incorporation of the mother figure (the maternal introject).

Under favorable conditions, this synthesis occurs with the evolvment of “object constancy” and the valuing of other persons for their true positive and negative qualities.

This makes for consolidation of the self-image, acceptance of “good-bad” qualities within oneself, and a solid sense of identity and positive self-esteem.

Failure in such integration is presumed to result in “splitting,” an absence of self-object differentiation and delusional merging of self and object images predisposing to psychosis (Kernberg, 1975; Kohut, 1971; Searles, 1966).

Causes of such failure are variably attributed to constitutional predisposition, severe maternal deprivation, inconsistencies in care, mishandling, and cruelty.

Milder difficulties in self-representation other than psychosis are said to occur where the damage is not so serious, resulting in (1) narcissistic personality disorders, who will require special kinds of management therapeutically (Kohut, 1971), and (2) borderline personality disorders (Kernberg, 1975) in whom “allgood” and “all-bad” self and object representations alternate and who also need special techniques.

Again, these concepts derive from work on borderline and psychotic adults and have not been substantiated by modern researchers in child development (Bower, 1977; Condon and Sander, 1974; Dunn, 1977; Kagan et al, 1978; Le-win, 1975; Thomas and Chess, 1980).

Alternative explanations have been offered founded on empirical studies.

For instance, detailed observations of infants have shown the presence of a need for investigation and exploration as a means to task mastery.

The urge for social relationships seems also to be basic, and this is satisfied by mutual interactions of the infant and mothering figures, not as a fused image, but as separate and individually functioning beings.

This does not detract from the ingenuity of the object relations formulations and even their usefulness in giving the therapist some model around which to organize interventions and to enhance confidence in what he or she is doing.

Attempts that have been made, however, to explicate these ideas in “metapsychological verifications of psychic structures, cathectic shifts, or fusions and defusions of drive-energies” (Stolorow and Atwood, 1979) are extremely confusing.

www.freepsychotherapybooks.org 486 PSYCHOANALYTICALLY ORIENTED PSYCHOTHERAPY The supportive and reeducative tactics within a psychoanalytic framework has come to be known by a number of designations, such as “dynamic psychotherapy,” “psychoanalytically oriented psychotherapy,” and “insight therapy.” Conceptually and operationally, psychoanalysis and psychoanalytically oriented psychotherapy are distinguishable (Gill, 1951, 1954; Bibring, 1954; Greenson, 1967).

Nevertheless, there has been a tendency to lump them together under the banner of psychoanalysis, some analysts quoting Freudian scripture to justify this amalgamation (Fromm-Reichmann, 1950, 1954).

Much debate and controversy has crystallized around these issues (Glover E, 1931; Alexander, 1954; Bibring, 1954; Fromm-Reichmann, 1954; Knight, 1954a & b; Rangell, 1954; Waelder, 1960; Tarachow, 1963; Wallerstein, 1966; DeWald, 1964).

Nevertheless, the clinical usefulness of modified approaches is conceded since classical analysis is contraindicated in certain conditions.

Uncovering and exploratory techniques that do not penetrate into infantile genetic sources or sponsor a regressive transference neurosis are considered helpful in patients with circumscribed symptom and character neuroses who possess sufficient ego strength to endure probing procedures.

Here the therapist does not fully present as an object through whom gratifications are sought and obtained, as in supportive therapy; nor does the therapist stimulate the patient's unconscious fantasy system by posing obdurate therapeutic interpretation barriers to all transference demands until a transference neurosis erupts, as in Freudian psychoanalysis. Rather, the therapist takes an intermediate position, offering himself or herself as a projective vehicle for some transference gratification while doing as much analytically interpretive work as the patient can tolerate.

Psychoanalytically oriented psychotherapy is thus the most active of all analytic therapies and maintains the greatest flexibility in the techniques employed (Stewart, 1985).

By focusing on pertinent data, using active means of dealing with resistance, and bringing unconscious conflicts to awareness without resorting to deep regression to invoke a transference neurosis, much unnecessary time is claimed to be saved.

The relationship is accordingly actively manipulated, transference is controlled, and certain aspects of the relationship are stimulated.

The focus is generally more on the present than the past and deals with limited rather than complete aspects of the personality.

Recent conflicts more than past ones are in the forefront.

Frequent weekly sessions, the couch position, and free association are only occasionally employed, if at all.

Practical goals of bolstering defenses, promoting enhanced adaptation, and encouraging more effective interpersonal relationships are considered adequate, and a complete overhauling of the personality structure is considered a fortunate but serendipitous happening.

Psychoanalytic psychotherapy therefore has a broader range of application than formal analysis and is adaptable both to problems of relatively healthy individuals and to sicker patients with whom psychoanalysis cannot be employed.

The use of adjuncts, such as drugs, family therapy, and behavior approaches, is often resorted to as indicated by the immediate needs of the patient.

Flexibility of operation is the keynote.

Consequently, there is no accepted format, the design of treatment being determined by the therapist based on what is indicated at the time and his or her expertise with special adjunctive methods.

Perhaps the best known system of psychoanalytically oriented psychotherapy has been set forth by Alexander, French, and other members of the Chicago Institute for Psychoanalysis (1946).

The therapy described, though of a short-term nature, is believed by the authors to yield results comparable to long-term standard psychoanalysis.

Alexander and French stress the fact that they utilize the therapeutic situation as a corrective emotional experience.

It provides a new and more favorable medium in which the patient is exposed to, relives, and finally masters the conflicts and emotional problems he or she was unable to handle as a child.

This is achieved either in the transference relationship or outside of therapy in real life.

Not only does the patient overcome unresolved childhood conflicts by reliving them—which makes them less acute—but the therapist also responds to the patient and to the patient's behavior in a manner totally different from that of the parent.

This gives the patient an opportunity to revive his or her past and to face conflicts over and over again under the guiding aegis of the therapist.

Activity in the therapeutic situation is said to accentuate the corrective experience.

The authors recommend such modifications in technique as direct interviewing, in www.freepsychotherapybooks.org 488 addition to free association; the regulating of the number of sessions each week; the offering to the patient of advice and suggestions about certain aspects of his or her life; the interrupting of therapy for a variable period prior to ending treatment; the manipulating of the transference in each patient in accordance with the needs of the patient; and the employing of real-life experiences as a part of the treatment process.

Flexibility in method is advocated with a change of technique periodically to suit the patient's personality as well as problems.

While the transference relationship is believed to be important, extratherapeutic experiences are considered of equal importance.

Positive transference is encouraged to establish rapport and to enhance therapeutic progress.

Negative transference is analyzed when it blocks the process of therapy.

Emphasis is more on the relationship than it is on the transference neurosis.

The experience of mutual frankness and sincerity in relationship to the therapist makes it possible for the patient to reorient to other human contacts.

F.

Alexander (1965) has reemphasized that psychoanalytic psychotherapy acts as a restitutive medium that the therapist actively manipulates, scrupulously avoiding repeating the traumatizing circumstances of the patient's childhood no matter how provocative he or she may be.

Once the therapist becomes alerted to the nature of past hurtful experiences with parental figures, the therapist may, by design, act in a way completely opposite to that of the parent so as not to bring to pass the patient's expectations.

Control of transference is vital.

Another technique is to change the frequency of interviews at certain phases of treatment to make the patient conscious of dependency needs by frustrating them.

Experimental temporary interruptions in therapy are also useful.

Overtreatment, with its regressive dependency, should be avoided by reminding the patient that treatment will be made as short as possible.

This discourages procrastination in facing up to important issues.

The search for early memories is not as important as dealing with the present and with immediate life problems.

Avoidance of a "parentifying" transference so as to enable the patient to take over his or her own life management as rapidly as is feasible is urgent.

While these techniques are not applicable in all cases, the majority of patients, according to Alexander, can benefit from them.

Results justify the modification of formal psychoanalysis in many patients.

www.freepsychotherapybooks.org 489 Psychoanalytic psychotherapy supports many different modes of operation unique to the personalities and styles of therapists.

These are sometimes unjustifiably described as revolutionary, but almost invariably they are merely adaptations of earlier forms of treatment.

For example, many have drawn inspiration from the "sector therapy" of Felix Deutsch (1949a & b; Deutsch & Murphy, 1955).

This is a goal-limited therapy “conducted so as to work on the unconscious factors that influence the reality situation rather than the reality situation itself.” Focus is on a limited portion of the total problem, the object being to adjust the patient to an aspect of life that hitherto he or she has been unable to master.

An “induced positive transference” is employed, and “associative anamnesis used in lieu of free association.” Here the technique essentially involves a focusing of the interview on symptoms and conflicts by accenting key words and phrases that the patient has used and that reflect basic problems (Deutsch, 1939).

The therapist picks a few of the most frequently employed words or expressions, incorporates them into the conversation, and observes the reactions of the patient.

The key words and phrases uttered by the therapist usually stimulate associative ramifications in the form of free associations.

The associations are guided, however, and their continuity is maintained.

Through this means, manifest symptoms and current problems are linked with underlying conflicts.

The constant confrontations by the therapist serve some of the purposes of interpretation.

Memories are revived; associative chains are broken up and replaced with new ones.

Through this form of interviewing, the patient purportedly learns to discriminate the past from the present and his or her ego is induced to alter its defensive attitudes.

Illustrative of the many innovations that have been introduced into psychoanalytically oriented psychotherapy is the “objective psychotherapy” described by Karpman (1949).

He assigns to the patient a series of written questions dealing with the patient’s history, attitudes, and feelings.

The patient is requested to write out detailed answers to these questions.

After reading them, the therapist picks out pertinent points and formulates new questions to which the patient is expected to reply.

Reading material may be given to the patient that is related to his or her problem, and the patient’s reactions in writing are requested.

Dreams are written by the patient, and interpretations to these are handed in written form back to the patient for leisurely study.

Formulations of www.freepsychotherapybooks.org 490 the dynamics are made in writing from time to time by the therapist, and the patient is requested to study these and to turn in written comments.

Focal therapy is the name given by Balint et al.

(1972) for a concentrated short-term form of insight therapy.

To qualify for this treatment according to the authors, a patient has to be able to establish a reliable therapeutic relationship with the therapist, the illness needs to be ego-dystonic and not a valuable part of the patient’s personality, interpretations made by the therapist must be acceptable to and acted on by the patient, and both parties should agree on a suitable specific focus (a “meeting of two minds”).

The focus often is on crucial unconscious conflicts, the resolution of which will, one hopes, make at least a limited impact on the patient’s character structure.

Occasionally, as therapy proceeds, the focus has to be modified and even changed.

The technique calls for greater manipulateness than is customary in psychoanalysis, although interpretation is still the basic intervention.

The theory of treatment and of the involved psychopathology are psychoanalytic, but the method is distinguished from true psychoanalysis by its inherent activity, the abandonment of free association, and the avoidance of a transference neurosis.

Since its early presentation, the term “focal therapy” has been appropriated by therapists of

various orientations as a label for short-term treatment targeted at various foci (symptoms, behavior difficulties, personality problems, situational crises), and psychoanalytic techniques are commonly employed.

George Goldman (1956) has written on "reparative psychotherapy," which is eclectic in nature, utilizing various supportive and insight approaches depending on the needs of the patient. The external manifestations of the problem are first handled in dealing with the disturbed behavior and reality situation.

Gradually there is a working toward emotional factors, including the defenses.

The primary aim of "reparative therapy" is the modification or relief of symptoms.

A secondary goal is improvement of adaptive functioning.

A hoped-for objective is growth of the individual toward greater maturity or capacity.

Deep-seated characterological change is not considered a target in reparative therapy, although reconstructive successes may on occasion be scored.

An interesting psychoanalytically oriented "humanistic" "third force" approach is www.freepsychotherapybooks.org 491 that of Gestalt therapy.

Drawing from gestalt theory and employing concepts from psychoanalysis, Perls, Hefferline, and Goodman (1951) evolved a transactional form of therapy that proposed to bring fragmented elements of personality into a unified whole.

Ideally, it was theorized, an object ("figure") and its field ("ground") should blend into a harmonious assemblage (Gestalt), the interplay constituting a balanced unit of emerging and receding "figure" and "ground." According to this theory, neurotic and psychotic individuals, because to their rigidity (fixation) or imperfect "figure" formation (repression), are burdened with defective split-off Gestalts.

These make for confusion, anxiety, and the various symptoms of emotional illness.

It follows that the psychotherapeutic focus must take into account both the individual and the environment as well as their subtle interaction with one another.

This task necessitates a search for repressed material, the mechanisms through which repression is maintained, as well as the specific needs for repression.

To organize a mature figure-ground Gestalt, which is the essence of maturity, it is essential that the individual restore to his or her total being the split-off and dissociated aspects of himself or herself.

Guidance in bringing these to awareness is indispensable.

More important, an adjustment to the present situation is fostered by presenting actively to the patient a series of "therapeutic experiments" that provide a dynamic educational experience.

Not only does the patient learn through this about the repressed and repressing self, but he or she is also faced with graded challenges that help in mastering fears and blocks.

The creative resolution of elements of unawareness, which is fostered by the experiments, is said to restore the individual to personality integration and constructive contact with reality.

The function of many of the experiments is to integrate alienated aspects of the self.

One technique is to animate and hold conversations with parts of one's body that feel tense or people and objects in dreams, or one with fantasized parents and other important personages. During this dialogue, the patient acts, feels and talks as if he or she is actually the object or part object.

Sometimes the "empty chair" technique is used, the patient being urged to imagine a parental or other important figure sitting in a nearby empty chair and then to talk to, upbraid, or question the visionary occupant.

This may be followed by changing seats and acting the part of the person or object with whom

there has been a www.freepsychotherapybooks.org 492 dialogue.

A great deal of emotion may be liberated in the process.

The therapist in the meantime questions, challenges, and confronts the patient about the behavior and evasions, at all times insisting that he or she take responsibility for his or her actions.

Group work may be an integral part of therapy, and many of the techniques of psychodrama, such as role reversal, are employed.

J.

A.

Greenwald (1972) has outlined a number of useful ground rules for Gestalt therapy.

During the last years of his life Perls was active at Esalen Institute in California, elaborating on a number of active techniques and gathering around him a large number of followers who enthusiastically worked with his methods, the flavor of which may be gathered only from reading recordings of his interviews, listening to his cassettes, or watching his films.

An interesting conversation with Perls is published by Clements (1968).

Gestalt therapy enjoys a great popularity among younger generations of psychotherapists principally because of its dramatic techniques.

Gestalt Therapy Institutes have been organized in various parts of the country offering workshops in the method.

Some of the Gestalt techniques are utilized periodically by therapists of various schools as a means of stirring up activity in treatment when movement is deemed necessary.

Not all therapists are capable of doing this, however, particularly if they have no confidence in the methods or believe that they are too contrived and artificial.

Not all patients can handle the challenging and confronting manner of the Gestalt therapist.

A good working relationship is required for this, and if attacks on neurotic ego-syntonic behavior are launched prematurely, the patient may be driven out of treatment.

Borderline patients particularly cannot tolerate the attacks.

The anti-intellectual stance of Gestalt therapy has as its emphasis the undiluted, direct experiencing of emotions.

The gimmicky quality of the techniques have come under attack by more conventional therapists.

Some of the Gestalt techniques have nevertheless been incorporated into other therapies.

Such therapies draw from the inventiveness of the leader in devising interventions toward integration of split-off aspects of the self.

Responsibility for one's actions is constantly stressed with the need to resolve the "unfinished business of the www.freepsychotherapybooks.org 493 past," to abandon "phoniness" for true self-awareness, to liberate oneself from the "computer" within one's mind which delivers false messages, to discard useless hoped-for illusions and goals, to stop looking for the "why" instead of the "what" and "how," and to give up banking on help from others (including the therapist) rather than oneself since that is a "sick game." Used by qualified professionals, selected Gestalt techniques may, during individual therapy or in groups, help break through resistance in rigid, guarded individuals who are then provided with opportunities to work through their liberated memories and emotions.

Rigid adherence to the theories of the founders of the Gestalt movement may tie the therapist into a bind, however, particularly in relation to the management of the ubiquitous transference and countertransference eruptions encouraged by the great activity inherent in the treatment process.

Other analytically oriented approaches have been designed, limited only by the originality of the authors, that employ various degrees of activity.

These range from a mere pointing out by the therapist of destructive coping mechanisms (which may be helpful to some patients with a strong readiness toward change) to interpersonal provocations within the therapeutic relationship that are calculated to mobilize defenses and resistances.

Instead of responding to these in traditional ways, the therapist challenges and even opposes them.

Deliberately created by the therapist in some cases is a state of crisis to which the patient will react with frustration, bewilderment, rage, and other reactions which are superimposed on his or her habitual defensive operations.

It is at this point that the skill and stability of the therapist are taxed to the limit.

If a working relationship and good communication with the patient has been established, with appropriate management of any countertransference, the therapist may be able to support the patient through the upheaval, firmly standing ground, while manifesting empathy and understanding.

The patient is confronted with two situations: either he or she leaves therapy or changes.

One hopes the latter contingency will prevail.

Criticism of Psychoanalytically Oriented Psychotherapy www.freepsychotherapybooks.org 494 The marriage of non-analytic and analytic techniques has, in some circles, not been too happily received, non-analytic therapy being regarded as a kind of ravenous cormorant who eventually swallows, digests, and destroys its marital partner (Waelder, 1960).

The progeny of this wedlock, too, in the form of specific procedures, are regarded by some analysts as monstrosities conceived through the mating of opportunism with clinical expediency. Criticism is expressed to the effect that transference is watered down by the active techniques employed.

The consequence of avoidance of a transference neurosis is said to be a limitation of the extent of insight achieved.

The inevitability of circumscribed goals in treatment is also presented as an objection.

Additionally, the concentrated short-term techniques employed are believed to be dangerous in the hands of any other than the most highly trained and experienced psychoanalysts.

They are unfortunately apt to appeal to therapists who, while not trained in psychoanalysis, are searching for dramatic unrealistic psychoanalytic shortcuts.

To these criticisms adherents of psychoanalytically oriented psychotherapy reply with accounts of greater effectiveness in the management of patients, over a relatively brief period, who would not be suited for the classical approach, which is applicable to a limited number of individuals.

TRANSACTIONAL MODELS OF PSYCHOTHERAPY In recent years a form of psychotherapy has come to prominence, the therapeutic focus being the ongoing transactions that take place between patient and therapist.

The elicitation of defenses that the patient employs, the understanding of the roles that he or she plays, and their "working through" within the relationship have been credited with influencing various dimensions of personality, including interpersonal and intrapsychic components.

In establishing the theoretical foundation for transactional therapy, concepts have been adapted from field theory, communication theory, game theory, and role theory, especially the latter two.

Game theory, which was developed by Emile Borel, John von Neumann, and Oskar Morgenstern for the study of competitive economic behavior, attempts to analyze conflict in mathematical terms by abstracting common strategic elements controlled by the www.freepsychotherapybooks.org 495 participants.

The theory is applied to various kinds of conflict situations, including neurotic behavior and patient-therapist transactions, conceiving of the maneuvers as chosen strategies employed by the "players" under established "rules" that will bring about a favorable outcome ("payoff"). Rationality is stressed, the players presumably taking into account in their "bargaining" situations various calculations of risk.

In role theory society is regarded as an aggregate of persons with common goals whose positions enjoin them to assume specialized roles.

The individual, constituted of values, traits, and attitudes (including "self"), develops action systems as a result of the interplay of self and role.

Role theory deals with reciprocal relationships that go on between people.

It also embraces the interaction of self and role.

It contemplates personality as a tapestry of role behavior, of role perception, and of self-perception in the matrix of role.

The conventional definition of role as employed G.

H.

Mead (1934) regards it as the pattern of attitudes and actions an individual exhibits in social situations.

This is molded by one's status or position in the social structure, which obliges one to behave in certain ways.

Special actions are expected of persons occupying certain roles ("role expectations") that meet some need in the social system (Parsons & Shils, 1951).

The individual consequently organizes behavior in order to fulfill role expectations.

Thus the student assumes with the teacher the role of learner; the teacher, the role of educator.

The child learns that under set circumstances behavior in certain ways is expected.

Conformity brings rewards: revolt results in punishment.

The little boy finds that his roles in society, his privileges, and his liberties differ from those of a little girl.

As he matures, his role expectations and behavior change.

Juveniles, adolescents, and adults perform differently both through intentional instruction and through incidental learning.

The individual ("ego") utilizes many roles at various times and under varied situations with different people ("alters").

When a role coordinates with that which the individual is expected to play, he or she is in relative equilibrium (a state of "complementarity").

When role complementarity is upset, disequilibrium follows with www.freepsychotherapybooks.org 496 ensuing tension and anxiety that nurture neurotic disturbances in self-evaluation and difficulties in interpersonal and group relations.

Roles may be conscious and explicit or shunted from awareness and more or less implicit.

In the latter case the roles usually reflect early identifications, such as mother-child, teacher-child relationships.

These identifications are only partly expressed in social roles; some are neutralized and locked in.

The expressed roles force the individual to repeat compulsively time-worn and habitual pathological patterns.

Ambiguous role expectations lead to conflict and to socially invalid role enactments.

For example, many women in our culture, tending to equate some aspects of the feminine role with inferiority, resist complying with certain role expectations.

Indeed they may seek a solution to their conflict by assuming a masculine role in some areas of

adjustment.

Role conflicts in such instances are inevitable.

To some extent role conflicts encompass all human beings, since in certain areas of functioning every individual occupies two or more positions simultaneously and is unable to live up to all of his or her role expectations.

The degree of role conflict and the inability of the individual to evolve adequate defenses to the dilemmas posed by one's ambivalencies will determine their pathogenicity.

The role an individual believes must be played in a social context is a product of perceptual and conceptual fields, which involve past experiences and embrace many intrapsychic processes. The overt factors of role perception (for instance, the acts and appearance of others) are colored and even distorted by internal needs and conflicts.

Thus, when excessive dependency is a lingering impulse not resolved in the course of maturation, the individual will implicitly assume the role of child with any person whom he or she perceives as an actual or potential parental substitute.

The specific position assigned to the parental symbol will be that of authority.

When the symbol does not come up to the demanded role expectation of being and acting as the authority, conflict may ensue.

Though the unconscious is not acknowledged by some role theorists, its operations are present, prompting motivations that drag the individual to act out roles in
www.freepsychotherapybooks.org 497 opposition to traditional role expectations.

Role enactment then may not validate the expectations of the person or persons with whom the individual is relating.

This can give rise to conflict leading to mutual retaliation and other defenses.

Multiple roles are the rule: the richer the repertory of role potential, the more flexible and integrated the individual (Gough, 1948; Cameron & Margaret, 1951).

Role theorists regard the self as an intervening variable that can be approached through role concepts.

The elusive entity of the self is an aspect of the total cognitive organization, an inference derived from interaction with other persons, objects, and events.

The self evolves as an organization of qualities, the result of maturational and personal-social experiences.

Its formation is involved with the principle of "constancy," the need for homeostasis in the child bringing about responses that invoke the aid and intervention of other individuals.

Awareness by the child of the "somatic self" that embraces tensions is the basis of the differentiation of self from nonself.

External events become associated with tension reduction, and the child's perception that certain motor activities on his or her part in relation to others lead to events that eliminate tension acts as further support for the foundation of self.

Toward the end of the first year a new cognitive structure is laid down in the form of gestures and other forms of communication that enable the child to differentiate persons and objects from the acts of persons.

At the same time there is discrimination between self-acts that are approved or disapproved. Perceiving, identifying, and conceptualizing of roles and role expectations are elaborated as symbolic development expands.

The self-concept in role theory is thus intricately related to the socialization process and to reciprocal role playing with many significant persons.

Out of such interactions there evolves the "social self." Difficulties in development result in "fixations" of the self concept on primitive or less mature levels than the "social self." An interesting finding issuing from psychoanalysis that relates to the development of role

expectations is that the child will divine, by uncanny perception, the verbally unexpressed but, nevertheless, obvious unconscious designs of the parent with whom he or she identifies. There is an acting out either as a child or later in life, some of the
www.freepsychotherapybooks.org 498 unconscious parental needs and demands.

Antisocial tendencies are often a reflection of the unconscious urges of parents, who, in their eagerness to conceal these promptings, give them undue emphasis in the form of warnings, reaction formations, defenses, and symbolized expressions of repudiated drives.

The parent may also confuse the child as to roles by alternately encouraging (seducing) and punishing (rejecting) the child.

Harmony between the self and role enactment is one of the measures of adaptation.

Performances to satisfy role expectations that are incongruent with the self (self-role conflicts), or two or more role expectations that clash with one another (role-role conflicts), will interfere with adjustment.

Ego-defense mechanisms are elaborated to reconcile such differences and to maintain a constancy of the self in the face of expressing such discordant role expectations.

Many self-maintaining mechanisms, such as the evolvment of a rationalizing philosophy, are implemented to support incongruent roles that are regarded as essential for adjustment.

When self-maintaining mechanisms fail, neurotic defenses may be exploited to bolster the self.

If "constancy" (homeostasis) is still precarious, autistic and desocialized role enactment may come about.

When two or more roles are incompatible, and institutionalized forms are not available to reconcile opposing role enactments, conflicts may also ensue with eventuating defensive and disorganizing tendencies.

A number of transactional models have appeared.

Illustrative are those elaborated by Berne, Grinker, Haley, J.

Rosen, and Whitaker and Malone.

The Transactional Model of Grinker A transactional approach is described by Roy Grinker (1961) that consists essentially of focusing on the roles the patient and therapist are playing with each other while taking into account the current environmental influences.

Posing the question as to whether it is possible to set up a model of psychotherapy based on empirical operations rather than on a theory of psychodynamics or diagnosis, Grinker contends that personality cannot be understood through intrapsychic processes; rather, it is best considered as a system of interactions among human beings within a social organization.

In therapy both explicit and implicit roles constitute the transactions

www.freepsychotherapybooks.org 499 between therapist and patient.

Complementarity of roles is evidence of stability and permits communication of information.

Noncomplementarity may lead to disequilibrium, which then promotes the establishment of a new system and modification of roles.

During the therapist-patient transaction old roles, preconscious and unconscious, are revived and their origins in past experiences come up for discussion.

Both therapist and patient are involved in this transactional process, each acting on the other with a constant feedback of respective roles.

As the patient reveals through communication the desired relationship with the therapist, the latter interprets the designs of the patient in the hope that this learning will generalize to other relationships.

In the communication with the patient the therapist freely discusses his or her positive and negative feelings.

Refusal to play the role demanded by the patient will mobilize defensive reactions in the patient.

But if the therapist stands firm, new solutions will be sought.

Among the techniques are the following: focusing of the interview on the ongoing transactions in therapy rather than past experiences; the use of pointed directiveness, confrontations, and challenges; a firm curbing of acting-out; avoidance of silences; employment of dreams only as a source of information without interpretation; the extension of support when needed; and avoidance of a transference neurosis.

The patient is discouraged from using childish and immature forms of communication by interpreting their function and is encouraged to experiment in relationships outside the therapeutic situation.

This orientation, writes Grinker, “facilitates a vivid, current understanding of the patient without recourse to reified variables, of unconscious, transference, countertransference, resistance, topological foci, processes involving energy, or any past functions of the human being in behavior.” The Transactional Model of Haley Another approach is that of Jay Haley (1963b), who considers the therapeutic process a communicative transaction between the therapist and patient.

According to Haley, the essence of what happens in all forms of treatment is the determined struggle for control between therapist and patient.

Each of the participants is bent on winning this contest: the patient, through the ploys of symptoms; the therapist, through varied contradictory confrontations (“therapeutic paradoxes”) presented to the patient www.freepsychotherapybooks.org 500 adversary.

Human beings relate through an interchange of messages that define the nature of their relationship.

A “symmetrical” relationship is one in which each person initiates action, criticizes, offers advice, and otherwise shows essentially the same type of behavior.

Such a relationship tends to be competitive.

A “complementary” relationship is one in which different types of behavior exist, interlocking with each other: for example, one teaching, the other learning; one giving advice, and so on. Relationships are never completely stable; they keep shifting as people grope for a definition of their relationship.

Maneuvers in a relationship consist of requests, commands, and suggestions that another person think, say, feel, and act in certain ways, as well as comments about the partner’s communicative behavior.

Relationships become pathological when one person attempts to circumscribe the other’s behavior while denying that he or she is doing this.

For example, a directive may be delivered, then qualified with the statement that it does not have to be followed.

Out of this conflict a series of paradoxical communication patterns evolve.

Psychopathology may be viewed as a set of maneuvers to gain an advantage, namely, the control of a relationship.

Psychiatric symptoms offer the patient advantages by setting the rules for relationships and by making the social world more predictable.

Under these circumstances therapy must aim at preventing this usage of symptoms while encouraging other ways of handling relationships.

Exposing himself or herself to therapy, however, poses for the patient the threat of giving up his or her habitual control over relationships.

A duel for control can be expected, therefore, though this may be vigorously denied by both patient and therapist.

It is vitally necessary, however, that the therapist deal successfully with the question as to who will control the relationship—the therapist or the patient.

The resolution of this question is the basis for therapeutic change.

If the patient wins, he or she will perpetuate the problems by governing the relationship with the symptoms.

If the therapist gains control, the patient will have an opportunity to change.

The stratagems that the therapist must employ in winning control are, therefore, the crux of treatment.

www.freepsychotherapybooks.org 501 Stratagems are used in all therapies.

In directive approaches, for instance, the therapist may tentatively align with the resistances of the patient and his or her need to retain symptoms.

Permission having been given the patient to embrace the symptoms on the therapist's terms, the patient is then extorted into yielding them by various devices.

In deconditioning techniques the principal agency is not desensitization, but the therapist's taking control over the patient's behavior and the patient obeying the prescriptions of the therapist and operating under the latter's terms.

In nondirective therapy, therapists, even though they imagine themselves to be uncontrolling, do not give in to the controllingness of the patient and actually establish themselves as the agency in control.

Insight leading to self-awareness is not essential; transference interpretations need not be made; and connections established between the patient's past and present are dispensable.

Thus, in evaluating the effects of psychoanalysis, Haley suggests that exploration of the human psyche in quest of insight is irrelevant to change.

It is not self-awareness that promotes health; interpersonal transactions alone are responsible for any transformation that develops.

Emphasis on internal processes (such as the unconscious), resistances, and mechanisms of defense are diversionary from what is more significant, namely, that in silence and nondirectiveness the therapist is subtly demonstrating that the patient cannot control the analyst's behavior.

From the start the analyst gains the upper hand, though there is denial that the patient is being directed, thereby facing the patient with a paradox.

Another way of establishing control is for the analyst to interpret that which the patient ostensibly knows nothing about—his or her unconscious.

Any disagreement is classified as resistance.

Moreover, crediting the patient's symptoms to deep symbolic forces within the patient prevents the patient from manipulating the therapist through the symptoms.

Even philosophical therapies, such as Zen Buddhism, operate through the principle of changing the student's conception of reality by controlling the student's thinking through the setting up of paradoxical situations.

In summary, Haley contends that what causes change in psychotherapy is not self-awareness brought about through the divulging of the operative dynamics, conscious or unconscious, or deconditioning, or any of the other factors commonly ascribed to the

www.freepsychotherapybooks.org 502 healing process, but the resolution of the therapeutic paradoxes that appear in the relationship between psychotherapist and patient.

During therapy a number of paradoxes are presented to the patient in the form of contradictory demands (these are deliberately planned therapeutic strategies that generally take the form of wresting control from the patient) to which adjustment will be required with completely different problem-solving tactics than are customary for him or her.

Cure is registered when the patient does not really care whether the therapist is controlling or whether the patient is at the helm.

Effectuated then are new and healthier strategies in interpersonal relationships.

In the process of resolving the paradoxes in therapy the individual stops employing symptoms as modes of control.

The first paradox that confronts the patient during treatment is that he or she enters “voluntarily” into a therapeutic relationship that, at the same time, is actually compulsory in terms of the rules that are being set.

The second paradox is that the therapist who professes an interest in the patient actually is performing a paid service.

Is he or she seeing the patient because of desire or because of discharging a duty?

Why, if as interested as the therapist seems to be, will the therapist not socialize with the patient?

The focus of the therapeutic process is the relationship with the therapist.

Regardless of the theoretical system and the techniques employed, the real drama is shifting the balance of control from the patient (by symptoms) to the therapist.

Gradually the balance tilts from a helper-recipient (complementary) relationship to a cooperative (symmetrical) one.

It matters little, according to Haley, what particular brand of therapy one practices so long as strategies employed wrest control from the patient.

Diverted from customary modes of manipulating relationships through the perpetuation of symptoms, the patient progresses toward different and more realistic approaches to problem solving.

The Transactional Model of Berne Eric Berne (1961), utilizing concepts from psychoanalysis, communication theory, and game theory, has succeeded in developing a system of “transactional analysis” that delineates some of the processes operating in interpersonal relationships in simplified terms, which makes them palatable to the average person.

By restricting and focusing transactional operations on several common and universal groupings, it is possible with www.freepsychotherapybooks.org 503 this technique to bring patients to an awareness of certain basic characterological defects in themselves and others in a rapid and dramatic way.

Because the transactions between people are described as “pastimes,” “games,” and “scripts,” and because they are tagged with humorous, salty, and quippish labels, the patient develops a tolerant and understanding attitude toward the frivolities of other human beings, while becoming critically understanding of his or her own histrionic interplay with people.

Ego-syntonic attitudes, ordinarily difficult to acknowledge, are then isolated as playful absurdities that yield a dubious “payoff.” The patient learns to laugh at himself or herself and soon to inhibit various operations as ego-alien.

As a short-term process, practiced individually and particularly in groups, easily taught to professionals, “transactional analysis” may serve a useful purpose for some patients, provided that it is not blown up as the ultimate answer to all the problems in psychotherapy.

Berne conceives of human relationships as repetitive sets of social maneuvers that serve a defensive function and yield important gratifications.

Such maneuvers take the form of “pastimes” or “games” that people play.

These may be simple stunts, or they may be elaborate exercises that follow an unconscious life plan or “script.” Manifested in all persons are three different “ego states”: first, the child within the person, a regressive relic of the individual’s archaic past, hence an aspect of his or her “archaeopsyché”; second, the external parental agency (parent), whom the person has incorporated through identification, the “exteroopsyché”; third, the grown-up, mature, reasonable “data-processing” adult self, the “neopsyché.” Each of these aspects of the person perceives reality differently: the child part, prelogically and distortedly; the parent part, judgmentally; the adult part, comprehensively on the basis of past experience.

The three states are constantly operating in response to the needs of the person and the kinds of pastimes and games that he or she is indulging at the time.

For example, in response to a story about embezzlers, the parent acts moralistically (i.e., plays the game "Blemish"), the adult is interested in how the embezzlement was managed (i.e., plays the game "Accountant"), the child acts naively and thinks of how interesting it would be to perpetrate the embezzlement (i.e., plays the game "Cops and Robbers").

The three ego states may come into conflict with each other, symptoms being the consequences of the child struggling with the parent, both "contaminating" the adult.

During therapy, the three ego states, displayed in the relationship with the therapist, are interpreted to the patient.

The function of child and parent within the patient and the origin of these states in the life history are ventilated.

The irrational defenses the patient employs to justify them as adult operations eventually become clear.

Ultimately the adult becomes stronger and displaces the child and parent.

Prior to engaging in transactional analysis, the therapist makes a diagnosis through "structural analysis," by becoming sensitized to the patient's demeanor, gestures, vocabulary, and voice in order to determine which ego state is exhibiting the symptom or is responsible for disturbing characterological features.

Psychopathology is designated and rephrased in terms of the parent-adult-child trilogy ("the cathexis of anthropomorphic precipitates").

Thus, hallucinations are usually exhibitions of the parents, utilizing the audience of the child and sometimes the contaminated adult.

Delusions are generally exhibitions of the child, which contaminate the adult.

Depersonalization is a manifestation of somatic stimuli, distorted by the confused child, which are incomprehensible to the adult.

If the stimuli become ego-syntonic to the adult, they are transformed into delusions of bodily change.

In hypomania the child excludes the parent with the cooperation of the contaminated adult.

If this changes to mania, the adult as well as the parent become overpowered by the "hypercathexed" child.

Conversion hysteria is an exhibition of the child that has excluded the adult through repression.

Character disorders and psychopathies are manifestations of the child living in agreement with the adult.

Impulse neuroses are eruptions of the child without the cooperation of the adult or parent.

The therapist also operates with maneuvers from his or her parent, adult, and child.

It is presumed that these will act in concert for the benefit of the patient.

Often the child in the therapist will be able to perceive aspects of the patient's child and parent intuitively and subconsciously.

The function of the therapist depends on the problem being considered; for instance, the therapist may, where repression dominates, have to break down the barrier to enable the child in the patient and therapist to talk together in the presence of the acting adult.

The internal operations of the patient are regarded as akin to those of a multiple personality, the patient eventually learning to view personal maneuvers through the lens of whichever of the three ego-states are active at the time.

The preliminary "structural analytic work" of the therapist (which incidentally is usually

accomplished in two or three individual sessions), in addition to making a diagnosis, consists of “decontamination,” “boundary work,” and “stabilization” to enable the adult to control the personality in the face of stress.

Thereafter “transactional analysis” is employed, often in a group setting, to help the adult maintain social control despite the activities of other people who seek to stimulate the patient’s child or parent.

Transactional analysis in a group is usually followed by “game analysis” and, ultimately, by “script analysis” in which complicated life plans exist.

In the group the transactions among the constituent members, consisting of a transactional stimulus and transactional response, are analyzed.

A stimulus from one adult may be made to another adult, but the response in the latter may be that of the child or parent (“crossed transaction”).

The stimulus may be from child to child or parent to parent.

As long as the vectors are not crossed, the conversation will proceed smoothly (“complementary transactions”).

Crossed transactions may be interrupted by the intervention of another member of the group or by the therapist, who attempts to bring the transactions back to the adult level.

In this way the group members learn to diagnose each other’s ego states as well as their own.

The engagements within the group (indeed in all social intercourse) are in the form of pastimes (when transactions are straightforward), games (in which dissimulation is introduced), and scripts (complex sets of transactions).

Pastimes are generally parental or adult.

For instance, a parental type of pastime is “PTA” in which projections (“Isn’t it awful?”) of delinquency by others are mouthed, such as delinquent juveniles, delinquent husbands, or delinquent wives; or they may be adult, in which introjections of “Me too!” are admitted (“Why can’t I be a good mother, father, hostess, whatever?”).

Other pastimes are “psychiatry” (“Here’s what you’re doing”) and its introjective parts (“Why do I do this?” and “Which part of me said that?”).

The “Small-Talk” pastime consists of bits like www.freepsychotherapybooks.org 506 “General Motors” (comparing cars), “Who Won” (man-talk), “Grocery,” and “Kitchen” (lady-talk).

Pastimes are indulged for a variety of reasons, for instance, as a way of diversion, getting satisfactions, warding off guilt, and easing quiet desperation.

In group therapy pastimes are considered a waste of efforts, although they are a preliminary means through which the group members size each other up before the “games” begin.

Transactions are of several types: complementary (well structured) and “ulterior.” Games are ulterior transactions, containing a concealed motivation (“gimmick”).

A common game played in a group is “Why don’t you ... yes, but.” (Parent: “Why don’t you go to school if you want to be better educated?” Child: “Yes, but then I won’t have time to bowl.”)

The “gimmick” in presenting an adultlike request for information or for solutions here is to gratify the child who presents himself or herself as inadequate to meet the situation.

Other games (which are infinite) are “Schlemiel,” “Alcoholic,” “Wooden Leg,” “Uproar,” “Ain’t it awful,” “If it weren’t for you,” “You got me into this,” “There I go again,” “Let’s put one over on Joey,” “Do me something,” “Gee, but you’re wonderful, professor,” “Harass,” “Rapo,” and “Now I’ve got the son-of-a-bitch.” Each game involves a goaldirected set of exploitative complementary transactions.

Patients may enter into various alternative roles in the games that they play.

Complex sets of games (scripts) are transference phenomena, manifestations of infantile reactions and experiences derived from the need to repeat in acts the transference drama.

For instance, women whose fathers were alcoholics often follow the script based on the “rescue

fantasy” of marrying one alcoholic after another.

Pastimes, games, and scripts are played out and analyzed as integral parts of the group process.

In summary, transactional analysis is a technique felt by Berne to be superior to current supportive and insight therapies, helping the patient rapidly to tolerate and control anxieties and to circumscribe acting-out, while retaining the values of rational therapy.

Moreover, he believes it to be applicable to problems that are difficult to reach by conventional therapies, for example, psychopathic personality, borderline cases, mental retardation, and manic-depressive disorders.

The technique is easy to grasp.

(Berne claims it may be learned in 10 weeks and perfected in a year of supervision.) In addition, it www.freepsychotherapybooks.org 507 is well accepted by many patients.

Its claim of universal applicability is, however, challengeable.

Several major schools of transactional analysis exist at present, most importantly (1) the Berne approach, (2) the Schiff “Reparenting” approach, (3) the Asklepion approach, and (4) the “Redecision” approach, the latter three introducing modifications in emphasis and techniques in the original Berne method (Goulding, 1976).

Experiential Therapy Experiential approaches downgrade experimental, technical, and intellectual methods as nonmeaningful in the treatment process.

What is considered important is bringing up and expressing feelings.

According to its exponents, only through this means can one actualize one’s potential and thereby cure emotional illness.

The traditional therapeutic doctor-patient alliance is rejected as the basis for personality change.

Instead of anonymity, manipulateness, or detached noninterference, the climate of the therapeutic relationship is one of empathic warmth, friendship, expressed emotion, and regard for the patient.

Neither the problems of the developmental past nor the promulgation of present insights are considered important.

The emotional encounter between client and therapist is the basis for significant movement, and defenses against proper relating are dealt with by confrontation and the assignment of tasks.

Experiencing the encounter as a different mode of relating eventually leads to self-actualization.

The role of the therapist, it is proclaimed, is to liberate the affects of the patient that have been frozen and cause paralysis.

This is best accomplished, not by interpretation, but by establishing a meaningful, deep, affective relationship through the therapist’s playing a role with the patient that is different from the conventional “You need me; I don’t need you” role.

Toward this goal the therapist does not set himself or herself up as a “healthy authority,” a paradigm of personality virtues who is falsely “objective” (usually a cover-up for the therapist’s neurotic omniscience and omnipotence), a pose that is rejecting of the patient and demeaning to himself or herself as an adult.

There is no reason, it is claimed, why therapy should not be a mutual growth process, the therapist exhibiting his or her www.freepsychotherapybooks.org 508 sickness and relating to what remains of the patient’s healthy soundness, as well as the reverse.

Such sincerity, it is presumed, can be remarkably ego-building for the patient.

Important exponents of this point of view are Whitaker and Malone (1963), who insist that the roots of psychotherapy are embedded in the matrix of the individual’s biological being.

Penetration to this dimension requires a unique management of the transactions between patient

and therapist in which heightened emotional tension is maintained.

Attitudinal and behavioral change are secured by a modification of the customary interpersonal relationship during the therapeutic hour.

This experience incorporates emotional penetrations into the therapist's own child-self, which, projected onto the patient, results in self-treatment of certain aspects of the therapist through the patient.

In the course of this complex interaction, the patient is said to liberate himself or herself from socially restricting reality-inhibiting roles.

Acting-out by both therapist and patient is accepted and even encouraged, releasing energies previously entrapped in emotional conflict.

An interlocking of the unconscious (id) aspects of the participants reflects itself in a joint fantasy experience characterized by mass body sensations and a primitive level of communication.

In depth, this encounter is believed to be of the profoundest therapeutic advantage.

The patient's acceptance of his or her unconscious fantasies and their unification with the conscious self is encouraged by the role of the therapist as a "good" parent who sees the patient as part of himself or herself.

The techniques employed by the therapist are not as important as the activities in facilitating symbolic unconscious meanings.

This is accomplished by the therapist's dissociating as many of the realities as possible from the immediate therapeutic experience.

At the start of treatment the therapist may respond to the patient with silence and nonverbal communication as a way of promoting transference and of understanding the symbolic meaning of the patient's communications and behavior.

The therapist encourages the patient's plunge into fantasy by pointing out to the patient the therapist's own limitations and immaturities.

Various forms of physical contact, including aggression, may be executed by the therapist. www.freepsychotherapybooks.org 509 experiential therapist.

Such objects as clay and rubber knives may be used, and sleeping during the session may take place, with a reporting of any dreams by therapist and patient as a way of promoting the mutual fantasy experience and "clearing things up." Out of the mutual encounter at the "gateways of unreality," therapist and patient are presumed to arrive at a better understanding of themselves and to achieve a heightened capacity to cope with reality.

Experiential therapy is often conducted in a group setting (see "Encounter and Marathon Group," Chapter 52) and may include many ordinary or unusual nonverbal interchanges.

In "direct psychoanalysis" J.

M.

Rosen (1947, 1962, 1964) has introduced a means of entering the private world of the psychotic and of interrupting the deeply neurotic or psychotic regression (pregenital and neoinfantile) in which the patient is seeking a hallucinatory or symbolic representation of the indispensable, exalted but deadly mother that he or she knew.

Instead of evading the parental role thrust on the therapist, as in conventional analysis, the "direct psychoanalyst" accepts it, functioning as "a loving, omnipotent protector and provider" for the patient.

With a psychotic person, the analyst acts like an idealized parent who has the responsibility of bringing up the child again.

With a neurotic person, the therapist guides him or her parentally to maturity.

Observing Rosen working with patients, a number of therapists have followed his methods and have attempted to detail in their writings the techniques of direct analysis (Brody, 1959;

Scheflen, 1961; English, et al, 1961).

While Rosen's technique in the hands of skilled, trained therapists establishes rapid communication with regressed patients, its advantage as a form of reconstructive therapy has been challenged.

For example, a controlled followup study at the Psychiatric Reception Center of the Philadelphia General Hospital showed that the percentage of improved patients treated by direct analysis was approximately the same as the control groups (Bookhammer, et al., 1966).

Criticism of Transactional Therapies The greater activity inherent in transactional methods, as in other www.freepsychotherapybooks.org 510 psychoanalytically oriented approaches, is believed by some observers to sponsor reparative and hence goal-limited tendencies that are more reeducative than reconstructive.

A more passive approach, on the other hand, is said to permit the patient more readily, from his or her own growing resources, to appreciate transference distortions and develop different ways of relating to others as a product of personal experience.

Censured, also, is the minimization of insight as a constructive therapeutic force.

Social insights acquired through manipulative procedures rarely effectuate permanent change.

The strongest criticism has been directed toward those transactional therapies that advocate a free, spontaneous interaction between therapist and patient, characterized by unrestrained physical and symbolic communication (aggression, physical contact, long periods of silence, joint fantasy), verbal and nonverbal.

The emerging irrational experience, which may reach a psychotic level, is presumed to get at the core of problems.

Since both the patient and therapist are encouraged to regress, there is said to issue out of this interaction mutual growth and maturity.

It is the "letting oneself go" and acting in an unrestrained way in the therapeutic relationship that have brought forth the greatest cries of disapprobation.

This is because some transactional therapists have interpreted freedom in the relationship as an invitation to play the kind of role with patients that they need transferentially to project onto them.

Thus, they assume the identity of the father, mother, sibling, God, devil, lover, persecutor, or whatever other figure is dredged up from their own or their patient's imagination.

The more inventive and histrionic-minded the therapist, the keener the fantasy life of the patient, the more the eventuating drama takes on the aspect of a folie-adeux.

While the patient may register benefits from such tactics, as one can from any other kind of relatedness, and though better rapport may be established, particularly with psychotic individuals (such as occurs in Rosen's direct psychoanalysis), one should not be deceived regarding the depth and permanence of changes effectuated by such play acting.

Freedom of participation without control can give the therapist with neurotic problems a license for personal emotional catharsis and acting-out.

This release has positive values for detached therapists who otherwise may find it difficult to relate and www.freepsychotherapybooks.org 511 who therefore respond more genuinely and spontaneously if they are able to express themselves verbally or to act with few or no restraints.

Some patients are willing to accept the unconventional behavior of therapists on the basis that they are now dealing with new, easier, more genuine kinds of authority who can admit to certain weaknesses: some patients tolerate a therapist's unconventional acts because they feel sorry for or protective toward the therapist.

They may even assume the role of "helper," considering the therapist as a patient, fulfilling in this way their own neurotic needs for control and dominant status.

As long as a working relationship exists, patients seem to be able to adjust to an astonishing variety of provocations.

In their eagerness to get well, they will make capital out of almost any kind of relationship. This criticism, of course, is not directed at all transactional therapists, but only at those whose personal problems are such that they interpret behaving naturally as meaning undisciplined expression of their feelings and impulses.

ANALYTIC GROUP THERAPY The spectacular growth of group psychotherapy throughout the world attests to its general acceptance as an effective treatment method.

Where its proposed objectives are toward reconstructive changes in the personality structure, methods are introduced that purportedly release growth potentials.

Among the earliest dynamic formulations was a paper by Lazell (1921), which described group work among schizophrenics in terms of psychoanalytic theory, and the studies by Burrow.

In the early 1930s Trigant Burrow (1926a, b, c, & d, 1927) organized a group of "participant observers" as part of a research project to investigate human basic motivations, particularly destructive and pathogenic impulses.

Observing his own and the reactions of his colleagues, Burrow (1924) evolved ideas about the "social neurosis" or "mass neurosis" (1926b), and he concluded that human beings were faced with phylic defect that fostered autistic self-reference (Burrow, 1933, Syz, 1957).

The study of this ubiquitous defect he called "phyloanalysis" (Burrow, 1930).

Since emotional prejudices prevented the individual from fully scrutinizing his or her own processes, Burrow www.freepsychotherapybooks.org 512 believed that these could best be approached through mutual interaction and selfdisclosure in a group ("consensual observation"). A group, he found, helped to orient and resolve "man's unacknowledged self-centered and socially disruptive trend" (Syz, 1963).

This was rooted in a preconscious primary identification with the mother, the basis for later stages in the child's development.

Obsessive libidinal strivings were the product of an intrusion on this primary identification by objectification and cognition.

Awareness of psychosomatic accompaniments of emotional blocking, in the form of oculofacial tensions, sometimes produced unexpected behavior changes, with a resolution of affect images (Burrow, 1941a & b).

"The drive for self-justification and social approval gave way to an inclusive feeling attitude, to enhanced communication and a more direct application to immediate tasks, which outlasted the consciously induced attentional shift" (Syz, 1963).

The focus on how attentional behavioral shifts could influence endo-organismic tension patterns differentiated Burrow's organismic from other group therapeutic approaches (Syz, 1936, 1957).

An integration of psychoanalytic theory into group process was to be expected as soon as psychoanalytically oriented psychotherapists became interested in groups.

Among the earliest group analysts was Paul Schilder (1936, 1939, 1940), who, in working with criminals, described a method that combined autobiographical material with free association and dream interpretation.

Wender (1940) employed a group analytic procedure in a hospital setting, and Sarlin and Berezin (1946) also reported an uncovering approach in group therapy.

Among the pioneer efforts to amalgamate psychoanalytic and sociological concepts were those of Moreno (1957), who very early in his career used the group method, and Wender (1940, 1963) and Slavson (1943), who outlined an analytic group process that they considered equivalent in every respect to psychoanalytically oriented psychotherapy.

Slavson brought out that transference in the group was multiple, being directed toward various persons at the same and at different times.

It could be positive or negative, but if progress was to occur, positive transference toward therapist and group members had to develop after analyzing and disposing of negative feelings. The group, essentially www.freepsychotherapybooks.org 513 patterned after the original family, fostered transference that was of parental, sibling, and “identification” types; in the latter case the therapist and certain group members served as identification models.

The effect of the group was to dilute the transference, although the total consequence to the individual was a heightening of emotional feeling.

Shortly before the end of World War II Alexander Wolf (1950), applying himself to the “psychoanalysis of groups,” endorsed the use of free association, dream analysis, exploration of transference and resistance, and the search for those early memories and experiences that were the forerunners of current characterological and symptomatic disturbances.

Countertransference was also to be considered.

Wolf advocated heterogeneous groups of approximately the same age with a balancing of males and females.

He believed this facilitated resolution of problems on the heterosexual gregarious plane.

Excluded were psychopaths, alcoholics, stutterers, mental defectives, hallucinating psychotics, and hypermanic patients.

The groups, which had eight to ten members, met two to three times weekly for 90 minutes.

So-called “alternate sessions” were also introduced by Wolf; i.e., scheduled meetings of a therapeutic group without the therapist present were alternated with regular sessions held with the therapist.

A number of individual interviews might be required before the patient was ready to enter the group.

This, the first stage of therapy, was diagnostic.

The patient was also prepared for group analysis by explanations regarding its theory and practice.

The second stage of therapy, conducted in the group setting, consisted of free associations organized around the dreams of the group members.

Once good rapport developed, the third stage of therapy began.

This consisted of each patient spontaneously free associating about the next, which resulted in a bombardment of the patient’s defenses and exposure of conflicts.

The patient learned which of the characteristics pleased the group and which disturbed them.

In the fourth stage of treatment, resistances mobilized by the free associations of the group members came up for analysis.

The fifth stage of treatment was concerned with analysis of transference—its identification and resolution.

Recreating his or her original family in the group, each patient learned how he or she projected parental and sibling images toward members and therapist.

Wolf claimed that www.freepsychotherapybooks.org 514 identification and resolution of transference proceeded more thoroughly and rapidly in a group setting.

The sixth and final stage of treatment was characterized by planned conscious responses in the interests of the group and the individual.

These replaced strivings of an irrational or compulsive nature engendered by the person’s habitual character structure.

More penetrating analytic explorations were possible in groups than in individual analysis because the “group ego,” with which each member identified, supported an impoverished ego and heightened tolerance of anxiety.

In his later contributions Wolf, in conjunction with E.

K.

Schwartz, expanded the advantages of group over individual analysis (Schwartz & Wolf, 1960; Wolf & Schwartz, 1962).

An important feature of group, they wrote, was the replacement of the omniscient ego ideal of the single therapist by the group ego ideal with which the patient could align.

In this way the group enabled the patient to face social reality and “to become aware that his fulfillment can only be realized in a social or interpersonal setting.” At the same time the authors depreciated the impact on the individual of group dynamics, considered by some other therapists to be a potent force in a therapeutic group.

They considered group analysis essentially the psychoanalysis of the individual within a group rather than the analysis of the group.

Other psychoanalysts also experimented with methods that drew on psychoanalytic theory.

Sutherland (1952), for example, stressed that psychoneurotics could be helped adequately in group therapy only if a change was brought about in them through the resolution of their unconscious conflicts.

He contended that group therapy could be conducted along strictly analytic lines in order to achieve this purpose.

Somewhat similar points of view were emphasized by S. H.

Foulkes (1948) and Ackerman (1950).

Ezriel (1950, 1952) listed three hypotheses related to a method of applying psychoanalytic theory to group therapy.

The first was that unconscious feelings toward past authoritative personages were projected onto figures in the social environment.

The group provided an opportunity for such multiple transferences.

The second was that a common group tension developed that involved all group members and elicited in each person unconscious fantasies that had a common denominator.

Each member assumed a role in a www.freepsychotherapybooks.org 515 “drama,” enacted in the session by the group.

This brought out the individual's particular defense mechanisms.

The third hypothesis dealt with interpretations that were made to the group members.

These were given in relation to the unconscious content of the area of common tension and to the specific defenses the patient employed.

Spotnitz (1952b) discussed the enhancing of ego functioning of the individual in the group setting.

Where selection of group members included a scattering of problems and personality types, an opportunity was afforded patients to experience and to observe a variety of reactions that helped them scrutinize their own reactions more objectively.

Individual resistances were handled rapidly by the group, and this resulted in symptomatic improvement.

Spotnitz believed that the instinctual forces at work in group could be understood in terms of the life and death instincts as outlined by Freud.

In England, as in the United States, fruitful investigations of analytic group therapy have been ongoing.

Bion and Rickman, around 1943, instituted group discussions among army personnel in an atmosphere stripped of the traditional authoritarianism.

Bion's work at the Northfield Military Hospital was particularly outstanding and was adopted as a model for psychoanalytic group practice in various parts of England.

All groups, Bion (1948—1951, 1961) claimed, operated on the basis of certain cultural assumptions that gave rise to heightened emotional states.

Contributions to the group were in accordance with whether individuals resisted or accepted the existing cultural standards.

Valences of pairing, dependency, or “fight-flight” developed with shifts in the emotional culture of the group.

Operating in a relatively nondirective way and purposefully avoiding structure, Bion utilized the initial resentment or confusion of the group members as material for analytic exploration. Throughout the group process interpretation was focused on the total group rather than on individual behavior.

Expounding on the importance of group factors as distinctive from individual phenomena. Foulkes (1948; Foulkes & Anthony, 1957) considered the group matrix the essential element. The group analyst, he insisted, had to deal, as a participant observer, with all communicative and relationship processes as aspects of the total interactional field.

Substantive regressive, infantile manifestations were, according to Foulkes, www.freepsychotherapybooks.org 516 minimized by the presence of the group.

Maintaining some Freudian concepts, Foulkes felt his approach drew largely from Gestalt and field theory, particularly from the ideas of Kurt Lewin (1945).

Analytic group therapy, he insisted, must concern itself with the “deep aspects of social interaction,” which is “preeminent in the uncovering of the deep unconscious group phenomena.” Though transference occurred in a group, it was not of paramount importance for the therapeutic trend of the group.

“The group is not the most suitable place to analyze a transference neurosis... .

Psychoanalysis provides the ideal solution for this....

It is not that group-analysis does less; it does something different....

It is consequently unprofitable to confuse the scope and field of the two therapies.” Due to the present-day rapid growth of group psychotherapy, methodologies, as M.

Rosenbaum (1965) has pointed out, have moved far ahead of theoretical explications.

Conceptual clarity of the process operative in groups is particularly lacking.

In this void it was to be expected that each school of analysis would project postulates regarding group phenomena, attempting to delineate in its own terms what happens to people in groups and how they best may be helped to achieve personality growth.

Thus, Freudian analysts conceive of the group as a family in which the individual acts out repetitively and compulsively infantile neurotic tendencies.

Catharsis and regression are accordingly considered to be significant therapeutic agencies, although Scheidlinger (1968) has emphasized that therapeutic usefulness rests in how well ego functions can overcome resistance to face ward-off infantile drives and accept and master them.

Psychoanalysts with an inclination toward “ego analysis” regard the group as a medium that supports mutual exploration of defensive characterological factors and healthy adaptive functions.

Horney analysts believe that a group helps to modify exaggerated self-idealization and other distorted concepts of the self and world.

In an atmosphere of mutual cooperation a healthy feeling of belongingness may be restored.

Sullivanian analysts look upon the group as a laboratory in which neurotic interpersonal patterns may be detected and constructive models elaborated.

Superimposed on adumbrations of unconscious mechanisms are group dynamics, held by some (such as Bach, 1954; Thelen, 1954; Thelen, et al, 1954: Whitaker & www.freepsychotherapybooks.org 517 Lieberman, 1964) to be, not an accessory, but a primary factor responsible for change.

Postulates from field theory and role theory are borrowed liberally by analysts who subscribe to group rather than patient-oriented methods (Bales, 1950, 1958; Bavelas, 1952; Cameron, N.

& Margaret, 1951; Cartwright, 1950, Deutsch, M, 1949a, 1951; Festinger, 1942, 1947; French, J, 1941, 1944; Gough, 1948; Homans, 1950; Lewin, K, 1947, 1948, 1951; Lippitt, 1948, 1952; Mead, G, 1934; Parsons & Schils, 1951).

These concepts, some of which were discussed in the last chapter in the exposition on reeducational group therapy, are combined with psychoanalytic formulations and make for a confounding diffuseness in ideas.

Accordingly, group dynamically oriented analysts have been subject to virulent attack. Indeed, the most vocal antagonists consider group processes to be obstructive rather than helpful during analytic group treatment.

In another direction are group analytic activities that are wedded to existential concepts, exploiting the “feeling” and “experiential” components of the group process (Mullan & Sangiuliano, 1958; Hora, 1959a & b; De Rosis, 1964).

A total involvement by the therapist in the group is a primary aim with communication of all of his or her authentically perceived experiences and reflections.

Innovations introduced into the field of analytic group therapy include the use of multiple therapists (Spitz & Kopp, 1957; Rosenbaum, M.

1963), of family members in groups (Ackerman, 1958b; Grotjahn, 1959; Bell, J.

1962), of marital couples (Whitaker, C.

1958; Perelman, 1960; Leichter, 1962; Grinker, 1966; Sager, 1966a) and of videotaping (Berger, 1970, 1971; Alger, 1972; Melnick & Tims, 1974).

The literature in group therapy reflects a paucity of substantial contributions in research and follow-up and a diffuseness of regard for the boundaries of group psychotherapy per se in relation to other types of group experience, such as group counseling, small-group interactions, family therapy, marital therapy, and the therapeutic community.

There is particularly a tendency to amalgamate reeducational with analytic groups.

This is understandable in that group dynamics operate in all group interactions; however, it tends to confuse issues relating to group theories and techniques.

Excellent www.freepsychotherapybooks.org 518 reviews of the yearly literature are found in annual issues of the International Journal of Group Psychotherapy, the official publication of the American Group Psychotherapy Association, under the title “The Group Psychotherapy Literature.” Overviews of the literature in group psychotherapy for a ten-year period are included in the books Group Therapy (Wolberg and Schwartz, 1973-1975; Wolberg and Aronson 1974-1979, 1980— 1983).

An excellent historical account may be found in the paper by Durkin (1974).

The books by Yalom (1975, 1980, 1983), Sager and Kaplan (1972), Grotjahn and Freedman (1983), and Kaplan and Sadock (1983) continue to enjoy deserved popularity.

Among the important past articles and publications are the reviews written by Taylor (1958), M. Rosenbaum (1965), and Durkin (1974).

A coordinated bibliography has been compiled by R.

Thorne (1966) and Strachstein (1965).

As may be expected, the writings reflect many cause controversial issues, not only in relation to theory, but also in regard to the organization and direction of the group, including the participation and role of the therapist, the therapist's relative activity or passivity, the value of alternate sessions, presessions and post-sessions, the management of acting-out, and the handling of resistance and transference.

The most important older general readings include the following: Ackerman (1957), Bach (1954), Berne (1961, 1963), T.

Bieber (1957, 1959), Bion (1961), Brock-bank (1966), Cartwright & Lippitt (1951), Cartwright and Zander (1960), Corsini (1957), Durkin (1954, 1957, 1964), Foulkes (1948, 1957, 1964),

Foulkes and Anthony (1957), Glatzer (1964), Hinckley (1951), Hobbs and Rogers (1951), Hulse (1960), J.

Johnson (1963), Kadis (1963), Klapman (1946), Locke (1961), Moreno (1957), Mullan (1962), Powdermaker (1951, 1953), Rosenbaum and Berger, (1963), Scheidlinger (1952, 1960), E. Schwartz and A.

Wolf (1957, 1960, 1961), Slavson (1943, 1947, 1956a & b, 1960, 1964), Spotnitz (1952a & b, 1961), Wender (1963), D.

Whitaker and Lieberman (1964) and A.

Wolf and E.

Schwartz (1962).

Recommended literature on topical problems subsume the following references: 1.

Special technical management—M.

Berger (1958), Hulse (1950) Markowitz and Kadis (1964), Munzer (1964), Scheflen (1964), Winick and Holt (1961) www.freepsychotherapybooks.org 519 2.

The alternate meeting—Kadis (1956, 1958, 1960), A.

Wolf and E.

Schwartz (1959, 1960) 3.

Selection of patients for groups —M.

Freedman and Sweet (1954), Furst (1950), Glatzer (1956), Leopold (1957), Slavson (1955), Stein (1963) 4.

Combined and joint therapy—Aronson (1964), Lipschutz (1957), H.

Papanek (1956), Sager (1959) 5.

The use of co-therapists—A.

Gans (1962), L.

Lundin and Aranov (1952), Solomon et al.

(1953) 6.

Transference—Bach (1957), Belinkoff (1964), Beukenkamp (1956), Chance (1952), Demarest and Teicher (1954), Ezriel (1959), Farrell (1962), S.

Foulkes (1961), Fried (1965), Glatzer (1952, 1965), Grotjahn (1953), Hadden (1953), Horowitz (1964), Loeser and Bry (1953), Mullan (1955), Slavson (1950), Spangaard (1959), A.

Wolf (1950) 7.

Countertransference—R.

Cohen (1961), Flescher (1953), G.

Frank (1953), Grotjahn (1950, 1953), Hadden (1953), Hora (1956), Mullan (1953b), L.

Rosenthal (1953), Schindler (1953), E.

Schwartz and A.

Wolf (1964), Shaton et al.

(1962), Slavson (1953), Stroh (1958) 8.

Resistance—Arsenian et al.

(1962), Belinkoff et al.

(1962), Bry (1951), Durkin (1951), Flescher (1957), Gadpaille (1959), Geller (1962), Glatzer (1953, 1959a & b, 1962), Goodman and Marks (1963), M.

Goodman et al.

(1964), Greenbaum (1957), Heckel et al.

(1962), G.

Hill and Armitage (1954), Hulse (1950), J.

Jackson and Grotjahn (1958), Kotkov (1957, 1958), Krause (1961), E.

McDonald (1951), R.

Menninger (1949), Morse et al.

(1955), Mullan (1953a), Ormont (1962, 1964), Redl (1948), J. Rosen and Chasen (1949), L. Rosenthal (1963), Spotnitz (1952b, 1958), Spotnitz and Gabriel (1950), A. Stein (1952), Stock (1962), J. White (1964), Wolman (1964) 9.

Acting-out—Abt (1965), Aronson (1964), Bry (1953), Burke and Lee (1964), Drukin (1955), Durkin et al. (1948), Eaton (1962), Slavson (1956), Vass (1965), A. Wolf et al. (1954), Ziferstein and Grotjahn (1957) 10.

Dreams, fantasies, and early recollections—Chalfen (1964), Kadis (1957), KleinLipshutz (1953), Locke (1957) 11.

Special clinical issues—Abrahams (1953), S. Brody (1961), Durkin (1954), www.freepsychotherapybooks.org 520 Ginott (1961), Godenne (1964), Grotjahn (1959), J. Krasner (1959), L. Kubie (1958a & b), S. Kubie and Landau (1953), L. Rosenthal and Nagelberg (1956), Slavson (1945, 1950), C. Whitaker (1958), A.

Wolberg (1960), Wolman (1964) Criticism of Analytic Group Therapy There are those who insist that individual therapy is doomed.

Some analytic group therapists have become so completely involved with their specialty that they forecast a bleak future for the dyadic model.

Because no person is an “island” and each individual is propelled by a feedback of stimuli from others, some predict that psychotherapy of the future will be experiential and concern itself exclusively with the “here and now,” with clusters and not single entities, with the “what” and “how” and not the “why.” History taking and ransacking childhood memories in search of “significant traumas” will be relegated to “the archives of psychological history” (Kempler, 1969).

This dire augury probably reflects the sentiment of therapists whose working styles are more attuned to groups than to individuals, and in all likelihood it refers to those patients whose learning patterns and cognitive modes synchronize with group interactions.

There are patients, however, who are inflexibly unmotivated to resolve their problems in groups, and there are some whose defenses operate against benefits accruing from therapeutic group experiences.

Such persons do very much better in individual therapy.

Moreover, the verdict of many therapists who do both group and individual therapy points to the fact that if the patient has a desire and capacity for reconstructive change, there is no substitute for intensive individual treatment.

Intrapsychic alterations, reflected in depth transformations of the character structure, may occur in patients in group therapy who possess a readiness for change.

In my opinion, however, there is a greater likelihood that this will happen with properly administered individual therapy or with combined therapy.

Individual therapy is here to stay despite the adaptations that undoubtedly will need to take place with massive programs for the delivery of mental health services.

www.freepsychotherapybooks.org 521 Criticisms of analytic group therapy are not so much levied at the concept of working with a group but rather at the theories of the different group therapists and the global claims that they make for their methods.

The controversy in group therapy between adherents of the regressive-re-constructive and the experiential-affect approaches (which interpret analytic group therapy as a blend of psychoanalytic and group dynamic processes) is as heated as the Freudian-neo-Freudian polemic in individual psychoanalysis.

Controversial also is the tendency on the part of some group analysts to consider the analytic group by itself an adequate substitute for individual analysis.

Indeed, in the opinion of a dedicated few, group analysis is psychoanalysis.

This idea has been disputed by other group analysts who differentiate analytic group process, irrespective of theoretical direction, from psychoanalysis; rather it is classified as a form of psychoanalytically oriented psychotherapy.

The group process, nonetheless, stimulates powerful projective and identification mechanisms in both patients and therapists that operate in the interests of cure as well as of resistance.

In a group setting, say the critics, it is more difficult to detect the resistance manifestations, since patients are capable of concealing themselves better behind vocalizations of more active members.

It is particularly inconvenient to unmask transference as it assumes the regressive form of claiming the right to a symbiotic tie with the mother.

In groups transference is often displaced from the therapist to the group members, undergoing such sharp splitting, "dilution," and distortion that it loses its therapeutic potential.

Defenses against transference are more easily elaborated in the group than in individual analysis.

The resolution of the regressive transference resistance, one of the most important elements in therapeutic change, may, as a consequence, be completely blocked.

This failure is abetted by the propensity of the group to sponsor the acting out of members with each other, particularly in alternate or post-therapy meetings, where pairings drain off transference tensions and leave the neurosis exactly where it was.

The therapist is put in an anomalous role here since to forbid interaction outside of the formal group places the therapist unhappily in the position of a punitive and nonneutral authority; to sanction it invites further acting-out.

Moreover, the nature of the group situation makes it arduous for the therapist to identify with each patient and then www.freepsychotherapybooks.org 522 to objectify the identification in the form of appropriate interpretations.

Some therapists try to overcome this by seeing patients in combined individual and group therapy or by avoiding placing patients into groups until they are well into the middle phases of therapy.

Reconstructive change presupposes intrapsychic restructuring.

This is often impeded in the group, it is claimed, by alliances that members make with each other, by encouragement of hostile verbal exchanges, and by interpersonal interlacings through which neurotic regressive gratifications are derived at the expense of mature growth.

Analysis of interpersonal operations, continue the critics, is important but often done on the sacrificial altar of true intrapsychic exploration.

The engaging of members in verbal combat as an outlet for "sibling-rivalry" hostility serves too readily as defensive resistance.

Complicating matters also are the reassuring, advice-giving directives that the reincarnated "family-members" extend to those undergoing stress who are seemingly in need of support. Such tactics, critics allege, water down the reconstructive potentials of treatment.

Analytic group therapy also poses a strain on the therapist, since the multiple impact of the group members will exaggerate the therapist's countertransference and interfere with objective interpretations.

A great deal of criticism is extended toward experiential therapists who abdicate their role as therapists and become themselves patients in the group.

This is said to play into their patients' defensive needs for omnipotence and control.

These criticisms, while authentic, do not really negate the value of analytic group therapy.

They merely accent the need for great experience and maturity on the part of the analytic group therapist.

If therapists are able to manage the resistances of their patients, they will have in analytic group therapy a most important, even indispensable, therapeutic instrument.

Most therapists endorse a combination of group and individual therapy (combined therapy) during which the benefits of both forms of treatment are used to advantage (Caligor, Fieldsteel & Brok, 1984).

www.freepsychotherapybooks.org 523 12 Miscellaneous Therapies Throughout the world a brotherhood of distraught people in search of peace of mind constantly exploit miscellaneous therapies held out to them as recipes for mental health.

Feeding this ever-increasing quest is a supermarket of psychotherapeutic methods that are designed to suit the most diverse tastes.

Some of these are carefully thought-out interventions, drawing their substance from established supportive, reeducative, and reconstructive therapies, put together into creative constellations.

Others exploit unconventional and dubious methods, sustaining these with fanciful theories, exploiting an uncritical eclecticism on the basis that "anything that works is acceptable" (Grinker, 1964).

Common sense here is usually buried under a landslide of optimism and is palpable testimony to humankind's need to yield its emotional miseries at any cost.

At the very time that Cassandras are bemoaning the death of psychoanalysis and other traditional therapies, these "new" treatments dip into the pool of mortal credulity to exploit anomalous "cures" for all imaginable ailments.

It is almost impossible to chronicle the myriad systems of helping that have appeared on the psychotherapeutic horizon.

Many of these are revivals of old and discarded methods; others are more novel and innovative. Still others are products of the counterculture, which spawns a variety of egregious approaches reflecting the anti-intellectualism of the day.

All inspire expectations of success, especially when promoted by charismatic leaders who seek to etch a lasting profile on posterity.

Most of these approaches, following a spectacular rise in popularity, fizzle out like a Fourth of July pinwheel.

Among the surviving systems are some that, blending with the personalities and working styles of certain therapists, actually enhance therapeutic outcomes.

They then acquire conventional respectability and become part of an eclectic schema.

Innovative therapies come about largely as a revolt against the semantics and rigidities of the traditional therapies.

Some of our present-day theories and methods are coached in language that is confusing not only to patients but also to professionals trying www.freepsychotherapybooks.org 524 to make sense out of them.

When we examine critically what the "new" therapies introduce, however, we usually find nothing essentially original.

Rather they take a small section of the familiar ideas long in use and merely explicate this area in terms that may be more comprehensible and hence acceptable to patients.

The great dedication and enthusiasm of the innovators come through to enhance the placebo

effect and to enliven other non-specific elements.

Because therapists practicing innovative therapies are so intensely convinced of the validity of what they do, they will score greater triumphs than if they were to use traditional methods about which they feel lukewarm.

The results of innovative therapies show that about two-thirds of the patients "improve," irrespective of the models used.

This compares favorably with the traditional therapies, which to some observers implies that the theories and even the methods we employ do not account for success in psychotherapy. Rather, the constructive use the patient makes out of the relationship with the therapist toward acquisition of a sense of mastery and more adaptive modes of coping is believed to count most.

All therapies are not the same, however, because there are differences in the dimensions and permanence of improvement with different treatment methods.

We cannot expect that short-term treatment with supportive educational interventions will influence the personality structure in as great depth as will properly conducted techniques aimed at the intrapsychic structure and dynamic interpersonal operations.

But irrespective of the kinds of therapy done, their alignment with the personality and style of the therapist and their coordination with the essential learning capacities of the patient are fundamental ingredients of success (see also Abroms, 1969; Corini, 1981; Dolliver, 1981; Halleck, 1974; Marmor, 1974, 1980.) Because the faith of the patient in the therapist's techniques is one of the most effective elements of cure, the virtue of the interventions themselves remains open to question.

This, however, should not discourage experimentation with a variety of methods.

There is no reason why a therapist should not cautiously test those techniques that possess a reasonable scientific propriety to see whether they blend with one's personality and style of operation, provided that the innovations introduced preserve www.freepsychotherapybooks.org 525 accepted bounds of patient-therapist decorum.

It is only too common to observe therapists so rigidly anchored to standard systems that they stubbornly resist learning innovations even though they have proven merit (Martin & Lief, 1973).

It is beyond the scope of this book to describe all or most of the miscellaneous treatment methods in use today.

Corsini (1981) has listed and Herink (1980) has described more than 250 different kinds of psychotherapy.

New techniques are evolved yearly.

Some come into the therapeutic arena like lions only to depart like lambs when their placebo effect has been expended.

Others persevere with ebbs and spurts of popularity.

Examples of surviving representative therapies are outlined below.

EMOTIVE RELEASE (BODY THERAPIES) The paradigm for emotive release was the "cathartic method" developed by Breuer and Freud in the late 1800s, which served as the precursor of psychoanalysis.

The theory behind the method was that neurosis developed as a product of psychical traumas that evoked fright, anxiety, shame, or physical pain if there had not been an adequate discharge of affect at the time.

Emotion then was so bound up that it could only be discharged subversively in the form of symptoms.

It was posited, however, that a cure might be achieved by reviving the memory of the event and putting the buried emotion into words.

The task of the therapist was to induce the patient to give utterance to the traumatic event "with an expression of affect." This could be done by eliminating the defense or resistance to permit the proper release or abreaction.

Neurosis was thus essentially a "splitting of consciousness" between memory and affect, the cure lying in the healing of this split.

These ideas still form a basic part of the philosophy behind later abreactive, emotionally oriented release methods, including those of scream therapy, active analysis, narcotherapy, primal therapy, expressive hypnotherapy, bioenergetic analysis, orgone therapy, direct analysis (Rosen J, 1947, 1962), and scientology among others.

Various rituals are organized around each of these therapies focused on breaking down defenses to the presumably barricaded early psychic traumas.

After a period of treatment with the release of a good deal of emotion and the lessening of tension we would expect an impact www.freepsychotherapybooks.org 526 on the physiological state of the individual, such as the pulse, temperature, and blood pressure.

This effect can be significant (Karle et al, 1973), and indicates a reduction of tension as registered on body organs.

In some patients, too, there is at least temporary improvement in their psychological state as a consequence of the total therapeutic experience.

Whether this improvement is any greater than in other kinds of treatment is open to question.

There are some patients whose learning patterns are singularly susceptible to release techniques, particularly when rendered by an enthusiastic and charismatic therapist.

Other patients fail to respond to these techniques and do better with nonrelease modalities to which they are more attuned.

Bioenergetics Bioenergetic therapy is a modification of orthodox Reichian therapy.

Reich (1942, 1949) contended that interruption of the life-energy flow through repression was aided by seven rings of "muscle armoring" and that this was the basis of neurosis.

By manipulating these rings progressively from head to pelvis (orgone therapy) conscious awareness of painful vegetative sensations occurred, the energy flow was encouraged, and health brought about.

The Lowenian emphasis (Lowen, 1958) in bioenergetics is also on muscular energy blockages, but rather than the therapist manipulating the muscles, as in orgone therapy, reliance is placed on the patient's own muscle activity exercises and verbalizations.

The exercises open up an awareness in the patient of malfunctioning of breathing and other muscular operations that are presumably crippled by blockage of the energy flow.

The loosening of the muscle armoring is believed to be the first step in personality reorganization.

The actual technique consists, at the start, of assumption of a painful "stress position," such as lying on the floor with the back arched so that support is maintained by the top of the head, the elbows, and the soles of the feet.

Other exercises consist of kicking or hitting the couch.

The "stress positions" assumed can actually cause the body to vibrate, and the discomfort may lead to yelling or screaming, which the therapist encourages.

A number of adaptations of Lowen's technique have appeared, especially in the www.freepsychotherapybooks.org 527 behavior therapy field.

One innovation is to fuse bioenergetics with assertive training.

An effort is made to elicit in various ways, as through facial, postural, verbal, and other expressive channels, patterns associated with emotion.

Assertive behavior, along with expression of appropriate feelings, is in this way gradually shaped.

The therapist here participates in the exercises given the patient by modeling an appropriate behavior and by role playing.

Emotional release is stimulated by such games as rolling up a towel, with the therapist grasping one end and the patient the other.

A tug of war then ensues, the patient being told the towel is his and that he must wrest it from the therapist, demanding "It is mine, give it to me," and maintaining eye contact while the therapist resists and goads the patient on (Palmer, RD, 1971).

Other "exercises" given the patient are punching a pillow violently in private, yelling such phrases as "you bitch," "Damn you," and the like.

The therapist handles the patient's reactions in the office, reassuring the patient, if necessary, not to be afraid, or forcibly insisting on the patient yelling louder.

Some therapists encourage the use of obscenities to cut into the patient's repressions.

Some provide the patient with a tennis racket with which to hit the couch, while uttering such statements as "Bastard, I hate you." Some encourage the patient to growl through bared teeth with jaw thrust forward, shaking fists at the therapist while maintaining eye contact.

Some ask the patient to repeat angrily, "Yes, I am mean and ugly and nasty, and I will become more so if you don't stop bothering me." Bioenergetic assertive techniques have also been used with proper organization to release affectionate and sexual feelings.

Scream Therapies Organized to thaw out and to liberate fundamental "survival-based" feelings frozen within the individual by society, scream therapies prescribe a special regime for the patient and subject him or her to maneuvers to melt resistance to "letting go." A typical example are the exercises devised by Daniel Casriel (1972).

In a group setting the emphasis is first placed on screaming to express feelings rather than to talk about them.

Casriel differentiates several types of screaming, each with different sounds and expressing different emotions.

"There are basic screams of fear, of pain, of need and entitlement and four screams of anger—the deepest one accompanied by strong feelings www.freepsychotherapybooks.org 528 of pleasure." In addition, there are joyous sounds of pleasure.

Confrontation, encounter methods, and marathons are utilized to loosen feelings as these come up.

The patient is encouraged to scream at the top of his or her lungs.

If the patient gets too disturbed, he or she is held or hugged for reassurance and comfort.

Once the sounds of repudiated emotions are released and accepted, the patient is encouraged to explore the reasons for them.

Role playing may be utilized here to open up pockets of understanding and to reprogram behavioral habits.

Progress with feeling emotion and dealing with its source, outside of therapy, is reported to the group at later sessions.

The use is made of floor mats for patients to express their "historical feelings" and then to connect these with presentday problems.

Character disorders as well as neuroses are said to respond readily to these methods.

Some scream therapy interventions are rationalized by the theory that the release of rage is a precursor to eliminating resistance to emotional growth and development.

In one technique to induce rage, the patient is placed on the laps of six or eight people who physically restrain him or her while the therapist, on whose lap the patient's head rests, fires pointed questions at the patient about his or her past.

Simultaneously or somewhat later the therapist tickles or prods the rib cage of the patient to induce discomfort or pain.

This usually releases cries of helpless protest and violent rage.

The "treatment" goes on for as long as 4-6 hours until the patient is literally exhausted.

The patient is then released and embraced affectionately by members of the group and the therapist.

EST Contracted from "Erhard Seminars Training," a brainchild of Werner Erhard, EST was founded in 1971 and essentially is organized around a two-weekend experience designed to transform the participants' ability to "clear up" disabling life situations.

Many thousands of people have graduated from EST training courses, which are offered throughout the country.

Sessions are conducted in a rigidly disciplinary way to stir up tension.

Discomfort and deprivations are deliberately organized, along with a grueling confrontation.

Honesty of revelation is stressed, and the emphasis is on assuming

www.freepsychotherapybooks.org 529 responsibility for one's own actions.

"Much of the effectiveness of training is based on surprise, or shock... .

You are simply caught off balance... .

Our bodies scream for relief" (Kettle, 1976).

The leader "harangues us, urges, cajoles, wheedles, roars ... one thing that happens in the est training is the attack on the armor with a consequent release of energy." The participants cry, laugh, scream, vomit.

Testimonials are ample from participants as to how much they have been helped by EST.

The marketing techniques used to publicize the program have made it extremely popular.

There is evidence that some persons with good ego strength and strong motivation derive benefits from participation (Simon, 1978), as they might with other experiential groups.

On the other hand, psychoses may be precipitated in vulnerable persons as a consequence of the intensive experience (Glass et al, 1977; Kirsch & Glass, 1977).

Primal Therapy The best known of the scream therapies is primal therapy which had a brief period of popularity.

According to Janov (1970), people are born with needs, a part of their "real selves." When these needs are unmet, the neurotic process begins.

When the pain of unfulfilled needs is not alleviated, the infant diverts the pain by shutting off the need that is unsatisfied; the child then separates himself or herself from the needs and feelings.

Substitute gratifications ensue, usually symbolically, and become the modes of functioning of the "unreal self." Sensations caused by unattended needs are relocated to areas where greater control or relief can be provided.

For instance, early weaning can find a symbolic satisfaction later in incessant smoking.

"Neurosis is a symbolic behavior in defense against excessive biological pain." But symbolic satisfactions never really gratify the needs from which they issue, which unfortunately are not available to the person because they have been buried.

Tension issues from unsatisfied needs.

A constant denial of real needs ("minor Primal Scenes") sensitizes the individual to a precipitating traumatic event (the "major Primal Scene"), usually between the ages of 5 and 7, that produces in the child the realization "There is no hope of being loved for what I am." The child then shields himself from this realization "by www.freepsychotherapybooks.org 530 becoming split from his feelings, and slips quietly into neurosis." Neurotic behavior becomes automatic.

"Primal Pains" are so intolerable that they, and the memories with which they are associated, become disconnected from consciousness.

This resulting tension promotes a life style or "personality" to minimize and dull suffering.

Primal therapy, continues Janov, constitutes a systematic assault on the unreal self “which eventually produces a new quality of being—normality—just as the original assaults on the real self produced a new state of being—neurosis.

Pain is both the way in and the way out.” There can be no resolution until the “Primal Pains” are reexperienced consciously.

This breaks open the memory bank associated with the “Primal Pains.” As with other therapeutic systems, splits have occurred among therapists who practice primal therapy on the basis of dissatisfaction with results.

Criticism of Emotive Release The concentration on causation in the early years to the exclusion of later etiological factors is a defect from which many therapies, including emotive-release therapies, are blighted.

We may rightfully be suspicious of any theory that proposes a unitary cause for all emotional problems.

Human behavior is extremely reticular, and unfortunate conditionings may occur anytime between birth and the grave.

The traumas of middle age can be no less endurable than those of early childhood.

An adult victimized by disastrous contemporary circumstances that are beyond the individual's existing coping capacities may show catastrophic responses that may permanently injure faith in oneself and open the door to emotional illness that plagues the individual for the remainder of life.

This does not minimize the cathartic effect of emotive release.

The ability to let go without restraint discharges a good deal of pent-up emotion, for example, fear, indignation, and rage.

Relief of tension may then ensue.

The ability to express uncurbed emotion without encountering anticipated punitive retaliation or rejection—indeed being encouraged to emote freely in the face of a new, accepting authority—can have an important impact on the superego.

The lessening of controls of the conscience and the www.freepsychotherapybooks.org 531 freedom in unleashing inner feeling have a strengthening effect on the ego.

With the discharge of affect a flood of insights may then be liberated.

Surfacing of repressed memories and of repudiated aspects of the self may occur where defenses are broken down catastrophically in the therapeutic process so that the individual has no way of dealing with the stress situation except through regressive parental invocation in screaming.

Where dependency needs are extraordinarily pronounced and where one does not encounter a parental agency who comes to one's aid by one's screaming, the individual may in some instances have to develop a more mature way of dealing with needs, such as by greater self-reliance and assertiveness.

In other cases the screaming furthers rather than lessens the regressive tendency, and, from a long-term standpoint, this does not help the patient's condition.

In borderline cases or schizophrenics, damage may be done to the defensive system so that a break with reality may occur.

The more drastic emotive-release therapies are tailor-made for patients who have a stake in feeling pain as a condition for well-being.

If they can accept the premise that suffering of pain is mandatory to overcome problems and that feeling pain fulfills certain needs, such as the propitiation of guilt, the therapy may constitute a tension-releasing experience for them.

The charisma of the therapist acts as a catalyst for these patients, but the basic need to accept pain is a primary requirement.

Obviously many patients react adversely to paying penance by suffering and will not abide by this premise.

Indeed, feeling pain for them will act as a deterrent to getting well.

One may call this, if one wishes, a resistance to getting well.

In the field of psychotherapy we too often credit an individual's inability to respond to therapeutic stratagems as resistance rather than acknowledge shortcomings in the system itself.

Despite defects in the system, for some individuals, such as severe character disorders, or those unmotivated for traditional therapies, assuming they are not borderline cases or schizophrenics, the dramatic treatment techniques such as emotive release may operate as means of penetrating otherwise unyielding defenses that isolate individuals from themselves or others.

www.freepsychotherapybooks.org 532 GUIDED IMAGERY About 1895, Sigmund Freud utilized the "concentration technique." He enjoined his patients to shut their eyes.

While he pressed on their foreheads, he instructed his patients to let images come to their minds and, without holding anything back, to verbalize what they imagined.

Because Freud felt that touching his patients interfered with the transference, he abandoned this technique in favor of free association and the use of dreams.

Since Freud's early use of fantasies, methods of image associations have been periodically introduced that the authors presented as original and unique, even though on scrutiny they were quite similar to Freud's old technique.

Some therapists find that imagery plays an important role in their treatment of a number of disorders (Horowitz 1970, 1976; Brett, 1985), and may serve as a vehicle to expedite treatment.

Psychoimagination Therapy Integrating phenomenology and imagination, Shorr (1972) has introduced what he describes as an existentially based therapy (psychoimagination therapy) organized around how patients view themselves from the inside and how they believe other people view them.

Imagery and imagination are used as a means of examining how patients regard themselves and how they see their phenomenological world.

Shorr discovered that when a person is asked to say how he or she imagines a thing in two different ways and there is a conflict between the two, a "bipolarization" in the meaning systems will be exposed.

A new more unified meaning system must then be organized.

The search for meaning is one of the most powerful strivings in the human being.

To survive in a hostile world as a child, the meaning of events may be perverted and this can distort reality.

If true facts about oneself and about past personal experiences can be laid bare, new and more adaptive meanings are made available to the patient.

The problem lies in how best to achieve true meanings.

Shorr believes this can be done through imagery and imagination, which can be used to reach out into the world and differentiate fantasy about oneself from reality.

In this way, one can alter one's sense of values.

The therapist tries to get from patients their definitions of themselves, how they think other people www.freepsychotherapybooks.org 533 define them, and confirmation of these ideas from those they are with.

A disparity in definitions about oneself from these various sources makes for conflict.

The job of therapy, Shorr alleges, is to change a distorted negative self-definition to a more positive one.

Thus, one may ask a patient to look into a mirror and to imagine looking at a different image than one's usual self.

Often the person fantasized is one toward whom the patient has a good deal of emotion. A dialogue may be set up between the person and the fantasized object in the mirror. Conflicts are easily elicited in this way.

By coming to an awareness of the conflicts within oneself, the individual is more capable of resolving confusion in personal meaning systems.

The patient may be assigned to do "task imagery." For instance, the patient may be asked to climb 1000 steps in his or her imagination.

Some will hesitate after a few steps; some will slowly ascent to the top but refuse to traverse the last few steps as if in fear of success; others will rapidly climb to the top.

This yields clues as to achievement motives, power motives, and so on.

Later the therapist encourages the patient to achieve the task in imagination (i.e., a person who cannot reach the top is enjoined to do so in fantasy).

As a way of getting rid of parental representations (the introjects), Shorr utilizes "body focusing." Here patients are queried in what part of their body their mother or father resides.

Once the body part is named the patient may be asked whether it can be removed and, if so, how this could best be achieved through imagery.

Imagery can be utilized in a group setting.

All members may be asked to close their eyes, imagine throwing a fishing line in the water, and fantasize what they catch.

This stirs up interaction between the members and brings out hidden wishes in each patient.

Should the patient feel improperly defined by a member in the group, verbal confrontation in the group is encouraged.

This may be more easily accomplished through "cathartic imagery," by setting up a psychodramatic scene in which one confronts a group member in the role of a significant personage who harbors a faulty image of the patient.

The therapist sides with the patient and with the corrected definition of the self-image to render support in releasing feelings against fancied retaliation.

Such expressions emerge as, "I am not what you say I am.

How dare you.

Never will you do this to me again," along with expressions of www.freepsychotherapybooks.org 534 outraged emotion.

These reactions are essential for changing the self-definition.

Reaction may vary from verbal outrage to violent screaming.

Shorr claims that his method is faster than analytic methods that depend solely on verbalization since through imagery one can see more clearly what is involved in change.

Psychosynthesis As described by Assagioli (1965), the patient in psychosynthesis is asked, while relaxed on a couch, to visualize and relive, as realistically as possible, the scene or situation that is stimulating his or her upset and to let the emotion come out as freely as possible.

This technique can also be applied to future events that the patient anticipates.

Verbal expression, writing, keeping a diary, and muscular exercises are other modes of discharge for the emotions.

Eclecticism of method is endorsed.

Tien (1972) defines a psychosynthesist as a psychiatrist who fuses "classical psychoanalysis, modern chemotherapy, improved electroconvulsive therapy, family therapy, group therapy and community psychiatry with television as the storage, amplification and feedback system." The electroconvulsive therapy (ELT) is for the purpose of erasing disturbing memories or information.

As in any other eclectic system, the practitioners of psychosynthesis design their own unique

combination of modalities.

Miscellaneous Guided Imagery Techniques There are other ways of working with guided imagery and a number of techniques have been evolved, including those taken from behavior therapy, hypnoanalysis, hypnogogic and “affective” imagery, eidetic analysis, and “in-the-body” daydreams.

In the behavioral technique of systematic desensitization a hierarchy of progressively more noxious phobic situations are constructed and the patient is gradually trained to fantasize and tolerate these situations from lowest to highest traumatic significance (Dyckman & Cowan, 1978).

Implosive therapy also depends on mobilization of vivid exaggerated fantasies (Wolpin, 1969).

In hypnoanalysis the patient is enjoined to visualize special scenes in order to elicit associations, dreams, and emotions (Wolberg, LR, 1964a; Rochkind & Conn, 1973).

In eidetic analysis, a method developed by Ahsen (1968), a series of images involving the patient’s parents in various situations are proposed and the patient is asked to tell a story about what he or she feels and sees (Ahsen, 1972).

Hypnogogic imagery is a normal phenomenon appearing in some people spontaneously in a drowsy state.

It has the same dynamic significance as dreams.

Images may be induced in subjects by relaxation exercises, meditation, autogenic training, hypnotic induction, and sensory deprivation during which an alteration of consciousness comes about.

A form has been described by Leuner (1969) as “symbol-drama-guided affective imagery.” Here the patient lies down on a couch or sits comfortably in a reclining chair and is exposed to relaxing suggestions.

The patient is told to picture himself or herself on a meadow and to describe what he or she sees and feels.

The patient is then asked to take a walk in these surroundings and to describe what he or she sees.

Thereafter the patient is enjoined to climb a mountain, to follow a stream, to picture and explore a house, to visualize family members, to see a rose bush and imagine a car coming by and stopping, to see a lion, and a number of other scenes that will bring up a variety of themes and associations.

A passive attitude on the part of the therapist is usually adopted unless there is a block when active “symbol-confrontation” is employed.

This technique is highly productive in dynamically oriented psychotherapy (Kosbab, 1974).

The directive “in-the-body daydream” focuses on bodily tensions and may travel to different zones with many rich fantasies about what is going on (Alexander ED, 1971).

The “Flomp Method” consists of daydreaming about a situation or event from one’s life where one was disappointed and/or afraid.

The patient is taught a relaxation method and asked to see himself or herself acting in the situation with the knowledge, power, and maturity that he or she now possesses “to make the done undone.” Other situations are suggested and a variety of techniques of exploration utilized (Hagelin & Lazar, 1973).

Criticism of Guided Imagery As part of an active psychoanalytically oriented psychotherapy, guided imagery may contribute substantially in the treatment of those patients who are capable of visualizing and working with fantasies and daydreams. Not all patients are proficient in doing this.

Moreover, in borderline or schizophrenic states imagery techniques may liberate explosive

emotions and even precipitate acute, though usually temporary, psychotic outbreaks. Reliance on guided imagery as the chief or only technique will bring many disappointments. It must be reinforced by other modes of operation.

The data emerging from imagery should never be taken at its face value since distortion, secondary elaboration, and defensive rationalization are common.

ERICKSONIAN PSYCHOTHERAPY (STRATEGIC THERAPY) "Ericksonian psychotherapy" is the name of a body of interventions most of which have been taken from the lectures, seminars, workshops, and writings of Milton H.

Erickson, perhaps the most outstanding practitioner of hypnosis in the United States.

More important than the actual techniques is the philosophy behind the methods, as well as the tactical interpersonal approaches to the patient, which are designed to liberate potentials for self-help in either the hypnotic or waking state (Erickson & Rossi, 1980; Haley, 1973).

Discounting the myths and anecdotes, that are so commonly expressed by devotees and detractors of any charismatic figure, Erickson has had a significant influence on thousands of professionals and is registering an imprint on American psychotherapy itself.

This is evidenced by the many volumes about Erickson that have been and continue to be published (Hammond, 1984; Rossi & Ryan, 1985; Rossi et al., 1983; Zeig, 1980, 1982, 1985a, b).

Some of Erickson's interventions were an outcome of coping techniques that he employed to moderate the pain and disabilities he suffered as a result of having had childhood poliomyelitis.

Struggles with his handicaps made for a unique blend of resourcefulness, flexibility, ingenuity, artfulness, and improvisation which, combined with an unorthodox style and a penchant for brinksmanship have created a model of psychotherapy that is exciting to read about but difficult for the traditional therapist to duplicate.

There are, nevertheless, lessons to be learned from the dexterous ways Erickson related to his patients and even from the dramatic, histrionic expediences devised by this talented innovator. www.freepsychotherapybooks.org 537 Acting variantly with each patient as counselor, analyst, referee, arbiter, advocate, prompter, mentor, accepting authority, or punitive parent, Erickson stressed the uniqueness of each individual, who, motivated by singular needs and idiomatic defenses, required an original mode of approach rather than orthodox, unimaginative, and doctrinal styles.

He considered himself and his words, intonations, manner of speaking, and body movements vehicles of influence that could promote change.

Interested in action rather than theory, he considered traditional theory a handicap that anchored therapists to a bedrock of hopeless imponderables.

Toward this end he suggested, cajoled, and maneuvered with a host of individual, multilevel communicative thrusts, verbal and nonverbal, that were fabricated to influence the patient without the latter's full awareness of being manipulated.

Sometimes he failed, but this merely provided him with new incentives to overcome the patient's reluctance to utilize latent resources and potentials for change.

Frequently he would join the manifest resistance and seemingly side with the patient's illness and defenses, or he would assign the patient what appeared to be peculiar, irrelevant tasks. He would offer homespun advice and common-sense remedies that made use of the obvious. Conversely, he would use metaphors and obtuse inferences that were not exactly to the point. He would set up situations "where people would spontaneously realize their previously unrecognized abilities to change" (Zeig, 1985).

But there was a design in these contrivances if no more than to confuse patients enough to force them to open their minds to a different way of looking at things.

Techniques were not selected in advance but were tailored to the exigencies of the immediate

situation.

Even though Erickson refused to identify himself with any of the well-known schools of psychotherapy, he often used behavioral, cognitive, analytic, and other methodologies within the framework of his unique modes of operation.

Hypnosis was employed when it was considered useful in expediting therapy.

His immediate objective was symptom relief and problem solving, although personality and value change were considered ideal goals that might sooner or later be achieved.

Some psychotherapists worship Erickson with a reverence that borders on idolatry.

Every word, sentiment, opinion, or act are presumed to have an inspired meaning.

Such deification, rooted in expectations of timeless power and omnipotence, can ultimately lead www.freepsychotherapybooks.org 538 to disillusionment.

Equally prejudiced are those who regard Erickson as a maverick whose egregious methods are a passing fancy that will eventually be consigned to the dustbin of outmoded schemes.

These attitudes do injustice to a highly creative, imaginative, and original mind who evolved novel approaches to some of the most baffling problems in psychotherapy.

Erickson was a marvelous influencing machine, crafted by years of struggle in mastering his painful physical disability.

His courage, sensitivity, perceptiveness, and unique modes of coping made him, in the words of Haley (1973), an "uncommon therapist." But his approaches, which blended with his "uncommon" personality and styles of operation, cannot be easily transposed, digested, and used by others.

A poignant criticism of Erickson's strategic therapy is that it is overvalued by those who believe that clever tactics can substitute for disciplined training.

Technical modes of operation are only a fragment of what goes into the gestalt of a psychotherapeutic program.

For one thing we must know how to deal with a host of variables related to patients' defenses, belief systems, and characterological peculiarities, which can negate and cancel out the effect of all our strategic interventions.

LIFESPRING An entry into the human potential movement is Lifespring, which, inspired by the work of Carl Rogers, Abraham Maslow, and Fritz Perls, aims to stimulate personal growth through "self-awareness and acceptance." The program is not designed for persons with severe emotional problems; nor is it recommended as adjunctive to an established therapeutic program.

Be that as it may, many people attracted to Lifespring seek help for emotional difficulties.

Such help may be forthcoming through the relationships that are established with the trainers and other participants, the emotional catharsis that takes place in "sharing" one's difficulties and opening up to others, the group support, persuasive suggestions, operant conditioning, educational awareness, and other bounties inherent in a helping situation.

The group leaders, who consider themselves trainers rather than psychotherapists, use a variety of techniques such as meditation, role playing, www.freepsychotherapybooks.org 539 guided fantasies, and group discussions.

Many of the graduates of the program maintain contact with each other and the organization, gaining the benefits of an extended family.